

2022 Standards and Guidelines for the

ACCREDITATION OF HEALTH PLANS

Effective July 1, 2022



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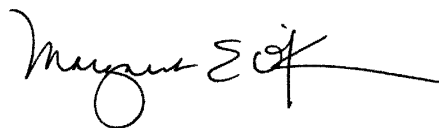
Updating this year's *Standards and Guidelines* would not have been possible without a team effort and invaluable input from so many:

- The staff of NCQA's Accreditation and Recognition Operations, Product Design and Support and Information Systems departments.
- The NCQA Standards Committee.
- The Long-Term Services & Supports Advisory Committee.
- The Committee on Performance Measurement.
- The Review Oversight Committee.
- Surveyors.
- The Consumer Advisory Council.
- The Public Sector Advisory Council.
- The Employer Advisory Council.
- The Health Plan Advisory Council.

NCQA also received suggestions on proposed changes from health plans, purchasers, consumers and policymakers during public comment.

We also want to express our gratitude to the John A. Hartford Foundation, without whose support this work would not be possible.

Sincerely,

A handwritten signature in black ink, appearing to read "Margaret E. O'Kane", with a stylized flourish at the end.

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Overview

Accreditation: A Symbol of Quality and Improvement

Why NCQA?

NCQA Health Plan Accreditation builds on more than 30 years of experience to provide a current, rigorous and comprehensive framework for essential quality improvement and measurement. It is the only program in the industry that bases results on clinical performance and consumer experience (HEDIS^{®1} and CAHPS^{®2}).

NCQA standards are a roadmap for improvement—organizations use them to perform gap analysis and align improvement activities with areas that are most important to states and employers, such as network adequacy and consumer protection. Standards evaluate organizations on:

- Quality Management and Improvement.
- Population Health Management.
- Network Management.
- Utilization Management.
- Credentialing and Recredentialing.
- Members' Rights and Responsibilities.
- Member Connections.
- Medicaid Benefits and Services.

The use of HEDIS data allows the Health Plan Accreditation to effectively measure care and service performance. This focuses attention on activities that keep members healthy.

Organizations choose NCQA Health Plan Accreditation because:

- **Employers want it.** Many employers—especially Fortune 500 employers—do business only with NCQA-Accredited organizations. They and other purchasers want to maximize the value of their health investment by focusing on quality care.
- **It helps meet regulatory requirements.** NCQA Accreditation contains many of the key elements that federal law and regulations require for State Health Insurance and Exchange plans. Forty-three states recognize NCQA Accreditation as meeting their requirements for Medicaid or commercial plans; 29 states mandate it for Medicaid. The Federal Employees Health Benefit Program accepts NCQA Accreditation.
- **Consumers are looking for quality.** The standards focus on key patient protections that consumers, regulators, public purchasers and employers value. 178 million people are enrolled in NCQA-Accredited health plans.
- **It's flexible and comprehensive.** NCQA standards apply to all types of organizations. Annual updates to standards support the evolving needs of regulators and public and private purchasers.

¹HEDIS[®] is a registered trademark of the National Committee for Quality Assurance (NCQA).

²CAHPS[®] is a registered trademark of the Agency for Healthcare Research and Quality (AHRQ).

Notable Changes for 2022**Clarifications to the Policies and Procedures**

Overview

- Added “Prevalidation programs” to the list of NCQA programs.

Section 1

- No notable clarifications in this section.

Section 2

- Clarified scoring in the “Resurvey (*applies to First and Renewal Evaluation Options*)” section.
- Added a new last paragraph explaining that NCQA does not conduct annual corporate surveys if the organization does not have any affiliates seeking corporate credit in the applicable standards year.
- Clarified that a CAP must detail how the organization intends to address identified deficiencies and when the CAP Survey is scheduled in the “CAP Survey” section.
- Clarified that corrective action requests are not specific to failed must-pass elements and are handled outside the formal survey process in the “Corrective Action Requests” section.
- Clarified that organizations may receive a provisional status if it does not receive a score of “Met” in three or more must-pass elements in Table 4.
- Added a bullet under “Scope of review” explaining which elements are scored multiple times (e.g., by program or for delegation oversight purposes) but not by product line in the “How Standards Are Scored” section.
- Added the third bullet under “Note” in the “Must-Pass Element and Corrective Action Plan” section.

Section 3

- Added “Natural Disaster and Cybercrime” subhead and text.

Section 4

- No notable clarifications in this section.

Section 5

- Clarified reporting obligations in “Notifying NCQA of Reportable Events.”

Section 6

- No notable clarifications in this section.

Section 7

- No notable clarifications in this section.

2022 Medicare Advantage Deeming Module

- Clarified that MA organizations may obtain MA Deemed status in any of the following categories:
 - Quality Improvement.
 - Advanced Directives.
 - Antidiscrimination.
 - Confidentiality.
 - Provider Participation.

- Clarified that the following standards are reviewed as part of the MA Deeming Program:
 - MA 1. – MA 7. – MA 16. – MA 22. – SNP 1.
 - MA 2. – MA 9. – MA 19. – MA 23. – SNP 2.
 - MA 4. – MA 11. – MA 20. – MA 24. – SNP 3.
 - MA 5. – MA 12. – MA 21. – MA 25.
- Clarified that the following standards are “Not applicable for MA Deeming”:
 - MA 3. – MA 8. – MA 13. – MA 15.
 - MA 6. – MA 10. – MA 14. – MA 21.
- Clarified that MA organizations must earn an overall score of 100% in each deeming category reviewed to obtain MA deemed status.

Quality Management and Improvement

Standards for Quality Management and Improvement

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QI 1: Program Structure and Operations

Refer to Appendix 1 for points

The organization clearly defines its quality improvement (QI) program structure and processes, assigns responsibility to appropriate individuals and operationalizes its QI program.

Intent

The organization has the QI infrastructure necessary to improve the quality and safety of clinical care and services it provides to its members and to oversee the QI program.

Element A: QI Program Structure

The organization's QI program description specifies:

1. The QI program structure.
2. The behavioral healthcare aspects of the program.
3. Involvement of a designated physician in the QI program.
4. Involvement of a behavioral healthcare practitioner in the behavioral aspects of the program.
5. Oversight of QI functions of the organization by the QI Committee.

Summary of Changes

Deletions

- Eliminated factor 6 and revised the "Met" scoring.

Scoring	Met	Partially Met	Not Met
	The organization meets 5 factors	The organization meets 3-4 factors	The organization meets 0-2 factors
Data source	Documented process		
Scope of review	Product lines		
	<i>For Interim Surveys and First Surveys: This element applies to all product lines.</i>		
	<i>For Renewal Surveys: This element applies to the Medicaid product line only.</i>		
Look-back period	Documentation		
	NCQA reviews the organization's QI program description that is in place throughout the look-back period.		
	<i>For Interim Surveys: Prior to the survey date.</i>		
	<i>For First Surveys: 6 months.</i>		
	<i>For Renewal Surveys: 24 months.</i>		

Explanation This element is a **structural requirement**. The organization must present its own documentation.

The QI program description

The QI program description is a comprehensive document, or a set of interconnected documents, that describes, in plain language, the QI program's governance, scope, goals, measurable objectives, structure and responsibilities.

Factor 1: Program structure

The program description includes the following information about the QI structure:

- The QI program's functional areas and their responsibilities.
- Reporting relationships of QI Department staff, QI Committee and any subcommittee.
- Resources and analytical support.
- Delegated QI activities, if the organization delegates QI activities.
- Collaborative QI activities, if any.
- How the QI and population health management (PHM) programs are related in terms of operations and oversight.

Factor 2: Behavioral healthcare

The program description outlines the organization's efforts to monitor and improve behavioral healthcare.

Factor 3: Involvement of designated physician

The program description describes the role of the designated physician in the QI program, which includes participating in or advising the QI Committee or a subcommittee that reports to the QI Committee.

The physician may be a medical director, associate medical director or other physician assigned this task.

Factor 4: Involvement of designated behavioral healthcare practitioner

The program description specifies the role of the designated behavioral healthcare practitioner in the QI program, which includes participating in or advising the QI Committee or a behavioral healthcare subcommittee that reports to the QI Committee.

The behavioral healthcare practitioner must be a medical doctor or have a clinical PhD or PsyD and may be a medical director, clinical director, participating practitioner from the organization or behavioral healthcare delegate (if applicable).

Factor 5: QI Committee oversight

The program description defines the role, function and reporting relationships of the QI Committee and subcommittees, including committees associated with oversight of delegated activities.

- If participating practitioners are not members of the QI committee, they are involved in a clinical subcommittee or relevant ad hoc task forces.
Note: *Participating practitioners are external to the organization and part of the organization's network.*
- If organization staff are not members of the QI committee, they are involved in multidisciplinary work groups or subcommittees.

Exceptions

This element is NA for Renewal Surveys for the commercial, Medicare and Exchange product lines.

Factors 2 and 4 are NA if all purchasers (e.g., a state) of the organization's services carve out behavioral healthcare services (i.e., contract for a service or function to be performed by an entity other than the organization).

Related information

Collaborative activities. If the organization collaborates with other organizations on QI activities:

- It includes information about the collaborative and QI activities performed in the QI program description.
- It has communication and feedback mechanisms between the collaborative group and its internal QI Committee.

If the collaborative group has its own QI committee for carrying out functions, the organization may consider it to be a subcommittee of the QI Committee.

Examples None.

Element B: Annual Work Plan

The organization documents and executes a QI annual work plan that reflects ongoing activities throughout the year and addresses:

1. Yearly planned QI activities and objectives.
2. Time frame for each activity's completion.
3. Staff members responsible for each activity.
4. Monitoring of previously identified issues.
5. Evaluation of the QI program.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 5 factors	No scoring option	The organization meets 0-4 factors

Data source Documented process, Reports

Scope of review **Product lines**

This element applies to Interim Surveys, First Surveys and Renewal Surveys for all product lines.

Documentation

For Interim Surveys: NCQA reviews the organization's QI program annual work plan. The work plan may be a plan for the upcoming year.

For First Surveys: NCQA reviews the organization's QI program annual work plan including any documented progress on activities.

	<i>For Renewal Surveys:</i> NCQA reviews the organization's most recent and the previous year's annual work plans including documented progress on activities. NCQA cites two data sources, because a work plan may be considered a documented process for future activities and a report that tracks past activities.
Look-back period	<i>For Interim Surveys:</i> Prior to the survey date. <i>For First Surveys:</i> At least once during the prior year. <i>For Renewal Surveys:</i> 24 months.
Explanation	This element is a structural requirement. The organization must present its own documentation. The work plan is a dynamic document that covers a full year of planned activities and objectives. Factor 1: Yearly planned QI activities and objectives The organization identifies and includes activities in the work plan that address: • Quality of clinical care. • Safety of clinical care. • Quality of service. • Members' experience. Factor 2: Time frame for each activity's completion A time frame for completion must be included for each activity. Time frames must be specific (e.g., date, month, quarter), as opposed to a frequency. Factor 3: Staff responsible for each activity The work plan must specify the staff responsible for activities. Staff are not required to be listed by name; they may be listed by title or role. The list may be limited to activity leads. Factor 4: Monitoring previously identified issues The QI work plan includes periodic or ongoing monitoring of issues identified in prior years that it determines require additional follow-up. Factor 5: Evaluation of the QI program Annual evaluation of the QI program must be listed as a specific activity on the work plan, with a stated time frame and identify the staff responsible for the evaluation. Staff are not required to be listed by name; they may be listed by title or role. Exceptions None.
Examples	None.

Element C: Annual Evaluation

The organization conducts an annual written evaluation of the QI program that includes the following information:

1. A description of completed and ongoing QI activities that address quality and safety of clinical care and quality of service.
2. Trending of measures of performance in the quality and safety of clinical care and quality of service.
3. Evaluation of the overall effectiveness of the QI program and of its progress toward influencing networkwide safe clinical practices.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 2-3 factors	No scoring option	The organization meets 0-1 factors
Data source	Reports		
Scope of review	Product lines <i>This element applies to First Surveys and Renewal Surveys for all product lines.</i>		
	Documentation <i>For First Surveys:</i> NCQA reviews the organization's most recent annual evaluation report. <i>For Renewal Surveys:</i> NCQA reviews the organization's most recent and the previous year's annual evaluation report.		
Look-back period	<i>For First Surveys:</i> At least once during the prior year. <i>For Renewal Surveys:</i> 24 months.		
Explanation	This is a structural requirement. The organization must present its own documentation. The organization annually evaluates how well it met its performance goals and objectives for improving the quality and safety of clinical care and services specified within its QI program description. The organization summarizes its findings in an annual report. Factor 1: Completed and ongoing QI activities The annual evaluation summarizes completed and ongoing QI activities outlined within the QI program description. Factor 2: Trending of QI measure results The annual evaluation includes trending of QI measures results over time and comparison against performance objectives defined within the QI program description. The organization uses charts, graphs or tables, or a combination of these, to display trended data.		

Results do not need to be trended for First Surveys.

Factor 3: Evaluation of effectiveness

The annual evaluation includes the organization's determination of the overall effectiveness of the QI program and its progress in meeting safe clinical practice goals, based on the performance in all aspects of the QI program. The summary of effectiveness addresses:

- Adequacy of QI program resources.
- QI Committee and subcommittee structure.
- Practitioner participation and leadership involvement in the QI program.
- Need to restructure or change the QI program for the subsequent year.

Exceptions

None.

Examples**Annual written evaluation contents**

- The title of each QI program initiative described in the work plan.
- A description of the program.
- The program's major accomplishments.
- Appropriate measures trended over time, including:
 - HEDIS data.
 - CAHPS data.
 - Organization-specific data.
- Barriers to achieving objectives.
- Recommended interventions for overcoming issues and barriers.
- Whether planned yearly activities were completed and objectives were met.
- Whether the QI program will be restructured or changed in the subsequent year.

Element D: QI Committee Responsibilities

The organization's QI Committee:

- 1. Recommends policy decisions.**
- 2. Analyzes and evaluates the results of QI activities.**
- 3. Ensures practitioner participation in the QI program through planning, design, implementation or review.**
- 4. Identifies needed actions.**
- 5. Ensures follow-up, as appropriate.**

Summary of Changes**Clarifications**

- Added a sentence under "Documentation" in the scope of review to clarify that all factors must be performed in each year of the look-back period.
- Revised the language throughout the Explanation to clarify the QI Committee's role in overseeing QI activities.

- Clarified in the Explanation under “Factor 3: Practitioner Participation” that participating practitioners must be representative of the organization’s network.
- Replaced “Factor 1: QI Committee common policy recommendations” with “Factor 1: Common QI Committee recommendations” in the Examples and revised the bulleted text.

Scoring	Met	Partially Met	Not Met
	The organization meets 4-5 factors	The organization meets 3 factors	The organization meets 0-2 factors
Data source	Documented process, Reports		
Scope of review	Product lines <i>This element applies to Interim Surveys, First Surveys and Renewal Surveys for all product lines.</i> Documentation <i>For All Survey Types:</i> Factors 1–5 must be performed in each year of the look-back period. <i>For Interim Surveys:</i> NCQA reviews up to three sets of QI Committee minutes or QI Committee charter and a timeline for operationalizing the committee, if the committee has not met. If three sets of meeting minutes are not available, NCQA reviews all committee minutes within the look-back period. <i>For First Surveys:</i> NCQA reviews up to three sets of QI Committee minutes within the look-back period. If three sets of meeting minutes are not available, NCQA reviews all committee minutes within the look-back period. <i>For Renewal Surveys:</i> NCQA reviews up to three sets of QI Committee minutes for each year of the look-back period. If three sets of meeting minutes are not available, NCQA reviews all committee minutes for each year of the look-back period.		
Look-back period	<i>For Interim Surveys:</i> Prior to the survey date. <i>For First Surveys:</i> 6 months. <i>For Renewal Surveys:</i> 24 months.		
Explanation	This is a structural requirement. The organization must present its own documentation. The organization’s QI Committee oversees the development and implementation of the QI program and QI functions in the organization. Associated committees and subcommittees may also participate in these activities. Factor 1: Policy recommendations The QI Committee recommends and revises, or oversees recommendations and revisions to, policies for effective operation of the QI program and achievement of QI program objectives. Factor 2: Analysis and evaluation of QI activities The QI Committee oversees the analysis and evaluation of the QI program and assesses the results. The committee is not required to be directly involved in		

quantitative and qualitative analysis, but in its oversight role, reviews the analysis and evaluation of QI activities of other committees or staff.

Factor 3: Practitioner participation

The QI Committee facilitates participating practitioner involvement in the QI program activities through attendance and discussion in relevant QI committee or QI subcommittee meetings or on ad hoc task forces.

Participating practitioners are representative of the specialties in the organization's network.

If participating practitioners are not members of the QI committee, they are involved in a clinical subcommittee or relevant ad hoc task force.

***Note:** Participating practitioners are external to the organization and part of the organization's network.*

Factor 4: Identify needed actions

The QI Committee identifies actions to improve quality, prioritizes them based on their significance and chooses which to pursue, or oversees these functions if performed by an associated committee or subcommittee.

Factor 5: Follow-up

The QI Committee reviews and evaluates the organization's actions to determine their effectiveness.

Exceptions

None.

Related information

Collaborative activities. If the organization collaborates with other organizations on QI activities:

- It includes information about the collaborative and QI activities performed in the QI program description.
- It has communication and feedback mechanisms between the collaborative group and its internal QI Committee.

If the collaborative group has its own QI committee for carrying out functions, the organization may consider it to be a subcommittee of the QI Committee.

Examples**Factor 1: Common QI Committee recommendations**

- Approve the QI program description and work plan.
- Approve practitioner availability and access standards.
- Approve selected quality performance measures or initiatives.
- Approve performance goals, objectives and thresholds.
- Approve criteria for complex case management and disease management participation.
- Approve policies and procedures covered in the QI, PHM, NET, UM, CR, ME, LTSS, MED and MA standard categories.
- Establish the composition of and the operating guidelines for QI subcommittees.

Factor 3: Relevant QI activities for practitioner participation

- Develop and review clinical practice guidelines.
- Analyze member experience results.
- Select QI initiatives to improve access to care and services.

Element E: Promoting Organizational Diversity, Equity and Inclusion

The organization:

1. Promotes diversity in recruiting and hiring.
2. Offers training to employees on cultural competency, bias or inclusion.

Summary of Changes

Additions

- This is a new element.

Scoring	Met	Partially Met	Not Met
	The organization meets 1-2 factors	No scoring option	The organization meets 0 factors

Data source Documented process, Materials

Scope of review **Product lines**
This element applies to Interim Surveys, First Surveys and Renewal Surveys for all product lines.

Documentation

For Interim Surveys: NCQA reviews the organization's policies and procedures for promoting diversity in recruiting and hiring and for offering training to employees.

For First and Renewal Surveys: NCQA reviews the organization's policies and procedures for promoting diversity in recruiting and hiring and reviews evidence demonstrating that the organization offers training to employees.

Look-back period *For Interim Surveys:* Prior to the survey date.
For First Surveys and Renewal Surveys: 6 months.

Explanation This element is a **structural requirement**. The organization must present its own documentation.

Factor 1: Promotes diversity in recruiting and hiring

The organization describes how its recruiting and hiring processes promote a diverse workforce.

Note: *This factor is specific to the organization's recruiting and hiring processes and does not apply to practitioner network management.*

Factor 2: Offer training on cultural competency, bias or inclusion

The organization offers at least one training to all employees on cultural competence, bias or inclusion. The organization determines training type and frequency.

Exceptions

None.

Examples**Factor 1: Promoting diversity in recruiting and hiring**

- Require that candidates interviewed for a position in the organization include at least one person from an underrepresented group and one woman.
- Create an inclusive job description:
 - Use gender neutral language.
 - Reduce requirements to “must-haves.”
 - Consider indicating a salary range.
 - Emphasize the organization’s commitment to diversity and inclusion.
- Blind-review resumes.
- Hold hiring decision makers responsible for representation growth (i.e., diversity) within teams and in the organization.
- Dedicate resources to recruiting underrepresented groups (e.g., individuals with disabilities).
- Deploy technology that screens for biases in job descriptions and postings.
- Require interview panels to include interviewers from underrepresented groups or genders.

QI 2: Health Services Contracting

The organization's contracts with individual practitioners and providers, including those making UM decisions, specify that contractors cooperate with its QI program.

Intent

The organization's contracts with practitioners and providers foster open communication and cooperation with QI activities.

Element A: Practitioner Contracts

Contracts with practitioners specifically require that:

1. Practitioners cooperate with QI activities.
2. Practitioners allow the organization to use their performance data.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 2 factors	The organization meets 1 factor	The organization meets 0 factors
Data source	Documented process, Materials		
Scope of review	Product lines <i>For Interim Surveys and First Surveys: This element applies to all product lines.</i> <i>For Renewal Surveys: This element applies to the Medicaid product line only.</i> Documentation <i>For Interim Surveys: NCQA reviews one primary care contract and one specialist contract. The contracts do not need to be executed.</i> <i>For First Surveys and Renewal Surveys: NCQA reviews three active primary care contracts and three active specialist contracts executed within the look-back period. If the contracts do not address the factors, NCQA reviews a practitioner manual or the organization's policies and procedures as an extension of the contract in certain circumstances. Refer to <i>Related information</i>.</i>		
Look-back period	<i>For Interim Surveys: Prior to the survey date.</i> <i>For First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	This is a structural requirement. The organization must present its own documentation. This element applies to contracts between the organization and its practitioners, medical groups, independent practice associations (IPA) and rental networks.		

Providing the contract for review

The organization may attach the entire contract or only the relevant pages from the contract that address this element (e.g., the first page with the practitioner name, text that meets the factors and the signature page). The organization may provide contract documents when it submits the completed survey tool or may have the documents reviewed onsite.

Factor 1: Cooperation with QI activities

Practitioner contracts specify that practitioners cooperate with the organization's QI activities to improve the quality of care and services and member experience. Cooperation includes collection and evaluation of data and participation in the organization's QI programs.

Factor 2: Use of performance data

Practitioner contracts, or contract addenda, state that the organization may use practitioner performance data for quality improvement activities.

Exceptions

This element is NA for Renewal Surveys for the commercial, Medicare and Exchange product lines.

This element is NA for Renewal Surveys for the Medicaid product line if the organization provides evidence that there were no new contracts executed during the look-back period.

Related information

Use of practitioner manual or organization's policies. The organization may use its practitioner manual or policies as evidence of performance against this element in the following circumstances.

- Practitioner contracts specify that the manual or policy is an extension of the contract and that practitioners must abide by the conditions set forth in the contract and in the manual or policy.
- The manual or policy includes the requirements specified in factors 1 and 2.

The organization includes an addendum addressing any factors not included in the contract.

Examples**Factor 2: Use of practitioner performance data**

- Public reporting to consumers.
- Preferred status designation in the network (tiering) for narrow networks.
- Cost sharing for using preferred providers.

Element B: Provider Contracts

Contracts with organization providers specifically require that:

1. Providers cooperate with QI activities.
2. Providers allow the plan to use their performance data.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 2 factors	The organization meets 1 factor	The organization meets 0 factors
Data source	Documented process, Materials		
Scope of review	Product lines <i>This element applies to Interim Surveys and First Surveys for all product lines.</i> Documentation <i>For Interim Surveys:</i> NCQA reviews one hospital contract and one contract from another provider type. The contracts do not need to be executed. <i>For First Surveys:</i> NCQA reviews one active hospital contract and one active contract from another provider type executed within the look-back period. If the contracts do not address the factors, NCQA reviews a provider manual or the organization's policies and procedures as an extension of the contract in certain circumstances. Refer to <i>Related information</i>.		
Look-back period	<i>For Interim Surveys:</i> Prior to the survey date. <i>For First Surveys:</i> 6 months.		
Explanation	This is a structural requirement. The organization must present its own documentation. This element is limited to nonspecialty acute care hospitals. Providing the contract for review The organization may attach the entire contract or only the relevant pages from the contract that address this element (e.g., the first page with the provider name, text that meets the factors and the signature page). The organization may provide contract documents when it submits the completed survey tool or may have the documents reviewed onsite. Factor 1: Cooperation with QI activities Provider contracts specify that providers cooperate with the organization's QI activities, including collection of performance measurement data and participation in the organization's clinical and service measure QI programs.		

Factor 2: Use of performance data

Provider contracts, or contract addenda, state:

- That the organization may use provider performance data for quality improvement activities, **or**
- That providers abide by the organization's policies or provider manual, which contains provisions for the use of provider performance data.

Exceptions

None.

Related information

Use of provider manual or organization policies. The organization may use its provider manual or policies as evidence of performance against this element in the following circumstances.

- Provider contracts specify that the manual or policy is an extension of the contract and that providers must abide by the conditions set forth in the contract, and in the manual or policy.
- The manual or policy includes the required language.

The organization includes an addendum addressing any factors not included in the contract.

Examples**Factor 2: Use of provider performance data**

- Public reporting to consumers.

QI 3: Continuity and Coordination of Medical Care

The organization monitors and takes action, as necessary, to improve continuity and coordination of care across the health care network.

Intent

The organization uses information at its disposal to facilitate continuity and coordination of medical care across its delivery system.

Element A: Identifying Opportunities

The organization annually identifies opportunities to improve coordination of medical care by:

1. Collecting data on member movement between practitioners.
2. Collecting data on member movement across settings.
3. Conducting quantitative and qualitative analysis of data to identify improvement opportunities.
4. Identifying and selecting one opportunity for improvement.
5. Identifying and selecting a second opportunity for improvement.
6. Identifying and selecting a third opportunity for improvement.
7. Identifying and selecting a fourth opportunity for improvement.

Summary of Changes

Clarifications

- Replaced “causal” with “qualitative” in factor 3.
- Added “and select” to the second paragraph in the Explanation.
- Revised the first paragraph under “Factors 4–7: Identifying and selecting opportunities” to clarify that organizations describe their reasons for selecting opportunities.
- Revised the “Partners in Quality” language under “Related information” to clarify that the organization receives automatic credit for identifying and selecting an opportunity for improvement.
- Added “and” to the first bullet under “Related information” to clarify that both bullets are required.
- Clarified in the last bullet in the Examples under “Factor 2: Movement across settings” that organizations may use one of two rates.

Scoring

Met	Partially Met	Not Met
The organization meets 5-7 factors	The organization meets 3-4 factors	The organization meets 0-2 factors

Data source

Reports

Scope of review

Product lines

This element applies to First Surveys and Renewal Surveys for all product lines.

Documentation

NCQA reviews the organization's medical care coordination data, quantitative and qualitative analysis and four opportunities for improvement selected from the data analysis.

For First Surveys: NCQA also reviews the organization's most recent annual data collection, analysis and identification of opportunities report.

For Renewal Surveys: NCQA also reviews the organization's most recent and the previous year's annual data collection, analysis and identification of opportunities reports.

Look-back period

For First Surveys: At least once during the prior year.

For Renewal Surveys: 24 months.

Explanation

Continuity and coordination of medical care services is the facilitation, across transitions and settings of care, of:

- Patients getting the care or services they need, **and**
- Practitioners or providers getting the information they need to provide the care patients need.

The organization uses valid data collection and analysis methodology to identify and select opportunities for improvement.

Factors 1, 2: Collecting data

Care transitions refers to members moving between health care practitioners and across settings as their conditions and care needs change during the course of a chronic or acute illness. The organization collects data to assess coordination of care during care transitions.

Movement of members between practitioners includes the inception or cessation of patient care by a practitioner and coordination of care across practitioners who are concurrently or intermittently providing ongoing care for members.

Member movement across settings usually occurs as a member's health status changes (e.g., moving from home to hospital, moving from the hospital to a rehabilitation facility).

Data collected for factors 1 and 2 are analyzed in factor 3 and result in opportunities for factors 4–7. Because each opportunity in factors 4–7 must address a unique area of measurement, at least four measures must be presented based on the data collected for factors 1 and 2.

Factor 3: Quantitative and qualitative analysis

For initial measurement, the organization conducts an annual quantitative and qualitative analysis of data from factors 1 and 2.

For remeasurement, the organization conducts a quantitative analysis, and conducts a qualitative analysis if quantitative analysis demonstrates that stated goals were not met.

Refer to *Appendix 5: Glossary* for the full definition of and requirements for *quantitative analysis* and *qualitative analysis*.

Factors 4–7: Identifying and selecting opportunities

The organization identifies multiple areas, or measures, for improvement, based on its analysis. The organization selects four opportunities for improvement and describes its reasons for the selections. Opportunities relate directly to findings of the qualitative analysis.

NCQA considers each area, or measure, selected for improvement as one opportunity, and counts only one opportunity per measure for factors 4–7. Opportunities may address areas, or measures, related to transitions or settings and are not required to address both.

Because opportunities for improvement can change with each analysis, the organization may present different opportunities for each annual analysis.

Exceptions

None.

Related information

Partners in Quality. The organization receives automatic credit for identifying and selecting one opportunity if it is an NCQA-designated Partner in Quality.

Collaboration through patient-centered medical home (PCMH) initiative. The use of a medical home initiative meets one opportunity for Element A if:

- The initiative is a direct result of the data collected and the analysis performed to meet factors 1–3, **and**
- The organization provides evidence of active support for the PCMH model during the previous 12 months.

NCQA defines “active support” as any of the following:

- Helping with application fees for NCQA PCMH Recognition (beyond the NCQA program’s sponsor discount).
- Helping practices transform into a medical home.
- Providing other incentives for NCQA PCMH Recognition, such as pay-for-performance.
- Using NCQA PCMH Recognition as a criterion for inclusion in a restricted or tiered network.
- Reporting Recognition status in the physician directory.

The organization may receive credit for a second or third opportunity if it can provide evidence of an analysis to determine whether the medical home initiatives can meet additional opportunities.

Automatic credit does not apply if the organization uses a medical home initiative to meet the requirements.

Use of HEDIS measures. Organizations may use HEDIS results that address movement between practitioners or movement across settings to identify relevant clinical issues. Although a HEDIS measure may be relevant to both movement between practitioners or across settings, each HEDIS measure may result in only one opportunity for factors 4–7.

Note: The use of HEDIS measure results alone may not meet the intent of the element, because the results may not evaluate continuity and coordination of care between practitioners or across settings. However, measure results may be used to identify a continuity and coordination of care area or measure that goes beyond HEDIS results.

Examples**Inpatient settings**

- Hospital:
 - Medical surgical.
 - Rehabilitation.
 - Labor and delivery.
 - Pediatrics.
 - Orthopedics.
- Skilled nursing.
- Extended care facility.
- Inpatient hospice.

Outpatient settings

- Rehabilitation center.
- Physician's office.
- Surgery center.
- Urgent care center.
- Emergency department (ED).
- Hospice.
- Home health.

Factor 1: Movement between practitioners

- Specialist to primary care practitioner.
- Practitioner surveys about communication and coordination issues.
- Consult report completeness.
- Consult reports received.
- HEDIS performance measure data:
 - Comprehensive Diabetes Care (CDC)—Eye Exam rate.

Factor 2: Movement across settings

- Readmission data.
- Inpatient facility to primary care practitioners.
- ED to primary care practitioners.
- Urgent care center to primary care practitioners.
- Inpatient to nursing home or post-acute care setting nursing home.
- ED visit follow-up calls.
- Post-discharge phone calls.
- Home health visits within seven days of inpatient discharge.
- HEDIS performance measure data:
 - Persistence of Beta-Blocker Treatment After a Heart Attack (PBH).
 - Plan All-Cause Readmissions (PCR).
 - Prenatal and Postpartum Care (PPC)—Postpartum rate.

- Use of Opioids From Multiple Providers (UOP).
- Transitions of Care (TRC)—either the *Patient Engagement After Inpatient Discharge* rate or the *Medication Reconciliation Post-Discharge* rate.

Element B: Acting on Opportunities

The organization annually acts to improve coordination of medical care by:

1. Acting on a first opportunity for improvement identified in Element A, factors 4–7.
2. Acting on a second opportunity for improvement identified in Element A, factors 4–7.
3. Acting on a third opportunity for improvement identified in Element A, factors 4–7.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 2-3 factors	The organization meets 1 factor	The organization meets 0 factors
Data source	Documented process, Reports, Materials		
Scope of review	Product lines <i>This element applies to Renewal Surveys for all product lines.</i> Documentation NCQA reviews the organization's most recent and the previous year's reports. Depending on the action taken, NCQA also reviews a documented process or materials.		
Look-back period	<i>For Renewal Surveys: 24 months.</i>		
Explanation	Activities related to coordination between medical care and behavioral healthcare do not meet the intent of this element. Factors 1–3: Acting to improve The organization takes at least one action for each of three distinct prioritized opportunities from Element A. Interventions relate directly to the cause of the deficiency identified in the qualitative analysis. Exceptions None. Related information <i>Selecting actions.</i> Actions are based on three distinct opportunities; however, it is possible that the same action may address more than one identified opportunity. <i>Partners in Quality.</i> The organization receives automatic credit for acting on one opportunity if it is an NCQA-designated Partner in Quality.		

Collaboration through PCMH initiative. The use of a medical home initiative meets one opportunity for Element B if:

- The initiative is a direct result of the data collected and the analysis performed to meet Element A, factors 1–3.
- The organization provides evidence of active support for the PCMH model during the previous 12 months.

NCQA defines “active support” as any of the following:

- Helping with application fees for NCQA PCMH Recognition (beyond the NCQA program’s sponsor discount).
- Helping practices transform into a medical home.
- Providing other incentives for NCQA PCMH Recognition, such as pay-for-performance.
- Using NCQA PCMH Recognition as a criterion for inclusion in a restricted or tiered network.
- Reporting Recognition status in the physician directory.

The organization may receive credit for a second or third opportunity if it provides evidence of an analysis to determine whether the medical home initiatives can meet additional opportunities.

Automatic credit does not apply if the organization uses a medical home initiative to meet the requirements.

Examples

Taking action

- Prompt specialists to send summaries of recommendations to practitioners who provide primary care services.
- Educate inpatient discharge planners or home health agencies on the use of discharge instructions.
- Use an ED case manager to follow up with members after an ED visit.
- Provide online tool kits and communicate their availability to practitioners.

Element C: Measuring Effectiveness

The organization annually measures the effectiveness of improvement actions taken for:

1. The first opportunity identified in Element B.
2. The second opportunity identified in Element B.
3. The third opportunity identified in Element B.

Summary of Changes

Clarifications

- Replaced “the rate of member contacts that resulted in scheduled appointments” with “the ‘percentage of member contacts that resulted in scheduled appointments’” in the “Intermediate measures” Example.

Scoring	Met	Partially Met	Not Met
	The organization meets 2-3 factors	The organization meets 1 factor	The organization meets 0 factors

Data source Reports

Scope of review **Product lines**

This element applies to Renewal Surveys for all product lines.

Documentation

NCQA reviews the organization’s most recent and the previous year’s measurement of effectiveness.

Look-back period *For Renewal Surveys: 24 months.*

Explanation **Measurement cycle**

To meet each factor, the organization must measure the effectiveness of its actions from each of the two annual cycles (e.g., remeasure twice). A baseline measurement, analysis and intervention must precede each remeasurement; therefore, the factor requires two full cycles of measurement, analysis and intervention, followed by one additional remeasurement and analysis.

Measuring effectiveness

The organization may measure effectiveness by repeating the original measurement or may use defined variables to measure performance of identified issues, collecting data on one of the following:

- Activities.
- Events.
- Occurrences.
- Outcomes.

Measures are based on standards of care or practice guidelines that include objective clinical criteria from authoritative sources, such as:

- Clinical literature.
- Consensus panels.

- HEDIS measures.
- Measures that are part of the organization's ongoing monitoring.

Measures may be designed for a focused study or for an activity targeted to improve a process of care.

Methodology

The organization uses valid methodology that allows for comparison to the initial measurement or direct measurement of the impact of an action, defining the following for each measure:

- The numerator and denominator.
- The sampling methodology.
- The sample size calculation.
- The measurement periods and seasonality effects.

Quantitative and qualitative analysis

For remeasurement, the organization conducts quantitative analysis and conducts qualitative analysis if quantitative analysis demonstrates that stated goals were not met.

Refer to *Appendix 5: Glossary* for the full definition of and requirements for *quantitative analysis* and *qualitative analysis*.

Exceptions

None.

Related information

Partners in Quality. The organization receives automatic credit for acting on one opportunity if it is an NCQA-designated Partner in Quality.

Collaborative activities. The organization receives credit in Element C for use of a PCMH initiative, for the conditions for which it received credit in Elements A and B. Data collection, analysis and measure of effectiveness may occur in the aggregate at the collaborative group level under the following conditions:

- The organization is involved in the collaborative efforts.
- The collaborative measure is relevant to the organization's population.
- If a sample is used, it is representative of the collaborative organizations but is not required to be stratified for each organization. All eligible members from each participating organization are eligible for the sampling frame.
- Opportunities for improvement can be identified at the collaborative level for all participating organizations.
- Collaborative interventions are performed in a pilot program that is made available to the entire eligible population.

Intermediate measures. The organization may evaluate effectiveness of an intervention by using the same measure specification used for the initial measurement to remeasure, or by conducting an intermediate measurement. An intermediate measurement can evaluate processes or outcomes related to the intervention.

Examples *Intermediate measure.* Organization A's goal is to increase its screening rate by encouraging members to schedule a screening appointment. Its intermediate measurement of effectiveness is the "percentage of member contacts that resulted in scheduled appointments." The organization will wait to measure the intervention's success at increasing screening until the next annual measurement cycle.

Element D: Transition to Other Care

The organization helps with members' transition to other care when their benefit ends, if necessary.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement

Data source Documented process, Reports, Materials, Records or files

Scope of review **Product lines**
This element applies to First Surveys and Renewal Surveys for all product lines.

Documentation

NCQA reviews a documented process and evidence that the organization helped members transition to other care, as needed, throughout the look-back period.

Look-back period *For First Surveys: 6 months.*
For Renewal Surveys: 24 months.

Explanation **Exhausted benefits**
 If covered benefits are exhausted while a member continues to need care, the organization notifies the member about alternatives and resources for continuing care and how to obtain it, as appropriate. The organization is not required to develop alternative resources.

Exceptions

None.

Examples **Identifying members whose benefit ended**
 The organization identifies qualified individuals using daily case manager reports or requests for extension of needed services that were denied due to benefit limitations.

QI 4: Continuity and Coordination Between Medical Care and Behavioral Healthcare

The organization collaborates with behavioral healthcare practitioners to monitor and improve coordination between medical care and behavioral healthcare.

Intent

The organization collaborates with behavioral healthcare practitioners and uses information at its disposal to coordinate medical care and behavioral healthcare.

Element A: Data Collection

The organization annually collects data about opportunities for collaboration between medical care and behavioral healthcare in the following areas:

1. Exchange of information.
2. Appropriate diagnosis, treatment and referral of behavioral disorders commonly seen in primary care.
3. Appropriate use of psychotropic medications.
4. Management of treatment access and follow-up for members with coexisting medical and behavioral disorders.
5. Primary or secondary preventive behavioral healthcare program implementation.
6. Special needs of members with severe and persistent mental illness.

Summary of Changes

Clarifications

- Removed the second paragraph under “Factor 3: Appropriate use of psychotropic medications” because this information is in the Examples.
- Added a paragraph to the Explanation under “Factor 4: Management of coexisting medical and behavioral conditions” to clarify the factor’s intent.
- Added a new second bullet in the Examples under “Factor 4: Managing coexisting conditions.”
- Removed the third bullet in the Examples under “Factor 6: Severe and persistent mental illness,” which read, “Results of the HEDIS measure Metabolic Monitoring for Children and Adolescents on Antipsychotics (APM).”

Scoring

Met	Partially Met	Not Met
The organization meets 4-6 factors	The organization meets 2-3 factors	The organization meets 0-1 factors

Data source Reports, Materials

Scope of review **Product lines**

This element applies to First Surveys and Renewal Surveys for all product lines.

Documentation

For First Surveys: NCQA reviews the organization's evidence of collaboration (e.g., joint meeting minutes, communications) on data collection and most recent annual data collection report.

For Renewal Surveys: NCQA reviews the organization's evidence of collaboration (e.g., joint meeting minutes, communications) on data collection and most recent and previous year's annual data collection reports.

Look-back period

For First Surveys: At least once during the prior year.

For Renewal Surveys: 24 months; at least once during the prior year for factor 1.

Explanation

The organization demonstrates collaboration between its behavioral healthcare delivery system and medical care delivery system.

Collaboration with a managed behavioral healthcare organization (MBHO) for this element is not considered delegation.

Factor 1: Exchange of information

The exchange of information is bidirectional. The organization collects data on exchange of information between behavioral healthcare and relevant medical delivery systems (e.g., medical/surgical specialists, organizational providers) measuring any or all of the following:

- Accuracy of information.
- Sufficiency of information.
- Timeliness of information.
- Clarity of information.
- Frequency of receiving information.

The organization meets the requirements of factor 1 if medical and behavioral healthcare practitioners can access each other's notes through a fully integrated electronic medical record (EMR). NCQA considers an EMR to be fully integrated if it is implemented for all participating medical and behavioral health practitioners.

Factor 2: Diagnosis, treatment and referral of behavioral disorders

The organization collects data on:

- Behavioral disorders that may have been misdiagnosed or treated improperly, **or**
- Referrals that were unnecessary, too early, too late or to the incorrect type of behavioral healthcare practitioner.

Factor 3: Appropriate use of psychotropic medications

The organization collects data on behavioral and medical practitioner adherence to prescribing guidelines.

Factor 4: Management of coexisting medical and behavioral conditions

The organization collects data on issues around management of multiple conditions where there are both medical and behavioral health conditions and management across the continuum of care is an issue.

The intent is to collect data on both treatment access and follow-up services for members with coexisting medical and behavioral conditions.

Factor 5: Prevention programs for behavioral healthcare

The organization collects data on issues that could be preventable if appropriate primary or secondary programs were developed and implemented. The organization identifies the programs that the collaboration deems most appropriate, but is not required to implement the program to meet the element.

Factor 6: Severe and persistent mental illness

The organization collects data on specific issues around the continuity and coordination of services for members with severe and persistent mental illness (SPMI).

Exceptions

This element is NA if all purchasers of the organization's services carve out or exclude behavioral healthcare.

Factor 6 is NA if the organization's membership does not include adult members.

Related information

Use of HEDIS measures. Organizations may use HEDIS results that address collaboration between behavioral healthcare and medical care to identify relevant clinical issues. Although a HEDIS measure may be relevant for more than one factor, the results of any HEDIS measure may be used for only one factor.

Note: The use of HEDIS measure results alone may not meet the intent of the element, because the results may not evaluate continuity and coordination of care between practitioners or across settings. However, measure results may be used to identify a continuity and coordination of care area or measure, that goes beyond HEDIS results.

Examples**Factor 1: Exchange of information**

- Surveys of behavioral healthcare practitioners and other practitioners regarding information exchanged.
- Evaluation of solicited or unsolicited practitioner reports on communication between behavioral healthcare practitioners and medical practitioners, including protection of privacy.

Factor 2: Diagnosis, treatment and referral

- Data on the use of primary care guidelines for treating or making referrals for treatment of problems such as eating disorders, depression, postpartum depression, substance abuse or attention deficit disorder.
- Results of the HEDIS measure Antidepressant Medication Management (AMM).
- Results of the HEDIS measure Follow-Up Care for Children Prescribed ADHD Medication (ADD).

Factor 3: Psychotropic medication use

- Analysis of pharmaceutical utilization data for appropriateness of a psychopharmacological medication.
- Evaluation of psychotropic medication utilization data on issues related to multiple-prescribing practitioners.
- Results of the HEDIS measure Antidepressant Medication Management (AMM).
- Results of the HEDIS measure Follow-Up Care for Children Prescribed ADHD Medication (ADD).

- Results of the organization's documented process for approving use of psychopharmacological medications in specific situations.
- Results of technology assessment to evaluate emerging psychopharmacological medications.

Factor 4: Managing coexisting conditions

- Data on the frequency of behavioral healthcare consultations for medical or surgical inpatients with secondary or tertiary mental health or substance abuse diagnoses.
- Data on the frequency of treatment and follow-up visits following mental health or substance abuse diagnoses.
- Pharmaceutical data on medication interactions to assess coordination of coexisting medical and behavioral problems.
- Results of the HEDIS measure Cardiovascular Monitoring for People With Cardiovascular Disease and Schizophrenia (SMC).
- Results of the HEDIS measure Diabetes Monitoring for People With Cardiovascular Diseases and Schizophrenia (SMD).
- Results of the HEDIS measure Diabetes Screening for People With Schizophrenia or Bipolar Disorder Who Are Using Antipsychotic Medications (SSD).

Factor 5: Primary preventive programs

- Data on the need for a prevention of substance abuse program.
- Data on the need for a parenting skills training program.
- Data on the need for a bereavement counseling program.
- Data on the need for a nutritional and body image program for adolescents.
- Data on the need for a stress management program for adults.

Factor 5: Secondary preventive programs

- Data on the need for developmental screening of children in primary care settings.
- Data on the need for ADHD screening of children in primary care settings.
- Data on the need for screening for eating disorders in adolescents in primary care settings.
- Data on the need for behavioral health consultations for members hospitalized for targeted medical or surgical conditions that are known to be associated with behavioral complications or comorbidities.
- Data on the need for postpartum depression screening.

Factor 6: Severe and persistent mental illness

- Results of the HEDIS measure Diabetes Screening for People With Schizophrenia or Bipolar Disorder Who Are Using Antipsychotic Medications (SSD).
- Results of HEDIS measure Diabetes Monitoring for People With Diabetes and Schizophrenia (SMD).
- Results of HEDIS measure Cardiovascular Monitoring for People With Cardiovascular Disease and Schizophrenia (SMC).
- Data on frequency of assessment of substance use disorders.

Element B: Collaborative Activities

The organization annually conducts activities to improve the coordination of behavioral healthcare and general medical care, including:

1. Collaborating with behavioral healthcare practitioners.
2. Quantitative and qualitative analysis of data to identify improvement opportunities.
3. Identifying and selecting one opportunity for improvement from Element A.
4. Identifying and selecting a second opportunity for improvement from Element A.
5. Taking collaborative action to address one identified opportunity for improvement from Element A.
6. Taking collaborative action to address a second identified opportunity for improvement from Element A.

Summary of Changes**Clarifications**

- Replaced “causal” with “qualitative” in factor 2.
- Revised the first paragraph of the Explanation under “Factors 3, 4: Identifying and selecting opportunities” to clarify that organizations describe their reasons for selecting opportunities.
- Clarified under “Collaboration through patient-centered medical home (PCMH) initiative” under “Related information” that opportunities come from Elements A and B.
- Added “and” to the first bullet under “Related information” to clarify that both bullets are required.

Scoring

Met	Partially Met	Not Met
The organization meets 4-6 factors	The organization meets 3 factors	The organization meets 0-2 factors

Data source Documented process, Reports, Materials

Scope of review**Product lines**

This element applies to First Surveys and Renewal Surveys for all product lines.

Documentation

For First Surveys: NCQA reviews the organization’s evidence of collaboration (e.g., joint meeting minutes, communications) on data collected in Element A, analyses, opportunities and actions.

For Renewal Surveys: NCQA reviews the organization’s most recent and the previous year’s annual collaborations (e.g., joint meeting minutes, communications) on data collected in Element A, analyses, opportunities and actions.

For factors 5 and 6: NCQA reviews reports. Depending on the action taken to address identified opportunities, NCQA may also review a documented process or materials.

Look-back period

For First Surveys: At least once during the prior year.

For Renewal Surveys: 24 months.

Explanation

Factor 1: Collaboration

The organization collaborates with behavioral healthcare practitioners to identify opportunities to improve continuity of care.

Factor 2: Quantitative and qualitative analysis

For initial measurement, the organization conducts quantitative analysis of data and collaborates to conduct qualitative analysis of data.

For remeasurement, the organization conducts quantitative analysis and collaborates to conduct qualitative analysis if quantitative analysis demonstrates that stated goals were not met.

Refer to *Appendix 5: Glossary* for the full definition of and requirements for *quantitative analysis* and *qualitative analysis*.

Factors 3, 4: Identifying and selecting opportunities

Working collaboratively, the organization identifies multiple areas for improvement based on its analysis. The organization selects two opportunities for improvement, and describes its reasons for the selections. Opportunities relate directly to findings of the qualitative analysis.

The organization may identify multiple opportunities per area (e.g., factor) from Element A for factors 3 and 4.

Because opportunities for improvement can change with each analysis, the organization may present different opportunities for each annual cycle.

Factors 5, 6: Acting to improve

The organization acts on at least two opportunities for improvement from Element A to improve coordination of care between medical care and behavioral healthcare during each annual cycle. Interventions relate directly to the cause of the deficiency identified in the qualitative analysis.

Exception

Factors 2–6 are NA if all purchasers of the organization's services carve out or exclude behavioral healthcare. Even if behavioral healthcare is carved out or excluded, the organization must meet factor 1.

Related information

Collaboration through patient-centered medical home (PCMH) initiative. The use of a medical home initiative meets one opportunity for collaboration between medical care and behavioral healthcare in Element B if:

- The initiative is a direct result of the data collected in Element A and the analysis performed to meet Element B, factors 1–2, **and**
- The organization provides evidence of active support for the PCMH model during the previous 12 months.

NCQA defines “active support” as any of the following:

- Helping with application fees for NCQA PCMH Recognition (beyond the NCQA program's sponsor discount).
- Helping practices transform into a medical home.
- Providing other incentives for NCQA PCMH Recognition, such as pay-for-performance.

- Using NCQA PCMH Recognition as a criterion for inclusion in a restricted or tiered network.
- Reporting Recognition status in the physician directory.

The organization can receive credit for a second opportunity if it can provide evidence of an analysis to determine whether the medical home initiatives can meet additional opportunities.

Automatic credit does not apply if the organization uses a medical home initiative to meet the requirements.

Examples

Opportunities for collaboration

- Improve the process for members to authorize sharing of behavioral health information with primary care practitioners.
- Develop or implement primary care guidelines for assessing, treating and referring common behavioral problems.
- Increase nonbehavioral healthcare practitioner satisfaction with feedback from behavioral healthcare practitioners.
- Improve procedures for treating hospitalized members with coexisting medical and behavioral conditions.
- Improve identification and management of elderly members with multiple behavioral healthcare medications and potentially inappropriate dosages.
- Educate primary care practitioners about appropriate indications for referring patients with hyperactivity disorder or depression to behavioral healthcare specialists.
- For Medicaid products in states that carve out behavioral healthcare services, identify methods to improve dialogue between inpatient and aftercare providers.
- Provide tools to facilitate communication between a medical practitioner and the behavioral healthcare practitioner who is treating the medical practitioner's patient.
- Hold quarterly meetings with the behavioral healthcare organization to develop coordination activities.

Documenting collaboration

- QI Committee evaluation of collaborative activities.
- Participation of behavioral healthcare specialists on the QI Committee or on QI teams that report to the QI Committee.
- Joint meetings of organization and MBHO QI Committees.
- Exchange of ideas between the organization and the MBHO QI Committees (e.g., minutes, reports, questionnaires).

Activities for continuity and coordination of care

- Establish a system with behavioral healthcare practitioners for communicating members' prescribed medications to their primary care practitioner.
- Develop and implement a prevention program for a behavioral disorder commonly managed in a primary care setting.
- Place behavioral healthcare practitioners in high-volume or key primary-care settings.

Element C: Measuring Effectiveness

The organization annually measures the effectiveness of improvement actions taken for:

1. The first opportunity identified in Element B.
2. The second opportunity identified in Element B.

Summary of Changes

Clarifications

- Replaced “the rate of member contacts that resulted in scheduled appointments” with “the ‘percentage of member contacts that resulted in scheduled appointments’” in the “Intermediate measures” Example.

Scoring	Met	Partially Met	Not Met
	The organization meets 2 factors	The organization meets 1 factor	The organization meets 0 factors

Data source Reports

Scope of review **Product lines**

This element applies to Renewal Surveys for all product lines.

Documentation

NCQA reviews the organization’s most recent and the previous year’s annual effectiveness report for two opportunities.

Look-back period *For Renewal Surveys: 24 months.*

Explanation **Measurement cycle**

To meet each factor, the organization must measure the effectiveness of its actions from each of the two annual cycles (e.g., remeasure twice). A baseline measurement, analysis and intervention must precede each remeasurement; therefore, the factor requires two full cycles of measurement, analysis and intervention, followed by one additional remeasurement and analysis.

Measuring effectiveness

The organization may measure effectiveness by repeating the original measurement or may use defined variables to measure performance of identified issues, collecting data on one of the following:

- Activities.
- Events.
- Occurrences.
- Outcomes.

Measures are based on standards of care or practice guidelines that include objective clinical criteria from authoritative sources, such as:

- Clinical literature.
- Consensus panels.

- HEDIS measures.
- Measures that are part of the organization's ongoing monitoring.

Measures may be designed for a focused study or for an activity targeted to improve a process of care.

Methodology

The organization uses valid methodology that allows for comparison to the initial measurement or direct measurement of the impact of an action, defining the following for each measure:

- The numerator and denominator.
- The sampling methodology.
- The sample size calculation.
- The measurement periods and seasonality effects.

Quantitative and qualitative analysis

For remeasurement, the organization conducts quantitative analysis and conducts qualitative analysis if quantitative analysis demonstrates that stated goals were not met.

Refer to *Appendix 5: Glossary* for the full definition of and requirements for *quantitative analysis* and *qualitative analysis*.

Exception

This element is NA if all purchasers of the organization's services carve out or exclude behavioral healthcare.

Related information

Collaborative activities. The organization receives credit in Element C for use of a PCMH initiative, for the conditions for which it received credit in Element B. Data collection, analysis and measure of effectiveness can occur in the aggregate at the collaborative group level under the following conditions:

- The organization is involved in the collaborative efforts.
- The collaborative measure is relevant to the organization's population.
- Samples are representative of the collaborative organizations, but are not required to be stratified for each organization. All eligible members from each participating organization are eligible for the sampling frame.
- Opportunities for improvement can be identified at the collaborative level for all participating organizations.

The organization may:

- Use a collaborative-level goal or benchmark.
- Perform practitioner-focused interventions at the collaborative level.
- Perform member-focused interventions that include all eligible members.
- Perform collaborative interventions in a pilot program that is made available to the entire eligible population.

Intermediate measures. The organization may evaluate effectiveness of an intervention by using the same measure specification used for the initial measurement to remeasure, or by conducting an intermediate measurement. An intermediate measurement can evaluate processes or outcomes related to the intervention.

Examples**Intermediate measure**

Organization A's goal is to increase its screening rate by encouraging members to schedule a screening appointment. Its intermediate measurement of effectiveness is "the percentage of member contacts that resulted in scheduled appointments." The organization will wait to measure the intervention's success at increasing screening until the next annual measurement cycle.

QI 5: Delegation of QI

If the organization delegates NCQA-required QI activities, there is evidence of oversight of the delegated activities.

Intent

The organization remains responsible for and has appropriate structures and mechanisms to oversee delegated QI activities.

Element A: Delegation Agreement

The written delegation agreement:

1. Is mutually agreed upon.
2. Describes the delegated activities and the responsibilities of the organization and the delegated entity.
3. Requires at least semiannual reporting by the delegated entity to the organization.
4. Describes the process by which the organization evaluates the delegated entity's performance.
5. Describes the process for providing member experience and clinical performance data to its delegates when requested.
6. Describes the remedies available to the organization if the delegated entity does not fulfill its obligations, including revocation of the delegation agreement.

Summary of Changes

Clarifications

- Revised the first sentence of the second paragraph under "Documentation" to clarify that the process from factor 5 must be included in the organization's documentation.
- Replaced "CAHPS 5.0H" with "CAHPS" in the factor 5 Explanation.

Scoring

Met	Partially Met	Not Met
The organization meets 5-6 factors	The organization meets 3-4 factors	The organization meets 0-2 factors

Data source Materials

Scope of review **Product lines**

This element applies to Interim Surveys, First Surveys and Renewal Surveys for all product lines.

Documentation

NCQA reviews delegation agreements in effect during the look-back period from up to four randomly selected delegates, or reviews all delegates if the organization has fewer than four.

For delegation agreements in place prior to January 1, 2019, the organization may provide documentation that it notified the delegate of the process required in factor 5. This documentation of notification is not required to be mutually agreed upon.

	Delegation agreements implemented on or after January 1, 2019, must include a description of the process required in factor 5.
	The score for the element is the average of the scores for all delegates.
Look-back period	<i>For Interim Surveys and First Surveys:</i> 6 months. <i>For Renewal Surveys:</i> 24 months.
Explanation	This element may not be delegated. This element applies to agreements that are in effect within the look-back period. The delegation agreement describes all delegated QI activities. A generic policy statement about the content of delegated arrangements does not meet this element.

Factor 1: Mutual agreement

Delegation activities are mutually agreed on before delegation begins, in a dated, binding document or communication between the organization and the delegated entity.

NCQA considers the effective date specified in the delegation agreement as the mutually agreed-upon effective date. The effective date may be before or after the signature date on the agreement. If the agreement has no effective date, NCQA considers the signature date (meaning the date of last signature) as the mutually agreed upon effective date.

NCQA may accept other evidence of the mutually agreed-upon effective date: a letter, meeting minutes or other form of communication between the organization and the delegate that references the parties' agreement on the effective date of delegated activities.

NCQA requires submitted evidence for all other delegation factors to consider the same mutually agreed-upon date as the effective date for the delegate's performance of delegated activities.

Factor 2: Assigning responsibilities

The delegation agreement or an addendum thereto or other binding communication between the organization and the delegate specifies the QI activities:

- Performed by the delegate, in detailed language.
- Not delegated, but retained by the organization.
 - The organization may include a general statement in the agreement addressing retained functions (e.g., the organization retains all other QI functions not specified in this agreement as the delegate's responsibility).

If the delegate subdelegates an activity, the delegation agreement must specify that the delegate or the organization is responsible for subdelegate oversight.

Factor 3: Reporting

The organization determines the method of reporting and the content of the reports, but the agreement must specify:

- That reporting is at least semiannual.
- What information is reported by the delegate about QI delegated activities.

- How, and to whom, information is reported (i.e., joint meetings or to appropriate committees or individuals in the organization).

The organization must receive regular reports from all delegates, even NCQA-Accredited or NCQA-Certified delegates.

Factor 4: Performance monitoring

The delegation agreement specifies how the organization evaluates the delegate's performance.

Factor 5: Providing member and clinical data

The organization's delegation agreement describes what the delegate must do to obtain the following information when it is needed or on an ongoing basis:

- *Member experience data:* Complaints, CAHPS survey results or other data collected on members' experience with the delegate's services.
- *Clinical performance data:* HEDIS measures, claims and other clinical data collected by the organization. The organization may provide data feeds for relevant claims data or clinical performance measure results.

The organization may provide the delegate with the data upon request or on an ongoing basis.

Factor 6: Consequences for failure to perform

The delegation agreement specifies consequences if a delegate fails to meet the terms of the agreement and, at a minimum, circumstances that would cause revocation of the agreement.

Exception

This element is NA if the organization does not delegate QI activities.

Examples None.

Element B: Predelegation Evaluation

For new delegation agreements initiated in the look-back period, the organization evaluated delegate capacity to meet NCQA requirements before delegation began.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization evaluated delegate capacity before delegation began	The organization evaluated delegate capacity after delegation began	The organization did not evaluate delegate capacity

Data source Reports

Scope of review **Product lines**

This element applies to Interim Surveys, First Surveys and Renewal Surveys for all product lines.

This element applies if delegation was implemented in the look-back period.

Documentation

NCQA reviews the organization's predelegation evaluation from up to four randomly selected delegates, or reviews all delegates if the organization has fewer than four.

The score for the element is the average of the scores for all delegates.

Look-back period

For Interim and First Surveys: 6 months.

For Renewal Surveys: 12 months.

Explanation

This element may not be delegated.

NCQA-Accredited/Certified delegates

Automatic credit is available for this element if all delegates are NCQA-Accredited health plans or MBHOs, unless the element is NA.

Predelegation evaluation

The organization evaluated the delegate's capacity to meet NCQA requirements within 12 months prior to implementing delegation. The evaluation may include a review of the organization's structure, processes, and staffing in order to determine its capability to perform the delegated function.

NCQA considers the date of the agreement to be the implementation date if the delegation agreement does not include an implementation date.

If the time between the predelegation evaluation and implementation of delegation exceeds the 12 months, the organization conducts another predelegation evaluation.

If the organization amends the delegation agreement to include additional QI activities within the look-back period, it performs a predelegation evaluation for the additional activities.

Exceptions

This element is NA if:

- The organization does not delegate QI activities.
- Delegation arrangements have been in effect for longer than the look-back period.

Examples

Predelegation evaluation

- Site visit.
- Telephone consultation.
- Documentation review.
- Committee meetings.
- Virtual review.

Element C: Review of QI Program

For arrangements in effect for 12 months or longer, the organization:

1. Annually reviews its delegate's QI program.
2. Annually evaluates delegate performance against NCQA standards for delegated activities.
3. Semiannually evaluates regular reports, as specified in Element A.

Summary of Changes**Clarifications**

- Removed "audits" from the scope of review for First Surveys and Renewal Surveys.

Scoring

Met	Partially Met	Not Met
The organization meets 2-3 factors	No scoring option	The organization meets 0-1 factors

Data source

Reports

Scope of review**Product lines**

Factor 1 applies to Interim Surveys for all product lines.

All factors in this element apply to First Surveys and Renewal Surveys for all product lines.

Documentation

NCQA reviews evidence of the organization's review from up to four randomly selected delegates, or all delegates if the organization has fewer than four.

For All Surveys: NCQA reviews the organization's review of the delegate's QI program (factor 1).

For First Surveys: NCQA reviews the organization's most recent annual review, performance evaluation and semiannual evaluation.

For Renewal Surveys: NCQA reviews the organization's most recent and the previous year's annual reviews, performance evaluations and four semiannual evaluations.

The score for the element is the average of the scores for all delegates.

Look-back period

For Interim Surveys and First Surveys: Once during the prior year.

For Renewal Surveys: 24 months.

Explanation

This element may not be delegated.

Automatic credit is available for factor 2 if all delegates are NCQA-Accredited health plans or MBHOs, unless the element is NA.

Factor 1: Review of the QI program

Appropriate organization staff or committee reviews the delegate's QI program. At a minimum, the organization reviews parts of the QI program that apply to the delegated functions.

Factor 2: Annual evaluation

No additional explanation required.

Factor 3: Evaluation of reports

No additional explanation required.

Exceptions

This element is NA if:

- The organization does not delegate QI activities.
- Delegation arrangements have been in effect for less than 12 months.

Factors 2 and 3 are NA for Interim Surveys.

Examples None.

Element D: Opportunities for Improvement

For delegation arrangements that have been in effect for more than 12 months, at least once in each of the past 2 years that delegation has been in effect, the organization identified and followed up on opportunities for improvement, if applicable.

Summary of Changes

- No changes to this element.

Scoring

Met	Partially Met	Not Met
The organization has acted on identified problems, if any, at least once in each of the past 2 years that the delegation arrangement has been in effect	The organization took inappropriate or weak action, or acted only in the past year	The organization has not acted on identified problems

Data source Documented process, Reports, Materials

Scope of review **Product lines**
This element applies to First Surveys and Renewal Surveys for all product lines.

Documentation

NCQA reviews reports for opportunities for improvement, if applicable, from up to four randomly selected delegates, or from all delegates, if the organization has fewer than four, and for evidence that the organization took appropriate action to resolve issues.

For First Surveys: NCQA reviews the organization's most recent annual review and follow-up on improvement opportunities.

For Renewal Surveys: NCQA reviews the organization's most recent and the previous year's annual reviews and follow-up on improvement opportunities.

The score for the element is the average of the scores for all delegates.

Look-back period	<i>For First Surveys:</i> At least once during the prior year. <i>For Renewal Surveys:</i> 24 months.
Explanation	This element may not be delegated. NCQA-Accredited/Certified delegates Automatic credit is available for this element if all delegates are NCQA-Accredited health plans or MBHOs, unless the element is NA. Identify and follow up on opportunities The organization uses information from its predelegation evaluation, ongoing reports or annual evaluation to identify areas of improvement. Exceptions This element is NA if: • The organization does not delegate QI activities. • Delegation arrangements have been in effect for less than 12 months. • The organization has no opportunities to improve performance. – NCQA evaluates whether this conclusion is reasonable, given assessment results.
Examples	None.

Population Health Management

Standards for Population Health Management

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PHM 1: PHM Strategy

Refer to Appendix 1 for points

The organization outlines its population health management (PHM) strategy for meeting the care needs of its member population.

Element A: Strategy Description

The strategy describes:

1. Goals and populations targeted for each of the four areas of focus.*
2. Programs or services offered to members.
3. Activities that are not direct member interventions.
4. How member programs are coordinated.
5. How members are informed about available PHM programs.
6. How the organization promotes health equity.

**Critical factors: These factors must be scored “yes” to score at least “Partially Met.”*

Summary of Changes

Additions

- Added a new factor 6 and explanation to address how the organization promotes health equity.

Clarifications

- Updated the “Met”, “Partially Met”, and “Not Met” scoring options.
- Revised the lookback period for factor 6 to 6 months for Renewal Surveys.
- Added examples for factor 4.
- Replaced all the text with new text under “Factors 1, 2: Goals, target populations, opportunities, programs or services” in the Examples.

Scoring	Met	Partially Met	Not Met
	The organization meets 4-6 factors	The organization meets 3 factors	The organization meets 0-2 factors
Data source	Documented process		
Scope of review	Product lines <i>This element applies to Interim Surveys, First Surveys and Renewal Surveys for all product lines.</i> NCQA reviews and scores this element for each product line brought forward for Accreditation. Documentation NCQA reviews a description of the organization’s comprehensive PHM strategy that is in place throughout the look-back period. The strategy may be fully described in one document or the organization may provide a summary document with references or links to supporting documents provided in other PHM elements. The organization may use a single document to describe a strategy that applies across all product lines if the document also describes differences in strategy to support different populations, by product line.		

Look-back period	<i>For Interim Surveys:</i> Prior to the survey date. <i>For First Surveys:</i> 6 months. <i>For Renewal Surveys:</i> 24 months, 6 months for factor 6.
Explanation	This element is a structural requirement. The organization must present its own materials. Factor 1 is a critical factor; the factor must be scored “yes” for the organization to score at least “Partially Met” on this element. Factors 1, 2: Four areas of focus The organization has a comprehensive strategy for population health management that, <i>at a minimum</i>, addresses member needs in the following four areas of focus: • Keeping members healthy. • Managing members with emerging risk. • Patient safety or outcomes across settings. • Managing multiple chronic illnesses. At a minimum, the description includes the following for each of the four areas of focus: • A goal (factor 1). • A target population (factor 1). • A program or service (factor 2). Goals are measurable and specific to a target population. A program is a collection of services or activities to manage member health. A service is an activity or intervention in which individuals can participate to help reach a specified health goal. Factor 2: Programs and services Programs and services offered to the organization’s members align with its comprehensive strategy and the areas of focus in factor 1. NCQA does not prescribe a specific number of programs or services that must be offered to members, nor does it require all programs and services to be included or limited to each focus area in factor 1. The organization must include a description of the programs and services that align with the goals in its comprehensive PHM strategy. This may include programs and services involving any level of member interactive contact. Factor 3: Activities that are not direct member interventions The organization describes the activities it offers in its PHM strategy, including activities not directed at individual members. The organization has at least one activity in place that supports the PHM strategy. An activity may be specific to one area of focus or apply to more than one area of focus. NCQA does not prescribe a specific number of activities that must be offered to members, nor does it require all activities unrelated to the PHM strategy to be included or limited to each focus area in factor 1. The organization must include a description of all activities that align with the goals in its comprehensive PHM strategy.

Factor 4: Coordination of member programs

The organization coordinates programs or services it directs and those facilitated by providers, external management programs and other entities. The PHM strategy describes how the organization coordinates programs across settings, providers and levels of care to minimize the confusion for members being contacted from multiple sources. Coordination activities are not required to be exclusive to one area of focus and may apply across the continuum of care and to initiatives in other organizations.

Factor 5: Informing members

The PHM strategy describes the organization's process for informing members about all available PHM programs and services, regardless of level of contact. The organization may make the information available on its website; by mail, email, text or other mobile application; by telephone; or in person.

The organization communicates the information to members by mail, telephone or in person.

Factor 6: Promote health equity

The World Health Organization defines health equity as “the absence of unfair and avoidable or remediable differences in health among population groups defined socially, economically, demographically or geographically.”

The organization has a comprehensive PHM strategy that describes its commitment to improving health equity and the actions it takes to promote equity in management of member care. The organization determines how, and in what areas, it will promote health equity, and describes a plan for at least one action.

Exceptions

None.

Examples**Factors 1, 2: Goals, target populations, opportunities, programs or services**Keeping members healthy

Seasonal influenza vaccinations: This program focuses on providing children and adult members with their annual influenza vaccinations.

- *Program Goal:* 55% of members in the target population report receiving annual influenza vaccinations within 12 months of enrollment.
- *Target Population:* Children 5–17; Adults 18 or older with no risk factors.
- *Programs or services:*
 - Distributing free influenza vaccinations at:
 - Community flu clinics (e.g., churches, recreation centers).
 - Mobile and drive-thru clinics.
 - Local pharmacies.
 - Sending email and mail reminders.
 - Radio and TV advertisement reminding the public to get vaccinated.

Managing members with emerging risk

Future Moms: Maternity program focused on providing care and treatment for high-risk maternity cases. The program helps expectant mothers focus on early prenatal interventions, risk assessments and education.

- *Program Goal:* 90% of members ages 18–45 identified as potential high-risk for pregnancy complications will enroll in the program within the next 12 months *from the date of identification*.
- *Target Population:* Members 18–45 with an obstetrics/gynecology claim and other medical or pharmacy data that stratifies them into high risk for pregnancy complications.
- *Programs or services:*
 - Prenatal and newborn education classes.
 - Lactation consultations.
 - Breastfeeding support groups.
 - How to prevent preeclampsia workshops.
 - Literature and follow-up appointments for recovery after emergency cesarean.

Patient safety

Combating Hospital-Acquired Infections: This program focuses on educating members to reduce the rate of hospital-acquired infections.

- *Program Goal:* Improve clinical safety by reducing hospital-acquired infections by 5% over 3 years.
- *Target Population:* Members receiving in-patient surgical procedures.
- *Programs or services:*
 - Distribute educational materials about proper hand hygiene, signs and symptoms of infection and when to seek care.
 - Post-discharge follow-up to confirm members receipt of post-surgical care instructions.

Outcomes across settings

Preventing Hospital Readmissions Program: Provide focused care management services to members identified as high risk for readmission.

- *Program Goal:* 75th percentile of clinical HEDIS® goal of Quality Compass for Ambulatory Follow-Up After Emergency Department Visit for Mental Illness within 6 months.
- *Target Population:* Members 18 or older admitted within the past 90 days who were diagnosed with a behavioral health disorder or mental illness.
- *Programs or services:*
 - Weekly outreach from nurse care manager who:
 - Completes post-discharge assessment and creates member goals and interventions.
 - Performs medication reconciliation and encourages medication adherence.
 - Confirms follow-up appointment.

Managing multiple chronic illnesses

Population Health Management: Designed to help maximize health status, improve health outcomes and reduce health care costs for members diagnosed with asthma (adult), diabetes (types 1 and 2, adult), coronary artery disease, heart failure and chronic obstructive pulmonary disease.

- *Program Goal:* 95% of members with a chronic illness report improved self-management of their illness on patient-reported outcome surveys within 24 months of enrollment.

- **Target Population:** Members 18–65 who have more than one chronic illness, identified through claims, lab and pharmacy data.
- **Programs or services:**
 - 24-hour inbound calling program: Informs members of identified gaps in care and actions to improve overall health.
 - Health Information Profile tool: Dynamic resource that captures and houses a member’s medical history, medication, self-monitoring and control/prevention information.
 - Mobile app with community support networks, condition-specific management education, trigger-tracking and on-demand coaching.

Factor 3: Activities that are not direct member interventions

- Sharing data and information with practitioners.
- Interacting with and integrating delivery systems (e.g., contract with accountable care organizations [ACO]).
- Providing technology support to or integrating with patient-centered medical homes (PCMH).
- Integrating with community resources.
- Value-based payment (VBP) arrangements.
- Collaborating with community-based organizations and hospitals to improve transitions of care from the post-acute setting to the home.
- Collaborating with hospitals to improve patient safety.

Factor 4: Coordination of member programs

- Coordinate care for hospitalized members between inpatient case management and transition of care programs.
- Share data with providers to coordinate closure of care gaps.
- Determine member eligibility in multiple programs and coordinate between programs to avoid duplication of services or member contacts.

Element B: Informing Members

The organization informs members eligible for programs that include interactive contact:

1. How members become eligible to participate.
2. How to use program services.
3. How to opt in or opt out of the program.

Summary of Changes

Clarifications

- Replaced “materials sent to members” with “materials used to communicate with members” under “Documentation” in the scope of review.
- Replaced “informing members” with “communicating the information to members” in the Explanation under “Distribution of materials.”

Scoring

Met	Partially Met	Not Met
The organization meets 2-3 factors	No scoring option	The organization meets 0-1 factors

Data source	Documented process, Materials
Scope of review	Product lines <i>This element applies to Interim Surveys, First Surveys and Renewal Surveys for all product lines.</i> NCQA reviews and scores this element for each program brought forward for Accreditation. The score for this element is the average of the scores for all programs or services. Documentation <i>For Interim Surveys:</i> NCQA reviews the organization's documented process in effect during the look-back period from up to four randomly selected programs or services that involve interactive contact, or reviews all programs if the organization has fewer than four. <i>For First Surveys and Renewal Surveys:</i> NCQA reviews materials used to communicate with members from up to four randomly selected programs or services that involve interactive contact, or reviews all programs if the organization has fewer than four.
Look-back period	<i>For Interim Surveys:</i> Prior to the survey date. <i>For First Surveys:</i> 6 months. <i>For Renewal Surveys:</i> 24 months.
Explanation	This element applies to PHM programs or services in the PHM strategy that require interactive contact with members, including those offered directly by the organization. Interactive contact Programs with interactive contact have two-way interaction between the organization and the member, during which the member receives self-management support, health education or care coordination through one of the following methods: • Telephone. • In-person contact (i.e., individual or group). • Online contact: – Interactive web-based module. – Live chat. – Secure email. – Video conference. Interactive contact does not include: • Completion of a health appraisal. • Contacts made only to make an appointment, leave a message or verify receipt of materials. • Contact made to inform members of the availability of affinity programs (e.g., subsidized gym memberships, device purchases, discounted weight loss subscriptions).

Distribution of materials

The organization distributes information to members by mail, fax or email, or through messages to members' mobile devices, through real-time conversation or on its website, if it informs members that the information is available online through another method listed here. The notice communicating the information to members is available online must include a description specific enough to give readers a clear idea of the topic and the general content and must include a link or direction to the specific information. The organization may group or summarize the information by theme. The organization mails information to members who do not have fax, email, telephone, mobile device or internet access. If the organization uses telephone or other verbal conversations, it provides a transcript of the conversation or script used to guide the conversation.

Factors 1–3: Member information

The organization provides eligible members with information on specific programs with interactive contact.

Exceptions

None.

Related information

Use of organizations that have interactive contact with members. Arrangements with contracted organizations to administer programs within the scope of the PHM strategy are considered delegation of PHM 1, Element B if those organizations perform the function required by this element and all delegation requirements apply, including oversight, even if the PHM functions are not considered delegation because those specific functions are not in the scope of the standards.

Examples

- A summary or notes from a wellness program documenting a discussion between a health coach and a member or caretaker about how the member became eligible for the program, how to use the program and how to opt in or opt out.
- A call script for initial outreach by the organization:

“Hello, <Member>! This is <Care Coordinator> from <Your Organization> and I’m calling because you are eligible for our Diabetes Management Program. We identify members for this program using medical and pharmacy data for services related to your condition. If you choose to participate in this program, you will receive <explain program services>. Are you interested in participating?”

Member agrees: “Great! We will enroll you in the program. You should expect a welcome call from your assigned care manager in the next two business days. At that time, you can discuss how you prefer to communicate: by phone, text or video chat.”

Member declines: “No problem! You have the option to join this program at any time. If you change your mind, please call <Your Organization> Care Management Department at <number> or visit our website at <Your Organization>.org/DM/signup.”

- A notice sent by mail, email, member account portal or text, inviting the member to contact the organization about their enrollment:

Dear Member,

Because you had a recent hospital stay, you have been selected to participate in our Transitions Case Management Program. Sometime in the next three days, a nurse will call you to make sure you understand the instructions you were given when you left the hospital, and to make sure you have an appropriate provider to see for follow-up care. To contact the nurse directly, call 555-555-1234. If you do not want to participate in the Transitions Case Management Program, let us know by calling 555-123-4567.

- *Delegation example*

The organization contracts with ABC PHM Company to provide telephone-based wellness coaching. As part of the arrangement, ABC PHM Company informs members of the information required in this element. Because the standards do not specifically review a wellness coaching function, NCQA does not consider review of the organization's delegation oversight for that function. However, because ABC PHM Company is performing the function required in PHM 1, Element B, the organization has delegated that function, and all delegation requirements apply, including oversight.

PHM 2: Population Identification

The organization systematically collects, integrates and assesses member data to inform its population health management programs.

Intent

The organization assesses the needs of its population and determines actionable categories for appropriate intervention.

Element A: Data Integration

The organization integrates the following data to use for population health management functions:

1. Medical and behavioral claims or encounters.
2. Pharmacy claims.
3. Laboratory results.
4. Health appraisal results.
5. Electronic health records.
6. Health services programs within the organization.
7. Advanced data sources.

Summary of Changes

Clarifications

- Added language to the “Related information” section about when a vendor relationship exists.

Scoring	Met	Partially Met	Not Met
	The organization meets 3-7 factors	The organization meets 2 factors	The organization meets 0-1 factors
Data source	Documented process, Reports, Materials		
Scope of review	Product lines <i>This element applies to Interim Surveys, First Surveys and Renewal Surveys for all product lines.</i> Documentation <i>For Interim Surveys:</i> NCQA reviews the organization’s documented process for the types and sources of integrated data. <i>For First and Renewal Surveys:</i> NCQA reviews reports or materials (e.g., screenshots) for evidence that the organization integrated data types and data from sources listed in the factors. The organization may submit multiple examples that collectively demonstrate integration from all data types and sources, or may submit one example that demonstrates integration of all data types and sources.		
Look-back period	<i>For Interim Surveys:</i> Prior to the survey date. <i>For First Surveys:</i> 6 months. <i>For Renewal Surveys:</i> 24 months.		

Explanation **Data integration** is combining data from multiple sources or databases. Data may be combined from multiple systems and sources (e.g., claims, pharmacy), across care sites (e.g., inpatient, ambulatory, home) and across domains (e.g., clinical, business, operational). The organization may limit data integration to the minimum necessary to identify eligible members and determine and support their care needs.

Factor 1: Claims or encounter data

Requires both medical and behavioral health claims or encounters. Behavioral health claims data are not required if all purchasers of the organization's services carve out behavioral healthcare services.

Factors 2, 3

No additional explanation required.

Factor 4: Health appraisals

The organization demonstrates the capability to integrate data from health appraisals and health appraisals should be integrated if elected by plan sponsor.

Factor 5: Electronic health records

Integrating EHR data from one practice or provider meets the intent of this requirement.

Factor 6: Health service programs within the organization

Relevant organization programs may include utilization management, care management or wellness coaching programs. The organization has a process for integrating relevant or necessary data from other programs to support identification of eligible members and determining care needs. Health appraisal results do not meet this factor.

Factor 7: Advanced data sources

Advanced data sources aggregate data from multiple entities such as all-payer claims systems, regional health information exchanges and other community collaboratives. The organization must have access to the data to meet the intent of this factor.

Exceptions

None.

Related information

The data sources that meet factors 1–6 may not be used to meet factor 7.

A vendor relationship exists if the organization delegates these functions to a NCQA-Prevalidated Health IT solution that receives a designation of "Organization Support."

Examples**Factor 5: EHR integration**

- Direct link from EHRs to data warehouse.
- Normalized data transfer or other method of transferring data from practitioner or provider EHRs.

Factor 6: Health services programs within the organization

- Case management.
- UM programs.
 - Daily hospital census data captured through UM.
 - Diagnosis and treatment options based on prior authorization data.
- Disease management.
- Wellness coaching.
- Health information line.

Factor 7: Advanced data sources

Advanced data sources may require two-way data transfer: The organization and other entities can submit data to the source and can use data from the same source. These include but are not limited to:

- Regional, community or health system health information exchanges (HIE).
- All-payer databases.
- Integrated data warehouses between providers, practitioners, and the organization with all parties contributing to and using data from the warehouse.
- State or regionwide immunization registries.

Element B: Population Assessment

The organization annually:

1. Assesses the characteristics and needs, including social determinants of health, of its member population.
2. Assesses the needs of child and adolescent members.
3. Assesses the needs of members with disabilities.
4. Assesses the needs of members with serious and persistent mental illness (SPMI).
5. Assesses the needs of members of racial or ethnic groups.
6. Assesses the needs of members with limited English proficiency.
7. Identifies and assesses the needs of relevant member subpopulations.

Summary of Changes**Additions**

- Added the following two new factors 5 and 6 and explanation on “assessing the needs of members of racial or ethnic groups and “assessing the needs of members with limited English proficiency” and reordered the factors.

Clarifications

- Updated the “Met” and “Partially Met” scoring options.
- Deleted “at least once during the prior year for factor 2” from the look-back period for Renewal Surveys.
- Revised the Renewal Surveys look-back period to be “at least once during the prior year” for factors 5 and 6.
- Added language to the “Related information” section about when a vendor relationship exists.
- Updated the Healthy People 2030 link in the “Related information” section.
- Added examples for factor 5 and 6.

Scoring	Met	Partially Met	Not Met
	The organization meets 5-7 factors	The organization meets 2-4 factors	The organization meets 0-1 factors
Data source Scope of review	Documented process, Reports Product lines <i>This element applies to Interim Surveys, First Surveys and Renewal Surveys for all product lines.</i> Documentation <i>For Interim Surveys:</i> NCQA reviews the organization’s policies and procedures. <i>For First Surveys:</i> NCQA reviews the organization’s most recent annual assessment reports. <i>For Renewal Surveys:</i> NCQA reviews the organization’s most recent and previous year’s annual assessment reports. Look-back period <i>For Interim Surveys:</i> Prior to the survey date. <i>For First Surveys:</i> At least once during the prior year. <i>For Renewal Surveys:</i> 24 months; at least once during the prior year for factors 5 and 6.		
Explanation	At least annually, the organization uses data at its disposal (e.g., claims, encounters, lab, pharmacy, utilization management, socioeconomic data, demographics) to identify the needs of its population. Factor 1: Characteristics and needs To determine the necessary structure and resources for its PHM program, the organization assesses its population, taking into consideration the characteristics of the population including social determinants of health. At a minimum, the organization assesses social determinants of health. Social determinants of health¹ are economic and social conditions that affect a wide range of health, functioning and quality-of-life outcomes and risks. The organization defines the determinants assessed. Factor 2: Needs of children and adolescents The organization assesses the needs of members 2–19 years of age (children and adolescents). If the organization’s regulatory agency’s definition of children and adolescents is different from NCQA’s, the organization uses the regulatory		

agency's definition. The organization provides the definition to NCQA, which determines whether the organization's needs assessment is consistent with the definition.

Factors 3, 4: Individuals with disabilities and SPMI

Members with disabilities and with serious and persistent mental illness (SPMI) have particularly acute needs for care coordination and intense resource use (e.g., prevalence of chronic diseases).

Factors 5: Members of racial or ethnic groups

The organization may use direct or indirect data collection to assess the racial or and ethnic needs groups of its population. The organization may collect data directly at various points of interaction with members or indirectly from third-party sources. The organization describes needs that may be relevant or specific to member experiences or cultures from identified racial or ethnic groups.

Note: *NET 1, Element A is specific to member needs relative to the network. The assessment may be used in PHM 2, Element B, factor 5 but the organization must go beyond to evaluate member needs in general, not specific to the network.*

Factor 6: Assess the needs of members with limited English proficiency

The organization assesses and describes the needs of its members with limited English proficiency. To assess limited English proficiency, the organization must first collect data on its population's language profile. The organization may use direct or indirect data collection to determine the languages spoken by its members. The organization then utilizes the data to determine the needs of members whose primary language is a language other than English.

Factor 7: Identifying and assessing characteristics and needs of subpopulations

A **subpopulation** is a group of individuals within the membership who share characteristics. The organization uses its assessment of the member population to identify and assess the characteristics and needs of relevant subpopulations. The organization includes at least two relevant subpopulations in its assessment, and considers at least two characteristics or needs for each. The organization's assessment describes how it determined that the subpopulation is relevant to its membership as a whole.

Exception

Factor 2 is NA for the Medicare product line.

Related information

A vendor relationship exists if the organization delegates these functions to a NCQA-Prevalidated Health IT solution that receives a designation of "Organization Support."

¹<https://health.gov/healthypeople/objectives-and-data/social-determinants-health>

Examples**Factors 1, 7: Relevant characteristics**

Social determinants of health include, but are not limited to:

- Resources to meet daily needs.
- Safe housing.
- Local food markets.
- Access to educational, economic and job opportunities.
- Access to health care services.
- Quality of education and job training.
- Availability of community-based resources in support of community living and opportunities for recreational and leisure-time activities.
- Transportation options.
- Public safety.
- Social support.
- Social norms and attitudes (e.g., discrimination, racism, and distrust of government).
- Exposure to crime, violence and social disorder (e.g., presence of trash and lack of cooperation in a community).
- Socioeconomic conditions.
- Residential segregation.
- Language/literacy.
- Access to mass media and emerging technologies.
- Culture.

Other characteristics:

- Natural environment, such as green space (e.g., trees and grass) or weather (e.g., climate change).
- Built environment, such as buildings, sidewalks, bike lanes and roads.
- Worksites, schools and recreational settings.
- Housing and community design.
- Exposure to toxic substances and other physical hazards.
- Physical barriers, especially for people with disabilities.
- Aesthetic elements (e.g., good lighting, trees, benches).
- Multiple chronic conditions or severe injuries.
- Eligibility categories included in Medicaid managed care (e.g., TANF, low-income, SSI, other disabled).
- Nature and extent of carved out benefits.
- Types of Special Needs Plan (SNP) (e.g., dual eligible, institutional, chronic).
- Age.
- Race.
- Ethnicity.
- Language preference.

Factor 3: Individuals with disabilities

Disabilities may include, but are not limited to:

- Vision or hearing loss.
- Ambulatory status.
- Partial or total loss of the use of limbs (quadriplegia, paraplegia).
- Back injury.
- Immune system disorder (e.g., HIV/AIDS, lupus, rheumatoid arthritis).
- Neurological disorder (e.g., multiple sclerosis, cerebral palsy, Parkinson's disease, epilepsy).

Factor 5: Assessment of race, ethnicity

- Direct data collection sources:
 - Enrollment forms, when not prohibited by state law.
 - Complex case management intake forms.
 - Member surveys or focus groups.
- Indirect data collection sources:
 - U.S. Census data on the racial/ethnic composition of the population.
 - Published health statistics, health services research, data provided by plan sponsors or government agencies.
 - Estimation methods such as:
 - Geocoding: Using an individual's home address to infer other information, including race/ethnicity.
 - Surname analysis: Using an individual's last name to infer other information, including race/ethnicity.

Factor 6: Assess the needs of members with limited English proficiency

- Direct data collection sources:
 - Member surveys and focus groups.
 - Usage of translation services by Member Services as an indicator of member language preference.
- Indirect data collection sources:
 - State-level census or community-level data.
 - The Modern Language Association Language Map (http://www.mla.org/map_main).
- Population language profile:
 - 65% of members speak English as their primary language, while 25% speak Spanish and 10% speak Mandarin.

Element C: Activities and Resources

The organization annually uses the population assessment to:

1. Review and update its PHM activities to address member needs.
2. Review and update its PHM resources to address member needs.
3. Review and update activities or resources to address health care disparities for at least one identified population.
4. Review community resources for integration into program offerings to address member needs.

Summary of Changes

Additions

- Added a new factor 3, explanation and examples.

Clarifications

- Updated the scoring for the “Met” category.
- Revised the look-back period for Renewal Surveys to be “at least once during the prior year” for factor 3.

Scoring	Met	Partially Met	Not Met
	The organization meets 3-4 factors	The organization meets 2 factors	The organization meets 0-1 factors
Data source	Documented process, Reports, Materials		
Scope of review	Product lines <i>This element applies to Interim Surveys, First Surveys and Renewal Surveys for all product lines.</i>		
	Documentation <i>For Interim Surveys:</i> NCQA reviews the organization’s policies and procedures. <i>For First and Renewal Surveys:</i> NCQA reviews committee minutes or similar documents showing process and resource review and updates.		
Look-back period	<i>For Interim Surveys:</i> Prior to the survey date. <i>For First Surveys :</i> At least once during the prior year. <i>For Renewal Surveys:</i> 24 months; at least once during the prior year for factor 3.		
Explanation	Factors 1, 2: PHM activities and resources The organization uses assessment results to review and update its PHM strategy, including programs, services, activities and resources to meet member needs. Updates are based on assessment of populations identified in PHM 2, Element B. The organization describes the populations and needs it addresses.		
	Factor 3: Address health care disparities for identified population The organization uses assessment results to identify health care disparities among members of racial or ethnic minority groups and members with limited English proficiency.		

The organization reviews and updates its programs, services, activities or resources to address disparities identified in Element B, factors 5 and 6. The organization reviews and updates at least one program, service, activity or resource, as needed, in response to at least one discovered disparity.

Factor 4: Community resources

The organization connects members with community resources or promotes community programs. Integrating community resources indicates that the organization actively and appropriately responds to members' needs. Community resources correlate with member needs discovered during the population assessment.

Actively responding to member needs is more than posting a list of resources on the organization's website; active response includes referral services and helping members access community resources.

Exceptions

None.

Examples

Factor 2: PHM resources

- Staffing ratios.
- Clinical qualifications.
- Job training.
- External resource needs and contacts.
- Cultural competency.

Factor 3: Address health care disparities for at least one identified population

- Improve pregnancy outcomes for African American women by providing health diaries for prepartum and postpartum women to track their symptoms.
- Reduce food insecurity to improve maternal health for Hispanic women by partnering with local farmers markets.
- Deliver culturally appropriate interventions for American Indian men and women with hypertension.

Factor 4: Community resources and programs

- Population assessment determines a high population of elderly members without social supports. The organization partners with the Area Agency on Aging to help with transportation and meal delivery.
- Connect at-risk members with shelters.
- Connect food-insecure members with food security programs or sponsor community gardens.
- Sponsor or set up fresh food markets in communities lacking access to fresh produce.
- Participate as a community partner in healthy community planning.
- Partner with community organizations promoting healthy behavior learning opportunities (e.g., nutritional classes at local supermarkets, free fitness classes).
- Support community improvement activities by attending planning meetings or sponsoring improvement activities and efforts.

- Social workers or other community health workers that contact members to connect them with appropriate community resources.
- Referrals to community resources based on member need.
- Discounts to health clubs or fitness classes.

Element D: Segmentation

At least annually, the organization segments or stratifies its entire population into subsets for targeted intervention.

1. Segments or stratifies its entire population into subsets for targeted intervention.
2. Assesses for racial bias in its segmentation or stratification methodology.

Summary of Changes

Additions

- Added a new factor 2, explanation, examples and adjusted the scoring to account for the new factor.

Clarifications

- Replaced “a description of the methods used” with “a description of the methods used, which include processes for assessing for racial bias in the methodology” under “Documentation” in the scope of review.
- Revised the look-back period for Renewal Surveys to be “at least once during the prior year” for factor 2.
- Added a “Related information” section about when a vendor relationship exists.

Scoring	Met	Partially Met	Not Met
	The organization meets 1-2 factors	No scoring option	The organization meets 0 factors
Data source	Documented process, Reports		
Scope of review	Product lines		
	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys for all product lines.</i>		
	Documentation		
	NCQA reviews a description of the methods used, which include processes for assessing for racial bias in the methodology.		
Look-back period	<i>For First Surveys:</i> NCQA also reviews the organization’s most recent annual report demonstrating implementation.		
	<i>For Renewal Surveys:</i> NCQA also reviews the organization’s most recent and previous year’s annual reports demonstrating implementation.		
	<i>For Interim Surveys:</i> Prior to the survey date.		
	<i>For First Surveys:</i> At least once during the prior year.		
	<i>For Renewal Surveys:</i> 24 months; at least once during the prior year for factor 2.		

Explanation	Population segmentation divides the population into meaningful subset using information collected through population assessment and other data sources. Risk stratification uses the potential risk or risk status of individuals to assign them to tiers or subsets. Members in specific subsets may be eligible for programs or receive specific services. Segmentation and risk stratification result in the categorization of individuals with care needs at all levels and intensities. Segmentation and risk stratification are means of targeting resources and interventions to individuals who can most benefit from them. Either process may be used to meet this element. The organization explains how it evaluates its methodology for racial bias in its segmentation or stratification methodology. If the organization does not discover racial bias, it states this in the report description. Methodology The organization describes its method for segmenting or stratifying its membership, including the subsets to which members are assigned. The organization may use more than one risk stratification methods to determine actionable subsets. Segmentation and stratification use the findings from the population assessment and data integration (e.g., clinical and behavioral data, population and social needs) to determine programs or services for which members are eligible. Although these methods may include utilization/resource use or cost information. Methods that use only cost information for segmentation and stratification do not meet the intent of this element. Reports The organization provides reports specifying the number of members in each category and the programs or services for which they are eligible. Reports are a “point-in-time” snapshot during the look-back period. Reports reflect the number of members eligible for each PHM program. They display data in raw numbers and as a percentage of the total enrolled member population, and may total more than 100% if members fall into more than one category. PHM programs or services provided to members include, but are not limited to, complex case management. Exceptions None. Related information A vendor relationship exists if the organization delegates these functions to a NCQA-Prevalidated Health IT solution that receives a designation of “Organization Support.”
Examples	Factor 2: Assess for racial bias Evaluate whether algorithms lead to bias (e.g., algorithms are based on costs). Due to unequal access to care, less money is spent on Black patients than on White patients, which leads to racial bias in risk scores.

- Describe testing conducted to evaluate risk score differences between racial groups.
- Evaluate test data sets.

Health Plan A

Subset of Population	Targeted Intervention for Eligible Members	Number of Members	Percentage of Membership
Pregnancy: Over 35 years, multiple gestation	High-risk pregnancy case management	80	5%
Type I Diabetes: Moderate risk	Diabetes management	2,320	20%
Tobacco use	Smoking cessation	580	5%
Behavioral health diagnosis in ages 15-19, rural	Telephone or video behavioral health counseling sessions	1,160	10%
Women of child-bearing age	Targeted women's health newsletter	2,900	25%
No risk factors	Routine member newsletters	1,740	15%
No associated data	None	2,320	20%

Health Plan B

Subset of Population	Targeted Interventions for Eligible Members	Number of Members	Percentage of Membership
Risk Tier 4	Intensive Complex Case Management: 10+ chronic conditions and comorbidities	2,400	7%
Risk Tier 3	Complex Case Management: 7-9 chronic conditions and comorbidities	2,800	8%
Risk Tier 2	Chronic Condition Management Program and 3-5 other comorbidities	1,800	5%
Risk Tier 1	Chronic Condition Management Program and 1-2 other comorbidities	8,800	24%
Risk Tier 0	Routine member newsletters, no conditions	12,200	34%
No associated data	None	8,000	22%

Health Plan C

Risk Score of Population	Targeted Intervention for Eligible Members	Number of Members	Percentage of Membership
Score = >20 (Terminal diagnosis with less than 6-month life expectancy)	Palliative Care	1,000	2%
Score = >20 (More than 20 risk factors, no terminal diagnosis)	Complex Case Management	3,000	6%
Score = 15–19 (15-19 risk factors)	Chronic Condition Management with case manager; social worker referral	5,000	10%
Score = 10–14 (10-14 risk factors)	Chronic Condition Management with case manager; social worker referral	7,000	14%

Risk Score of Population	Targeted Intervention for Eligible Members	Number of Members	Percentage of Membership
Score = 7–9 (7-9 risk factors)	Condition Management app; social worker referral	9,000	18%
Score = 4–6 (4-6 risk factors)	Condition Management app	11,000	22%
Score = 1–3 (1-3 risk factors)	Prevention Education emails	7,000	14%
Score = 0 (0 risk factors or no associated data)	Routine member newsletters	7,000	14%

PHM 3: Delivery System Supports

The organization describes how it supports the delivery system, patient-centered medical homes and use of value-based payment arrangements.

Intent

The organization works with practitioners or providers to achieve population health management goals.

Element A: Practitioner or Provider Support

The organization supports practitioners or providers in its network to achieve population health management goals by:

1. Sharing data.
2. Offering evidence-based or certified decision-making aids.
3. Providing practice transformation support to primary care practitioners.
4. Providing comparative quality information on selected specialties.
5. Providing comparative pricing information on selected services.
6. Providing training on equity, cultural competency, bias, diversity and inclusion.

Summary of Changes

Additions

- Added a new factor 6, explanation, examples and adjusted the scoring to account for the new factor.

Clarifications

- Replaced “third entity” with “third-party entity” for factor 2 under “Documentation” in the scope of review.
- Revised the look-back period for Renewal Surveys to be 6 months for factor 6.
- Replaced “Factor 6: Another activity” subhead and text with “Factor 6: Training on equity, culturally competency, bias, diversity and inclusion” subhead and text in the Explanation and Examples.

Deletions

- Deleted the previous factor 6 (One additional activity to support practitioners or providers in achieving PHM goals).

Scoring	Met	Partially Met	Not Met
	The organization meets 3-6 factors	The organization meets 2 factors.	The organization meets 0-1 factors

Data source Documented process, Reports, Materials

Scope of review **Product lines**

This element applies to Interim Surveys, First Surveys and Renewal Surveys for all product lines.

Documentation

For Interim Surveys: NCQA reviews the organization's description of how it supports practitioners or providers.

For First Surveys and Renewal Surveys: NCQA reviews the organization's description that is in place throughout the look-back period of how it supports practitioners or providers and materials demonstrating implementation. If the support involves sharing or providing information, NCQA may also review reports showing the information.

For factor 2: NCQA reviews materials for evidence that they were developed using established criteria or certified by a third-party entity.

Look-back period

For Interim Surveys: Prior to the survey date.

For First Surveys: 6 months.

For Renewal Surveys: 24 months; 6 months for factor 6.

Explanation

The organization identifies and implements activities that support practitioners and providers in meeting population health goals. Practitioners and providers may include accountable care entities, primary or specialty practitioners, PCMHs, or other providers included in the organization's network. Organizations may determine the practitioners or providers they support.

Factor 1: Data sharing

Data sharing is transmission of member data from the health plan to the provider or practitioner that assists in delivering services, programs, or care to the member. The organization determines the frequency for sharing data.

Factor 2: Evidence-based or certified decision-making aids

Shared decision-making (SDM) aids provide information about treatment options and outcomes. SDM aids are designed to complement practitioner counselling, not replace it. SDM aids facilitate member and practitioner discussion on treatment decisions.

SDM aids may focus on preference-sensitive conditions, chronic case management or lifestyle changes, to encourage patient commitment to self-care and treatment regimens.

SDM aids are certified by a third party that evaluates quality, or are created using evidence-based criteria. If certified, the organization provides information about how, when, under what conditions and to whom certified SDM aids are offered. If created using evidence-based criteria, criteria must be cited. At least one certified or evidence-based SDM aid must be offered to meet the intent.

Factor 3: Practice transformation support

Transformation includes movement to becoming a more-integrated or advanced practice (e.g., ACO, PCMH) and toward value-based care delivery.

The organization provides documentation that it supports practice transformation. Organizations may offer financial incentives, learning collaboratives, Maintenance of Certification (MOC) credits and other methods.

Factor 4: Comparative quality and cost information on selected specialties

The organization provides comparative quality and, if available, cost information about selected specialties to practitioners or providers to help them make referral decisions. The organization provides comparative quality information about selected specialties to practitioners or providers and reports cost information if it is available. Comparative cost information may be cost or efficiency information and may be represented as relative rates or as a relative range.

Cost refers to a financial amount determined using actual unit prices per service or unit prices from a standardized fee schedule

Note: For this factor, “specialties” and “specialty” refers to nonprimary care (specialties other than pediatrics, internal medicine and general or family medicine).

Factor 5: Comparative pricing information for selected services

Price is the actual monetary amount paid for a given service. The organization provides comparative pricing information to practitioners or providers to help them make referral decisions. Comparative pricing information may contain actual unit prices per service or relative prices per service, compared across practitioners or providers.

To meet this requirement, the organization must provide comparative pricing information on at least one service and show that it has provided the information to at least one provider that prescribes the service to members.

Factor 6: Training on equity, culturally competency, bias, diversity and inclusion

The organization provides at least one training to practitioners in its network on topics of health equity, including cultural competence, bias, diversity and inclusion. The organization chooses the training delivery type and frequency.

Exceptions

None.

Related information

Partners in Quality. The organization receives automatic credit for factors 3 and 6 if it is an NCQA-designated Partner in Quality.

The organization must provide documentation of its status.

Practice transformation support. The organization can support its practitioners or providers in meeting their population health management goals through any of the following methods:

- Incentive payments for PCMH arrangement.
- Technology support.
- Best practices.
- Supportive educational information, including webinars or other education sessions.
- Help with application fees for NCQA PCMH Recognition (beyond the NCQA program's sponsor discount).
- Help practices transform into a medical home.

- Provide incentives for NCQA PCMH Recognition, such as pay-for-performance.
- Use NCQA PCMH Recognition as a criterion for inclusion in a restricted or tiered network.

Examples

Factor 1: Data sharing

- Sharing patient-specific data listed below that the practitioner or provider does not have access to:
 - Pharmacy data.
 - ED reports.
 - Enrollment data.
 - Eligibility in the organization's intervention programs (e.g., enrollment in a wellness or complex case management program).
 - Reports on gaps in preventive services (e.g., a missed mammogram, need for a colonoscopy).
 - Claims data indicate if these services were not done; practitioners or staff can remind members to receive services.
 - Claims data.
 - Data generated by specialists, urgent care clinics or other care providers.
 - Lab results.
 - Social determinants of health assessment results.
 - Health appraisal results.
- Methods of data sharing:
 - Transmitted through electronic channels as “raw” data to practitioners who conduct data analysis to drive improved patient outcomes.
 - Practitioner or provider portals that have accessible patient-specific data.
 - Submit data to a regional HIE.
 - Reports created for practitioners or providers about patients or the attributed population.
 - A direct link to EHRs, to automatically populate recent claims for relevant information and alert practitioners or providers to changes in a patient's health status.
 - Secure email or secure file transfer.

Factor 2: Decision-making aids

- Certification bodies:
 - National Quality Forum.
 - Washington State Health Care Authority.
- Methods of data sharing:
 - Email PDF versions of certified or evidence-based shared decision aids to practitioners or providers.
 - Notify providers how they can use the provider portal to access links to evidence-based shared decision aids.
 - A semiannual article in the organization's provider newsletter, describing available evidence-based SDM tools and how, when, under what conditions and to whom the tools should be offered.

Factor 3: Practice transformation support

- Pay application fees for practices to earn PCMH Recognition.
- Supply staff, consultants or resources to help the practice with the transformation process.

- Notify practices that earning PCMH Recognition will make them eligible for pay-for-performance incentives.
- Support or finance efforts to establish a relationship with an accountable care organization (ACO).

Factor 4: Quality and cost information

- Selected specialties:
 - Specialties to which a primary care practitioner refers members most frequently.
- Quality information:
 - Organization-developed performance measures based on evidence-based guidelines.
 - AHRQ patient safety indicators associated with a provider.
 - In-patient quality indicators.
 - Risk-adjusted measures of mortality, complications and readmission.
 - Quality Payment Program (QPP) measures.
 - Non-QPP Qualified Clinical Data Registry (QCDR) measures.
 - CAHPS Clinician and Group Survey.
 - The American Medical Association's Physician Consortium for Performance Improvement (PCPI) measures.
 - Cost information:
 - Relative cost of episode of care.
 - Relative cost of practitioner services.
 - In-office procedures.
 - Relative dollars per member per month (PMPM), overall or by type of service or dollars per procedure.
 - Care pattern reports that include quality and cost information.
 - External cost-comparison databases with information about the potential cost of members seeking care out-of-network.

Factor 5: Pricing information

- Selected services:
 - Services for which the organization has unit price information.
 - Services commonly requested by primary care practitioners that are not conducted in-office.
 - Radiology services.
 - Outpatient procedures.
 - Pharmaceutical costs.
 - In-office procedures performed by specialty practitioners.
- Pricing information:
 - Types of pricing used for comparison:
 - Price ranges or tiers.
 - Bundled payments.
 - Unit price.

Factor 6: Training on equity, culturally competency, bias, diversity or inclusion

- Delivery types:
 - Practitioner portal.
 - Newsletter or other practitioner communications during initial credentialing or recredentialing cycles.
- Practitioner training:
 - U.S. Department of Health and Human Services, Office of Minority Health has free, [continuing education e-learning](#) (Culturally and Linguistically Appropriate Services in Maternal Health Care, Behavioral Health, Oral Health) programs to help health care professionals provide culturally competent care.
- Training provided by Johns Hopkins University of Medicine Office of Diversity, Inclusion and Health Equity:
 - Unconscious Bias Collection (via LinkedIn Learning).

Element B: Value-Based Payment Arrangements

The organization demonstrates that it has a value-based payment (VBP) arrangement(s) and reports the percentages of total payments tied to VBP.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Reports		
Scope of review	Product lines <i>This element applies to First Surveys and Renewal Surveys for all product lines.</i> NCQA reviews and scores this element for each product line brought forward for Accreditation. Documentation <i>For First Surveys and Renewal Surveys:</i> NCQA reviews the VBP worksheet to demonstrate that the organization has VBP arrangements in each product line. Worksheets reflect a continuous 12-month period within the look-back period.		
Look-back period	<i>For First Surveys and Renewal Surveys:</i> At least once during the prior 24 months.		
Explanation	This element may not be delegated. There is broad consensus that payment models need to evolve from payment based on volume of services provided to models that consider value or outcomes. The fee-for-service (FFS) model does not adequately address the importance of non-visit-based care, care coordination and other functions that are proven to support achievement of population health goals.		

The organization demonstrates that it has at least one VBP arrangement by reporting the percentage of total payments made to providers and practitioners associated with each type of VBP arrangement for a continuous 12-month period within the look-back period.

The organization uses the following VBP types, sourced from *CMS Report to Congress: Alternative Payment Models and Medicare Advantage* to report arrangements to NCQA. The organization is not required to use them for internal purposes. If the organization uses different labels for its VBP arrangements, it categorizes them using the NCQA provided definitions.

- *Pay-for-performance*: Payments are for individual units of service and triggered by care delivery, as under the FFS approach, but providers or practitioners can qualify for bonuses or be subject to penalties for cost and/or quality related performance. Foundational payments or payments for supplemental services also fall under this payment approach.
- *Shared savings*: Payments are FFS, but provider/practitioners who keep medical costs below the organization's established expectations retain a portion (up to 100 percent) of the savings generated. Providers/practitioners who qualify for a shared savings award must also meet standards for quality of care, which can influence the portion of total savings the provider or practitioner retains.
- *Shared risk*: Payments are FFS, but providers/practitioners whose medical costs are above expectations, as predetermined by the organization, are liable for a portion (up to 100 percent) of cost overruns.
- *Two-sided risk sharing*: Payments are FFS, but providers/practitioners agree to share cost overruns in exchange for the opportunity to receive shared savings.
- *Capitation/population-based payment*: Payments are not tied to delivery of services, but take the form of a fixed per patient, per unit of time sum paid in advance to the provider/practitioner for delivery of a set of services (partial capitation) or all services (full or global capitation). The provider/practitioner assumes partial or full risk for costs above the capitation/ population-based payment amount and retains all (or most) savings if costs fall below the capitation/population-based payment amount. Payments, penalties and awards depend on quality of care.

Calculating VBP reach

Percentage of payments is calculated by:

- Numerator: The value-based payments, *divided by*
- Denominator: All payments (including FFS).

The percentage of payments reflects 12 months of payments within the look-back period and can be based on allowed amounts, actual payments or forecasted payments.

Types of providers/practitioners

For each type of VBP arrangement, the organization reports a percentage of total payments and indicates the provider/practitioner types included in the arrangement.

Exceptions

None.

Examples**Calculating VBP reach**

The denominator is 12 months of all payments, but if there are 3 months of VBPs in the look-back period, the numerator is the 3 months of VBPs.

PHM 4: Wellness and Prevention

The organization offers wellness services focused on preventing illness and injury, promoting health and productivity and reducing risk.

Intent

The organization helps members identify and manage health risks through evidence-based tools that maintain member privacy and explain how the organization uses collected information.

Element A: Frequency of Health Appraisal Completion

The organization has the capability to administer an health appraisal (HA) annually.

Summary of Changes

Clarifications

- Replaced “adult members” with “members” in the standard intent.
- Clarified that HAs apply to adults only under “Documentation” in the scope of review and in the first paragraph of the Explanation.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement

Data source Documented process, Reports, Materials

Scope of review **Product lines**
This element applies to First Surveys for all product lines.

Documentation

NCQA reviews the organization’s documented process for administering annual HAs, or documentation that the organization administered an annual HA to adult members.

Look-back period *For First Surveys: At least once during the prior year.*

Explanation The organization provides evidence that it has the capability to administer HAs to adult members, even if it does not provide services to any employer or plan sponsor.

Exception

This element is NA for the Medicaid product line if the state conducts its own HA or mandates a tool for the organization to conduct HAs. The organization must present documentation demonstrating the state requirement.

Related information

Use of vendors for HA services. If the organization contracts with a non-accredited/noncertified vendor to provide HA services, it provides access to the vendor’s HA. NCQA does not consider the relationship to be delegation, and

delegation oversight is not required under PHM 7. NCQA evaluates the vendor's HA against the requirements. Refer to *Vendor Relationships* in Appendix 2.

Examples Evidence of capability to administer

- Contracts that specify at least annual administration of the HA.
- Reports that demonstrate at least annual administration of the HA.

Element B: Topics of Self-Management Tools

The organization offers self-management tools, derived from available evidence, that provide members with information on at least the following wellness and health promotion areas:

1. Healthy weight (BMI) maintenance.
2. Smoking and tobacco use cessation.
3. Encouraging physical activity.
4. Healthy eating.
5. Managing stress.
6. Avoiding at-risk drinking.
7. Identifying depressive symptoms.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 5-7 factors	The organization meets 3-4 factors	The organization meets 0-2 factors
Data source	Documented process, Materials		
Scope of review	Product lines <i>This element applies to First Surveys and Renewal Surveys for all product lines.</i> Documentation NCQA reviews the organization's policies and procedures for developing evidence based self-management tools, and reviews the organization's self-management tools. Both must be available throughout the look-back period. If the organization can provide a "test" or "demo" log-on ID, NCQA reviews the organization's performance through that mechanism. If the organization cannot provide a test or demo log-on, NCQA reviews the organization's website or screen shots, supplemented with documents specifying the required features and functions of the site. If screen shots provided include detailed explanations of how the site works, there is no need to provide supplemental documents.		
Look-back period	<i>For First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		

Explanation The organization provides evidence that it can perform all activities required by this element, even if it does not provide services to any employer or plan sponsor.

Self-management tools

Self-management tools help members determine risk factors, provide guidance on health issues, recommend ways to improve health or support reducing risk or maintaining low risk. They are interactive resources that allow members to enter specific personal information and provide immediate, individual results based on the information. This element addresses self-management tools that members can access directly from the organization's website or through other methods (e.g., printed materials, health coaches).

Evidence-based information

The organization meets the requirement of "evidenced-based" information if recognized sources are cited prominently in the self-management tools.

If the organization's materials do not cite recognized sources, NCQA also reviews the organization's documented process detailing the sources used, and how they were used in developing the self-management tools.

Factors 1–7

No additional explanation required.

Exceptions

None.

Related information

Use of vendors for self-management tool services. If the organization contracts with a vendor to provide self-management tools, it provides access to the vendor's self-management tools. NCQA does not consider the relationship to be delegation, and delegation oversight is not required under PHM 7. NCQA evaluates the vendor's self-management tools against the requirements. Refer to *Vendor* in Appendix 2.

Examples

Self-management tools

- Interactive quizzes.
- Worksheets that can be personalized.
- Online logs of physical activity.
- Caloric intake diary.
- Mood log.

PHM 5: Complex Case Management

The organization coordinates services for its highest-risk members with complex conditions and helps them access needed resources.

Intent

The organization helps members with multiple or complex conditions to obtain access to care and services, and coordinates their care.

Element A: Access to Case Management

The organization has multiple avenues for members to be considered for complex case management services, including:

1. Medical management program referral.
2. Discharge planner referral.
3. Member or caregiver referral.
4. Practitioner referral.

Summary of Changes

Clarifications

- Added text to the sixth paragraph in the Explanation to clarify that NCQA reviews communication of referral options to members and practitioners.

Scoring	Met	Partially Met	Not Met
	The organization meets 3-4 factors	The organization meets 2 factors	The organization meets 0-1 factors
Data source	Documented process, Reports, Materials		
Scope of review	Product lines <i>This element applies to Interim Surveys, First Surveys and Renewal Surveys for all product lines.</i> Documentation NCQA reviews the organization's policies and procedures. <i>For First Surveys and Renewal Surveys:</i> NCQA also reviews evidence that the organization has multiple referral avenues in place throughout the look-back period and that it communicates the referral options to members and practitioners at least once during the look-back period.		
Look-back period	<i>For Interim Surveys:</i> Prior to the survey date. <i>For First Surveys:</i> 6 months. <i>For Renewal Surveys:</i> 24 months.		
Explanation	The overall goal of complex case management is to help members regain optimum health or improved functional capability, in the right setting and in a cost-effective manner. It involves comprehensive assessment of the member's condition; determination of available benefits and resources; and development and		

implementation of a case management plan with performance goals, monitoring and follow-up.

NCQA considers complex case management to be an opt-out program: All eligible members have the right to participate or to decline to participate.

The organization offers a variety of programs to its members and does not limit eligibility to one complex condition or to members already enrolled in other organization's program.

Identification is how members are segmented or stratified into the complex case management program using the process described in PHM 2, Element D. Members are considered "identified" on the date when they are assigned to a segment or stratum.

In addition to the process described in PHM 2, Element D, multiple referral avenues can minimize the time between identification of a need and delivery of complex case management services.

The organization has a process for facilitating referrals listed in the factors, even if it does not currently have access to the source. NCQA reviews communication of referral options to members and practitioners.

Factor 1: Medical management program referral

Medical management program referrals include referrals that come from other organization programs or through a vendor or delegate. These may include disease management programs, UM programs, health information lines or similar programs that can identify needs for complex case management and are managed by organization or vendor staff.

Factor 2: Discharge planner referral

No additional explanation required.

Factors 3, 4: Member, caregiver and practitioner referral

The organization communicates referral options to members (factor 3) and practitioners (factor 4).

Exceptions

None.

Examples

Facilitating referrals

- Correspondence from members, caregivers or practitioners about potential eligibility.
- Monthly or quarterly reports, from various sources, of the number of members identified for complex case management.
- Brochures or mailings to referral sources about the complex case management program and instructions for making referrals.
- Web-based materials with information about the complex case management program and instructions for making referrals.

Element B: Case Management Systems

The organization uses case management systems that support:

1. Evidence-based clinical guidelines or algorithms to conduct assessment and management.
2. Automatic documentation of staff ID, and the date and time of action on the case or when interaction with the member occurred.
3. Automated prompts for follow-up, as required by the case management plan.

Summary of Changes

Clarifications

- Added a “Related information” section about when a vendor relationship exists.

Scoring	Met	Partially Met	Not Met
	The organization meets 3 factors	The organization meets 2 factors	The organization meets 0-1 factors
Data source	Documented process, Reports, Materials		
Scope of review	Product lines <i>This element applies to Interim Surveys, First Surveys and Renewal Surveys for all product lines.</i> Documentation NCQA reviews the organization’s documented process. <i>For First Surveys and Renewal Surveys:</i> NCQA also reviews the organization’s complex case management system or annotated screenshots of system functionality. The system must be in place throughout the look-back period.		
Look-back period	<i>For Interim Surveys:</i> Prior to the survey date. <i>For First Surveys:</i> 6 months. <i>For Renewal Surveys:</i> 24 months.		
Explanation	Factor 1: Evidence-based clinical guidelines or algorithms The organization develops its complex case management system using one of the following sources: • Clinical guidelines, or • Algorithms, or • Other evidence-based materials. NCQA does not require the entire evidence-based guideline or algorithm to be embedded in the automated system, but the components used to conduct assessment and management of patients must be embedded in the system.		

Factor 2: Automated documentation

The complex case management system includes automated features that provide accurate documentation for each entry (record of actions or interaction with members, practitioners or providers) and use automatic date, time and user (user ID or name) stamps.

Factor 3: Automated prompts

The complex case management system includes prompts and reminders for next steps or follow-up care.

Exceptions

None.

Related information

A vendor relationship exists if the organization delegates these functions to a NCQA-Prevalidated Health IT solution that receives a designation of “Organization Support.”

Examples None.

Element C: Case Management Process

The organization’s complex case management procedures address the following:

1. Initial assessment of member health status, including condition-specific issues.
2. Documentation of clinical history, including medications.
3. Initial assessment of the activities of daily living.
4. Initial assessment of behavioral health status, including cognitive functions.
5. Initial assessment of social determinants of health.
6. Initial assessment of life-planning activities.
7. Evaluation of cultural and linguistic needs, preferences or limitations.
8. Evaluation of visual and hearing needs, preferences or limitations.
9. Evaluation of caregiver resources and involvement.
10. Evaluation of available benefits.
11. Evaluation of community resources.
12. Development of an individualized case management plan, including prioritized goals and considers member and caregiver goals, preferences and desired level of involvement in the case management plan.
13. Identification of barriers to the member meeting goals or complying with the case management plan.
14. Facilitation of member referrals to resources and a follow-up process to determine whether members act on referrals.
15. Development of a schedule for follow-up and communication with members.
16. Development and communication of a member self-management plan.
17. A process to assess member progress against the case management plan.

Summary of Changes

Clarifications

- Revised the text in the Explanation under “Factor 13: Identification of barriers” to clarify that policies and procedures address barriers to member participation in a case management plan.
- Revised the bullets in the Examples under “Factor 3: Activities of daily living” to align with the Explanation.
- Added a bullet to the Examples under “Factor 7: Cultural needs, preferences or limitations.”

Scoring	Met	Partially Met	Not Met
	The organization meets 12-17 factors	The organization meets 8-11 factors	The organization meets 0-7 factors

Data source Documented process

Scope of review **Product lines**
This element applies to Interim Surveys and First Surveys for all product lines.

Documentation

NCQA reviews the organization’s policies and procedures in place throughout the look-back period.

Look-back period *For Interim Surveys: Prior to the survey date.*
For First Surveys: 6 months.

Explanation This is a **structural requirement**. The organization must present its own documentation.

Assessment and evaluation

Assessment and evaluation each require the case manager or other qualified individual to draw and document a conclusion about data or information collected. If the organization’s CM system automatically generates suggestions, the case manager or other individual must still document their own conclusions. It is not sufficient to just have raw data or answers to questions. Policies describe the process to both collect information and document a summary of the meaning or implications of that data or information to the member’s situation, so that it can be used in the case management plan.

The organization must draw a conclusion for each factor (unless otherwise stated in the explanation). This may be in separate summaries for each factor or in a combined summary, or in a combination of these.

Complex case management policies and procedures state why an assessment might not be appropriate for a factor (e.g., life-planning activities, in pediatric cases). The organization records the specific factor and the reason in the case management system and file.

Factor 1: Initial assessment of member’s health status

Complex case management policies and procedures specify the process for initial assessment of health status, specific to an identified condition and likely comorbidities (e.g., high-risk pregnancy and heart disease, for members with diabetes). The assessment includes:

- Screening for presence or absence of comorbidities and their current status.
- Member's self-reported health status.
- Information on the event or diagnosis that led to the member's identification for complex case management.
- Current medications, including schedules and dosages.

Factor 2: Documentation of clinical history

Complex case management policies and procedures specify the process for documenting clinical history, including:

- Past hospitalization and major procedures, including surgery.
- Significant past illnesses and treatment history.
- Relevant past medications related to the member's condition.

Dates are a necessary component of accurate documentation of the member's clinical history. To the extent possible, the organization collects dates as part of documenting clinical history; however, NCQA does not penalize an organization if a member or other individual providing the information cannot provide dates. If dates are not present in the file, NCQA reviews the organization's complex case management policies and procedures. If the organization has a process for collecting dates as part of the clinical history, NCQA assumes the file does not include dates because the member or other individual giving information did not provide dates. The requirement is not met if the organization does not have a process for collecting dates as part of the clinical history.

Factor 2 does not require assessment or evaluation.

Factor 3: Initial assessment of activities of daily living

Complex case management policies and procedures specify the process for assessing functional status related to at least the six basic ADLs: bathing, dressing, going to the toilet, transferring, feeding and continence.

Factor 4: Initial assessment of behavioral health status

Complex case management policies and procedures specify the process for assessing behavioral health status, including:

- Cognitive functions:
 - The member's ability to communicate and understand instructions.
 - The member's ability to process information about an illness.
- Mental health conditions.
- Substance use disorders.

Factor 5: Initial assessment of social determinants of health

Complex case management policies and procedures specify the process for assessing social determinants of health,² which are economic and social conditions that affect a wide range of health, functioning and quality-of-life outcomes and risks that may affect a member's ability to meet case management goals.

Because social determinants of health are a combination of influences, the organization considers more than one social determinant of health, for a comprehensive overview of the member's health.

²<https://health.gov/healthypeople/objectives-and-data/social-determinants-health>

Factor 6: Initial assessment of life-planning activities

Complex case management policies and procedures specify the process for assessing whether members have completed life-planning activities such as wills, living wills or advance directives, health care powers of attorney and Medical or Physician Orders of Life-Sustaining Treatment (MOLST or POLST) forms.

If life planning activities are determined to be appropriate, the case manager documents what activities the member has taken and what documents are in place. If determined not to be appropriate, the case manager documents the reason in the case management record or file.

Providing life-planning information (e.g., brochure, pamphlet) to all members in case management meets the intent of this factor.

Factor 7: Evaluation of cultural and linguistic needs

Complex case management policies and procedures specify a process for assessing culture and language to identify potential barriers to effective communication or care and acceptability of specific treatments. Policies and procedures also include consideration of cultural health beliefs and practices, preferred languages, health literacy and other communication needs.

Factor 8: Evaluation of visual and hearing needs

Complex case management policies and procedures specify a process for assessing vision and hearing to identify potential barriers to effective communication or care.

Factor 9: Evaluation of caregiver resources

Complex case management policies and procedures specify a process for assessing the adequacy of caregiver resources (e.g., family involvement in and decision making about the care plan) during member evaluation.

Factor 10: Evaluation of available benefits

Complex case management policies and procedures specify a process for assessing the adequacy of health benefits regarding the ability to fulfill a treatment plan. Assessment includes a determination of whether the resources available to the member are adequate to fulfill the treatment plan.

Factor 11: Evaluation of community resources

Complex case management policies and procedures specify a process for assessing eligibility for community resources that supplement those for which the organization has been contracted to provide. At a minimum, these include:

- Community mental health.
- Transportation.
- Wellness organizations.
- Palliative care programs.
- Nutritional support.

Factor 12: Individual case management plan and goals

Complex case management policies and procedures specify a process for creating a personalized case management plan that meets member needs and includes:

- Prioritized goals.
 - Prioritized goals consider member and caregiver needs and preferences; they may be documented in any order, as long as the level of priority is clear.
- Time frames for reevaluation of goals.
 - Time frames are specified in the complex case management plan.
- Resources to be utilized, including appropriate level of care.
- Planning for continuity of care, including transition of care and transfers between settings.
- Collaborative approaches to be used, including level of family participation.

Factor 13: Identification of barriers

Complex case management policies and procedures address barriers to a member receiving or participating in a case management plan. A barrier analysis can assess:

- Language or literacy level.
- Access to reliable transportation.
- Understanding of a condition.
- Motivation.
- Financial or insurance issues.
- Cultural or spiritual beliefs.
- Visual or hearing impairment.
- Psychological impairment.

The organization documents that it assessed barriers, even if none were identified.

Factor 14: Referrals to available resources

Complex case management policies and procedures specify a process for facilitating referral to other health organizations, when appropriate.

Factor 15: Follow-up schedule

Complex case management policies and procedures specify a process for determining if follow-up is appropriate or necessary (for example, after a member is referred to a disease management program or health resource). The complex case management plan contains a schedule for follow-up that includes, but is not limited to:

- Counseling.
- Follow-up after referral to a DM program.
- Follow-up after referral to a health resource.
- Member education.
- Self-management support.
- Determining when follow-up is not appropriate.

Factor 16: Development and communication of self-management plans

Complex case management policies and procedures specify a process for communicating the self-management plan to the member or caregiver (i.e., verbally, in writing). **Self-management plans** are activities that help members manage a condition and are based on instructions or materials provided to them or to their caregivers.

Factor 17: Assessing progress

Complex case management policies and procedures specify a process for assessing progress toward overcoming barriers to care and to meeting treatment goals, and for assessing and adjusting the care plan and its goals, as needed.

Exceptions

None.

Examples**Factor 3: Activities of daily living**

- Dressing.
- Bathing.
- Toileting.
- Eating.
- Transferring (e.g., getting in and out of chairs).

Factor 4: Cognitive functioning assessment

- Alert/oriented, able to focus and shift attention, comprehends and recalls direction independently.
- Requires prompting (cuing, repetition, reminders) only under stressful situations or unfamiliar conditions.
- Requires assistance and some direction in specific situations (e.g. on all tasks involving shifting attention) or consistently requires low stimulus environment due to distractibility.
- Requires considerable assistance in routine situations. Is not alert and oriented or is unable to shift attention and recall directions more than half the time.
- Totally dependent due to disturbances such as constant disorientation, coma, persistent vegetative state or delirium.

Factor 5: Social determinants of health

- Current housing and housing security.
- Access to local food markets.
- Exposure to crime, violence and social disorder.
- Residential segregation and other forms of discrimination.
- Access to mass media and emerging technologies.
- Social support, norms and attitudes.
- Access, transportation and financial barriers to obtaining treatment.

Factor 7: Cultural needs, preferences or limitations

- Health care treatments or procedures that are discouraged or not allowed for religious or spiritual reasons.
- Family traditions related to illness, death and dying.

- Health literacy assessment.
- Dietary restrictions.

Factor 9: Caregiver assessment

- Member is independent and does not need caregiver assistance.
- Caregiver currently provides assistance.
- Caregiver needs training, supportive services.
- Caregiver is not likely to provide assistance.
- Unclear if caregiver will provide assistance.
- Assistance needed but no caregiver available.

Factor 10: Assessment of available benefits

- Benefits covered by the organization and by providers.
- Services carved out by the purchaser.
- Services that supplement those the organization has been contracted to provide, such as:
 - Community mental health.
 - Medicaid.
 - Medicare.
 - Long-term care and support.
 - Disease management organizations.
 - Palliative care programs.

Factor 13: Assessment of barriers

- Does the member understand the condition and treatment?
- Does the member want to participate in the complex case management plan?
- Does the member believe that participation will improve health?
- Are there financial or transportation limitations that may hinder the member from participating in care?
- Does the member have the mental and physical capacity to participate in care?

Source: Lorig, K., Patient Education, A Practical Approach (Thousand Oaks, CA: Sage Publications, 2001) 186–92.

Factor 16: Self-management

- Self-management includes ensuring that the member can:
 - Perform activities of daily living (e.g., transfer/ambulation, bathing, dressing, toileting, eating/feeding).
 - Perform instrumental activities of daily living (e.g., meals, housekeeping, laundry, telephone, shopping, finances).
 - Self-administer medication (e.g., oral, inhaled or injectable).
 - Self-administer medical procedures/treatments (e.g., change wound dressing).
 - Manage equipment (e.g., oxygen, IV/infusion equipment, enteral/ parenteral nutrition, ventilator therapy equipment or supplies).
 - Maintain a prescribed diet.
 - Chart daily weight, blood sugar.

Element D: Initial Assessment

An NCQA review of a sample of the organization's complex case management files demonstrates that the organization follows its documented processes for completing the following within 60 calendar days:

1. Initial assessment of member health status, including condition-specific issues.
2. Documentation of clinical history, including medications.
3. Initial assessment of the activities of daily living (ADL).
4. Initial assessment of behavioral health status, including cognitive functions.
5. Initial assessment of social determinants of health.
6. Evaluation of cultural and linguistic needs, preferences or limitations.
7. Evaluation of visual and hearing needs, preferences or limitations.
8. Evaluation of caregiver resources and involvement.
9. Evaluation of available benefits.
10. Evaluation of available community resources.
11. Assessment of life-planning activities.
12. Beginning the assessment for at least one factor within 30 calendar days of identifying a member for complex case management.

Summary of Changes**Additions**

- Added a new factor 12, subhead and text to the Explanation.

Clarifications

- Added “completing the following within 60 days” to the element stem.
- Replaced “assessment” with “assessment components” in the second paragraph of the Explanation.
- Removed “Timeliness of assessment” subhead and text from the Explanation.
- Revised the fifth paragraph in the Explanation to clarify the time frame for assessment when a member is admitted to a facility and the stay is longer than 30 calendar days.
- Added text to the Explanation under “Factor 5: Initial assessment of social determinants of health” to clarify that the file or case record documents the assessment.

Scoring

Met	Partially Met	Not Met
High (90-100%) on file review for at least 8 factors and medium (60-89%) on file review for any remaining factors	High (90%-100%) or medium (60-89%) on file review for 12 factors	Low (0-59%) on file review for any factor

Data source Records or files

Scope of review**Product lines**

This element applies to First Surveys and Renewal Surveys for all product lines.

Documentation

NCQA reviews initial assessments in a random sample of up to 40 complex case management files. Files are selected from active or closed member cases that were identified during the look-back period and remained open for at least 60 calendar days during the look-back period, from the date when the member was identified for complex case management.

The organization must provide the identification date for each case in the file universe.

Look-back period

For First Surveys: 6 months.

For Renewal Surveys: 12 months.

Explanation

Initial assessment files are reviewed on the requirements outlined in PHM 5, Element C.

Documentation to meet the factors includes evidence that the assessment components were completed and documented results of each assessment. A checklist of assessment components without documentation of results does not meet the requirement.

Assessment components may be completed by other members of the care team and with the assistance of the member's family or caregiver. Assessment results for each factor must be clearly documented in case management notes, even if a factor does not apply.

If the member is unable to communicate because of infirmity, assessment may be completed by professionals on the care team, with assistance from the patient's family or caregiver.

If case management stops when a member is admitted to a facility and the stay is longer than 30 calendar days, a new assessment must be performed after discharge if the member is identified again for case management.

Assessment and evaluation

Assessment and evaluation each require that the case manager or other qualified individual draw and document a conclusion about data or information collected. If the organization's CM system automatically generates suggestions, the case manager or other individual must still document their own conclusions. It is not sufficient to just have raw data or answers to questions. There is a documented summary of the meaning or implications of that data or information to the member's situation, so that it can be used in the case management plan.

The organization must draw a conclusion for each factor (unless otherwise stated in the explanation). This may be in separate summaries for each factor or in a combined summary, or in a combination of these.

Note: "Identification" is the point where a member is segmented or stratified into the complex case management program using the process described in PHM 2, Element D.

Dispute of file review results

Onsite file review is conducted in the presence of the organization's staff. The survey team works to resolve disputes that arise during the onsite survey. In the event that a dispute cannot be resolved, the organization must contact NCQA before the end of the onsite survey. File review results may not be disputed or appealed once the onsite survey is complete.

Files excluded from review

The organization excludes files from review that meet one of the following criteria:

- Eligible members whom it cannot locate or contact after three or more attempts across a 2-week period, within the first 30 calendar days after identification, through at least two of the following mechanisms:
 - Telephone.
 - Regular mail.
 - Email.
 - Fax.
- Members in complex case management for less than 60 calendar days during the look-back period.
 - The organization provides evidence of the member's identification date and that the member was in complex case management for less than 60 calendar days during the look-back period.
- Employees of the organization and their dependents.

Files that meet these criteria and are inadvertently included in the organization's file review are scored NA for all factors.

NCQA confirms that the files met the criteria for an NA score.

Factor 1: Initial assessment of member's health status

The file or case record documents the case manager's assessment of the member's current health status, including:

- Information on presence or absence of comorbidities and their current status.
- Self-reported health status.
- Information on the event or diagnosis that led to identification for complex case management.
- Current medications, including dosages and schedule.

Factor 2: Documentation of clinical history

The file or case record contains information on the member's clinical history, including:

- Past hospitalization and major procedures, including surgery.
- Significant past illnesses and treatment history.
- Past medications.

Dates are a necessary component of accurate documentation of the member's clinical history. To the extent possible, the organization collects dates as part of documenting clinical history; however, NCQA does not penalize an organization if a member or other individual providing the information cannot provide dates. If dates are not present in the file, NCQA reviews the organization's complex case management policies and procedures. If the organization has a process for collecting dates as part of the clinical history, NCQA assumes the file does not

include dates because the member or other individual giving information did not provide dates. The requirement is not met if the organization does not have a process for collecting dates as part of the clinical history.

Factor 2 does not require assessment or evaluation.

Factor 3: Initial assessment of activities of daily living

The file or case record documents the results of the ADL assessment.

For activities with which the member needs assistance, the case manager documents the reason and type of assistance. The case manager is not required to describe activities for which the member does not need assistance.

If the member needs no assistance with any ADLs, the case file or case notes reflect this (e.g., "Member is fully independent with ADLs").

Factor 4: Initial assessment of behavioral health status

The file or case record documents the case manager's assessment of:

- Cognitive functions.
 - The member's ability to communicate and understand instructions.
 - The member's ability to process information about an illness.
- Mental health conditions.
- Substance use disorders.

Factor 5: Initial assessment of social determinants of health

The file or case record documents the case manager's assessment of social determinants of health, which are economic and social conditions that affect a wide range of health, functioning and quality-of-life outcomes, and assessment of risks that may affect the member's ability to meet goals.

Because social determinants of health are a combination of influences, the organization considers more than one social determinant of health, for a comprehensive overview of the member's health.

Factor 6: Evaluation of cultural and linguistic needs

The file or case record documents the case manager's evaluation of the member's culture and language needs and their impact on communication, care or acceptability of specific treatments. At a minimum, the case manager evaluates:

- Cultural health beliefs and practices.
- Preferred languages.

Factor 7: Evaluation of visual and hearing needs

The file or case record documents the case manager's evaluation of the member's vision and hearing. The document describes specific needs to consider in the case management plan and barriers to effective communication or care.

Factor 8: Evaluation of caregiver resources

The file or case record documents the case manager's evaluation of the adequacy of caregiver resources (e.g., family involvement in and decision making about the care plan) during member evaluation. Documentation describes the resources in place and whether they are sufficient for the member's needs, and notes specific gaps to address.

Factor 9: Evaluation of available benefits

The file or case record documents the case manager's evaluation of the adequacy of the member's health insurance benefits in relation to the needs of the treatment plan. The evaluation goes beyond checking insurance coverage; it includes a determination of whether the resources available to the member are adequate to fulfill the treatment plan.

Factor 10: Evaluation of community resources

The file or case record documents the case manager's evaluation of the member's eligibility for community resources and the availability of those resources and documents which the member may need.

For the community resources the member needs, the availability and member's eligibility is also recorded in the file. The case manager does not need to address community resources the member does not need.

If the member does not need community resources, the case file or case notes reflect this (e.g., "Member does not need any of the available community resources").

Factor 11: Initial assessment of life planning activities

The file or case record documents the case manager's assessment of whether the member has in place or has considered the need for wills, living wills or advance directives, Medical or Physician Orders of Life-Sustaining Treatment (MOLST or POLST) forms and health care powers of attorney.

If life planning activities are determined to be appropriate, the case manager documents what activities the member has taken and what documents are in place. If life planning activities are determined not to be appropriate, the case manager documents the reason in the case management record or file.

As an alternative to an assessment of life planning needs, the organization may provide life planning information (e.g., brochure, pamphlet) to members in complex case management during the time frame allowed for completing the initial assessment. The file must document that the information was provided and the date.

Factor 12: Beginning the assessment within 30 calendar days of identification

The organization begins initial assessment within 30 calendar days of identifying a member for complex case management and completes the assessment within 60 calendar days of identifying the member. If the initial assessment begins after the first 30 calendar days of identifying the member, NCQA scores only factor 12 "No"; the remaining factors are not marked down.

NCQA scores any factor "No" if the initial assessment is completed more than 60 calendar days from identifying the member, unless the delay was due to circumstances beyond the organization's control;

- The member is hospitalized during the initial assessment period.
- The member cannot be contacted or reached through telephone, letter, email or fax.
- Natural disaster.
- The member is deceased.

The organization documents the reasons for the delay and actions it took to complete the assessment. The assessment may be derived from care or encounters occurring up to 30 calendar days before the member was identified if the information is related to the current episode of care (e.g., health history taken as part of disease management or during a hospitalization). Members are considered eligible once they are identified, unless they subsequently opt out or additional information reveals that they are ineligible.

Exceptions

None.

Examples None.

Element E: Case Management—Ongoing Management

An NCQA review of a sample of the organization's complex case management files demonstrates that the organization follows its documented processes for:

1. Development of case management plans that include prioritized goals, that take into account member and caregiver goals, preferences and desired level of involvement in the complex case management program.
2. Identification of barriers to meeting goals and complying with the case management plan.
3. Development of schedules for follow-up and communication with members.
4. Development and communication of member self-management plans.
5. Assessment of progress against case management plans and goals, and modification as needed.

Summary of Changes

Clarifications

- Replaced "The organization" with "The file or case record" in the Explanation text for factors 1, 3, 4 and 5.

Scoring	Met	Partially Met	Not Met
	High (90%-100%) on file review for at least 3 factors and medium (60-89%) on file review for any remaining factors	High (90%-100%) or medium (60-89%) on file review for 5 factors	Low (0-59%) on file review for any factors

Data source Records or files

Scope of review **Product lines**

This element applies to First Surveys and Renewal Surveys for all product lines.

Documentation

NCQA reviews initial assessments in a random sample of up to 40 complex case management files. Files are selected from active or closed cases that were identified during the look-back period and remained open for at least 60 calendar days during the look-back period, from the date when the member was identified for complex case management.

	The organization must provide the identification date for each case in the file universe.
Look-back period	<i>For First Surveys:</i> 6 months. <i>For Renewal Surveys:</i> 12 months.
Explanation	Assessment files are based on the requirements outlined in PHM 5, Element C.

Dispute of file review results

Onsite file review is conducted in the presence of the organization's staff. The survey team works to resolve disputes that arise during the onsite survey. In the event that a dispute cannot be resolved, the organization must contact NCQA before the end of the onsite survey. File review results may not be disputed or appealed once the onsite survey is complete.

Files excluded from review

The organization excludes files from review that meet one of the following criteria:

- Identified members whom it cannot locate or contact after three or more attempts across a 2-week period, within the first 30 calendar days after identification, through at least two of the following mechanisms:
 - Telephone.
 - Regular mail.
 - Email.
 - Fax.
- Members in complex case management for less than 60 calendar days during the look-back period.
 - The organization provides evidence of the member's identification date and that the member was in complex case management for less than 60 calendar days during the look-back period.
- Employees of the organization and their dependents.

Files that meet these criteria and are inadvertently included in the organization's file review are scored NA for all factors.

NCQA reserves the right to confirm that the files met the criteria for an NA score.

Factor 1: Case management plans and goals

The file or case record documents a plan for case management that:

- Is specific to the member's situation and needs, **and**
- Includes goals that reflect issues identified in the member assessment and the supporting rationale for each selected goal.
 - Goals are specific, measurable and timebound (i.e., have a target completion date).
 - Case management goals are prioritized.
 - The organization prioritizes goals using high/low, numeric rank or other similar designation. Priorities reflect input from the member or a caregiver, demonstrating the member or caregiver's preferences and priorities. Designating goals as long-term or short-term is not sufficient to meet the requirement. The organization must rank or prioritize goals.

Factor 2: Identification of barriers

The file or case record identifies barriers related to the member or to the member's circumstances (not to the case management process). The organization documents barriers to the member meeting the goals specified in the case management plan.

Factor 3: Follow-up and communication with members

The file or case record documents the next scheduled contact with the member, including the scheduled time or time frame, which may be an exact or relative date (e.g., "October 15," "in two weeks"), and the contact method.

Factor 4: Self-management plan

The file or case record documents a self-management plan that includes actions the member agrees to take to manage a condition or circumstances. The organization documents that the plan has been communicated to the member. Communication may be verbal or written. The self-management plan documents the member's acknowledgment of and agreement with actions the member will take.

Factor 5: Assessment of progress

The file or case record documents the member's progress toward goals. If the member does not demonstrate progress over time, the organization reassesses the applicability of the goals to the member's circumstances and modifies the goals, as appropriate.

Exceptions

None.

Examples Factors 1–5: Case Management—Ongoing Management

Member Diagnosis: Severe mental illness (depression); chronic homelessness (unstable housing for 8 months)	
Identification date: 1/5/2022	Initial Assessment Completed: 1/30/2022
Goal 1:	Secure stable housing for member by 2/11/2022. (Factor 1)
<i>Goal case notes:</i> Member did not identify a family or friend caregiver. Member expresses a desire for a home and is willing to accept case manager's help to manage other conditions, once in stable housing. (Factor 1) <i>Strategies to achieve goal:</i> Referral to community housing resources; secure temporary safe housing, pending a more permanent solution; accompany member to housing services. <i>Barriers to goal:</i> Member was previously evicted from temporary shelter due to unwillingness to comply with shelter staff rules. (Factor 2) <i>Progress assessment:</i> Member moved out of initial temporary shelter because he felt his belongings were unsafe. Asked for help getting into a home where he can lock up his belongings. CM adjusted completion date to 2/21/2022 and investigated group housing. (Factor 5)	
Goal 1 completed:	2/16/2022. Note: Member was accepted into adult male group housing, once he understood and accepted house rules, is comfortable with secure locker for belongings. (Factor 5)

Goal 2:	• Improve member's Patient Health Questionnaire-9 (PHQ-9) score from baseline (23 at initial assessment 1/30/2022) over 3–6 months. • Improve 5 points from baseline by 4/30/2022. • Improve 11 points from baseline by 7/30/2022. (Factor 1)
<i>Goal case notes:</i> Member did not identify a family or friend caregiver. Member expresses a desire for a home and is willing to accept case manager's help to manage other conditions, once in stable housing. Member feels that stable housing will help depression and is willing to attend therapy sessions. (Factor 1) <i>Strategies to achieve goal:</i> Implement a reminder system for taking medications; arrange transportation for therapist visits; check in weekly to discuss progress. <i>Barriers to goal:</i> Member uncertain about how to get to therapy sessions and states that he feels overwhelmed by having to change buses and remember schedules. Member said his medication has been stolen in shelters before. (Factor 2) <i>Progress assessment:</i> Member feels his medications are safe in group home lockers. CM helped the member set up a calendar pill case and clock alarm as medication reminders. CM arranged van transportation to twice weekly therapy sessions. CM assessed PHQ score at weekly call on 4/28/2022. Score was 16 (9 less than baseline). Member stated that housing greatly improved depression. Therapy sessions adjusted to weekly. CM assessed PHQ score at weekly call on 7/28/2022. Score was 12 (11 less than baseline). (Factor 5)	
<i>Goal 2 completed:</i>	7/28/2022. Note: Member attends therapy. Member can navigate bus lines without anxiety; assisted transportation to sessions discontinued. (Factor 5)
Follow-up and communication plan:	CM scheduled weekly follow-up calls at 5pm on Fridays via the group home's phone line. CM gave member direct emergency line and is working to secure cell phone for member. (Factor 3)
Self-management plan:	• Member will attend weekly follow-up calls on Fridays at 5pm via ***-***-****. • Member will continue to follow rules of group home. • Member will alert CM if changes to housing occur. • Member will use alarm clock reminders to take medication on schedule. Member and CM will discuss monthly refills to medications box. • CM arranges medication to be mailed to group home; member agrees to verify medication with CM during weekly calls. • Member attends therapy sessions and alerts group home staff to dramatic changes in mood (e.g., suicidal ideation). • Member will work with group home staff and other residents to learn bus routes and how to change buses on route. (Factor 4)

PHM 6: Population Health Management Impact

The organization measures the effectiveness of its PHM strategy.

Intent

The organization has a systematic process to evaluate whether it has achieved its goals and to gain insights into areas needing improvement.

Element A: Measuring Effectiveness

At least annually, the organization conducts a comprehensive analysis of the impact of its PHM strategy that includes the following:

1. Quantitative results for relevant clinical, cost/utilization and experience measures.
2. Comparison of results with a benchmark or goal.
3. Interpretation of results.

Summary of Changes

Clarifications

- Replaced “materials” with “documentation” in the first paragraph of the Explanation.
- Added “and” to the first bullet in the Explanation under “Factor 1: Measurement.”
- Replaced “at a minimum” with “and includes at least one of the following measures” under “Experience” in the Explanation for “Factor 1: Measurement.”

Scoring	Met	Partially Met	Not Met
	The organization meets 3 factors	The organization meets 2 factors	The organization meets 0-1 factors
Data source	Reports		
Scope of review	Product lines <i>This element applies to First Surveys and Renewal Surveys for all product lines.</i> NCQA reviews and scores this element for each product line brought forward for Accreditation. Documentation <i>For First Surveys:</i> NCQA reviews the organization’s plan for annual comprehensive analysis of its PHM strategy impact or the organization’s most recent annual comprehensive analysis of PHM strategy impact. <i>For Renewal Surveys:</i> NCQA reviews the organization’s most recent and previous annual comprehensive analysis of PHM strategy impact.		
Look-back period	<i>For First Surveys:</i> At least once during the prior year. <i>For Renewal Surveys:</i> 24 months.		

Explanation This element is a **structural requirement**. The organization must present its own documentation.

The organization conducts an annual comprehensive evaluation of the impact of the organization's PHM strategy.

Factor 1: Measurement

The organization's evaluation of the impact of its strategy includes at least four measures, that includes:

- One clinical measure, **and**
- One cost or utilization measure **and**
- One member feedback measure from two different programs.

Relevant measures align with the goals for the areas of focus, activities or programs described in PHM 1, Element A. The organization describes why measures are relevant. Measures may focus on one segment of the population or on populations across the organization.

Clinical measures. Measures can be activities, events, occurrences or outcomes for which data can be collected for comparison with a threshold, benchmark or prior performance. Clinical measures may be:

1. *Outcome measures:* Incidence or prevalence rates for desirable or undesirable health status outcomes (e.g., infant mortality), **or**
2. *Process measures:* Measures of clinical performance based on objective clinical criteria defined from practice guidelines or other clinical specifications (e.g., immunization rates).

Cost/Utilization measures. Utilization is an unweighted count of services (e.g., inpatient discharges, inpatient days, office visits, prescriptions). Utilization measures capture the frequency of services provided by the organization. Cost-related measures can be used to demonstrate utilization. The organization measures cost, resource use or utilization.

Cost of care considers the mix and frequency of services, and is determined using actual unit price per service or unit prices found on a standardized fee schedule. Examples of cost of care measurement include:

- Dollars per episode, overall or by type of service.
- Dollars per member, per month (PMPM), overall or by type of service.
- Dollars per procedure.

Resource use considers the cost of services in addition to the count of services across the spectrum of care, such as the difference between a major surgery and a 15-minute office visit.

Experience. The organization obtains member feedback from at least two programs (e.g., disease management or wellness programs), using focus groups or satisfaction surveys. Feedback is specific to the programs being evaluated and includes at least one of the following measures:

- Information about the overall program.
- The program staff.
- Usefulness of the information disseminated.

- Members' ability to adhere to recommendations.
- Percentage of members indicating that the program helped them achieve health goals.

The organization may supplement member survey or focus group data with member complaint data.

CAHPS and other general survey questions do not meet the intent of this element.

Factor 2: Quantitative analysis

The organization performs a quantitative analysis of the four measure results from factor 1.

Factor 3: Qualitative analysis

For initial measurement, the organization conducts qualitative analysis of each measure result from factor 1.

For remeasurement, the organization conducts qualitative analysis of each measure result from factor 1 if quantitative analysis demonstrates that stated goals were not met.

The organization assesses measure results together for a comprehensive qualitative analysis of the effectiveness of its PHM strategy. The analysis includes an interpretation of results, to give the organization insight into its PHM programs and strategy, and helps it understand the programs' effectiveness and impact on areas of focus.

Refer to *Appendix 5: Glossary* for the full definition of and requirements for *quantitative analysis* and *qualitative analysis*.

Note:

- *Participation rates do not qualify for this element.*
- *If the organization uses SF-8®, SF-12® or SF-36® to measure health status, results may count for two measures of effectiveness: one each for physical and mental health functioning.*

Exceptions

None.

Examples

Factor 1: Quantitative results

Utilization includes measures of waste, overutilization, access, cost or underutilization.

Experience

- Patient Health Questionnaire (PHQ-9).
- Patient-Reported Outcomes Measurement Information System (PROMIS) tools.
- Program-specific surveys.

Element B: Improvement and Action

The organization uses results from the PHM impact analysis to annually:

1. Identify opportunities for improvement.
2. Act on one opportunity for improvement.

Summary of Changes**Clarifications**

- Added “materials” as a data source and replaced “or reports” with “reports or materials” in the scope of review.
- Replaced “materials” with “documentation” in the first sentence of the Explanation.

Scoring	Met	Partially Met	Not Met
	The organization meets 2 factors	The organization meets 1 factor	The organization meets 0 factors
Data source	Documented process, Reports, Materials		
Scope of review	Product lines <i>This element applies to Renewal Surveys for all product lines.</i> NCQA reviews and scores this element for each product line brought forward for Accreditation. Documentation NCQA reviews the organization’s most recent and previous annual comprehensive analysis of PHM strategy impact. For factor 2, NCQA reviews a documented process, reports, or materials depending on the action taken to address identified opportunities.		
Look-back period	<i>For Renewal Surveys: 24 months.</i>		
Explanation	This element is a structural requirement. The organization must present its own documentation Factor 1: Opportunities for improvement The organization uses the results of its analysis to identify opportunities for improvement, which may be different each time data are measured and analyzed. NCQA does not prescribe a specific number of improvement opportunities. Factor 2: Act on opportunity for improvement The organization acts on at least one identified opportunity for improvement. Exceptions None.		
Examples	None.		

PHM 7: Delegation of PHM

If the organization delegates NCQA-required PHM activities, there is evidence of oversight of the delegated activities.

Intent

The organization remains responsible for and has appropriate structures and mechanisms to oversee delegated PHM activities.

Element A: Delegation Agreement

The written delegation agreement:

1. Is mutually agreed upon.
2. Describes the delegated activities and the responsibilities of the organization and the delegated entity.
3. Requires at least semiannual reporting by the delegated entity to the organization.
4. Describes the process by which the organization evaluates the delegated entity's performance.
5. Describes the process for providing member experience and clinical performance data to its delegates when requested.
6. Describes the remedies available to the organization if the delegated entity does not fulfill its obligations, including revocation of the delegation agreement.

Summary of Changes

Clarifications

- Revised the first sentence of the third paragraph under "Documentation" to clarify that the process from factor 5 must be included in the organization's documentation.
- Revised "CAHPS 5.0H" to "CAHPS" in the factor 5 explanation.

Scoring	Met	Partially Met	Not Met
	The organization meets 5-6 factors	The organization meets 3-4 factors	The organization meets 0-2 factors
Data source	Materials		
Scope of review	Product lines <i>This element applies to Interim Surveys, First Surveys and Renewal Surveys for all product lines.</i>		
	Documentation NCQA reviews delegation agreements in effect during the look-back period of up to four randomly selected delegates, or reviews all delegates if the organization has fewer than four. Delegation agreements implemented on or after January 1, 2019, must include a description of the process required in factor 5.		

For delegation agreements in place prior to January 1, 2019, the organization may provide documentation that it notified the delegate of the process required in factor 5. This documentation of notification is not required to be mutually agreed upon.

The score for the element is the average of the scores for all delegates.

Look-back period

For Interim Surveys and First Surveys: 6 months.

For Renewal Surveys: 24 months.

Explanation

This element may not be delegated.

This element applies to agreements that are in effect during the look-back period.

The delegation agreement describes all delegated PHM activities. A generic policy statement about the content of delegated arrangements does not meet this element.

Factor 1: Mutual agreement

Delegation activities are mutually agreed on before delegation begins, in a dated, binding document or communication between the organization and the delegated entity.

NCQA considers the effective date specified in the delegation agreement as the mutually agreed-upon effective date. The effective date may be before or after the signature date on the agreement. If the agreement has no effective date, NCQA considers the signature date (the date of last signature) as the mutually agreed upon effective date.

NCQA may accept other evidence of the mutually agreed-upon effective date: a letter, meeting minutes or other form of communication between the organization and the delegate that references the parties' agreement on the effective date of delegated activities.

NCQA requires submitted evidence for all other delegation factors to consider the same mutually agreed-upon date as the effective date for the delegate's performance of delegated activities.

Factor 2: Assigning responsibilities

The delegation agreement or an addendum thereto or other binding communication between the organization and the delegate specifies the PHM activities:

- Performed by the delegate, in detailed language.
- Not delegated, but retained by the organization.
- The organization may include a general statement in the agreement addressing retained functions (e.g., the organization retains all other PHM functions not specified in this agreement as the delegate's responsibility).

If the delegate subdelegates an activity, the delegation agreement must specify that the delegate or the organization is responsible for subdelegate oversight.

Factor 3: Reporting

The organization determines the method of reporting and the content of the reports, but the agreement must specify:

- That reporting is at least semiannual.
- What information is reported by the delegate about PHM delegated activities.

- How, and to whom, information is reported (i.e., joint meetings or to appropriate committees or individuals in the organization).

The organization must receive regular reports from all delegates, even NCQA-Accredited/Certified delegates.

Factor 4: Performance monitoring

The delegation agreement specifies how the organization evaluates the delegate's performance.

Factor 5: Providing member and clinical data

The organization's delegation agreement describes what the delegate must do to obtain the following data when it is needed or on an ongoing basis:

- *Member experience data:* Complaints, CAHPS survey results or other data collected on members' experience with the delegate's services.
- *Clinical performance data:* HEDIS measures, claims and other clinical data collected by the organization. The organization may provide data feeds for relevant claims data or clinical performance measure results.

Factor 6: Consequences for failure to perform

The delegation agreement specifies consequences if a delegate fails to meet the terms of the agreement and, at a minimum, circumstances that would cause revocation of the agreement.

Exception

This element is NA if the organization does not delegate PHM activities.

Examples None.

Element B: Predelegation Evaluation

For new delegation agreements initiated in the look-back period, the organization evaluated delegate capacity to meet NCQA requirements before delegation began.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization evaluated delegate capacity before delegation began	The organization evaluated delegate capacity after delegation began	The organization did not evaluate delegate capacity

Data source Reports

Scope of review **Product lines**

This element applies to Interim Surveys, First Surveys and Renewal Surveys for all product lines.

This element applies if delegation was implemented in the look-back period.

Documentation

NCQA reviews the organization's predelegation evaluation of up to four randomly selected delegates, or reviews all delegates if the organization has fewer than four.

The score for the element is the average of the scores for all delegates.

Look-back period

For Interim and First Surveys: 6 months.

For Renewal Surveys: 12 months.

Explanation

This element may not be delegated.

NCQA-Accredited/Certified delegates

Automatic credit is available for this element if all delegates are NCQA-Accredited health plans, MBHOs or CMOs, NCQA-Accredited/Certified DMOs, NCQA-Accredited PHP Organizations or NCQA-Prevalidated Health IT Solutions, unless the element is NA.

Predelegation evaluation

The organization evaluated the delegate's capacity to meet NCQA requirements within 12 months prior to implementing delegation. The evaluation may include a review of the organization's structure, processes, and staffing in order to determine its capability to perform the delegated function.

NCQA considers the date of the agreement to be the implementation date if the delegation agreement does not include an implementation date.

If the time between the predelegation evaluation and implementation of delegation exceeds the 12 months, the organization conducts another predelegation evaluation.

If the organization amends the delegation agreement to include additional PHM activities within the look-back period, it performs a predelegation evaluation for the additional activities.

Exceptions

This element is NA if:

- The organization does not delegate PHM activities.
- Delegation arrangements have been in effect for longer than the look-back period.

Related information

Use of collaboratives. The organization may enter into a statewide collaboration to perform any or all of the following:

- Predelegation evaluation.
- Annual evaluation.
- Annual audit of files.

The collaborative must agree on the use of a consistent audit tool and must share data. Each organization is responsible for meeting NCQA delegation standards, but may use the shared data collection process to reduce burden.

Examples Predelegation evaluation

- Site visit.
- Telephone consultation.
- Documentation review.
- Committee meetings.
- Virtual review.

Element C: Review of PHM Program

For arrangements in effect for 12 months or longer, the organization:

1. Annually reviews its delegate's PHM program.
2. Annually audits complex case management files against NCQA standards for each year that delegation has been in effect, if applicable.
3. Annually evaluates delegate performance against NCQA standards for delegated activities.
4. Semiannually evaluates regular reports, as specified in Element A.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 3-4 factors	The organization meets 2 factors	The organization meets 0-1 factors

Data source Reports

Scope of review**Product lines**

Factor 1 applies to Interim Surveys for all product lines.

All factors in this element apply to First Surveys and Renewal Surveys for all product lines.

Documentation

NCQA reviews evidence of the organization's review from up to four randomly selected delegates, or all delegates if the organization has fewer than four.

For All Surveys: NCQA reviews the organization's review of the delegate's PHM program (factor 1).

For First Surveys: NCQA reviews the organization's most recent annual review, audit, performance evaluation and semiannual evaluation.

For Renewal Surveys: NCQA reviews the organization's most recent and the previous year's annual reviews, audits, performance evaluations and four semiannual evaluations.

The score for the element is the average of the scores for all delegates.

Look-back period

For Interim Surveys and First Surveys: Once during the prior year.

For Renewal Surveys: 24 months.

Explanation This element may not be delegated.

Automatic credit is available for factors 2 and 3 if all delegates are NCQA-Accredited health plans, MBHOs or CMOs, or NCQA-Accredited/Certified DMOs, unless the element is NA.

Automatic credit is available for factor 3 if all delegates are NCQA-Prevalidated Health IT Solutions or NCQA-Accredited PHP Organizations, unless the element is NA.

Factor 1: Review of the PHM program

Appropriate organization staff or committee reviews the delegate's PHM program. At a minimum, the organization reviews parts of the PHM program that apply to the delegated functions.

Factor 2: Annual file audit

If the organization delegates complex case management, it audits the delegate's complex case management files against NCQA standards. The organization uses either of the following to audit the files:

- 5 percent or 50 of its files, whichever is less.
- The NCQA "8/30 methodology" available at <http://www.ncqa.org/Programs/Accreditation/PolicyUpdatesSupportingDocuments.aspx>

The organization bases its annual audit on the responsibilities described in the delegation agreement and the appropriate NCQA standards.

Factor 3: Annual evaluation

No additional explanation required.

Factor 4: Evaluation of reports

No additional explanation required.

Exceptions

This element is NA if:

- The organization does not delegate PHM activities.
- Delegation arrangements have been in effect for less than 12 months.

Factor 2 is NA if the organization does not delegate complex case management activities.

Factors 2–4 are NA for Interim Surveys.

Related information

Use of collaboratives. The organization may enter into a statewide collaboration to perform any or all of the following:

- Predelegation evaluation.
- Annual evaluation.
- Annual audit of files.

The collaborative must agree on the use of a consistent audit tool and must share data. Each organization is responsible for meeting NCQA delegation standards, but may use the shared data collection process to reduce burden.

Examples None.

Element D: Opportunities for Improvement

For delegation arrangements that have been in effect for more than 12 months, at least once in each of the past 2 years that delegation has been in effect, the organization identified and followed up on opportunities for improvement, if applicable.

Summary of Changes

- No changes to this element.

Scoring

Met	Partially Met	Not Met
The organization has acted on identified problems, if any, at least once in each of the past 2 years that the delegation arrangement has been in effect	The organization took inappropriate or weak action, or acted only in the past year	The organization has not acted on identified problems

Data source

Documented process, Reports, Materials

Scope of review**Product lines**

This element applies to First Surveys and Renewal Surveys for all product lines.

Documentation

NCQA reviews reports for opportunities for improvement if applicable of up to four randomly selected delegates, or from all delegates, if the organization has fewer than four, and for evidence that the organization took appropriate action to resolve issues.

For First Surveys: NCQA reviews the organization's most recent annual review and follow-up on improvement opportunities.

For Renewal Surveys: NCQA reviews the organization's most recent and previous year's annual reviews and follow-up on improvement opportunities.

The score for the element is the average of the scores for all delegates.

Look-back period

For First Surveys: At least once during the prior year.

For Renewal Surveys: 24 months.

Explanation

This element may not be delegated.

NCQA-Accredited/Certified delegates

Automatic credit is available for this element if all delegates are NCQA-Accredited health plans, MBHOs or CMOs, NCQA-Accredited/Certified DMOs, NCQA-Accredited PHP Organizations or NCQA-Prevalidated Health IT Solutions, unless the element is NA.

Identify and follow up on opportunities

The organization uses information from its predelegation evaluation, ongoing reports, or annual evaluation to identify areas of improvement.

Exceptions

This element is NA if:

- The organization does not delegate PHM activities.
- Delegation arrangements have been in effect for less than 12 months.
- The organization has no opportunities to improve performance.
 - NCQA evaluates whether this conclusion is reasonable, given assessment results.

Examples None.

Network Management

Standards for Network Management

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NET 1: Availability of Practitioners*Refer to Appendix 1 for points*

The organization maintains sufficient numbers and types of primary care, behavioral health and specialty care practitioners in its network.

Intent

The organization maintains an adequate network of primary care, behavioral healthcare and specialty care practitioners (SCP) and monitors how effectively this network meets the needs and preferences of its membership.

Element A: Cultural Needs and Preferences

The organization annually:

1. Assesses the cultural, ethnic, racial and linguistic needs of its members.
2. Adjusts the availability of practitioners within its network, if necessary.

Summary of Changes**Clarifications**

- Added First and Renewal Survey text under “Documentation” in the scope of review.

Policy Changes

- Revised the element stem to require annual frequency of assessment of member needs and adjustment of the practitioner network.
- Replaced “at least once within the look-back period” with “annually” under “Documentation” in the scope of review.
- Revised the look-back period for First and Renewal Surveys from 6 months to at least once during the prior year.

Scoring	Met	Partially Met	Not Met
	The organization meets 2 factors	The organization meets 1 factor	The organization meets 0 factors

Data source Documented process, Reports

Scope of review **Product lines**
This element applies to First Surveys and Renewal Surveys for all product lines.

Documentation

NCQA reviews the organization’s data collection methodology (presented as a documented process or within the report), assessment of unmet member needs and characteristics of the practitioner network and documentation of any adjustments made in the network to meet identified needs annually.

For First and Renewal Surveys: NCQA reviews the organization’s most recently completed report.

Look-back period *For First Surveys and Renewal Surveys:* At least once during the prior year.

Explanation **Factor 1: Assessing members' needs**

Data collection. To assess the cultural, ethnic, racial and linguistic needs of its members relative to its network, the organization must first collect data on ethnic, racial and linguistic characteristics of its members. A separate source of data specific to cultural characteristics (e.g., employer demographics, member surveys or focus groups) is not required.

Assessment. The organization assesses the unmet needs of its members relative to its network. To meet the factor, the organization must address all four needs—cultural, ethnic, racial and linguistic.

Cultural preferences and beliefs may be assessed from members (e.g., member surveys or focus groups) or other sources. If using other sources, aspects of culture can be initially inferred from ethnic, racial and linguistic characteristics but must also be supplemented with information about the cultural needs and preferences (e.g., religion, family traditions, customs) of its population or populations with similar characteristics. The organization may use existing health services research.

Factor 2: Practitioner availability

In order to determine if member needs are met by the network or if adjustments are needed to meet member needs, the organization assesses the characteristics (culture, ethnicity, race, spoken language) of network practitioners.

The organization adjusts the practitioner network to provide the types and number of practitioners necessary to meet the cultural, ethnic, racial and linguistic needs of its members within defined geographical areas. Adjustment of the practitioner network may include requiring existing practitioners to complete cultural competency training, providing practitioners with culturally and linguistically appropriate health education materials, or recruiting practitioners whose cultural and ethnic backgrounds are similar to the underrepresented member population. The organization determines what adjustments are appropriate based on identified needs.

The organization receives credit for factor 2 if it demonstrates that, based on its assessment of members' unmet needs and the applicable characteristics of the network, it is not necessary to adjust the practitioner network.

Exceptions

None.

Examples **Five-step process for meeting the intent of this element**

1. Collect data on ethnic, racial and linguistic needs of members from U.S. Census and enrollment data.
2. Conduct research or review literature on cultural needs and preferences based on the characteristics of the organization's members.
3. Correlate data with members' preferences based on member feedback or complaint data.
4. Assess the cultural, ethnic, racial and linguistic characteristics of network practitioners to evaluate whether network practitioners meet members' needs.
5. Take action to adjust the practitioner network if it does not meet members' cultural, ethnic, racial and linguistic needs.

Data sources

- Data from survey questions or focus groups that identify the health-related preferences or beliefs from specific ethnic groups.
- U.S. Census data on the racial/ethnic composition of the population within a service area or region.
- Practitioner race, ethnicity and language data collected during the credentialing process.
- Published health statistics, health services research, data provided by plan sponsors or government agencies.

Actions resulting from assessment

- Recruit, credential and contract with practitioners who speak a language that reflects members' linguistic needs.
- Recruit, credential and contract with practitioners whose cultural and ethnic backgrounds are similar to the underrepresented member population.
- Require practitioners to complete cultural competency training courses based on the racial/ethnic composition of the member population.

Element B: Practitioners Providing Primary Care

To evaluate the availability of practitioners who provide primary care services, including general medicine or family practice, internal medicine and pediatrics, the organization:

1. Establishes measurable standards for the number of each type of practitioner providing primary care.
2. Establishes measurable standards for the geographic distribution of each type of practitioner providing primary care.
3. Annually analyzes performance against the standards for the number of each type of practitioner providing primary care.
4. Annually analyzes performance against the standards for the geographic distribution of each type of practitioner providing primary care.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 3-4 factors	The organization meets 2 factors	The organization meets 0-1 factors

Data source Documented process, Reports

Scope of review **Product lines**

This element applies to First Surveys and Renewal Surveys for all product lines.

Documentation

NCQA reviews policies and procedures in place throughout the look-back period for factors 1 and 2.

For First Surveys: NCQA also reviews the organization's most recent analysis report for factors 3 and 4.

For Renewal Surveys: NCQA also reviews the most recent and the previous year's annual analysis reports for factors 3 and 4.

Look-back period

For First Surveys: At least once during the prior year.

For Renewal Surveys: 24 months.

Explanation

Factors 1 and 2 are **structural requirements**. The organization must present its own documentation.

For each of the following types of practitioners, the organization establishes availability standards that are realistic for the community and the delivery system, and consider clinical safety:

- General practitioners/family practitioners.
- Internists.
- Pediatricians.

The organization may include nurse practitioners in its practitioner availability standards if they are within the scope of credentialing and provide primary care as licensed independent practitioners.

Factor 1: Standards for number of primary care practitioners

The organization expresses the standard for number of practitioners in one of two ways:

- The ratio of number of each type of practitioners to number of members.
- The ratio of number of primary care practices accepting new patients to number of members.

Factor 2: Standards for geographic distribution of primary care practitioners

The organization expresses the standard for geographic distribution of practitioners in one of two ways:

- Acceptable distance to a practitioner's office.
- Acceptable driving time.

Factors 3, 4: Quantitative and qualitative analysis

For initial measurement, the organization conducts quantitative and qualitative analysis of data.

For remeasurement, the organization conducts quantitative analysis and conducts qualitative analysis if quantitative analysis demonstrates that stated goals were not met.

To analyze its performance against the availability standards from factors 1–2, the organization collects data using measures that allow direct comparison against standards defined in the factors.

The analyses are refined enough to identify access concerns in subpopulations, specific products/product lines or geographic areas.

Refer to *Appendix 5: Glossary* for the full definition of and requirements for *quantitative analysis* and *qualitative analysis*.

Exception

The pediatric components of this element are NA for Medicare product lines.

Examples Factors 1, 2: Availability standards**Table 1: Ratio of practitioners to members (factor 1)**

Practitioner Type	Measure	Standard and Performance Goal
Family practitioner	Family practitioners to members	1:2,000
General practitioner	General practitioners to members	1:2,000
Pediatrician	Pediatricians to members	1:2,000
Internist	Internists to members	1:2,000

Table 2: Number and geographic distribution of practitioners, miles (factors 1, 2)

Practitioner Type	Measure	Performance Goal
Family practitioner	Two within 5 miles of member's home	95%
General practitioner	One within 5 miles of member's home	95%
Pediatrician	Two within 5 miles of member's home	95%
Internist	Three within 5 miles of member's home	95%

Table 3: Number and geographic distribution of practitioners, driving time (factors 1, 2)

Practitioner Type	Measure	Performance Goal
Family practitioner	One within 10 minutes driving time from member's home	95%
General practitioner	One within 10 minutes driving time from member's home	95%
Pediatrician	One within 10 minutes driving time from member's home	95%
Internist	One within 10 minutes driving time from member's home	95%

Element C: Practitioners Providing Specialty Care

To evaluate the availability of specialists in its delivery system, the organization:

1. Defines the types of high-volume and high-impact specialists.
2. Establishes measurable standards for the number of each type of high-volume specialists.
3. Establishes measurable standards for the geographic distribution of each type of high-volume specialists.
4. Establishes measurable standards for the geographic distribution of each type of high-impact specialist.
5. Analyzes its performance against the established standards at least annually.

Summary of Changes

Clarifications

- Replaced “CAHPS 5.0H data” with “CAHPS data” and “to see” with “with” in the CAHPS question wording in the first paragraph under “Conducting data analysis of specialist availability” in the Examples.

Scoring	Met	Partially Met	Not Met
	The organization meets 3-5 factors	The organization meets 2 factors	The organization meets 0-1 factors
Data source	Documented process, Reports		
Scope of review	Product lines <i>This element applies to First Surveys and Renewal Surveys for all product lines.</i>		
	Documentation NCQA reviews policies and procedures in place throughout the look-back period for factors 1–4. <i>For First Surveys:</i> NCQA also reviews the organization’s most recent annual analysis report for factor 5. <i>For Renewal Surveys:</i> NCQA also reviews the most recent and the previous year’s annual analysis reports for factor 5.		
Look-back period	<i>For First Surveys:</i> 6 months for factors 1–4 and at least once in the prior year for factor 5. <i>For Renewal Surveys:</i> 24 months.		
Explanation	Factors 1–4 are structural requirements . The organization must present its own documentation. The organization’s standards for specialist availability are realistic for the community and the delivery system, and consider clinical safety. The organization may have the same measurable standards for high-volume and high-impact specialists. The documentation must be clear that stated standards apply to both.		

Factor 1: High-volume and high-impact specialists

The organization's policies and procedures explain how it defines high-volume and high-impact specialists. The organization identifies specialties that are considered high-volume and specialties that are considered high-impact. At a minimum, high-volume specialties include obstetrics/gynecology and high-impact specialties include oncology. Even if the organization only identifies the minimum specialties as high-volume and high-impact, the organization must state this in its policies and procedures.

If obstetricians are not appropriate for the population (i.e., Medicare), the organization may measure only gynecologists to meet the requirement.

Factor 2: Standards for number of high-volume specialists

The organization expresses the standard for number of high-volume specialists in one of two ways:

- The ratio of the number of each type of specialist to the number of members.
- The ratio of the number of each type of specialist accepting new patients to the number of members.

Factors 3, 4: Standards for geographic distribution of high-volume and high-impact specialists

The organization expresses the standard for geographic distribution of specialists in one of two ways:

- Acceptable distance to a specialist's office.
- Acceptable driving time to a specialist's office from the member's home.

Factor 5: Quantitative and qualitative analysis

For initial measurement, the organization conducts quantitative and qualitative analysis of data.

For remeasurement, the organization conducts quantitative analysis and conducts qualitative analysis if quantitative analysis demonstrates that stated goals were not met.

To analyze its performance against the availability standards from factors 2–4, the organization collects data using measures that allow direct comparison against standards defined in the factors.

The analyses are refined enough to identify access concerns in subpopulations, specific products/product lines or geographic areas.

Refer to *Appendix 5: Glossary* for the full definition of and requirements for *quantitative analysis* and *qualitative analysis*.

Exceptions

None.

Examples**Identifying high-volume SCPs**

- Identify practitioners located in an expected high-volume geographic area or in high-volume specialties, or both.
- Use available prior encounter data.

- Identify certain types of practitioners most likely to provide services to the largest segment of the membership; for example, orthopedic surgeons might see more members for treatment than neurosurgeons.

Identifying high-impact specialists

- Identify practitioner types that treat conditions that have high mortality and morbidity rates.
- Identify practitioner types where treatment requires significant resources.

Table 1: Ratio of SCPs to member (factor 2)

NUMBER OF PRACTITIONERS		
Practitioner Type	Measure: Ratio	Performance Goal
OB-GYN	OB-GYN to members	1:2,000
General surgeon	General surgeons to members	1:5,000
Orthopedic surgeon	Orthopedic surgeons to members	1:10,000
Neurologist	Neurologists to members	1:15,000

Table 2: Number and geographic distribution of SCPs, miles (factors 2, 3)

GEOGRAPHIC DISTRIBUTION OF PRACTITIONERS, MILES		
Practitioner Type	Measure: Miles	Performance Goal
OB-GYN	One within 10 miles of member's home	95%
General surgeon	One within 15 miles of member's home	95%
Orthopedic surgeon	One within 15 miles of member's home	95%
Neurologist	One within 15 miles of member's home	95%

Table 3: Number and geographic distribution of SCPs, driving time (factors 2, 3)

GEOGRAPHIC DISTRIBUTION OF PRACTITIONERS, DRIVING TIME		
Practitioner Type	Measure: Driving Time	Performance Goal
OB-GYN	One within 20 minutes driving time of member's home	95%
General surgeon	One within 25 minutes driving time of member's home	95%
Orthopedic surgeon	One within 25 minutes driving time of member's home	95%
Neurologist	One within 25 minutes driving time of member's home	95%

Conducting data analysis of specialist availability

The organization collected CAHPS data on the question, “In the last 12 months, how often did you get an appointment with a specialist as soon as you needed?” and the ratio of specialist types to members for each zip code in its service area, and found no change in the number of complaints regarding specialist availability in two suburban zip codes in the same geographic area.

Further analysis showed that although most specialist-to-member ratios remained steady, this geographic area lost two orthopedic surgeons in the last year. Results of a survey of the membership in the area showed that 37 percent of members

were not satisfied with the availability of orthopedic surgeons (the acceptable standard was 10 percent). All other data showed complaints within acceptable limits.

The organization performed a causal analysis through a focus group of practitioner relations and QI staff and physician leadership, and identified the following causes:

- One orthopedic surgeon retired in the past year.
- One orthopedic surgeon terminated his contract with the organization because of reimbursement issues.
- The organization is finding it difficult to recruit orthopedic surgeons to the network.

The organization developed a plan to increase the number of orthopedic surgeons in the affected area and to conduct a review of reimbursement policy.

Element D: Practitioners Providing Behavioral Healthcare

To evaluate the availability of high-volume behavioral healthcare practitioners in its delivery system, the organization:

1. Defines the types of high-volume behavioral healthcare practitioners.
2. Establishes measurable standards for the number of each type of high-volume behavioral healthcare practitioner.
3. Establishes measurable standards for the geographic distribution of each type of high-volume behavioral healthcare practitioner.
4. Analyzes performance against the standards annually.

Summary of Changes

Clarifications

- Deleted “number of” in the second bullet under “Factor 2: Standards for number of behavioral healthcare practitioners.”

Scoring	Met	Partially Met	Not Met
	The organization meets 3-4 factors	The organization meets 2 factors	The organization meets 0-1 factors

Data source Documented process, Reports

Scope of review **Product lines**

This element applies to First Survey and Renewal Surveys for all product lines.

Documentation

NCQA reviews policies and procedures in place throughout the look-back period for factors 1–3.

For First Surveys: NCQA also reviews the organization’s most recent annual analysis report for factor 4.

For Renewal Surveys: NCQA also reviews the most recent and the previous year’s annual analysis reports for factor 4.

Look-back period *For First Surveys:* At least once during the prior year.
For Renewal Surveys: 24 months.

Explanation Factors 1–3 are **structural requirements**. The organization must present its own documentation.

The organization's standards for behavioral healthcare practitioner availability are realistic for the community and the delivery system, and consider clinical safety.

Factor 1: Types of behavioral healthcare practitioners

The organization's policies and procedures explain how it defines high-volume behavioral healthcare practitioners. The organization defines the types of behavioral healthcare practitioners across the continuum of care.

Factor 2: Standards for number of behavioral healthcare practitioners

The organization expresses the standard for number of behavioral healthcare practitioners in one of the following ways:

- The ratio of each type of behavioral healthcare practitioners to the number of members.
- The ratio of each type of behavioral healthcare practitioner accepting new patients to the number of members.

Factor 3: Standards for geographic distribution of behavioral healthcare practitioners

The organization expresses the standard for geographic distribution of behavioral healthcare practitioners in one of two ways:

- Acceptable distance to a behavioral healthcare practitioner's office.
- Acceptable driving time to a behavioral healthcare practitioner's office from the member's home.

Factor 4: Quantitative and qualitative analysis

For initial measurement, the organization conducts quantitative and qualitative analysis of data.

For remeasurement, the organization conducts quantitative analysis and conducts qualitative analysis if quantitative analysis demonstrates that stated goals were not met.

To analyze its performance against the availability standards from factors 2–3, the organization collects data using measures that allow direct comparison against standards defined in the factors.

The analyses are refined enough to identify access concerns in subpopulations, specific products/product lines or geographic areas.

Refer to *Appendix 5: Glossary* for the full definition of and requirements for *quantitative analysis* and *qualitative analysis*.

Exception

This element is NA if all purchasers of the organization's services carve out or exclude behavioral healthcare.

Examples**Common types of practitioners across the continuum of behavioral healthcare delivery**

- Psychiatrist.
- Addiction medicine specialist.
- Clinical psychologist.
- Clinical social worker.
- Psychiatric clinical nurse specialist.
- Substance abuse counselor.
- Marriage and family therapist.

Identifying high-volume behavioral healthcare practitioners

- Identify practitioners located in an expected high-volume geographic area or in high-volume disciplines, or both.
- Use available prior-encounter data.
- Identify certain types of practitioners most likely to provide services to the largest segment of the membership; for example, social workers might see more members for treatment than psychiatrists.

Table 1: Ratio of behavioral healthcare practitioner type to member (factor 2)

Practitioner Type	Measure	Standard and Performance Goal
Psychiatrist	Psychiatrists to members	1:2,000
Clinical psychologist	Clinical psychologists to members	1:15,000
Licensed clinical social worker	Licensed clinical social workers to members	1:10,000
Marriage and family counselor	Marriage and family counselors to members	1:3,000

Table 2: Number and geographic distribution of behavioral healthcare practitioners, miles (factors 2, 3)

Practitioner Type	Standard	Performance Goal
Psychiatrist	One within 10 miles of member's home	95%
Clinical psychologist	One within 20 miles of member's home	95%
Licensed clinical social worker	One within 10 miles of member's home	95%
Marriage and family counselor	One within 10 miles of member's home	95%

Table 3: Number and geographic distribution of behavioral healthcare practitioners, driving time (factors 2, 3)

Practitioner Type	Standard	Performance Goal
Psychiatrist	One within 15 minutes driving time of member's home	95%
Clinical psychologist	One within 25 minutes driving time of member's home	95%
Licensed clinical social worker	One within 25 minutes driving time of member's home	95%
Marriage and family counselor	One within 15 minutes driving time of member's home	95%

NET 2: Accessibility of Services

The organization establishes mechanisms to provide access to appointments for primary care services, behavioral healthcare services and specialty care services.

Intent

The organization provides and maintains appropriate access to primary care services, behavioral healthcare services and specialty care services.

Element A: Access to Primary Care

Using valid methodology, the organization collects and performs an annual analysis of data to measure its performance against its standards for access to:

1. Regular and routine care appointments.
2. Urgent care appointments.
3. After-hours care.

Summary of Changes

Clarifications

- Replaced “CAHPS 5.0H data” with “CAHPS data” in the Examples.
- Removed “at a doctor’s office or clinic” from the second bullet under “Acceptable CAHPS commercial questions” in the Examples.

Scoring	Met	Partially Met	Not Met
	The organization meets 2-3 factors	The organization meets 1 factor	The organization meets 0 factors
Data source	Reports		
Scope of review	Product lines		
	<i>This element applies to First Surveys and Renewal Surveys for all product lines.</i>		
	Documentation		
	<i>For First Surveys:</i> NCQA reviews the organization’s most recent annual data collection and analysis report. <i>For Renewal Surveys:</i> NCQA reviews the organization’s most recent and the previous year’s annual data collection and analysis reports.		
Look-back period	<i>For First Surveys:</i> At least once during the prior year.		
	<i>For Renewal Surveys:</i> 24 months.		
Explanation	This is a structural requirement . The organization must present its own documentation.		
	Data collection methods The organization determines the data collection methodology. The methodology allows identification of issues at the organizational level.		

The organization may collect data across the entire practitioner or member population or from a statistically valid sample. If the organization collects data using surveys or practitioner self-reported information, it supplements the data with an analysis of complaints regarding access. Supplemental complaint data is used to validate survey findings and self-reported information and assists in qualitative analysis of the primary data. The organization is not required to conduct a complete quantitative and qualitative analysis on supplemental data.

Data collection, quantitative and qualitative analyses

To analyze its performance against the access standards from factors 1–3, the organization collects data using measures that allow direct comparison against standards it defines.

For initial measurement, the organization conducts quantitative and qualitative analysis of data.

For remeasurement, the organization conducts quantitative analysis and conducts qualitative analysis if quantitative analysis demonstrates that stated goals were not met.

The analyses may be conducted at the organizational level (i.e., primary care practitioners and practices may be grouped together), but if the analyses reveals issues, the organization conducts a practitioner-level analysis (by individual primary care practitioner) across all primary care practitioners and practices or from a statistically valid sample of them to determine if members are able to get an appointment to see a practitioner.

The analyses are refined enough to identify access concerns in subpopulations, specific products/product lines or geographic areas.

Refer to *Appendix 5: Glossary* for the full definition of and requirements for *quantitative analysis* and *qualitative analysis*.

Factors 1–3: Access to appointments and after-hour care

Factors 1–3 apply to access to primary care services. Standards may be quantified in a specific number of hours or days, or the number or percentage of complaints about access to each type of appointment stated in the factors. The measure of performance must reflect the standard. For example, if the standard is hours or days, then the measure of performance must be hours or days.

Exceptions

None.

Examples

Timeliness standards

- An appointment for regular or routine care within 30 days.
- 90 percent of members reported that they “always” or “usually” get an appointment for health care at a doctor’s office or clinic as soon as they need it.
- An appointment for urgent care within 48 hours.
- An after-hours phone call from an appropriate practitioner within an hour of the member contacting the organization.

Sources of data used to assess against the standards

- Applicable results of the HEDIS/CAHPS survey.
- Site-specific surveys regarding access to offices of practitioners providing primary care.

Acceptable CAHPS commercial questions

- In the last 12 months, when you needed care right away, how often did you get care as soon as you needed?
- In the last 12 months, how often did you get an appointment for a check-up or routine care as soon as you needed?

Element B: Access to Behavioral Healthcare

Using valid methodology, the organization annually collects and analyzes data to evaluate access to appointments for behavioral healthcare for:

1. Care for a non-life-threatening emergency within 6 hours. *
2. Urgent care within 48 hours.*
3. Initial visit for routine care within 10 business days.
4. Follow-up routine care.

**Critical factors: These factors must be scored “yes” to score at least “Partially Met.”*

Summary of Changes**Clarifications**

- Deleted the definition of “follow-up routine care appointments” from the Exception.

Scoring	Met	Partially Met	Not Met
	The organization meets 3-4 factors	The organization meets 2 factors	The organization meets 0-1 factors

Data source Documented process, Reports, Materials

Scope of review **Product lines**
This element applies to First Surveys and Renewal Surveys for all product lines.

Documentation

For First Surveys: NCQA reviews the organization’s most recent annual data collection and analysis report for all factors.

For Renewal Surveys: NCQA reviews the organization’s most recent and the previous year’s annual data collection and analysis reports.

For factor 1 for both survey types, if the organization directs members with non-life-threatening emergencies to the emergency department (ED), NCQA reviews the organization’s report, policies or other documentation.

Look-back period *For First Surveys:* At least once during the prior year.
For Renewal Surveys: 24 months.

Explanation This is a **structural requirement**. The organization must present its own documentation.

Factors 1 and 2 are **critical factors**; both factors must be scored “yes” for the organization to score at least “Partially Met” on this element.

Data collection methods

The organization determines its data collection methodology. The data collection methodology allows identification of issues at the organizational level.

The organization may collect data across the entire practitioner or member population or from a statistically valid sample. If the organization collects data using surveys or practitioner self-reported information, it supplements the data with an analysis of complaints regarding behavioral healthcare access. Supplemental complaint data is used to validate survey findings and self-reported information and assists in qualitative analysis of the primary data. The organization is not required to conduct a complete quantitative and qualitative analysis on supplemental data.

Data collection, quantitative and qualitative analyses

To analyze its performance against the access standards from factors 1–4, the organization collects data using measures that allow direct comparison against standards it defines.

For initial measurement, the organization conducts quantitative and qualitative analysis of data.

For remeasurement, the organization conducts quantitative analysis and conducts qualitative analysis if quantitative analysis demonstrates that stated goals were not met.

The analyses may be conducted at the organizational level (behavioral healthcare practitioners and practices may be grouped together), but if the analyses reveals issues, the organization conducts a practitioner-level analysis (by individual behavioral healthcare practitioner) across all behavioral healthcare practitioners and practices, or from a statistically valid sample, to determine if members are able to get an appointment to see a practitioner.

The analyses are refined enough to identify access concerns in subpopulations, specific products/product lines or geographic areas.

Refer to *Appendix 5: Glossary* for the full definition of and requirements for *quantitative analysis* and *qualitative analysis*.

Factors 1–4: Access to behavioral healthcare and appointments

The organization meets factor 1 if it directs members with non-life-threatening emergencies to the ED or behavioral health crisis units.

The organization's report includes separate analyses of appointment availability for behavioral healthcare practitioners who prescribe medications (e.g., psychiatrists) and for behavioral healthcare practitioners who do not prescribe (e.g., psychologists) for each factor.

Factor 3: Initial routine care

Initial routine care appointments do not include follow-up care for an existing problem.

Factor 4: Follow-up routine care appointments

Follow-up routine care appointments are visits at later, specified dates to evaluate patient progress and other changes that have taken place since a previous visit.

Exception

This element is NA if all purchasers of the organization's services carve out or exclude behavioral healthcare.

Examples**Factor 1: Care for a non-life-threatening emergency within 6 hours**

- Report showing that all members who contacted the organization for a non-life-threatening emergency were directed to the ED instead of being scheduled for a doctor's visit.
- Customer Service telephone script that instructs the staff to direct members with non-life-threatening emergencies to the ED or behavioral health crisis unit.

Factor 4: Follow-up of routine care*Setting timeliness standards (step 1)*

1. 90% of sites have slots for routine follow-up appointments with prescribers within 30 days and with nonprescribers within 20 days.
2. 75% of members have a follow-up visit with a prescriber within 30 days of initial visit for a specific condition and with a nonprescriber within 20 days of initial visit for a specific condition.
3. 90% of members report that they "always" or "usually" get a follow-up appointment with a prescriber. 90% of members report that they "always" or "usually" get a follow-up appointment with a nonprescriber.

Data sources to assess reasonable access (step 2)

1. Site surveys indicate:
 - 80% of sites reported having slots for routine follow-up appointments with prescribers within 30 days.
 - 85% of sites reported having slots for routine follow-up appointments with nonprescribers within 20 days.
2. Claims data analysis indicates:
 - 50% of members had routine follow-up appointments with a prescriber within 30 days of an initial visit for a specific condition.
 - 60% of members had routine follow-up appointments with nonprescribers within 20 days of initial visit for a specific condition.
3. Complaint data analysis indicates:
 - 70% of members reported that they "always" or "usually" get a follow-up appointment with a prescriber.
 - 80% of members reported that they "always" or "usually" get a follow-up appointment with a nonprescriber.

Element C: Access to Specialty Care

Using valid methodology, the organization annually collects and analyzes data to evaluate access to appointments for:

1. High-volume specialty care.
2. High-impact specialty care.

Summary of Changes**Clarifications**

- Removed a duplicate sentence about collecting data in the second paragraph, under “data collection methods” in the Explanation.
- Deleted the fourth paragraph regarding annual quantitative analysis in the Explanation under “Data collection, quantitative and qualitative analyses.”

Scoring	Met	Partially Met	Not Met
	The organization meets 2 factors	The organization meets 1 factor	The organization meets 0 factors
Data source	Reports		
Scope of review	Product lines <i>This element applies to First Surveys and Renewal Surveys for all product lines.</i>		
	Documentation <i>For First Surveys:</i> NCQA reviews the organization’s most recent data collection and analysis report. <i>For Renewal Surveys:</i> NCQA reviews the organization’s most recent and the previous year’s data collection and analysis reports.		
Look-back period	<i>For First Surveys:</i> At least once during the prior year. <i>For Renewal Surveys:</i> 24 months.		
Explanation	Data collection methods The organization determines the data collection methodology. The data collection methodology allows identification of issues at the organizational level. The organization may collect data across the entire practitioner or member population or from a statistically valid sample. If the organization collects data using surveys or self-reported practitioner information, it supplements the data with an analysis of complaints about access. Supplemental complaint data is used to validate survey findings and self-reported information and assists in qualitative analysis of the primary data. The organization is not required to conduct a complete quantitative and qualitative analysis on supplemental data. Data collection, quantitative and qualitative analyses The organization collects data using measures that allow direct comparison against standards it defines. <i>For initial measurement,</i> the organization conducts quantitative and qualitative analysis of data.		

For remeasurement, the organization conducts quantitative analysis and conducts qualitative analysis if quantitative analysis demonstrates that stated goals were not met.

The analyses may be conducted at the organizational level (specialists and specialty practices may be grouped together), but if the analyses reveals issues, the organization conducts a practitioner-level analysis (by individual specialist) across all affected high-volume and high-impact specialty practitioners and practices or from a statistically valid sample to determine if members are able to get an appointment to see a practitioner.

Refer to *Appendix 5: Glossary* for the full definition of and requirements for *quantitative analysis* and *qualitative analysis*.

Factor 1: High-volume specialists

The organization analyzes data to determine if access to appointments with high-volume specialists identified in NET 1, Element C is sufficient for members.

Factor 2: High-impact specialists

The organization analyzes data to determine if access to appointments with high-impact specialists identified in NET 1, Element C is sufficient for members.

Exceptions

None.

Examples

Sources of data

- Member surveys.
- Self-reported access data from practitioners, supplemented with analysis of complaints.

NET 3: Assessment of Network Adequacy

The organization monitors access to health care services and takes action to improve it.

Intent

The organization provides members adequate network access for needed health care services.

Element A: Assessment of Member Experience Accessing the Network

The organization annually identifies gaps in networks specific to geographic areas or types of practitioners or providers by:

1. Using analysis results related to member experience with network adequacy for nonbehavioral healthcare services from ME 7, Element C and Element D.
2. Using analysis results related to member experience with network adequacy for behavioral healthcare services from ME 7, Element E.
3. Compiling and analyzing nonbehavioral requests for and utilization of out-of-network services.
4. Compiling and analyzing behavioral healthcare requests for and utilization of out-of-network services.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 3-4 factors	The organization meets 2 factors	The organization meets 0-1 factors
Data source	Reports		
Scope of review	Product lines <i>This element applies to First Surveys and Renewal Surveys for all product lines.</i> NCQA reviews and scores this element for each product line brought forward for Accreditation; only the commercial and Exchange product lines may be combined. Documentation <i>For First Surveys:</i> NCQA reviews the organization's most recent report. <i>For Renewal Surveys:</i> NCQA reviews the organization's most recent and the previous year's reports.		
Look-back period	<i>For First Surveys:</i> At least once during the prior year. <i>For Renewal Surveys:</i> 24 months.		
Explanation	Quantitative and qualitative analysis <i>For initial measurement,</i> the organization conducts quantitative and qualitative analysis of data.		

For remeasurement, the organization conducts quantitative analysis and conducts qualitative analysis if quantitative analysis demonstrates that stated goals were not met.

Refer to *Appendix 5: Glossary* for the full definition of and requirements for *quantitative analysis* and *qualitative analysis*.

Factors 1, 2: Analysis of data from member experience, complaints and appeals

The organization completes a quantitative and qualitative analysis, by product/product line, of member complaint and appeal data related to network adequacy (e.g., requests for out-of-network services, appeals, complaints specific to access) and member experience (CAHPS or member experience survey).

Analysis of complaints, appeals and experience related to network adequacy may be included in the overall analysis of member experience for ME 7, Elements C–E. Documentation must clearly reflect results/analysis/opportunities by product line.

Factor 3: Nonbehavioral requests for and utilization of out-of-network services

The organization compiles data on member requests for out-of-network services and data on actual out-of-network utilization to identify and monitor issues with access to non-behavioral healthcare services, practitioners and providers. The organization reports data per thousand members at the product-line level.

The organization conducts quantitative and qualitative analysis to identify possible causes of out-of-network requests and utilization and opportunities to improve network adequacy.

Factor 4: Behavioral requests for and utilization of out-of-network services

The organization compiles data on member requests for out-of-network services and data on actual out-of-network utilization to identify and monitor issues with access to behavioral healthcare services practitioners and providers. The organization reports data per thousand members at the product-line level.

The organization conducts quantitative and qualitative analysis to identify possible causes of out-of-network requests and utilization and opportunities to improve network adequacy.

Exception

Factors 2 and 4 are NA if all purchasers of the organization's services carve out or exclude behavioral healthcare.

Related information

Organizations are required to include CAHPS in their analysis if they submit CAHPS results to NCQA. If an organization submits CAHPS to NCQA but is unable to include CAHPS in its analysis because of a small denominator and an inability to report valid rates, or the organization does not meet the 15,000 minimum-enrollment threshold as noted in the *Policies and Procedures*, then the organization is not required to include CAHPS in its analysis report for factor 1.

Examples **Factor 3: Requests for and utilization of out-of-network services***UM reports*

- Member/practitioner requests for out-of-network services, including:
 - Urgent concurrent, urgent preservice, nonurgent preservice and post-service requests.
 - Final determinations resulting from these requests (approvals and denials, regardless of reason code).
- For PPO products, organizations may compile and analyze requests and final determinations for in-network level of benefit coverage.

Claims data

- Claims denied with the reason “services available in network” or other out-of-network indicator.
- For PPO products, organizations may compile and analyze claims paid with out-of-network cost sharing applied or at price tiers higher than the lowest cost-sharing level.

Requests for and utilization of out-of-network services—nonbehavioral healthcare (First Survey)

Table 1: Annual out-of-network authorization requests per 1,000 members

	Total OON Requests	Total OON Requests per 1,000 Members	Approved OON Requests	Approved OON Requests per 1,000 Members	Denied OON Requests	Denied OON Requests per 1,000 Members	Goal Met (<10 OON Requests per 1,000 Members)
Commercial/Exchange PPO	1,590	7.95	1,050	5.25	540	2.70	Y
Medicaid HMO	2,462	12.31	620	3.10	1,842	9.21	N

Table 2: Annual utilization of out-of-network services per 1,000 members

	Total Claims for OON Services	Total OON Claims per 1,000 Members	Percentage/Number of OON Claims Paid at In-Network Level	Percentage/Number of OON Claims Paid at OON Level	Percentage/Number of OON Claims Denied	Goal Met (<10 OON Claims per 1,000 Members)
Commercial/Exchange PPO	1,790	8.95	59% (1,050)	39% (700)	2% (40)	Y
Medicaid HMO	658	3.29	94% (620)	No OON Benefit	6% (38)	Test

Quantitative analysis

Table 1 includes all approved and denied requests for out-of-network services compiled using authorization data supplied from the UM system on a per 1,000-member basis (total membership population of 200,000). The organization met its performance goal of less than 10 out-of-network requests per 1,000 members for commercial/Exchange PPOs but not for the Medicaid HMO.

For commercial/Exchange PPOs, nearly twice as many out-of-network requests were approved as were denied. Most approved requests were in rural areas. For the Medicaid HMO, there were three times more out-of-network denials than approvals. The majority of approvals were for dermatology and ophthalmology.

Claims data were evaluated for actual use of out-of-network services (Table 2), regardless of approval status. Claims were evaluated in three ways: paid at the in-network level, paid at the out-of-network level and denied. The goal of less than 10 out-of-network claims per 1,000 members was met for the commercial/ Exchange PPO and the Medicaid HMO. This appears to demonstrate that authorization review processes are effective in identifying and redirecting out-of-network requests to qualified in-network providers and practitioners.

Qualitative analysis

The QI Program Director and the QI Committee reviewed the data and participated in the analysis. Analysis revealed gaps in the Medicaid HMO practitioner network for specific specialties and geographic regions. A large percentage of Medicaid HMO requests were for out-of-network dermatology services. Some requests were made due to excessive wait times for appointment or excessive driving distance, and approvals were allocated in those instances. However, a large volume of requests were due to personal preference for a practitioner who was not in network. Where services were available in network, those requests were denied. Analysis of data on claims paid did not reveal any additional insights related to the practitioner network.

The commercial/Exchange PPO network is broader than the Medicaid HMO network and does not have the same gaps in specialty care. Most approved requests were for members in rural areas (approved due to driving time); others were for highly specialized services. There were no significant trends by specialty type.

For the commercial/Exchange PPO, members may seek out-of-network services without authorization and coverage is at the lower benefit level. Authorization requests to cover out-of-network services at the in-network level resulted in 59% of out-of-network claims being paid at the in-network level. 39% of the claims were paid at the out-of-network level, either because no authorization was requested, or the authorization request was denied. Only 2% of out-of-network claims were denied in full; these were for non-emergency out-of-area services, where no benefit exists. In looking closer at the claims paid at the out-of-network level, there were no trends in geographic region or practitioner specialty.

Claims data are supplemented by results of the annual member survey, which has a question about the reasons for using out-of-network practitioners. The primary member-reported reason for using an out-of-network practitioner was preference (practitioner reputation, referred by a friend, convenience to work).

The need to increase Medicaid HMO network resources for dermatology services in rural areas was identified as an opportunity for improvement by the QI Committee, specifically as it relates to the number of available practitioners and access to timely appointments. Increasing member access to telehealth services for dermatology is one solution to address this opportunity; improving member knowledge regarding in-network ophthalmologists is another.

An opportunity for improvement may exist in the commercial/Exchange PPO product line/product to recruit additional practitioners across a variety of specialties. Further analysis will be conducted by the QI Director and QI Committee.

Element B: Opportunities to Improve Access to Nonbehavioral Healthcare Services

The organization annually:

1. Prioritizes opportunities for improvement identified from analyses of availability (NET 1, Elements A, B and C), accessibility (NET 2, Elements A and C) and member experience accessing the network (NET 3, Element A, factors 1 and 3).
2. Implements interventions on at least one opportunity, if applicable.
3. Measures the effectiveness of interventions, if applicable.

Summary of Changes**Clarifications**

- Added “Factor 3: Measuring effectiveness” subhead and text to the Explanation.

Scoring	Met	Partially Met	Not Met
	The organization meets 3 factors	The organization meets 2 factors	The organization meets 0-1 factors
Data source	Documented process, Reports, Materials		
Scope of review	Product lines <i>This element applies to First Surveys and Renewal Surveys for all product lines.</i> NCQA reviews and scores this element for each product line brought forward for Accreditation; only the commercial and Exchange product lines may be combined. Documentation <i>For First Surveys:</i> NCQA reviews the organization’s most recently completed report. <i>For Renewal Surveys:</i> NCQA reviews the organization’s most recent and previous year’s completed reports. <i>For both survey types for factor 2:</i> NCQA reviews a documented process, reports or materials, depending on the action taken to address identified opportunities.		
Look-back period	<i>For First Surveys:</i> At least once during the prior year. <i>For Renewal Surveys:</i> 24 months.		
Explanation	Factors 1–2 The organization summarizes opportunities identified from analysis of nonbehavioral healthcare data from NET 3, Element B, factor 1 to show a comprehensive overview of network access issues. Data may be reported individually, but must be evaluated collectively in a single, comprehensive analysis to meet this element. The organization prioritizes opportunities by importance to member need and risk to member access to nonbehavioral healthcare services. Factor 3: Measuring effectiveness The organization must measure the effectiveness of its actions each year. The organization may measure the same issue for both years or can select a new issue for the second year.		

Exception

Factor 3 is NA for First Surveys.

Related information

Organizations are required to include CAHPS in their analysis if they submit CAHPS results to NCQA. If an organization submits CAHPS to NCQA but is unable to include CAHPS in its analysis because of a small denominator and an inability to report valid rates, or the organization does not meet the 15,000 minimum-enrollment threshold as noted in the *Policies and Procedures*, then the organization is not required to include CAHPS in its analysis report for factor 1.

Examples None.

Element C: Opportunities to Improve Access to Behavioral Healthcare Services

The organization annually:

1. **Prioritizes opportunities for improvement identified from analyses of availability (NET 1, Elements A and D), accessibility (NET 2, Element B) and member experience accessing the network (NET 3, Element A, factors 2 and 4).**
2. **Implements interventions on at least one opportunity, if applicable.**
3. **Measures the effectiveness of the interventions, if applicable.**

Summary of Changes
Clarifications

- Added “Factor 3: Measuring effectiveness” subhead and text to the Explanation.

Corrections

- Replaced “nonbehavioral healthcare services” with “behavioral healthcare services” in the Explanation under “Factors 1–2.”

Scoring

Met	Partially Met	Not Met
The organization meets 3 factors	The organization meets 2 factors	The organization meets 0-1 factors

Data source Documented process, Reports, Materials

Scope of review**Product lines**

This element applies to First Surveys and Renewal Surveys for all product lines.

NCQA reviews and scores this element for each product line brought forward for Accreditation; only the commercial and Exchange product lines may be combined.

Documentation

For First Surveys: NCQA reviews the organization’s most recently completed report.

For Renewal Surveys: NCQA reviews the organization’s most recent and previous year’s completed report.

	<i>For factor 2 for both survey types:</i> NCQA reviews a documented process, reports or materials, depending on the action taken to address identified opportunities.
Look-back period	<i>For First Surveys:</i> At least once during the prior year. <i>For Renewal Surveys:</i> 24 months.
Explanation	Factors 1–2 The organization summarizes opportunities identified from analysis of behavioral healthcare data from NET 3, Element C, factor 1 to show a comprehensive overview of network access issues. Data may be reported individually, but must be evaluated collectively in a single, comprehensive analysis to meet this element. The organization prioritizes opportunities by importance to member need and risk to member access to behavioral healthcare services. Factor 3: Measuring effectiveness The organization must measure the effectiveness of its actions each year. The organization may measure the same issue for both years or can select a new issue for the second year. Exceptions This element is NA if all purchasers of the organization’s services carve out or exclude behavioral healthcare. Factor 3 is NA for First Surveys.
Examples	None.

NET 4: Continued Access to Care

The organization monitors and takes action, as necessary, to improve continuity and coordination of care across the health care network.

Intent

The organization uses information at its disposal to facilitate continuity and coordination of medical care across its delivery system.

Element A: Notification of Termination

The organization notifies members affected by the termination of a practitioner or practice group in general, family or internal medicine or pediatrics, at least 30 calendar days prior to the effective termination date, and helps them select a new practitioner.

Summary of Changes

Clarifications

- Deleted “and practitioners” in the Explanation under “Distribution of termination notice to members.”

Scoring

Met	Partially Met	Not Met
The organization notifies members at least 30 calendar days prior to the effective date of termination	No scoring option	The organization does not notify members at least 30 calendar days prior to the effective date of termination

Data source Documented process, Reports, Materials

Scope of review

Product lines

This element applies to First Surveys and Renewal Surveys for all product lines.

Documentation

NCQA reviews the organization’s policies and procedures or decision process in place throughout the look-back period, and evidence that members were notified of practitioner termination.

Look-back period

For First Surveys: 6 months.

For Renewal Surveys: 24 months.

Explanation

Even if there were no terminations, the organization must have policies and procedures for notifying members of practitioner terminations.

The termination date is the date when a termination becomes effective.

If a practitioner or practice group notifies the organization of termination less than 30 calendar days prior to the effective date, the organization notifies the affected members as soon as possible, but no later than 30 calendar days after receipt of the notification.

All member notifications include:

- The practitioner or group name.
- The effective termination date.
- Procedures for selecting another practitioner or group.

NCQA does not require the organization to notify members of practitioner terminations if members select a practitioner group rather than an individual practitioner.

Distribution of termination notice to members

The organization distributes the notification to members by mail, fax or email, or on its website if it informs members that the information is available online. The organization mails the notification to members who do not have fax, email or internet access.

Exceptions

None.

Related information

The organization is not required to notify members of practitioner relocations or office closures if the practitioner or office remain available to members as part of the network.

Examples

Methods to define affected members

- Claims data (e.g., number of visits to a practitioner within a specified period; receipt of periodic preventive care by the same practitioner or practice site).
- Practitioner or practice site medical records.

Element B: Continued Access to Practitioners

If a practitioner's contract is discontinued, the organization allows affected members continued access to the practitioner, as follows:

1. Continuation of treatment through the current period of active treatment, or for up to 90 calendar days, whichever is less, for members undergoing active treatment for a chronic or acute medical condition.
2. Continuation of care through the postpartum period for members in their second or third trimester of pregnancy.

Summary of Changes

- No changes to this element.

Scoring

Met	Partially Met	Not Met
The organization meets 2 factors	The organization meets 1 factor	The organization meets 0 factors

Data source

Documented process, Reports, Materials, Records or files

Scope of review	Product lines <i>This element applies to First Surveys and Renewal Surveys for all product lines.</i> Documentation NCQA reviews the organization's policies and procedures, and evidence that the organization meets the requirements throughout the look-back period.
Look-back period	<i>For First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>
Explanation	All practitioner specialties are included in factor 1. The organization has a process for identifying members seen by practitioners and practice groups in its network, and notifies members about the opportunity for continued access. Even if no contracts were discontinued, the organization must have a process for allowing members to have access to care and treatment. The organization works with practitioners who are no longer under contract to develop a reasonable transition plan for each member in active treatment or postpartum period. An active course of treatment is when a member has regular visits with the practitioner to monitor the status of an illness or disorder, provide direct treatment, prescribe medication or other treatment or modify a treatment protocol. Active treatment does not include routine monitoring for a chronic condition (e.g., monitoring chronic asthma, not for an acute phase of the condition). The postpartum period begins immediately after childbirth and extends for approximately six weeks. This element applies if the practitioner agrees to: • Continue treatment for an appropriate period of time (based on transition plan goals). • Share information about the treatment plan with the organization. • Continue to follow the organization's UM policies and procedures. • Charge only the required copayment. Exceptions The organization is not required to provide continued access if: • The practitioner is unwilling to continue to treat the member or accept the organization's payment or other terms. • The member is assigned to a practitioner group, rather than to an individual practitioner, and has continued access to practitioners in the contracted group. • The organization discontinued a contract based on a professional review action, as defined in the Health Care Quality Improvement Act of 1986 (as amended, 42 U.S.C. section 11101 et seq.).

Examples**Documentation**

- Letters to members showing continued access.
- UM cases showing continued access.
- Claims paid showing continued access.
- Case management records showing continued access.
- Report for a terminated practitioner, stating why continued access does not apply.
- Contracts with practitioners include continued access for the periods specified in factors 1 and 2.

NET 5: Physician and Hospital Directories

The organization provides information to help members and prospective members choose physicians and hospitals.

Intent

The organization's directories offer information to members and prospective members that is useful in selecting a physician and hospital.

Element A: Physician Directory Data

The organization has a web-based physician directory that includes the following physician information:

1. Name.
2. Gender.
3. Specialty.
4. Hospital affiliations.
5. Medical group affiliations.
6. Board certification.
7. Accepting new patients.
8. Languages spoken by the physician or clinical staff.
9. Office locations and phone numbers.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 8-9 factors	The organization meets 5-7 factors	The organization meets 0-4 factors
Data source	Materials		
Scope of review	Product lines <i>For First Surveys, this element applies to all product lines.</i> <i>For Renewal Surveys, this element applies to the Medicaid product line only.</i>		
	Documentation NCQA reviews the organization's web-based directory or screenshots of the website that is in place throughout the look-back period.		
Look-back period	<i>For First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	This requirement applies only to network physicians. Every field must be populated. If a factor does not apply to a physician, the data field indicates "None" or "NA."		

The directory must include all physicians who fall under the scope of credentialing defined in CR 1, Element A, with the following exceptions:

- Rental network physicians who are exclusively for out-of-area care.
- Behavioral healthcare physicians in a delegated MBHO, if the organizations directs members to the MBHO but not to specific physicians.

Factors 1, 2

No additional explanation required.

Factor 3: Specialty

The directory lists:

- All applicable specialties, for physicians.
 - The directory is not required to list subspecialty.
- All applicable disciplines, for contracted behavioral healthcare physicians.

Factor 4: Hospital affiliations

The directory lists hospitals in the organization's network where physicians have admitting or attending privileges.

Factor 5: Medical group affiliations

The directory lists medical groups in the organization's network with which the physicians are affiliated.

Factor 6: Board certification

The directory lists physicians' board certification from ABMS or AOA, **and** either:

- A link directly to ABMS or AOA to verify current status, **or**
- Instructions for checking the most current board certification status on the ABMS or AOA website.

Factor 7: Accepting new patients

The directory indicates whether physicians who practice in the following areas are accepting new patients:

- General or family medicine.
- Internal medicine.
- Pediatrics.
- Obstetrics/gynecology.
- High-volume behavioral healthcare.

Factor 8: Languages spoken by the physician or clinical staff

The directory may list languages spoken by the nonclinical staff in a separate field.

The organization is not required to include English in the list of spoken languages.

Factor 9: Office location and phone numbers

The directory lists the physical addresses and phone numbers of office locations where physicians practice.

Exception

This element is NA for Renewal Surveys for the commercial, Medicare and Exchange product lines.

Examples None.

Element B: Physician Directory Updates

The organization updates its web-based physician directory within 30 calendar days of receiving new information from the physician.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Documented process, Reports, Materials		
Scope of review	Product lines		
	<i>For First Surveys, this element applies to all product lines.</i>		
	<i>For Renewal Surveys, this element applies to the Medicaid product line only.</i>		
Look-back period	Documentation		
	NCQA reviews the organization's policies and procedures in place throughout the look-back period that states the frequency of updates and evidence that the directory was updated within 30 calendar days of receipt of new information.		
	<i>For First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	This requirement applies only to network physicians.		
	Exception		
Examples	This element is NA for Renewal Surveys for the commercial, Medicare and Exchange product lines.		
	None.		

Element C: Assessment of Physician Directory Accuracy

Using valid methodology, the organization performs an annual evaluation of its physician directories for:

1. Accuracy of office locations and phone numbers.
2. Accuracy of hospital affiliations.
3. Accuracy of accepting new patients.
4. Awareness of physician office staff of physician's participation in the organization's networks.

Summary of Changes

- No changes to this element.

Scoring	Met The organization meets 3-4 factors	Partially Met The organization meets 2 factors	Not Met The organization meets 0-1 factors
Data source	Reports		
Scope of review	Product lines <i>This element applies to First Surveys and Renewal Surveys for all product lines.</i> Documentation <i>For First Surveys:</i> NCQA reviews the organization's most recent annual report. <i>For Renewal Surveys:</i> NCQA reviews the organization's most recent and the previous year's annual reports.		
Look-back period	<i>For First Surveys:</i> At least once during the prior year. <i>For Renewal Surveys:</i> 24 months.		
Explanation	Factors 1–3: Measurement and analysis of accuracy The organization describes its methodology and provides the accuracy rate for each factor. The organization uses valid methodology to collect data on whether the information listed in its physician directories for factors 1–4 is correct. The organization may include its entire physician network in its measurement or draw statistically valid samples. If the organization uses a sample, it describes the sample universe and the sampling methodology. The organization may use data from surveys, practitioner self-reported information or member complaints regarding the accuracy of physician directories. If the organization collects information from surveys, its methodology description includes the process for physician outreach if its response rates are low. The organization is not required to conduct a quantitative analysis, but must conduct a qualitative analysis to examine the underlying reasons for or cause of results. Refer to <i>Appendix 5: Glossary</i> for the full definition of and requirements for <i>qualitative analysis</i> .		

Factor 4: Awareness of physician participation in the organization's networks

The organization provides members with accurate information on in-network physicians to avoid barriers to access. The organization determines if there is a lack of awareness on the part of physician office staff or if the organization has incorrectly listed a physician as in-network.

The organization assesses that network physician office staff can identify every product/product line and organization network in which the physician participates.

Exception

Factor 4 is NA for an integrated HMO model (all practitioners and office staff are employees of the organization).

Examples

The organization conducted an analysis of information frequently found to be out of date in its physician directories. It drew a single, representative sample of primary care practitioners and specialists across all of its networks using a 95 percent confidence level and a 5 percent confidence interval (margin of error).

For factors 1–3, the organization conducted a two-phase mailing to validate the accuracy of the data. The organization compared the survey responses with the information listed in the web directory within 30 calendar days of receiving a response, to minimize the chance that differences were due to real changes in physician information over time. If the provider's survey response matched the information published in its web directory, the information was considered accurate.

For factor 4, the organization conducted telephone surveys on practices selected in the sample and compared its directory information to information in its physician contracts.

Factor 1: Office location/phone numbers

Numerator: Number of respondents with correct addresses/phone numbers listed in the directory.

Denominator: Total number of physician offices sampled.

Office Location/ Phone Numbers	Primary Care Practitioners		Specialists	
	Commercial	Medicaid	Commercial	Medicaid
Numerator	90	75	40	31
Denominator	120	87	45	36
<i>Rate</i>	75%	86%	89%	86%

Factor 2: Hospital affiliations

Numerator: Number of respondents with correct hospital affiliations listed in the directory.

Denominator: Total number of physicians sampled.

Hospital Affiliations	Primary Care Practitioners		Specialists	
	Commercial	Medicaid	Commercial	Medicaid
Numerator	111	82	39	30
Denominator	120	87	45	36
<i>Rate</i>	93%	94%	87%	83%

Factor 3: Accepting new patients

Numerator: Number of respondents correctly listed in the directory as accepting new patients.

Denominator: Total number of physician offices sampled.

Accepting New Patients	Primary Care Practitioners		Specialists	
	Commercial	Medicaid	Commercial	Medicaid
Numerator	97	82	42	34
Denominator	120	87	45	36
<i>Rate</i>	81%	94%	93%	94%

Factor 4: Current network participation

Numerator: Number of respondents who correctly identified the networks in which they participate and the directory matched its contracts.

Denominator: Number of physician offices in the sample.

Active Network Contracts	Primary Care Practitioners		Specialists	
	Commercial	Medicaid	Commercial	Medicaid
Numerator	100	76	42	33
Denominator	120	87	45	36
<i>Rate</i>	83%	87%	93%	92%

Element D: Identifying and Acting on Opportunities

Based on results of the analysis performed in Element C, at least annually, the organization:

1. Identifies opportunities to improve the accuracy of the information in its physician directories.
2. Takes action to improve the accuracy of the information in its physician directories.

Summary of Changes**Clarifications**

- Moved the second paragraph under “Factor 1: Identifying opportunities” to be the second paragraph under “Factor 2: Acting on opportunities.”

Scoring	Met	Partially Met	Not Met
	The organization meets 2 factors	No scoring option	The organization meets 0-1 factors

Data source Documented process, Reports

Scope of review **Product lines**

This element applies to First Surveys and Renewal Surveys for all product lines.

Documentation

For First Surveys: NCQA reviews the organization’s most recent annual report.

For Renewal Surveys: NCQA reviews the organization’s most recent and the previous year’s annual reports.

For factor 2 for both survey types: NCQA may also review a documented process, depending on the action taken to address identified opportunities.

Look-back period *For First Surveys:* At least once during the prior year.
For Renewal Surveys: 24 months.

Explanation **Factor 1: Identifying opportunities**

The organization identifies opportunities to pursue from the qualitative analysis performed in Element C. The identified opportunities relate directly to the findings of the qualitative analysis.

Factor 2: Acting on opportunities

The organization demonstrates that it has acted to improve the accuracy of its physician directories.

The organization takes at least one action to improve the accuracy of information in its physician directories. Actions relate directly to the cause of the deficiency identified in the qualitative analysis.

Exception

Factor 2 is NA if the organization has no opportunities for improvement. NCQA evaluates whether this conclusion is reasonable, given assessment results.

Examples None.

Element E: Searchable Physician Web-Based Directory

The organization's web-based physician directory includes search functions with instructions for finding the following physician information:

1. Name.
2. Gender.
3. Specialty.
4. Hospital affiliations.
5. Medical group affiliations.
6. Accepting new patients.
7. Languages spoken by the physician or clinical staff.
8. Office locations.

Summary of Changes

- No changes to this element.

Scoring

Met	Partially Met	Not Met
The organization meets 7-8 factors	The organization meets 4-6 factors	The organization meets 0-3 factors

Data source Materials

Scope of review	Product lines <i>This element applies to First Surveys and Renewal Surveys for all product lines.</i> Documentation NCQA reviews the organization's web-based directory or screenshots of its web-based directory that is in place throughout the look-back period.
Look-back period	<i>For First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>
Explanation	This requirement applies only to network physicians. Factors 1–8: Physician directory data The directory: • Allows customized searches based on the information most relevant for members, or • Allows searches by each field or by zip code and specialty and contains: – An advanced-search option using multiple variables required in factors 1–8. – Instructions on using the advanced-search function and provides a direct link to the function. Exception Factors marked “No” in Element A are scored NA in this element.
Examples	None.

Element F: Hospital Directory Data

The organization has a web-based hospital directory that includes the following information:

1. Hospital name.
2. Hospital location and phone number.
3. Hospital accreditation status.
4. Hospital quality data from recognized sources.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 3-4 factors	The organization meets 2 factors	The organization meets 0-1 factors
Data source	Materials		
Scope of review	Product lines		
	For First Surveys, this element applies to all product lines.		
	For Renewal Surveys, this element applies to the Medicaid product line only.		

Documentation

NCQA reviews the organization's web-based directory or screenshots of its web-based directory that is in place throughout the look-back period.

Look-back period

For First Surveys: 6 months.

For Renewal Surveys: 24 months.

Explanation

This requirement applies only to network hospitals.

Every field must be populated. If information is not available, the data field indicates "None" or "NA."

A hospital is an institution that primarily provides diagnostic and therapeutic services to patients admitted for medical diagnosis, treatment and care of injured, disabled, or ill individuals by or under the supervision of a physician.

This element is limited to acute care hospitals including specialty acute care such as children's hospitals or Veteran Affairs hospitals.

Factor 1: Hospital name

No additional explanation required.

Factor 2: Hospital location

The directory lists the physical addresses and phone numbers of primary and affiliated locations.

Factor 3: Hospital accreditation status

The directory indicates whether the hospital is accredited, the accrediting body and the accreditation status by one of the following methods:

- Lists hospital accreditation status and the accrediting body, **or**
- Provides a link to the accrediting body's web page that displays the hospital's accreditation status.
 - A link to the general website does not meet factor 3.

Factor 4: Hospital quality data from recognized sources

The hospital directory contains quality data from recognized national or state sources, or contains a link (if it exists) to a recognized source for quality data specific to each hospital. If there are limitations (e.g., a requirement to accept terms of agreement) to linking to the data, a link to the quality data landing page is acceptable. A link to the source's general website home page is not acceptable.

Exception

This element is NA for Renewal Surveys for the commercial, Medicare and Exchange product lines.

Related information

The organization is not required to include rental network hospitals in the directory if it does not contract with the hospitals in the network.

Examples**Factor 4: Nationally recognized hospital quality sources**

- Hospital Compare.
- The Leapfrog Group.
- Quality Check.

Element G: Hospital Directory Updates

The organization updates its web-based hospital directory information within 30 calendar days of receiving new information from the hospital.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Documented process, Reports, Materials		
Scope of review	Product lines		
	<i>For First Surveys, this element applies to all product lines.</i>		
	<i>For Renewal Surveys, this element applies to the Medicaid product line only.</i>		
Look-back period	Documentation		
	NCQA reviews the organization's policies and procedures in place throughout the look-back period that states the frequency of updates and evidence showing that the directory was updated within 30 calendar days of receipt of new information.		
	<i>For First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	This requirement applies only to network hospitals.		
	Exception		
	This element is NA for Renewal Surveys for the commercial, Medicare and Exchange product lines.		
Examples	None.		

Element H: Searchable Hospital Web-Based Directory

The organization's web-based directory includes search functions for specific data types and instructions for searching for the following information:

1. Hospital name.
2. Hospital location.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 2 factors	The organization meets 1 factor	The organization meets 0 factors

Data source	Materials
Scope of review	Product lines <i>This element applies to First Surveys and Renewal Surveys for all product lines.</i> Documentation NCQA reviews the organization’s web-based directory or screenshots of its web-based directory that is in place throughout the look-back period.
Look-back period	<i>For First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>
Explanation	This requirement applies only to network hospitals. The hospital directory: • Allows searches by zip code. • Allows customized searches. • Has clear instructions for finding hospitals using the searchable fields. Factors 1, 2 No additional explanation required. Exception Corresponding factors marked “No” in Element F are scored NA in this element.
Examples	None.

Element I: Usability Testing

The organization evaluates its web-based physician and hospital directories for understandability and usefulness to members and prospective members at least every three years, and considers the following:

1. Reading level.
2. Intuitive content organization.
3. Ease of navigation.
4. Directories in additional languages, if applicable to the membership.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 3-4 factors	The organization meets 2 factors	The organization meets 0-1 factors
Data source	Documented process, Reports		
Scope of review	Product lines <i>This element applies to First Surveys and Renewal Surveys for all product lines.</i> Documentation NCQA reviews the organization's policies and procedures in place throughout the look-back period and evidence that it conducted usability testing.		
Look-back period	<i>For First Surveys and Renewal Surveys:</i> At least once in the prior 36 months.		
Explanation	The organization conducts usability testing: • When there are significant changes to member demographics. • When there are changes to the layout or design of the directory. The audience for the usability testing reflects the population that will use the directories. The organization is not required to conduct usability testing with an external audience. Testing with internal staff that were not involved in development of the physician and hospital directories meets the intent. Factors 1–4: Usability testing No additional explanation required. Exception Factor 4 is NA if the membership does not warrant directories in additional languages.		

Related information

Information on usability testing. For additional information on usability testing, refer to www.usability.gov.

Use of vendors for usability testing services. If the organization contracts with a vendor to provide usability testing services, it provides access to the vendor's documentation. NCQA does not consider the relationship to be delegation, and delegation oversight is not required under NET 6. NCQA evaluates the vendor's documentation against the requirements. Refer to *Vendors* in Appendix 2.

Examples None.

Element J: Availability of Directories

The organization makes web-based physician and hospital directory information available to members and prospective members through alternative media, including:

1. Print.
2. Telephone.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 2 factors	The organization meets 1 factor	The organization meets 0 factors
Data source	Documented process, Materials		
Scope of review	Product lines		
	<i>This element applies to First Surveys and Renewal Surveys for all product lines.</i>		
Look-back period	Documentation		
	NCQA reviews the organization's policies and procedures for making the directories available, other evidence (e.g., scripts for telephone directories, printed screenshots of screens accessed during calls) and a printed sample of the directories available throughout the look-back period.		
	<i>For First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	Factors 1, 2		
	The print and telephone directories include the same information available in the web-based physician and hospital directories.		
Examples	Exceptions		
	None.		

NET 6: Delegation of NET

If the organization delegates NCQA-required Network Management activities, there is evidence of oversight of the delegated activities.

Intent

The organization remains responsible for and has appropriate structures and mechanisms to oversee delegated Network Management activities.

Element A: Delegation Agreement

The written delegation agreement:

1. Is mutually agreed upon.
2. Describes the delegated activities and the responsibilities of the organization and the delegated entity.
3. Requires at least semiannual reporting by the delegated entity to the organization.
4. Describes the process by which the organization evaluates the delegated entity's performance.
5. Describes the process for providing member experience and clinical performance data to its delegates when requested.
6. Describes the remedies available to the organization if the delegated entity does not fulfill its obligations, including revocation of the delegation agreement.

Summary of Changes

Clarifications

- Revised the first sentence of the third paragraph under "Documentation" to clarify that the process from factor 5 must be included in the organization's documentation.
- Replaced "CAHPS 5.0H data" with "CAHPS data" in the first bullet under factor 5 in the Explanation.

Scoring	Met	Partially Met	Not Met
	The organization meets 5-6 factors	The organization meets 3-4 factors	The organization meets 0-2 factors

Data source Materials

Scope of review **Product lines**

This element applies to First Surveys and Renewal Surveys for all product lines.

Documentation

NCQA reviews delegation agreements in effect during the look-back period from up to four randomly selected delegates, or reviews all delegates if the organization has fewer than four.

Delegation agreements implemented on or after January 1, 2019, must include a description of the process required in factor 5.

For delegation agreements in place prior to January 1, 2019, the organization may provide documentation that it notified the delegate of the process required in factor 5. This documentation of notification is not required to be mutually agreed upon.

The score for the element is the average of the scores for all delegates.

Look-back period

For First Surveys: 6 months.

For Renewal Surveys: 24 months.

Explanation

This element may not be delegated.

This element applies to agreements that are in effect within the look-back period.

The delegation agreement describes all delegated NET activities. A generic policy statement about the content of delegated arrangements does not meet this element.

Factor 1: Mutual agreement

Delegation activities are mutually agreed on before delegation begins, in a dated, binding document or communication between the organization and the delegated entity.

NCQA considers the effective date specified in the delegation agreement as the mutually agreed-upon effective date. The effective date may be before or after the signature date on the agreement. If the agreement has no effective date, NCQA considers the signature date (meaning the date of last signature) as the mutually agreed upon effective date.

NCQA may accept other evidence of the mutually agreed-upon effective date: a letter, meeting minutes or other form of communication between the organization and the delegate that references the parties' agreement on the effective date of delegated activities.

NCQA requires submitted evidence for all other delegation factors to consider the same mutually agreed-upon date as the effective date for the delegate's performance of delegated activities.

Factor 2: Assigning responsibilities

The delegation agreement or an addendum thereto or other binding communication between the organization and the delegate specifies the NET activities:

- Performed by the delegate in detailed language.
- Not delegated, but retained by the organization.
 - The organization may include a general statement in the agreement addressing retained functions (e.g., the organization retains all other NET functions not specified in this agreement as the delegate's responsibility).

If the delegate subdelegates an activity, the delegation agreement must specify that the delegate or organization is responsible for subdelegate oversight.

Factor 3: Reporting

The organization determines the method of reporting and the content of the reports, but the agreement must specify:

- That reporting is at least semiannual.
- What information is reported by the delegate about delegated activities.

- How, and to whom, information is reported (i.e., joint meetings or to appropriate committees or individuals in the organization).

The organization must receive regular reports from all delegates, even NCQA-Accredited or NCQA-Certified delegates.

Factor 4: Performance monitoring

The delegation agreement specifies how the organization evaluates the delegate's performance.

Factor 5: Providing member and clinical data

The organization's delegation agreement describes what the delegate must do to obtain the following data when it is needed or on an ongoing basis:

- *Member experience data:* Complaints, CAHPS survey results or other data collected on members' experience with the delegate's services.
- *Clinical performance data:* HEDIS measures, claims and other clinical data collected by the organization. The organization may provide data feeds for relevant claims data or clinical performance measure results.

Factor 6: Consequences for failure to perform

The delegation agreement specifies consequences if a delegate fails to meet the terms of the agreement and, at a minimum, circumstances that would cause revocation of the agreement.

Exceptions

This element is NA if the organization does not delegate NET activities.

The clinical performance aspect of factor 5 is NA if clinical performance data are not relevant to the delegated activities.

Examples None.

Element B: Predelegation Evaluation

For new delegation agreements initiated in the look-back period, the organization evaluated delegate capacity to meet NCQA requirements before delegation began.

Summary of Changes

- No changes to this element.

Scoring

Met	Partially Met	Not Met
The organization evaluated delegate capacity before delegation began	The organization evaluated delegate capacity after delegation began	The organization did not evaluate delegate capacity

Data source Reports

Scope of review**Product lines**

This element applies to First Surveys and Renewal Surveys for all product lines.

This element applies if delegation was implemented in the look-back period.

Documentation

NCQA reviews the organization's predelegation evaluation from up to four randomly selected delegates, or reviewed all delegates if the organization has fewer than four.

The score for the element is the average of the scores for all delegates.

Look-back period

For First Surveys: 6 months.

For Renewal Surveys: 12 months.

Explanation

This element may not be delegated.

NCQA-Accredited/Certified delegates

Automatic credit is available if all delegates are NCQA-Accredited health plans, MBHOs or NCQA-Certified HIP organizations, unless the element is NA. NCQA-Certified HIPs must be certified in the activity being delegated by the organization.

Predelegation evaluation

The organization evaluated the delegate's capacity to meet NCQA requirements within 12 months prior to implementing delegation. The evaluation may include a review of the organization's structure, processes, and staffing in order to determine its capability to perform the delegated function.

NCQA considers the date of the agreement to be the implementation date if the delegation agreement does not include an implementation date.

If the time between the predelegation evaluation and implementation of delegation exceeds the 12 months, the organization conducts another predelegation evaluation.

If the organization amends the delegation agreement to include additional NET activities within the look-back period, it performs a predelegation evaluation for the additional activities.

Exceptions

This element is NA if:

- The organization does not delegate NET activities.
- Delegation arrangements have been in effect for longer than the look-back period.

Related information

Use of collaboratives. An organization may collaborate in a statewide, predelegation evaluation with other organizations that have overlapping practitioner and provider networks. The organizations in the collaborative use the same audit tool and share data.

Examples Predelegation evaluation

- Site visit.
- Telephone consultation.
- Documentation review.
- Committee meetings.
- Virtual review.

Element C: Review of Delegated Activities

For arrangements in effect for 12 months or longer, the organization:

1. Annually reviews its delegate's network management procedures.
2. Annually evaluates delegate performance against NCQA standards for delegated activities.
3. Semiannually evaluates regular reports, as specified in Element A.

Summary of Changes**Clarifications**

- Removed "audits" from the scope of review for First Surveys and Renewal Surveys.

Scoring	Met	Partially Met	Not Met
	The organization meets 2-3 factors	The organization meets 1 factor	The organization meets 0 factors

Data source Reports

Scope of review **Product lines**

This element applies to First Surveys and Renewal Surveys for all product lines.

Documentation

NCQA reviews evidence of the organization's review from up to four randomly selected delegates, or all delegates if the organization has fewer than four.

For First Surveys: NCQA reviews the organization's most recent annual review, performance evaluation and semiannual evaluation.

For Renewal Surveys: NCQA reviews the organization's most recent and the previous year's annual reviews, performance evaluations and four semiannual evaluations.

The score for the element is the average of the scores for all delegates.

Look-back period *For First Surveys:* At least once during the prior year.

For Renewal Surveys: 24 months.

Explanation This element may not be delegated.

Automatic credit is available for factor 2 if all delegates are NCQA-Accredited health plans, MBHOs or NCQA-Certified HIPs, unless the element is NA. NCQA-Certified HIPs must be certified in the activity being delegated by the organization.

Factor 1: Review of the delegate's network management procedures

Appropriate organization staff or committee reviews the delegate's network management procedures. At a minimum, the organization reviews parts of the network management program that apply to the delegated functions.

Factors 2, 3

No additional explanation required.

Exceptions

This element is NA if:

- The organization does not delegate NET activities.
- Delegation arrangements have been in effect for less than 12 months.

Factor 1 is NA if the organization only delegates directory functions.

Examples None.

Element D: Opportunities for Improvement

For delegation arrangements that have been in effect for more than 12 months, at least once in each of the past 2 years that delegation has been in effect, the organization identified and followed up on opportunities for improvement, if applicable.

Summary of Changes

- No changes to this element.

Scoring

Met	Partially Met	Not Met
The organization has acted on identified problems, if any, at least once in each of the past 2 years that the delegation arrangement has been in effect	The organization took inappropriate or weak action, or acted only in the past year	The organization has not acted on identified problems

Data source Documented process, Reports, Materials

Scope of review**Product lines**

This element applies to First Surveys and Renewal Surveys for all product lines.

Documentation

NCQA reviews reports of opportunities for improvement, if applicable, from up to four randomly selected delegates, or from all delegates, if the organization has fewer than four, and for evidence that the organization took appropriate action to resolve issues.

For First Surveys: NCQA reviews the organization's most recent annual review and follow-up on improvement opportunities.

	<i>For Renewal Surveys:</i> NCQA reviews the organization's most recent and previous year's annual reviews and follow-up on improvement opportunities. The score for the element is the average of the scores for all delegates.
Look-back period	<i>For First Surveys:</i> At least once during the prior year. <i>For Renewal Surveys:</i> 24 months.
Explanation	This element may not be delegated. NCQA-Accredited/Certified delegates Automatic credit is available if all delegates are NCQA-Accredited health plans, MBHOs or NCQA-Certified HIP organizations, unless the element is NA. NCQA-Certified HIPs must be certified in the activity being delegated by the organization. Identify and follow-up on opportunities The organization uses information from its predelegation evaluation, ongoing reports, or annual evaluation to identify areas of improvement, if any. Exceptions This element is NA if: • The organization does not delegate NET activities. • Delegation arrangements have been in effect for less than 12 months. • The organization has no opportunities to improve performance. – NCQA evaluates whether this conclusion is reasonable, given assessment results.
Examples	None.

Utilization Management

Standards for Utilization Management

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UM 1: Program Structure

Refer to Appendix 1 for points

The organization's UM program has clearly defined structures and processes, and assigns responsibility to appropriate individuals.

Intent

The organization has a well-structured UM program and makes utilization decisions affecting the health care of members in a fair, impartial and consistent manner.

Element A: Written Program Description

The organization's UM program description includes the following:

1. A written description of the program structure.
2. The behavioral healthcare aspects of the program.
3. Involvement of a designated senior-level physician in UM program implementation.
4. Involvement of a designated behavioral healthcare practitioner in the implementation of the behavioral healthcare aspects of the UM program.
5. The program scope and process used to determine benefit coverage and medical necessity.
6. Information sources used to determine benefit coverage and medical necessity.

Summary of Changes

Clarifications

- Removed the second bullet under "Documentation" in the scope of review.
- Removed the fourth bullet and added two subbullets under "Decisions that require medical necessity review" to clarify the care or services that require medical necessity review.
- Revised the second bullet under "Decisions that require medical necessity review" to clarify when review is needed for preexisting conditions.
- Replaced "deemed medically necessary" with "covered" in the fifth bullet under "Decisions that do not require medical necessity review."
- Added "Dental and vision services not covered under a member's medical benefits are not within the scope of denial and appeal file review" to the text under the bullets in "Decisions that do not require medical necessity review."
- Revised the paragraph under "File review universe" to clarify how federal regulations define an overturned denial.

Scoring

Met	Partially Met	Not Met
The organization meets 4-6 factors	The organization meets 3 factors	The organization meets 0-2 factors

Data source Documented process, Reports

Scope of review **Product lines**

For Interim Surveys and First Surveys, this element applies to all product lines.

For Renewal Surveys, this element applies to the Medicaid product line only.

Documentation

For Interim Surveys: NCQA reviews the organization's UM program description.

For First Surveys and Renewal Surveys: NCQA reviews the organization's UM program description.

For factors 3 and 4: NCQA also reviews UM Committee minutes or other reports that document active involvement of a senior-level physician and a designated behavioral healthcare practitioner in the UM program throughout the look-back period.

Look-back period

For Interim Surveys: Prior to the survey date.

For First Surveys: 6 months.

For Renewal Surveys: 24 months.

Explanation

This element is a **structural requirement**. The organization must present its own documentation.

The UM program description is organized and written so that staff members and others can understand the program's structure, scope, processes and information sources used to make UM determinations.

Medical necessity review

Medical necessity review is a process to consider whether services that are covered only when medically necessary meet criteria for medical necessity and clinical appropriateness. A medical necessity review requires consideration of the member's circumstances, relative to appropriate clinical criteria and the organization's policies.

NCQA's UM standards specify the steps in the medical necessity review. Medical necessity review requires that denial decisions be made only by an appropriate clinical professional as specified in NCQA standards.

Decisions about the following require medical necessity review:

- Any covered medical benefits defined by the organization's Certificate of Coverage or Summary of Benefits, including, but not limited to:
 - Dental and vision services covered under medical benefits, including dental care or services associated with procedures that occur within or adjacent to the oral cavity or sinuses.
 - If medical and dental benefits are not differentiated in the benefits plan, the organization includes requests for care or services associated with dental procedures that occur within or adjacent to the oral cavity or sinuses for medical necessity review.
 - Pharmaceuticals covered under medical or pharmacy benefits.
- Preexisting conditions when the organization has a policy to deny coverage for care or services related to preexisting conditions.
- Care or services whose coverage depends on specific circumstances.
- Out-of-network services that are only covered in clinically appropriate situations.
- Prior authorizations for pharmaceuticals and pharmaceutical requests requiring prerequisite drug for a step therapy program.
- "Experimental" or "investigational" requests covered by the organization.

Decisions about the following do not require medical necessity review:

- Services in the member's benefits plan that are limited by number, duration or frequency.
- Extension of treatments beyond the specific limitations and restrictions imposed by the member's benefits plan.
- Care or services whose coverage does not depend on any circumstances.
- Requests for personal care services, such as cooking, grooming, transportation, cleaning and assistance with other activities of daily living (ADL).
- "Experimental" or "investigational" requests that are always excluded and never covered under any circumstances. In these instances, the organization either:
 - Identifies the specific service or procedure excluded from the benefits plan, **or**
 - If benefits plan materials include broad statements about exclusions but do not specify excluded services or procedures, the materials state that members have the opportunity to request information on excluded services or procedures and the organization maintains internal policies or criteria for these services or procedures.

If the services above, which do not require medical necessity review, are covered benefits and are denied and subsequently appealed, they are within the scope of *UM 8: Policies for Appeals* and *UM 9: Appropriate Handling of Appeals*.

Dental and vision services not covered under a member's medical benefits are not within the scope of denial and appeal file review.

NCQA does not have any additional classifications of denials, such as administrative.

Medical necessity review of requests for out-of-network coverage

Requests for coverage of out-of-network services that are only covered when medically necessary or in clinically appropriate situations require medical necessity review. Such requests indicate the member has a specific clinical need that the requestor believes cannot be met in-network (e.g., a service or procedure not provided in-network; delivery of services closer or sooner than provided or allowed by the organization's access or availability standards).

If the certificate of coverage or summary of benefits specifies that the organization never covers an out-of-network service for any reason or if the request does not indicate the member has a specific clinical need for which out-of-network coverage may be warranted, the request does not require medical necessity review.

File review universe

Although medical necessity review may result in approvals or denials, NCQA reviews only denials resulting from medical necessity review, as defined above, in UM 4–UM 7, with the exception of UM 5, Element E, which applies to both approvals and denials. NCQA reviews denials, whether or not the member is at financial risk, excluding postservice payment disputes initiated by a practitioner or provider where the member is not at financial risk.

Members are considered to be at financial risk if:

- They have financial liability (co-insurance, deductibles, charges in excess of allowed amounts, differentials in cost between in-network care and out-of-network care, costs that vary within the formulary) for services beyond a flat

copay that is always the same fixed dollar amount, Copays may vary across a range of services, but must not be different within the same service category (e.g., \$15 for primary care office visits and \$25 for specialist office visits is acceptable) **or**

- They may be balance-billed by a practitioner, provider or other party.

Classification of overturned denials. Although federal regulations may define an overturned denial based on the discussion between the member's treating practitioner and another physician or other appropriate reviewer (as described in UM 7) as an appeal, such an approval does not fall under the scope of NCQA's appeal standards. The case is considered a denial if a denial notice was issued.

Organization employees and their dependents: The organization may exclude employees and their dependents from the denial and appeal file universe.

Factor 1: Program structure

The written UM description includes all the following information about the UM program structure:

- UM staff's assigned activities.
- UM staff who have the authority to deny coverage.
- Involvement of a designated physician and a designated behavioral healthcare practitioner.
- The process for evaluating, approving and revising the UM program, and the staff responsible for each step.
- The UM program's role in the QI program, including how the organization collects UM information and uses it for QI activities.
- The organization's process for handling appeals and making appeal determinations.

Staff size. NCQA does not prescribe staff size or a method or criteria for determining staff size.

Factor 2: Behavioral healthcare aspects of the program

The program description specifies how the organization addresses sites of behavioral healthcare services (e.g., psychology groups) and the levels of behavioral healthcare services (e.g., inpatient psychiatric care, outpatient psychiatrist visits.) If the organization has a process for triage and referral to behavioral health services, the program description specifies the process.

Factor 3: Senior-level physician involvement

The program description specifies how a senior-level physician (a medical director, associate medical director or equivalent) is involved in UM activities, including implementation, supervision, oversight and evaluation of the UM program.

Factor 4: Designated behavioral healthcare practitioner involvement

The program description specifies how a designated behavioral healthcare physician or a doctoral-level behavioral healthcare practitioner is involved in implementing and evaluating the behavioral health aspects of the UM program.

The behavioral healthcare practitioner must be a physician or have a clinical PhD or PsyD, and may be a medical director, clinical director, participating practitioner from the organization or behavioral healthcare delegate (if applicable).

Factors 5, 6: Processes and information sources used to make determinations

The program description specifies:

- The UM functions, the services covered by each function or protocol and the criteria used to determine medical necessity, including:
 - How the organization develops and selects criteria.
 - How the organization reviews, updates and modifies criteria.
- How medical necessity and benefits coverage for inpatient and outpatient services are determined.
- The description of the data and information the organization uses to make determinations (e.g., patient records, conversations with appropriate physicians) and guide the UM decision-making process.
 - The description should not be burdensome for the member, the practitioner or the health delivery organization's staff.
- The triage and referral process for behavioral healthcare services (if applicable).
- How sites of service and levels of care are evaluated for behavioral healthcare services (if applicable).

The program description lists the information (e.g., patient records, conversations with appropriate physicians) the organization uses to make UM determinations.

Exceptions

This element is NA for Renewal Surveys for the commercial, Medicare and Exchange product lines.

Factors 2, 4 and behavioral healthcare aspects of factor 5 are NA if all purchasers of the organization's services carve out or exclude behavioral healthcare.

Related information

Benefit plan exceptions. If the organization authorizes a service, grants an extension of benefits or makes an exception to a limitation in the benefits plan (e.g., the organization covers up to 20 visits but allows 21 visits), the granting of an exception does not set precedent such that a subsequent denial of the same service or a request for an extension or exception is considered a medical necessity determination.

Examples

Factor 3: Senior-level physician involvement

The senior-level physician's responsibilities may include, but are not limited to:

- Setting UM policies.
- Supervising program operations.
- Reviewing UM cases.
- Participating on the UM Committee.
- Evaluating the overall effectiveness of the UM program.

Factor 4: Behavioral healthcare practitioner involvement

The designated behavioral healthcare practitioner's responsibilities may include, but are not limited to:

- Setting UM behavioral healthcare policies.
- Reviewing UM behavioral healthcare cases.
- Participating on the UM Committee.

Element B: Annual Evaluation

The organization annually evaluates and updates the UM program, as necessary.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Reports		
Scope of review	Product lines <i>This element applies to all product lines for First Surveys and Renewal Surveys.</i> Documentation <i>For First Surveys:</i> NCQA reviews the organization's most recent annual evaluation reports and updates (e.g., updates report, updated UM program description), if applicable. <i>For Renewal Surveys:</i> NCQA reviews the organization's most recent and previous year's annual evaluation reports and updates (e.g., updates report, updated UM program description), if applicable.		
Look-back period	<i>For First Surveys:</i> At least once during the prior year. <i>For Renewal Surveys:</i> 24 months.		
Explanation	This element is a structural requirement. The organization must present its own documentation. To determine if its UM program remains current and appropriate, the organization annually evaluates: • The program structure. • The program scope, processes, information sources used to determine benefit coverage and medical necessity. • The level of involvement of the senior-level physician and designated behavioral healthcare practitioner in the UM program. The organization considers member and practitioner experience data when evaluating its UM program, and updates the program based on its evaluation.		

Exceptions

None.

Examples

None.

UM 2: Clinical Criteria for UM Decisions

The organization uses written criteria based on sound clinical evidence to make utilization decisions, and specifies procedures for appropriately applying the criteria.

Intent

The organization applies objective and evidence-based criteria and takes individual circumstances and the local delivery system into account when determining the medical appropriateness of health care services.

Element A: UM Criteria

The organization:

1. Has written UM decision-making criteria that are objective and based on medical evidence.
2. Has written policies for applying the criteria based on individual needs.
3. Has written policies for applying the criteria based on an assessment of the local delivery system.
4. Involves appropriate practitioners in developing, adopting and reviewing criteria.
5. Annually reviews the UM criteria and the procedures for applying them, and updates the criteria when appropriate.

Summary of Changes

Clarifications

- Revised the look-back period from 6 months to 12 months for Renewal Surveys, for factor 4.

Scoring

Met	Partially Met	Not Met
The organization meets 4-5 factors	The organization meets 3 factors	The organization meets 0-2 factors

Data source Documented process, Reports, Materials

Scope of review

Product lines

This element applies to all product lines for Interim Surveys, First Surveys and Renewal Surveys.

Documentation

For Interim Surveys: NCQA reviews the organization's policies and procedures for factors 1–5.

For First Surveys and Renewal Surveys: NCQA reviews the following:

- *For factors 1–3:* Policies and procedures in place throughout the look-back period.
- *For factor 4:* Meeting minutes or other reports documenting the involvement of appropriate practitioners throughout the look-back period.
- *For factor 5:* Most recent annual review and update (for *First Surveys*) or most recent and previous year's annual reviews and updates (for *Renewal Surveys*).

Look-back period *For Interim Surveys:* Prior to the survey date.
For First Surveys: 6 months for factors 1–4; at least once during the prior year for factor 5.
For Renewal Surveys: 24 months for factors 1–3 and 5; 12 months for factor 4.

Explanation This element is a **structural requirement**. The organization must present its own documentation.

Factor 1: Written UM decision-making criteria

The organization has specific criteria to determine the medical necessity and clinical appropriateness of medical, behavioral healthcare and pharmaceutical services requiring approval. The organization may address factors 2 and 3 as part of the UM criteria, or in separate, overriding documented processes for staff (e.g., standing instructions for staff to use when determining whether UM guidelines are appropriate for a specific situation).

Factor 2: Consideration of individual needs

The organization considers at least the following characteristics when applying criteria to each individual:

- Age.
- Comorbidities.
- Complications.
- Progress of treatment.
- Psychosocial situation.
- Home environment, when applicable.

Factor 3: Assessment of the local delivery system

The organization's UM policies and procedures require consideration of available services in the local delivery system and their ability to meet the member's specific health care needs, when UM criteria are applied.

Factor 4: Practitioner involvement

Practitioners with clinical expertise in the area being reviewed have the opportunity to advise or comment on development or adoption of UM criteria, and on instructions for applying criteria. Although the organization may use practitioners that are its staff, non-staff network practitioners must also be involved in developing, adopting and reviewing criteria, because they are subject to application of the criteria. If an organization has been unable to involve network practitioners, it must document its attempts and provide the documentation to NCQA during the survey.

The organization may have practitioners review criteria if it does not develop its own UM criteria and obtains criteria from external entities.

Factor 5: Reviewing and updating criteria

The organization reviews its UM criteria and procedures against current clinical and medical evidence and updates them, when appropriate. If new scientific evidence is not available, a designated group may determine if further review of a criterion is necessary.

Exception

Factor 5 is NA for UM criteria in use for less than 12 months.

Related information

Factors 2, 3: Applying criteria. Nationally developed procedures for applying criteria, particularly those for length of hospital stay, are often designed for “uncomplicated” patients and for a comprehensive delivery system; they may not be appropriate for patients with complications or for a delivery system with insufficient alternatives to inpatient care. Written UM procedures direct decision makers to alternative procedures or approaches (e.g., a secondary set of UM criteria and individual case discussions) when assessment indicates that UM guidelines are not appropriate.

Examples**Factor 3: Assessment of the local delivery system**

Assessment of available services in the local delivery system and their ability to meet a member’s health care needs could include:

- Availability of inpatient outpatient and transitional facilities.
- Availability of outpatient services in lieu of inpatient services such as surgicenters vs. inpatient surgery.
- Availability of highly specialized services, such as transplant facilities or cancer centers.
- Availability of skilled nursing facilities, subacute care facilities or home care in the organization’s service area to support the patient after hospital discharge.
- Local hospitals’ ability to provide all recommended services within the estimated length of stay.

Factor 4: Practitioner involvement

The organization solicits opinions about the UM criteria through either of the following:

- Practitioner participation on a committee.
- Distributing the UM criteria to applicable practitioners.

In large regional or national organizations, a central office may develop or adopt criteria if practitioners with clinical expertise are involved in development or adoption.

Element B: Availability of Criteria

The organization:

1. States in writing how practitioners can obtain UM criteria.
2. Makes the criteria available to its practitioners upon request.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 2 factors	The organization meets 1 factor	The organization meets 0 factors
Data source	Documented process, Reports, Materials		
Scope of review	Product lines <i>This element applies to all product lines for Interim Surveys and First Surveys.</i> Documentation <i>For Interim Surveys:</i> NCQA reviews the organization's Documented process for making criteria available. <i>For First Surveys:</i> NCQA reviews the organization's communication of the criteria availability to each practitioner at least once during the look-back period and that the criteria were made available upon request throughout the look-back period.		
Look-back period	<i>For Interim Surveys:</i> Prior to the survey date. <i>For First Surveys:</i> 24 months for factor 1 and 6 months for factor 2.		
Explanation	The organization distributes criteria by mail, fax or email, or on its website, if it informs practitioners that the information is available on line. The organization mails the criteria to practitioners who do not have fax, email or internet access. Factor 1: How to obtain criteria No additional explanation required. Factor 2: Availability of the UM criteria upon request The organization makes criteria available, upon request, through any of the distribution methods listed above or through either of the following methods: • In person, at the organization. • By telephone. Exception Factor 2 is NA if the organization demonstrates that it informed practitioners of the UM criteria's availability upon request, but that no practitioners requested the criteria.		
Examples	None.		

Element C: Consistency in Applying Criteria

At least annually, the organization:

1. Evaluates the consistency with which health care professionals involved in UM apply criteria in decision making.
2. Acts on opportunities to improve consistency, if applicable.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 2 factors	The organization meets 1 factor	The organization meets 0 factors
Data source	Documented process, Reports, Materials		
Scope of review	Product lines <i>This element applies to all product lines for First Surveys and Renewal Surveys.</i> Documentation NCQA reviews evaluation results or similar documentation, and evidence (e.g., minutes, policies, procedural updates) that the organization acted on opportunities.		
Look-back period	<i>For First Surveys: At least once during the prior year.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	The evaluation of interrater reliability applies only to determinations made as part of a UM process. Factor 1: Evaluation of consistency The organization evaluates the consistency with which physician and nonphysician reviewers apply UM criteria: • Using hypothetical UM test cases, or • Using a sample of UM determination files. – If the organization uses a sample of UM determination files, it uses one of the following auditing methods: ▪ 5 percent or 50 of its UM determination files, whichever is fewer, or ▪ NCQA “8/30 methodology,” available at https://www.ncqa.org/programs/health-plans/policy-accreditation-and-certification/ or ▪ Another statistically valid method.		
	Factor 2: Acting on opportunities No additional explanation required.		

Exception

Factor 2 is NA if the organization has no opportunities to improve consistency. NCQA evaluates whether this conclusion is reasonable, given evaluation results.

Examples

None.

UM 3: Communication Services

The organization provides access to staff for members and practitioners seeking information about the UM process and the authorization of care.

Intent

Members and practitioners can access staff to discuss UM issues.

Element A: Access to Staff

The organization provides the following communication services for members and practitioners:

1. Staff are available at least eight hours a day during normal business hours for inbound collect or toll-free calls regarding UM issues.
2. Staff can receive inbound communication regarding UM issues after normal business hours.
3. Staff are identified by name, title and organization name when initiating or returning calls regarding UM issues.
4. TDD/TTY services for members who need them.
5. Language assistance for members to discuss UM issues.

Summary of Changes

Clarifications

- Moved “(such as dated contract or call script)” from the second paragraph under “Documentation” in the scope of review to be bulleted text in the Examples.
- Replaced “describe” with “state” in the paragraph under “Factor 3: Staff identification.”

Scoring

Met	Partially Met	Not Met
The organization meets 4-5 factors	The organization meets 3 factors	The organization meets 0-2 factors

Data source Documented process, Materials

Scope of review

Product lines

This element applies to all product lines for Interim Surveys, First Surveys and Renewal Surveys.

Documentation

For factors 1–3: NCQA reviews the organization’s policies and procedures that are in place throughout the look-back period.

For factors 4, 5: NCQA reviews materials or other evidence that demonstrate services provided to members at least once during the look-back period.

Look-back period

For Interim Surveys: Prior to the survey date.

For First Surveys: 6 months.

For Renewal Surveys: 24 months.

Explanation	Member Services staff may triage communications to UM staff. The organization is not required to offer TDD/TTY (factor 4) and language services (factor 5) to practitioners. Factor 1: Communication during business hours No additional explanation required. Factor 2: Communication after business hours The organization uses any of the following methods for after-hours communication, as appropriate: • Telephone. • Email. • Fax. Communications received after normal business hours are returned on the next business day and communications received after midnight on Monday–Friday are responded to on the same business day. Factor 3: Staff identification The organization’s policies and procedures state that when organization staff initiate or return calls to members or practitioners regarding UM issues, they identify themselves by name, title and the organization’s name. Factor 4: TDD/TTY services TDD (telecommunications device for the deaf) or TTY (telephone typewriter, or teletypewriter) are electronic devices for text communication via a telephone line, used when one or more parties have hearing or speech difficulties. The organization provides a separate phone number for receiving TDD/TTY messages or uses the State/711 Relay Services. Factor 5: Language assistance For all members who request language services, the organization provides services, free of charge, in the requested language through bilingual staff or an interpreter. This factor does not apply to after-hours communications. Use of contracted translation services is not considered delegation.
Exceptions	None.
Examples	Factors 4, 5: TDD/TTY services and language assistance • Dated contracts. • Dated call scripts.

UM 4: Appropriate Professionals

Qualified licensed health professionals assess the clinical information used to support UM decisions.

Intent

UM decisions are made by qualified health professionals.

Element A: Licensed Health Professionals

The organization has written procedures:

1. Requiring appropriately licensed professionals to supervise all medical necessity decisions.
2. Specifying the type of personnel responsible for each level of UM decision making.

Summary of Changes

- No changes to this element.

Scoring

Met	Partially Met	Not Met
The organization meets 2 factors	The organization meets 1 factor	The organization meets 0 factors

Data source Documented process

Scope of review **Product lines**
For Interim Surveys and First Surveys, this element applies to all product lines.
For Renewal Surveys, this element applies to the Medicaid product line only.

Documentation

NCQA reviews the organization's policies and procedures.

Look-back period *For Interim Surveys:* Prior to the survey date.
For First Surveys: 6 months.
For Renewal Surveys: 24 months.

Explanation This element is a **structural requirement**. The organization must present its own documentation.

Factor 1: Supervision

The policies and procedures specify that the organization uses licensed health care professionals to supervise UM activities. These licensed health care professionals:

- Provide day-to-day supervision of assigned UM staff.
- Participate in staff training.
- Monitor for consistent application of UM criteria by UM staff, for each level and type of UM decision.

- Monitor documentation for adequacy.
- Are available to UM staff on site or by telephone.

Factor 2: UM personnel and appropriate professionals

The policies and procedures specify that the organization uses licensed health care professionals to make UM decisions that require clinical judgment. The following staff may approve services:

- Staff who are not qualified health care professionals and are under the supervision of appropriately licensed health professionals, when there are explicit UM criteria and no clinical judgment is required.
- Licensed health care professionals.

Exception

This element is NA for Renewal Surveys for the commercial, Medicare and Exchange product lines.

Related information

UM oversight. As specified in UM 1, Element A, a senior-level physician oversees the UM program and a designated behavioral healthcare practitioner is involved in the behavioral health aspects of the UM program. These individuals are not required to have day-to-day involvement in UM activities.

For doctoral-level clinical psychologists, such authority must be in the scope of their license to practice.

Examples None.

Element B: Use of Practitioners for UM Decisions

The organization has a written job description with qualifications for practitioners who review denials of care based on medical necessity. Practitioners are required to have:

1. Education, training or professional experience in medical or clinical practice.
2. A current clinical license to practice or an administrative license to review UM cases.

Summary of Changes

Clarifications

- Replaced “meets” with “includes” in the “Not Met” column of the scoring table.

Scoring

Met	Partially Met	Not Met
The description includes 2 factors for all appropriate practitioners	The description includes 1 factor for all appropriate practitioners	The description includes 0 factors

Data source Materials

Scope of review **Product lines**

For Interim Surveys and First Surveys, this element applies to all product lines.

For Renewal Surveys, this element applies to the Medicaid product line only.

Documentation

NCQA reviews the organization's practitioner job descriptions in place during the look-back period.

Look-back period

For Interim Surveys: Prior to the survey date.

For First Surveys: 6 months.

For Renewal Surveys: 24 months.

Explanation

This element is a **structural requirement**. The organization must present its own Materials.

The written description applies to all types of practitioners who may review denials of care based on medical necessity for the organization. NCQA considers the following practitioner types to be appropriate for review of the specified UM denial decisions:

- *Physicians, all types:* Medical, behavioral healthcare, pharmaceutical, dental, chiropractic and vision denials.
- *Nurse practitioners*:* Medical, behavioral healthcare, pharmaceutical, dental, chiropractic and vision denials.
- *Doctoral-level clinical psychologists or certified addiction-medicine specialists:* Behavioral healthcare denials.
- *Pharmacists:* Pharmaceutical denials.
- *Dentists:* Dental denials.
- *Chiropractors:* Chiropractic denials.
- *Physical therapists:* Physical therapy denials.
- *Doctoral-level board-certified behavioral analysts:* Applied behavioral analysis denials.

* In states where the organization has determined that practice acts or regulations allow nurse practitioners to practice independently, nurse practitioners may review requests that are within the scope of their license.

Factor 1: Education, training or professional experience

No additional explanation required.

Factor 2: Current clinical or administrative license

An **administrative license** is a limited license that allows practitioners to use clinical skills and knowledge to make judgment on the medical or clinical appropriateness of requested care or services but does not convey the authority to practice clinical medicine or to prescribe medications.

Exception

This element is NA for Renewal Surveys for the commercial, Medicare and Exchange product lines.

Examples

None.

Element C: Practitioner Review of Nonbehavioral Healthcare Denials

The organization uses a physician or other health care professional, as appropriate, to review any nonbehavioral healthcare denial based on medical necessity.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	High (90-100%) on file review	Medium (60-89%) on file review	Low (0-59%) on file review
Data source	Records or files		
Scope of review	Product lines		
	<i>This element applies to all product lines for First Surveys and Renewal Surveys.</i>		
	NCQA reviews and scores this element for each product line brought forward for Accreditation; only the commercial and Exchange product line files may be combined.		
Look-back period	Documentation		
	<i>For First Surveys and Renewal Surveys:</i> NCQA reviews a random sample of up to 40 nonbehavioral healthcare denial files resulting from medical necessity review for evidence that the files were reviewed by an appropriate practitioner.		
	<i>For First Surveys:</i> 6 months. <i>For Renewal Surveys:</i> 12 months.		
Explanation	THIS IS A MUST-PASS ELEMENT.		
	This element applies to all nonbehavioral healthcare denial determinations resulting from medical necessity review (as defined in UM1, Element A).		
	Dispute of file review results		
	NCQA conducts onsite file review in the presence of the organization's staff and works with the organization to resolve any disputes during the onsite survey. An organization that is unable to resolve a dispute with the survey team must contact NCQA before the onsite survey is complete. File review results may not be disputed or appealed once the onsite survey is complete.		
	Appropriate practitioner review		
	Appropriate practitioners review all medical necessity denials for requested health care services offered under the organization's medical benefit. NCQA does not require practitioner review of requests for medical services that are specifically excluded from the benefits plan or that exceed the limitations or restrictions stated in the benefits plan.		

The UM denial file includes any of the following documentation of appropriate professional review:

- The reviewer's handwritten signature or initials.
- The reviewer's unique electronic signature or identifier on the denial letter or on the notation of denial in the file.
- A signed or initialed note from a UM staff person, attributing the denial decision to the professional who reviewed and decided the case.

Exceptions

None.

Related information

UM denial file exclusions. NCQA does not include UM decisions that do not require medical necessity review within the UM denial file sample.

Dental benefits. If medical and dental benefits are not differentiated in the benefits plan, the organization identifies dental care or services associated with procedures that occur within or adjacent to the oral cavity or sinuses. NCQA requires medical necessity review of requests for these services.

Examples None.

Element D: Practitioner Review of Behavioral Healthcare Denials

The organization uses a physician or appropriate behavioral healthcare practitioner, as appropriate, to review any behavioral healthcare denial of care based on medical necessity.

Summary of Changes

- No changes to this element.

Scoring

Met	Partially Met	Not Met
High (90-100%) on file review	Medium (60-89%) on file review	Low (0-59%) on file review

Data source Records or files

Scope of review

Product lines

This element applies to all product lines for First Surveys and Renewal Surveys.

NCQA reviews and scores this element for each product line brought forward for Accreditation; only the commercial and Exchange product line files may be combined.

Documentation

For First Surveys and Renewal Surveys: NCQA reviews a random sample of up to 40 behavioral healthcare denial files resulting from medical necessity review for evidence that the files were reviewed by an appropriate practitioner.

Look-back period

For First Surveys: 6 months.

For Renewal Surveys: 12 months.

Explanation THIS IS A MUST-PASS ELEMENT

This element applies to all behavioral healthcare denial determinations resulting from medical necessity review (as defined in UM 1, Element A).

Dispute of file review results

NCQA conducts onsite file review in the presence of the organization's staff and works with the organization to resolve any disputes during the onsite survey. An organization that is unable to resolve a dispute with the survey team must contact NCQA before the onsite survey is complete. File review results may not be disputed or appealed once the onsite survey is complete.

Appropriate practitioner review

Appropriate practitioners review all medical necessity denials for requested health care services offered under the organization's behavioral healthcare benefit. NCQA does not require practitioner review of requests for medical services that are specifically excluded from the benefits plan or that exceed the limitations or restrictions stated in the benefits plan.

The UM denial file includes any of the following documentation of appropriate professional review:

- The reviewer's handwritten signature or initials.
- The reviewer's unique electronic signature or identifier on the denial letter or on the notation of denial in the file.
- A signed or initialed note from a UM staff person, attributing the denial decision to the professional who reviewed and decided the case.

Exception

This element is NA if all purchasers of the organization's services carve out or exclude behavioral healthcare.

Related information

UM denial file exclusions. NCQA does not include UM decisions in the UM denial file sample that do not require medical necessity review.

Examples None.

Element E: Practitioner Review of Pharmacy Denials

The organization uses a physician or a pharmacist to review pharmacy denials based on medical necessity.

Summary of Changes**Clarifications**

- Removed the second sentence in the third bullet under "Appropriate practitioner review" because it duplicated the text in the first bullet.

Scoring

Met	Partially Met	Not Met
High (90-100%) on file review	Medium (60-89%) on file review	Low (0-59%) on file review

Data source Records or files

Scope of review **Product lines**

This element applies to all product lines for First Surveys and Renewal Surveys.

NCQA reviews and scores this element for each product line brought forward for Accreditation; only the commercial and Exchange product line files may be combined.

Documentation

For First Surveys and Renewal Surveys: NCQA reviews a random sample of up to 40 pharmaceutical denial files resulting from medical necessity review for evidence that the files were reviewed by an appropriate practitioner.

Look-back period *For First Surveys:* 6 months.

For Renewal Surveys: 12 months.

Explanation **THIS IS A MUST-PASS ELEMENT.**

This element applies to all pharmaceutical denial determinations resulting from medical necessity review (as defined in UM 1, Element A), whether the pharmaceutical is covered under an organization's medical benefit or its pharmacy benefit.

Dispute of file review results

NCQA conducts onsite file review in the presence of the organization's staff and works with the organization to resolve any disputes during the onsite survey. An organization that is unable to resolve a dispute with the survey team must contact NCQA before the onsite survey is complete. File review results may not be disputed or appealed once the onsite survey is complete.

Appropriate practitioner review

Appropriate practitioners review all medical necessity denials for requested pharmaceuticals offered under the organization's benefits. NCQA does not require practitioner review of requests for medical services that are specifically excluded from the benefits plan or that exceed the limitations or restrictions stated in the benefits plan.

The UM denial file includes any of the following as documentation of appropriate professional review:

- The reviewer's handwritten signature or initials.
- The reviewer's unique electronic signature or identifier on the denial letter or on the notation of denial in the file.
- A signed or initialed note from UM staff, attributing the denial decision to the professional who reviewed and decided the case.

Exception

This element is NA if all purchasers of the organization's services carve out or exclude pharmaceutical management.

Related information

UM denial file exclusions. NCQA does not include the following UM decisions in the UM denial file sample:

- UM decisions that do not require medical necessity review.
- UM approvals, including substitution of a generic pharmaceutical for a name-brand pharmaceutical.
- Pharmacy benefit denials resulting from closed formularies and related to tiered formularies, copayments, generic substitution and therapeutic interchange.

Examples None.

Element F: Use of Board-Certified Consultants

The organization:

1. Has written procedures for using board-certified consultants to assist in making medical necessity determinations.
2. Provides evidence that it uses board-certified consultants for medical necessity determinations.

Summary of Changes
Clarifications

- Revised the look-back period from 24 months to 6 months for Renewal Surveys.
- Replaced “Factor 1: Procedures for using board-certified consultants” with “Factor 1: Policies and procedures for using board-certified consultants” subhead and text.
- Added text under “Factor 2: Evidence of use of board-certified consultants.”
- Deleted the exception for factor 2 for when organizations have all specialties available.
- Added an exception for factor 2 for Interim Surveys.

Policy Changes

- Revised the two paragraphs under “Documentation” in the scope of review to specify the data sources NCQA reviews for factors 1 and 2.

Scoring

Met	Partially Met	Not Met
The organization meets 2 factors	The organization meets factor 2 only	The organization meets factor 1 only, or meets 0 factors

Data source Documented process, Reports, Materials, Records or files

Scope of review**Product lines**

This element applies to all product lines for Interim Surveys, First Surveys and Renewal Surveys.

Documentation

For All Surveys: For factor 1, NCQA reviews the organization’s written policies and procedures for using all board-certified consultants, including internal and external board-certified consultants, and reviews the list of board-certified consultants used.

For First Surveys and Renewal Surveys: For factor 2, NCQA also reviews three cases showing the use of external board-certified consultants during the look-back period. If there are not three external cases, NCQA also reviews internal cases, for a total of three cases. If the organization does not use external board-certified consultants, NCQA reviews three internal cases.

Look-back period

For Interim Surveys: Prior to the survey date.

For First and Renewal Surveys: 6 months.

Explanation

This element applies to medical necessity determinations.

Factor 1: Policies and procedures for using board-certified consultants

The organization has written policies and procedures for using board-certified consultants, including internal and external board-certified consultants. The organization maintains a list of board-certified consultants that includes contact information (e.g., names, specialties) and makes the list available to UM staff as a reference for contacting those consultants.

Factor 2: Evidence of use of board-certified consultants

NCQA reviews three cases showing the use of external board-certified consultants for medical necessity determinations. If the organization does not have three cases of the use of external board-certified consultants, NCQA also reviews internal cases, for a total of three cases.

If the organization does not use external consultants for medical necessity determinations because all specialties are available in the organization, NCQA reviews three cases of the use of internal consultants. Network practitioners are not considered part of the organization.

Exception

Factor 2 is NA for Interim Surveys.

Related information

NCQA does not consider it delegation if a board-certified consultant reviews cases and makes a recommendation for medical necessity determinations, if the organization makes the final decision. If the consultant makes the final decision, NCQA considers this to be delegation.

Examples**Factors 1, 2: Use of board-certified consultant**

An attending physician believes a newborn is suffering from a neurological disorder. The physician requests approval for the infant to be treated by a pediatric neurologist. The organization does not have a pediatric neurologist on staff, but it does have access to a board-certified pediatric neurologist through a consulting firm. The organization collects the necessary clinical information and sends it to the consulting neurologist, who replies with a recommendation for authorization to an out-of-network pediatric neurologist within 24 hours.

UM 5: Timeliness of UM Decisions

The organization makes UM decisions in a timely manner to accommodate the clinical urgency of the situation.

Intent

The organization makes UM decisions in a timely manner to minimize any disruption in the provision of health care.

Element A: Notification of Nonbehavioral Healthcare Decisions

The organization adheres to the following time frames for notification of non-behavioral healthcare UM decisions:

1. For commercial and Exchange urgent concurrent decisions, the organization gives electronic or written notification of the decision to members and practitioners within 24 hours of the request.
2. For Medicare and Medicaid urgent concurrent decisions, the organization gives electronic or written notification of the decision to members and practitioners within 72 hours of the request.
3. For urgent preservice decisions, the organization gives electronic or written notification of the decision to members and practitioners within 72 hours of the request.
4. For commercial and Exchange nonurgent preservice decisions, the organization gives electronic or written notification of the decision to members and practitioners within 15 calendar days of the request.
5. For Medicare and Medicaid nonurgent preservice decisions, the organization gives electronic or written notification of the decision to members and practitioners within 14 calendar days of the request.
6. For postservice decisions, the organization gives electronic or written notification of the decision to members and practitioners within 30 calendar days of the request.

Summary of Changes

Clarifications

- Replaced “Notification of Nonbehavioral Decisions” with “Notification of Nonbehavioral Healthcare Decisions” in the element title.
- Added “commercial and Exchange” to factor 4.
- Moved the explanation text about Medicare and Medicaid time frames for nonurgent preservice requests to the stem as a new factor 5 and renumbered the references throughout the element to align with the addition.
- Replaced “Classification of UM requests” with “Definitions used when classifying UM requests” in the Explanation.
- Revised the last paragraph under “Definitions used for classifying UM requests” to clarify requests that may be classified as preservice or postservice.
- Removed “Factor 4: Nonurgent preservice” subhead and text from the Explanation.
- Revised the Medicaid extension condition under “Factors 2, 3: Urgent concurrent and urgent preservice requests” to align with regulations.
- Replaced “Factors 4, 5: Nonurgent preservice and postservice requests” with “Factor 4: Nonurgent preservice requests for commercial and Exchange product lines.”

- Added “Factor 5: Nonurgent preservice for Medicare and Medicaid product lines” subhead and text.
- Added “Factor 6: Postservice requests for commercial, Exchange and Medicaid product lines” subhead and text.
- Added a sentence under “Extension for other reasons” that for Medicare, extensions are not allowed for postservice requests.
- Added a last paragraph under “Failure to follow filing procedures” to clarify that the organization must provide a reason for denying a postservice request for failure to follow filing procedures.

Scoring

Met	Partially Met	Not Met
High (90-100%) on file review	Medium (60-89%) on file review	Low (0-59%) on file review

Data source Records or files

Scope of review**Product lines**

This element applies to all product lines for First Surveys and Renewal Surveys.

NCQA reviews and scores this element for each product line brought forward for Accreditation; only the commercial and Exchange product line files may be combined.

Documentation

For First Surveys and Renewal Surveys: NCQA reviews a random sample of up to 40 nonbehavioral healthcare denial files resulting from medical necessity review for evidence of timeliness of notification.

Look-back period

For First Surveys: 6 months.

For Renewal Surveys: 12 months.

Explanation**THIS IS A MUST-PASS ELEMENT**

This element applies to all nonbehavioral healthcare denial determinations resulting from medical necessity review (as defined in UM 1, Element A).

Dispute of file review results

NCQA conducts onsite file review in the presence of the organization’s staff and works with the organization to resolve any disputes during the onsite survey. An organization that is unable to resolve a dispute with the survey team must contact NCQA before the onsite survey is complete. File review results may not be disputed or appealed once the onsite survey is complete.

Definitions used when classifying UM requests

Urgent request: A request for medical care or services where application of the time frame for making routine or non-life threatening care determinations:

- Could seriously jeopardize the life or health of the member or the member’s ability to regain maximum function, based on a prudent layperson’s judgment, **or**
- Could seriously jeopardize the life, health or safety of the member or others, due to the member’s psychological state, **or**

- In the opinion of a practitioner with knowledge of the member's medical or behavioral condition, would subject the member to adverse health consequences without the care or treatment that is the subject of the request.

Concurrent request: A request for coverage of medical care or services made while a member is in the process of receiving the requested medical care or services, even if the organization did not previously approve the earlier care.

Nonurgent request: A request for medical care or services for which application of the time periods for making a decision does not jeopardize the life or health of the member or the member's ability to regain maximum function and would not subject the member to severe pain.

Preservice request: A request for coverage of medical care or services that the organization must approve in advance, in whole or in part.

Postservice request: A request for coverage of medical care or services that have been received (e.g., retrospective review).

Reclassification of nonbehavioral requests that do not meet the definition of "urgent." All types of requests received while the member is receiving care may be reclassified as preservice or postservice if the request does not meet the definition of "urgent." This includes a request to extend a course of treatment beyond the time period or number of treatments previously approved by the organization. The request may be handled as a new request and decided within the time frame appropriate for the type of decision notification (i.e., preservice or postservice).

Factors 1–6: Timeliness of notification

NCQA considers 24 hours to be equivalent to 1 calendar day and 72 hours to be equivalent to 3 calendar days.

NCQA measures timeliness of notification from the date when the organization receives the request from the member or the member's authorized representative, even if the organization does not have all the information necessary to make a decision, to the date when the notice was provided to the member and practitioner, as applicable.

For Medicare only: NCQA measures timeliness of notification for urgent requests from the date when the appropriate department receives the request.

The organization documents the date when it receives the request, and the date of the decision notification, in the UM file. The request is received when it arrives at the organization, even if it is not received by the UM department.

If the organization sends written notice, NCQA uses the date on the notice as the notification date. If the organization does not retain copies of the written notice, it has other methods of documenting the notification date. If the organization uses electronic notification, NCQA uses the date when the notification was posted in the electronic system.

An organization may have procedures for ongoing review of urgent concurrent care it approved initially. For ongoing reviews, the notification period begins on the day of the review. The organization documents the date of the ongoing review and the decision notification in the UM denial file.

The organization may extend the decision notification time frame under certain circumstances. Refer to *Related information*.

Exceptions

Exceptions to member notification. NCQA does not require the organization to notify a member of:

- An urgent concurrent denial.
- An urgent preservice denial.
- A postservice (retrospective) denial if the member is not at financial risk.

For urgent denials, NCQA considers the attending or treating practitioner to be acting as the member's representative. During the file review process, NCQA assesses whether the decision notification time frames to the practitioner were appropriate.

Factors 1, 4 are NA for the Medicare and Medicaid product lines.

Factors 2, 5 are NA for the commercial and Exchange product lines.

Related information

Notifying the practitioner. If information on the attending or treating practitioner was not provided with the request, the organization attempts to identify the practitioner. The organization documents its efforts to identify the practitioner.

For urgent concurrent decisions, the organization may inform the hospital Utilization Review (UR) department staff without attempting to identify the treating practitioner, with the understanding that staff will inform the attending/treating practitioner.

In all cases, if the practitioner is not known, the organization must address the notification to the attention of the attending or treating practitioner; the practitioner name is not required.

Receiving requests after normal business hours. Due to the nature of urgent requests, the organization has procedures for accepting them after normal business hours. NCQA counts the time from the date when the organization receives the request, whether or not it is during business hours.

Postservice payment disputes. Postservice requests for payment initiated by a practitioner or a facility are not subject to review if the practitioner or facility has no recourse against the member for payment (i.e., the member is not at financial risk). Exclude denials of such requests from the file review universe.

Approving alternative services. If the organization approves an alternative to the service being requested and the member or the member's authorized representative does not request or agree to the alternative service, the organization would be denying care that was originally requested; therefore, this is considered a denial and should be included in the file review universe. However, if the member or the member's authorized representative agrees to the alternative and the care is authorized, the practitioner has essentially withdrawn the initial request; therefore, this is not considered a denial and should not be included in the file review universe.

Extending time frames. Members or their authorized representatives may agree to extend the time frame for urgent, preservice and postservice requests.

*Extension conditions*Factor 1: Urgent concurrent requests for commercial and Exchange product lines.

- The organization may extend the decision notification time frame if the request to extend urgent concurrent care was made less than 24 hours prior to the expiration of the previously approved time period or number of treatments. The organization may treat the request as urgent preservice and send a decision notification within 72 hours.
- The organization may extend the decision notification time frame if the request to approve additional days for urgent concurrent care is related to care not previously approved by the organization and the organization documents that it made at least one attempt and was unable to obtain the needed clinical information within the initial 24 hours after the request for coverage of additional days. In this case, the organization has up to 72 hours to make the decision.

Factors 2, 3: Urgent concurrent and urgent preservice requests for Medicare and Medicaid product lines.

For Medicare, the organization may extend the time frame once, by up to 14 calendar days, under the following conditions:

- The member requests an extension, **or**
- The organization needs additional information, **and**
 - Documents that it made at least one attempt to obtain the necessary information.
 - Notifies the member or the member's authorized representative of the delay.

The organization notifies the member or the member's authorized representative of its decision as expeditiously as the member's health condition requires, but no later than the expiration of the extension.

For Medicaid, the organization may extend the time frame once, by up to 14 calendar days, under the following conditions:

- The member requests an extension, **or**
- The organization needs additional information, provided it documents that it made at least one attempt to obtain the necessary information.

The organization notifies the member or the member's authorized representative of its decision as expeditiously as the member's health condition requires, but no later than the expiration of the extension.

Factor 3: Urgent preservice requests for commercial and Exchange product lines.

The organization may extend the urgent preservice time frame once due to lack of information, for 48 hours, under the following conditions:

- Within 24 hours of receipt of the urgent preservice request, the organization asks the member or the member's representative for the information necessary to make the decision, **and**
- The organization gives the member or the member's authorized representative at least 48 hours to provide the information, **and**
- The extension period, within which a decision must be made by the organization, begins on the sooner of:
 - The date when the organization receives the member's response (even if not all of the information is provided), **or**

- The last date of the time period given to the member to provide the information, even if no response is received from the member or the member's authorized representative.

Factor 4: Nonurgent preservice requests for commercial and Exchange product lines. If the request lacks clinical information, the organization may extend the nonurgent preservice time frame for up to 15 calendar days, under the following conditions:

- Before the end of the time frame, the organization asks the member or the member's representative for the information necessary to make the decision, **and**
- The organization gives the member or the member's authorized representative at least 45 calendar days to provide the information.
- The extension period, within which a decision must be made by the organization, begins on the sooner of:
 - The date when the organization receives the member's response (even if not all of the information is provided), **or**
 - The last date of the time period given to the member to supply the information, even if no response is received from the member or the member's authorized representative.

The organization may deny the request if it does not receive the information within the time frame, and the member may appeal the denial.

Factor 5: Nonurgent preservice requests for Medicare and Medicaid product lines.

For Medicare, the organization may extend the time frame once, by up to 14 calendar days, under the following conditions:

- The member requests an extension, **or**
- The organization needs additional information, **and**
 - Documents that it made at least one attempt to obtain the necessary information.
 - Notifies the member or the member's authorized representative of the delay.

The organization notifies the member or the member's authorized representative of its decision as expeditiously as the member's health condition requires, but no later than the expiration of the extension.

For Medicaid, the organization may extend the time frame once, by up to 14 calendar days, under the following conditions:

- The member requests an extension, **or**
- The organization needs additional information, provided it documents that it made at least one attempt to obtain the necessary information.

The organization notifies the member or the member's authorized representative of its decision as expeditiously as the member's health condition requires, but no later than the expiration of the extension.

Factor 6: Postservice requests for commercial, Exchange and Medicaid product lines. If the request lacks clinical information, the organization may extend the postservice time frame for up to 15 calendar days, under the following conditions:

- Before the end of the time frame, the organization asks the member or the member's representative for the information necessary to make the decision, **and**
- The organization gives the member or the member's authorized representative at least 45 calendar days to provide the information.
- The extension period, within which a decision must be made by the organization, begins on the sooner of:
 - The date when the organization receives the member's response (even if not all of the information is provided), **or**
 - The last date of the time period given to the member to supply the information, even if no response is received from the member or the member's authorized representative.

The organization may deny the request if it does not receive the information within the time frame, and the member may appeal the denial.

Extension for other reasons. In a situation beyond the organization's control (e.g., waiting for an evaluation by a specialist), it may extend the nonurgent preservice and postservice time frames once, for up to 15 calendar days.

- Within 15 calendar days of a preservice request, the organization notifies the member (or the member's authorized representative) of the need for an extension and the expected date of the decision.
- Within 30 calendar days of a postservice request, the organization notifies the member (or the member's authorized representative) of the need for an extension and the expected date of the decision.

For Medicare, extensions are not allowed for postservice requests.

Factors 1–3: Verbal notification of denials. Verbal notification does not replace electronic or written notification of denial decisions, but when provided, the organization may extend the time frame for electronic or written notification for commercial, Medicare and Exchange decisions as described below.

- Verbal notification requires communication with a live person; the organization may not leave a voicemail, **and**
- The organization records the time and date of the notification and the staff member who spoke with the practitioner or member, **and**
- The organization provides verbal notification within the time frames specified for an urgent concurrent or urgent preservice request.

For commercial, Medicare and Exchange decisions, if the organization provides verbal notification of a denial decision as specified for an urgent concurrent or urgent preservice request, it has an additional 3 calendar days following verbal notification to provide electronic or written notification.

For Medicaid decisions, providing verbal notification does not extend the electronic or written notification time frame.

Failure to follow filing procedures. If the member (or the member's authorized representative) does not follow the organization's reasonable filing procedures for requesting preservice or urgent concurrent coverage, the organization notifies the member (or the member's authorized representative) of the failure and informs them of the proper procedures to follow when requesting coverage.

- *For urgent preservice and concurrent decisions*, the organization notifies the practitioner (member's authorized representative) or member within 24 hours of receiving the request. Notification may be verbal, unless the practitioner or member requests written notification.
- *For nonurgent preservice decisions*, the organization notifies the member or the member's authorized representative within 5 calendar days of receiving the request.

The organization may not deny a nonurgent preservice, urgent preservice or urgent concurrent request that requires medical necessity review for failure to follow filing procedures.

The organization may deny a postservice request if the member (or the member's authorized representative) does not follow the organization's reasonable filing procedures, but must provide the reason for the denial.

Use of practitioner web portals. The organization may provide electronic denial notifications to practitioners through a web portal if:

- The organization informs practitioners of the notification mechanism and their responsibility to check the portal regularly, **and**
- The organization documents the date and time when the information was posted in the portal, **and**
- The information posted in the portal meets the requirement in UM 4–UM 7, **and**
- The organization has an alternative notification method for practitioners who do not have access to the web portal.

Use of member web portals. The organization may provide electronic denial notifications to members through a web portal if all the following criteria are met:

- The organization documents the member's agreement to receive electronic notifications via the portal.
- The organization documents the date and time when the information was posted in the portal.
- Members receive notification that a new document or update is available in the portal when it is posted (e.g., text, email, other electronic notification).
- The information posted in the portal meets the requirements in UM 4–UM 7.
- The organization has an alternative notification method for members who do not have access to the web portal or who do not agree to receive notifications via the portal.

Examples

Failure to follow filing procedures

- An organization's procedure is that members or practitioners submit UM requests in writing but the member or practitioner files a request over the phone.
- An organization's procedure is that members or practitioners submit requests within a specific time frame but the member or practitioner submits the request outside the time frame.

Element B: Notification of Behavioral Healthcare Decisions

The organization adheres to the following time frames for notification of behavioral healthcare UM decisions:

1. For commercial and Exchange urgent concurrent decisions, the organization gives electronic or written notification of the decision to members and practitioners within 24 hours of the request.
2. For Medicare and Medicaid urgent concurrent decisions, the organization gives electronic or written notification of the decision to members and practitioners within 72 hours of the request.
3. For urgent preservice decisions, the organization gives electronic or written notification of the decision to members and practitioners within 72 hours of the request.
4. For commercial and Exchange nonurgent preservice decisions, the organization gives electronic or written notification of the decision to members and practitioners within 15 calendar days of the request.
5. For Medicare and Medicaid nonurgent preservice decisions, the organization gives electronic or written notification of the decision to members and practitioners within 14 calendar days of the request.
6. For postservice decisions, the organization gives electronic or written notification of the decision to practitioners and members within 30 calendar days of the request.

Summary of Changes

Clarifications

- Replaced “Notification of BH Decisions” with “Notification of Behavioral Healthcare Decisions” in the element title.
- Added “commercial and Exchange” to factor 4.
- Moved the explanation text about Medicare and Medicaid time frames for nonurgent preservice requests to the stem as a new factor 5 and renumbered the references throughout the element to align with the addition.
- Replaced “Classification of UM requests” with “Definitions used when classifying UM requests” in the Explanation.
- Revised the last paragraph under “Definitions used for classifying UM requests” to clarify requests that may be classified as preservice or postservice.
- Removed “Factor 4: Nonurgent preservice” subhead and text from the Explanation.
- Revised the Medicaid extension condition under “Factors 2, 3: Urgent concurrent and urgent preservice requests” to align with regulations.
- Replaced “Factors 4, 5: Nonurgent preservice and postservice requests” with “Factor 4: Nonurgent preservice requests for commercial and Exchange product lines.”
- Added “Factor 5: Nonurgent preservice for Medicare and Medicaid product lines” subhead and text.
- Added “Factor 6: Postservice requests for commercial, Exchange and Medicaid product lines” subhead and text.
- Added a sentence under “Extension for other reasons” that for Medicare, extensions are not allowed for postservice requests.
- Added a last paragraph under “Failure to follow filing procedures” to clarify that the organization must provide a reason for denying a postservice request for failure to follow filing procedures.

Scoring	Met	Partially Met	Not Met
	High (90-100%) on file review	Medium (60-89%) on file review	Low (0-59%) on file review

Data source Records or files

Scope of review **Product lines**

This element applies to all product lines for First Surveys and Renewal Surveys.

NCQA reviews and scores this element for each product line brought forward for Accreditation; only the commercial and Exchange product line files may be combined.

Documentation

For First Surveys and Renewal Surveys: NCQA reviews a random sample of up to 40 behavioral healthcare denial files resulting from medical necessity review for evidence of timeliness of notification.

Look-back period *For First Surveys:* 6 months.

For Renewal Surveys: 12 months.

Explanation **THIS IS A MUST-PASS ELEMENT.**

This element applies to all behavioral healthcare denial determinations resulting from medical necessity review (as defined in UM 1, Element A).

Dispute of file review results

NCQA conducts onsite file review in the presence of the organization's staff and works with the organization to resolve any disputes during the onsite survey. An organization that is unable to resolve a dispute with the survey team must contact NCQA before the onsite survey is complete. File review results may not be disputed or appealed once the onsite survey is complete.

Definitions used when classifying UM requests

The organization uses the definitions stated in Element A.

Reclassification of behavioral requests that do not meet the definition of "urgent."

All types of requests received while the member is receiving care may be reclassified as preservice or postservice if the request does not meet the definition of "urgent." This includes a request to extend a course of treatment beyond the time period or number of treatments previously approved by the organization. The request may be handled as a new request and decided within the time frame appropriate for the type of decision notification (i.e., preservice or postservice).

Factors 1–6: Timeliness of notification

NCQA considers 24 hours to be equivalent to 1 calendar day and 72 hours to be equivalent to 3 calendar days.

NCQA measures timeliness of notification from the date when the organization receives the request from the member or the member's authorized representative, even if the organization does not have all the information necessary to make a decision, to the date when it notifies the member and practitioner, as applicable.

For Medicare only: NCQA measures timeliness of notification for urgent requests from the date when the appropriate department receives the request.

The organization documents the date when it receives the request, and the date of the decision notification, in the UM file. The request is received when it arrives at the organization, even if it is not received by the UM department.

If the organization sends written notice, NCQA uses the date on the notice as the notification date. If the organization does not retain copies of the written notice, it has other methods of documenting the notification date. If the organization uses electronic notification, NCQA uses the date when the notification was posted in the electronic system.

An organization may have procedures for ongoing review of urgent concurrent care it approved initially. For ongoing reviews, the notification period begins on the day of the review. The organization documents the date of the ongoing review and the decision notification in the UM denial file.

The organization may extend the decision time frame under certain circumstances. Refer to *Related information*.

Exceptions

This element is NA if all purchasers of the organization's services carve out or exclude behavioral healthcare.

Exceptions to member notification. NCQA does not require the organization to notify a member of:

- An urgent concurrent denial.
- An urgent preservice denial.
- A postservice (retrospective) denial if the member is not at financial risk.

For urgent denials, NCQA considers the attending or treating practitioner to be acting as the member's representative. During the file review process, NCQA assesses whether the decision notification time frames to the practitioner were appropriate.

Factors 1, 4 are NA for the Medicare and Medicaid product lines.

Factors 2, 5 are NA for the commercial and Exchange product lines.

Related information

Notifying the practitioner. If information on the attending or treating practitioner was not provided with the request, the organization attempts to identify the practitioner. The organization documents its efforts to identify the practitioner.

For urgent concurrent decisions, the organization may inform the hospital Utilization Review (UR) department staff without attempting to identify the treating practitioner, with the understanding that staff will inform the attending/treating practitioner.

In all cases, if the practitioner is not known, the organization must address the notification to the attention of the attending or treating practitioner; the practitioner name is not required.

Receiving requests after normal business hours. Due to the nature of urgent requests, the organization has procedures for accepting them after normal business hours. NCQA counts the time from the date when the organization receives the request, whether or not it is during business hours.

Postservice payment disputes. Postservice requests for payment initiated by a practitioner or a facility are not subject to review if the practitioner or facility has no recourse against the member for payment (i.e., the member is not at financial risk). Exclude denials of such requests from the file review universe.

Approving alternative services. If the organization approves an alternative to the service being requested and the member or the member's authorized representative does not request or agree to the alternative service, the organization would be denying care that was originally requested; therefore, this is considered a denial and should be included in the file review universe. However, if the member or the member's authorized representative agrees to the alternative and the care is authorized, the practitioner has essentially withdrawn the initial request; therefore, this is not considered a denial and should not be included in the file review universe.

Extending time frames. Members or their authorized representatives may agree to extend the decision-making time frame for urgent, preservice and postservice requests.

Extension conditions

Factor 1: Urgent concurrent requests for commercial and Exchange product lines.

- The organization may extend the decision notification time frame if the request to extend urgent concurrent care was made less than 24 hours prior to the expiration of the previously approved time period or number of treatments. The organization may treat the request as urgent preservice and send a decision notification within 72 hours.
- The organization may extend the decision notification time frame if the request to approve additional days for urgent concurrent care is related to care not previously approved by the organization and the organization documents that it made at least one attempt and was unable to obtain the needed clinical information within the initial 24 hours after the request for coverage of additional days. In this case, the organization has up to 72 hours to make the decision.

Factors 2, 3: Urgent concurrent and urgent preservice requests for Medicare and Medicaid product lines.

For Medicare, the organization may extend the time frame once, by up to 14 calendar days, under the following conditions:

- The member requests an extension, **or**
- The organization needs additional information, **and**
 - Documents that it made at least one attempt to obtain the necessary information.
 - Notifies the member or the member's authorized representative of the delay.

The organization notifies the member or the member's authorized representative of its decision as expeditiously as the member's health condition requires, but no later than the expiration of the extension.

For Medicaid, the organization may extend the time frame once, by up to 14 calendar days, under the following conditions:

- The member requests an extension, **or**
- The organization needs additional information, provided it documents that it made at least one attempt to obtain the necessary information.

The organization notifies the member or the member's authorized representative of its decision as expeditiously as the member's health condition requires, but no later than the expiration of the extension.

Factor 3: Urgent preservice requests for commercial and Exchange product lines.

The organization may extend the urgent preservice time frame once due to lack of information, for 48 hours, under the following conditions:

- Within 24 hours of receipt of the urgent preservice request, the organization asks the member or the member's representative for the information necessary to make the decision, **and**
- The organization gives the member or the member's authorized representative at least 48 hours to provide the information, **and**
- The extension period, within which a decision must be made by the organization, begins on the sooner of:
 - The date when the organization receives the member's response (even if not all of the information is provided), **or**
 - The last date of the time period given to the member to provide the information, even if no response is received from the member or the member's authorized representative.

Factor 4: Nonurgent preservice requests for commercial and Exchange product lines. If the request lacks clinical information, the organization may extend the nonurgent preservice time frame for up to 15 calendar days, under the following conditions:

- Before the end of the time frame, the organization asks the member or the member's representative for the information necessary to make the decision, **and**
- The organization gives the member or the member's authorized representative at least 45 calendar days to provide the information.
- The extension period, within which a decision must be made by the organization, begins on the sooner of:
 - The date when the organization receives the member's response (even if not all of the information is provided), **or**
 - The last date of the time period given to the member to supply the information, even if no response is received from the member or the member's authorized representative.

The organization may deny the request if it does not receive the information within the time frame, and the member may appeal the denial.

Factor 5: Nonurgent preservice requests for Medicare and Medicaid product lines.

For Medicare, the organization may extend the time frame once, by up to 14 calendar days, under the following conditions:

- The member requests an extension, **or**
- The organization needs additional information, **and**

- Documents that it made at least one attempt to obtain the necessary information.
- Notifies the member or the member's authorized representative of the delay.

The organization notifies the member or the member's authorized representative of its decision as expeditiously as the member's health condition requires, but no later than the expiration of the extension.

For Medicaid, the organization may extend the time frame once, by up to 14 calendar days, under the following conditions:

- The member requests an extension, **or**
- The organization needs additional information, provided it documents that it made at least one attempt to obtain the necessary information.

The organization notifies the member or the member's authorized representative of its decision as expeditiously as the member's health condition requires, but no later than the expiration of the extension.

Factor 6: Postservice requests for commercial, Exchange and Medicaid product lines. If the request lacks clinical information, the organization may extend the postservice time frame for up to 15 calendar days, under the following conditions:

- Before the end of the time frame, the organization asks the member or the member's representative for the information necessary to make the decision, **and**
- The organization gives the member or the member's authorized representative at least 45 calendar days to provide the information.
- The extension period, within which a decision must be made by the organization, begins on the sooner of:
 - The date when the organization receives the member's response (even if not all of the information is provided), **or**
 - The last date of the time period given to the member to supply the information, even if no response is received from the member or the member's authorized representative.

The organization may deny the request if it does not receive the information within the time frame, and the member may appeal the denial.

Extension for other reasons. In a situation beyond the organization's control (e.g., waiting for an evaluation by a specialist), it may extend the nonurgent preservice and postservice time frames once, for up to 15 calendar days.

- Within 15 calendar days of a preservice request, the organization notifies the member (or the member's authorized representative) of the need for an extension and the expected date of the decision.
- Within 30 calendar days of a postservice request, the organization notifies the member (or the member's authorized representative) of the need for an extension and the expected date of the decision.

For Medicare, extensions are not allowed for postservice requests.

Factors 1–3: Verbal notification of denials. Verbal notification does not replace electronic or written notification of denial decisions, but when provided, the organization may extend the time frame for electronic or written notification for commercial, Medicare and Exchange decisions as described below.

- Verbal notification requires communication with a live person; the organization may not leave a voicemail, **and**
- The organization records the time and date of the notification and the staff member who spoke with the practitioner or member, **and**
- The organization provides verbal notification within the time frames specified for an urgent concurrent or urgent preservice request.

For commercial, Medicare and Exchange decisions, if the organization provides verbal notification of a denial decision as specified for an urgent concurrent or urgent preservice request, it has an additional 3 calendar days following verbal notification to provide electronic or written notification.

For Medicaid decisions, providing verbal notification does not extend the electronic or written notification time frame.

Failure to follow filing procedures. If the member (or the member's authorized representative) does not follow the organization's reasonable filing procedures for requesting preservice or urgent concurrent coverage, the organization notifies the member (or the member's authorized representative) of the failure and informs them of the proper procedures to follow when requesting coverage.

- *For urgent preservice and concurrent decisions*, the organization notifies the practitioner (member's authorized representative) or member within 24 hours of receiving the request. Notification may be verbal, unless the practitioner or member requests written notification.
- *For nonurgent preservice decisions*, the organization notifies the member or the member's authorized representative within 5 calendar days of receiving the request.

The organization may not deny a nonurgent preservice, urgent preservice or urgent concurrent request that requires medical necessity review for failure to follow filing procedures.

The organization may deny a postservice request if the member (or the member's authorized representative) does not follow the organization's reasonable filing procedures, but must provide the reason for the denial.

Use of practitioner web portals. The organization may provide electronic denial notifications to practitioners through a web portal if:

- The organization informs practitioners of the notification mechanism and their responsibility to check the portal regularly, **and**
- The organization documents the date and time when the information was posted in the portal, **and**
- The information posted in the portal meets the requirements in UM 4–UM 7, **and**
- The organization must have an alternative method for practitioners who do not have access to the web portal.

Use of member web portals. The organization may provide electronic denial notifications to members through a web portal if all the following criteria are met:

- The organization documents the member's agreement to receive electronic notifications via the portal.
- The organization documents the date and time when the information was posted in the portal.
- Members receive notification that a new document or update is available in the portal when it is posted (e.g., text, email, other electronic notification).
- The information posted in the portal meets the requirements in UM 4–UM 7.
- The organization has an alternative notification method for members who do not have access to the web portal or who do not agree to receive notifications via the portal.

Examples Failure to follow filing procedures

- An organization's procedure is that members or practitioners submit UM requests in writing but the member or practitioner files a request over the phone.
- An organization's procedure is that members or practitioners submit requests within a specific time frame but the member or practitioner submits the request outside the time frame.

Element C: Notification of Pharmacy Decisions

The organization adheres to the following time frames for notifying members and practitioners of pharmacy UM decisions:

1. For commercial and Exchange urgent concurrent decisions, the organization gives electronic or written notification of the decision to members and practitioners within 24 hours of the request.
2. For Medicare Part B and Medicaid urgent concurrent decisions, the organization gives electronic or written notification of the decision to members and practitioners within 24 hours of the request.
3. For commercial and Exchange urgent preservice decisions, the organization gives electronic or written notification of the decision to members and practitioners within 72 hours of the request.
4. For Medicare Part B and Medicaid urgent preservice decisions, the organization gives electronic or written notification of the decision to members and practitioners within 24 hours of the request.
5. For commercial and Exchange nonurgent preservice decisions, the organization gives electronic or written notification of the decision to members and practitioners within 15 calendar days of the request.
6. For Medicare Part B nonurgent preservice decisions, the organization gives electronic or written notification of the decision to members and practitioners within 72 hours of the request.
7. For Medicaid nonurgent preservice decisions, the organization gives electronic or written notification of the decision to members and practitioners within 24 hours of the request.
8. For postservice decisions, the organization gives electronic or written notification of the decision to members and practitioners within 30 calendar days of the request.

9. For Medicare Part D urgent preservice decisions, the organization gives electronic or written notification of the decision to members and practitioners within 24 hours of receipt of the request.
10. For Medicare Part D nonurgent preservice decisions, the organization gives electronic or written notification of the decision to members and practitioners within 72 hours of receipt of the request.
11. For Medicare Part D postservice decisions, the organization gives electronic or written notification of the decision to members and practitioners within 14 calendar days of receipt of the request.

Summary of Changes

Clarifications

- Added “commercial and Exchange” to factors 1, 3, and 5.
- Added factors 2, 4, 6 and 7 and renumbered the factors and the references throughout the element to align with the additions.
- Replaced “1–4” with “1–8” under “Product lines” in the scope of review.
- Replaced “Classification of UM pharmaceutical requests” with “Definitions used for classifying UM requests” in the Explanation.
- Revised the Exceptions to align with the factors.
- Replaced “Factors 1, 2: Urgent concurrent and urgent preservice requests for Medicare and Medicaid product lines” with “Factors 2, 4: Urgent concurrent and urgent preservice requests for Medicaid product line” under “Related information” and replaced original text.
- Replaced “Factors 3, 4: Nonurgent preservice and postservice requests” with “Factor 5: Nonurgent preservice for commercial and Exchange product lines” under “Related information.”
- Added “Factor 7: Nonurgent preservice for Medicaid product line” and text under “Related information.”
- Added “Factor 8: Postservice for commercial, Exchange and Medicaid product lines” and text under “Related information.”
- Replaced “Extending time frames for factors 5–7—Alignment with CMS” with “Extending time frames for Medicare Part B and D for factors 2, 4, 6, 9–11—Alignment with CMS” under “Related information” and revised text.
- Added a sentence under “Extension for other reasons” that for Medicare, extensions are not allowed for postservice requests.
- Added a last paragraph under “Failure to follow filing procedures” to clarify that the organization must provide a reason for denying a postservice request for failure to follow filing procedures.

Scoring

Met	Partially Met	Not Met
High (90-100%) on file review	Medium (60-89%) on file review	Low (0-59%) on file review

Data source Records or files

Scope of review

Product lines

This element applies to all product lines for First Surveys and Renewal Surveys.

NCQA reviews and scores this element for each product line brought forward for Accreditation; only the commercial and Exchange product line files may be combined.

Factors 1–8 apply to commercial, Medicaid and Exchange product lines and Medicare Part B drugs.

Documentation

For First Surveys and Renewal Surveys: NCQA reviews a random sample of up to 40 pharmaceutical denial files resulting from medical necessity review for evidence of timeliness of notification.

Look-back period

For First Surveys: 6 months.

For Renewal Surveys: 12 months.

Explanation

THIS IS A MUST-PASS ELEMENT.

This element applies to all pharmaceutical denial determinations resulting from medical necessity review (as defined in UM 1, Element A), whether the pharmaceutical is covered under an organization's medical benefit or its pharmacy benefit.

Dispute of file review results

NCQA conducts onsite file review in the presence of the organization's staff and works with the organization to resolve any disputes during the onsite survey. An organization that is unable to resolve a dispute with the survey team must contact NCQA before the onsite survey is complete. File review results may not be disputed or appealed once the onsite survey is complete.

Definitions used when classifying UM requests

The organization uses the definitions stated in Element A.

Reclassification of pharmaceutical requests that do not meet the definition of "urgent." All types of requests received while the member is receiving care may be reclassified as preservice or postservice if the request does not meet the definition of "urgent." This includes a request to extend a course of treatment beyond the time period or number of treatments previously approved by the organization. The request may be handled as a new request and decided within the time frame appropriate for the type of decision notification (i.e., preservice or postservice).

Factors 1–11: Timeliness of pharmacy notification

NCQA considers 24 hours to be equivalent to 1 calendar day and 72 hours to be equivalent to 3 calendar days.

NCQA measures timeliness of notification from the date when the organization receives the request from the member or the member's authorized representative, even if the organization does not have all the information necessary to make a decision, to the date when it notifies the member and practitioner, as applicable.

For Medicare Part B drugs only: NCQA measures timeliness of notification for urgent requests from the date when the appropriate department receives the request.

The organization documents the date when it receives the request, and the date of the decision notification, in the UM file. The request is received when it arrives at the organization, even if it is not received by the UM department.

If the organization sends written notice, NCQA uses the date on the notice as the notification date. If the organization does not retain copies of the written notice, it has other methods of documenting the notification date. If the organization uses

electronic notification, NCQA uses the date when the notification was posted in the electronic system.

An organization may have procedures for ongoing review of urgent concurrent care it approved initially. For ongoing reviews, the notification period begins on the day of the review. The organization documents the date of the ongoing review and the decision notification in the UM denial file.

The organization may extend the decision time frame under certain circumstances. Refer to *Related information*.

Exceptions

This element is NA:

- If all purchasers of the organization's services carve out or exclude pharmaceutical management.
- For pharmaceutical approvals made at the pharmacy.

Exceptions to member notification. NCQA does not require the organization to notify a member of:

- An urgent concurrent denial.
- An urgent preservice denial.
- A postservice (retrospective) denial if the member is not at financial risk.

For urgent denials, NCQA considers the attending or treating practitioner to be acting as the member's representative. During the file review process, NCQA assesses whether the decision notification time frames to the practitioner were appropriate.

Factors 1–8 are NA for Medicare Part D drugs.

Factors 1, 3 and 5 are NA for Medicare and Medicaid product lines.

Factors 2 and 4 are NA for commercial and Exchange product lines.

Factors 6 is NA for commercial, Medicaid and Exchange product lines.

Factor 7 is NA for commercial, Medicare and Exchange product lines.

Factors 9–11 are NA for commercial, Medicaid and Exchange product lines and Medicare Part B drugs.

Related information

Notifying the practitioner. If information on the attending or treating practitioner was not provided with the request, the organization attempts to identify the practitioner. The organization documents its efforts to identify the practitioner.

For urgent concurrent decisions, the organization may inform the hospital Utilization Review (UR) department staff without attempting to identify the treating practitioner, with the understanding that staff will inform the attending/treating practitioner.

In all cases, if the practitioner is not known, the organization must address the notification to the attention of the attending or treating practitioner; the practitioner name is not required.

Medicare Part B drugs (factors 2, 4, 6). For guidance on drugs covered under Medicare Part B, refer to the *Medicare Managed Care Manual, Chapter 4 - Benefits and Beneficiary Protections, 10.8 – Drugs that are Covered Under Original Medicare Part B*.

Medicare Part D drugs (factors 9–11). For guidance on drugs covered under Medicare Part D, refer to the *Medicare Prescription Drug Benefit Manual Chapter 6 – Part D Drugs and Formulary Requirements, 10.2 - Covered Part D Drugs*.

Alignment with CMS time frames.

- Factor 9: Medicare Part D urgent preservice decisions. The time frame corresponds to the CMS “expedited coverage” determination time frame.
- Factor 10: Medicare Part D nonurgent preservice decisions. The time frame corresponds to the CMS “standard coverage” determination time frame.
- Factor 11: Medicare Part D postservice decisions. The time frame corresponds to the CMS “request for reimbursement” time frame.

Receiving requests after normal business hours. Due to the nature of urgent requests, the organization has procedures for accepting them after normal business hours. NCQA counts the time from the date when the organization receives the request, whether or not it is during business hours.

Medications dispensed at the pharmacy. Requests for coverage of medications dispensed at a pharmacy may only be classified as urgent concurrent, urgent preservice or nonurgent preservice, depending on whether the request meets NCQA’s definition of “urgent.” Medications dispensed at the pharmacy may not be classified as postservice requests.

Postservice payment disputes. Postservice requests for payment initiated by a practitioner or a facility are not subject to review if the practitioner or facility has no recourse against the member for payment (i.e., the member is not at financial risk). Exclude denials of such requests from the file review universe.

Approving alternative pharmaceuticals. If the organization approves an alternative to the service being requested and the member or the member’s authorized representative does not request or agree to the alternative service, the organization would be denying care that was originally requested; therefore, this is considered a denial and should be included in the file review universe. However, if the member or the member’s authorized representative agrees to the alternative and the care is authorized, the practitioner has essentially withdrawn the initial request; therefore, this is not considered a denial and should not be included in the file review universe.

Extending time frames for factors 1–8. Members or their authorized representatives may agree to extend the decision-making time frame for urgent, preservice and postservice requests.

Extension conditions

Factor 1: Urgent concurrent requests for commercial and Exchange product lines.

- The organization may extend the decision notification time frame if the request to extend urgent concurrent care was made less than 24 hours prior to the expiration of the previously approved time period or number of treatments. The organization may treat the request as urgent preservice and send a decision notification within 72 hours.

- The organization may extend the decision notification time frame if the request to approve additional days for urgent concurrent care is related to care not previously approved by the organization and the organization documents that it made at least one attempt and was unable to obtain the needed clinical information within the initial 24 hours after the request for coverage of additional days. In this case, the organization has up to 72 hours to make the decision.

Factors 2, 4: Urgent concurrent and urgent preservice requests for Medicaid product line. The organization may extend the time frame once, by up to 14 calendar days, under the following conditions:

- The member requests an extension, **or**
- The organization needs additional information, provided it documents that it made at least one attempt to obtain the necessary information.

The organization notifies the member or the member's authorized representative of its decision as expeditiously as the member's health condition requires, but no later than the expiration of the extension.

Factor 3: Urgent preservice requests for commercial and Exchange product lines. The organization may extend the urgent preservice time frame once due to lack of information, for 48 hours, under the following conditions:

- Within 24 hours of receipt of the urgent preservice request, the organization asks the member or the member's representative for the information necessary to make the decision, **and**
- The organization gives the member or the member's authorized representative at least 48 hours to provide the information, **and**
- The extension period, within which a decision must be made by the organization, begins on the sooner of:
 - The date when the organization receives the member's response (even if not all of the information is provided), **or**
 - The last date of the time period given to the member to provide the information, even if no response is received from the member or the member's authorized representative.

Factor 5: Nonurgent preservice requests for commercial and Exchange product lines. If the request lacks clinical information, the organization may extend the nonurgent preservice time frame for up to 15 calendar days, under the following conditions:

- Before the end of the time frame, the organization asks the member or the member's representative for the information necessary to make the decision, **and**
- The organization gives the member or the member's authorized representative at least 45 calendar days to provide the information.
- The extension period, within which a decision must be made by the organization, begins on the sooner of:
 - The date when the organization receives the member's response (even if not all of the information is provided), **or**
 - The last date of the time period given to the member to supply the information, even if no response is received from the member or the member's authorized representative.

The organization may deny the request if it does not receive the information within the time frame, and the member may appeal the denial.

Factor 7: Nonurgent preservice requests for Medicaid product line. The organization may extend the time frame once, by up to 14 calendar days, under the following conditions:

- The member requests an extension, **or**
- The organization needs additional information, provided it documents that it made at least one attempt to obtain the necessary information.

The organization notifies the member or the member's authorized representative of its decision as expeditiously as the member's health condition requires, but no later than the expiration of the extension.

Factor 8: Postservice requests for commercial, Exchange and Medicaid product lines. If the request lacks clinical information, the organization may extend the postservice time frame for up to 15 calendar days, under the following conditions:

- Before the end of the time frame, the organization asks the member or the member's representative for the information necessary to make the decision, **and**
- The organization gives the member or the member's authorized representative at least 45 calendar days to provide the information.
- The extension period, within which a decision must be made by the organization, begins on the sooner of:
 - The date when the organization receives the member's response (even if not all of the information is provided), **or**
 - The last date of the time period given to the member to supply the information, even if no response is received from the member or the member's authorized representative.

The organization may deny the request if it does not receive the information within the time frame, and the member may appeal the denial.

Extension for other reasons. In a situation beyond the organization's control (e.g., waiting for an evaluation by a specialist), it may extend the nonurgent preservice and postservice time frames once, for up to 15 calendar days.

- Within 15 calendar days of a preservice request, the organization notifies the member (or the member's authorized representative) of the need for an extension and the expected date of the decision.
- Within 30 calendar days of a postservice request, the organization notifies the member (or the member's authorized representative) of the need for an extension and the expected date of the decision.

Extending time frames for Medicare Part B and D for factors 2, 4, 6, 9–11—Alignment with CMS: In accordance with the Medicare Prescription Drug Manual, Parts C & D Enrollee Grievances, Organization/Coverage Determinations, and Appeals Guidance, extensions are not allowed.

For Medicare, extensions are not allowed for postservice requests.

Factors 1–4: Verbal notification of denials. Verbal notification does not replace electronic or written notification of denial decisions, but when provided, the organization may extend the time frame for electronic or written notification for commercial, Medicare and Exchange decisions as described below.

- Verbal notification requires communication with a live person; the organization may not leave a voicemail, **and**
- The organization records the time and date of the notification and the staff member who spoke with the practitioner or member, **and**
- The organization provides verbal notification within the time frames specified for an urgent concurrent or urgent preservice request.

For commercial, Medicare and Exchange decisions, if the organization provides verbal notification of a denial decision as specified for an urgent concurrent or urgent preservice request, it has an additional 3 calendar days following verbal notification to provide electronic or written notification.

For Medicaid decisions, providing verbal notification does not extend the electronic or written notification time frame.

For Medicare Part D plans, initial verbal notification of a decision may be made within the specified time frames. Written notification must be made no later than 3 calendar days after verbal notification.

Failure to follow filing procedures. If the member (or the member's authorized representative) does not follow the organization's reasonable filing procedures for requesting preservice or urgent concurrent coverage, the organization notifies the member (or the member's authorized representative) of the failure and informs them of the proper procedures to follow when requesting coverage.

- *For urgent preservice and concurrent decisions*, the organization notifies the practitioner (member's authorized representative) or member within 24 hours of receiving the request. Notification may be verbal, unless the practitioner or member requests written notification.
- *For nonurgent preservice decisions*, the organization notifies the member or the member's authorized representative within 5 calendar days of receiving the request.

The organization may not deny a nonurgent preservice, urgent preservice or urgent concurrent request that requires medical necessity review for failure to follow filing procedures.

The organization may deny a postservice request if the member (or the member's authorized representative) does not follow the organization's reasonable filing procedures, but must provide the reason for the denial.

Use of practitioner web portals. The organization may provide electronic denial notifications to practitioners through a web portal if:

- The organization informs practitioners of the notification mechanism and their responsibility to check the portal regularly, **and**
- The organization documents the date and time when the information was posted in the portal, **and**
- The information posted in the portal meets the requirements in UM 4–UM 7, **and**
- The organization must have an alternative method for practitioners who do not have access to the web portal.

Use of member web portals. The organization may provide electronic denial notifications to members through a web portal if all the following criteria are met:

- The organization documents the member's agreement to receive electronic notifications via the portal.
- The organization documents the date and time when the information was posted in the portal.
- Members receive notification that a new document or update is available in the portal when it is posted (e.g., text, email, other electronic notification).
- The information posted in the portal meets the requirements in UM 4–UM 7.
- The organization has an alternative notification method for members who do not have access to the web portal or who do not agree to receive notifications via the portal.

Examples

Failure to follow filing procedures

- An organization's procedure is that members or practitioners submit UM requests in writing but the member or practitioner files a request over the phone.
- An organization's procedure is that members or practitioners submit requests within a specific time frame but the member or practitioner submits the request outside the time frame.

Element D: UM Timeliness Report

The organization monitors and submits a report for timeliness of:

1. Notification of nonbehavioral UM decisions.
2. Notification of behavioral UM decisions.
3. Notification of pharmacy UM decisions.

Summary of Changes

Clarifications

- Renumbered the factor references throughout the element to align with revised factors.
- Revised the scoring table to account for the revised factors.
- Replaced the first sentence in the Explanation to reflect that approvals are no longer in the element scope.
- Replaced "Factors 1–6" with "Factors 1–3" in the Explanation.
- Revised the text under "Excluded from the timeliness report" to clarify that timeliness of notifications sent for approvals is not required to be included in factors 1–3.
- Removed the first example of timeliness reports and renumbered the second example as factor 1 in the Examples.

Deletions

- Removed factors 1, 3 and 5 and text and renumbered the remaining factors.

Scoring

Met	Partially Met	Not Met
The organization meets 2-3 factors	No scoring option	The organization meets 0-1 factors

Data source	Reports
Scope of review	Product lines <i>This element applies to all product lines for First Surveys and Renewal Surveys.</i> Documentation <i>For First Surveys and Renewal Surveys:</i> NCQA reviews the organization's timeliness reports.
Look-back period	<i>For First Surveys:</i> 6 months. <i>For Renewal Surveys:</i> 12 months.
Explanation	This element applies to all UM denial determinations resulting from medical necessity review (as defined in UM 1, Element A).

Factors 1–3

The organization monitors the timeliness of notification for all requests and, using at least six months of data, calculates the percentage of decisions that adhere to time frames specified in Elements A–C. The six months of data can extend beyond the look back period, however the report must be completed within the look back period. At a minimum, the timeliness report calculates rates of adherence to time frames for each category of request (urgent concurrent, urgent preservice, nonurgent preservice, post-service) for each factor. The organization generates reports to reflect differences if its processes or staff vary by product/product line.

Excluded from the timeliness report

For all product lines, the organization excludes decisions and notifications for nonemergency transportation approvals.

Timeliness of notifications sent for approvals is not required to be included in factors 1–3.

Exceptions

Factor 2 is NA if all purchasers of the organization's services carve out or exclude behavioral healthcare.

Factor 3 is NA if all purchasers of the organization's services carve out or exclude pharmaceutical management.

Examples Timeliness reports

Factor 1: Timeliness of nonbehavioral UM decision making—commercial product line

	Urgent concurrent	Urgent preservice	Nonurgent preservice	PostsERVICE
Numerator ¹	350	560	875	689
Denominator ²	400	575	880	689
Rate	87.5%	97.4%	99.4%	100%

¹Numerator: The number of cases meeting the decision time frame.

²Denominator: The total number of requests.

Element E: Interim: Policies and Procedures

The organization has written policies and procedures in place for the following time frames for timeliness of nonbehavioral healthcare, behavioral healthcare and pharmacy UM decision making:

1. For commercial and Exchange urgent concurrent decisions, the organization gives electronic or written notification of the decision to members and practitioners within 24 hours of receipt of the request.
2. For Medicare and Medicaid urgent concurrent decisions, the organization gives electronic or written notification of the decision to members and practitioners within 72 hours of receipt of the request.
3. For Medicare Part B and Medicaid pharmaceutical urgent concurrent decisions, the organization gives electronic or written notification of the decision to members and practitioners within 24 hours of the request.
4. For urgent preservice decisions, the organization gives electronic or written notification of the decision to members and practitioners within 72 hours of receipt of the request.
5. For Medicare Part B and Medicaid pharmaceutical urgent preservice decisions, the organization gives electronic or written notification of the decision to members and practitioners within 24 hours of the request.
6. For commercial and Exchange nonurgent preservice decisions, the organization gives electronic or written notification of the decision to members and practitioners within 15 calendar days of receipt of the request.
7. For Medicare and Medicaid nonurgent preservice decisions, the organization gives electronic or written notification of the decision to members and practitioners within 14 calendar days of the request.
8. For Medicare Part B pharmaceutical nonurgent preservice decisions, the organization gives electronic or written notification of the decision to members and practitioners within 72 hours of the request.
9. For Medicaid pharmaceutical nonurgent preservice decisions, the organization gives electronic or written notification of the decision to members and practitioners within 24 hours of the request.
10. For postservice decisions, the organization gives electronic or written notification of the decision to members and practitioners within 30 calendar days of receipt of the request.
11. For Medicare Part D urgent preservice decisions, the organization gives electronic or written notification of the decision to members and practitioners within 24 hours of receipt of the request.
12. For Medicare Part D nonurgent preservice decisions, the organization gives electronic or written notification of the decision to members and practitioners within 72 hours of receipt of the request.
13. For Medicare Part D postservice decisions, the organization gives electronic or written notification of the decision to members and practitioners within 14 calendar days of receipt of the request.

Summary of Changes

Clarifications

- Added text in factors to clarify that the organization gives electronic or written notification of the decision to members and practitioners.
- Revised the factor language; added factors 3, 5, 7–9 and 11–13 and renumbered the factors throughout the element text to align.
- Added “commercial and Exchange” to factor 6.
- Revised the scoring table to account for the added factors.
- Replaced “Classification of UM requests” with “Definitions used when classifying UM requests” in the Explanation.
- Removed the last paragraph under “Factors 1–13: Timeliness of decision making” in the Explanation.
- Removed “Factor 4: Nonurgent preservice” subhead and text in the Explanation.
- Removed “Medicare Part D drugs” subhead and text in the Explanation.
- Revised the Exceptions to align with the factors.
- Replaced “Factors 4,5: Nonurgent preservice and postservice requests” with “Factor 6: Nonurgent preservice requests for commercial and Exchange product lines” under “Related information.”
- Added “Factors 7,9: Nonurgent preservice requests for Medicare and Medicaid product lines” and text under “Related information.”
- Added “Factor 10: Postservice requests for commercial, Exchange and Medicaid product lines” and text under “Related information.”
- Added “Extending time frames for Medicare Part B and D for factors 3, 5, 8, 11–13—Alignment with CMS” and text under “Related information” to clarify that extensions are not allowed for Medicare Part B and D.
- Added a sentence under “Extension for other reasons” that for Medicare, extensions are not allowed for postservice requests.
- Added a last paragraph under “Failure to follow filing procedures” to clarify that the organization must provide a reason for denying a postservice request for failure to follow filing procedures.

Scoring

Met	Partially Met	Not Met
The organization meets 10-13 factors	The organization meets 7-9 factors	The organization meets 0-6 factors

Data source Documented process

Scope of review **Product lines**

This element applies to all product lines for Interim Surveys.

Documentation

For Interim Surveys: NCQA reviews the organization’s policies and procedures for nonbehavioral healthcare, behavioral healthcare and pharmacy UM decision making.

Look-back period *For Interim Surveys:* Prior to the survey date.

Explanation This element applies to all UM determinations resulting from medical necessity review, whether they are approvals or denials (as defined in UM 1, Element A).

Definitions used when classifying UM requests

The organization uses the definitions stated in Element A.

Reclassification of requests that do not meet the definition of “urgent.” All types of requests received while the member is receiving care may be reclassified as preservice or postservice if the request does not meet the definition of “urgent.” This includes a request to extend a course of treatment beyond the time period or number of treatments previously approved by the organization. The request may be handled as a new request and decided within the time frame appropriate for the type of decision notification (i.e., preservice or postservice).

Factors 1–13: Timeliness of decision making

NCQA considers 24 hours to be equivalent to 1 calendar day and 72 hours to be equivalent to 3 calendar days.

The organization’s policies and procedures specify that it makes nonbehavioral healthcare, behavioral healthcare and pharmacy UM decisions within the time frames specified in factors 1–13.

Policies include information on the timeliness of notification decisions, which is measured from the date when the organization receives the request from the member or from the member’s authorized representative, even if the organization does not have all the information necessary to make the decision to the date when the notice was provided to the member and practitioner, as applicable.

For Medicare only: NCQA measures timeliness of notification for urgent requests from the date when the appropriate department receives the request.

The organization’s policies documents the date when it receives the request, and the date of the decision notification, in the UM file. The request is received when it arrives at the organization, even if it is not received by the UM department.

If the organization sends written notice, NCQA uses the date on the notice as the notification date. If the organization does not retain copies of the written notice, it has other methods of documenting the notification date. If the organization uses electronic notification, NCQA uses the date when the notification was posted in the electronic system.

The organization may have procedures for ongoing review of urgent concurrent care it approved initially. For ongoing reviews, the notification period begins on the day of the review. The organization documents the date of the ongoing review and the decision notification in the UM denial file.

Exceptions

Factors 1–10 are NA for Medicare Part D drugs.

Factors 1 and 6 are NA for Medicare and Medicaid product lines.

Factors 3 and 5 are NA for pharmaceuticals under the commercial and Exchange product lines.

Factors 2 and 7 are NA for commercial and Exchange product lines.

Factor 8 is NA for pharmaceuticals under the commercial, Medicaid and Exchange product lines.

Factor 9 is NA for pharmaceuticals under the commercial, Medicare and Exchange product lines.

Factors 11–13 are NA for commercial, Medicaid and Exchange product lines and for Medicare Part B drugs.

Related information

Notifying the practitioner. If information on the attending or treating practitioner was not provided with the request, the organization attempts to identify the practitioner. The organization documents its efforts to identify the practitioner.

For urgent concurrent decisions, the organization may inform the hospital Utilization Review (UR) department staff without attempting to identify the treating practitioner, with the understanding that staff will inform the attending/treating practitioner.

In all cases, if the practitioner is not known, the organization must address the notification to the attention of the attending or treating practitioner; the practitioner name is not required.

Receiving requests after normal business hours. Due to the nature of urgent requests, the organization has procedures for accepting them after normal business hours. NCQA counts the time from the date when the organization receives the request, whether or not it is during business hours.

Postservice payment disputes. Postservice requests for payment initiated by a practitioner or a facility are not subject to review if the practitioner or facility has no recourse against the member for payment (i.e., the member is not at financial risk). Exclude denials of such requests from the file review universe.

Approving alternative services. If the organization approves an alternative to the service being requested and the member or the member's authorized representative does not request or agree to the alternative service, the organization would be denying care that was originally requested; therefore, this is considered a denial and should be included in the file review universe. However, if the member or the member's authorized representative agrees to the alternative and the care is authorized, the practitioner has essentially withdrawn the initial request; therefore, this is not considered a denial and should not be included in the file review universe.

Extending time frames. Members or their authorized representatives may agree to extend the time frame for urgent, preservice and postservice requests.

Extension conditions

Factor 1: Urgent concurrent requests for commercial and Exchange product lines.

- The organization may extend the decision notification time frame if the request to extend urgent concurrent care was made less than 24 hours prior to the expiration of the previously approved time period or number of treatments. The organization may treat the request as urgent preservice and send a decision notification within 72 hours.
- The organization may extend the decision notification time frame if the request to approve additional days for urgent concurrent care is related to care not previously approved by the organization and the organization documents that it made at least one attempt and was unable to obtain the

needed clinical information within the initial 24 hours after the request for coverage of additional days. In this case, the organization has up to 72 hours to make the decision.

Factors 2–5: Urgent concurrent and urgent preservice requests for Medicare and Medicaid product lines.

For Medicare, the organization may extend the time frame once, by up to 14 calendar days, under the following conditions:

- The member requests an extension, **or**
- The organization needs additional information, **and**
 - Documents that it made at least one attempt to obtain the necessary information.
 - Notifies the member or the member's authorized representative of the delay.

The organization notifies the member or the member's authorized representative of its decision as expeditiously as the member's health condition requires, but no later than the expiration of the extension.

For Medicaid, the organization may extend the time frame once, by up to 14 calendar days, under the following conditions:

- The member requests an extension, **or**
- The organization needs additional information, provided it documents that it made at least one attempt to obtain the necessary information.

The organization notifies the member or the member's authorized representative of its decision as expeditiously as the member's health condition requires, but no later than the expiration of the extension.

Factor 4: Urgent preservice requests for commercial and Exchange product lines.

The organization may extend the urgent preservice time frame once due to lack of information, for 48 hours, under the following conditions:

- Within 24 hours of receipt of the urgent preservice request, the organization asks the member or the member's representative for the information necessary to make the decision, **and**
- The organization gives the member or the member's authorized representative at least 48 hours to provide the information, **and**
- The extension period, within which a decision must be made by the organization, begins on the sooner of:
 - The date when the organization receives the member's response (even if not all of the information is provided), **or**
 - The last date of the time period given to the member to provide the information, even if no response is received from the member or the member's authorized representative.

Factor 6: Nonurgent preservice requests for commercial and Exchange product lines.

If the request lacks clinical information, the organization may extend the nonurgent preservice time frame for up to 15 calendar days, under the following conditions:

- Before the end of the time frame, the organization asks the member or the member's representative for the information necessary to make the decision, **and**

- The organization gives the member or the member's authorized representative at least 45 calendar days to provide the information.
- The extension period, within which a decision must be made by the organization, begins on the sooner of:
 - The date when the organization receives the member's response (even if not all of the information is provided), **or**
 - The last date of the time period given to the member to supply the information, even if no response is received from the member or the member's authorized representative.

The organization may deny the request if it does not receive the information within the time frame, and the member may appeal the denial.

Factors 7, 9: Nonurgent preservice requests for Medicare and Medicaid product lines.

For Medicare, the organization may extend the time frame once, by up to 14 calendar days, under the following conditions:

- The member requests an extension, **or**
- The organization needs additional information, **and**
 - Documents that it made at least one attempt to obtain the necessary information.
 - Notifies the member or the member's authorized representative of the delay.

The organization notifies the member or the member's authorized representative of its decision as expeditiously as the member's health condition requires, but no later than the expiration of the extension.

For Medicaid, the organization may extend the time frame once, by up to 14 calendar days, under the following conditions:

- The member requests an extension, **or**
- The organization needs additional information, provided it documents that it made at least one attempt to obtain the necessary information.

The organization notifies the member or the member's authorized representative of its decision as expeditiously as the member's health condition requires, but no later than the expiration of the extension.

Factor 10: Postservice requests for commercial, Exchange and Medicaid product lines. If the request lacks clinical information, the organization may extend the postservice time frame for up to 15 calendar days, under the following conditions:

- Before the end of the time frame, the organization asks the member or the member's representative for the information necessary to make the decision, **and**
- The organization gives the member or the member's authorized representative at least 45 calendar days to provide the information.
- The extension period, within which a decision must be made by the organization, begins on the sooner of:
 - The date when the organization receives the member's response (even if not all of the information is provided), **or**
 - The last date of the time period given to the member to supply the information, even if no response is received from the member or the member's authorized representative.

The organization may deny the request if it does not receive the information within the time frame, and the member may appeal the denial.

Extension for other reasons. In a situation beyond the organization's control (e.g., waiting for an evaluation by a specialist), it may extend the nonurgent preservice and postservice time frames once, for up to 15 calendar days.

- Within 15 calendar days of a preservice request, the organization notifies the member (or the member's authorized representative) of the need for an extension and the expected date of the decision.
- Within 30 calendar days of a postservice request, the organization notifies the member (or the member's authorized representative) of the need for an extension and the expected date of the decision.

Extending time frames for Medicare Part B and D for factors 3, 5, 8, 11–13—Alignment with CMS: In accordance with the Medicare Prescription Drug Manual, Parts C & D Enrollee Grievances, Organization/Coverage Determinations, and Appeals Guidance, extensions are not allowed.

For Medicare, extensions are not allowed for postservice requests.

Factors 1–5: Verbal notification of denials. Verbal notification does not replace electronic or written notification of denial decisions, but when provided, the organization may extend the time frame for electronic or written notification for commercial, Medicare and Exchange decisions as described below.

- Verbal notification requires communication with a live person; the organization may not leave a voicemail, **and**
- The organization records the time and date of the notification and the staff member who spoke with the practitioner or member, **and**
- The organization provides verbal notification within the time frames specified for an urgent concurrent or urgent preservice request.

For commercial, Medicare and Exchange decisions, if the organization provides verbal notification of a denial decision as specified for an urgent concurrent or urgent preservice request, it has an additional 3 calendar days following verbal notification to provide electronic or written notification.

For Medicaid decisions, providing verbal notification does not extend the electronic or written notification time frame.

Failure to follow filing procedures. If the member (or the member's authorized representative) does not follow the organization's reasonable filing procedures for requesting preservice or urgent concurrent coverage, the organization notifies the member (or the member's authorized representative) of the failure and informs them of the proper procedures to follow when requesting coverage.

- *For urgent preservice and concurrent decisions*, the organization notifies the practitioner (member's authorized representative) or member within 24 hours of receiving the request. Notification may be verbal, unless the practitioner or member requests written notification.
- *For nonurgent preservice decisions*, the organization notifies the member or the member's authorized representative within 5 calendar days of receiving the request.

The organization may not deny a nonurgent preservice, urgent preservice or urgent concurrent request that requires medical necessity review for failure to follow filing procedures.

The organization may deny a postservice request if the member (or the member's authorized representative) does not follow the organization's reasonable filing procedures, but must provide the reason for the denial.

Use of practitioner web portals. The organization may provide electronic denial notifications to practitioners through a web portal if:

- The organization informs practitioners of the notification mechanism and their responsibility to check the portal regularly, **and**
- The organization documents the date and time when the information was posted in the portal, **and**
- The information posted in the portal meets the requirement in UM 4–UM 7, **and**
- The organization has an alternative notification method for practitioners who do not have access to the web portal.

Use of member web portals. The organization may provide electronic denial notifications to members through a web portal if all the following criteria are met:

- The organization documents the member's agreement to receive electronic notifications via the portal.
- The organization documents the date and time when the information was posted in the portal.
- Members receive notification that a new document or update is available in the portal when it is posted (e.g., text, email, other electronic notification).
- The information posted in the portal meets the requirements in UM 4–UM 7.
- The organization has an alternative notification method for members who do not have access to the web portal or who do not agree to receive notifications via the portal.

Examples

Failure to follow filing procedures

- An organization's procedure is that members or practitioners submit UM requests in writing but the member or practitioner files a request over the phone.
- An organization's procedure is that members or practitioners submit requests within a specific time frame but the member or practitioner submits the request outside the time frame.

UM 6: Clinical Information

When determining coverage based on medical necessity, the organization obtains relevant clinical information and consults with the treating practitioner.

Intent

The organization uses all information relevant to a member's care when it makes coverage decisions.

Element A: Relevant Information for Nonbehavioral Healthcare Decisions

There is documentation that the organization gathers relevant clinical information consistently to support nonbehavioral healthcare UM decision making.

Summary of Changes

- No changes to this element.

Scoring

Met	Partially Met	Not Met
High (90-100%) on file review	Medium (60-89%) on file review	Low (0-59%) on file review

Data source

Records or files

Scope of review

Product lines

This element applies to all product lines for First Surveys and Renewal Surveys.

NCQA reviews and scores this element for each product line brought forward for Accreditation; only the commercial and Exchange product line files may be combined.

Documentation

For First Surveys and Renewal Surveys: NCQA reviews a random sample of up to 40 nonbehavioral healthcare denial files resulting from medical necessity review (as defined in UM 1, Element A) for evidence of using relevant clinical information to support UM decision making.

Look-back period

For First Surveys: 6 months.

For Renewal Surveys: 12 months.

Explanation

Although NCQA only reviews denial files during the file review process, this element applies to all nonbehavioral healthcare determinations resulting from medical necessity review, whether they are approvals or denials.

Dispute of file review results

NCQA conducts onsite file review in the presence of the organization's staff and works with the organization to resolve any disputes during the onsite survey. An organization that is unable to resolve a dispute with the survey team must contact NCQA before the onsite survey is complete. File review results may not be disputed or appealed once the onsite survey is complete.

Relevant clinical information

Denial files contain clinical information appropriate to each case.

The relevance of clinical information is considered in terms of the criteria used by the organization to make its decision (i.e., the clinical information must be related to the criteria the organization said were not met in its denial notice). Organizations must gather clinical information when determining medical necessity. If enough clinical information relevant to the criteria is not provided with the request, the organization must document in the denial file its attempts to gather the clinical information needed to make a decision.

Exceptions

None.

Related information

Refer to UM 1, Element A for the definition of “medical necessity review.”

Examples**Clinical information for determining coverage**

Clinical information may include, but is not limited to:

- Office and hospital records.
- A history of the presenting problem.
- Physical exam results.
- Diagnostic testing results.
- Treatment plans and progress notes.
- Patient psychosocial history.
- Information on consultations with the treating practitioner.
- Evaluations from other health care practitioners and providers.
- Operative and pathological reports.
- Rehabilitation evaluations.
- A printed copy of criteria related to the request.
- Information regarding benefits for services or procedures.
- Information regarding the local delivery system.
- Patient characteristics and information.
- Information from family members.
- Diagnosis codes.

Element B: Relevant Information for Behavioral Healthcare Decisions

There is documentation that the organization gathers relevant clinical information consistently to support behavioral healthcare UM decision making.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	High (90-100%) on file review	Medium (60-89%) on file review	Low (0-59%) on file review
Data source	Records or files		
Scope of review	Product lines <i>This element applies to all product lines for First Surveys and Renewal Surveys.</i> NCQA reviews and scores this element for each product line brought forward for Accreditation; only the commercial and Exchange product line files may be combined. Documentation <i>For First Surveys and Renewal Surveys:</i> NCQA reviews a random sample of up to 40 behavioral healthcare denial files resulting from medical necessity review (as defined in UM 1, Element A) for evidence of using relevant clinical information to support UM decision making.		
Look-back period	<i>For First Surveys:</i> 6 months. <i>For Renewal Surveys:</i> 12 months.		
Explanation	Although NCQA only reviews denial files during the file review process, this element applies to all behavioral healthcare determinations resulting from medical necessity review, whether they are approvals or denials. Dispute of file review results NCQA conducts onsite file review in the presence of the organization's staff and works with the organization to resolve any disputes during the onsite survey. An organization that is unable to resolve a dispute with the survey team must contact NCQA before the onsite survey is complete. File review results may not be disputed or appealed once the onsite survey is complete. Relevant clinical information Denial files contain clinical information appropriate to each case. The relevance of clinical information is considered in terms of the criteria used by the organization to make its decision (i.e., the clinical information must be related to the criteria the organization said were not met in its denial notice). Organizations must gather clinical information when determining medical necessity. If enough clinical information relevant to the criteria is not provided with the request, the organization must document in the denial file its attempts to gather the clinical information needed to make a decision.		

Exception

This element is NA if all purchasers of the organization's services carve out or exclude behavioral healthcare.

Related information

Refer to UM 1, Element A for the definition of "medical necessity review."

Examples**Clinical information for determining coverage**

Clinical information may include, but is not limited to:

- Office and hospital records.
- A history of the presenting problem.
- Physical exam results.
- Diagnostic testing results.
- Treatment plans and progress notes.
- Patient psychosocial history.
- Information on consultations with the treating practitioner.
- Evaluations from other health care practitioners and providers.
- Operative and pathological reports.
- Rehabilitation evaluations.
- A printed copy of criteria related to the request.
- Information regarding benefits for services or procedures.
- Information regarding the local delivery system.
- Patient characteristics and information.
- Information from family members.
- Diagnosis codes.

Element C: Relevant Information for Pharmacy Decisions

The organization documents that it consistently gathers relevant information to support pharmacy UM decision making.

Summary of Changes

- No changes to this element.

Scoring

Met	Partially Met	Not Met
High (90-100%) on file review	Medium (60-89%) on file review	Low (0-59%) on file review

Data source

Records or files

Scope of review**Product lines**

This element applies to all product lines for First Surveys and Renewal Surveys.

NCQA reviews and scores this element for each product line brought forward for Accreditation; only the commercial and Exchange product line files may be combined.

Documentation

For First Surveys and Renewal Surveys: NCQA reviews a random sample of up to 40 pharmaceutical denial files resulting from medical necessity review (as defined in UM 1, Element A) for evidence of using relevant clinical information to support UM decision making.

Look-back period

For First Surveys: 6 months.

For Renewal Surveys: 12 months.

Explanation

Although NCQA only reviews denial files during the file review process, this element applies to all pharmaceutical determinations resulting from medical necessity review, whether they are approvals or denials. This includes all pharmaceuticals covered under an organization's medical benefit or its pharmacy benefit.

Dispute of file review results

NCQA conducts onsite file review in the presence of the organization's staff and works with the organization to resolve any disputes during the onsite survey. An organization that is unable to resolve a dispute with the survey team must contact NCQA before the onsite survey is complete. File review results may not be disputed or appealed once the onsite survey is complete.

Relevant clinical information

Pharmaceutical denial files contain clinical information appropriate to each case.

The relevance of clinical information is considered in terms of the criteria used by the organization to make its decision (i.e., the clinical information must be related to the criteria the organization said were not met in its denial notice). Organizations must gather clinical information when determining medical necessity. If enough clinical information relevant to the criteria is not provided with the request, the organization must document in the denial file its attempts to gather the clinical information needed to make a decision.

Exception

This element is NA if all purchasers of the organization's services carve out or exclude pharmaceutical management.

Related information

Refer to UM 1, Element A for the definition of "medical necessity review."

Examples

None.

UM 7: Denial Notices

The organization documents and communicates the reasons for a denial.

Intent

Members and practitioners receive enough information to help them understand a decision to deny care or coverage and to decide whether to appeal the decision.

Element A: Discussing a Denial With a Nonbehavioral Healthcare Reviewer

The organization gives practitioners the opportunity to discuss nonbehavioral healthcare UM denial decisions with a physician or other appropriate reviewer.

Summary of Changes

Clarifications

- Replaced “Discussing a Denial With a Review” with “Discussing a Denial With a Nonbehavioral Healthcare Reviewer” in the element title.
- Clarified in the second and fourth bullets under “Opportunity to discuss denial decisions” that the name of the individual who notified the practitioner via telephone must be documented.
- Clarified in the fourth bullet under “Opportunity to discuss denial decisions” that if a voicemail is left, the denial file must include who left the message and the date and time.

Scoring

Met	Partially Met	Not Met
High (90-100%) on file review	Medium (60-89%) on file review	Low (0-59%) on file review

Data source Records or files

Scope of review **Product lines**

This element applies to all product lines for First Surveys and Renewal Surveys.

NCQA reviews and scores this element for each product line brought forward for Accreditation; only the commercial and Exchange product line files may be combined.

Documentation

For First Surveys and Renewal Surveys: NCQA reviews a random sample of up to 40 nonbehavioral healthcare denial files resulting from medical necessity review for evidence that a practitioner has the opportunity to discuss a denial with a reviewer.

Look-back period *For First Surveys:* 6 months.

For Renewal Surveys: 12 months.

Explanation This element applies to all nonbehavioral healthcare denial determinations resulting from medical necessity review (as defined in UM 1, Element A).

Dispute of file review results

NCQA conducts onsite file review in the presence of the organization’s staff and works with the organization to resolve any disputes during the onsite survey. An organization that is unable to resolve a dispute with the survey team must contact

NCQA before the onsite survey is complete. File review results may not be disputed or appealed once the onsite survey is complete.

Opportunity to discuss denial decisions

The organization notifies the treating practitioner about the opportunity to discuss a medical necessity denial:

- In the denial notification, **or**
- By telephone, including leaving a voicemail, if the organization documents the name of the individual at the organization who notified the treating practitioner or left the voicemail, and the date and time of the notification or voicemail, **or**
- In materials sent to the treating practitioner, informing the practitioner of the opportunity to discuss a specific denial with a reviewer.

The organization includes the following information in the denial file:

- The denial notification, if the treating practitioner was notified in the denial notification.
- The time and date of the notification and the name of the individual at the organization, if the treating practitioner was notified by telephone.
 - If the treating practitioner was notified by voicemail, the name of the individual who left the voicemail, and the date and time.
- Evidence that the treating practitioner was notified that a physician or other reviewer is available to discuss the denial, if notified in materials sent to the treating practitioner.

NCQA does not require evidence of discussion with an attending or treating practitioner, and does not consider the discussion to be an appeal.

For the Medicare product line, the organization may provide the treating practitioner with an opportunity to discuss a UM request with a physician or other appropriate reviewer prior to the decision to meet the intent of this element. The organization must provide documentation in the denial file.

Exceptions

None.

Examples

None.

Element B: Written Notification of Nonbehavioral Healthcare Denials

The organization's written notification of nonbehavioral healthcare denials, provided to members and their treating practitioners, contains the following information:

1. The specific reasons for the denial, in easily understandable language.
2. A reference to the benefit provision, guideline, protocol or other similar criterion on which the denial decision is based.
3. A statement that members can obtain a copy of the actual benefit provision, guideline, protocol or other similar criterion on which the denial decision was based, upon request.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	High (90-100%) on file review for at least 2 factors and medium (60-89%) on file review for any remaining factor	High (90-100%) or medium (60-89%) on file review for 3 factors	Low (0-59%) on file review for any factor
Data source	Records or files		
Scope of review	Product lines <i>This element applies to all product lines for First Surveys and Renewal Surveys.</i> NCQA reviews and scores this element for each product line brought forward for Accreditation; only the commercial and Exchange product line files may be combined. Documentation <i>For First Surveys and Renewal Surveys:</i> NCQA reviews a random sample of up to 40 nonbehavioral healthcare denial files resulting from medical necessity review for evidence that denial notices meet all three factors.		
Look-back period	<i>For First Surveys:</i> 6 months. <i>For Renewal Surveys:</i> 12 months.		
Explanation	THIS IS A MUST-PASS ELEMENT. This element applies to all nonbehavioral healthcare denial determinations resulting from medical necessity review (as defined in UM 1, Element A). Dispute of file review results NCQA conducts onsite file review in the presence of the organization's staff and works with the organization to resolve any disputes during the onsite survey. An organization that is unable to resolve a dispute with the survey team must contact NCQA before the onsite survey is complete. File review results may not be disputed or appealed once the onsite survey is complete.		

Factor 1: Reason for denial

The denial notification states the reason for the denial in terms specific to the member's condition or request and in language that is easy to understand, so the member and practitioner understand why the organization denied the request and have enough information to file an appeal.

An appropriately written notification includes a complete explanation of the grounds for the denial, in language that a layperson would understand, and does not include abbreviations, acronyms or health care procedure codes that a layperson would not understand.

The organization is not required to spell out abbreviations/acronyms if they are clearly explained in lay language.

To illustrate, for the acronym DNA, spelling out would be "a DNA (deoxyribonucleic acid)" whereas explaining would be "a DNA test looks at your genetic information."

Denial notifications sent only to practitioners may include technical or clinical terms.

The organization may send a single notice to the member and practitioner that includes the specific reason for the denial, in language that would be easily understood by the member. The notice may also include, in a separate section, additional clinical or technical language directed toward a practitioner.

For denials resulting from medical necessity review of out-of-network requests, the reason for the denial must explicitly address the reason for the request (e.g., if the request is related to accessibility issues, that may be impacted by the clinical urgency of the situation, the denial must address whether or not the requested service can be obtained within the organization's accessibility standards).

Factor 2: Reference to UM criterion

The denial notification references the specific criterion used to make the denial decision. The criterion used and referenced is specific to the member's condition or to the requested services.

The criterion referenced must be identifiable by name and must be specific to an organization or source (e.g., ABC PBM's Criteria for Treatment of Hypothyroidism with Synthroid or Criteria Company Inc.'s Guidelines for Wound Treatment). If it is clear that the criterion is attributable to the organization, it is acceptable to state "our Criteria for XXX" (e.g., our Criteria for Treating High Cholesterol with Lipitor).

If the organization uses a trademarked criterion name, it does not need to cite the organization that holds the trademark. If it is clear that the criterion is attributable to the organization, it is acceptable to state, "our Criteria for XXX" (e.g., our Criteria for Treating High Cholesterol with Lipitor).

Because benefit documents are often large and complex, the organization must direct members to the information using the section title or page number.

For denials resulting from medical necessity review of out-of-network requests, criteria may be excerpted from benefit documents that govern out-of-network coverage, organization policies specifying circumstances where out-of-network coverage will be approved or clinical criteria used to evaluate the member's clinical need relative to available network providers and services. The reference must specifically support the rationale for the decision and must relate to the reason for the request.

Factor 3: Availability of criterion

The denial notification informs the member, and the practitioner acting as the member's authorized representative, that the criterion used to make the decision is available upon request. Providing the criterion, or an excerpt specific to the denial reason with the denial notification is also acceptable. NCQA scores this factor "Yes" if the criterion or excerpt is included in the decision notice or if the notification states that the criterion or excerpt is included as an attachment.

Exception

Factor 3 is NA for Medicare denials.

Related information

Refer to *Related information* in UM 5, Element A for "Member Notification Exceptions" and "Notifying the Practitioner."

Denials due to insufficient clinical information. If the organization denies a request due to lack of clinical information, the denial notice must meet factors 1–3. If the organization does not have enough clinical information to reference a specific criterion, the denial notice must state this and specify the information needed.

Examples**Factor 1: Acceptable language documenting the reason for a denial**

After speaking with your physician, Dr. Jones, and reviewing your medical record, we have concluded that the requested rhinoplasty procedure (a surgery to change the shape of your nose) is not associated with a medical condition; it is for cosmetic purposes only. We cover this procedure when it is done to correct a deviated septum (where the wall between the nasal passage is crooked). The information provided by your doctor does not indicate that this is the case. As indicated in XYZ Plan's Summary of Covered Benefits, in "Exclusions and Limitations on Covered Benefits," rhinoplasty for cosmetic purpose is excluded; therefore, we cannot approve your request for this procedure.

Insufficient language documenting the reason for a denial

- The treatment is determined not to be medically necessary.
- The treatment is not a covered benefit.

Factors 2, 3: Acceptable language referencing decision-making criteria

After reviewing our UM Criteria for Wound Treatment, which considers age, progress of treatment and assessment of your home environment, we have determined your treatment of IV antibiotics and dressing changes can be provided at home.

You may obtain a copy of the criteria on which this decision was based by sending a request to us at the following address or contacting us by telephone.

Element C: Written Notification of Nonbehavioral Healthcare Appeal Rights/Process

The organization's written nonbehavioral healthcare denial notification to members and their treating practitioners contains the following information:

1. A description of appeal rights, including the right to submit written comments, documents or other information relevant to the appeal.
2. An explanation of the appeal process, including members' rights to representation and appeal time frames.
3. A description of the expedited appeal process for urgent preservice or urgent concurrent denials.
4. Notification that expedited external review can occur concurrently with the internal appeals process for urgent care.

Summary of Changes

Clarifications

- Replaced "Nonbehavioral Healthcare Notice of Appeal Rights/Process" with "Written Notification of Nonbehavioral Healthcare Appeal Rights/Process" in the element title.
- Revised the text for the "Met" scoring option to read: "High (90-100%) on file review for 3 factors and medium (60-89%) on file review for any remaining factors" to align with scoring options in Elements F and I.
- Revised the text for the "Partially Met" scoring option to read: "High (90-100%) or medium (60-89%) on file review for 4 factors to align with Elements F and I.

Scoring

Met	Partially Met	Not Met
High (90-100%) on file review for 3 factors and medium (60-89%) on file review for any remaining factor	High (90-100%) or medium (60-89%) on file review for 4 factors	Low (0-59%) on file review for any factor

Data source Records or files

Scope of review **Product lines**

This element applies to all product lines for First Surveys and Renewal Surveys.

NCQA reviews and scores this element for each product line brought forward for Accreditation; only the commercial and Exchange product line files may be combined.

Documentation

For First Surveys and Renewal Surveys: NCQA reviews a random sample of up to 40 nonbehavioral healthcare denial files resulting from medical necessity review for evidence that denial notices meet all four factors.

Look-back period *For First Surveys:* 6 months.

For Renewal Surveys: 12 months.

Explanation THIS IS A MUST-PASS ELEMENT.

This element applies to all nonbehavioral healthcare denial determinations resulting from medical necessity review (as defined in UM 1, Element A).

Dispute of file review results

NCQA conducts onsite file review in the presence of the organization's staff and works with the organization to resolve any disputes during the onsite survey. An organization that is unable to resolve a dispute with the survey team must contact NCQA before the onsite survey is complete. File review results may not be disputed or appealed once the onsite survey is complete.

Factor 1: Description of appeal rights

No additional explanation required.

Factor 2: Right to representation and appeal time frames

The denial notification:

- Includes a statement that members may be represented by anyone they choose, including an attorney.
- Provides contact information for a state office of health insurance consumer assistance or ombudsperson, if applicable.
Note: This is not required for members covered by the Federal Employee Health Benefits (FEHB) program.
- States the time frame for filing an appeal.
- States the organization's time frame for deciding the appeal.
- States the procedure for filing an appeal, including where to direct the appeal and information to include in the appeal.

The notification to the practitioner is not required to include the member's right to representation if the practitioner is not acting as an authorized representative.

Factor 3: Expedited appeal process

The denial notification describes the expedited appeals process for urgent preservice or urgent concurrent denials. If the same process applies to standard and expedited appeals, there must be a description included in the letter that makes it clear that the process applies to both. Factor 3 is met if the organization includes a description of the expedited appeals process in denial notices for every type of request.

The denial notification states:

- The time frame for filing an expedited appeal.
- The organization's time frame for deciding the expedited appeal.
- The procedure for filing an expedited appeal, including where to direct the appeal and information to include in the appeal.

Factor 4: Concurrent expedited external review

The denial notification states that for urgent care situations, expedited external review may occur at the same time as the internal appeal process.

The organization may discontinue the internal appeal for all member requests addressed by the external review if it is not required to continue the internal appeal process under state law. The organization continues the internal appeal process for components of the request that are not addressed in the external review.

Exceptions

Factor 4 is NA for:

- Members covered by Medicare, Medicaid or the FEHB Program and for members in self-funded accounts.
- Nonurgent preservice and postservice denial decisions.

Related information

Refer to *Related information* in UM 5, Element A for “Member Notification Exceptions” and “Notifying the Practitioner.”

Medicare denials and Fully Integrated Dual Eligible (FIDE) denials. CMS requires organizations to issue an Integrated Denial Notice (IDN) for non-inpatient medical service denials for Medicare and FIDE members. The IDN meets factors 1–3 for these members.

Examples None.

Element D: Discussing a Behavioral Healthcare Denial With a Reviewer

The organization provides practitioners with the opportunity to discuss any behavioral healthcare UM denial decision with a physician, appropriate behavioral healthcare reviewer or pharmacist reviewer.

Summary of Changes

Clarifications

- Clarified in the second and fourth bullets under “Opportunity to discuss denial decisions” that the name of the individual who notified the practitioner via telephone must be documented.
- Clarified in the fourth bullet under “Opportunity to discuss denial decisions” that if a voicemail is left, the denial file must include who left the message and the date and time.

Scoring

Met	Partially Met	Not Met
High (90-100%) on file review	Medium (60-89%) on file review	Low (0-59%) on file review

Data source Records or files

Scope of review**Product lines**

This element applies to all product lines for First Surveys and Renewal Surveys.

NCQA reviews and scores this element for each product line brought forward for Accreditation; only the commercial and Exchange product line files may be combined.

Documentation

For First Surveys and Renewal Surveys: NCQA reviews a random sample of up to 40 behavioral healthcare denial files resulting from medical necessity review for evidence of opportunity for a practitioner to discuss a denial with a reviewer.

Look-back period

For First Surveys: 6 months.

For Renewal Surveys: 12 months.

Explanation This element applies to all behavioral healthcare denial determinations resulting from medical necessity review (as defined in UM 1, Element A).

Dispute of file review results

NCQA conducts onsite file review in the presence of the organization's staff and works with the organization to resolve any disputes during the onsite survey. An organization that is unable to resolve a dispute with the survey team must contact NCQA before the onsite survey is complete. File review results may not be disputed or appealed once the onsite survey is complete.

Opportunity to discuss denial decisions

The organization notifies the treating practitioner about the opportunity to discuss a medical necessity denial:

- In the denial notification, **or**
- By telephone, including leaving a voicemail, if the organization documents the name of the individual at the organization who notified the treating practitioner or left the voicemail, and the date and time of the notification or voicemail, **or**
- In materials sent to the treating practitioner, informing the practitioner of the opportunity to discuss a specific denial with a reviewer.

The organization includes the following information in the denial file:

- The denial notification, if the treating practitioner was notified in the denial notification.
- The time and date of the notification and the name of the individual at the organization, if the treating practitioner was notified by telephone.
 - If the treating practitioner was notified by voicemail, the name of the individual who left the voicemail, and the date and time.
- Evidence that the treating practitioner was notified that a physician or other reviewer is available to discuss the denial, if notified in materials sent to the treating practitioner.

NCQA does not require evidence of discussion with an attending or treating practitioner, and does not consider the discussion to be an appeal.

For the Medicare product line, the organization may provide the treating practitioner with an opportunity to discuss a UM request with a physician or other appropriate reviewer prior to the decision to meet the intent of this element. The organization must provide documentation in the denial file.

Exception

This element is NA if all purchasers of the organization's services carve out or exclude behavioral healthcare.

Examples None.

Element E: Written Notification of Behavioral Healthcare Denials

The organization's written notification of behavioral healthcare denials, that it provided to members and their treating practitioners, contains:

1. The specific reasons for the denial, in easily understandable language.
2. A reference to the benefit provision, guideline, protocol or other similar criterion on which the denial decision was based.
3. A statement that the member can obtain a copy of the actual benefit provision, guideline, protocol or other similar criterion on which the denial decision was based, upon request.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	High (90-100%) on file review for at least 2 factors and medium (60-89%) on file review for any remaining factor	High (90-100%) or medium (60-89%) on file review for 3 factors	Low (0-59%) on file review for any factor

Data source Records or files

Scope of review **Product lines**

This element applies to all product lines for First Surveys and Renewal Surveys.

NCQA reviews and scores this element for each product line brought forward for Accreditation; only the commercial and Exchange product line files may be combined.

Documentation

For First Surveys and Renewal Surveys: NCQA reviews a random sample of up to 40 behavioral healthcare denial files resulting from medical necessity review for evidence that denial notices meet all three factors.

Look-back period *For First Surveys:* 6 months.

For Renewal Surveys: 12 months.

Explanation **THIS IS A MUST-PASS ELEMENT.**

This element applies to all behavioral healthcare denial determinations resulting from medical necessity review (as defined in UM 1, Element A).

Dispute of file review results

NCQA conducts onsite file review in the presence of the organization's staff and works with the organization to resolve any disputes during the onsite survey. An organization that is unable to resolve a dispute with the survey team must contact NCQA before the onsite survey is complete. File review results may not be disputed or appealed once the onsite survey is complete.

Factor 1: Reason for denial

The denial notification states the reason for the denial in terms specific to the member's condition or request and in language that is easy to understand, so the member and practitioner understand why the organization denied the request and have enough information to file an appeal.

An appropriately written notification includes a complete explanation of the grounds for the denial, in language that a layperson would understand, and does not include abbreviations, acronyms or health care procedure codes that a layperson would not understand.

The organization is not required to spell out abbreviations/acronyms if they are clearly explained in lay language.

To illustrate, for the acronym DNA, spelling out would be "a DNA (deoxyribonucleic acid)" whereas explaining would be "a DNA test that looks at your genetic information."

Denial notifications sent only to practitioners may include technical or clinical terms.

The organization may send a single notice to the member and practitioner that includes the specific reason for the denial, in language that would be easily understood by the member. The notice may also include, in a separate section, additional clinical or technical language directed toward a practitioner.

For denials resulting from medical necessity review of out-of-network requests, the reason for the denial must explicitly address the reason for the request (e.g., if the request is related to accessibility issues that may be impacted by the clinical urgency of the situation, the denial must address whether or not the requested service can be obtained within the organization's accessibility standards).

Factor 2: Reference to UM criterion

The denial notification references the specific criterion used to make the denial decision. The criterion used and referenced is specific to the member's condition or to the requested services.

The criterion referenced must be identifiable by name and must be specific to an organization or source (e.g., ABC PBM's Criteria for Treatment of Hypothyroidism with Synthroid or Criteria Company Inc.'s Guidelines for Wound Treatment).

If the organization uses a trademarked criterion name, it does not need to cite the organization that holds the trademark. If it is clear that the criterion is attributable to the organization, it is acceptable to state, "our Criteria for XXX" (e.g., our Criteria for Treating High Cholesterol with Lipitor).

Because benefit documents are often large and complex, the organization must direct members to the information using the section title or page number.

For denials resulting from medical necessity review of out-of-network requests, criteria may be excerpted from benefit documents that govern out-of-network coverage, organization policies specifying circumstances where out-of-network coverage will be approved or clinical criteria used to evaluate the member's clinical need relative to available network providers and services. The reference must specifically support the rationale for the decision and must relate to the reason for the request.

Factor 3: Availability of criterion

The denial notification informs the member, and the practitioner acting as the member's authorized representative, that the criterion used to make the decision is available upon request. Providing the criterion, or an excerpt specific to the denial reason with the denial notification is also acceptable. NCQA scores this factor "Yes" if the criterion or excerpt is included in the decision notice or if the notification states that the criterion or excerpt is included as an attachment.

Exceptions

This element is NA if all purchasers of the organization's services carve out or exclude behavioral healthcare.

Factor 3 is NA for Medicare denials.

Related information

Refer to *Related information* in UM 5, Element B for "Member Notification Exceptions" and "Notifying the Practitioner."

Denials due to insufficient clinical information. If the organization denies a request due to lack of clinical information, the denial notice must meet factors 1–3.

If the organization does not have enough clinical information to reference a specific criterion, the denial notice must state this and specify the information needed.

Examples**Factor 1: Acceptable language documenting the reason for denial**

Unless it is not medically recommended, less intense treatments, such as a trial of different antidepressant medications, should be used prior to considering ECT. Your physician has not indicated that you have tried antidepressant medications; therefore, the requested ECT is denied at this time. We recommend that you discuss other treatment options with your physician.

Insufficient language documenting reason for denial

The treatment is determined to not be medically necessary.

Factors 1, 2: Denying an out-of-network exception request and referencing UM criteria

A member's primary care practitioner requests out-of-network coverage for treatment of ADHD, explaining that only a specific pediatric psychiatrist can meet the member's needs. Medical records demonstrate initial screening by the primary care practitioner; no other medical or behavioral diagnoses are noted.

The organization's denial notification states, "Our medical director has reviewed your child's primary care physician's request for coverage of treatment for attention deficit hyperactivity disorder (or "ADHD") with Dr. Jones, an out-of-network pediatric psychiatrist. As stated in your Certificate of Coverage under "Out of Network Coverage," your plan covers out-of-network practitioners only when your clinical needs cannot be met in-network. Your primary care physician did not provide evidence that your child has special needs related to the ADHD diagnosis or treatment. Several in-network pediatric psychiatrists are trained to diagnose and treat ADHD. Please work with your primary care physician to select an in-network practitioner.

Factors 2, 3: Acceptable language referencing decision-making criteria

After reviewing and discussing your current symptoms with your practitioner, we believe that your condition has improved to the point where you no longer pose a danger to yourself or to others and can be managed appropriately in an intensive outpatient program. Because your symptoms no longer meet our Criteria for Inpatient Detoxification Monitoring, your request for further days in the hospital is denied.

You can obtain a copy of the criteria on which this decision was based by sending a request to us at the following address or contacting us by telephone.

Element F: Written Notification of Behavioral Healthcare Appeal Rights/Process

The organization's written notification of behavioral healthcare denials, which it provides to members and their treating practitioners, contains the following information:

1. A description of appeal rights, including the right to submit written comments, documents or other information relevant to the appeal.
2. An explanation of the appeal process, including members' right to representation and appeal time frames.
3. A description of the expedited appeal process for urgent preservice or urgent concurrent denials.
4. Notification that expedited external review can occur concurrently with the internal appeals process for urgent care.

Summary of Changes**Clarifications**

- Replaced "Behavioral Healthcare Notice of Appeal Rights/Process" with "Written Notification of Behavioral Healthcare Appeal Rights/Process" in the element title.

Scoring

Met	Partially Met	Not Met
High (90-100%) on file review for 3 factors and medium (60-89%) on file review for any remaining factor	High (90-100%) or medium (60-89%) on file review for 4 factors	Low (0-59%) on file review for any factor

Data source

Records or files

Scope of review**Product lines**

This element applies to all product lines for First Surveys and Renewal Surveys.

NCQA reviews and scores this element for each product line brought forward for Accreditation; only the commercial and Exchange product line files may be combined.

Documentation

For First Surveys and Renewal Surveys: NCQA reviews a random sample of up to 40 behavioral healthcare denial files resulting from medical necessity review for evidence that denial notices meet all four factors.

Look-back period *For First Surveys:* 6 months.
 For Renewal Surveys: 12 months.

Explanation **THIS IS A MUST-PASS ELEMENT.**

This element applies to all behavioral healthcare denial determinations resulting from medical necessity review (as defined in UM 1, Element A).

Dispute of file review results

NCQA conducts onsite file review in the presence of the organization's staff and works with the organization to resolve any disputes during the onsite survey. An organization that is unable to resolve a dispute with the survey team must contact NCQA before the onsite survey is complete. File review results may not be disputed or appealed once the onsite survey is complete.

Factor 1: Description of appeal rights

No additional explanation required.

Factor 2: Right to representation and appeal time frames

The denial notification:

- Includes a statement that members may be represented by anyone they choose, including an attorney.
- Provides contact information for a state office of health insurance consumer assistance or ombudsperson, if applicable.
Note: *This is not required for members covered by the Federal Employee Health Benefits (FEHB) program.*
- States the time frame for filing an appeal.
- States the organization's time frame for deciding the appeal.
- States the procedures for filing an appeal, including where to direct the appeal and information to include in the appeal.

The notification to the practitioner does not have to include the member's right to representation if the practitioner is not the member's authorized representative.

Factor 3: Expedited appeal process

The denial notification describes the expedited appeals process for urgent preservice or urgent concurrent denials. If the same process applies to standard and expedited appeals, there must be a description included in the letter that makes it clear that the process applies to both. Factor 3 is met if the organization includes a description of the expedited appeals process in denial notices for every type of request.

The denial notification states:

- The time frame for filing an expedited appeal.
- The organization's time frame for deciding the appeal.
- The procedure for filing an expedited appeal, including where to direct the appeal and information to be included in the appeal.

Factor 4: Concurrent expedited external review

The denial notification states that for urgent care situations, expedited external review may occur at the same time as the internal appeal process.

The organization may discontinue the internal appeal for all member requests that were addressed by the external review if it is not required to continue the internal appeal process under state law. (The organization continues the internal appeal process for components of the request that are not addressed in the external review.)

Exceptions

This element is NA if all purchasers of the organization's services carve out or exclude behavioral healthcare.

Factor 4 is NA for:

- Members covered by Medicare, Medicaid or the FEHB Program and for members in self-funded accounts.
- Nonurgent preservice and postservice denial decisions.

Related information

Refer to *Related information* in UM 5, Element B for "Member Notification Exceptions" and "Notifying the Practitioner."

Medicare denials and Fully Integrated Dual Eligible (FIDE) denials. CMS requires organizations to issue an Integrated Denial Notice (IDN) for non-inpatient medical service denials for Medicare and FIDE members. The IDN meets factors 1–3 for these members.

Examples None.

Element G: Discussing a Pharmacy Denial With a Reviewer

The organization gives practitioners the opportunity to discuss pharmacy UM denial decisions with a physician or pharmacist.

Summary of Changes**Clarifications**

- Clarified in the second and fourth bullets under "Opportunity to discuss denial decisions" that the name of the individual who notified the practitioner via telephone must be documented.
- Clarified in the fourth bullet under "Opportunity to discuss denial decisions" that if a voicemail is left, the denial file must include who left the message and the date and time.

Scoring

Met	Partially Met	Not Met
High (90-100%) on file review	Medium (60-89%) on file review	Low (0-59%) on file review

Data source Records or files

Scope of review**Product lines**

This element applies to all product lines for First Surveys and Renewal Surveys.

NCQA reviews and scores this element for each product line brought forward for Accreditation; only the commercial and Exchange product line files may be combined.

Documentation

For First Surveys and Renewal Surveys: NCQA reviews a random sample of up to 40 pharmaceutical denial files resulting from medical necessity review for evidence of opportunity for a practitioner to discuss a denial with a reviewer.

Look-back period

For First Surveys: 6 months.

For Renewal Surveys: 12 months.

Explanation

This element applies to all pharmaceutical denial determinations resulting from medical necessity review (as defined in UM 1, Element A), whether the pharmaceutical is covered under an organization's medical benefit or its pharmacy benefit.

Dispute of file review results

NCQA conducts onsite file review in the presence of the organization's staff and works with the organization to resolve any disputes during the onsite survey. An organization that is unable to resolve a dispute with the survey team must contact NCQA before the onsite survey is complete. File review results may not be disputed or appealed once the onsite survey is complete.

Opportunity to discuss pharmaceutical denial decisions

The organization notifies the treating practitioner about the opportunity to discuss a pharmaceutical medical necessity denial:

- In the denial notification, **or**
- By telephone, including leaving a voicemail, if the organization documents the name of the individual at the organization who notified the treating practitioner or left the voicemail, and the date and time of the notification or voicemail, **or**
- In materials sent to the treating practitioner, informing the practitioner of the opportunity to discuss a specific denial with a reviewer.

The organization includes the following information in the denial file:

- The denial notification, if the treating practitioner was notified in the denial notification.
- The time and date of the notification, and the name of the individual at the organization, if the treating practitioner was notified by telephone.
 - If the treating practitioner was notified by voicemail, the name of the individual who left the voicemail, and the date and time.
- Evidence that the treating practitioner was notified that a physician or pharmacist reviewer is available to discuss the denial, if notified in materials sent to the treating practitioner.

NCQA does not require evidence of discussion with an attending or treating practitioner and does not consider the discussion to be an appeal.

For the Medicare product line, the organization may provide the treating practitioner with an opportunity to discuss a UM request with a physician or other appropriate reviewer prior to the decision to meet the intent of this element. The organization must provide documentation in the denial file.

Exception

This element is NA if all purchasers of the organization's services carve out or exclude pharmaceutical management.

Examples None.

Element H: Written Notification of Pharmacy Denials

The organization's written notification of pharmacy denials to members and their treating practitioners contains the following information:

1. The specific reasons for the denial, in language that is easy to understand.
2. A reference to the benefit provision, guideline, protocol or similar criterion on which the denial decision is based.
3. A statement that members can obtain a copy of the actual benefit provision, guideline, protocol or similar criterion on which the denial decision was based, upon request.

Summary of Changes

- No changes to this element.

Scoring

Met	Partially Met	Not Met
High (90-100%) on file review for at least 2 factors and medium (60-89%) on file review for any remaining factor	High (90-100%) or medium (60-89%) on file review for 3 factors	Low (0-59%) on file review for any factor

Data source Records or files

Scope of review

Product lines

This element applies to all product lines for First Surveys and Renewal Surveys.

NCQA reviews and scores this element for each product line brought forward for Accreditation; only the commercial and Exchange product line files may be combined.

Documentation

For First Surveys and Renewal Surveys: NCQA reviews a random sample of up to 40 pharmaceutical denial files resulting from medical necessity review for evidence that denial notices meet all three factors.

Look-back period

For First Surveys: 6 months.

For Renewal Surveys: 12 months.

Explanation THIS IS A MUST-PASS ELEMENT.

This element applies to all pharmaceutical denial determinations resulting from medical necessity review (as defined in UM 1, Element A), whether the pharmaceutical is covered under an organization's medical benefit or its pharmacy benefit.

Dispute of file review results

NCQA conducts onsite file review in the presence of the organization's staff and works with the organization to resolve any disputes during the onsite survey. An organization that is unable to resolve a dispute with the survey team must contact NCQA before the onsite survey is complete. File review results may not be disputed or appealed once the onsite survey is complete.

Factor 1: Reason for denial

The denial notification states the reason for the denial in terms specific to the member's condition or request and in language that is easy to understand, so the member and practitioner understand why the organization denied the request and have enough information to file an appeal.

An appropriately written notification includes a complete explanation of the grounds for the denial, in language that a layperson would understand, and does not include abbreviations, acronyms or health care procedure codes that a layperson would not understand.

The organization is not required to spell out abbreviations/acronyms if they are clearly explained in lay language.

To illustrate, for the acronym DNA, spelling out would be "a DNA (deoxyribonucleic acid)" whereas explaining would be "a DNA test looks at your genetic information."

Denial notifications sent only to practitioners may include technical or clinical terms.

The organization may send a single notice to the member and practitioner that includes the specific reason for the denial, in language that would be easily understood by the member. The notice may also include, in a separate section, additional clinical or technical language directed toward a practitioner.

For denials resulting from medical necessity review of out-of-network requests, the reason for the denial must explicitly address the reason for the request (e.g., if the request is related to accessibility issues, that may be impacted by the clinical urgency of the situation, the denial must address whether or not the requested service can be obtained within the organization's accessibility standards).

Factor 2: Reference to UM criterion

The denial notification references the specific criterion used to make the denial decision. The criterion used and referenced is specific to the member's condition or to the requested services.

The criterion referenced must be identifiable by name and must be specific to an organization or source (e.g., ABC PBM's Criteria for Treatment of Hypothyroidism with Synthroid or Criteria Company Inc.'s Guidelines for Wound Treatment).

If the organization uses a trademarked criterion name, it does not need to cite the organization that holds the trademark. If it is clear that the criterion is attributable to the organization, it is acceptable to state, "our Criteria for XXX" (e.g., our Criteria for Treating High Cholesterol with Lipitor).

Because benefit documents are often large and complex, the organization must direct members to the information using the section title or page number.

For denials resulting from medical necessity review of out-of-network requests, criteria may be excerpted from benefit documents that govern out-of-network coverage, organization policies specifying circumstances where out-of-network coverage will be approved or clinical criteria used to evaluate the member's clinical need relative to available network providers and services. The reference must specifically support the rationale for the decision and must relate to the reason for the request.

Factor 3: Availability of criterion

The denial notification informs the member, and the practitioner acting as the member's authorized representative, that the criterion used to make the decision is available upon request. Providing the criterion, or an excerpt specific to the denial reason with the denial notification is also acceptable. NCQA scores this factor "Yes" if the criterion or excerpt is included in the decision notice or if the notification states that the criterion or excerpt is included as an attachment.

Exceptions

This element is NA if all purchasers of the organization's services carve out or exclude pharmaceutical management.

Factor 3 is NA for Medicare denials.

Related information

Refer to *Related information* in UM 5, Element C for "Member Notification Exceptions" and "Notifying the Practitioner."

Denials due to insufficient clinical information. If the organization denies a request due to lack of clinical information, the denial notice must meet factors 1–3.

If the organization does not have enough clinical information to reference a specific criterion, the denial notice must state this and specify the information needed.

Examples

None.

Element I: Written Notification of Pharmacy Appeal Rights/Process

The organization's written notification of pharmacy denials to members and their treating practitioners contains the following information:

1. A description of appeal rights, including the member's right to submit written comments, documents or other information relevant to the appeal.
2. An explanation of the appeal process, including the member's right to representation and the appeal time frames.
3. A description of the expedited appeal process for urgent preservice or urgent concurrent denials.
4. Notification that expedited external review can occur concurrently with the internal appeal process for urgent care.

Summary of Changes

Clarifications

- Replaced "Pharmacy Notice of Appeal Rights/Process" with "Written Notification of Pharmacy Appeal Rights/Process" in the element title.

Scoring

Met	Partially Met	Not Met
High (90-100%) on file review for 3 factors and medium (60-89%) on file review any remaining factor	High (90-100%) or medium (60-89%) on file review for 4 factors	Low (0-59%) on file review for any factor

Data source Records or files

Scope of review

Product lines

This element applies to all product lines for First Surveys and Renewal Surveys.

NCQA reviews and scores this element for each product line brought forward for Accreditation; only the commercial and Exchange product line files may be combined.

Documentation

For First Surveys and Renewal Surveys: NCQA reviews a random sample of up to 40 pharmaceutical denial files resulting from medical necessity review for evidence that denial notices meet all four factors.

Look-back period

For First Surveys: 6 months.

For Renewal Surveys: 12 months.

Explanation

THIS IS A MUST-PASS ELEMENT.

This element applies to all pharmaceutical denial determinations resulting from medical necessity review (as defined in UM 1, Element A), whether the pharmaceutical is covered under an organization's medical benefit or its pharmacy benefit.

Dispute of file review results

NCQA conducts onsite file review in the presence of the organization's staff and works with the organization to resolve any disputes during the onsite survey. An organization that is unable to resolve a dispute with the survey team must contact NCQA before the onsite survey is complete. File review results may not be disputed or appealed once the onsite survey is complete.

Factor 1: Description of appeal rights

No additional explanation required.

Factor 2: Right to representation and appeal time frames

The denial notification:

- Includes a statement that members may be represented by anyone they choose, including an attorney.
- Provides contact information for a state office of health insurance consumer assistance or ombudsman, if applicable.
***Note:** This is not required for members covered by the Federal Employee Health Benefits (FEHB) program.*
- States the time frame for filing an appeal.
- States the organization's time frame for deciding the appeal.
- States the procedure for filing an appeal, including where to direct the appeal and information to include in the appeal.

The notification to the practitioner is not required to include the member's right to representation if the practitioner is not acting as an authorized representative.

Factor 3: Expedited appeal process

The denial notification describes the expedited appeals process for urgent preservice or urgent concurrent denials. If the same process applies to standard and expedited appeals, there must be a description included in the letter that makes it clear that the process applies to both. Factor 3 is met if the organization includes a description of the expedited appeals process in denial notices for every type of request.

The denial notification states:

- The time frame for filing an expedited appeal.
- The organization's time frame for deciding the appeal.
- The procedure for filing an expedited appeal, including where to direct the appeal and information to include in the appeal.

Factor 4: Concurrent expedited external review

The denial notification states that for urgent care situations, expedited external review may occur at the same time as the internal appeal process. The organization may discontinue the internal appeal for all member requests addressed by the external review if it is not required to continue the internal appeal process under state law. The organization continues the internal appeal process for components of the request that are not addressed in the external review.

Exceptions

This element is NA if all purchasers of the organization's services carve out or exclude pharmaceutical management.

Factor 4 is NA for:

- Members covered by Medicare, Medicaid or the FEHB Program and for members in self-funded accounts.
- Nonurgent preservice and postservice denial decisions.

Related information

Medicare denials and FIDE denials. CMS requires organizations to:

- Issue an IDN for non-inpatient medical service denials for Medicare and FIDE members.
 - The IDN meets factors 1–3 for these members.
- Issue a Notice of Denial of Medicare Prescription Drug Coverage for Medicare Part D members.
 - The notice meets factors 1–3 for these members.

Examples None.

UM 8: Policies for Appeals

The organization has written policies and procedures for thorough, appropriate and timely resolution of member appeals.

Intent

There is an established, impartial process for resolving members' disputes and responding to member requests to reconsider a decision they find unacceptable regarding their care and service.

Element A: Internal Appeals

The organization's written policies and procedures for registering and responding to written internal appeals include the following:

1. For commercial and Exchange, allowing at least 180 calendar days after notification of the denial for the member to file an appeal.
2. For Medicare and Medicaid, allowing at least 60 calendar days after notification of the denial for the member to file an appeal.
3. Documenting the substance of the appeal and any actions taken.
4. Full investigation of the substance of the appeal, including any aspects of clinical care involved.
5. The opportunity for the member to submit written comments, documents or other information relating to the appeal.
6. Appointment of a new person to review an appeal who was not involved in the initial determination and who is not the subordinate of any person involved in the initial determination.
7. Appointment of at least one person to review an appeal who is a practitioner in the same or a similar specialty.
8. The decision for a preservice appeal and notification to the member within 30 calendar days of receipt of the request.
9. The commercial, Exchange and Medicare decision for a postservice appeal and notification to the member within 60 calendar days of receipt of the request.
10. For Medicaid, the decision for a postservice appeal and notification to the member within 30 calendar days of receipt of the request.
11. The decision for an expedited appeal and notification to the member within 72 hours of receipt of the request.
12. Notification to the member about further appeal rights.
13. Referencing the benefit provision, guideline, protocol or other similar criterion on which the appeal decision is based.
14. Giving members reasonable access to and copies of all documents relevant to the appeal, free of charge, upon request.
15. Including a list of titles and qualifications, including specialties, of individuals participating in the appeal review.
16. Allowing an authorized representative to act on behalf of the member.

17. Providing notices of the appeals process to members in a culturally and linguistically appropriate manner.
18. Continued coverage pending the outcome of an appeal.

Summary of Changes

Clarifications

- Added a new factor 2 and factor 10 to align with language in the Explanation and renumbered the factors.
- Clarified factors 1 and 9 to include the applicable product lines, to align with the Explanation section.
- Revised the factor numbering in the Explanation section to align with the factors.
- Added applicable Exceptions.

Scoring

Met	Partially Met	Not Met
The organization meets 14-18 factors	The organization meets 9-13 factors	The organization meets 0-8 factors

Data source Documented process

Scope of review

Product lines

For Interim Surveys and First Surveys, this element applies to all product lines.

For Renewal Surveys, this element applies to the Medicaid product line only.

Documentation

For Interim Surveys, First Surveys and Renewal Surveys: NCQA reviews the organization's policies and procedures.

Look-back period

For Interim Surveys: Prior to the survey date.

For First Surveys: 6 months.

For Renewal Surveys: 24 months.

Explanation

This element is a **structural requirement**. The organization must present its own documentation.

This element applies to appeal decisions related to coverage or rescission of coverage, whether or not the denial resulted from medical necessity review (e.g., medical, behavioral health, pharmacy or personal care services).

A member may appeal any adverse medical necessity or benefit decision.

With a member's permission, the organization may refer an appeal directly to an IRO without conducting an internal review.

Classification of appeal requests

Preservice appeal: An appeal of an adverse decision for coverage of care or services in advance of the member obtaining care or services.

Postservice appeal: A request to change an adverse determination for care or services that have been received by the member.

Expedited appeal: An appeal on an adverse decision for coverage for urgent care.

Factor 1, 2: Appeal filing time frames

Appeal policies and procedures include the following time frames for members (or their authorized representatives) to file an appeal, as applicable:

- *For commercial and Exchange product lines:* 180 calendar days or more.
- *For Medicare and Medicaid product lines:* 60 calendar days.

Factor 3: Documenting the substance of an appeal and actions taken

Appeal policies and procedures specify that documentation of the substance of the appeal includes, but is not limited to:

- The member's reason for appealing the previous decision.
- Additional clinical or other information provided with the appeal request.

Appeal policies and procedures specify that documentation of actions taken includes, but is not limited to:

- Previous denial or appeal history.
- Follow-up activities associated with the denial and conducted before the current appeal.

The organization determines the extent of documentation in the appeal file.

Factor 4: Investigating the substance of an appeal

Appeal policies and procedures specify that the organization fully investigates the content of the appeal and documents its findings. The organization's appeal review does not give deference to the denial decision.

Factor 5: Right to submit comments and other information

Appeal policies and procedures specify that members have the opportunity to submit information relevant to the appeal. The organization documents when members fail to submit relevant information by the specified deadline.

Factor 6: Person or people deciding the appeal

Appeal policies and procedures specify who in the organization decides appeals.

The organization may designate any individual or group (e.g., a panel) in its policies and procedures to overturn appeals and to uphold appeals that do not require medical necessity review.

However, for appeals that require medical necessity review, the final decision to uphold an appeal must be made by an appropriate practitioner or a group (e.g., a panel) that includes an appropriate practitioner who was not involved in the initial denial decision and is not subordinate to the practitioner who made the initial denial decision.

NCQA considers the following practitioner types to be appropriate for review of the specified UM denial decisions:

- *Physicians, all types:* Medical, behavioral healthcare, pharmaceutical, dental, chiropractic and vision denials.
- *Nurse practitioners*:* Medical, behavioral healthcare, pharmaceutical, dental, chiropractic and vision denials.
- *Doctoral-level clinical psychologists or certified addiction-medicine specialists:* Behavioral healthcare denials.
- *Pharmacists:* Pharmaceutical denials.

- *Dentists*: Dental denials.
- *Chiropractors*: Chiropractic denials.
- *Physical therapists*: Physical therapy denials.
- *Doctoral-level board-certified behavioral analysts*: Applied behavioral analysis denials.

* In states where the organization has determined that practice acts or regulations allow nurse practitioners to practice independently, nurse practitioners may review requests that are within the scope of their license.

Factor 7: Same-or-similar-specialist review

Appeal policies and procedures require same-or-similar-specialist review as part of the process to uphold the initial decision in an appeal that requires medical necessity review.

The purpose of same-or-similar specialist review of appeals is to apply specific clinical knowledge and experience when determining if an appeal meets criteria for medical necessity and clinical appropriateness.

The same-or-similar specialist may be the same individual designated to make the appeal decision or may be a separate reviewer who provides a recommendation to the individual making the decision. The same-or-similar specialist may be any of the practitioner types specified in factor 5, with the exception of pharmacists, because pharmacists generally treat patients only in limited situations and therefore are not considered same-or-similar specialists for the purposes of deciding appeals.

To be considered a same-or-similar specialist, the reviewing specialist's training and experience must meet the following criteria:

- Includes treating the condition.
- Includes treating complications that may result from the service or procedure.
- Is sufficient for the specialist to determine if the service or procedure is medically necessary or clinically appropriate.

"Training and experience" refers to the practitioner's clinical training and experience.

When reviewing appeal files, NCQA reviews whether the specialist's training and experience aligns with the condition, service or procedure in question, as opposed to requiring an exact match to the referring or treating practitioner type or specialty.

The intent is that the specialist reviewing the appeal would have encountered a patient with this condition who is considering or has received the service or procedure in a clinical setting. Because of this, more complex services and procedures require review by practitioners with more specialized training and experience. For example, while a decision to uphold a denial of hospital admission for arrhythmia might be reviewed by any number of practitioners, including, but not limited to, a cardiologist, cardiothoracic surgeon, internist, family practitioner, geriatrician or emergency medicine physician, a decision to uphold a denial of surgery to repair an atrial septal defect in a newborn would require review by a cardiothoracic surgeon with pediatric experience.

NCQA accepts board certification in a specialty as a proxy for clinical training and experience. A specialist who maintains board certification in a general and specialty area (e.g., internal medicine and pulmonology) is considered to have training and experience in both areas. NCQA does not require that the same-or-similar specialist reviewer be actively practicing.

Experience with the condition, service or procedure that is limited to UM decision making in cases similar to the appeal in question is not considered sufficient experience, nor do UM decision-making criteria supersede the requirement for same-or-similar specialist review.

If the organization's clinical criteria limits who can perform a service or procedure, or who can prescribe a pharmaceutical to specific practitioner types or specialties, then only those practitioner types or specialties may be considered same-or-similar specialist reviewers.

Factors 8–11: Appeal decisions

Appeal policies and procedures specify that appeal decisions and notifications are timely. The appeal decision notification states the reason for upholding the denial in terms specific to the member's condition or request and in language that is easy to understand, so the member and practitioner understand why the organization upheld the appeal decision and have enough information to file the next level of appeal.

An appropriately written notification includes a complete explanation of the ground for the upheld appeal decision, in language that a layperson would understand, and does not include abbreviations, acronyms or health care procedure codes that a layperson would not understand.

The organization is not required to spell out abbreviations/acronyms if they are clearly explained in lay language.

To illustrate, for the acronym DNA, spelling out would be "a DNA (deoxyribonucleic acid)" whereas explaining would be "a DNA test looks at your genetic information."

Upheld appeal notifications sent only to practitioners may include technical or clinical terms.

The organization may send a single notice to the member and practitioner that includes the specific reason for the upheld appeal, in language that would be easily understood by the member. The notice may also include, in a separate section, additional clinical or technical language directed toward a practitioner.

For expedited appeals (factor 11), appeal policies and procedures specify that the organization grants an expedited review for all requests concerning admissions, continued stay or other health care services for a member who has received emergency services but has not been discharged from a facility.

Factor 12: Notification of further appeal rights

Appeal policies and procedures include a description of the next level of appeal, either within the organization or to an external organization, as applicable, and relevant written procedures.

Factor 13: Reference to and excerpt from criteria

Appeal policies and procedures specify that the appeal notification references the specific criterion used to make the appeal decision.

Appeal policies and procedures specify that the organization informs the member, and the practitioner acting as the member's authorized representative, that the criterion used to make the decision is available upon request. The criterion availability component of factor 13 is met if the policies and procedures specify that the criterion, or an excerpt of the criterion, is included in the decision notice or as an attachment.

Factor 14: Access to and copies of documents

No additional explanation required.

Factor 15: Titles and qualifications

Appeal policies and procedures require the appeal notice to identify all reviewers who participated in making the appeal decision, including the same-or-similar specialist reviewer, when applicable, as they provide specific clinical knowledge and experience that affects the decision.

For each individual, the notice includes:

- *For a benefit appeal:* The title (position or role in the organization).
- *For a medical necessity appeal:* The title (position or role in the organization), qualifications (clinical credentials such as MD, DO, PhD, physician) and specialty (e.g., pediatrician, general surgeon, neurologist, clinical psychologist).

The organization is not required to include individuals' names in the written notification.

Factor 16: Authorized representative

No additional explanation required.

Factor 17: Culturally and linguistically appropriate notification

Appeal policies and procedures specify that appeal notices are based on members' cultural and linguistic needs.

Factor 18: Continued coverage pending outcome of appeal

Note: *This factor applies to appeals of denials, reduction or termination of coverage for an ongoing course of treatment for which coverage was previously approved. It does not apply to requests for extensions.*

Appeal policies and procedures allow continued coverage, pending the outcome of an internal appeal of a concurrent care decision until:

- The end of the approved treatment period, **or**
- Determination of the appeal, subject to regulatory and contractual obligations.

If the outcome of the appeal is in the organization's favor, NCQA standards do not prohibit the organization from seeking reimbursement from the member for payments made.

Medicare appeals for factors 8, 9, 11–15

The organization's policies and procedures describe its process for sending an upheld denial to MAXIMUS.

Medicare Part D appeals for factors 8, 9, 11–13

The organization describes its process for notifying members that an upheld denial should be sent to MAXIMUS.

Exceptions

This element is NA for Renewal Surveys for the commercial, Medicare and Exchange product lines.

Factor 1 is NA for Medicare and Medicaid product lines.

Factor 2 is NA for commercial and Exchange product lines.

Factor 9 is NA for Medicaid product lines.

Factor 10 is NA for commercial, Medicare and Exchange product lines.

Factors 14 and 15 are NA for Medicare Part D appeals.

For factor 14, the “free of charge” component is NA for Medicare appeals.

Related information

Discussion with the practitioner. NCQA does not consider a request by a treating practitioner to discuss the decision with an appropriate practitioner reviewer in UM 7, Elements A and D, to be an initiation of the appeal process.

Extending the time frame to obtain additional information. Although there are allowable extensions for initial UM decisions, the organization may only extend the appeal time frame to obtain additional information when:

- The member agrees to extend the appeal time frame, **or**
- Federal program regulations allow the organization to request additional information from the member.

Allowable extensions for the Medicare and Medicaid product line only:

- For Medicare and Medicaid, the organization may allow a 14-day extension if the member requests the extension or the organization demonstrates that more information is needed and the delay is in the member’s interest.

Verbal notification. Verbal notification does not replace electronic or written notification of expedited appeal decisions, but when provided, the organization may extend the time frame for commercial, Medicare and Exchange decisions as described below.

- Verbal notification requires communication with a live person; the organization may not leave a voicemail, **and**
- The organization records the time and date of the notification and the staff member who spoke with the practitioner or member, **and**
- The organization provides verbal notification within 72 hours.

For commercial, Medicare and Exchange appeals, if the organization provides verbal notification for an expedited appeal, it has an additional 3 calendar days following verbal notification to provide electronic or written notification.

For Medicaid appeals, verbal notification is appropriate for nonurgent preservice, postservice and expedited appeals. Verbal notification of a decision does not extend the electronic or written notification time frame. Organizations may verbally inform members if there is a delay and must resolve appeals as expeditiously as the member’s health requires.

Expedited appeals. The organization may inform the hospital Utilization Review (UR) department staff of its decision, with the understanding that staff will inform the attending/treating practitioner. Notifications may be addressed to the hospital UR department, but must be to the attention of the attending or treating practitioner.

FEHB member appeals. For Federal Employees Health Benefits (FEHB) Program member appeals for which the organization requested additional information, NCQA gives the organization credit for factor 7 if its policies state that it makes appeal decisions within 30 calendar days after the date when the information was received.

Note: This may extend the normal 30-calendar-day preservice time frame.

Other levels of internal appeal. An organization may have two levels of internal appeal and may also have voluntary levels of appeal beyond the internal appeal process. The organization informs members of the process. Policies and procedures specify the time frames allocated for each level of internal appeal; however, the total of both time frames must equal or be less than the time frame specified for the type of appeal under review. For example, for a preservice appeal where NCQA allows 30 calendar days, the organization may allocate 10 calendar days to complete first-level appeals and 20 calendar days to complete second-level appeals.

The 180-calendar-day allowance for filing an appeal applies to first-level appeals.

NCQA standards do not specify a minimum time frame for members to file a second level appeal following the decision on a first level appeal.

Policies and procedures specify the level of review that includes same-or-similar-specialist review.

The organization suspends the statute of limitations or other legal or equitable basis for denial of the claim based on timeliness while a voluntary appeal is pending.

Use of practitioner web portals. The organization may provide electronic upheld appeal decision notifications to practitioners through a web portal if:

- The organization informs practitioners of the notification mechanism and their responsibility to check the portal regularly, **and**
- The organization documents the date and time when the information was posted in the portal, **and**
- The information posted in the portal meets the requirement in UM 8 and UM 9, **and**
- The organization has an alternative notification method for practitioners who do not have access to the web portal.

Use of member web portals. The organization may provide electronic upheld appeal decision notifications to members through a web portal if all the following criteria are met:

- The organization documents the member's agreement to receive electronic notifications via the portal.
- The organization documents the date and time when the information was posted in the portal.
- Members receive notification that a new document or update is available in the portal when it is posted (e.g., text, email, other electronic notification).
- The information posted in the portal meets the requirements in UM 8 and 9.
- The organization has an alternative notification method for members who do not have access to the web portal or who do not agree to receive notifications via the portal.

Examples None.

UM 9: Appropriate Handling of Appeals

The organization adjudicates member appeals in a thorough, appropriate and timely manner.

Intent

The organization has a full and fair process for resolving member disputes and responding to members' requests to reconsider a decision they find unacceptable regarding their care and service.

Element A: Preservice and Postservice Appeals

An NCQA review of the organization's appeal files indicates that they contain the following information:

1. Documentation of the substance of appeals.
2. Investigation of appeals.
3. Appropriate response to the substance of appeals.

Summary of Changes

Clarifications

- Revised the text under "Documentation" in the scope of review to clarify that appeal files include all medical necessity and benefit decision appeals.

Scoring

Met	Partially Met	Not Met
High (90-100%) on file review for at least 2 factors and medium (60-89%) on file review for any remaining factor	High (90-100%) or medium (60-89%) on file review for 3 factors	Low (0-59%) on file review for any factor

Data source Records or files

Scope of review

Product lines

This element applies to all product lines for First Surveys and Renewal Surveys.

NCQA reviews and scores this element for each product line brought forward for Accreditation; only the commercial and Exchange product line files may be combined.

Documentation

For First Surveys and Renewal Surveys: NCQA reviews a random sample of up to 40 upheld appeal files for evidence the appeal file contains all three factors. Appeal files include appeals of any denial of a request for coverage, whether or not the denial resulted from medical necessity review (e.g., medical, behavioral health, pharmacy or personal care services). This includes all medical necessity and benefit decision appeals.

Look-back period

For First Surveys: 6 months.

For Renewal Surveys: 12 months.

Explanation This element evaluates handling of appeals according to the policies required by *UM 8: Policies for Appeals*.

Dispute of file review results

NCQA conducts onsite file review in the presence of the organization's staff and works with the organization to resolve any disputes during the onsite survey. An organization that is unable to resolve a dispute with the survey team must contact NCQA before the onsite survey is complete. File review results may not be disputed or appealed once the onsite survey is complete.

Factor 1: Documentation of the substance

The organization's documentation of the appeal includes:

- The member's reason for appealing the previous decision.
- Actions taken include, including, but not limited to:
 - Previous denial or appeal history.
 - Follow-up activities associated with the denial and conducted before the current appeal.

Factor 2: Documentation of the investigation of appeals

The organization investigates the content of the appeal, including all aspects of clinical care involved, and documents its findings. The investigation does not defer to the denial decision.

Factor 3: Appropriate response to the substance of appeals

The organization's response is commensurate with the seriousness and urgency of the appeal. It directly responds to the reasons given by the member when appealing and addresses new information provided by the member or practitioner as part of the appeal.

Exceptions

None.

Related information

Personal care services. Benefit appeal files include appeals for personal care services such as cooking, grooming, transportation, cleaning and assistance with other activities of daily living.

Examples None.

Element B: Timeliness of the Appeal Process

The organization adheres to the following time frames for notification of preservice, postservice and expedited appeal decisions.

1. For preservice appeals, the organization gives electronic or written notification within 30 calendar days of receipt of the request.
2. For commercial, Exchange and Medicare postservice appeals, the organization gives electronic or written notification within 60 calendar days of receipt of the request.
3. For Medicaid postservice appeals, the organization gives electronic or written notification within 30 calendar days of the request.
4. For expedited appeals, the organization gives electronic or written notification within 72 hours of receipt of the request.

Summary of Changes

Clarifications

- Revised the text under “Documentation” in the scope of review to clarify that appeal files include all medical necessity and benefit decision appeals.
- Revised factors 1, 2 and 4 and added a new factor 3 to align with language in the Explanation; revised the factor numbering throughout the Explanation.
- Removed “Resolved appeals” sub-subhead and text under “Factors 1–4: Timeliness of appeal process” in the Explanation.
- Removed “Factor 2: Postservice appeals” subhead and text in the Explanation.
- Added Exceptions.
- Added the “Other levels of internal appeal” Explanation language from UM 8, Element A to the UM 9, Element B Explanation section.

Scoring

Met	Partially Met	Not Met
High (90-100%) on file review	Medium (60-89%) on file review	Low (0-59%) on file review

Data source Records or files

Scope of review

Product lines

This element applies to all product lines for First Surveys and Renewal Surveys.

NCQA reviews and scores this element for each product line brought forward for Accreditation; only the commercial and Exchange product line files may be combined.

Documentation

For First Surveys and Renewal Surveys: NCQA reviews a random sample of up to 40 upheld appeal files for evidence of timeliness of decision making. Appeal files include appeals of any denial of a request for coverage, whether or not the denial resulted from medical necessity review (e.g., medical, behavioral health, pharmacy or personal care services). This includes all medical necessity and benefit decision appeals.

Look-back period

For First Surveys: 6 months.

For Renewal Surveys: 12 months.

Explanation THIS IS A MUST-PASS ELEMENT.

This element evaluates handling of appeals according to the policies required by *UM 8: Policies for Appeals*.

Dispute of file review results

NCQA conducts onsite file review in the presence of the organization's staff and works with the organization to resolve any disputes during the onsite survey. An organization that is unable to resolve a dispute with the survey team must contact NCQA before the onsite survey is complete. File review results may not be disputed or appealed once the onsite survey is complete.

Factors 1–4: Timeliness of appeal process

NCQA considers 72 hours to be equivalent to 3 calendar days.

NCQA measures timeliness of notification from the date when the organization receives the request from the member or the member's authorized representative, even if the organization does not have all the information necessary to make a decision, to the date when the notice was provided to the member or member's authorized representative, as applicable.

For Medicare only: NCQA measures timeliness of notification for urgent requests from the date when the appropriate department receives the request.

The organization documents the date when it receives the request, and the date it resolves the appeal, in the appeal file. The request is received when it arrives at the organization, even if it is not received by the appeals department.

If the organization sends written notice, NCQA uses the date on the notice as the notification date. If the organization does not retain copies of the written notice, it has other methods of documenting the notification date. If the organization uses electronic notification, NCQA uses the date when the notification was posted in the electronic system.

Exceptions

Factor 2 is NA for Medicaid product lines.

Factor 3 is NA for commercial, Medicare and Exchange product lines.

Related information

Extending the decision time frame. The organization may extend the appeal time frame to obtain additional information if:

- The member agrees to extend the appeal time frame, **or**
- Federal program regulations allow the organization to request additional information from the member.

The organization may deny the appeal and notify the member if it does not receive the information within the time frames.

The organization documents the extension in the appeal file.

Allowable extensions for the Medicare and Medicaid product line only:

- For Medicare and Medicaid, the organization may allow a 14-day extension if the member requests the extension or the organization demonstrates that more information is needed and the delay is in the member's interest.

Verbal notification. Verbal notification does not replace electronic or written notification of expedited appeal decisions, but when provided, the organization may extend the time frame for commercial, Medicare and Exchange decisions as described below.

- Verbal notification requires communication with a live person; the organization may not leave a voicemail, **and**
- The organization records the time and date of the notification and the staff member who spoke with the practitioner or member, **and**
- The organization provides verbal notification within 72 hours.

For commercial, Medicare and Exchange appeals, if the organization provides verbal notification for an expedited appeal, it has an additional 3 calendar days following verbal notification to provide electronic or written notification.

For Medicaid appeals, verbal notification is appropriate for nonurgent preservice, postservice and expedited appeals. Verbal notification of a decision does not extend the electronic or written notification time frame. Organizations may verbally inform members if there is a delay and must resolve appeals as expeditiously as the member's health requires.

Expedited appeals. The organization may inform the hospital Utilization Review (UR) department staff of its decision, with the understanding that staff will inform the attending/treating practitioner. Notifications may be addressed to the hospital UR department, but must be to the attention of the attending or treating practitioner.

FEHB member appeals. For Federal Employees Health Benefits (FEHB) Program member appeals for which the organization requested additional information, preservice and postservice appeal files meet factors 1 and 2 if the decision was made within 30 calendar days after the date when the information was received.

Note: This may extend the preservice and postservice appeal time frames.

Other levels of internal appeal. An organization may have two levels of internal appeal and may also have voluntary levels of appeal beyond the internal appeal process. The organization informs members of the process. Policies and procedures specify the time frames allocated for each level of internal appeal; however, the total of both time frames must equal or be less than the time frame specified for the type of appeal under review. For example, for a preservice appeal where NCQA allows 30 calendar days, the organization may allocate 10 calendar days to complete first-level appeals and 20 calendar days to complete second-level appeals.

The 180-calendar-day allowance for filing an appeal applies to first-level appeals.

NCQA standards do not specify a minimum time frame for members to file a second level appeal following the decision on a first-level appeal.

Policies and procedures specify the level of review that includes same-or-similar-specialist review.

The organization suspends the statute of limitations or other legal or equitable basis for denial of the claim based on timeliness while a voluntary appeal is pending.

Use of practitioner web portals. The organization may provide electronic upheld appeal decision notifications to practitioners through a web portal if:

- The organization informs practitioners of the notification mechanism and their responsibility to check the portal regularly, **and**
- The organization documents the date and time when the information was posted in the portal, **and**
- The information posted in the portal meets the requirement in UM 8 and UM 9, **and**
- The organization has an alternative notification method for practitioners who do not have access to the web portal.

Use of member web portals. The organization may provide electronic upheld appeal decision notifications to members through a web portal if all the following criteria are met:

- The organization documents the member's agreement to receive electronic notifications via the portal.
- The organization documents the date and time when the information was posted in the portal.
- Members receive notification that a new document or update is available in the portal when it is posted (e.g., text, email, other electronic notification).
- The information posted in the portal meets the requirements in UM 8 and UM 9.
- The organization has an alternative notification method for members who do not have access to the web portal or who do not agree to receive notifications via the portal.

Examples None.

Element C: Appeal Reviewers

The organization provides nonsubordinate reviewers who were not involved in the previous determination and same-or-similar-specialist review, as appropriate.

Summary of Changes

Clarifications

- Revised the text under "Documentation" in the scope of review to clarify that appeal files include all medical necessity and benefit decision appeals.

Scoring

Met	Partially Met	Not Met
High (90-100%) on file review	Medium (60-89%) on file review	Low (0-59%) on file review

Data source Records or files

Scope of review **Product lines**

This element applies to all product lines for First Surveys and Renewal Surveys.

NCQA reviews and scores this element for each product line brought forward for Accreditation; only the commercial and Exchange product line files may be combined.

Documentation

For First Surveys and Renewal Surveys: NCQA reviews a random sample of up to 40 upheld appeal files for evidence of involvement of nonsubordinate and same-or-similar specialist reviewers. Appeal files include appeals of any denial of a request for coverage, whether or not the denial resulted from medical necessity review (e.g., medical, behavioral health, pharmacy or personal care services). This includes all medical necessity and benefit decision appeals.

Look-back period

For First Surveys: 6 months.

For Renewal Surveys: 12 months.

Explanation

This element evaluates handling of appeals according to the policies required by *UM 8: Policies for Appeals*.

Dispute of file review results

NCQA conducts onsite file review in the presence of the organization's staff and works with the organization to resolve any disputes during the onsite survey. An organization that is unable to resolve a dispute with the survey team must contact NCQA before the onsite survey is complete. File review results may not be disputed or appealed once the onsite survey is complete.

Person or people deciding the appeal

The organization may designate any individual or group (e.g., a panel) to overturn appeals and to uphold appeals that do not require medical necessity review.

However, for appeals that require medical necessity review, the final decision to uphold an appeal must be made by an appropriate practitioner or a group (e.g., a panel) that includes an appropriate practitioner who was not involved in the initial denial decision and is not subordinate to the practitioner who made the initial denial decision.

NCQA considers the following practitioner types to be appropriate for review of the specified UM denial decisions:

- *Physicians, all types:* Medical, behavioral healthcare, pharmaceutical, dental, chiropractic and vision denials.
- *Nurse practitioners*:* Medical, behavioral healthcare, pharmaceutical, dental, chiropractic and vision denials.
- *Doctoral-level clinical psychologists or certified addiction-medicine specialists:* Behavioral healthcare denials.
- *Pharmacists:* Pharmaceutical denials.
- *Dentists:* Dental denials.
- *Chiropractors:* Chiropractic denials.
- *Physical therapists:* Physical therapy denials.
- *Doctoral-level board-certified behavioral analysts:* Applied behavioral analysis denials.

* In states where the organization has determined that practice acts or regulations allow nurse practitioners to practice independently, nurse practitioners may review requests that are within the scope of their license.

Same-or-similar-specialist review

Same-or-similar-specialist review is a required part of the process to uphold the initial decision in an appeal that requires medical necessity review.

The purpose of same-or-similar specialist review of appeals is to apply specific clinical knowledge and experience when determining if an appeal meets criteria for medical necessity and clinical appropriateness.

The same-or-similar specialist may be the same individual designated to make the appeal decision or may be a separate reviewer who provides a recommendation to the individual making the decision. The same-or-similar specialist may be any of the practitioner types specified above, with the exception of pharmacists, because pharmacists generally treat patients only in limited situations and therefore are not considered same-or-similar specialists for the purposes of deciding appeals.

To be considered a same-or-similar specialist, the reviewing specialist's training and experience must meet the following criteria:

- Includes treating the condition.
- Includes treating complications that may result from the service or procedure.
- Is sufficient for the specialist to determine if the service or procedure is medically necessary or clinically appropriate.

"Training and experience" refers to the practitioner's clinical training and experience.

When reviewing appeal files, NCQA reviews whether the specialist's training and experience aligns with the condition, service or procedure in question, as opposed to requiring an exact match to the referring or treating practitioner type or specialty.

The intent is that the specialist reviewing the appeal would have encountered a patient with this condition who is considering or has received the service or procedure in a clinical setting. Because of this, more complex services and procedures require review by practitioners with more specialized training and experience. For example, while a decision to uphold a denial of hospital admission for arrhythmia might be reviewed by any number of practitioners, including, but not limited to, a cardiologist, cardiothoracic surgeon, internist, family practitioner, geriatrician or emergency medicine physician, a decision to uphold a denial of surgery to repair an atrial septal defect in a newborn would require review by a cardiothoracic surgeon with pediatric experience.

NCQA accepts board certification in a specialty as a proxy for clinical training and experience. A specialist who maintains board certification in a general and specialty area (e.g., internal medicine and pulmonology) is considered to have training and experience in both areas. NCQA does not require that the same-or-similar specialist reviewer be actively practicing.

Experience with the condition, service or procedure that is limited to UM decision making in cases similar to the appeal in question is not considered sufficient experience, nor do UM decision-making criteria supersede the requirement for same-or-similar specialist review.

If the organization's clinical criteria limits who can perform a service or procedure, or who can prescribe a pharmaceutical to specific practitioner types or specialties, then only those practitioner types or specialties may be considered same-or-similar specialist reviewers.

Exceptions

None.

Related information

Same-or-similar specialist review with multiple levels of appeal. If a second-level appeal file is chosen for review, NCQA reviews the file for all requirements, including same-or-similar specialist, even if same-or-similar specialist review occurs at the first level of appeal. If a first-level appeal file is chosen, and the specialist review occurs at the second level:

- NCQA scores the same-or-similar specialist review NA if the second-level appeal has not taken place.
- NCQA scores all requirements, including same-or-similar specialist, if the second-level appeal has taken place.

Because in general pharmacists treat patients in very limited situations, pharmacists are not considered same-or-similar specialists for the purposes of deciding appeals.

Automated systems. If the initial denial decision was made by an automated system (e.g., claims system), any reviewer is considered new and non-subordinate.

Contracting with a board-certified consultant. NCQA does not consider it delegation if the organization contracts with board-certified consultants who make recommendations and provide same-or-similar specialty review. If the consultant makes the appeal decision, NCQA considers this to be delegation.

Examples None.

Element D: Notification of Appeal Decision/Rights

An NCQA review of the organization's internal appeal files indicates notification to members and their treating practitioners of the following:

1. Specific reasons for the appeal decision, in easily understandable language.
2. A reference to the benefit provision, guideline, protocol or other similar criterion on which the appeal decision was based.
3. Notification that the member can obtain a copy of the actual benefit provision, guideline, protocol or other similar criterion on which the appeal decision was based, upon request.
4. Notification that the member is entitled to receive reasonable access to and copies of all documents, free of charge, upon request.
5. A list of titles and qualifications, including specialties, of individuals participating in the appeal review.
6. A description of the next level of appeal, either within the organization or to an independent external organization, as applicable, along with any relevant written procedures.

Summary of Changes

Clarifications

- Revised the text under “Documentation” in the scope of review to clarify that appeal files include all medical necessity and benefit decision appeals.
- Added “and their treating practitioners” to the element stem to align with the Explanation.

Scoring

Met	Partially Met	Not Met
High (90-100%) on file review for at least 4 factors and medium (60-89%) on file review for any remaining factors	High (90-100%) or medium (60-89%) on file review for 6 factors	Low (0-59%) on file review for any factor

Data source Records or files

Scope of review

Product lines

This element applies to all product lines for First Surveys and Renewal Surveys.

NCQA reviews and scores this element for each product line brought forward for Accreditation; only the commercial and Exchange product line files may be combined.

Documentation

For First Surveys and Renewal Surveys: NCQA reviews a random sample of up to 40 upheld appeal files for evidence that appeal notices meet all 6 factors. Appeal files include appeals of any denial of a request for coverage, whether or not the denial resulted from medical necessity review (e.g., medical, behavioral health, pharmacy or personal care services). This includes all medical necessity and benefit decision appeals.

Look-back period

For First Surveys: 6 months.

For Renewal Surveys: 12 months.

Explanation

THIS IS A MUST-PASS ELEMENT.

This element evaluates handling of appeals according to the policies required by *UM 8: Policies for Appeals*.

Dispute of file review results

NCQA conducts onsite file review in the presence of the organization's staff and works with the organization to resolve any disputes during the onsite survey. An organization that is unable to resolve a dispute with the survey team must contact NCQA before the onsite survey is complete. File review results may not be disputed or appealed once the onsite survey is complete.

Factor 1: The appeal decision

The appeal decision notification states the reason for upholding the denial in terms specific to the member's condition or request and in language that is easy to understand, so the member and practitioner understand why the organization upheld the appeal decision and have enough information to file the next level of appeal.

An appropriately written notification includes a complete explanation of the grounds for the upheld appeal decision, in language that a layperson would understand, and does not include abbreviations, acronyms or health care procedure codes that a layperson would not understand.

The organization is not required to spell out abbreviations/acronyms if they are clearly explained in lay language.

To illustrate, for the acronym DNA, spelling out would be “a DNA (deoxyribonucleic acid)” whereas explaining would be “a DNA test is a test that looks at your genetic information.”

Upheld appeal notifications sent only to practitioners may include technical or clinical terms.

The organization may send a single notice to the member and practitioner that includes the specific reason for upholding the denial, in language that would be easily understood by the member. The notice may also include, in a separate section, additional clinical or technical language directed toward a practitioner.

For appeals resulting from medical necessity review of out-of-network requests, the reason for upheld appeal decision must explicitly address the reason for the request (e.g., if the request is related to accessibility issues, that may be impacted by the clinical urgency of the situation, the appeal decision must address whether or not the requested service can be obtained within the organization’s accessibility standards).

Factor 2: Reference to UM criterion

The appeal notification references the specific criterion used to make the denial decision. The criterion used and referenced is specific to the member’s condition or to the requested services.

The criterion must be identifiable by name and must be specific to an organization or source (e.g., ABC PBM’s Criteria for Treatment of Hypothyroidism with Synthroid or CriteriaCompany Inc.’s Guidelines for Wound Treatment).

If the organization uses a trademarked criterion name, it does not need to cite the organization that holds the trademark. If it is clear that the criterion is attributable to the organization, it is acceptable to state, “our Criteria for XXX” (e.g., our Criteria for Treating High Cholesterol with Lipitor).

Because benefit documents are often large and complex, the organization must direct members to the information using the section title or page number.

For appeals of denials resulting from medical necessity review of out-of-network requests, criteria may be excerpted from benefit documents that govern out-of-network coverage, organization policies specifying circumstances where out-of-network coverage will be approved or clinical criteria used to evaluate the member’s clinical need relative to available network providers and services. The reference must specifically support the rationale for the decision and must relate to the reason for the request.

Factor 3: Availability of criterion

The appeal notification informs the member, and the practitioner acting as the member’s authorized representative, that the criterion used to make the decision is available upon request. Providing the criterion, or an excerpt specific to the denial reason with the appeal decision notification is also acceptable. NCQA scores this factor “Yes” if the criterion or excerpt is included in the decision notice or if the notification states that the criterion or excerpt is included as an attachment.

Factor 4: Access to and copies of documents

No additional explanation required.

Factor 5: Titles and qualifications

The upheld appeal decision notification identifies all reviewers who participated in making the appeal decision, including the same-or-similar specialist reviewer, when applicable, as they provide specific clinical knowledge and experience that affects the decision.

For each individual, the notice includes:

- *For a benefit appeal:* The title (position or role in the organization).
- *For a medical necessity appeal:* The title (position or role in the organization), qualifications (clinical credentials such as MD, DO, PhD, physician) and specialty (e.g., pediatrician, general surgeon, neurologist, clinical psychologist).

The organization is not required to include individuals' names in the written notification.

Factor 6: Additional appeal rights

The notification describes members' additional appeal rights if their appeal is denied.

If the next level of appeal is independent external review, the notification includes a statement that members are not required to bear costs of the IRO, including any filing fees, unless state law mandates that members pay an IRO filing fee. The statement that members are not required to bear costs of the IRO, including filing fees, does not apply to appeals by members in self-funded accounts or to members covered by Medicare, Medicaid or the FEHB Program.

If the organization instructs the member to send an appeal directly to an IRO, including MAXIMUS, this is sufficient to meet factor 6, if it provides information on where to send the appeal and any relevant time frames, if applicable.

Exceptions

Factors 3, 4 and 5 are NA for Medicare Part D appeals.

For factor 4, the "free of charge" component is NA for Medicare appeals.

Related information

Medicare appeals. For Medicare appeal files, factors 1–6 are met if there is evidence that the organization sent the upheld denial to MAXIMUS.

Examples

None.

Element E: Final Internal and External Appeal Files

In an NCQA review of denials overturned by the IRO or of the organization's final internal denials, the files contained the following:

1. Member notification of independent appeal rights.
2. Member notification about obtaining more information regarding independent appeal rights.
3. A statement that members are not required to bear costs of the IRO, including any filing fees.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 2-3 factors	The organization meets 1 factor	The organization meets 0 factors
Data source	Records or files		
Scope of review	Product lines <i>This element applies to commercial and Exchange product lines for First Surveys and Renewal Surveys.</i> Documentation <i>For First Surveys and Renewal Surveys:</i> NCQA reviews five of the most recently overturned appeals by the IRO, or reviews five final internal denials, if no appeals were overturned by the IRO. If there are fewer than five files, NCQA reviews all files. This file review is independent of the appeal file review performed for UM 9, Elements A–D.		
Look-back period	<i>For First Surveys:</i> 6 months. <i>For Renewal Surveys:</i> 24 months.		
Explanation	This element evaluates handling of appeals according to the policies required by <i>UM 8: Policies for Appeals</i>. Dispute of file review results NCQA conducts onsite file review in the presence of the organization's staff and works with the organization to resolve any disputes during the onsite survey. An organization that is unable to resolve a dispute with the survey team must contact NCQA before the onsite survey is complete. File review results may not be disputed or appealed once the onsite survey is complete. Factor 1: Notification of independent appeal rights The denial letter includes a statement about eligibility for external review. Factor 2: Information about independent appeal rights No additional explanation required.		

Factor 3: Cost of the review

If state law mandates that members pay an IRO filing fee, the organization receives credit for this factor if it provides the state's language.

Exceptions

This element is NA:

- If the organization had no final internal denials during the look-back period.
- For appeals by members covered by Medicare, Medicaid or the Federal Employees Health Benefit (FEHB) Program.
- For appeals by members in self-funded accounts.
- If the employer mandates that its employees go through its external appeal process.

Examples None.

Element F: Appeals Overturned by the IRO

In an NCQA review of the organization's files of appeals overturned by the IRO, there is evidence that the organization implemented the IRO's decision in all cases reviewed.

Summary of Changes

- No changes to this element.

Scoring

Met	Partially Met	Not Met
The organization meets the requirement	No scoring option	The organization does not meet the requirement

Data source Records or files

Scope of review**Product lines**

This element applies to commercial and Exchange product lines for First Surveys and Renewal Surveys.

Documentation

For First Surveys and Renewal Surveys: NCQA reviews five of the most recently overturned appeals by the IRO, or reviews five final internal denials, if no appeals were overturned by the IRO. If there are fewer than five files, NCQA reviews all files.

This file review is independent of the appeal file review performed for UM 9, Elements A–D.

Look-back period

For First Surveys: 6 months.

For Renewal Surveys: 24 months.

Explanation**Dispute of file review results**

NCQA conducts onsite file review in the presence of the organization's staff and works with the organization to resolve any disputes during the onsite survey. An organization that is unable to resolve a dispute with the survey team must contact

NCQA before the onsite survey is complete. File review results may not be disputed or appealed once the onsite survey is complete.

Exceptions

This element is NA:

- If the organization has had no final denials overturned by the IRO.
- For appeals by members covered by Medicare, Medicaid or the Federal Employees Health Benefits (FEHB) program.
- For appeals by members in self-funded accounts.

Examples

None.

UM 10: Evaluation of New Technology

The organization evaluates the inclusion of new technology and the new application of existing technology in its benefits plan, including medical and behavioral healthcare procedures, pharmaceuticals and devices.

Intent

The organization has a formal mechanism to evaluate and address new developments in technology and new applications of existing technology for inclusion in its benefits plan to keep pace with changes and to ensure that members have equitable access to safe and effective care.

Element A: Written Process

The organization's written process for evaluating new technology and the new application of existing technology for inclusion in its benefits plan includes an evaluation of the following:

1. Medical procedures.
2. Behavioral healthcare procedures.
3. Pharmaceuticals.
4. Devices.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 3-4 factors	The organization meets 2 factors	The organization meets 0-1 factors
Data source	Documented process		
Scope of review	Product lines <i>This element applies to all product lines for Interim Surveys, First Surveys and Renewal Surveys.</i> Documentation <i>For Interim Surveys, First Surveys and Renewal Surveys: NCQA reviews the organization's policies and procedures in place throughout the look-back period.</i>		
Look-back period	<i>For Interim Surveys: Prior to the survey date.</i> <i>For First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	Factors 1–4 No explanation required. Exceptions Factor 2 is NA if all purchasers of the organization's services carve out or exclude behavioral healthcare services.		

Factor 3 is NA if all purchasers of the organization's services carve out or exclude the pharmacy benefit.

This element is NA:

- For Medicaid product lines if the state mandates all benefits and new technology determinations.
 - The organization provides the state's language.
- If the organization does not determine which technologies, pharmaceuticals, devices, procedures or other services are included in benefits plans it offers to members.
 - For example, when these determinations are made by all purchasers of the organization's services.

Examples None.

Element B: Description of the Evaluation Process

The organization's written evaluation process includes the following:

1. The process and decision variables the organization uses to make determinations.
2. A review of information from appropriate government regulatory bodies.
3. A review of information from published scientific evidence.
4. A process for seeking input from relevant specialists and professionals who have expertise in the technology.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 3-4 factors	The organization meets 2 factors	The organization meets 0-1 factors
Data source	Documented process		
Scope of review	Product lines <i>This element applies to all product lines for Interim Surveys, First Surveys and Renewal Surveys.</i> Documentation <i>For Interim Surveys, First Surveys and Renewal Surveys: NCQA reviews the organization's policies and procedures in place throughout the look-back period.</i>		
Look-back period	<i>For Interim Surveys: Prior to the survey date.</i> <i>For First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		

Explanation **Factor 1: Process and decision variables**

The organization's process includes a committee or other formal body that reviews new technology. Behavioral healthcare professionals are involved in the decision-making process for behavioral healthcare services.

Factor 2: Review of information from appropriate regulatory bodies

The organization reviews information from appropriate government regulatory bodies (e.g., FDA, CMS).

Factor 3: Review of information from scientific evidence

The organization reviews relevant scientific information that may include, but is not limited to:

- Articles in peer-reviewed literature.
- Recommendations from professional societies.
- Summaries from organizations that rely on the judgment of experts when determining the effectiveness of new technology.

Factor 4: Input from specialists

No explanation required.

Exceptions

This element is NA for Medicaid product lines if the state mandates all benefits and new technology determinations. The organization must produce documentation that demonstrates this.

This element is NA if the organization does not determine which technologies, pharmaceuticals, devices, procedures or other services are included in benefits plans it offers to members. For example, when these determinations are made by all purchasers of the organization's services.

Related information

Practitioner input. If the organization's corporate office makes decisions about technology without input from local practitioners, but appropriate practitioners review its technology assessment policy, NCQA does not require the local organization to obtain input from practitioners before implementing the corporate office's decision.

Examples None.

UM 11: Procedures for Pharmaceutical Management

The organization ensures that its procedures for pharmaceutical management, if any, promote the clinically appropriate use of pharmaceuticals.

Intent

The organization develops, regularly reviews and updates policies and procedures for pharmaceutical management based on sound clinical evidence.

Element A: Pharmaceutical Management Procedures

The organization's policies and procedures for pharmaceutical management include the following:

1. The criteria used to adopt pharmaceutical management procedures.
2. A process to use clinical evidence from appropriate external organizations.
3. A process to include pharmacists and appropriate practitioners in the development of procedures.
4. A process to provide procedures to practitioners annually and when it makes changes.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 3-4 factors	The organization meets 2 factors	The organization meets 0-1 factors
Data source	Documented process		
Scope of review	Product lines <i>This element applies to all product lines for Interim Surveys, First Surveys and Renewal Surveys.</i> Documentation <i>For Interim Surveys, First Surveys and Renewal Surveys: NCQA reviews the organization's policies and procedures in place throughout the look-back period.</i>		
Look-back period	<i>For Interim Surveys: Prior to the survey date.</i> <i>For First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	Factor 1: Criteria for adoption Criteria for adopting pharmaceutical procedures are different from the clinical criteria used in medical necessity review and required in UM 2: <i>Clinical Criteria for UM Decisions</i> . In this element, NCQA reviews the organization's decision criteria for developing pharmaceutical management procedures as a whole.		

The organization's policies and procedures specify that the criteria used when adopting the pharmaceutical management procedures, includes:

- Pharmaceutical classes.
- Classes preferred or covered at any level.
- Lists of preferred pharmaceuticals or formularies.
- Considerations for limiting access to drugs in certain classes.
- Prior authorization criteria.
- Generic substitution, therapeutic interchange, step therapy or other management methods to which the practitioner's prescribing decisions are subject.
- Within each class of pharmaceuticals:
 - Pharmaceuticals preferred or covered at any level.
 - An exceptions process available to members.
 - Substitutions made automatically or with permission of the prescribing practitioner.
 - Evidence that preferred-status pharmaceuticals can produce similar or better results for a majority of the population than other pharmaceuticals in the same class.
 - Other requirements, restrictions, limitations or incentives that apply to the use of certain pharmaceuticals.

Factor 2: Use of clinical evidence

The organization's policies and procedures specify that to make pharmaceutical decisions, the organization uses clinical evidence from the following sources, as appropriate:

- Government agencies.
- Medical associations.
- National commissions.
- Peer-reviewed journals.
- Authoritative compendia.

Factor 3: Involvement of pharmacists and appropriate practitioners

When reviewing and making periodic updates to the pharmaceutical management policies and procedures, the organization's pharmaceutical management committee involves:

- Clinical pharmacists.
- Appropriate practitioners.

The committee may be local, regional or national. If the committee is national, involvement of local practitioners is not required.

Factor 4: Distributing pharmaceutical management procedures

The organization's policies and procedures specify that it distributes pharmaceutical management procedures to practitioners by mail, fax, email, or on its website, if it informs practitioners that the information is available on line. The notice must include a description specific enough to give readers a clear idea of the topic and the general content and must include a link or direction to the specific

information. The organization may group or summarize the information by theme. The organization has a process to mail pharmaceutical management procedures to practitioners who do not have fax, email or internet access.

Exception

This element is NA if all purchasers of the organization's services carve out or exclude pharmaceutical management.

Examples None.

Element B: Pharmaceutical Restrictions/Preferences

Annually and after updates, the organization communicates to members and prescribing practitioners:

1. A list of pharmaceuticals, including restrictions and preferences.
2. How to use the pharmaceutical management procedures.
3. An explanation of limits or quotas.
4. How prescribing practitioners must provide information to support an exception request.
5. The organization's process for generic substitution, therapeutic interchange and step-therapy protocols.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 4-5 factors	The organization meets 3 factors	The organization meets 0-2 factors
Data source	Documented process, Reports, Materials		
Scope of review	Product lines <i>This element applies to all product lines for Interim Surveys, First Surveys and Renewal Surveys.</i> Documentation NCQA reviews the organization's pharmaceutical procedures and lists. <i>For First Surveys and Renewal Surveys, NCQA also reviews materials distributed to members and prescribing practitioners. The organization may also provide reports to show evidence of distribution to members and practitioners.</i>		
Look-back period	<i>For Interim Surveys: Prior to the survey date.</i> <i>For First Surveys: At least once during the prior year.</i> <i>For Renewal Surveys: 24 months.</i>		

Explanation This element applies to open and closed formularies.

Pharmaceutical and pharmaceutical management procedures communicated to members and prescribers include information on, as applicable:

- Covered pharmaceuticals.
- Copayment information, including tiers.
- Pharmaceuticals that require prior authorization.
- Limits on refills, doses or prescriptions.
- Use of generic substitution, therapeutic interchange or step-therapy protocols.
- How formulary updates are communicated, and how often, if the organization has scheduled formulary updates.

Distribution of pharmaceutical procedures and updates

The organization may limit communication of updates to “negative” formulary changes (i.e., changes that result in restrictions or replacements) and may limit such communications to affected members and their practitioners.

The organization distributes pharmaceutical management procedures and updates by mail, fax, email, or on its website if it informs members and prescribing practitioners that the information is available online. The notice must include a description specific enough to give readers a clear idea of the topic and the general content and must include a link or direction to the specific information. The organization may group or summarize the information by theme. The organization mails pharmaceutical management procedures and updates to prescribing practitioners and affected members who do not have fax, email or internet access.

Factors 1–3

No additional explanation required.

Factor 4: Exception request

If the organization administers a closed formulary, there is an exception process for circumstances where the formulary does not adequately accommodate members’ clinical needs, and a process for prescribing practitioners to submit information that supports exception requests.

Factor 5: Process for generic substitution, therapeutic interchange and step-therapy protocols

The organization’s procedures regarding generic substitution, therapeutic interchange and step-therapy are outlined and communicated to members and prescribers. The organization is not required to communicate procedures that are not in use within its benefit.

Exceptions

This element is NA if all purchasers of the organization’s services carve out or exclude pharmaceutical management.

Factor 4 is NA for organizations that do not administer a closed formulary for their members.

Examples Pharmaceutical management procedures

The member and practitioner newsletters or handbooks describe changes to generic substitution, therapeutic interchange or step-therapy protocols.

Element C: Pharmaceutical Patient Safety Issues

The organization's pharmaceutical procedures include:

1. Identifying and notifying members and prescribing practitioners affected by a Class II recall or voluntary drug withdrawals from the market for safety reasons within 30 calendar days of the FDA notification.
2. An expedited process for prompt identification and notification of members and prescribing practitioners affected by a Class I recall.

Summary of Changes

- No changes to this element.

Scoring

Met	Partially Met	Not Met
The organization meets 2 factors	The organization meets 1 factor	The organization meets 0 factors

Data source

Documented process, Materials

Scope of review**Product lines**

This element applies to all product lines for Interim Surveys, First Surveys and Renewal Surveys.

Documentation

NCQA reviews the organization's policies and procedures in place throughout the look-back period.

For First Surveys and Renewal Surveys: NCQA also reviews communications to members and prescribing practitioners, as applicable.

Look-back period

For Interim Surveys: Prior to the survey date.

For First Surveys: 6 months.

For Renewal Surveys: 24 months.

Explanation**Factors 1, 2: Class II recalls, Class I recalls and market withdrawal**

- *Class II recall:* Removal of a distributed product where use of or exposure to the product may cause temporary or medically reversible adverse health consequences or where the probability of serious, adverse health consequences is remote.
- *Class I recall:* Removal of a distributed product due to reasonable probability that use of or exposure to the product will cause serious, adverse health consequences or death.
- *Market withdrawal:* Removal or correction of a distributed product which involves a minor violation that would not be subject to legal action by the FDA.

Factor 1: Withdrawals for safety reason or Class II recalls

The organization is not required to communicate to members and practitioners about wholesale-only withdrawals. The organization is only required to notify affected members that it can identify. The organization identifies affected members using batch or lot numbers or NDC codes of the withdrawn or recalled pharmaceuticals.

Exception

This element is NA if all purchasers of the organization's services carve out or exclude pharmaceutical management.

Related information

Information regarding drug recalls and withdrawals. Definitions and safety alerts of Class I, Class II and market withdrawals are posted on the FDA website: <http://www.fda.gov/>.

Examples**Content of notification**

Communication to affected members regarding recalled or withdrawn pharmaceuticals may include:

- The name of the drug/pharmaceutical.
- The date of the withdrawal/recall.
- The reason for the withdrawal/recall.
- Instructions such as:
 - Return the pharmaceutical to the pharmacy.
 - See the prescribing practitioner for directions or a new prescription.

Communication to affected practitioners may include:

- The name of the pharmaceutical.
- The date of the withdrawal/recall.
- Patients affected (if known).

Element D: Reviewing and Updating Procedures

With the participation of physicians and pharmacists, the organization annually:

1. Reviews the procedures.
2. Reviews the list of pharmaceuticals.
3. Updates the procedures as appropriate.
4. Updates the list of pharmaceuticals as appropriate.

Summary of Changes

- No changes to this element.

Scoring

Met	Partially Met	Not Met
The organization meets 3-4 factors	The organization meets 2 factors	The organization meets 0-1 factors

Data source Documented process, Reports, Materials

Scope of review	Product lines This element applies to all product lines for Interim Surveys, First Surveys and Renewal Surveys. Documentation NCQA reviews the organization's policies and procedures. <i>For First Surveys and Renewal Surveys:</i> NCQA also reviews pharmaceutical management committee minutes or similar documentation, and updates to the pharmaceutical procedures, if applicable.
Look-back period	<i>For Interim Surveys:</i> Prior to the survey date. <i>For First Surveys:</i> At least once during the prior year. <i>For Renewal Surveys:</i> 24 months.
Explanation	This element applies to pharmaceutical management procedures described in Elements A–C. The organization may space its review and updates of pharmaceutical management procedures and list of approved pharmaceuticals over the course of the year. Involvement of physicians and pharmacists When reviewing and making periodic updates to the pharmaceutical management policies and procedures, the organization's pharmaceutical management committee involves: • Clinical pharmacists. • Appropriate physicians. The committee may be local, regional or national. If the committee is national, involvement of local physicians or pharmacists is not required. Factors 1–4 No additional explanation required. Exception This element is NA if all purchasers of the organization's services carve out or exclude pharmaceutical management.
Examples	None.

Element E: Considering Exceptions

The organization has exceptions policies and procedures that describe the process for:

1. Making an exception request based on medical necessity.*
2. Obtaining medical necessity information from prescribing practitioners.*
3. Using appropriate pharmacists and practitioners to consider exception requests.
4. Timely handling of exception requests.
5. Communicating the reason for a denial and an explanation of the appeal process when it does not approve an exception request.

**Critical factors: These factors must be scored “yes” to score at least “Partially Met.”*

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 4-5 factors	The organization meets 3 factors	The organization meets 0-2 factors
Data source	Documented process		
Scope of review	Product lines <i>This element applies to all product lines for Interim Surveys, First Surveys and Renewal Surveys.</i> Documentation NCQA reviews the organization’s policies and procedures in place throughout the look-back period.		
Look-back period	<i>For Interim Surveys: Prior to the survey date.</i> <i>For First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	Factors 1 and 2 are critical factors: Both factors must be scored “yes” for the organization to score at least “Partially Met” on this element. Formulary exceptions are requests by members or their authorized representatives to obtain a pharmaceutical that is not included as part of the organization’s closed formulary. Factor 1: Exception for medical necessity The organization’s policies and procedures allow members to request exceptions based on medical necessity if needed pharmaceuticals are not on the list of approved pharmaceuticals. Factor 2: Obtaining information from prescribing practitioners The organization’s policies and procedures detail the clinical information needed from prescribing practitioners in order to determine medical necessity.		

Factor 3: Review by appropriate practitioner

No additional explanation required.

Factor 4: Timely handling of exception requests

The organization's policies and procedures provide time frames for exception requests. Time frames are not required to be the same as those stated in *UM 5: Timeliness of UM Decisions*, but must consider clinical urgency.

Factor 5: Denial notification and appeal rights

The organization's policies and procedures specify that if the organization denies a request for an exception based on medical necessity, internal and external appeal processes are available on the same basis as for denials of other services.

Exceptions

This element is NA if:

- The organization does not administer a closed formulary for its members.
- All purchasers of the organization's services carve out or exclude pharmaceutical management.

Related information

Formulary exception appeals. A denial of a formulary exception request is a coverage decision subject to appeal under the policies and procedures required under UM 8. Upheld appeals of formulary exception denials are included in the file universe in *UM 9: Appropriate Handling of Appeals*.

Existing exception policies. Many organizations and PBMs have exception policies in place; some use online or telephone systems to communicate with pharmacists and consider coverage for the prescribed pharmaceutical based on medical necessity. It is not the intent of this element to interfere with these processes; they meet the requirements of Element E if the organization's formal appeal process is available to members whose exception request was denied.

Examples**Factor 1: Reasons for exception requests**

- Intolerance or allergy to pharmaceuticals on the organization's list of approved drugs.
- Inadequate or inappropriate response to pharmaceuticals on the organization's list of approved drugs.

UM 12: UM System Controls

The organization has UM system controls to protect data from being altered outside of prescribed protocols.

Intent

The organization develops policies and procedures to monitor compliance of system controls specific to UM denial and appeal notification and receipt dates.

Element A: UM Denial System Controls

The organization has policies and procedures describing its system controls specific to UM denial notification dates that:

1. Define the date of receipt consistent with NCQA requirements.
2. Define the date of written notification consistent with NCQA requirements.
3. Describe the process for recording dates in systems.
4. Specify titles or roles of staff who are authorized to modify dates once initially recorded and circumstances when modification is appropriate.
5. Specify how the system tracks modified dates.
6. Describe system security controls in place to protect data from unauthorized modification.
7. Describe how the organization monitors its compliance with the policies and procedures in factors 1–6 at least annually and takes appropriate action, when applicable.

Summary of Changes

Clarifications

- Replaced “for system controls” with “to monitor compliance of system controls” in the standard intent.
- Replaced “Specify staff” with “Specify titles or roles of staff” in factor 4.
- Removed “Materials” as a data source.
- Removed the second paragraph under “Documentation” in the scope of review addressing factor 6.
- Revised the look-back period for First Surveys from “6 months” to “6 months for factors 1–6; prior to the survey date for factor 7.”
- Revised the look-back period for Renewal Surveys from “12 months” to “24 months for factors 1–6; prior to the survey date for factor 7.”
- Clarified the third paragraph in the Explanation that the element applies to UM denial determinations resulting from medical necessity review.
- Added “Factors 1, 2” subhead and text to the Explanation.
- Revised the text under “Factor 4: Authorization to modify dates” to clarify authorization to modify dates.
- Revised the text under “Factor 6: Securing system data” to clarify physical access.
- Added “Factor 7: Annually monitoring UM system controls process” subhead and text to the Examples.

Policy Changes

- Revised factor 7 to read, “How the organization monitors its compliance with the policies and procedures in factors 1-6 at least annually and takes appropriate action when applicable.”
- Replaced “Factor 7: UM process audit” with “Factor 7: Annually monitoring UM system controls process” and revised the text to clarify the monitoring process.

Scoring

Met	Partially Met	Not Met
The organization meets 7 factors	No scoring option	The organization meets 0-6 factors

Data source Documented process

Scope of review**Product lines**

This element applies to all product lines for Interim Surveys, First Surveys and Renewal Surveys.

Documentation

NCQA reviews the organization’s policies and procedures for its UM denial system security controls.

Look-back period

For Interim Surveys: Prior to the survey date.

For First Surveys: 6 months for factors 1–6; prior to the survey date for factor 7.

For Renewal Surveys: 24 months for factors 1–6; prior to the survey date for factor 7.

Explanation**THIS IS A MUST-PASS ELEMENT.**

This element is a **structural requirement**. The organization must present its own documentation.

This element applies to the systems used to manage UM denial determinations resulting from medical necessity review in scope of *UM 5: Timeliness of UM Decisions*.

Factors 1, 2

The organization defines the dates of receipt and written notification for UM denial determinations resulting from medical necessity review consistent with the requirements in *UM 5: Timeliness of UM Decisions*.

Factor 3

No additional explanation.

Factor 4: Authorization to modify dates

The organization’s policies and procedures identify:

- All staff titles or roles authorized to modify dates.
 - Policies and procedures state if no staff are authorized to modify dates under any circumstances.
- The circumstances when modification is appropriate.

Factor 5: Tracking modifications

The organization's policies and procedures describe how the system tracks:

- Date modifications.
- When the date was modified.
- The staff who modified the date.
- Why the date was modified.

Factor 6: Securing system data

This factor applies to all UM system data and it is not limited to the dates specified in factors 1–5. The organization's policies and procedures describe the process for:

- Limiting physical access to the operating environment that houses utilization management data, including, but not limited to, the organization's computer servers, hardware and physical records and files.
 - “Physical access” does not refer to the organization's building or office location.
- Preventing unauthorized access and changes to system data.
- Password-protecting electronic systems, including requirements to:
 - Use strong passwords.
 - Discourage staff from writing down passwords.
 - User IDs and passwords unique to each user.
 - Change passwords when requested by staff or if passwords are compromised.

Note: *If the organization's policies and procedures state that it follows the National Institute of Standards and Technology guidelines, this is acceptable to describe the process for password-protecting electronic systems.*

- Disabling or removing passwords of employees who leave the organization and alerting appropriate staff who oversee computer security.

Factor 7: Annually monitoring the UM system controls process

The policies and procedures describe the organization's process for at least annually:

- Demonstrating that specified policies and procedures for factors 1–6 are followed.
- Analyzing date modifications that do not meet the organization's established policy.

At a minimum, the description includes:

- The method used to monitor compliance with the organization's policies and procedures described in factors 1–6.
 - If the UM system does not allow date modifications under any circumstances, the description includes the functionality of the system that ensures compliance with established policy.
 - If the UM system allows date modifications only under specific circumstances established by policy, the description includes the process for monitoring compliance with established policy.
 - If the organization uses system alerts or flags to identify noncompliance, the description indicates how this process is conducted and monitored.

- If the organization conducts auditing, sampling is not an allowable method.
 - The description specifies the staff roles or department involved in the audit and the audit frequency.
- The staff titles or roles responsible for oversight of the monitoring process.
- The organization's process for taking actions if it identifies date modifications that do not meet its established policy, including:
 - A quarterly monitoring process to assess the effectiveness of its actions on all findings until it demonstrates improvement for one finding over at least three consecutive quarters.
 - The staff roles or department responsible for the actions.
 - The process for documenting and reporting date modifications that do not meet its established policy.

Examples

Factor 7: Annually monitoring the UM system controls process

Frequency and methods of monitoring activities. The organization has a process for monitoring its compliance with factors 1–6 at least annually, but may use a frequency and/or method other than a single annual process to review UM denial date modifications; for example:

- A description of the system functionality that prevents or disallows modifications to UM system data.
- If the UM system allows date changes only under specific circumstances, an annual process for identifying all changes to established policies within the past 12 months and updating the UM system controls accordingly.
- A review of automatic system alerts or flags for date modifications or events in real time and a separate process for annually testing the performance of the system's automatic alerts or flags.
- A monthly, quarterly or semiannual process to audit files identified by a system-generated report or event log of all date modifications that occurred since the previous audit

Element B: UM Denial System Controls Oversight

At least annually, the organization demonstrates that it monitors compliance with its UM denial controls, as described in Element A, factor 7, by:

- 1. Identifying all modifications to receipt and decision notification dates that did not meet the organization's policies and procedures for date modifications.**
- 2. Analyzing all instances of date modifications that did not meet the organization's policies and procedures for date modifications.**
- 3. Acting on all findings and implementing a quarterly monitoring process until it demonstrates improvement for one finding over three consecutive quarters.**

Summary of Changes

- This is a new element.

Scoring	Met	Partially Met	Not Met
	The organization meets 3 factors	No scoring option	The organization meets 0-2 factors
Data source	Reports		
Scope of review	Product lines <i>This element applies to all product lines for First Surveys and Renewal Surveys.</i>		
	Documentation <i>For First and Renewal Surveys, NCQA reviews the organization's analysis report.</i> This element is scored Met if the organization provides evidence, in lieu of monitoring and analysis Reports, of advanced system control capabilities that automatically record dates and prevent changes that do not meet the organization's policies and procedures for date modifications.		
Look-back period	<i>For All Surveys: Prior to the survey date.</i>		
Explanation	This element is a structural requirement. The organization must present its own documentation. This element applies to the systems used to manage UM denial determinations resulting from medical necessity review in scope of <i>UM 5: Timeliness of UM Decisions</i>. Factor 1: Identifying all modifications that did not meet the policies and procedures The organization demonstrates that at least annually, it identifies all modifications to UM denial receipt and notification dates that did not meet the organization's policies and procedures outlined in UM 12, Element A. Factor 2: Analyzing all modifications that did not meet the policies and procedures The organization demonstrates that at least annually, it conducts a qualitative and quantitative analysis of all modifications that did not meet its policies and procedures outlined in UM 12, Element A. Refer to <i>Appendix 5: Glossary</i> for the full definitions of and requirements for quantitative and qualitative analysis. Factor 3: Acting on all findings The organization identifies and documents all actions it has taken, or plans to take, to address all date modifications (factors 1 and 2) that did not meet its policies and procedures, if applicable. One action may be used to address more than one finding, if appropriate. The organization also implements a quarterly monitoring process to assess the effectiveness of its actions on all findings. The organization must continue to monitor until it demonstrates improvement of at least one finding over three consecutive quarters.		

If the organization did not demonstrate improvement of at least one finding during the look-back period, it submits all quarterly monitoring reports demonstrating ongoing monitoring.

If the organization identified findings less than three quarters before the survey submission date, it submits all monitoring information it has available.

Exception

Factor 3 is NA if the organization did not identify any date modifications or if all identified date modifications met the organization's policies and procedures.

Examples None.

Element C: UM Appeal System Controls

The organization has policies and procedures describing its system controls specific to UM appeal dates that:

1. Define the date of receipt consistent with NCQA requirements.
2. Define the date of written notification consistent with NCQA requirements.
3. Describe the process for recording dates in systems.
4. Specify titles or roles of staff who are authorized to modify dates once initially recorded and circumstances when modification is appropriate.
5. Specify how the system tracks modified dates.
6. Describe system security controls in place to protect data from unauthorized modification.
7. Describe how the organization monitors its compliance with the policies and procedures in factors 1–6 at least annually and takes appropriate action, when applicable.

Summary of Changes

Clarifications

- This element was formerly Element B.
- Replaced “for system controls” with “to monitor compliance of system controls” in the standard intent.
- Replaced “Specify staff” with “Specify titles or roles of staff” in factor 4.
- Removed “Materials” as a data source.
- Removed the second paragraph under “Documentation” in the scope of review addressing factor 6.
- Revised the look-back period for First Surveys from “6 months” to “6 months for factors 1–6; prior to the survey date for factor 7.”
- Revised the look-back period for Renewal Surveys from “12 months” to “24 months for factors 1–6; prior to the survey date for factor 7.”
- Clarified the third paragraph in the Explanation that the element applies to “appeal decisions related to coverage or rescission of coverage, whether or not the denial resulted from UM medical necessity decisions review.”
- Added “Factors 1, 2” subhead and text to the Explanation.
- Revised the text under “Factor 4: Authorization to modify dates” to clarify authorization to modify dates.

- Revised the text under “Factor 6: Securing system data” to clarify physical access.
- Added “Factor 7: Annually monitoring UM system controls process” subhead and text to the Examples.

Policy Changes

- Revised factor 7 to read, “How the organization monitors its compliance with the policies and procedures in factors 1-6 at least annually and takes appropriate action when applicable.”
- Replaced “Factor 7: UM process audit” with “Factor 7: Annually monitoring UM system controls process” and revised the text to clarify the monitoring process.

Scoring	Met	Partially Met	Not Met
	The organization meets 7 factors	No scoring option	The organization meets 0-6 factors

Data source Documented process

Scope of review

Product lines

This element applies to all product lines for Interim Surveys, First Surveys and Renewal Surveys.

Documentation

NCQA reviews the organization’s policies and procedures for its UM appeal system security controls.

Look-back period

For Interim Surveys: Prior to the survey date.

For First Surveys: 6 months for factors 1–6; prior to the survey date for factor 7.

For Renewal Surveys: 24 months for factors 1–6; prior to the survey date for factor 7

Explanation

THIS IS A MUST-PASS ELEMENT.

This element is a **structural requirement**. The organization must present its own documentation.

This element applies to the systems used to manage appeal decisions related to coverage or rescission of coverage, whether or not the denial resulted from medical necessity review in scope of *UM 8: Policies for Appeals* and *UM 9: Appropriate Handling of Appeals*.

Factors 1, 2

The organization defines the dates of receipt and written notification for UM denial determinations resulting from medical necessity review consistent with the requirements in *UM 8: Policies for Appeals* and *UM 9: Appropriate Handling of Appeals*.

Factor 3

No additional explanation.

Factor 4: Authorization to modify dates

The organization's policies and procedures identify:

- All staff titles or roles authorized to modify dates.
 - Policies and procedures state if no staff are authorized to modify dates under any circumstances.
- The circumstances when modification is appropriate.

Factor 5: Tracking modifications

The organization's policies and procedures describe how the system tracks:

- Date modifications.
- When the date was modified.
- The staff who modified the date.
- Why the date was modified.

Factor 6: Securing system data

This factor applies to all UM system data and it is not limited to the dates specified in factors 1–5. The organization's policies and procedures describe the process for:

- Limiting physical access to the operating environment that houses utilization management data, including, but not limited to, the organization's computer servers, hardware and physical records and files.
 - "Physical access" does not refer to the organization's building or office location.
- Preventing unauthorized access and changes to system data.
- Password-protecting electronic systems, including requirements to:
 - Use strong passwords.
 - Discourage staff from writing down passwords.
 - User IDs and passwords unique to each user.
 - Change passwords when requested by staff or if passwords are compromised.

Note: *If the organization's policies and procedures state that it follows the National Institute of Standards and Technology guidelines, this is acceptable to describe the process for password-protecting electronic systems.*

- Disabling or removing passwords of employees who leave the organization and alerting appropriate staff who oversee computer security.

Factor 7: Annually monitoring the UM system controls process

The policies and procedures describe the organization's process for at least annually:

- Demonstrating that specified policies and procedures for factors 1–6 are followed.
- Analyzing date modifications that do not meet the organization's established policy.

At a minimum, the description includes:

- The method used to monitor compliance with the organization's policies and procedures described in factors 1–6.
 - If the UM system does not allow date modifications under any circumstances, the description includes the functionality of the system that ensures compliance with established policy.
 - If the UM system allows date modifications only under specific circumstances established by policy, the description includes the process for monitoring compliance with established policy.
 - If the organization uses system alerts or flags to identify noncompliance the description indicates how this process is conducted and monitored.
 - If the organization conducts auditing, sampling is not an allowable method.
 - The description specifies the staff roles or department involved in the audit and the audit frequency.
- The staff titles or roles responsible for oversight of the monitoring process.
- The organization's process for taking actions if it identifies date modifications that do not meet its established policy, including:
 - A quarterly monitoring process to assess the effectiveness of its actions on all findings until it demonstrates improvement for one finding over at least three consecutive quarters.
 - The staff roles or department responsible for the actions.
 - The process for documenting and reporting date modifications that do not meet its established policy.

Examples

Factor 7: Annually monitoring the UM controls process

Frequency and methods of monitoring activities. The organization has a process for monitoring its compliance with factors 1–6 at least annually, but may use a frequency and/or method other than a single annual process to review UM appeal date modifications; for example:

- A description of the system functionality that prevents or disallows modifications to UM system data.
- If the UM system allows date changes only under specific circumstances, an annual process for identifying all changes to established policies within the past 12 months and updating the UM system controls accordingly.
- A review of automatic system alerts or flags for date modifications or events in real-time and a separate process for annually testing the performance of the system's automatic alerts or flags.
- A monthly, quarterly or semiannual process to audit files identified by a system-generated report or event log of all date modifications that occurred since the previous audit.

Element D: UM Appeal System Controls Oversight

At least annually, the organization demonstrates that it monitors compliance with its UM appeal controls, as described in Element C, factor 7, by:

1. Identifying all modifications to receipt and decision notification dates that did not meet the organization's policies and procedures for date modifications.
2. Analyzing all instances of date modifications that did not meet the organization's policies and procedures for date modifications.
3. Acting on all findings and implementing a quarterly monitoring process until it demonstrates improvement for one finding over three consecutive quarters.

Summary of Changes

- This is a new element.

Scoring	Met	Partially Met	Not Met
	The organization meets 3 factors	No scoring option	The organization meets 0-2 factors
Data source	Reports		
Scope of review	Product lines <i>This element applies to all product lines for First Surveys and Renewal Surveys.</i>		
	Documentation <i>For First and Renewal Surveys:</i> NCQA reviews the organization's analysis report. This element is scored Met if the organization provides evidence, in lieu of monitoring and analysis Reports, of advanced system control capabilities that automatically record dates and prevent changes that do not meet the organization's policies and procedures for date modifications.		
Look-back period	<i>For All Surveys:</i> Prior to the survey date.		
Explanation	This element is a structural requirement . The organization must present its own documentation. This element applies to the systems used to manage appeal decisions related to coverage or rescission of coverage, whether or not the denial resulted from medical necessity review in scope of <i>UM 8: Policies for Appeals</i> and <i>UM 9: Appropriate Handling of Appeals</i> . Factor 1: Identifying all modifications that did not meet the policies and procedures. The organization demonstrates that at least annually, it identifies all modifications to UM appeal receipt and notification dates that did not meet the organization's policies and procedures outlined in UM 12, Element C.		

Factor 2: Analyzing all modifications that did not meet the policies and procedures

The organization demonstrates that at least annually, it conducts a qualitative and quantitative analysis of all modifications that did not meet its policies and procedures outlined in UM 12, Element C.

Refer to *Appendix 5: Glossary* for the full definitions of and requirements for quantitative and qualitative analysis.

Factor 3: Acting on findings

The organization identifies and documents all actions it has taken, or plans to take, to address all date modifications (factors 1 and 2) that did not meet its policies and procedures, if applicable. One action may be used to address more than one finding, if appropriate.

The organization also implements a quarterly monitoring process to assess the effectiveness of its actions on all findings.

The organization must continue to monitor until it demonstrates improvement of at least one finding over three consecutive quarters.

If the organization did not demonstrate improvement of at least one finding during the look-back period, it submits all quarterly monitoring reports demonstrating ongoing monitoring.

If the organization identified findings less than three quarters before the survey submission date, it submits all monitoring information it has available.

Exception

Factor 3 is NA if the organization did not identify any date modifications or if all identified date modifications met the organization's policies and procedures.

Examples

None.

UM 13: Delegation of UM

If the organization delegates UM activities, there is evidence of oversight of the delegated activities.

Intent

The organization remains responsible for and has appropriate structures and mechanisms to oversee delegated UM activities.

Element A: Delegation Agreement

The written delegation agreement:

1. Is mutually agreed upon.
2. Describes the delegated activities and the responsibilities of the organization and the delegated entity.
3. Requires at least semiannual reporting by the delegated entity to the organization.
4. Describes the process by which the organization evaluates the delegated entity's performance.
5. Describes the process for providing member experience and clinical performance data to its delegates when requested.
6. Describes the remedies available to the organization if the delegated entity does not fulfill its obligations, including revocation of the delegation agreement.

Summary of Changes

Clarifications

- Added "Documented process" as a data source.
- Added text under "Documentation" in the scope of review to clarify when NCQA reviews the delegate's system controls policies and procedures.
- Revised the first sentence of the fifth paragraph under "Documentation" to clarify that the process from factor 5 must be included in the organization's documentation.
- Revised the look-back period for Renewal Surveys from "24 months" to "24 months for factors 1–6; prior to the survey date for the system controls component under factor 4."
- Added text to Factor 4: "Performance monitoring" to clarify system control requirements.
- Replaced "CAHPS 5.0H" with "CAHPS" in the factor 5 explanation.

Scoring

Met	Partially Met	Not Met
The organization meets 5-6 factors	The organization meets 3-4 factors	The organization meets 0-2 factors

Data source Documented process, Materials

Scope of review **Product lines**

This element applies to all product lines for Interim Surveys, First Surveys and Renewal Surveys.

Documentation

For Interim Surveys, First Surveys and Renewal Surveys: NCQA reviews delegation agreements in effect during the look-back period from up to four randomly selected delegates, or reviews all delegates if the organization has fewer than four.

For factor 4, if UM denial and appeal system security controls are not addressed in the delegation agreement, NCQA reviews the delegate's UM denial and appeal system controls policies and procedures.

Delegation agreements implemented on or after January 1, 2022, must include a description of the delegate's UM system security controls. For the 2022 HPA standards only, for delegation agreements in place prior to January 1, 2022, the organization may provide other mutually agreed upon documentation or the delegate's system controls policies and procedure.

For factor 5, delegation agreements implemented on or after January 1, 2019, must include a description of the process required in the factor. For delegation agreements in place prior to January 1, 2019, the organization may provide documentation that it notified the delegate of the process required in factor 5. This documentation of notification is not required to be mutually agreed upon.

The score for the element is the average of the scores for all delegates.

Look-back period

For Interim Surveys and First Surveys: 6 months.

For Renewal Surveys: 24 months for factors 1–6; prior to the survey date for the system controls component under factor 4.

Explanation

This element may not be delegated.

This element applies to agreements that are in effect within the look-back period. The delegation agreement describes all delegated UM activities. A generic policy statement about the content of delegated arrangements does not meet this element.

Factor 1: Delegation agreement

Delegation activities are mutually agreed on before delegation begins, in a dated, binding document or communication between the organization and the delegated entity.

NCQA considers the effective date specified in the delegation agreement as the mutually agreed-upon effective date. The effective date may be before or after the signature date on the agreement. If the agreement has no effective date, NCQA considers the signature date (the date of the last signature) as the mutually agreed upon effective date.

NCQA may accept other evidence of the mutually agreed-upon effective date: a letter, meeting minutes or other form of communication between the organization and the delegate that references the parties' agreement on the effective date of delegated activities.

NCQA requires submitted evidence for all other delegation factors to consider the same mutually agreed-upon date as the effective date for the delegate's performance of delegated activities.

Factor 2: Assigning responsibilities

The delegation agreement or an addendum thereto or other binding communication between the organization and the delegate specifies the UM activities:

- Performed by the delegate, in detailed language.
- Not delegated, but retained by the organization.
 - The organization may include a general statement in the agreement addressing retained functions (e.g., the organization retains all other UM functions not specified in this agreement as the delegate's responsibility).

If the delegate subdelegates an activity, the delegation agreement must specify that the delegate or the organization is responsible for subdelegate oversight.

Factor 3: Reporting

The organization determines the method of reporting and the content of the Reports, but the agreement must specify:

- That reporting is at least semiannual.
- What information is reported by the delegate about delegated activities.
- How, and to whom, information is reported (i.e., joint meetings or to appropriate committees or individuals in the organization).

The organization must receive regular reports from all delegates, even NCQA-Accredited or NCQA-Certified delegates.

Factor 4: Performance monitoring

The delegation agreement states the organization's process for monitoring and evaluating the delegate's performance.

If the organization contracts with delegates that store, create, modify or use UM denial or appeal data covered by UM 5: *Timeliness of UM Decisions*, UM 8: *Policies for Appeals* or UM 9: *Appropriate Handling of Appeals* on the organization's behalf, the delegation agreement describes:

- The delegate's UM system security controls in place to protect data from unauthorized modification, as outlined in UM 12, Element A (UM Denial System Controls) and Element C (UM Appeal System Controls), factor 6.
- How the delegate monitors its UM denial and appeal system security controls at least annually, as required in UM 13, Element C, factor 5.
- How the organization monitors the delegate's UM denial and appeal system security controls at least annually, as required in UM 13, Element C, factor 5.

Factor 5: Providing member and clinical data

The organization's delegation agreement describes what the delegate must do to obtain the following information when it is needed or on an ongoing basis:

- *Member experience data*: Complaints, CAHPS survey results or other data collected on members' experience with the delegate's services.
- *Clinical performance data*: HEDIS measures, claims and other clinical data collected by the organization. The organization may provide data feeds for relevant claims data or clinical performance measure results.

The organization may provide the delegate with the data upon request or on an ongoing basis.

Factor 6: Consequences for failure to perform

The delegation agreement specifies consequences if a delegate fails to meet the terms of the agreement and, at a minimum, circumstances that would cause revocation of the agreement.

Exception

This element is NA if the organization does not delegate UM activities.

Examples**Factor 3: Reporting**

- Number of UM cases handled by type (preservice, urgent concurrent or postservice) and by service (inpatient or outpatient).
- Number of denials issued.
- Number of denials appealed.

Element B: Predelegation Evaluation

For new delegation agreements initiated in the Look-back period, the organization evaluated delegate capacity to meet NCQA requirements before delegation began.

Summary of Changes

- No changes to this element.

Scoring

Met	Partially Met	Not Met
The organization evaluated delegate capacity before delegation began	The organization evaluated delegate capacity after delegation began	The organization did not evaluate delegate capacity

Data source

Reports

Scope of review**Product lines**

This element applies to all product lines for Interim Surveys, First Surveys and Renewal Surveys.

This element applies if delegation was implemented in the look-back period.

Documentation

NCQA reviews the organization's predelegation evaluation from up to four randomly selected delegates, or reviews all delegates if the organization has fewer than four.

The score for the element is the average of the scores for all delegates.

Look-back period

For Interim Surveys and First Surveys: 6 months.

For Renewal Surveys: 12 months.

Explanation This element may not be delegated.

NCQA-Accredited delegates

Automatic credit is available for this element if all delegates are NCQA-Accredited health plans or MBHOs, or are NCQA Accredited in UM, unless the element is NA.

Predelegation evaluation

The organization evaluated the delegate's capacity to meet NCQA requirements within 12 months prior to implementing delegation. The evaluation may include a review of the organization's structure, processes, and staffing in order to determine its capability to perform the delegated function.

NCQA considers the date of the agreement to be the implementation date if the delegation agreement does not include an implementation date.

If the time between the predelegation evaluation and implementation of delegation exceeds the 12 months, the organization conducts another predelegation evaluation.

If the organization amends the delegation agreement to include additional UM activities within the look-back period, it performs a predelegation evaluation for the additional activities.

Exceptions

This element is NA if:

- The organization does not delegate UM activities.
- Delegation arrangements have been in effect for longer than the look-back period.

Related information

Use of collaborative. The organization may enter into a statewide collaboration to perform any or all of the following:

- Predelegation evaluation.
- Annual evaluation.
- Annual audit of files.

The collaborative must agree on the use of a consistent audit tool and must share data. Each organization is responsible for meeting NCQA delegation standards, but may use the shared data collection process to reduce burden.

Examples

Predelegation evaluation

- Site visit.
- Telephone consultation.
- Documentation review.
- Committee meetings.
- Virtual review.

Element C: Review of the UM Program

For arrangements in effect for 12 months or longer, the organization:

1. Annually reviews its delegate's UM program.
2. Annually audits UM denials and appeals files against NCQA standards for each year that delegation has been in effect.
3. Annually evaluates delegate performance against NCQA standards for delegated activities.
4. Semiannually evaluates regular reports, as specified in Element A.
5. Annually monitors the delegate's UM denial and appeal system security controls to ensure that the delegate monitors its compliance with the delegation agreement or with the delegate's policies and procedures at least annually.
6. Annually acts on all findings from factor 5 for each delegate and implements a quarterly monitoring process until each delegate demonstrates improvement for one finding over three consecutive quarters.

Summary of Changes

Additions

- Added factors 5 and 6 and adjusted the scoring to reflect the new factors.

Clarifications

- Revised the look-back period for Renewal Surveys from "24 months" to "24 months for factors 1–4; at least once during the prior year for factors 5–6."
- Added a second paragraph under "NCQA-Accredited delegates" in the Explanation to clarify when NCQA awards automatic credit to delegates.
- Added "Factor 5: Annual monitoring of UM systems" subhead and text.
- Added "Factor 6: Takes action" subhead and text.

Policy Changes

- Added exceptions for factors 5 and 6.

Scoring

Met	Partially Met	Not Met
The organization meets 5-6 factors	The organization meets 3-4 factors	The organization meets 0-2 factors

Data source

Reports

Scope of review

Product lines

Factor 1 applies to Interim Surveys for all product lines.

All factors in this element apply to First Surveys and Renewal Surveys for all product lines.

Documentation

NCQA reviews evidence of the organization's review from up to four randomly selected delegates, or all delegates if the organization has fewer than four.

For All Surveys: NCQA reviews the organization's evaluation of the delegate's UM program (factor 1).

For First Surveys: NCQA also reviews the organization's most recent annual review, audit, performance evaluation and semiannual evaluation.

For Renewal Surveys: NCQA also reviews the organization's most recent and the previous year's annual reviews, audits, performance evaluations and four semiannual evaluations.

The score for the element is the average of the scores for all delegates.

Look-back period

For Interim Surveys and First Surveys: Once during the prior year.

For Renewal Surveys: 24 months for factors 1–4; at least once during the prior year for factors 5–6.

Explanation

This element may not be delegated.

NCQA-Accredited delegates

Automatic credit is available for factors 2 and 3 if all delegates are NCQA-Accredited health plans or MBHOs, or are NCQA Accredited in UM, unless the element is NA.

For factors 5 and 6, automatic credit is available for delegates that are NCQA Accredited under 2021 (or later) standards for Health Plan Accreditation, MBHO Accreditation or UM-CR-PN Accreditation.

Factor 1: Review of the UM program

The appropriate organization staff or committee review the delegate's UM program. At a minimum, the organization reviews parts of the UM program that apply to the delegated functions.

Factor 2: Annual file audit

If the organization delegates the denial and appeal processes, it audits denial and appeal files against NCQA standards.

The organization uses one of the following to audit the delegate's files:

- 5 percent or 50 of its files, whichever is less, **or**
- The NCQA "8/30 methodology," available at <http://www.ncqa.org/Programs/Accreditation/PolicyUpdatesSupportingDocuments.aspx>

The organization bases its annual audit on the responsibilities described in the delegation agreement and the appropriate NCQA standards.

Factor 3: Annual evaluation

No additional explanation required.

Factor 4: Evaluation of reports

No additional explanation required.

Factor 5: Annual monitoring of UM systems

The organization's process for monitoring system security controls covers delegates that store, create, modify or use UM denial or appeal data covered by *UM 5: Timeliness of UM Decisions*, *UM 8: Policies for Appeals* or *UM 9: Appropriate Handling of Appeals* on its behalf. If the organization contracts with such delegates, it has a process for annually monitoring the delegate's UM denial

and appeal system security controls in place to protect data from unauthorized modification, as outlined in UM 12, Element A (UM Denial System Controls) and Element C (UM Appeal System Controls), factor 6. The organization also ensures that the delegate annually monitors that it follows the delegation agreement or its own policies and procedures.

The organization reviews all modifications made in all delegates' UM denial and appeal systems during the look-back period that did not meet the modification criteria allowed by the delegation agreement or by the delegates' policies and procedures. The organization provides documentation (which may be a report or other type of evidence) that it completed the monitoring process at least annually during the look-back period.

If the delegate's UM system does not allow date modifications, the delegate:

- Describes the functionality of the system that ensures compliance with established policy.
- Provides documentation or evidence of advanced system control capabilities that automatically record dates and prevent modifications that do not meet modification criteria.

Audit. Auditing is allowed only if the organization or delegate does not use a UM system that can identify all noncompliant modifications.

Documentation indicates the staff roles or department involved in the audit.

- The organization or delegate identifies all UM date modifications, but may use sampling to identify potential noncompliant changes for the audit.
- The organization uses one of the following methods to audit files:
- 5 percent or 50 of its files, whichever is less, to ensure that information is verified appropriately.
- The NCQA "8/30 methodology" available at <https://www.ncqa.org/programs/health-plans/policy-accreditation-and-certification/>

Factor 6: Annual actions

The organization identifies and documents all actions it has taken, or plans to take, to address all modifications (findings from factor 5) that did not meet the delegation agreement or the delegate's policies and procedures, if applicable. One action may be used to address more than one finding for each delegate or across multiple delegates, if appropriate.

The organization also implements a quarterly monitoring process for each delegate to assess the effectiveness of its actions on all findings.

- The organization must continue to monitor each delegate until the delegate demonstrates improvement of at least one finding over three consecutive quarters.
- If a delegate did not demonstrate improvement of at least one finding during the look-back period, the organization submits all quarterly monitoring reports demonstrating ongoing monitoring.
- If the organization identified findings less than three quarters before the survey submission date, it submits all monitoring information it has available.

Exceptions

This element is NA if:

- The organization does not delegate UM activities.
- Delegation arrangements have been in effect for less than 12 months.

Factors 2–4 are NA for Interim Surveys.

Factor 5 is NA if the delegate only provides cloud-based UM data storage functions and does not provide services that create, modify or use UM data.

Factor 6 is NA if the organization did not identify any date modifications or if all identified date modifications met the delegation agreement or the delegate's policies and procedures.

Related information

Use of collaborative. The organization may enter into a statewide collaboration to perform any or all of the following:

- Predelegation evaluation.
- Annual evaluation.
- Annual audit of files.

The collaborative must agree on the use of a consistent audit tool and must share data. Each organization is responsible for meeting NCQA delegation standards, but may use the shared data collection process to reduce burden.

Examples None.

Element D: Opportunities for Improvement

For delegation arrangements that have been in effect for more than 12 months, at least once in each of the past 2 years the organization identified and followed up on opportunities for improvement, if applicable.

Summary of Changes

- No changes to this element.

Scoring

Met	Partially Met	Not Met
The organization has acted on identified problems, if any, at least once in each of the past 2 years that the delegation arrangement has been in effect.	The organization took inappropriate or weak action, or acted only in the past year	The organization has not acted on identified problems

Data source Documented process, Reports, Materials

Scope of review **Product lines**

This element applies to all product lines for First Surveys and Renewal Surveys.

Documentation

For First Surveys and Renewal Surveys: NCQA reviews reports for opportunities for improvement from up to four randomly selected delegates, or from all delegates, if the organization has fewer than four, and for evidence that the organization took appropriate action to resolve issues.

For First Surveys: NCQA reviews the organization's most recent annual review and follow-up on improvement opportunities.

For Renewal Surveys: NCQA reviews the organization's most recent and previous year's annual reviews and follow-up on improvement opportunities.

The score for the element is the average of the scores for all delegates.

Look-back period

For First Surveys: At least once during the prior year.

For Renewal Surveys: 24 months.

Explanation

This element may not be delegated.

NCQA-Accredited delegates

Automatic credit is available for this element if all delegates are NCQA Accredited health plans or MBHOs, or are NCQA Accredited in UM, unless the element is NA.

Identify and follow-up on opportunities

The organization uses information from its predelegation evaluation, ongoing reports or annual evaluation to identify areas of improvement.

Exceptions

This element is NA if:

- The organization does not delegate UM activities.
- Delegation arrangements have been in effect for less than 12 months.
- The organization has no opportunities to improve performance.
 - NCQA evaluates whether this conclusion is reasonable, given assessment results.

Examples

None.

Credentialing and Recredentialing

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CR 1: Credentialing Policies

Refer to Appendix 1 for points

The organization has a well-defined credentialing and recredentialing process for evaluating and selecting licensed independent practitioners to provide care to its members.

Intent

The organization has a rigorous process to select and evaluate practitioners.

Element A: Practitioner Credentialing Guidelines

The organization specifies:

1. The types of practitioners it credentials and recredentials.
2. The verification sources it uses.
3. The criteria for credentialing and recredentialing.
4. The process for making credentialing and recredentialing decisions.
5. The process for managing credentialing files that meet the organization's established criteria.
6. The process for requiring that credentialing and recredentialing are conducted in a nondiscriminatory manner.
7. The process for notifying practitioners if information obtained during the organization's credentialing process varies substantially from the information they provided to the organization.
8. The process for notifying practitioners of the credentialing and recredentialing decision within 60 calendar days of the credentialing committee's decision.
9. The medical director or other designated physician's direct responsibility and participation in the credentialing program.
10. The process for securing the confidentiality of all information obtained in the credentialing process, except as otherwise provided by law.
11. The process for confirming that listings in practitioner directories and other materials for members are consistent with credentialing data, including education, training, board certification and specialty.

Summary of Changes

Clarifications

- Moved text regarding dated information from under "Automated credentialing" to "Related information" under the subhead "Verification from a report."
- Added a subbullet under "Use of web crawlers" to clarify "web crawler."
- Revised the fifth bullet under "Practitioners who do not need to be credentialed" to clarify that "treatment setting" does not refer to how care is provided.

Scoring

Met	Partially Met	Not Met
The organization meets 8-11 factors	The organization meets 5-7 factors	The organization meets 0-4 factors

Data source Documented process

Scope of review **Product lines**

For Interim Surveys and First Surveys, this element applies to all product lines.

For Renewal Surveys, this element applies to the Medicaid product line only.

Documentation

NCQA reviews the organization's policies and procedures in effect throughout the look-back period.

Look-back period *For Interim Surveys:* Prior to the survey date.

For First Surveys: 6 months.

For Renewal Surveys: 24 months.

Explanation This element is a **structural requirement**. The organization must present its own documentation.

Practitioners within the scope of credentialing

Practitioners are within the scope of credentialing if all criteria listed below are met:

- Practitioners are licensed, certified or registered by the state to practice independently (without direction or supervision).
- Practitioners have an independent relationship with the organization.
 - An independent relationship exists when the organization directs its members to see a specific practitioner or group of practitioners, including all practitioners whom member can select as primary care practitioners.
- Practitioners provide care to members under the organization's medical benefits.

The listed criteria apply to practitioners in the following settings:

- Individual or group practices.
- Facilities.
- Rental networks:
 - That are part of the organization's primary network and the organization has members who reside in the rental network area.
 - Specifically for out-of-area care and members may see only those practitioners or are given an incentive to see rental network practitioners.
- Telemedicine.

Factor 1: Types of practitioners

Credentialing policies and procedures include the following types of practitioners.

- *Medical practitioners:*
 - Medical doctors.
 - Oral surgeons.
 - Chiropractors.
 - Osteopaths.
 - Podiatrists.
 - Nurse practitioners.

- Other medical practitioners who may be within the scope of credentialing.
 - NCQA does not include these practitioners in the credentialing file review.
- *Behavioral healthcare practitioners:*
 - Psychiatrists and other physicians.
 - Addiction medicine specialists.
 - Doctoral or master's-level psychologists.
 - Master's-level clinical social workers.
 - Master's-level clinical nurse specialists or psychiatric nurse practitioners.
 - Other behavioral healthcare specialists who may be within the scope of credentialing.

Factor 2: Verification sources

Credentialing policies and procedures describe the sources the organization uses to verify credentialing information. The organization uses any of the following sources to verify credentials:

- The primary source (or its website).
- A contracted agent of the primary source (or its website).
 - The organization obtains documentation indicating a contractual relationship between the primary source and the agent that entitles the agent to verify credentials on behalf of the primary source.
- An NCQA-accepted source listed for the credential (or its website).

Factors 3, 4: Decision-making criteria and process

The organization:

- Credentials practitioners before they provide care to members.
- Has a process for making credentialing decisions, and defines the criteria it requires to reach a credentialing decision.
 - Criteria are designed to assess a practitioner's ability to deliver care.
- Determines which practitioners may participate in its network.

Factor 5: Managing files that meet the criteria

Credentialing policies and procedures describe the process used to determine and approve files that meet criteria (i.e., clean files). The organization may present all practitioner files to the Credentialing Committee or may designate approval authority of clean files to the medical director or to an equally qualified practitioner.

Factor 6: Nondiscriminatory credentialing and recredentialing

Credentialing policies and procedures:

- State that the organization does not base credentialing decisions on an applicant's race, ethnic/national identity, gender, age, sexual orientation or patient type (e.g., Medicaid) in which the practitioner specializes.
- Specify the process for preventing discriminatory practices.
 - Preventing involves taking proactive steps to protect against discrimination occurring in the credentialing and recredentialing processes.
- Specify how the organization monitors the credentialing and recredentialing processes for discriminatory practices, at least annually.
 - Monitoring involves tracking and identifying discrimination in credentialing and recredentialing processes.

Factor 7: Discrepancies in credentialing information

Credentialing policies and procedures describe the organization's process for notifying practitioners when credentialing information obtained from other sources varies substantially from that provided by the practitioner.

Factor 8: Notification of decisions

Credentialing policies and procedures specify that the organization's time frame for notifying applicants of initial credentialing decisions and recredentialing denials does not exceed 60 calendar days from the Credentialing Committee's decision. The organization is not required to notify practitioners regarding recredentialing approvals.

Factor 9: Participation of a medical director or designated physician

Credentialing policies and procedures describe the medical director or other designated physician's overall responsibility and participation in the credentialing process.

Factor 10: Ensuring confidentiality

Credentialing policies and procedures describe the organization's process for ensuring confidentiality of the information collected during the credentialing process and the procedures it uses to keep this information confidential.

Factor 11: Practitioner directories and member materials

Credentialing policies and procedures describe the organization's process for ensuring that information provided in member materials and practitioner directories is consistent with the information obtained during the credentialing process.

Exception

This element is NA for Renewal Surveys for the commercial, Medicare and Exchange product lines.

Related information

Appropriate documentation. Credentialing policies and procedures define the organization's process for documenting information and activities in credentialing files. The organization documents verification in the credentialing files using any of the following methods or a combination:

- Credentialing documents signed (or initialed) and dated by the verifier.
- A checklist that includes for each verification:
 - The source used.
 - The date of verification.
 - The signature or initials of the person who verified the information.
 - Typed initials are only acceptable if there is a unique electronic signature or identifier on the checklist.
 - The report date, if applicable.
- A checklist with a single signature and a date for all verifications that has a statement confirming the signatory verified all of the credentials on that date and that includes for each verification:
 - The source used.
 - The report date, if applicable.

Verification from a report. NCQA uses the date generated by the source when the information is retrieved. If the source report does not generate a date, NCQA uses the date noted in the credentialing file by the organization staff who verified the credentials. Staff who verified the credentials must sign or initial the verification.

Automated credentialing system. The organization may use an electronic signature or unique electronic identifier of staff to document verification if it can demonstrate that the electronic signature or unique identifier can only be entered by the signatory. The organization provides its security and login policies and procedures to confirm the unique identifier and the signature can only be entered by the signatory. The system must identify the individual verifying the information, the date of verification, the source and the report date, if applicable.

- Faxed, digital, electronic, scanned or photocopied signatures are acceptable. Signature stamps are not acceptable.
- If the checklist does not include checklist requirements listed above, appropriate credentialing information must be included.

Use of web crawlers. The organization may use web crawlers to verify credentialing information from approved sources. A “web crawler” is software that retrieves information directly from a website; in this case, the state licensing or certification agency (i.e., the primary source). The organization provides documentation that the web crawler collects information only from approved sources, and documents that staff reviewed the credentialing information.

Provisional credentialing. If the organization decides to provisionally credential practitioners, it:

- Has a process for one-time provisional credentialing of practitioners applying to its network for the first time.
- Verifies the following within the required time limits:
 - A current, valid license to practice (CR 3: *Credentialing Verification*, Element A, factor 1).
 - The past five years of malpractice claims or settlements from the malpractice carrier, or the results of the National Practitioner Data Bank (NPDB) query (CR 3, Element A, factor 6).
 - A current and signed application with attestation (CR 3, Element C, factors 1–6).
- Follows the same process for presenting provisional credentialing files to the Credentialing Committee or medical director as it does for its regular credentialing process.
- Does not perform provisional credentialing for practitioners who were credentialed by a delegate on behalf of the organization.
- Does not hold practitioners in provisional status for longer than 60 calendar days.
- Does not list provisionally credentialed practitioners in the directory.
- Does not allow practitioners to deliver care prior to completion of provisional credentialing.

Practitioners who do not need to be credentialed.

- Practitioners who practice exclusively in an inpatient setting and provide care for organization members only because members are directed to the hospital or another inpatient setting.
- Practitioners who practice exclusively in free-standing facilities and provide care for organization members only because members are directed to the facility.
- Pharmacists who work for a pharmacy benefits management (PBM) organization to which the organization delegates utilization management (UM) functions.
- Covering practitioners (e.g., locum tenens).
 - Locum tenens who do not have an independent relationship with the organization are outside NCQA's scope of credentialing.
- Practitioners who do not provide care for members (e.g., board-certified consultants who may provide a professional opinion to the treating practitioner).
- Rental network practitioners who provide out-of-area care only, and members are not required or given an incentive to seek care from them.

Practitioner termination and reinstatement. The organization:

- Initially credentials a practitioner again if the break in network participation is more than 30 calendar days.
- Re-verifies credentials that are no longer within verification time limits.
- Re-verifies credentials that will not be in effect when the Credentialing Committee or medical director makes the credentialing decision.

Examples**Factor 6: Nondiscriminatory credentialing and recredentialing**

The organization monitors credentialing decisions to prevent discrimination. Monitoring includes, but is not limited to:

- Maintaining a heterogeneous credentialing committee membership and the requirement for those responsible for credentialing decisions to sign a statement affirming that they do not discriminate.
- Periodic audits of credentialing files (in-process, denied and approved files) that suggest potential discriminatory practice in selecting practitioners.
- Annual audits of practitioner complaints for evidence of alleged discrimination.

Automated credentialing systems

- Adobe Sign.
- DocuSign.

Element B: Practitioner Rights

The organization notifies practitioners about their right to:

1. Review information submitted to support their credentialing application.
2. Correct erroneous information.
3. Receive the status of their credentialing or recredentialing application, upon request.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 2-3 factors	No scoring option	The organization meets 0-1 factors
Data source	Documented process, Materials		
Scope of review	Product lines <i>For Interim Surveys and First Surveys, this element applies to all product lines.</i> <i>For Renewal Surveys, this element applies to the Medicaid product line only.</i> Documentation NCQA reviews the organization's policies and procedures for all three factors. <i>For First Surveys and Renewal Surveys:</i> NCQA also reviews the information sent to affected practitioners throughout the look-back period.		
Look-back period	<i>For Interim Surveys:</i> Prior to the survey date. <i>For First Surveys:</i> 6 months. <i>For Renewal Surveys:</i> 24 months.		
Explanation	This element is a structural requirement. The organization must present its own documentation. Factor 1: Review information The organization notifies practitioners of their right to review information obtained from outside sources (e.g., malpractice insurance carriers, state licensing boards) to support their credentialing application. The organization is not required to make available: • References. • Recommendations. • Peer-review protected information. Factor 2: Correct erroneous information from other sources The organization notifies practitioners of their right to correct erroneous information and: • The time frame for making corrections. • The format for submitting corrections. • Where to submit corrections.		

The organization is not required to reveal the source of information that was not obtained to meet verification requirements or if federal or state law prohibits disclosure.

The organization documents receipt of corrected information in the practitioner's credentialing file.

Factor 3: Application status

The organization notifies practitioners of:

- Their right to be informed of the status of their application, upon request.
- The information it is allowed to share with practitioners.
- Its process for responding to requests for application status.

Exception

This element is NA for Renewal Surveys for the commercial, Medicare and Exchange product lines.

Examples**Avenues for notification**

- Application.
- Contract.
- Provider manual.
- Other information distributed to practitioners.
- Website.
- Letter to practitioners.

Factor 2: Areas where variation from information provided may occur

- Actions on a license.
- Malpractice claims history.
- Board certification.

Element C: Credentialing System Controls

The organization's credentialing process describes:

1. How primary source verification information is received, dated and stored.
2. How modified information is tracked and dated from its initial verification.
3. Titles or roles of staff who are authorized to review, modify and delete information, and circumstances when modification or deletion is appropriate.
4. The security controls in place to protect the information from unauthorized modification.
5. How the organization monitors its compliance with the policies and procedures in factors 1–4 at least annually and takes appropriate action when applicable.

Summary of Changes**Clarifications**

- Replaced "Staff" with "Titles or roles of staff" in factor 3.
- Revised factor 5 to read, "How the organization monitors its compliance with the policies and procedures in factors 1–4 at least annually and takes appropriate action when applicable."

- Removed “materials” as a data source.
- Removed the second paragraph under “Documentation” in the scope of review addressing factor 4.
- Revised the look-back period for First Surveys from “6 months” to “6 months for factors 1–4; prior to the survey date for factor 5.”
- Revised the look-back period for Renewal Surveys from “12 months” to “24 months for factors 1–4; prior to the survey date for factor 5.”
- Revised the text under “Factor 3: Authorization to modify dates” to clarify authorization to modify dates.
- Revised the text under “Factor 4: Securing information” to clarify physical access.
- Replaced “Factor 5: Credentialing process audit” with “Factor 5: Annually monitoring the credentialing process” and revised the text to clarify the monitoring process.
- Added “Factor 5: Annually monitoring the credentialing process” subhead and text to the Examples.

Scoring	Met	Partially Met	Not Met
	The organization meets 5 factors	No scoring option	The organization meets 0-4 factors
Data source	Documented process		
Scope of review	Product lines <i>This element applies to Interim Surveys, First Surveys and Renewal Surveys for all product lines.</i> Documentation NCQA reviews the organization’s policies and procedures for its CR system security controls.		
Look-back period	<i>For Interim Surveys:</i> Prior to the survey date. <i>For First Surveys:</i> 6 months for factors 1–4; prior to the survey date for factor 5. <i>For Renewal Surveys:</i> 24 months for factors 1–4; prior to the survey date for factor 5.		
Explanation	THIS IS A MUST-PASS ELEMENT. This element is a structural requirement. The organization must present its own documentation. This element applies to both paper and electronic credentialing processes. Factor 1: Primary source verification information The organization’s policies and procedures describe how credentialing information is received, stored, reviewed, tracked and dated. Factor 2: Tracking modifications The organization’s policies and procedures describe how it tracks modifications made to credentialing information: • When the information was modified. • How the information was modified.		

- Staff who made the modification.
- Why the information was modified.

Factor 3: Authorization to modify information

The organization's policies and procedures identify:

- All staff titles or roles authorized to access, modify and delete information.
 - Policies and procedures state if no staff are authorized to modify dates under any circumstances.
- The circumstances when modification or deletion of information is appropriate.

Factor 4: Securing information

The organization's policies and procedures describe the process for:

- Limiting physical access to the operating environment that houses credentialing information, to protect the accuracy of information gathered from primary sources and NCQA-approved sources.
 - Physical access may include, but is not limited to, the organization's computer servers, hardware and physical records and files.
 - "Physical access" does not refer to the organization's building or office location.
- Preventing unauthorized access, changes to and release of credentialing information.
- Password-protecting electronic systems, including user requirements to:
 - Use strong passwords.
 - Discourage staff from writing down passwords.
 - Use IDs and passwords unique to each user.
 - Change passwords when requested by staff or if passwords are compromised.

Note: *If the organization's policies and procedures state that it follows the National Institute of Standards and Technology guidelines, this is acceptable to describe the process for password-protecting electronic systems.*

- Disabling or removing passwords of employees who leave the organization and alerting appropriate staff who oversee computer security.

Factor 5: Annually monitoring the credentialing process

The policies and procedures describe the organization's process for at least annually:

- Demonstrating that specified policies and procedures for factors 1–4 are followed.
- Analyzing modifications that do not meet the organization's established policy.

At a minimum, the description includes:

- The method used to monitor compliance with the organization's policies and procedures described in factors 1–4.
 - If the CR system does not allow modifications under any circumstances, the description includes the functionality of the system that ensures compliance with established policy.
 - If the CR system allows modifications only under specific circumstances established by policy, the description includes the process for monitoring compliance with established policy.

- If the organization uses system alerts or flags to identify noncompliance, the description indicates how this process is conducted and monitored.
- If the organization conducts auditing, sampling is not an allowable method.
 - The description specifies the staff roles or department involved in the audit and the audit frequency.
- The staff titles or roles responsible for oversight of the monitoring process.
- The organization's process for taking actions if it identifies modifications that do not meet its established policy, including:
 - A quarterly monitoring process to assess the effectiveness of its actions on all findings until it demonstrates improvement for one finding over at least three consecutive quarters.
 - The staff roles or department responsible for the actions.
 - The process for documenting and reporting modifications that do not meet established policy.

Exceptions

None.

Examples

Factor 5: Annually monitoring the credentialing process

Frequency and methods of monitoring activities. The organization has a process for monitoring its compliance with factors 1–4 at least annually, but may use a frequency and/or method other than a single annual process to review credentialing modifications; for example:

- A description of the system functionality that prevents or disallows modifications to credentialing information.
- If the CR system allows modifications only under specific circumstances, an annual process for identifying all changes to established policies within the past 12 months and updating the credentialing system controls accordingly.
- A review of automatic system alerts or flags for modifications or events in real time and a separate process for annually testing the performance of the system's automatic alerts or flags.
- A monthly, quarterly or semiannual audit of all modifications that occurred since the previous audit.

Element D: Credentialing System Controls Oversight

At least annually, the organization demonstrates that it monitors compliance with its CR controls, as described in Element C, factor 5, by:

1. Identifying all modifications to credentialing and recredentialing information that did not meet the organization's policies and procedures for modifications.
2. Analyzing all instances of modifications that did not meet the organization's policies and procedures for modifications.
3. Acting on all findings and implementing a quarterly monitoring process until it demonstrates improvement for one finding over three consecutive quarters.

Summary of Changes**Additions**

- This is a new element.

Scoring	Met	Partially Met	Not Met
	The organization meets 3 factors	No scoring option	The organization meets 0-2 factors

Data source Reports

Scope of review **Product lines**
This element applies to all product lines for First Surveys and Renewal Surveys.

Documentation

For First and Renewal Surveys: NCQA reviews the organization's analysis report.

This element is scored Met if the organization provides evidence, in lieu of monitoring and analysis reports, of advanced system control capabilities that automatically record dates and prevent changes that do not meet the organization's policies and procedures for modifications.

Look-back period *For All Surveys:* Prior to the survey date.

Explanation This element is a **structural requirement**. The organization must present its own documentation.

This element applies to both paper and electronic credentialing processes.

Factor 1: Identifying all modifications that did not meet the policies and procedures

The organization demonstrates that at least annually, it identifies all modifications to credentialing and recredentialing information that did not meet the organization's policies and procedures outlined in CR 1, Element C.

Factor 2: Analyzing all modifications that did not meet the policies and procedures

The organization demonstrates that at least annually, it conducts a qualitative and quantitative analysis of all modifications that did not meet its policies and procedures outlined in CR 1, Element C.

Refer to *Appendix 5: Glossary* for the full definitions of and requirements for quantitative analysis and qualitative analysis.

Factor 3: Acting on all findings

The organization identifies and documents all actions it has taken, or plans to take, to address all modifications (factors 1 and 2) that did not meet its policies and procedures, if applicable. One action may be used to address more than one finding, if appropriate.

The organization also implements a quarterly monitoring process to assess the effectiveness of its actions on all findings.

- The organization must continue to monitor until it demonstrates improvement of at least one finding over three consecutive quarters.
- If the organization did not demonstrate improvement of at least one finding during the look-back period, it submits all quarterly monitoring reports demonstrating ongoing monitoring.
- If the organization identified findings less than three quarters before the survey submission date, it submits all monitoring information it has available.

Exception

Factor 3 is NA if the organization did not identify any modifications or if all identified modifications met the organization's policies and procedures.

Examples

None.

CR 2: Credentialing Committee

The organization designates a Credentialing Committee that uses a peer-review process to make recommendations regarding credentialing decisions.

Intent

The organization obtains meaningful advice and expertise from participating practitioners when it makes credentialing decisions.

Element A: Credentialing Committee

The organization's Credentialing Committee:

1. Uses participating practitioners to provide advice and expertise for credentialing decisions.
2. Reviews credentials for practitioners who do not meet established thresholds.
3. Ensures that files that meet established criteria are reviewed and approved by a medical director, designated physician or Credentialing Committee.

Summary of Changes

Clarifications

- Added "or Credentialing Committee" to factor 3.

Scoring

Met	Partially Met	Not Met
The organization meets 2-3 factors	No scoring option	The organization meets 0-1 factors

Data source

Documented process, Reports

Scope of review

Product lines

This element applies to Interim Surveys, First Surveys and Renewal Surveys for all product lines.

Documentation

For Interim Surveys: NCQA reviews Credentialing Committee minutes from three different meetings, or reviews the Credentialing Committee charter, and reviews a timeline for operationalizing the committee, if the committee has not met. If the required meeting minutes are not available for review, NCQA reviews the meeting minutes that are available within the look-back period.

For First Surveys and Renewal Surveys: NCQA reviews Credentialing Committee meeting minutes from three different meetings within the look-back period. If the required meeting minutes are not available for review, NCQA reviews the meeting minutes that are available within the look-back period.

Look-back period

For Interim Surveys: Prior to the survey date.

For First Surveys: 6 months.

For Renewal Surveys: 24 months.

Explanation **Factor 1: Participating practitioners**

The Credentialing Committee is a peer-review body with members from the types of practitioners participating in the organization's network.

The organization may have separate review bodies for each practitioner type (e.g., physician, oral surgeon, psychologist), specialty or multidisciplinary committee, with representation from various specialties.

If the organization is part of a regional or national organization, a regional or national Credentialing Committee that meets the criterion may serve as the peer review committee for the local organization.

Note: *Participating practitioners are external to the organization and are part of the organization's network.*

Factor 2: Committee review

The Credentialing Committee:

- Reviews the credentials of practitioners who do not meet the organization's criteria for participation in the network.
- Gives thoughtful consideration to credentialing information.
- Documents discussions about credentialing in meeting minutes.

Meetings and decisions may take place in real-time, virtual meetings (i.e., through video conference or web conference with audio), but may not be conducted only through email.

Factor 3: Review of files that meet established thresholds

For files that meet the organization's credentialing criteria, the organization:

- Submits all practitioner files to the Credentialing Committee for review, **or**
- Has a process for medical director or qualified physician review and approve clean files.
 - Evidence of review and approval is a handwritten signature, handwritten initials or unique electronic identifier, if the organization has appropriate controls for ensuring that only the designated medical director or qualified physician can enter the electronic signature.
 - An individual signature is not required in each practitioner file if there is one report with a signature that lists all required credentials for all practitioners with clean files.

NCQA scores this factor "yes" if the organization presents all files (including clean files) to the Credentialing Committee.

Exceptions

None.

Related information

Assessment of timeliness. NCQA considers a practitioner to be credentialed as of the Credentialing Committee or medical director decision date and uses this date to assess timeliness in the file review elements, even if a review board or governing body reviews decisions made by the Credentialing Committee or medical director.

Providing care to members. The organization does not permit uncredentialed practitioners to provide care to members.

Some states require reimbursement of practitioners back to the date the application, for members seen during the credentialing process period, if the organization subsequently decides to credential the practitioner. Such retropayment is outside the scope of NCQA's credentialing requirement.

Examples None.

CR 3: Credentialing Verification

The organization verifies credentialing information through primary sources, unless otherwise indicated.

Intent

The organization conducts timely verification of information to ensure that practitioners have the legal authority and relevant training and experience to provide quality care.

Element A: Verification of Credentials

The organization verifies that the following are within the prescribed time limits:

1. A current and valid license to practice.
2. A valid DEA or CDS certificate, if applicable.
3. Education and training as specified in the explanation.
4. Board certification status, if applicable.
5. Work history.
6. A history of professional liability claims that resulted in settlement or judgment paid on behalf of the practitioner.

Summary of Changes

Clarifications

- Replaced “2021 standards” with “2022 standards” in the third paragraph under the look-back period.
- Clarified in the Explanation that all credentials must be current when presented to the Credentialing Committee.
- Removed NTIS as an appropriate verification source under “Factor 2: DEA or CDS certificates.”
- Added NBPAS as an example of a board that is not a member of ABMS/AOA under “Factor 3: Education and training.”
- Added the Accreditation Council for Graduate Medical Education—International to the list of acceptable verification sources under “Factor 3: Education and training.”

Scoring

Met	Partially Met	Not Met
High (90-100%) on file review for at least 4 factors and medium (60-89%) on file review for any remaining factors	High (90-100%) or medium (60-89%) on file review for 6 factors	Low (0-59%) on file review for any factor

Data source Records or files

Scope of review **Product lines**

This element applies to First Surveys and Renewal Surveys for all product lines.

Documentation

NCQA reviews verification of credentials within a random sample of up to 40 initial credentialing files and up to 40 recredentialing files for practitioners that were due for recredentialing during the look-back period.

Look-back period

For First Surveys: 6 months.

For Renewal Surveys: 36 months.

For All Surveys: For credentialing files where verification of DEA or CDS is before June 1, 2020, and a practitioner who is DEA- or CDS- eligible does not have a DEA or CDS certificate, NCQA accepts either the verification process required in the 2022 standards or the applicable prior year's standards, which state, "If a qualified practitioner does not have a valid DEA or CDS certificate, the organization notes this in the credentialing file and arranges for another practitioner to fill prescriptions."

Explanation**THIS IS A MUST-PASS ELEMENT.**

This element applies to:

- Practitioners in the scope of credentialing as defined in CR 1, Element A.
- Initial credentialing and recredentialing files, unless an exception noted below applies.

For factors with verification time limits, NCQA counts back from the decision date to the verification date to assess timeliness of verification.

All credentials must be current at the time of the credentialing committee decision.

Appropriate documentation

Each file contains evidence of verification from a listed source and review by organization staff. Refer to CR 1, Element A, *Related information*, "Appropriate documentation."

Dispute of file review results

NCQA conducts onsite file review in the presence of the organization's staff and works with the organization to resolve any disputes during the onsite survey. An organization that is unable to resolve a dispute with the survey team must contact NCQA before the onsite survey is complete. File review results may not be disputed or appealed once the onsite survey is complete.

Factor 1: Licensure

Verification time limit: 180 calendar days.

The organization verifies that the practitioner has a valid and current license to practice at the time of the credentialing decision. The organization verifies license in all states where the practitioner provides care to members. The organization must verify license directly from state licensing or certification agency (or its website).

Factor 2: DEA or CDS certificates

Verification time limit: Prior to the credentialing decision.

This factor applies to practitioners who are qualified to write prescriptions. The organization verifies that the practitioner's Drug Enforcement Administration (DEA) or Controlled Dangerous Substances (CDS) certificate is valid and current in each

state where the practitioner provides care to members. Acceptable verification sources:

- DEA or CDS agency.
- DEA or CDS certificate, or a photocopy of the certificate.
- Documented visual inspection of the original DEA or CDS certificate.
- Confirmation from the American Medical Association (AMA) Physician Masterfile (DEA only).
- American Osteopathic Association Official Osteopathic Physician Profile Report or Physician Master File (DEA only).
- Confirmation from the state pharmaceutical licensing agency, where applicable.

Pending DEA certificates. The organization may credential a practitioner whose DEA certificate is pending if it has a documented process for allowing a practitioner with a valid DEA certificate to write all prescriptions requiring a DEA number for the prescribing practitioner whose DEA is pending until the practitioner has a valid DEA certificate.

DEA- and CDS- eligible practitioners who do not have a certificate. The organization verifies that all DEA- and CDS-eligible practitioners who do not have a valid DEA/CDS certificate, and for whom prescribing controlled substance is in the scope of their practice, have in place a designated practitioner to write prescriptions on their behalf. The organization documents the practitioner's lack of DEA/CDS certificate in the credentialing file and obtains the name of a designated alternate prescriber from the practitioner. If the alternate prescriber is a practice rather than an individual, the file may include the practice name. The organization is not required to arrange an alternate prescriber.

If the practitioner states in writing that they do not prescribe controlled substances and that in their professional judgment, the patients receiving their care do not require controlled substances, they are therefore not required to have a DEA/CDS certificate, but must describe their process for handling instances when a patient requires a controlled substance. The organization includes the practitioner's statement and process description in the credentialing file.

Factor 3: Education and training

Verification time limit: Prior to the credentialing decision.

The organization verifies the highest of the following three levels of education and training obtained by the practitioner as appropriate:

1. Board certification.
2. Residency.
3. Graduation from medical or professional school.

The organization uses any of the following to verify education and training:

- The primary source.
- The state licensing agency, specialty board or registry, if it performs primary source verification.
- The organization:
 - Obtains written confirmation of primary source verification from the primary source at least annually, **or**

- Provides a printed, dated screenshot of the state licensing agency, specialty board or registry website displaying the statement that it performs primary source verification of practitioner education and training information, **or**
- Provides evidence of a state statute requiring the licensing agency, specialty board or registry to obtain verification of education and training directly from the institution.
- Sealed transcripts, if the organization provides evidence that it inspected the contents of the envelope and confirmed that the practitioner completed (graduated from) the appropriate training program.

Verification of fellowship does not meet the intent of this factor.

Other acceptable verification sources for physicians (MD, DO)

Board certification

- For physicians (MD, DO):
 - ABMS or its member boards or an official ABMS Display Agent, where a dated certificate of primary-source authenticity has been provided.
Note: The ABMS “Is Your Doctor Board Certified,” accessible through the ABMS website, is intended for consumer reference only and is not an acceptable source for verifying board certification.
 - AMA Physician Masterfile.
 - AOA Official Osteopathic Physician Profile Report or AOA Physician Master File.
 - Boards in the United States that are not members of the ABMS or AOA (e.g., NBPAS), if the organization documents within its policies and procedures which specialty boards it accepts and obtains annual written confirmation from the board that the board performs primary source verification of completion of education and training.
- For other health care professionals:
 - Registry that performs primary source verification of board status if the organization obtains annual written confirmation that the registry performs primary source verification of board certification status.

Graduation from medical school

- AMA Physician Masterfile.
- American Osteopathic Association (AOA) Official Osteopathic Physician Profile Report or AOA Physician Master File.
- Educational Commission for Foreign Medical Graduates (ECFMG) for international medical graduates licensed after 1986.

Completion of residency training

- AMA Physician Masterfile.
- AOA Official Osteopathic Physician Profile Report or AOA Physician Master File.
- FCVS for closed residency programs.

NCQA only recognizes residency programs accredited by the Accreditation Council for Graduate Medical Education (ACGME), the Accreditation Council for Graduate Medical Education—International, the American Osteopathic Association (in the United States), the College of Family Physicians of Canada or the Royal College of Physicians and Surgeons of Canada.

Factor 4: Board certification status

Verification time limit: 180 calendar days.

NCQA does not require board certification; however, the organization verifies current certification status of practitioners who state that they are board certified.

The organization documents the expiration date of the board certification in the credentialing file. If a practitioner has a certification that does not expire (e.g., a lifetime certification status), the organization verifies that board certification is current and documents the date of verification.

Verification sources

The organization uses any of the following to verify board certification:

- For all practitioner types:
 - The primary source (appropriate specialty board).
 - The state licensing agency if it primary source verifies board certification.
- For physicians (MD, DO), the sources listed under *Factor 3: Education and Training*.

Note: *Verification of board certification does not apply to nurse practitioners or other health care professionals unless the organization communicates board certification to members.*

Factor 5: Work history

Verification time limit: 365 calendar days.

Employment dates. The organization obtains a minimum of the most recent five years of work history as a health professional through the practitioner's application or CV. If the practitioner has fewer than five years of work history, the time frame starts at the initial licensure date.

The application or CV includes the beginning and ending month and year for each position of employment experience, unless the practitioner has had continuous employment for five years or more with no gap. In such a case, providing the year meets the intent of this factor.

Gaps in work history. The organization documents its review of the practitioner's work history and any gaps on the application, CV, checklist or other identified documentation methods (i.e., signature or initials of staff who reviewed the history and the date of review).

- *If a gap in employment exceeds six months*, the practitioner clarifies the gap verbally or in writing. The organization documents a verbal clarification or includes the written notice in the practitioner's credentialing file.
- *If the gap in employment exceeds one year*, the practitioner clarifies the gap in writing and the organization documents review.

Factor 6: Malpractice history

Verification time limit: 180 calendar days.

The organization obtains confirmation of the past five years of malpractice settlements from the malpractice carrier or queries the National Practitioner Databank (NPDB). The five-year period may include residency or fellowship years. The organization is not required to obtain confirmation from the carrier for practitioners who had a hospital insurance policy during a residency or fellowship.

Exceptions

Factors 3 and 5 are NA for recredentialing files.

Factor 4 is NA if the practitioner is:

- Not board certified, **or**
- A board-certified nurse practitioner or other health care professional, but the organization does not communicate board certification to members.

Related information

Query results. The organization is not required to share query results with NCQA. NCQA accepts documentation of the query and of the organization's receipt of the information.

Examples**DEA- and CDS- eligible practitioner who do not have a certificate**

Practitioner's statement. I do not prescribe controlled substances for my patients. If I determine that a patient may require a controlled substance, I refer the patient to their PCP or to another practitioner for evaluation and management.

Element B: Sanction Information

The organization verifies the following sanction information for credentialing:

1. State sanctions, restrictions on licensure and limitations on scope of practice.
2. Medicare and Medicaid sanctions.

Summary of Changes

- No changes to this element.

Scoring

Met	Partially Met	Not Met
High (90-100%) on file review for at least 1 factor and medium (60-89%) on file review for any remaining factor	Medium (60-89%) on file review for 2 factors	Low (0-59%) on file review for any factor

Data source

Records or files

Scope of review**Product lines**

This element applies to First Surveys and Renewal Surveys for all product lines.

Documentation

NCQA reviews verification of sanctions information within a random sample of up to 40 initial credentialing files and up to 40 recredentialing files for practitioners that were due for recredentialing during the look-back period.

Look-back period

For First Surveys: 6 months.

For Renewal Surveys: 36 months.

Explanation THIS IS A MUST-PASS ELEMENT.

This element applies to:

- Practitioners in the scope of credentialing as defined in CR 1, Element A.
- Initial credentialing and recredentialing files, unless an exception noted below applies.

NCQA counts back from the decision date to the verification date to assess timeliness of verification.

Dispute of file review results

NCQA conducts onsite file review in the presence of the organization's staff and works with the organization to resolve any disputes during the onsite survey. An organization that is unable to resolve a dispute with the survey team must contact NCQA before the onsite survey is complete. File review results may not be disputed or appealed once the onsite survey is complete.

Appropriate documentation

Verification time limit: 180 calendar days.

Each file contains evidence of verification of sanction information from a listed source and review by organization staff. Refer to CR 1, Element A, *Related information*, "Appropriate documentation."

Factor 1: Scope of review for sanctions and limitations on licensure

The organization verifies state sanctions, restrictions on licensure and limitations on scope of practice in all states where the practitioner provides care to members. The organization may obtain verification from the NPDB for all practitioner types listed below.

The organization verifies the most recent five-year period available through any of the following sources:

- *Physicians:*
 - Appropriate state agencies.
 - Federation of State Medical Boards (FSMB).
- *Chiropractors:*
 - State Board of Chiropractic Examiners.
 - Federation of Chiropractic Licensing Boards' Chiropractic Information Network-Board Action Databank (CIN-BAD).
- *Oral surgeons:*
 - State Board of Dental Examiners or State Medical Board.
- *Podiatrists:*
 - State Board of Podiatric Examiners.
 - Federation of Podiatric Medical Boards.
- *Other nonphysician health care professionals:*
 - State licensure or certification board.
 - Appropriate state agency.

Factor 2: Sources for Medicare/Medicaid sanctions

The organization may obtain verification from any of the following sources:

- State Medicaid agency or intermediary.
- Medicare intermediary.
- List of Excluded Individuals and Entities (maintained by OIG and available over the internet).
- Medicare Exclusion Database.
- Federal Employees Health Benefits Plan (FEHB) Program department record, published by the Office of Personnel Management, Office of the Inspector General.
- AMA Physician Master File.
- FSMB.
- NPDB.

Exceptions

None.

Related information

Use of verifications in CR 5: Ongoing Monitoring. The organization may use sanctions information in CR 5, Element A, factors 1 and 2 to meet CR 3, Element B if the information is no more than 180 calendar-days old and the organization provides documentation that the practitioner was enrolled in the alert services at the time of the cited report.

Query results. The organization is not required to share query results with NCQA. NCQA accepts documentation of the query and of the organization's receipt of the information.

Examples None.

Element C: Credentialing Application

Applications for credentialing include the following:

1. Reasons for inability to perform the essential functions of the position.
2. Lack of present illegal drug use.
3. History of loss of license and felony convictions.
4. History of loss or limitation of privileges or disciplinary actions.
5. Current malpractice insurance coverage.
6. Current and signed attestation confirming the correctness and completeness of the application.

Summary of Changes**Clarifications**

- Clarified that factor 5 is met if the practitioner attests to having employer malpractice insurance and the organization verifies the employer's coverage.

Scoring	Met	Partially Met	Not Met
	High (90-100%) on file review for at least 4 factors and medium (60-89%) on file review for any remaining factors	High (90-100%) or medium (60-89%) on file review for 6 factors	Low (0-59%) on file review for any factor
Data source	Records or files		
Scope of review	Product lines <i>This element applies to First Surveys and Renewal Surveys for all product lines.</i>		
Look-back period	Documentation NCQA reviews application and attestation within a random sample of up to 40 initial credentialing files and up to 40 recredentialing files for practitioners that were due for recredentialing during the look-back period.		
	<i>For First Surveys: 6 months.</i> <i>For Renewal Surveys: 36 months.</i>		
Explanation	THIS IS A MUST-PASS ELEMENT. This element applies to: • Practitioners in the scope of credentialing as defined in CR 1, Element A. • Initial credentialing and recredentialing files, unless an exception noted below applies. NCQA counts back from the decision date to the verification date to assess timeliness of credentialing and recredentialing decisions.		
	Dispute of file review results NCQA conducts onsite file review in the presence of the organization's staff and works with the organization to resolve any disputes during the onsite survey. An organization that is unable to resolve a dispute with the survey team must contact NCQA before the onsite survey is complete. File review results may not be disputed or appealed once the onsite survey is complete.		
	Appropriate documentation <i>Attestation verification time limit: 365 calendar days.</i>		
	Each file contains the application and attestation, and evidence of review by the organization's staff. Refer to CR 1, Element A, <i>Related information</i> , "Appropriate documentation."		
	Factor 1: Inability to perform essential functions The inquiry regarding inability to perform essential functions may vary or may exceed NCQA standards, depending on the organization's interpretation of applicable legal requirements such as the Americans with Disabilities Act (ADA).		
	Factor 2: Illegal drug use Practitioners may use language other than "drug" to attest they do not use illegal substances. The organization may use more general or extensive language to		

query practitioners about impairment; language is not required to refer exclusively to the present or only to illegal substances.

Factor 3: History of loss of license

At initial credentialing, practitioners attest to any loss of license since their initial licensure. At recredentialing, practitioners attest to any loss of licensure since the last credentialing cycle.

Factor 3: History of felony convictions

At initial credentialing, practitioners attest to any felony convictions since their initial licensure. At recredentialing, practitioners attest to any felony convictions since the last credentialing cycle.

Factor 4: Limitation of privileges or disciplinary actions

At initial credentialing, practitioners attest to any loss or limitation of privileges or disciplinary actions since their initial licensure. At recredentialing, practitioners attest to any loss or limitation of privileges or disciplinary actions since the last credentialing cycle.

Factor 5: Current malpractice coverage

The application states the amount of a practitioner's current malpractice insurance coverage (even if the amount is \$0) and the date when coverage expires.

If the practitioner's malpractice insurance coverage is current and is provided in the application, it must be current as of the date when the practitioner signed the attestation and include the amount of coverage the practitioner has on the date when the attestation was signed. If the practitioner does not have current malpractice coverage, then it is acceptable to include future coverage with the effective and expiration dates.

Documentation of malpractice insurance coverage may also be a face sheet or a federal tort letter as an addendum to the application. In this case, the practitioner is not required to attest to malpractice coverage on the application. The face sheet or federal tort letter must include the insurance effective and expiration dates (the future effective date is acceptable).

If a practitioner attests to having coverage under an employer's professional liability insurance policy, factor 5 is met if the organization verifies the coverage.

Factor 6: Correctness and completeness of the application

If the application and attestation must be updated, only the practitioner may attest to the update; organization staff may not. If a copy of an application from an entity external to the organization is used, it must include an attestation to the correctness and completeness of the application. NCQA does not count the associated attestation elements as present if the practitioner did not sign the application within the required time frame.

Faxed, digital, electronic, scanned or photocopied signatures are acceptable. Signature stamps are not acceptable unless the practitioner is physically impaired and the disability is documented in the practitioner's file.

CAQH. CAQH has a system that allows the practitioner to update application information electronically. NCQA accepts the last attestation date generated by this system as the date when the practitioner signed and dated the application to attest to its completeness and correctness.

Exceptions

None.

Related information

Meeting time limits. NCQA does not require receipt of the attestation before the organization conducts credentialing verification and queries required for other elements. If the signature attestation exceeds the time limit before the credentialing decision, the practitioner must attest that the information on the application remains correct and complete, but is not required to complete another application. NCQA recommends that the organization send a copy of the completed application with the new attestation form when it requests the practitioner to update the attestation.

Use of other applications. The organization may use a state application or an application from another entity if it meets the factors in this element.

If state regulations require the organization to use a credentialing application that does not contain an attestation, or all information in factors 1–6, the organization attaches the attestation or additional information as an addendum to the application. If state regulations prohibit addenda to the application, the organization attaches a copy of the relevant regulations when it submits the survey tool.

Examples

None.

CR 4: Recredentialing Cycle Length

The organization formally recredentials its practitioners at least every 36 months.

Intent

The organization conducts timely recredentialing.

Element A: Recredentialing Cycle Length

The length of the recredentialing cycle is within the required 36-month time frame.

Summary of Changes

- No changes to this element.

Scoring

Met	Partially Met	Not Met
High (90-100%) on file review	Medium (60-89%) on file review	Low (0-59%) on file review

Data source Records or files

Scope of review **Product lines**

This element applies to Renewal Surveys for all product lines.

Documentation

NCQA reviews the timeliness of recredentialing within a random sample of up to 40 recredentialing files for practitioners that were due for recredentialing during the look-back period.

Look-back period *For Renewal Surveys: 36 months.*

Explanation **THIS IS A MUST-PASS ELEMENT.**

This element applies to:

- Practitioners in the scope of credentialing as defined in CR 1, Element A.
- Recredentialing files, unless an exception noted below applies.

Each file contains the Credentialing Committee decision date. The 36-month recredentialing cycle begins on the date of the previous credentialing decision. NCQA counts the 36-month cycle to the month, not to the day.

Dispute of file review results

NCQA conducts onsite file review in the presence of the organization's staff and works with the organization to resolve any disputes during the onsite survey. An organization that is unable to resolve a dispute with the survey team must contact NCQA before the onsite survey is complete. File review results may not be disputed or appealed once the onsite survey is complete.

Exceptions

None.

Related information

Extending the recredentialing cycle length. The organization may extend a practitioner's recredentialing cycle time frame (beyond 36 months) if the practitioner is:

- On active military assignment.
- On medical leave (e.g., maternity leave).
- On sabbatical.

The organization documents this and recredentials the practitioner within 60 calendar days of the practitioner's return to practice.

Termination and reinstatement. If the organization terminates a practitioner for administrative reasons (e.g., the practitioner failed to provide complete credentialing information) and not for quality reasons, it may reinstate the practitioner within 30 calendar days of termination and is not required to perform initial credentialing. The organization performs initial credentialing if reinstatement is more than 30 calendar days after termination.

If the organization does not have the necessary information for recredentialing, it informs the practitioner that this information is needed at least 30 calendar days before the recredentialing deadline and that without this information, the practitioner will be administratively terminated. The organization includes this notification in the practitioner's credentialing file. If the practitioner is subsequently terminated for lack of information, the termination notice should be in the practitioner's file.

Failure to recredential within 36 months. The organization will be scored down if it missed the 36-month time frame for recredentialing a practitioner but did not terminate the practitioner. The organization may recredential the practitioner within 30 calendar days of missing the deadline, but if recredentialing is not completed within 30 calendar days, the organization must initial credential the practitioner.

Examples

None.

CR 5: Ongoing Monitoring and Interventions

The organization develops and implements policies and procedures for ongoing monitoring of practitioner sanctions, complaints and quality issues between recredentialing cycles and takes appropriate action against practitioners when it identifies occurrences of poor quality.

Intent

The organization identifies and, when appropriate, acts on important quality and safety issues in a timely manner during the interval between formal credentialing.

Element A: Ongoing Monitoring and Interventions

The organization implements ongoing monitoring and makes appropriate interventions by:

1. Collecting and reviewing Medicare and Medicaid sanctions.
2. Collecting and reviewing sanctions and limitations on licensure.
3. Collecting and reviewing complaints.
4. Collecting and reviewing information from identified adverse events.
5. Implementing appropriate interventions when it identifies instances of poor quality related to factors 1–4.

Summary of Changes

Clarifications

- Replaced “release” with “alert” in the last sentence of the last paragraph under “Factors 1, 2: Time frame for reviewing sanction information.”

Scoring

Met	Partially Met	Not Met
The organization meets 4-5 factors	The organization meets 3 factors	The organization meets 0-2 factors

Data source Documented process, Reports, Materials, Records or files

Scope of review **Product lines**
This element applies to First Surveys and Renewal Surveys for all product lines.

Documentation

NCQA reviews the organization’s policies and procedures, monitoring reports and documentation of interventions throughout the look-back period.

Look-back period *For First Surveys: 6 months.*
For Renewal Surveys: 24 months.

Explanation This element applies to practitioners in the scope of credentialing as defined in CR 1, Element A.

The organization conducts ongoing monitoring between recredentialing cycles.

Factor 1: Sources for Medicare/Medicaid sanctions

The organization collects and reviews information from any of the following sources:

- State Medicaid agency or intermediary.
- Medicare intermediary.
- AMA Physician Master File entry.
- *FEHB Program Department Record*, published by the Office of Personnel Management, Office of the Inspector General.
- *List of Excluded Individuals and Entities* (maintained by OIG), available online.
- Medicare Exclusion Database.
- FSMB.
- NPDB.

Factor 2: Sources for sanctions and limitations on licensure

The organization collects and reviews information from any of the following sources:

- *Physicians:*
 - Appropriate state agencies.
 - FSMB.
 - NPDB.
- *Chiropractors:*
 - State Board of Chiropractic Examiners.
 - Federation of Chiropractic Licensing Boards' Chiropractic Information Network-Board Action Databank (CIN-BAD).
 - NPDB.
- *Oral surgeons:*
 - State Board of Dental Examiners or State Medical Board, depending on the state.
 - NPDB.
- *Podiatrists:*
 - State Board of Podiatric Examiners.
 - Federation of Podiatric Medical Boards.
 - NPDB.
- *Nonphysician healthcare practitioners:*
 - Appropriate state agency.
 - State licensure or certification board.
 - NPDB.

Factors 1, 2: Time frame for reviewing sanction information

The organization reviews information within 30 calendar days of its release by the reporting entity.

If the reporting entity does not publish sanction information on a set schedule, the organization:

- Documents that the reporting entity does not release information on a set schedule.
- Queries for this information at least every six months.

If the reporting entity does not release sanction information reports, the organization conducts individual queries of credentialed practitioners every 12–18 months.

An organization that subscribes to a sanctions alert service reviews the information within 30 calendar days of a new alert. The organization shows evidence of its subscription to the sanctions alert service during the look-back period and reviews of the information within 30 calendar days of a new alert.

Factor 3: Investigating complaints

The organization:

- Investigates practitioner-specific member complaints upon their receipt and evaluates the practitioner's history of complaints, if applicable.
- Evaluates the history of complaints for all practitioners at least every six months.

Factor 4: Adverse events

The organization monitors for adverse events at least every six months.

The organization may limit monitoring of adverse events to primary care practitioners and high-volume behavioral healthcare practitioners.

Factor 5: Implementing interventions

The organization implements interventions based on its policies and procedures if there is evidence of poor quality that could affect the health and safety of its members.

Exception

Factor 5 is NA if there are no sanctions, complaints or adverse events that require the organization to implement an intervention.

Examples

None.

CR 6: Notification to Authorities and Practitioner Appeal Rights

An organization that has taken action against a practitioner for quality reasons reports the action to the appropriate authorities and offers the practitioner a formal appeal process.

Intent

The organization uses objective evidence and patient-care considerations when deciding on a course of action for dealing with a practitioner who does not meet its quality standards.

Element A: Actions Against Practitioners

The organization has policies and procedures for:

1. The range of actions available to the organization.
2. Making the appeal process known to practitioners.

Summary of Changes

- No changes to this element.

Scoring

Met	Partially Met	Not Met
The organization meets 2 factors	The organization meets 1 factor	The organization meets 0 factors

Data source Documented process

Scope of review **Product lines**
This element applies to Interim Surveys and First Surveys for all product lines.

Documentation

NCQA reviews the organization's policies and procedures.

Look-back period *For Interim Surveys: Prior to the survey date.*
For First Surveys: 6 months.

Explanation This element is a **structural requirement**. The organization must present its own documentation.

This element applies to practitioners in the scope of credentialing as defined in CR 1, Element A.

Factor 1: Range of actions available

Policies and procedures:

- Specify that the organization reviews participation of practitioners whose conduct could adversely affect members' health or welfare.
- Specify the range of actions that may be taken to improve practitioner performance before termination.
- Specify that the organization reports its actions to the appropriate authorities.

Factor 2: Making the appeal process known

No additional explanation required.

Exceptions

None.

Examples

None.

CR 7: Assessment of Organizational Providers

The organization has written policies and procedures for the initial and ongoing assessment of providers with which it contracts.

Intent

The organization evaluates the quality of providers with which it contracts.

Element A: Review and Approval of Provider

The organization's policy for assessing a health care delivery provider specifies that before it contracts with a provider, and for at least every 36 months thereafter, it:

1. Confirms that the provider is in good standing with state and federal regulatory bodies.
2. Confirms that the provider has been reviewed and approved by an accrediting body.
3. Conducts an onsite quality assessment if the provider is not accredited.

Summary of Changes

- No changes to this element.

Scoring

Met	Partially Met	Not Met
The organization meets 2-3 factors	The organization meets 1 factor	The organization meets 0 factors

Data source Documented process

Scope of review **Product lines**
This element applies to Interim Surveys and First Surveys for all product lines.

Documentation

NCQA reviews the organization's policies and procedures in place throughout the look-back period.

Look-back period *For Interim Surveys: Prior to the survey date.*
For First Surveys: 6 months.

Explanation An organizational provider refers to facilities providing services to members and where members are directed for services rather than being directed to a specific practitioner. This element applies to all organizational providers with which the organization contracts (e.g., telemedicine providers, urgent care centers).

Factor 1: Confirmation with state and federal regulatory bodies

The organization's policies and procedures specify sources used to confirm that providers are in good standing with state and federal requirements, including:

- Applicable state or federal agency.
- Agent of the applicable state or federal agency.
- Copies of credentials (e.g., state licensure) from the provider.

NCQA does not accept an attestation from a provider to the organization regarding the provider's regulatory status.

Factor 2: Confirmation of review and approval by an accrediting body

The organization's policies and procedures specify sources used to confirm the provider's accreditation status, including:

- Applicable accrediting body for each type of organizational provider
- Agent of the applicable accrediting body.
- Copies of credentials (e.g., accreditation report, certificate or decision letter) from the provider.

NCQA does not accept an attestation from a provider to the organization regarding the provider's accreditation status.

Factor 3: Site visits for unaccredited facilities

The organization's policies and procedures include:

- Onsite quality assessment criteria for each type of provider.
- A process ensuring that the providers credential their practitioners.

The organization receives credit for this factor if its policies and procedures specify that it contracts only with accredited providers.

If a provider has satellite facilities that follow the same policies and procedures as the provider, the organization may limit site visits to a main facility.

State or federal review in lieu of a site visit. The organization may have a policy to substitute a CMS or state quality review in lieu of a site visit under the following circumstances:

- The CMS or state review is no more than three years old.
 - If the CMS or state review is older than three years, the organization conducts its own onsite quality review.
- The organization obtains a survey report or letter from CMS or the state, from either the provider or the agency, stating that the facility was reviewed and passed inspection.
 - The report meets the organization's quality assessment criteria or standards.

The organization is not required to conduct a site visit if the provider is in a rural area, as defined by the U.S. Census Bureau (<https://www.hrsa.gov/rural-health/about-us/definition/datafiles.html>), and the state or CMS has not conducted a site review.

Exceptions

None.

Related information

Time frame. NCQA does not prescribe a time frame for gathering data to use for assessing organizational providers (e.g., the 180-calendar-day rule, applied against the verification of credentials of individual practitioners, is NA).

Examples

None.

Element B: Medical Providers

The organization includes at least the following medical providers in its assessment:

1. Hospitals.*
2. Home health agencies.
3. Skilled nursing facilities.
4. Free-standing surgical centers.

***Critical factors:** These factors must be scored “yes” to score at least “Partially Met.”

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 3-4 factors	The organization meets 2 factors	The organization meets 0-1 factors
Data source	Documented process		
Scope of review	Product lines <i>This element applies to Interim Surveys and First Surveys for all product lines.</i> Documentation NCQA reviews an organization’s policies and procedures in place throughout the look-back period.		
Look-back period	<i>For Interim Surveys:</i> Prior to the survey date. <i>For First Surveys:</i> 6 months.		
Explanation	Factor 1 is a critical factor . This factor must be scored “yes” for the organization to score at least “Partially Met” on this element. Factors 2–4 No additional explanation required. Exceptions None. Related information <i>Nonskilled home health agencies.</i> Home health agencies that only provide home aides (e.g., for help with cooking, dressing, medical appointments) are not within the scope of CR 7.		
Examples	None.		

Element C: Behavioral Healthcare Providers

The organization includes behavioral health care facilities providing mental health or substance abuse services in the following settings:

1. Inpatient.
2. Residential.
3. Ambulatory.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 3 factors	The organization meets 1-2 factors	The organization meets 0 factors
Data source	Documented process		
Scope of review	Product lines <i>This element applies to Interim Surveys and First Surveys for all product lines.</i> Documentation NCQA reviews the organization's policies and procedures in place throughout the look-back period.		
Look-back period	<i>For Interim Surveys:</i> Prior to the survey date. <i>For First Surveys:</i> 6 months.		
Explanation	Assessment policies and procedures address all applicable types of providers, regardless of how many members are treated at the facilities. The organization is not required to credential organizational providers that operate only as 12-step programs. Exceptions This element is NA if all purchasers of the organization's services carve out or exclude behavioral healthcare. Factor 2 is NA if residential treatment facilities are not part of the organization's benefit package or are unavailable in the service area.		
Examples	Behavioral healthcare providers • Psychiatric hospitals and clinics. • Addiction disorder facilities. • Residential treatment centers for psychiatric and addiction disorders.		

Element D: Assessing Medical Providers

The organization assesses contracted medical health care providers against the requirements and within the time frame in Element A.

Summary of Changes

- No changes to this element.

Scoring

Met	Partially Met	Not Met
The organization meets the requirement	No scoring option	The organization does not meet the requirement

Data source

Reports, Records or files

Scope of review**Product lines**

This element applies to First Surveys and Renewal Surveys for all product lines.

Documentation

NCQA reviews evidence that the organization assessed the providers in Element B. The organization provides documentation of a tracking mechanism(s) (checklist or spreadsheet); a separate tracking mechanism or report is not required for each provider.

Look-back period

For First Surveys: 6 months.

For Renewal Surveys: 24 months.

Explanation

The organization is not required to conduct a site visit if the provider is in a rural area, as defined by the U.S. Census Bureau (<https://www.hrsa.gov/rural-health/about-us/definition/datafiles.html>), and the state or CMS has not conducted a site review.

Exceptions

None.

Examples

Table 1: Assessment of organizational providers tracking log

Org. Name	Org. Type	Confirmation Dates and Statuses		
		Licensing & Regulatory	Accrediting Body	Site Visit
Mega	Hospital	4/1/2019; Active	4/10/2019; Name; Active	Rural provider, visit not conducted by state or CMS.
		4/5/2022; Active	4/15/2022; Name; Active	Rural provider, visit not conducted by state or CMS.

Org. Name	Org. Type	Confirmation Dates and Statuses		
		Licensing & Regulatory	Accrediting Body	Site Visit
Downtown Surgery Center	Free-Standing Surgical Center	3/2/2019 Active	None	2/10/2019; CMS Compliant
		3/15/2022; Active	None	2/2/2022; CMS Compliant
District Physicians	Home Health	3/2/2019; Active	3/20/2019; Name; Active	Rural provider, visit not conducted by state or CMS.
		3/17/2022; Active	3/20/2022; Name; Active	Rural provider, visit not conducted by the state or CMS.

Element E: Assessing Behavioral Healthcare Providers

The organization assesses contracted behavioral healthcare providers against the requirements and within the time frame in Element A.

Summary of Changes

- No changes to this element.

Scoring

Met	Partially Met	Not Met
The organization meets the requirement	No scoring option	The organization does not meet the requirement

Data source Reports, Records or files

Scope of review

Product lines

This element applies to First Surveys and Renewal Surveys for all product lines.

Documentation

NCQA reviews evidence that the organization assessed the providers in Element C. The organization provides documentation of a tracking mechanism(s) (checklist or spreadsheet); a separate tracking mechanism or report is not required for each provider.

Look-back period

For First Surveys: 6 months.

For Renewal Surveys: 24 months.

Explanation

The organization is not required to conduct a site visit if the provider is in a rural area, as defined by the U.S. Census Bureau (<https://www.hrsa.gov/rural-health/about-us/definition/datafiles.html>), and the state or CMS has not conducted a site review.

The organization is also not required to conduct a site visit of ambulatory facilities that are not part of the organization's benefits package or are not available in the service area.

Exceptions

This element is NA if all purchasers of the organization's services carve out or exclude behavioral healthcare.

The organization provides evidence to support the score of "NA."

Examples**Table 2: Assessment of behavioral healthcare organizational providers tracking log**

Org. Name	Org. Type	Confirmation Dates and Statuses		
		Licensing & Regulatory	Accrediting Body	Site Visit
Mega X	Ambulatory	4/1/2019; Active	4/10/2019; Name; Active	NA
		4/5/2022; Active	4/15/2022; Name; Active	NA
Getting Better	Residential	3/2/2019; Active	None	2/2/2019; CMS Compliant
		3/15/2022; Active	None	2/10/2022; CMS Compliant

CR 8: Delegation of CR

If the organization delegates any NCQA-required credentialing activities, there is evidence of oversight of the delegated activities.

Intent

The organization remains responsible for credentialing and recredentialing its practitioners, even if it delegates all or part of these activities.

Element A: Delegation Agreement

The written delegation agreement:

1. Is mutually agreed upon.
2. Describes the delegated activities and the responsibilities of the organization and the delegated entity.
3. Requires at least semiannual reporting by the delegated entity to the organization.
4. Describes the process by which the organization evaluates the delegated entity's performance.
5. Specifies that the organization retains the right to approve, suspend and terminate individual practitioners, providers and sites, even if the organization delegates decision making.
6. Describes the remedies available to the organization if the delegated entity does not fulfill its obligations, including revocation of the delegation agreement.

Summary of Changes

Clarifications

- Added "documented process" as a data source.
- Added text under "Documentation" in the scope of review to clarify when NCQA reviews the delegate's system controls policies and procedures.
- Revised the look-back period for Renewal Surveys from "24 months" to "24 months for factors 1–6; prior to the survey date for the system controls component under factor 4."
- Added text to Factor 4: "Performance monitoring" to clarify system controls requirements.

Scoring

Met	Partially Met	Not Met
The organization meets 5-6 factors	The organization meets 3-4 factors	The organization meets 0-2 factors

Data source Documented process, Materials

Scope of review

Product lines

This element applies to Interim Surveys, First Surveys and Renewal Surveys for all product lines.

Documentation

NCQA reviews delegation agreements in effect during the look-back period from up to four randomly selected delegates, or reviews all delegates if the organization has fewer than four.

For factor 4, if credentialing system security controls are not addressed in the delegation agreement, NCQA reviews the delegate's credentialing system controls policies and procedures.

Delegation agreements implemented on or after January 1, 2022, must include a description of the delegate's credentialing system security controls. For the 2022 HPA standards only, for delegation agreements in place prior to January 1, 2022, the organization may provide other mutually agreed upon documentation or the delegate's system controls policies and procedures.

The score for the element is the average of the scores for all delegates.

Look-back period

For Interim Surveys and First Surveys: 6 months.

For Renewal Surveys: 24 months for factors 1–6; prior to the survey date for the system controls component under factor 4.

Explanation

This element may not be delegated.

This element applies to agreements that are in effect within the look-back period.

The delegation agreement describes all delegated CR activities. A generic policy statement about the content of delegated arrangements does not meet this element.

Factor 1: Mutual agreement

Delegation activities are mutually agreed on before delegation begins, in a dated, binding document or communication between the organization and the delegated entity.

NCQA considers the effective date specified in the delegation agreement as the mutually agreed-upon effective date. The effective date may be before or after the signature date on the agreement. If the agreement has no effective date, NCQA considers the signature date (the date of the last signature) as the mutually agreed upon effective date.

NCQA may accept other evidence of the mutually agreed-upon effective date: a letter, meeting minutes or other form of communication between the organization and the delegate that references the parties' agreement on the effective date of delegated activities.

NCQA requires submitted evidence for all other delegation factors to consider the same mutually agreed-upon date as the effective date for the delegate's performance of delegated activities.

Factor 2: Assigning responsibilities

The delegation agreement or an addendum thereto or other binding communication between the organization and the delegate specifies the CR activities:

- Performed by the delegate in detailed language.
- Not delegated, but retained by the organization.
 - The organization may include a general statement in the agreement addressing retained functions (e.g., the organization retains all other CR functions not specified in this agreement as the delegate's responsibility).

If the delegate subdelegates an activity, the delegation agreement must specify which organization is responsible for oversight of the subdelegate.

Factor 3: Reporting

The organization determines the method of reporting and the content of the reports, but the agreement must specify:

- That reporting is at least semiannual.
- What information is reported by the delegate about delegated activities.
- How, and to whom, information is reported (i.e., joint meetings or to appropriate committees or individuals in the organization).

The organization must receive regular reports from all delegates, even NCQA-Accredited delegates. NCQA scores this factor “yes” if the organization delegates CR activities to an NCQA-Certified CVO that is certified to perform the delegated activity.

Factor 4: Performance monitoring

The delegation agreement states the organization’s process for monitoring and evaluating the delegate’s performance.

If the organization contracts with delegates that store, create, modify or use credentialing data on the organization’s behalf, the delegation agreement describes:

- The delegate’s CR system security controls in place to protect data from unauthorized modification as outlined in CR 1, Element C (Credentialing System Controls), factor 4.
- How the delegate monitors its credentialing system security controls at least annually, as required in CR 8, Element C, factor 5.
- How the organization monitors the delegate’s credentialing system security controls at least annually, as required in CR 8, Element C, factor 5.

Factor 5: Right to approve, suspend and terminate

No additional explanation required.

Factor 6: Consequences for failure to perform

The delegation agreement specifies consequences if a delegate fails to meet the terms of the agreement and, at a minimum, circumstances that result in revocation of the agreement.

Exceptions

This element is NA if the organization does not delegate CR activities.

Examples**Factor 3: Reporting**

- Lists of credentialed and recertified practitioners.
- Committee meeting minutes.
- List of providers assessed.

Element B: Predelegation Evaluation

For new delegation agreements initiated in the look-back period, the organization evaluated delegate capacity to meet NCQA requirements before delegation began.

Summary of Changes**Clarifications**

- Revised the paragraph under “NCQA-Accredited/Certified delegates” to include Accredited health plans or MBHOs.

Scoring

Met	Partially Met	Not Met
The organization evaluated delegate capacity before delegation began	The organization evaluated delegate capacity after delegation began	The organization did not evaluate delegate capacity

Data source

Reports

Scope of review**Product lines**

This element applies to Interim Surveys, First Surveys and Renewal Surveys for all product lines.

This element applies if delegation was implemented in the look-back period.

Documentation

NCQA reviews the organization’s predelegation evaluation from up to four randomly selected delegates, or reviews all delegates if the organization has fewer than four.

The score for the element is the average of the scores for all delegates.

Look-back period

For Interim Surveys and First Surveys: 6 months.

For Renewal Surveys: 12 months.

Explanation

This element may not be delegated.

NCQA-Accredited/Certified delegates

Automatic credit is available for this element if all delegates are NCQA-Accredited health plans or MBHOs, NCQA Accredited in CR or NCQA Certified in CVO, unless the element is NA. NCQA-Certified CVOs must be certified to perform the activity being delegated by the organization.

Predelegation evaluation

The organization evaluated the delegate’s capacity to meet NCQA requirements within 12 months prior to implementing delegation. The evaluation may include a review of the organization’s structure, processes and staffing in order to determine its capability to perform the delegated function.

NCQA considers the date of the agreement to be the implementation date if the delegation agreement does not include an implementation date.

If the time between the predelegation evaluation and implementation of delegation exceeds 12 months, the organization conducts another predelegation evaluation.

If the organization amends the delegation agreement to include additional CR activities within the look-back period, it performs a predelegation evaluation for the additional activities.

Exceptions

This element is NA if:

- The organization does not delegate CR activities.
- Delegation arrangements have been in effect for longer than the look-back period.

Related information

Use of collaboratives. An organization may collaborate in a statewide predelegation evaluation with other organizations that have overlapping practitioner and provider networks. The organizations in the collaborative use the same audit tool and share data. Each organization is responsible for meeting NCQA delegation standards, but may use the shared data collection process to reduce burden.

Examples**Predelegation evaluation**

- Site visit.
- Telephone consultation.
- Documentation review.
- Committee meetings.
- Virtual review.

Element C: Review of Delegate's Credentialing Activities

For delegation arrangements in effect for 12 months or longer, the organization:

1. Annually reviews its delegate's credentialing policies and procedures.
2. Annually audits credentialing and recredentialing files against NCQA standards for each year that delegation has been in effect.
3. Annually evaluates delegate performance against NCQA standards for delegated activities.
4. Semiannually evaluates regular reports, as specified in Element A.
5. Annually monitors the delegate's credentialing system security controls to ensure that the delegate monitors its compliance with the delegation agreement or with the delegate's policies and procedures at least annually.
6. Annually acts on all findings from factor 5 for each delegate and implements a quarterly monitoring process until each delegate demonstrates improvement for one finding over three consecutive quarters.

Summary of Changes**Additions**

- Added factors 5 and 6 and adjusted the scoring to reflect the new factors.

Clarifications

- Revised the look-back period for Renewal Surveys from "24 months" to "24 months for factors 1–4; at least once during the prior year for factors 5–6."

- Revised the first paragraph under “NCQA-Accredited/Certified delegates” to include Accredited health plans or MBHOs.
- Added a third paragraph under “NCQA-Accredited/Certified delegates” in the Explanation to clarify when NCQA awards automatic credit to delegates.
- Added “Factor 5: Annual monitoring of CR systems” subhead and text.
- Added “Factor 6: Takes action” subhead and text.
- Added a second paragraph under “Related information” to clarify that organizations are not required to audit a CVO against timeliness requirements.

Policy Changes

- Added Exceptions for factors 5 and 6.

Scoring	<table><tr><th>Met</th><th>Partially Met</th><th>Not Met</th></tr><tr><td>The organization meets 5-6 factors</td><td>The organization meets 3-4 factors</td><td>The organization meets 0-2 factors</td></tr></table>	Met	Partially Met	Not Met	The organization meets 5-6 factors	The organization meets 3-4 factors	The organization meets 0-2 factors
Met	Partially Met	Not Met					
The organization meets 5-6 factors	The organization meets 3-4 factors	The organization meets 0-2 factors					
Data source	Reports						
Scope of review	Product lines <i>Factor 1 applies to Interim Surveys for all product lines.</i> <i>All factors in this element apply to First Surveys and Renewal Surveys for all product lines.</i> Documentation NCQA reviews evidence of the organization’s review from up to four randomly selected delegates, or all delegates if the organization has fewer than four. <i>For All Surveys:</i> NCQA reviews the organization’s evaluation of the delegate’s CR activities (factor 1). <i>For First Surveys:</i> NCQA also reviews the organization’s most recent annual audit, performance evaluation and semiannual report evaluation. <i>For Renewal Surveys:</i> NCQA also reviews the most recent and the previous year’s annual reviews, audits, performance evaluations and four semiannual evaluations. The score for the element is the average of the scores for all delegates.						
Look-back period	<i>For Interim Surveys and First Surveys:</i> At least once during the prior year. <i>For Renewal Surveys:</i> 24 months for factors 1–4; at least once during the prior year for factors 5–6.						
Explanation	This element may not be delegated. NCQA-Accredited/Certified delegates Automatic credit is available for factors 2 and 3 if all delegates are NCQA-Accredited health plans or MBHOs, NCQA Accredited in CR or NCQA Certified in CVO, unless the element is NA. NCQA-Certified CVOs must be certified to perform the activity being delegated by the organization. Automatic credit is available for factor 4 for NCQA-Certified CVOs that are certified to perform the delegated activity.						

For factors 5 and 6, automatic credit is available for delegates that are NCQA Accredited under 2021 (or later) standards for Health Plan Accreditation, MBHO Accreditation or UM-CR-PN Accreditation.

Factor 1: Review of the CR policies and procedures

The appropriate organization staff or committee reviews the delegate's credentialing policies and procedures. At a minimum, the organization reviews the sections of the policies and procedures that apply to the delegated functions.

Factor 2: Annual file audit

If the organization delegates credentialing, it audits the delegate's credentialing and recredentialing files against NCQA standards. The organization uses one of the following methods to audit the files:

- 5 percent or 50 of its files, whichever is less, to ensure that information is verified appropriately.
 - At a minimum, the sample includes at least 10 credentialing files and 10 recredentialing files. If fewer than 10 practitioners were credentialed or recredentialed since the last annual audit, the organization audits the universe of files rather than a sample.
- The NCQA "8/30 methodology" available at <https://www.ncqa.org/programs/health-plans/policy-accreditation-and-certification/>

The organization bases its annual audit on the responsibilities of the delegate described in the delegation agreement and the appropriate NCQA standards.

Factor 3: Annual evaluation

No additional explanation required.

Factor 4: Evaluation of reports

For delegates that are NCQA Accredited in CR, the only NCQA-required reporting is the names or files of practitioners or providers processed by the delegate.

Factor 5: Annual monitoring of CR systems

The organization's process for monitoring system security controls covers delegates that store, create, modify or use CR data on its behalf. If the organization contracts with such delegates, it has a process for annually monitoring the delegate's CR system security controls in place to protect data from unauthorized modification as outlined in CR 1, Element C (Credentialing System Controls), factor 4. The organization also ensures that the delegate annually monitors that it follows the delegation agreement or its own policies and procedures.

The organization reviews all modifications made in all delegates' CR systems during the look-back period that did not meet the modification criteria allowed by the delegation agreement or by the delegates' policies and procedures. The organization provides documentation (which may be a report or other type of evidence) that it completed the monitoring process at least annually during the look-back period.

If the delegate's CR system does not allow modifications, the delegate:

- Describes the functionality of the system that ensures compliance with established policy.

- Provides documentation or evidence of advanced system control capabilities that automatically record dates and prevent modifications that do not meet modification criteria.

Audit. Auditing is allowed only if the organization or delegate does not use a CR system that can identify all noncompliant modifications.

- Documentation indicates the staff roles or department involved in the audit.
- The organization or delegate identifies all CR system modifications, but may use sampling to identify potential noncompliant changes for the audit.
- The organization uses one of the following methods to audit files:
 - 5 percent or 50 of its files, whichever is less, to ensure that information is verified appropriately.
 - At a minimum, the sample includes at least 10 credentialing files and 10 recredentialing files. If fewer than 10 practitioners were credentialed or recredentialed since the last annual audit, the organization audits the universe of files rather than a sample.
 - The NCQA “8/30 methodology” available at <https://www.ncqa.org/programs/health-plans/policy-accreditation-and-certification/>

Factor 6: Annual actions

The organization identifies and documents all actions it has taken, or plans to take, to address all modifications (findings from factor 5) that did not meet the delegation agreement or the delegate’s policies and procedures, if applicable. One action may be used to address more than one finding for each delegate or across multiple delegates, if appropriate.

The organization also implements a quarterly monitoring process for each delegate to assess the effectiveness of its actions on all findings.

- The organization must continue to monitor each delegate until the delegate demonstrates improvement of at least one finding over three consecutive quarters.
- If a delegate did not demonstrate improvement of at least one finding during the look-back period, it submits all quarterly monitoring reports demonstrating ongoing monitoring.
- If the organization identified findings less than three quarters before the survey submission date, it submits all monitoring information it has available.

Exceptions

The element is NA if:

- The organization does not delegate CR activities.
- Delegation arrangements have been in effect for less than 12 months.

Factor 2 is NA if none of the practitioners were initial credentialed or were due for recredentialing.

Factors 2–4 are NA for Interim Surveys.

Factor 5 is NA if the delegate only provides cloud-based CR data storage functions and does not provide services that create, modify or use CR data.

Factor 6 is NA if the organization did not identify any modifications or if all identified modifications met the delegation agreement or the delegate’s policies and procedures.

Related information

Use of collaborative. An organization may collaborate in a statewide, annual file audit and annual evaluation with other organizations that have overlapping practitioner and provider networks. The organizations in the collaborative use the same audit tool and share data.

Auditing CVOs. The organization is not required to audit the CVO against timeliness requirements during the delegation audit, because NCQA does not recognize CVOs for decision making. If the organization delegates decision making, NCQA assesses the organization for timeliness of the credentialing decision.

Examples**Factor 2: Annual evaluation**

- Site visit.
- Telephone consultation.
- Documentation review.
- Committee meetings.
- Virtual review.

Element D: Opportunities for Improvement

For delegation arrangements that have been in effect for more than 12 months, at least once in each of the past 2 years, the organization identified and followed up on opportunities for improvement, if applicable.

Summary of Changes**Clarifications**

- Revised the paragraph under “NCQA-Accredited/Certified delegates” to include Accredited health plans or MBHOs.

Scoring

Met	Partially Met	Not Met
The organization has acted on identified problems, if any, at least once in each of the past 2 years that the delegation arrangement has been in effect	The organization took inappropriate or weak action, or acted only in the past year	The organization has not acted on identified problems

Data source Documented process, Reports, Materials

Scope of review**Product lines**

This element applies to First Surveys and Renewal Surveys for all product lines.

Documentation

NCQA reviews reports of opportunities for improvement, from up to four randomly selected delegates, or from all delegates, if the organization has fewer than four, and for evidence that the organization took appropriate action to resolve issues.

For First Surveys: NCQA reviews the organization's most recent annual review and follow-up on improvement opportunities.

For Renewal Surveys: NCQA reviews the organization's most recent and the previous year's annual reviews and follow-up on improvement opportunities.

The score for the element is the average of the scores for all delegates.

Look-back period

For First Surveys: At least once during the prior year.

For Renewal Surveys: 24 months.

Explanation

This element may not be delegated.

NCQA-Accredited/Certified delegates

Automatic credit is available for this element if all delegates are NCQA-Accredited health plans or MBHOs, NCQA Accredited in CR or NCQA Certified in CVO, unless the element is NA. NCQA-Certified CVOs must be certified to perform the delegated activity.

Identify and follow-up on opportunities

The organization uses information from its predelegation evaluation, ongoing reports, or annual evaluation to identify areas of improvement.

Exceptions

This element is NA if:

- The organization does not delegate CR activities.
- Delegation arrangements have been in effect for less than 12 months.
- The organization has no opportunities to improve performance.
 - NCQA evaluates whether this conclusion is reasonable, given assessment results.

Examples

None.

Member Experience

Standards for Member Experience

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ME 1: Statement of Members' Rights and Responsibilities

Refer to Appendix 1 for points

The organization has a written policy that states its commitment to treating members in a manner that respects their rights, and its expectations of members' responsibilities.

Intent

The organization recognizes the specific needs of and maintains a mutually respectful relationship with members.

Element A: Rights and Responsibilities Statement

The organization's member rights and responsibilities statement specifies that members have:

1. A right to receive information about the organization, its services, its practitioners and providers and member rights and responsibilities.
2. A right to be treated with respect and recognition of their dignity and their right to privacy.
3. A right to participate with practitioners in making decisions about their health care.
4. A right to a candid discussion of appropriate or medically necessary treatment options for their conditions, regardless of cost or benefit coverage.
5. A right to voice complaints or appeals about the organization or the care it provides.
6. A right to make recommendations regarding the organization's member rights and responsibilities policy.
7. A responsibility to supply information (to the extent possible) that the organization and its practitioners and providers need in order to provide care.
8. A responsibility to follow plans and instructions for care that they have agreed to with their practitioners.
9. A responsibility to understand their health problems and participate in developing mutually agreed-upon treatment goals, to the degree possible.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 6-9 factors	No scoring option	The organization meets 0-5 factors

Data source Documented process, Materials

Scope of review **Product lines**
For Interim Surveys and First Surveys: This element applies to all product lines.
For Renewal Surveys: This element applies to the Medicaid product line only.

Documentation

NCQA reviews the organization's policies and procedures or rights and responsibilities statement that is in place throughout the look-back period.

Look-back period	<i>For Interim Surveys:</i> Prior to the survey date. <i>For First Surveys:</i> 6 months. <i>For Renewal Surveys:</i> 24 months.
Explanation	Authority for this activity may not be delegated and must be performed by the organization; however, an organization may adopt the rights and responsibilities statement of its customers or payers (e.g., a Medicaid organization adopts the rights and responsibilities statement required by the state Medicaid agency). Factors 1–9 No additional explanation required. Exception This element is NA for Renewal Surveys for the commercial, Medicare and Exchange product lines.
Examples	None.

Element B: Distribution of Rights Statement

The organization distributes its member rights and responsibilities statement to the following groups:

1. New members, upon enrollment.
2. Existing members, if requested.
3. New practitioners, when they join the network.
4. Existing practitioners, if requested.

Summary of Changes

Clarifications

- Added a “Related Information” section and language about use of vendors/mail service organizations for distribution of member rights and responsibilities statement.

Scoring	Met	Partially Met	Not Met
	The organization meets 3-4 factors	The organization meets 2 factors	The organization meets 0-1 factors
Data source	Reports, Materials		
Scope of review	Product line		
	This element applies to First Surveys and Renewal Surveys for all product lines.		
	Documentation		
	For First Surveys and Renewal Surveys: NCQA reviews organization materials containing the rights statement distributed to members at enrollment and to practitioners who joined the network during the look-back period.		

NCQA also reviews the organization's distribution of materials containing the rights statement to existing members and practitioners during the look-back period, if requested.

Look-back period

For First Surveys: 6 months.

For Renewal Surveys: 24 months.

Explanation

The organization notifies members and practitioners of policy revisions as they occur.

Distribution of rights statement

The organization distributes information to members and practitioners by mail, fax or email, or on its website if it informs members and practitioners that the information is available online. The notice must include a description specific enough to give readers a clear idea of the topic and the general content and must include a link or direction to the specific information. The organization may group or summarize the information by theme. The organization mails the information to members and practitioners who do not have fax, email or internet access.

Factors 1, 3

No additional explanation required.

Factors 2, 4

The organization provides documentation of an instance in the look-back period when it has distributed the rights and responsibilities statement to a member and practitioner upon request. If the organization has had no requests, it states so in writing as part of its survey submission. The organization is not required to track requests for existing members and practitioners during the look-back period in order to demonstrate the fact that there are none.

Documentation that the organization distributes the rights and responsibilities statement to all members and practitioners annually meets factors 2 and 4.

Exceptions

Factor 2 is NA if no existing members request the information.

Factor 4 is NA if no existing practitioners request the information.

Related information

Use of vendors/mail service organizations for distribution of member rights and responsibilities statement. If the organization contracts with a mail service organization to provide distribution services, it provides access to the mail service organization's documentation for evaluation. NCQA does not consider the relationship to be delegation, and delegation oversight is not required under ME 8. Refer to *Vendors* in Appendix 2.

Examples

None.

ME 2: Subscriber Information

The organization provides each subscriber with the information necessary to understand benefit coverage and obtain care.

Intent

The organization informs subscribers about benefits and access to medical services.

Element A: Subscriber Information

The organization distributes the following written information to its subscribers upon enrollment and annually thereafter:

1. Benefits and services included in, and excluded from, coverage.
2. Pharmaceutical management procedures, if they exist.
3. Copayments and other charges for which members are responsible.
4. Benefit restrictions that apply to services obtained outside the organization's system or service area.
5. How to obtain language assistance.
6. How to submit a claim for covered services, if applicable.
7. How to obtain information about practitioners who participate in the organization.
8. How to obtain primary care services, including points of access.
9. How to obtain specialty care and behavioral healthcare services and hospital services.
10. How to obtain care after normal business hours.
11. How to obtain emergency care, including the organization's policy on when to directly access emergency care or use 911 services.
12. How to obtain care and coverage when subscribers are out of the organization's service area.
13. How to submit a complaint.
14. How to appeal a decision that adversely affects coverage, benefits or a subscriber's relationship with the organization.
15. Availability of independent, external review of internal UM final determinations.
16. How the organization evaluates new technology for inclusion as a covered benefit.

Summary of Changes

Clarifications

- Added "and" to the bullets for First and Renewal Surveys under "Documentation" in the scope of review to clarify that all bullets must be met.
- Replaced "The most recent distribution" with "The most recent distribution and the previous year's distribution" in the first bullet under "Documentation" in the scope of review for Renewal Surveys.
- Revised the look-back period for Renewal Surveys to 24 months for all factors.
- Revised the first paragraph in the Explanation to clarify that factor 5 may be delegated to an organization with NCQA MHC Distinction.

- Added a “Related information” section and language about use of vendors/mail service organizations for distribution of subscriber information.

Scoring	Met	Partially Met	Not Met
	The organization meets 12-16 factors	The organization meets 8-11 factors	The organization meets 0-7 factors

Data source	Reports, Materials
Scope of review	Product lines <i>This element applies to Interim Surveys, First Surveys and Renewal Surveys for all product lines.</i> NCQA reviews and scores this element for each product line brought forward for Accreditation. Documentation <i>For Interim Surveys:</i> NCQA reviews information that will be distributed to subscribers. <i>For First Surveys:</i> NCQA reviews the following: • The most recent distribution of information to subscribers for all factors, and • Evidence that the organization distributed the information to subscribers. <i>For Renewal Surveys:</i> NCQA reviews the following: • The most recent and the previous year’s distribution of information to subscribers for all factors, and • The previous year’s distribution of information to subscribers for factors 1–14 and 16. • Evidence that the organization distributed the information to subscribers.
Look-back period	<i>For Interim Surveys:</i> Prior to the survey date. <i>For First Surveys:</i> 6 months. <i>For Renewal Surveys:</i> 24 months.
Explanation	This element may not be delegated, with the exception of factor 5, which may be delegated to an organization with NCQA MHC Distinction. <i>For First Surveys and Renewal Surveys,</i> NCQA does not accept or review materials presented in draft form or not yet distributed. Information about subscriber benefits and services can be accessed easily and is written in user-friendly language. Distribution of subscriber information The organization distributes information to subscribers by mail, fax or email. The organization may include the information on its website if it informs subscribers that the information is available online. The notice must include a description specific enough to give readers a clear idea of the topic and the general content and must include a link or direction to the specific information. The organization may group or summarize the information by theme. The organization also informs subscribers that the statement is available through alternative media on request.

Factor 1: Benefits and services

Information distributed to subscribers explains the scope and limitations of benefits and services. The materials may include broad statements about exclusions (e.g., experimental or investigation services) without specifying the service or procedure provided the materials state that members have the opportunity to request information on excluded services or procedures and the organization maintains internal policies or criteria for these services or procedures.

Factor 2: Pharmaceutical management

Information distributed to subscribers includes a description of the following components of the organization's pharmaceutical management policies and procedures:

- Procedures that affect coverage of pharmaceuticals.
- How to obtain or review lists of pharmaceuticals.
- How to obtain or review limitations on prescribing or on access to pharmaceuticals.
- The copayment structure for restricted pharmaceuticals.
- The exceptions policy for coverage of nonformulary pharmaceuticals (if there is a closed formulary).

Factors 3, 4

No additional explanation required.

Factor 5: Language assistance

The organization provides language services to all subscribers who request them, through bilingual staff or interpreter services, to help subscribers obtain information about benefits and access to medical services.

Use of contracted translation services is not considered delegation.

Factor 6: Claims for covered services

No additional explanation required.

Factor 7: Information about practitioners

The organization tells subscribers how to obtain the following practitioner information:

- Name, address, telephone numbers.
- Professional qualifications.
- Specialty.
- Medical school attended.
- Residency completion.
- Board certification status.

Factor 8: Primary care services

No additional explanation required.

Factor 9: Specialty care, behavioral healthcare and hospital services

If the organization uses a primary care gatekeeper system, it tells subscribers how to access specialists through primary care practitioners. If the organization allows self-referrals for specialist services, such as behavioral healthcare or cardiology, it tells subscribers how to access these services.

The organization tells subscribers if they are restricted from certain specialists in its network.

The organization tells subscribers how to access hospital services.

Factors 10, 11: After-hours and emergent care

No additional explanation required.

Factor 12: Care and coverage outside the service area

The organization tells subscribers how to access services outside the service area, including information on covered and noncovered benefits.

Factor 13: Submitting a complaint

The organization informs subscribers how to submit complaints both orally and in writing.

Factor 14: Appealing a decision

The organization provides details about its appeal process, for coverage and noncoverage appeals, which may include:

- Time frames for members to file an appeal.
- Time frames for deciding the appeal.
- The procedures for filing an appeal, including where to direct the appeal and information to include in the appeal.

Factor 15: External review rights

The organization provides written notification to subscribers of the availability of independent, external review of internal UM final determinations.

Factor 16: Evaluating new technology for inclusion as a covered benefit

No additional explanation required.

Exceptions

Factor 2 is NA if all purchasers of the organization's services carve out or exclude pharmaceutical management.

Factor 6 is NA if the organization does not process claims.

Factor 15 is NA for appeals:

- By members covered by Medicare, Medicaid or the Federal Employees Health Benefits (FEHB) Program.
- By members in self-funded accounts.
- By members whose employer has arranged for employees to have access to employer-mandated independent review.

Related information

Use of vendors/mail service organizations for distribution of subscriber information. If the organization contracts with a mail service organization to provide distribution services, it provides access to the mail service organization's documentation for evaluation. NCQA does not consider the relationship to be delegation, and delegation oversight is not required under ME 8. Refer to *Vendors* in Appendix 2.

Examples**Factors 1–16: Sources of information for subscribers**

- Subscriber handbook.
- Practitioner and provider directory.
- Benefits summary materials.
- Subscriber ID card.

Element B: Interpreter Services

Based on the linguistic need of its subscribers, the organization provides interpreter or bilingual services in its Member Services department and telephone functions.

Summary of Changes

- No changes to this element.

Scoring	Met The organization meets the requirement	Partially Met No scoring option	Not Met The organization does not meet the requirement
Data source	Reports, Materials, Records or files		
Scope of review	Product lines <i>This element applies to Interim Surveys, First Surveys and Renewal Surveys for all product lines.</i> Documentation <i>For Interim Surveys:</i> NCQA reviews information that will be sent to subscribers. <i>For First Surveys and Renewal Surveys:</i> NCQA reviews an assessment of subscribers' linguistic needs and contracts (or similar documents) for evidence that the organization provides services throughout the look-back period.		
Look-back period	<i>For Interim Surveys:</i> Prior to the survey date. <i>For First Surveys:</i> 6 months. <i>For Renewal Surveys:</i> 24 months.		
Explanation	Use of contracted translation services is not considered delegation. This element applies to organizations with subscribers whose principal spoken and written language is not English. The organization may use the linguistic data used to satisfy NET 1, Element A.		

Exception

This element is NA if the organization can show that all subscribers' principal spoken and written language is English.

Examples**Sources of data to determine linguistic needs**

- Census Bureau data.
- Subscriber surveys.
- Information from employer groups.
- Analysis of subscribers' complaints.
- For Medicaid and Medicare product lines, results of the Race/Ethnicity of Membership (RDM) and Language Diversity of Membership (LDM) HEDIS measures.

ME 3: Marketing Information

The organization ensures that communication with prospective members correctly and thoroughly represents the benefits and operating procedures of the organization.

Intent

Prospective members receive an accurate description of the organization's benefits and operating procedures.

Element A: Materials and Presentations

All organizational materials and presentations accurately describe the following information:

1. Covered benefits.
2. Noncovered benefits.
3. Practitioner and provider availability.
4. Key UM procedures the organization uses.
5. Potential network, service or benefit restrictions.
6. Pharmaceutical management procedures.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 5-6 factors	The organization meets 3-4 factors	The organization meets 0-2 factors
Data source	Materials		
Scope of review	Product lines <i>This element applies to Interim Surveys, First Surveys and Renewal Surveys for all product lines.</i> Documentation NCQA reviews the organization's marketing information made available to prospective members within the look-back period.		
Look-back period	<i>For Interim Surveys: Prior to the survey date.</i> <i>For First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	This element may not be delegated. The organization ensures that members enroll with a realistic expectation of how it operates. Factors 1, 2 No additional explanation required.		

Factor 3: Availability

The organization's marketing materials describe the networks or subnetworks of practitioners and providers that prospective members may access, and how to access them.

Factor 4: UM procedures

UM procedures include, but are not limited to:

- Preservice review.
- Urgent concurrent review.
- Postservice review.
- Filing an appeal.

Factor 5: Potential restrictions

No additional explanation required.

Factor 6: Pharmaceutical management procedures

Marketing materials:

- Describe the organization's pharmaceutical management procedures.
- List pharmaceutical restrictions.
- Provide instructions for obtaining of the pharmaceutical management procedures or checking coverage regarding a pharmaceutical.
- Describe the exceptions process for coverage of nonformulary pharmaceuticals, if the organization has a closed formulary.

Exceptions

This element is NA for Medicare or Exchange product lines if the organization has no control over marketing materials. The organization must provide documentation demonstrating this restriction.

This element is NA for the Medicaid product line if:

- The organization has no control over its marketing materials, **or**
- The organization does not communicate with or market to prospective members and does not submit information to a centralized location for prospective members to compare plans.

The organization has documentation of the restriction or a policy stating that it does not market to prospective members.

Examples

None.

Element B: Communicating With Prospective Members

The organization uses easy-to-understand language in communications to prospective members about its policies and practices regarding collection, use and disclosure of PHI:

1. In routine notification of privacy practices.
2. The right to approve the release of information (use of authorizations).
3. Access to medical records.
4. Protection of oral, written and electronic information across the organization.
5. Information for employers.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 4-5 factors	The organization meets 3 factors	The organization meets 0-2 factors
Data source	Materials		
Scope of review	Product lines <i>This element applies to Interim Surveys, First Surveys and Renewal Surveys for all product lines.</i> Documentation NCQA reviews the organization's materials containing PHI use and disclosure policies and procedures communicated to prospective members throughout the look-back period.		
Look-back period	<i>For Interim Surveys:</i> Prior to the survey date. <i>For First Surveys:</i> 6 months. <i>For Renewal Surveys:</i> 24 months.		
Explanation	This element may not be delegated. Factors 1–4 No additional explanation required. Factor 5: Information for employers The organization explains what information it shares with employers (or purchasers). Exceptions This element is NA for Medicare or Exchange product lines if the organization has no control over marketing materials. The organization must provide documentation demonstrating this restriction. This element is NA for the Medicaid product line if: • The organization has no control over its marketing materials, <i>or</i>		

- The organization does not communicate with or market to prospective members and does not submit information to a centralized location for prospective members to compare plans.
 - The organization has documentation of the restriction or a policy stating that it does not market to prospective members.

Factor 5 is NA:

- For product lines sponsored by state or federal government (e.g., Medicare and Medicaid).
- If the organization does not disclose information to plan sponsors.
- For individual plans and family plans offered under the Exchange product line.

Examples None.

Element C: Assessing Member Understanding

The organization systematically takes the following steps:

1. Assesses how well new members understand policies and procedures.
2. Implements procedures to maintain accuracy of marketing communication.
3. Acts on opportunities for improvement, if applicable.

Summary of Changes

Clarifications

- Revised the look-back period for First Surveys to 24 months.
- Revised the Explanation to state that the organization initiates a quality improvement process to clarify its policies and procedures.
- Replaced “analysis” with “assessment” in the last bullet under “Factor 1: Assessment” in the Examples.

Scoring	Met	Partially Met	Not Met
	The organization meets 2-3 factors	The organization meets 1 factor	The organization meets 0 factors

Data source Reports

Scope of review **Product lines**

This element applies to First Surveys and Renewal Surveys for all product lines.

Documentation

For factors 1, 2: NCQA reviews evidence that the organization completed the required activities at least once during the look-back period.

For factor 3: NCQA reviews evidence that the organization took action at least once during the look-back period, if applicable.

Look-back period *For First Surveys and Renewal Surveys: 24 months.*

Explanation This element may not be delegated.

The organization specifies how often it collects member feedback. If assessment of feedback shows that new members do not have an accurate understanding of the organization's policies and procedures, the organization initiates a quality improvement process to clarify its policies and procedures.

Factors 1–3

No additional explanation required.

Exceptions

This element is NA for Medicare or Exchange product lines if the organization has no control over marketing materials. The organization must provide documentation demonstrating this restriction.

This element is NA for the Medicaid product line if:

- The organization has no control over its marketing materials, **or**
- The organization does not communicate with or market to prospective members and does not submit information to a centralized location for prospective members to compare plans.
 - The organization has documentation of the restriction or a policy stating that it does not market to prospective members.

Factor 3 is NA if the organization has no opportunities to improve member understanding. NCQA evaluates whether this conclusion is reasonable, given assessment results.

Examples

Factor 1: Assessment

- Surveys of new members.
- Surveys conducted through employers.
- Focus groups.
- Assessment of inquiries and complaints by new members.

ME 4: Functionality of Claims Processing

The organization provides members with timely and accurate information about their claims.

Intent

The organization allows members to access and track claims through the claims process on its website and by telephone.

Element A: Functionality—Website

Members can track the status of their claims in the claims process and obtain the following information on the organization's website in one attempt or contact:

1. The stage in the process.
2. The amount approved.
3. The amount paid.
4. The member's cost.
5. The date paid.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 4-5 factors	The organization meets 2-3 factors	The organization meets 0-1 factors
Data source	Documented process, Reports, Materials		
Scope of review	Product lines <i>This element applies to First Surveys for all product lines.</i>		
	Documentation NCQA reviews the organization's website content and functionality against the requirements of this element. Both must be in place throughout the look-back period. If the organization can provide a "test" or "demo" log-on ID, NCQA reviews the organization's performance through that mechanism. If the organization cannot provide a test or demo log-on, NCQA reviews the organization's website or screen shots of web functionality, supplemented with documents specifying the required features and functions of the site. If screen shots provided include detailed explanations of how the site works, there is no need to provide supplemental documents.		
Look-back period	<i>For First Surveys: 6 months.</i>		

Explanation The organization provides evidence that it can perform all activities required by this element, even if no employers or plans sponsors purchase the services.

The organization meets the requirement of “one attempt or contact” if:

- Members can access all required website functions in one session without the need to sign in again or contact the organization.
- The website contains links to other organizations that provide the information stated in the factors.

Factor 1: The stage in the process

Members can access and track claims in all stages of the claim process on the organization’s website. NCQA does not review individual claims, and expects that data are present only if applicable to the specific stage in the claim process.

Factors 2–4

No additional explanation required.

Factor 5: Date paid

“Date paid” is the date when the claim is processed for payment or the date when the check is cut.

Exceptions

This element is NA:

- If 90 percent or more of the organization’s claims payments (measured in dollars, not in the number of claims processed or encounters) are under capitation and members have no financial responsibility beyond a flat copay.
- For services that are carved out for a specific employer group, and the organization does not process claims for carved-out services.
- If members have no financial liability (i.e., co-insurance, deductibles, charges in excess of allowed amounts, differentials in cost between in-network care and out-of-network care, costs that vary for the formulary) for services beyond a flat copay that is always the same fixed dollar amount and cannot be balance-billed by a practitioner, provider or other party.
 - The flat copayment amount is specified on the organization’s website. It may be different across a range of services, but must not be different within the same service category (e.g., \$15 for primary care office visits and \$25 for specialist office visits is acceptable).

The organization provides documentation to support a score of “NA.”

Related information

Use of vendors for claims processing services. If the organization contracts with a vendor to provide web-based claims processing services, it provides access to the vendor’s claims processing system. NCQA does not consider the relationship to be delegation, and delegation oversight is not required under ME 8. NCQA evaluates the vendor’s system against the requirements. Refer to *Vendors* in Appendix 2.

Examples None.

Element B: Functionality—Telephone Requests

Members can track the status of their claims in the claims process and obtain the following information over the telephone in one attempt or contact:

1. The stage in the process.
2. The amount approved.
3. The amount paid.
4. The member's cost.
5. The date paid.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 4-5 factors	The organization meets 3 factors	The organization meets 0-2 factors
Data source	Documented process, Reports, Materials		
Scope of review	Product lines <i>This element applies to First Surveys for all product lines.</i>		
	Documentation NCQA reviews evidence of how the organization's telephone system meets each factor throughout the look-back period. <i>For live-person systems:</i> NCQA reviews policies and procedures, scripts and other resources used by Member Services staff. <i>For automated systems:</i> NCQA reviews evidence of functional capability or scripts, supplemented with documents specifying the required features and stating that the telephone system functions as required.		
Look-back period	<i>For First Surveys:</i> 6 months.		
Explanation	The organization provides evidence that it can perform the activities required by this element, even if no employers or plan sponsors purchase the services. The organization meets the requirements for all five factors, even if a factor does not apply to a specific claim. A notation may be made in the organization's system regarding claim status (e.g., pending, denied). One attempt or contact The organization meets the requirement of "one attempt or contact" if members can complete all the required functions over the telephone without needing to make more than one call. The organization may have an automated system that answers and triages an initial call, but once the member reaches a live person, providing the member with another number to call, transferring the member to voicemail or into a phone queue does not meet the requirement.		

The organization may use:

- A live-person or automated telephone system to provide the information, **or**
- A live-person telephone transfer to another person or organization.

After-hours calls

Calls received after normal business hours are returned within one business day. Calls received after midnight are responded to the same business day. If the organization does not have a voicemail system, it has another method to track calls that are received after normal business hours, and returns those calls on the next business day.

Factor 1: Stage in the process

Members can access and track claims over the telephone for all stages of the payment process. NCQA does not review individual claims, and expects that data are present only if applicable to the specific stage in the claim process.

Factors 2–4

No additional explanation required.

Factor 5: Date paid

“Date paid” is the date when the claim is processed for payment or the date when the check is cut.

Exceptions

None.

Examples**Evidence of the claims process by telephone**

- Tracking logs that contain responses to applicable factors.
- Decision-tree script provided to Member Services staff.

ME 5: Pharmacy Benefit Information

The organization provides members with the information they need to understand and use their pharmacy benefit.

Intent

The organization uses its website and telephone communication to inform members about their pharmacy benefit, their financial responsibility for medications and the operations of network pharmacies.

Element A: Pharmacy Benefit Information—Website

Members can complete the following actions on the organization's website in one attempt or contact:

1. Determine their financial responsibility for a drug, based on the pharmacy benefit.
2. Initiate the exceptions process.
3. Order a refill for an existing, unexpired mail-order prescription.
4. Find the location of an in-network pharmacy.
5. Conduct a pharmacy proximity search based on zip code.
6. Determine the availability of generic substitutes.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 4-6 factors	The organization meets 2-3 factors	The organization meets 0-1 factors
Data source	Documented process, Reports, Materials		
Scope of review	Product lines		
	<i>This element applies to First Surveys for all product lines.</i>		
	Documentation		
	NCQA reviews the organization's website content and functionality against the requirements of this element. Both must be in place throughout the look-back period.		
	If the organization can provide a "test" or "demo" log-on ID, NCQA reviews the organization's performance through that mechanism. If the organization cannot provide a test or demo log-on, NCQA reviews the organization's website or screen shots of web functionality, supplemented with documents specifying the required features and functions of the site. If screen shots provided include detailed explanations of how the site works, there is no need to provide supplemental documents.		
Look-back period	<i>For First Surveys: 6 months.</i>		

Explanation The organization has the capability to meet the requirements of this element, even if no employers or plan sponsors purchase the services.

One attempt or contact

The organization meets the requirement of “one attempt or contact” if:

- Members can access all required website functions in one session without the need to sign in again or contact the organization.
- The website contains a direct link to the organization’s pharmacy benefit manager or to a similar organization that performs the functions in factors 1–6.

Factor 1: Financial responsibility

The organization’s website:

- Allows members to enter a pharmaceutical name, the National Drug Code (NDC) or another identifier.
- Calculates a member’s financial responsibility (e.g., out-of-pocket cost associated with filling a prescription) for a listed medication, based on the member’s pharmacy benefit.
 - The organization’s website may display a statement that because of frequent changes in the price of medications, financial information provided on the site may not be exact.

If members have no financial responsibility for any medications, the organization meets the requirement if it posts a generic statement on its website informing members that they have no financial responsibility.

If members have the same flat copay for all medications (copay does not vary by tier, drug class, injectables), the organization meets the requirement if it posts a generic statement on its website informing members that there is a flat copay, including the dollar amount, for all medications. The only acceptable variation is days supply (e.g., 30-days vs. 90-days supply).

Factor 2: Exceptions process

The organization’s website has an exceptions process for reviewing the requests of members for noncovered pharmaceuticals when the formulary does not adequately accommodate their clinical needs. The exceptions process can resolve an issue before it reaches the level of a formal appeal.

Members (or their authorized representative) can initiate the exceptions process on their own behalf without having to go through a dispensing pharmacist’s online system. Limiting the process to initiate the exceptions process to only practitioners does not meet the factor.

The organization has an exceptions process, even if it has a process for communicating online with a dispensing pharmacist about an exception to a prescribed pharmaceutical.

Factor 3: Mail-order prescription refills

Members can use the organization’s website to order a refill of an existing, unexpired mail-order prescription, if allowed by law. The organization determines which pharmaceuticals are available by mail, and is not required to make all prescription medications available by mail.

Factors 4, 5: Location and proximity search

No additional explanation required.

Factor 6: Availability of generic substitutes

Members can enter the name of a specific pharmaceutical on the organization's website to retrieve a list of available generic medication substitutes for a specific pharmaceutical.

Exceptions

This element is NA if all purchasers of the organization's services carve out or exclude the pharmacy benefit.

Factor 2 is NA if the organization has open formularies where all drugs are allowed.

Factor 3 is NA if the organization does not offer a mail-order service.

Related information

Factors 3–5. NCQA does not consider it to be delegation if the organization contracts with pharmacies and provides a link to the pharmacies' website.

Use of vendors. NCQA considers the use of a PBM or contracting with a similar organization to be delegation, except for factors 3–5.

Examples**Factor 1: Financial responsibility**

The organization's website states:

- *Medicaid members:* There is no copay for any prescription.
CHIP members: There is a \$10 copay for each prescription, each time it is filled.
- *Medicaid members:* There is no copay for any prescription.
CHIP members: There is a \$5 copay for prescriptions up to a 30-days supply and \$10 for prescriptions with a 31–90 days supply, for each prescription, each time it is filled.

Element B: Pharmacy Benefit Information—Telephone

Members can complete the following actions via telephone in one attempt or contact:

1. Determine their financial responsibility for a drug, based on the pharmacy benefit.
2. Initiate the exceptions process.
3. Order a refill for an existing, unexpired, mail-order prescription.
4. Find the location of an in-network pharmacy.
5. Conduct a proximity search based on zip code.
6. Determine the availability of generic substitutes.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 4-6 factors	The organization meets 2-3 factors	The organization meets 0-1 factors
Data source	Documented process, Reports, Materials		
Scope of review	Product lines <i>This element applies to First Surveys for all product lines.</i> Documentation NCQA reviews evidence of how the organization’s telephone system meets each factor throughout the look-back period. <i>For live-person systems:</i> NCQA reviews policies and procedures, scripts and other resources used by Member Services staff. <i>For automated system:</i> NCQA reviews evidence of functional capability or scripts, supplemented with documents specifying the required features and stating that the telephone system functions as required.		
Look-back period	<i>For First Surveys:</i> 6 months.		
Explanation	The organization has the capability to meet the requirements, even if it does not provide services to any employer or plan sponsor. One attempt or contact The organization meets the requirement of “one attempt or contact” if members can complete all the required functions over the telephone without the need to make more than one call. The organization may have an automated system that answers and triages an initial call, but once the member reaches a live person, providing the member with another number to call, transferring the member to voicemail or into a phone queue does not meet the requirement. The organization may use: • A live-person or automated telephone system to provide the information, or • A live-person telephone transfer to another person or organization. After-hours calls The organization may implement a system where calls received after normal business hours are returned on the next business day, but calls received after midnight on Monday–Friday are responded to on the same business day. If the organization does not have a voicemail system, it has another method of tracking calls that are received after normal business hours, and returns those calls on the next business day. Factor 1: Financial responsibility Members can determine their financial responsibility (e.g., out-of-pocket cost associated with filling a prescription) for specified pharmaceuticals by telephone. The organization may inform members that because of frequent changes in the price of medications, financial information provided over the telephone may not be exact.		

Factor 2: Exceptions process

The organization has an exceptions process for reviewing the requests of members for noncovered pharmaceuticals when the formulary does not adequately accommodate their clinical needs. The exceptions process can resolve an issue before it reaches the level of a formal appeal.

Eligible individuals (or their authorized representative) can initiate the exceptions process on their own behalf without having to go through a dispensing pharmacist's telephone system. Limiting the process to initiate the exceptions process to only practitioners does not meet the factor.

Factor 3: Mail-order prescription refills

Members can order a refill of an existing, unexpired mail-order prescription, if allowed by law.

The organization determines which pharmaceuticals are available by mail, and is not required to make all prescription medications available by mail.

Factors 4, 5: Location and proximity search

No additional explanation required.

Factor 6: Availability of generic substitutes

Members can access information by telephone on the availability of generic substitutes for specific pharmaceuticals.

Exceptions

This element is NA if all purchasers of the organizations carve out or exclude the pharmacy benefit.

Factor 2 is NA if the organization has open formulary where all drugs are allowed.

Factor 3 is NA if the organization does not offer mail-order services.

Related information

Factor 6: Clinical information. Member Services staff are not expected to answer clinical questions. Members may receive a return call from a pharmacist or clinician within 24 hours, or may be transferred directly to a pharmacist or clinician.

Examples

None.

Element C: QI Process on Accuracy of Information

The organization seeks to improve pharmacy benefit information provided on its website and by telephone, by:

1. Collecting data on quality and accuracy of information.
2. Analyzing data results.
3. Acting to improve identified deficiencies.

Summary of Changes**Clarifications**

- Revised the element stem and factor language.
- Replaced the “Factor 3: Acting to improve” subhead with “Factor 3: Acting to improve identified deficiencies” in the Explanation.

Scoring

Met	Partially Met	Not Met
The organization meets 2-3 factors	The organization meets 1 factor	The organization meets 0 factors

Data source Documented process, Reports

Scope of review**Product lines**

This element applies to First Surveys and Renewal Surveys for all product lines.

Documentation

For First Surveys and Renewal Surveys: NCQA reviews the organization’s data collection methodology and most recent data collection, analysis and action reports completed at least once during the look-back period. The methodology may be a separate documented process or described as part of the organization’s reports.

Look-back period

For First Surveys and Renewal Surveys: At least once in the prior 24 months.

Explanation

Both quality and accuracy must be addressed to receive credit. When the organization measures quality, it measures how useful or understandable the information provided is. Accuracy is related to how correct or precise the information provided is.

Factor 1: Data collection

The organization collects data on both the quality and accuracy of the pharmacy benefit information available on its website and provided by phone.

The data collection methodology that produces valid and reliable results and includes information addressed in all factors of Elements A and B.

Factor 2: Quantitative and qualitative analysis

For initial measurement, the organization conducts quantitative and qualitative analysis of data.

For remeasurement, the organization conducts quantitative analysis and conducts qualitative analysis if quantitative analysis demonstrates that stated goals were not met.

Refer to *Appendix 5: Glossary* for the full definition of and requirements for *quantitative analysis* and *qualitative analysis*.

Factor 3: Acting to improve identified deficiencies

The organization takes at least one action to improve deficiencies identified from the analyses conducted for factor 2. Interventions relate directly to the cause of the deficiency identified in the qualitative analysis.

Exceptions

This element is NA if all purchasers of the organization's services carve out or exclude the pharmacy benefit.

Factor 3 is NA if no deficiencies are identified. NCQA evaluates whether this conclusion is reasonable, given assessment results.

Examples

Evidence of the QI process

- Periodic audits of information provided via the organization's website and by telephone.

Quality assessment—website

- How easily members can find the information they need.
- How easy is the information to understand?
- Member feedback on the usefulness or understandability of information (e.g., surveys, focus groups).

Quality assessment—telephone

- Quality assurance monitoring of telephone interactions (e.g., clarity of response to questions).
- Member feedback on the usefulness or understandability of information (e.g., surveys, focus groups).

Accuracy assessment—website

- For interactive components, is the correct information returned when data is entered (e.g., a member benefit search returned correct benefit information)?

Accuracy assessment—telephone

- Quality assurance monitoring of telephone interactions (e.g., correct greeting and member identification, HIPAA verification).

Element D: Pharmacy Benefit Updates

The organization updates member pharmacy benefit information on its website and in materials used by telephone staff, as of the effective date of a formulary change and as new drugs are made available or are recalled.

Summary of Changes**Clarifications**

- Replaced “as soon as possible” with “as of the effective date” under “Pharmacy benefit information updates” in the Explanation.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Documented process, Materials		
Scope of review	Product lines <i>This element applies to First Surveys and Renewal Surveys for all product lines.</i>		
Look-back period	Documentation NCQA reviews the organization’s policies and procedures for updating pharmacy benefit information, and evidence of updates throughout the look-back period.		
	<i>For First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	Pharmacy benefit information updates The organization updates the pharmacy benefit information of members as of the effective date when there are changes to the formulary due to the availability of new drugs and the withdrawal or recall of pharmaceuticals. Updates include those related to formulary tiers, copays, limitations and exclusions. The organization updates both its website and materials used by staff to meet the requirement. Exception This element is NA if all purchasers of the organization’s services carve out or exclude the pharmacy benefit. Related information <i>Immediate formulary changes, recalled or withdrawn drugs.</i> For formulary changes occurring immediately or for recalled or withdrawn drugs, NCQA does not require the organization to update pharmacy benefit information on its website as of the effective date. Updates to benefit information should occur as soon as possible.		
Examples	None.		

ME 6: Personalized Information on Health Plan Services

The organization provides members with the information they need to easily understand and use health plan benefits.

Intent

The organization makes it easy for members to decide how to use their benefits.

Element A: Functionality—Website

Members can complete each of the following activities on the organization’s website in one attempt or contact:

1. Change a primary care practitioner, as applicable.
2. Determine how and when to obtain referrals and authorizations for specific services, as applicable.
3. Determine benefit and financial responsibility for a specific service or treatment from a specified provider or institution, if applicable.

Summary of Changes

Clarifications

- Revised the first sentence under “Factor 3: Benefit and financial responsibility” in the Explanation to clarify that the organization provides information about member financial responsibility for services under its medical benefit.

Scoring	Met	Partially Met	Not Met
	The organization meets 3 factors	The organization meets 2 factors	The organization meets 0-1 factors

Data source Documented process, Reports, Materials

Scope of review **Product lines**
This element applies to First Surveys and Renewal Surveys for all product lines.

Documentation

NCQA reviews the organization’s website content and functionality against the requirements of this element. Both must be in place throughout the look-back period.

If the organization can provide a “test” or “demo” log-on ID, NCQA reviews the organization’s performance through that mechanism. If the organization cannot provide a test or demo log-on, NCQA reviews the organization’s website or screen shots of web functionality, supplemented with documents specifying the required features and functions of the site. If screen shots provided include detailed explanations of how the site works, there is no need to provide supplemental documents.

Look-back period *For First Surveys: 6 months.*
For Renewal Surveys: 24 months.

- Explanation** The organization meets the requirement of “one attempt or contact” if:
- Members can access all required website functions in one session without the need to sign in again or contact the organization.
 - The website contains links to other organizations that provide the information stated in the factors.

Factor 1: Changing a primary care practitioner

A member can change or select a primary care practitioner on the organization’s website in one attempt or contact.

The organization receives credit for this factor if it does not require members to select a primary care practitioner (i.e., members may see any primary care practitioner in the benefits plan). The organization must provide documentation to support this.

Factor 2: Referrals and authorizations

If the organization requires a referral for services other than primary care, its website contains instructions for obtaining a referral and lists necessary authorizations.

The organization receives credit for this factor if it does not require referrals or authorizations for members for any services or procedures. The organization must provide documentation to support this.

Factor 3: Benefit and financial responsibility

The organization provides pricing for all services available to members under its medical benefit. Financial estimates may be a range based on local market cost or average negotiated rates. The organization may communicate that such information does not constitute approval or authorization for the service.

An interactive website that allows members to enter specific information meets the requirements of this factor. In order to provide accurate information, the organization may require a process where members provide the relevant CPT code.

The organization is not required to consider members’ deductible or out-of-pocket maximum expense when estimating financial responsibility, but it should communicate this to members and explicitly state the difference in financial responsibility between in-network benefits and out-of-network benefits, and that members may be responsible for charges above approved rates.

Exceptions

Factor 3 is NA if:

- 90 percent or more of the organization’s claims payments (as measured in dollars, rather than number of claims processed or number of members) are for services under capitation and the member has no financial responsibility beyond a flat co-pay.
- Members have no financial liability (i.e., co-insurance, deductibles, charges in excess of allowed amounts, differentials in cost between in-network care and out-of-network care, costs that vary for the formulary) for services beyond a flat copay that is always the same fixed dollar amount and cannot be balance-billed by a practitioner, provider or other party.

- The flat copayment amount is specified on the organization's website. It may be different across a range of services, but must not be different within the same service category (e.g., \$15 for primary care office visits and \$25 for specialist office visits is acceptable).

The organization provides documentation to support a score of "NA."

Examples None.

Element B: Functionality—Telephone

To support financial decision making, members can complete each of the following activities over the telephone within one business day:

1. Determine how and when to obtain referrals and authorizations for specific services, as applicable.
2. Determine benefit and financial responsibility for a specific service or treatment from a specified provider or institution.

Summary of Changes

Clarifications

- Revised the first sentence under "Factor 2: Benefit and financial responsibility" in the Explanation to clarify that the organization provides information about member financial responsibility for services under its medical benefit.

Scoring

Met	Partially Met	Not Met
The organization meets 2 factors	The organization meets 1 factor	The organization meets 0 factors

Data source Documented process, Reports, Materials

Scope of review

Product lines

This element applies to First Surveys and Renewal Surveys for all product lines.

Documentation

NCQA reviews evidence of how the organization's telephone system meets each factor throughout the look-back period.

For live-person systems, NCQA reviews policies and procedures, scripts and other resources used by Member Services staff.

Look-back period

For First Surveys: 6 months.

For Renewal Surveys: 24 months.

Explanation

The organization provides evidence that it can perform all activities evaluated by this element, even if it does not provide services to any employer or plan sponsor.

Members can complete the activity within one business day. The organization may use live-person telephone transfers to another person or organization.

Calls received after normal business hours are returned within one business day. Calls received after midnight are responded to on the same business day. If the organization does not have a voicemail system, it has another method of tracking

calls received after normal business hours, and returns those calls on the next business day.

Factor 1: Referrals and authorizations

The organization receives credit for this factor if it does not require referrals or authorizations.

Factor 2: Benefit and financial responsibility

The organization provides pricing for all services available to members under its medical benefit. Financial estimates may be a range based on local market cost or average negotiated rates. The organization may communicate that such information does not constitute approval or authorization for the service.

In order to provide accurate information, the organization may require a process where members provide the relevant CPT code.

The organization is not required to consider members' deductible or out-of-pocket maximum expense when estimating financial responsibility, but it should communicate this to members and explicitly state the difference in financial responsibility between in-network benefits and out-of-network benefits, and that members may be responsible for charges above approved rates.

Exceptions

None.

Examples

Evidence

- Policies and procedures, supplemented with call scripts.
- Documentation of resources available to Member Services.

Element C: Quality and Accuracy of Information

At least annually, the organization must evaluate the quality and accuracy of the information it provides to its members via the web and telephone, by:

1. Collecting data on quality and accuracy of information provided.
2. Analyzing data against standards or goals.
3. Determining causes of deficiencies, as applicable.
4. Acting to improve identified deficiencies, as applicable.

Summary of Changes

Clarifications

- Clarified in the Exception that NCQA evaluates whether an NA score for factors 3 and 4 is reasonable, given assessment results.

Scoring

Met	Partially Met	Not Met
The organization meets 4 factors	The organization meets 2-3 factors	The organization meets 0-1 factors

Data source Documented process, Reports

Scope of review	Product lines <i>This element applies to First Surveys and Renewal Surveys for all product lines.</i> Documentation NCQA reviews the organization's data collection methodology. The methodology may be a separate documented process or described as part of the organization's reports. <i>For First Surveys:</i> NCQA also reviews the organization's most recent annual data collection, analysis and action reports. <i>For Renewal Surveys:</i> NCQA also reviews the organization's most recent and the previous year's annual data collection, analysis and action reports.
Look-back period	<i>For First Surveys:</i> At least once during the prior year. <i>For Renewal Surveys:</i> 24 months.
Explanation	The organization analyzes: • Functionality in Element A, factor 1 to ascertain that the web application produces the desired result. • Quality and accuracy of information in Element A, factors 2 and 3, and Element B, factors 1 and 2. Both quality and accuracy must be addressed to receive credit. When the organization measures quality, it measures how useful or understandable the information provided is. Accuracy is related to how correct or precise the information provided is. Factor 1: Data collection The organization collects data on both the quality and accuracy of the information available on its website and provided by phone. The data collection methodology produces valid and reliable results and includes information addressed in all factors of Elements A and B. Factors 2, 3: Quantitative and qualitative analysis <i>For initial measurement,</i> the organization conducts quantitative and qualitative analysis of data. <i>For remeasurement,</i> the organization conducts quantitative analysis and conducts qualitative analysis if quantitative analysis demonstrates that stated goals were not met. Refer to <i>Appendix 5: Glossary</i> for the full definition of and requirements for <i>quantitative analysis</i> and <i>qualitative analysis</i>. Factor 4: Acting to improve The organization takes at least one action to improve deficiencies identified from the analyses conducted for factor 3. Interventions relate directly to the cause of the deficiency identified in the qualitative analysis. Exception Factors 3 and 4 are NA if no deficiencies are identified. NCQA evaluates whether this conclusion is reasonable, given assessment results.

Examples**Evidence of the QI process**

- Periodic audits of Member Services email communication and calls regarding the quality and accuracy of benefit information provided on the web and by telephone.

Quality assessment—website

- How easily members can find the information they need.
- How easy is the information to understand?
- Member feedback on the usefulness or understandability of information (e.g., surveys, focus groups).

Quality assessment—telephone

- Quality assurance monitoring of telephone interactions (e.g., clarity of response to questions).
- Member feedback on the usefulness or understandability of information (e.g., surveys, focus groups).

Accuracy assessment—website

- For interactive components, is the correct information returned when data is entered (e.g., a member benefit search returned correct benefit information)?

Accuracy assessment—telephone

- Quality assurance monitoring of telephone interactions (e.g., correct greeting and member identification, HIPAA verification).

Element D: Email Response Evaluation**The organization:**

1. Has a process for responding to member email inquiries within one business day of submission.
2. Has a process for annually evaluating the quality of email responses.
3. Annually collects data on email turnaround time.
4. Annually collects data on the quality of email responses.
5. Annually analyzes data.
6. Annually acts to improve identified deficiencies.

Summary of Changes**Clarifications**

- Replaced “NCQA reviews” with “NCQA also reviews” under “Documentation” in the scope of review.

Scoring

Met	Partially Met	Not Met
The organization meets 4-6 factors	The organization meets 2-3 factors	The organization meets 0-1 factors

Data source Documented process, Reports

Scope of review	Product lines <i>This element applies to First Surveys and Renewal Surveys for all product lines.</i> Documentation NCQA reviews the organization's policies and procedures in place throughout the look-back period for factors 1 and 2. <i>For First Surveys:</i> NCQA also reviews the organization's most recent annual evaluation report and actions completed within the look-back period for factors 3–6. <i>For Renewal Surveys:</i> NCQA also reviews the organization's previous and most recent annual evaluation report and actions completed within the look-back period for factors 3–6.
Look-back period	<i>For First Surveys:</i> 6 months. <i>For Renewal Surveys:</i> 24 months.
Explanation	This requirement applies to any email address that is communicated to members (e.g., in member materials, website, newsletter) as a method for contacting the organization. Although the intent of the element is to provide an electronic customer service function via email, if the organization does not provide this email function but uses a member portal to communicate with members instead, it may opt to have NCQA review and score the portal functions. Services provided by the portal must meet all requirements in Element D. Factor 1: Email inquiries Responding to member emails verbally does not meet the intent of the requirement. A system-generated, automatic email acknowledgment of receipt of a question or notification of when a member can expect a researched response does not meet the intent of the requirement. If responding to member email inquiries requires additional time, it is acceptable to send the member an email that describes the member's specific issue and the investigation that is in process. Factors 2–4: Process to measure quality The data collection methodology produces valid and reliable results. Factors 3–4: Data collection The organization collects information data on both the turnaround time and the email responses. When the organization measures quality, it measures how useful or understandable the email is. Factor 5: Quantitative and qualitative analysis <i>For initial measurement,</i> the organization conducts quantitative and qualitative analysis of data. <i>For remeasurement,</i> the organization conducts quantitative analysis and conducts qualitative analysis if quantitative analysis demonstrates that stated goals were not met.

Refer to *Appendix 5: Glossary* for the full definition of and requirements for *quantitative analysis* and *qualitative analysis*.

Factor 6: Acting to improve

The organization takes at least one action to improve deficiencies identified from the analyses conducted for factor 5. Interventions relate directly to the cause of the deficiency identified in the qualitative analysis.

Exception

Factor 6 is NA if no deficiencies are identified. NCQA evaluates whether this conclusion is reasonable, given assessment results.

Examples

None.

ME 7: Member Experience

The organization has written policies and procedures for thorough, appropriate and timely resolution of member complaints and appeals.

Intent

The organization has a thorough and consistent process for addressing member complaints and appeals.

Element A: Policies and Procedures for Complaints

The organization has policies and procedures for registering and responding to oral and written complaints that include:

1. Documentation of the substance of complaints and actions taken.
2. Investigation of the substance of complaints.
3. Notification to members of the resolution of the complaint and, if there is an adverse decision, the right to appeal.
4. Standards for timeliness, including standards for urgent situations.
5. Provision of language services for the complaint process.

Summary of Changes

Clarifications

- Clarified in the third paragraph of the Explanation that complaints resolved during first contact are included in the analysis for ME 7, Elements C and E.
- Added text under “Factor 1: Documentation” in the Explanation.
- Revised the first paragraph under “Factor 2: Investigation” in the Explanation to clarify that the organization investigates complaints and documents its findings.
- Added a last paragraph under “Factor 3: Notification of resolution and appeal rights” in the Explanation to clarify that for the Medicare product line, organizations are not required to notify members of the right to appeal a grievance decision.

Scoring	Met	Partially Met	Not Met
	The organization meets 3-5 factors	The organization meets 2 factors	The organization meets 0-1 factors

Data source Documented process

Scope of review **Product lines**
This element applies to Interim Surveys, First Surveys and Renewal Surveys for all product lines.

Documentation

NCQA reviews the organization’s policies and procedures.

Look-back period *For Interim Surveys: Prior to the survey date.*
For First Surveys: 6 months.
For Renewal Surveys: 24 months.

Explanation This is a **structural requirement**. The organization must present its own documentation.

This element applies to all complaints that do not become requests for coverage or requests to overturn a decision.

Complaints resolved during the first contact must be categorized as complaints and included in the analysis in ME 7, Elements C and E.

Factor 1: Documentation

The organization's documentation of the complaint includes:

- The member's reason for making the complaint.
- Actions taken, including, but not limited to:
 - The member's previous complaint history.
 - Follow-up activities associated with the complaint.

Factor 2: Investigation

The organization investigates the content of the complaint, including all issues relevant to the complaint, and documents its findings.

The organization's policies and procedures for resolving quality-of-care complaints specify when practitioner review is required.

Factor 3: Notification of resolution and appeal rights

Members have the right to appeal an adverse decision. If the organization makes an adverse decision as part of resolving a complaint, it notifies members of the decision and of their right to appeal.

If the organization cannot resolve a complaint within the time frame stated in its policies or cannot notify the member of the final decision for legal or statutory reasons, at a minimum, it must notify the member that the complaint was received and investigated.

For the Medicare product line, the organization is not required to notify members of the right to appeal a grievance decision.

Factor 4: Timeliness

The organization sets timeliness standards for registering and responding to complaints. The organization's timeliness and notification standards consider urgency, as defined by the organization.

Factor 5: Language services

The organization provides language services through bilingual staff or interpreter services to help members through the complaint process.

Use of contracted translation services is not considered delegation.

Exceptions

None.

Examples **Factor 5: Language services**

- Oral interpretation of documents written in English into a member's preferred language.
- Member notification documents are available in languages other than English.
- Language-line interpretation services are available for registering oral complaints.

Element B: Policies and Procedures for Appeals

The organization has policies and procedures for registering and responding to oral and written appeals of decisions that are not about coverage that include:

1. Documentation of the substance of appeals and actions taken.
2. Investigation of the substance of appeals.
3. Notification to members of the disposition of appeals and the right to further appeal, as appropriate.
4. Standards for timeliness, including standards for urgent situations.
5. Provision of language services for the appeal process.

Summary of Changes

Clarifications

- Added text under “Factor 1: Documentation” in the Explanation.
- Revised the first paragraph under “Factor 2: Investigation” to clarify that the organization investigates appeal issues and documents its findings.
- Added an exception for the Medicare product line.

Scoring

Met	Partially Met	Not Met
The organization meets 3-5 factors	The organization meets 2 factors	The organization meets 0-1 factors

Data source Documented process

Scope of review

Product lines

This element applies to Interim Surveys, First Surveys and Renewal Surveys for all product lines.

Documentation

NCQA reviews the organization’s policies and procedures.

Look-back period

For Interim Surveys: Prior to the survey date.

For First Surveys: 6 months.

For Renewal Surveys: 24 months.

Explanation

This is a **structural requirement**. The organization must present its own documentation.

“Appeal” in this element refers to appeals of decisions that are not about coverage. Appeals for coverage are assessed in *UM 8: Policies for Appeals* and *UM 9: Appropriate Handling for Appeals*. Members or their authorized representatives may appeal any adverse decision.

Factor 1: Documentation

The organization’s documentation of the appeal includes:

- The member’s reason for appealing the previous decision.
- Actions taken, including, but not limited to:

- The member's previous appeal history.
- Follow-up activities associated with the appeal.

Factor 2: Investigation

The organization investigates the content of the appeal, including all issues relevant to the appeal, and documents its findings.

Factor 3: Notification of resolution and appeal rights

The organization notifies members of its decision and of their right to appeal the resolution further within the time frame specified in its policies.

Factor 4: Timeliness

The organization sets timeliness standards for registering and responding to appeals. The organization's timeliness and notification standards consider urgency, as defined by the organization.

Factor 5: Language services

The organization provides language services through bilingual staff or interpreter services to help members through the appeal process.

Use of contracted translation services is not considered delegation.

Exception

This element is NA for the Medicare product line.

Examples**Appeals of decisions that are not about coverage**

- The organization denied a member's sixth request in 12 months to change primary care practitioners.
- A member's coverage was terminated for non-payment of premium, but the member had cancelled checks as proof of payment.
- Member appeals being placed in a restricted pharmacy program (the member must get all based on prescription history).
- A member appealed the organization's payment to a practitioner because of a significant concern with the quality of care.
- A member whose primary language was not English requested language assistance. The organization denied the request, stating that the population of members who spoke the language was too small to support language assistance. The member appealed the decision.

Factor 5: Language services

- Oral interpretation of documents that are written in English into a member's preferred language.
- Member notification documents are available in languages other than English.
- Language-line interpretation services are available for registering oral appeals.

Element C: Annual Assessment of Nonbehavioral Healthcare Complaints and Appeals

Using valid methodology, the organization annually analyzes nonbehavioral complaints and appeals for each of the five required categories.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Reports		
Scope of review	Product lines <i>This element applies to First Surveys and Renewal Surveys for all product lines.</i> NCQA reviews and scores this element for each product line brought forward for Accreditation; only the commercial and Exchange product lines may be combined. Documentation <i>For First Surveys:</i> NCQA reviews the organization's most recent annual data collection and analysis report. <i>For Renewal Surveys:</i> NCQA reviews the organization's most recent and the previous year's annual data collection and analysis report.		
Look-back period	<i>For First Surveys:</i> At least once during the prior year. <i>For Renewal Surveys:</i> 24 months.		
Explanation	Analysis of nonbehavioral complaints and appeals The organization collects and analyzes data for complaints and appeals separately. The organization collects data on member complaints and appeals from the entire population of members or draws statistically valid samples from the member population. If the organization use a member sample, it describes the sample universe and the sampling methodology. The organization collects data from all sources of members' complaints and appeals, including: • Non-coverage appeals addressed in ME 7, Element A: Policies and Procedures for Complaints and ME 7, Element B: Policies and Procedures for Appeals. • UM coverage appeals addressed in UM 8: Policies for Appeals and UM 9: Appropriate Handling of Appeals. The organization aggregates all complaints and appeals into the following required categories: • Quality of Care. • Access. • Attitude and Service.		

- Billing and Financial Issues.
- Quality of Practitioner Office Site.

The organization may use a different coding system for internal purposes, but it must aggregate and analyze all complaint and appeal data in the reporting categories above for NCQA evaluation purposes.

The organization reports results on each category, even if there are no complaints or appeals for a category.

Quantitative and qualitative analysis

For initial measurement, the organization conducts quantitative and qualitative analysis of data.

For remeasurement, the organization conducts quantitative analysis and conducts qualitative analysis if quantitative analysis demonstrates that stated goals were not met.

Refer to *Appendix 5: Glossary* for the full definition of and requirements for *quantitative analysis* and *qualitative analysis*.

Exceptions

None.

Examples

Complaint and appeal types

Quality of Care

- A member complained that a practitioner misdiagnosed a condition.

Access

- A member believed in-network practitioners did not have the expertise necessary to deal with an issue, and requested an out-of-network referral, which was denied. The member appealed the decision.
- Citing a shortage of Spanish-speaking practitioners, a member requested to go out of network. The request was denied and the member appealed the decision.
- A member complained that participating practitioners lacked available appointments.
- A member complained that a primary care practitioner refused to make a specialist referral. The member appealed to the organization to allow the referral.

Attitude and Service

- A member complained that the physician practitioner was rude and used abusive language.
- A member complained that there was a 30-minute wait to checkout after an appointment.
- A member complained about the tone and attitude of the customer service representative.
- A member complained that a customer service representative provided inaccurate information.

Billing/Financial

- Out-of-network services where members are balance billed.
- Disputes of deductibles and copayments.

Quality of Practitioner Office Site

A member sought out-of-network care because the participating practitioner's offices lacked wheelchair accessibility. The organization identified other practitioners with wheelchair access, but the member appealed to go out of network.

Table 1: Complaint volume report

Category	PREVIOUS YEAR		CURRENT MEASUREMENT YEAR	
	Complaints, Total	Complaints per 1,000 Members (Total: 300,000)	Complaints, Total	Complaints per 1,000 Members (Total: 240,000)
Quality of Care	1,462	4.87	1,323	5.51
Access	1,075	3.58	1,416	5.90
Attitude/Service	946	3.15	951	3.96
Billing/Financial	817	2.72	785	3.27
Quality of Practitioner Office Site	431	1.44	413	1.72
Total/Number per 1,000	4,731	15.77	4,888	20.37

Table 2: Appeal volume report

Category	PREVIOUS YEAR		CURRENT MEASUREMENT YEAR	
	Complaints, Total	Complaints per 1,000 Members (Total: 300,000)	Complaints, Total	Complaints per 1,000 Members (Total: 240,000)
Quality of Care	203	0.68	185	0.77
Access	121	0.40	98	0.41
Attitude/Service	83	0.28	86	0.36
Billing/Financial	91	0.30	78	0.33
Quality of Practitioner Office Site	68	0.23	63	0.26
Total/Average	566	1.89	510	2.13

Element D: Nonbehavioral Opportunities for Improvement

The organization annually identifies opportunities for improvement, sets priorities and decides which opportunities to pursue based on analysis of the following information:

1. Member complaint and appeal data from Element C.
2. CAHPS survey results and/or QHP Enrollee Experience Survey results.

Summary of Changes**Clarifications**

- Deleted text under “Documentation” in the scope of review stating that NCQA reviews documentation identifying opportunities selected by the organization.
- Replaced “CAHPS 5.0H” with “CAHPS” in the Explanation.

Scoring

Met	Partially Met	Not Met
The organization meets 2 factors	The organization meets 1 factor	The organization meets 0 factors

Data source

Reports

Scope of review**Product lines**

This element applies to Renewal Surveys for all product lines.

Documentation

NCQA reviews the organization’s most recent and the previous year’s reports.

Look-back period

For Renewal Surveys: 24 months.

Explanation

This element may not be delegated.

Factors 1, 2: Identification of opportunities

To identify opportunities for improvement, set priorities and decide which opportunities to pursue, the organization uses the results of its:

- Analyses of complaints and appeals (Element C), **and**
- *For commercial, Medicare or Medicaid product lines:* Quantitative and qualitative analysis of CAHPS survey results.
- *For the Exchange product line:* Quantitative and qualitative analysis of QHP Enrollee Experience Survey results if it brings forward for Accreditation a product line.

The organization identifies at least one opportunity to pursue from the analysis of data for each factor.

The organization must include the results of analyses reviewed in Element C as well as those of CAHPS and QHP results with its documentation of identified opportunities so that NCQA may review the relationship between the analyses and the identified opportunities. NCQA only reviews and scores that the identified opportunities relate directly to the findings of the qualitative analysis.

Refer to *Appendix 5: Glossary* for the full definition of and requirements for *quantitative analysis* and *qualitative analysis*.

Exceptions

Factor 2 is NA for the commercial, Medicare and Medicaid product lines if:

- The organization does not have CAHPS data because the denominators were too small to report valid rates, **or**
- The organization does not meet the 15,000 minimum-enrollment threshold and decides not to submit CAHPS results. Refer to Section 1 of the *Policies and Procedures*.

Factor 2 is NA for the Exchange product line if:

- The organization does not bring forward an Exchange product line for Accreditation.
- The Centers for Medicare & Medicaid Services (CMS) has exempted the organization from conducting the QHP Enrollee Experience Survey.

The organization must provide documentation of the exemption.

Examples Identifying and selecting opportunities

Table 1: CAHPS data by category

CAHPS Composite	Previous Year	Current Measurement Year	Benchmark*
Claims Processing	2.40	2.45	2.56
Customer Service	2.25	2.12	2.67
Getting Care Quickly	2.15	2.11	2.33
Getting Needed Care	2.43	2.49	2.77
How Well Doctors Communicate	2.27	2.22	2.58
Rating of Doctor	2.41	2.46	2.47
Rating of Health Care	2.35	2.46	2.50
Rating of Health Plan	2.10	2.25	2.31
Rating of Specialist	2.50	2.55	2.53

*90th Percentile National Benchmark

Causal analysis

Assembling the team. The organization arranged a causal analysis meeting with the medical director and assistant medical director, representatives from the Member Services, Provider Relations, Quality Improvement and Claims departments and member volunteers. Participants reviewed complaints data and CAHPS data, as well as complaints within the categories.

Qualitative analysis

1. Both the complaint and appeal and CAHPS data (Getting Care Quickly and Getting Needed Care) showed that access to practitioners who provide primary care services is an issue, but only in specific geographic areas.
2. Most of the service complaints and appeals were about slow service in Member Services, which resulted from significant turnover in the department; in addition, the department began to use a new telephone system that had problems when it first became operational.

3. Complaints and appeals about physician communication are a concern; many are about not having enough practitioners who speak Spanish and Chinese. These populations continue to grow in the service area and are becoming a significant percentage of the organization's membership.

Identified opportunities

1. Recruit practitioners who provide primary care services to the geographic areas where analysis found that the member-to-practitioner ratio is below the standard.
2. Recruit Spanish- and Chinese-speaking practitioners in areas where a large number of members speak those languages and where the organization has received complaints.
3. Analyze Member Services staffing needs and increase staff, if appropriate.
4. Train Member Services staff in communication skills.
5. Recruit Spanish- and Chinese-speaking Member Services staff.
6. Develop and implement a program to help practitioners communicate with non-English-speaking patients.

Element E: Annual Assessment of Behavioral Healthcare and Services

Using valid methodology, the organization annually:

1. Evaluates behavioral healthcare member complaints and appeals for each of the five required categories.
2. Conducts a member experience survey.

Summary of Changes

Clarifications

- Replaced "CAHPS 5.0H" with "CAHPS" in the factor 2 Explanation.

Scoring	Met	Partially Met	Not Met
	The organization meets 2 factors	The organization meets 1 factor	The organization meets 0 factors

Data source Reports

Scope of review **Product lines**

This element applies to First Surveys and Renewal Surveys for all product lines.

NCQA reviews and scores this element for each product line brought forward for Accreditation; only the commercial and Exchange product lines may be combined.

Documentation

For First Surveys: NCQA reviews the organization's most recent annual data collection and member experience survey report.

For Renewal Surveys: NCQA reviews the organization's most recent and the previous year's annual data collection and member experience survey reports.

Look-back period	<i>For First Surveys:</i> At least once during the prior year. <i>For Renewal Surveys:</i> 24 months.
Explanation	Factor 1: Member complaints and appeals The organization collects data for complaints and appeals separately. The organization collects data on member complaints and appeals from the entire population of members who have used behavioral healthcare services, or draws statistically valid samples from that population of members. If the organization uses a member sample, it describes the sample universe and the sampling methodology. The organization collects data from all sources of members' complaints and appeals, including: • Noncoverage appeals addressed in ME 7, Element A: Policies and Procedures for Complaints and ME 7, Element B: Policies and Procedures for Appeals. • UM coverage appeals addressed in <i>UM 8: Policies for Appeals</i> and <i>UM 9: Appropriate Handling of Appeals</i>. The organization aggregates all complaints and appeals into the following required categories: • Quality of Care. • Access. • Attitude and Service. • Billing and Financial Issues. • Quality of Practitioner Office Site. The organization may use a different coding system for internal purposes, but it must aggregate all complaint and appeal data in the reporting categories above for NCQA evaluation purposes. The organization reports results on each category, even if there are no complaints or appeals for a category. Factor 2: Member experience survey The organization identifies the population, sample size, sampling technique, method of administration (e.g., telephone or mail, administered by an outside vendor or by the organization) and response rates. The organization's report shows member experience results by product line, even if the response rate is low. A separate member experience survey for each product line is not required. An experience survey that focuses on limited populations (e.g., hospitalized patients, users of partial hospitalization programs, members in a specific geographic area) does not meet the requirements of this element. The CAHPS survey does not meet this factor; however, supplemental survey questions regarding behavioral healthcare may meet this factor if a question identifies members who have accessed behavioral healthcare services. Exception This element is NA if all purchasers of the organization's services carve out or exclude behavioral healthcare services.

Examples**Complaint and appeal types***Quality of Care*

- Dissatisfaction with care provided by a behavioral healthcare practitioner.

Access

- Denial of visits to a nonparticipating practitioner resulting from the member's difficulty finding or scheduling visits with a participating provider.

Attitude and Service

- Failure to release medical records.
- Long office wait time.

Billing and Financial Issues

- Denials due to incorrect coding by practice staff.

Quality of Practitioner Office Site

- Exam rooms do not provide enough privacy.
- Inadequate seating in the waiting room.
- The treatment setting is not safe.

Element F: Behavioral Healthcare Opportunities for Improvement

The organization works to improve members' experience with behavioral healthcare and service by annually:

1. Assessing data from complaints and appeals or from member experience surveys.
2. Identifying opportunities for improvement.
3. Implementing interventions, if applicable.
4. Measuring effectiveness of interventions, if applicable.

Summary of Changes**Clarifications**

- Added factor 2 as an exception if no opportunities are identified.

Scoring

Met	Partially Met	Not Met
The organization meets 3-4 factors	The organization meets 2 factors	The organization meets 0-1 factors

Data source

Documented process, Reports

Scope of review**Product lines**

This element applies to Renewal Surveys for all product lines.

Documentation

NCQA reviews the organization's most recent and the previous year's reports, interventions and effectiveness evaluation.

For factor 3, depending on the action taken, NCQA also reviews a documented process.

Look-back period	<i>For Renewal Surveys:</i> 24 months.
Explanation	Factor 1: Quantitative and qualitative analysis The organization analyzes data collected from either ME 7, Element E factor 1 or factor 2. <i>For initial measurement</i>, the organization conducts quantitative and qualitative analysis of data. <i>For remeasurement</i>, the organization conducts quantitative analysis and conducts qualitative analysis if quantitative analysis demonstrates that stated goals were not met. Refer to <i>Appendix 5: Glossary</i> for the full definition of and requirements for <i>quantitative analysis</i> and <i>qualitative analysis</i>. Factor 2: Identification of opportunities The organization identifies at least two opportunities to pursue from the analysis of data. The identified opportunities relate directly to the findings of the qualitative analysis. Factor 3: Implementing interventions Interventions are of sufficient strength and specificity that there is the likelihood they contribute to a measurable improvement and relate directly to causes of dissatisfaction identified in the analysis. Factor 4: Measuring effectiveness The organization measures the effectiveness of interventions to determine whether the interventions improved performance. The organization may measure effectiveness by repeating the original measurement or may use defined variables to measure performance of identified issues, collecting data on one of the following: 1. Activities. 2. Events. 3. Occurrences. 4. Outcomes. Exceptions This element is NA if all purchasers of the organization's services carve out or exclude behavioral healthcare services. Factors 2, 3 and 4 are NA if the organization's assessment does not identify opportunities for improvement. NCQA evaluates whether this conclusion is reasonable, given assessment results.

Examples Assessing and improving members' experience**Table 1: Annual complaint data (complaints per 1,000 members)**

Category	Previous Year	Current Year	Performance Goals	Performance Goals Met?
Quality of Care	4.50	4.07	<3.0	No
Access	3.31	4.36	<3.0	No
Attitude/Service	2.91	3.73	<3.0	No
Billing/Financial	2.51	2.42	<3.0	Yes
Quality of Practitioner Office Site	1.25	1.31	<3.0	Yes
<i>Total Average</i>	2.9	3.18	<3.0	NA

Table 2: Annual complaints data (total member complaints)

Category	Previous Year	Current Year	Performance Goal (% Volume Reduction)	Percentage Change
Quality of Care	1,462	1,323	20%	-10%
Access	1,075	1,175	20%	9%
Attitude/Service	946	1,121	25%	18%
Billing/Financial	817	785	20%	-4%
Quality of Practitioner Office Site	431	413	20%	-4%
<i>Total</i>	1,462	1,323	20%	-10%

ME 8: Delegation of ME

If the organization delegates any ME activities, there is evidence of oversight of the delegated activities.

Intent

The organization remains responsible for and has appropriate structures and mechanisms to oversee delegated Member Experience functions.

Element A: Delegation Agreement

The written delegation agreement:

1. Is mutually agreed upon.
2. Describes the delegated activities and the responsibilities of the organization and the delegated entity and the delegated activities.
3. Requires at least semiannual reporting by the delegated entity to the organization.
4. Describes the process by which the organization evaluates the delegated entity's performance.
5. Describes the process for providing member experience and clinical performance data to its delegates when requested.
6. Describes the remedies available to the organization if the delegated entity does not fulfill its obligations, including revocation of the delegation agreement.

Summary of Changes

Clarifications

- Revised the first sentence of the second paragraph under "Documentation" in the scope of review to clarify that the process from factor 5 must be included in the organization's documentation.
- Replaced "CAHPS 5.0H" with "CAHPS" in the factor 5 Explanation.

Scoring

Met	Partially Met	Not Met
The organization meets 5-6 factors	The organization meets 3-4 factors	The organization meets 0-2 factors

Data source Materials

Scope of review **Product line**

This element applies to Interim Surveys, First Surveys and Renewal Surveys for all product lines.

Documentation

NCQA reviews delegation agreements in effect during the look-back period from up to four randomly selected delegates, or reviews all delegates if the organization has fewer than four.

For delegation agreements in place prior to January 1, 2019, the organization may provide documentation that it notified the delegate of the process required in factor 5. This documentation of notification is not required to be mutually agreed upon.

Delegation agreements implemented on or after January 1, 2019, must include a description of the process required in factor 5.

The score for the element is the average of the scores for all delegates.

Look-back period

For Interim Surveys and First Surveys: 6 months.

For Renewal Surveys: 24 months.

Explanation

This element may not be delegated.

This element applies to agreements in effect within the look-back period.

The delegation agreement describes all delegated ME activities. A generic policy statement about the content of delegated arrangements does not meet this element.

Factor 1: Mutual agreement

Delegation activities are mutually agreed on or before delegation begins, in a dated, binding document or communication between the organization and the delegated entity.

NCQA considers the effective date specified in the delegation agreement as the mutually agreed-upon effective date. The effective date may be before or after the signature date on the agreement. If the agreement has no effective date, NCQA considers the signature date (meaning the date of last signature) as the mutually agreed upon effective date.

NCQA may accept other evidence of the mutually agreed-upon effective date: a letter, meeting minutes or other form of communication between the organization and the delegate that references the parties' agreement on the effective date of delegated activities.

NCQA requires submitted evidence for all other delegation factors to consider the same mutually agreed-upon date as the effective date for the delegate's performance of delegated activities.

Factor 2: Assigning responsibilities

The delegation agreement or an addendum thereto or other binding communication between the organization and the delegate specifies the ME activities:

- Performed by the delegate.
- Not delegated, but retained by the organization.
 - The organization may include a general statement in the agreement addressing the retained functions (e.g., the organization retains all other ME functions not specified in this agreement as the delegate's responsibility).

If the delegate subdelegates an activity, the delegation agreement must specify that the delegate or the organization is responsible for subdelegate oversight.

Factor 3: Reporting

The organization determines the method of reporting and the content of the reports, but the agreement must specify:

- That reporting is at least semiannual.
- What information is reported by the delegate about delegated activities.

- How, and to whom, information is reported (i.e., joint meetings or to appropriate committees or individuals in the organization).

The organization must receive regular reports from all delegates, including NCQA-Accredited or NCQA-Certified delegates.

Factor 4: Performance monitoring

The delegation agreement states the organization's process for monitoring and evaluating the delegate's performance.

Factor 5: Providing member and clinical data

The organization's delegation agreement describes what the delegate must do to obtain the following information when it is needed or on an ongoing basis:

- *Member experience data:* Complaints, CAHPS survey results or other data collected on members' experience with the delegate's services.
- *Clinical performance data:* HEDIS measures, claims and other clinical data collected by the organization. The organization may provide data feeds for relevant claims data or clinical performance measure results.

Factor 6: Consequences for failure to perform

The delegation agreement specifies consequences if a delegate fails to meet the terms of the agreement and, at a minimum, circumstances that would cause revocation of the agreement.

Exceptions

This element is NA if the organization does not delegate ME activities.

The clinical performance aspect of factor 5 is NA if clinical performance data are not relevant to the delegated activities.

Examples None.

Element B: Predelegation Evaluation

For new delegation agreements initiated in the look-back period, the organization evaluates delegate capacity to meet NCQA requirements before delegation began.

Summary of Changes

- No changes to this element.

Scoring

Met	Partially Met	Not Met
The organization evaluated delegate capacity before delegation began	The organization evaluated delegate capacity after delegation began	The organization did not evaluate delegate capacity

Data source Reports

Scope of review **Product line**

This element applies to Interim Surveys, First Surveys and Renewal Surveys for all product lines.

This element applies if delegation was implemented in the look-back period.

Documentation

NCQA reviews the organization's predelegation evaluation from up to four randomly selected delegates, or reviews all delegates if the organization has fewer than four.

The score for the element is the average of the scores for all delegates.

Look-back period

For Interim Surveys and First Surveys: 6 months.

For Renewal Surveys: 12 months.

Explanation

This element may not be delegated.

NCQA-Accredited/Certified delegates

Automatic credit is available if all delegates are NCQA-Accredited health plans, MBHOs or NCQA-Certified HIP organizations, unless the element is NA. NCQA-Certified HIPs must be certified in the activity being delegated by the organization.

Predelegation evaluation

The organization evaluated the delegate's capacity to meet NCQA requirements within 12 months prior to implementing delegation. The evaluation may include a review of the organization's structure, processes, and staffing in order to determine its capability to perform the delegated function.

NCQA considers the date of the agreement to be the implementation date if the delegation agreement does not include an implementation date.

If the time between the predelegation evaluation and implementation of delegation exceeds 12 months, the organization conducts another predelegation evaluation.

If the organization amends the delegation agreement to include additional ME activities within the look-back period, it performs a predelegation evaluation for the additional activities.

Exceptions

This element is NA if:

- The organization does not delegate ME activities.
- Delegation arrangements have been in effect for longer than the look-back period.

Examples**Predelegation evaluation**

- Site visit.
- Telephone consultation.
- Documentation review.
- Committee meetings.
- Virtual review.

Element C: Review of Performance

For delegation arrangements in effect for 12 months or longer, the organization:

1. Semiannually evaluates regular reports, as specified in Element A.
2. Annually evaluates delegate performance against NCQA standards for delegated activities.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 2 factors	The organization meets 1 factor	The organization meets 0 factors
Data source	Reports		
Scope of review	Product line <i>This element applies to First Surveys and Renewal Surveys for all product lines.</i> Documentation NCQA reviews evidence of the organization's review from up to four randomly selected delegates, or all delegates if the organization has fewer than four. <i>For First Surveys:</i> NCQA reviews the organization's most recent semiannual and annual performance evaluation reports completed during the look-back period. <i>For Renewal Surveys:</i> NCQA reviews the organization's four most recent semi-annual reports and the most recent and previous performance evaluation reports completed during the look-back period. The score for the element is the average of the scores for all delegates.		
Look-back period	<i>For First Surveys:</i> At least once during the prior year. <i>For Renewal Surveys:</i> 24 months.		
Explanation	This element may not be delegated. NCQA-Accredited/Certified delegates Automatic credit is available for factor 2 if all delegates are NCQA-Accredited health plans, MBHOs or NCQA-Certified HIP organizations, unless the element is NA. NCQA-Certified HIPs must be certified in the activity being delegated by the organization. Factor 1: Evaluation of reports No additional explanation required. Factor 2: Annual evaluation Annual evaluation is based on the delegation agreement and the appropriate NCQA standards, and includes a review of the delegate's policies and procedures.		

Exceptions

The element is NA if:

- The organization does not delegate ME activities.
- Delegation arrangements have been in effect for less than 12 months.

Examples**Factor 2: Annual evaluation**

- Site visit.
- Telephone consultation.
- Documentation review.
- Committee meetings.
- Virtual review.

Element D: Opportunities for Improvement

For delegation arrangements that have been in effect for more than 12 months, at least once in each of the past 2 years, the organization identified and followed up on opportunities for improvement, if applicable.

Summary of Changes

- No changes to this element.

Scoring

Met	Partially Met	Not Met
The organization has acted on identified problems, if any, at least once in each of the past 2 years that the delegation arrangement has been in effect	The organization took inappropriate or weak action, or acted only in the past year	The organization has not acted on identified problems

Data source

Documented process, Reports, Materials

Scope of review**Product line**

This element applies to First Surveys and Renewal Surveys for all product lines.

Documentation

NCQA reviews reports of opportunities for improvement from up to four randomly selected delegates, or from all delegates, if the organization has fewer than four, and for evidence that the organization took appropriate action to resolve issues.

For First Surveys: NCQA reviews the organization's most recent annual review and follow-up on improvement opportunities.

For Renewal Surveys: NCQA reviews the organization's most recent and the previous year's annual reviews and follow-up on improvement opportunities.

The score for the element is the average of the scores for all delegates.

Look-back period

For First Surveys: At least once during the prior year.

For Renewal Surveys: 24 months.

Explanation This element may not be delegated.

NCQA-Accredited/Certified delegates

Automatic credit is available if all delegates are NCQA-Accredited health plans, MBHOs or NCQA-Certified HIP organizations, unless the element is NA. NCQA-Certified HIPs must be certified in the activity being delegated by the organization.

Identify and follow-up on opportunities

The organization uses information from its predelegation evaluation, ongoing reports, or annual evaluation to identify areas of improvement.

Exceptions

This element is NA if:

- The organization does not delegate ME activities.
- Delegation arrangements have been in effect for less than 12 months.
- The organization has no opportunities to improve performance.
 - NCQA evaluates whether this conclusion is reasonable, given assessment results.

Examples None.

Long-Term Services and Supports

Standards for Long-Term Services and Supports

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LTSS 1: Core Features

Refer to Appendix 1 for points

The organization has processes in place that organizations can use as a foundation for coordinating long-term services and supports (LTSS).

Intent

The organization uses current best practices to coordinate LTSS to eligible members.

Element A: Program Description

The description of the organization's case management program includes:

1. Criteria for identifying members who are eligible for the program.
2. Services offered to members.
3. Evidence and professional standards used for program operations.*
4. Defined program goals.
5. How case management services are coordinated with the services of others involved in members' care.

**Critical factors: These factors must be scored "yes" to score at least "Partially Met."*

Summary of Changes

Clarifications

- Updated the organization names and alphabetically reordered the factor 3 examples.

Scoring	Met	Partially Met	Not Met
	The organization meets 4-5 factors	The organization meets 2-3 factors	The organization meets 0-1 factors
Data source	Documented process		
Scope of review	Product lines <i>This element applies to First Surveys and Renewal Surveys for all product lines.</i> Documentation NCQA reviews the organization's program description. NCQA scores this element for each program the organization brings forward for distinction. The score for the element is the average of the scores for all programs.		
Look-back period	<i>For First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	This element may not be delegated. Factor 3 is a critical factor; this factor must be scored "yes" for the organization to score at least "Partially Met" on this element. Case management is a collaborative process of assessment, planning, facilitation, coordination, evaluation and advocacy for supports and services to meet the needs of a member while promoting quality and cost-effective outcomes.		

The overall goal of case management LTSS programs is to help members function optimally in their preferred setting.

Factor 1: Eligibility criteria

A **purchaser** is an entity (e.g., state, health plan) that purchases services provided by the organization.

The program description states the eligibility criteria for the case management program. Eligibility criteria may be set by the purchaser. NCQA does not require the organization to use specific criteria.

Factor 2: Services

The program description specifies services available to eligible members across all programs the organization brings forward for distinction. The organizations may provide the services directly or may arrange for the services to be provided by other entities.

Factor 3: Evidence and professional standards

Professional standards of care are stated ethical or legal requirements to exercise the level of care, diligence and skill prescribed in a profession's code of practice.

The program description specifies the evidence and professional standards the organization uses to determine which services it offers to members and how it provides services.

Evidence derives from:

- Scientific evidence from technical literature or government research sources.
- Literature reviews on best practices (e.g., motivational interviewing, methods to improve health literacy).

Evidence includes:

- Guidelines.
- Statements of recommendation.
- Algorithms or materials created through an unbiased and transparent process of systematic review, appraisal and best practices to aid in the delivery of optimum care.

The program description includes professional standards that may be derived from:

- MCOs or state waiver requirements.
- Standardized techniques.
- Specialized models (e.g., chronic care, patient-centered care nursing model).

Factor 4: Program goals

The program description includes the organization's desired level of achievement expressed in explicit, measurable objectives and targets for the case management program. Goals must go beyond mission and vision statements.

Factor 5: Case management coordination

Coordination is essential to optimizing care delivery for members receiving services through multiple programs. The program description includes how care is coordinated with other programs and services the member is receiving outside the case management organization.

Exceptions

None.

Related information

Factors 2, 3: Services and evidence of professional standards. The organization may have a contractual relationship with a state or other purchaser that specifies services it must offer or tools it must use to operate the program (e.g., a specific needs assessment tool). In this case, the organization provides requirements of the state or purchaser and presents evidence for the models of care or tools it uses for program components that are not dictated by contractual or regulatory requirements.

Examples**Factor 2: Services**

- Care coordination, including arranging appointments and referrals to community resources.
- Case management plan development, with person-centered goals.
- Self-management plan development and monitoring.
- Self-directed services.
- Personal care assistance.
- Housekeeping and chore services.
- Money management.
- Transportation.
- Housing-related services.

Factor 3: Evidence and professional standards

The organization uses a combination of evidence and professional standards to support staff interactions with members, which may be derived from its research or from research organizations such as:

- Administration for Community Living.
- Administration on Aging.
- ADvancing States (formerly National Association of States United for Aging and Disabilities).
- American Case Management Association.
- American Nurse Association Guidelines.
- American Society on Aging.
- Association for Behavioral Analysis International.
- Case Management Society of America.
- Leadership Council of Aging Organizations.
- National Association of Social Workers.
- National Association of State Directors of Developmental Disabilities Services.

- National Coalition of Care Coordination.
- National Core Indicators—Aging and Disabilities.
- National Council on Independent Living.
- The John A. Hartford Foundation.
- The SCAN Foundation.

Factor 4: Program goals

- 30% of participants served for 6 months or longer report that their quality of life improved since the initial assessment.
- Improve member experience with the program by 20% during the calendar year.
- 85% of members served in the community for at least 6 months have no long-term nursing home stays.
- Reduce 30-day hospital readmissions by 10%.
- Increase rates of supported employment by 25%.

Factor 5: How case management services are coordinated

Coordination of case management program and services may include:

- Medical providers, including palliative care providers.
- Behavioral healthcare providers.
- Social services providers, such as housing, employment supports, nutritional assistance.
- Case managers from other organizations the member is affiliated with (discharge care managers, health plan care managers).
- Caregivers.

Element B: Assessment of Health, Functioning and Communication Needs

The organization's case management process includes the following assessments:

1. Health status, including condition-specific issues.
2. Clinical history, including medications.
3. Activities of daily living, including use of supports.
4. Instrumental activities of daily living, including use of supports.
5. Behavioral health status.
6. Cognitive functioning.
7. Social determinants of health.
8. Social functioning.
9. Health beliefs and behaviors.
10. Cultural and linguistic needs, preferences or limitations.
11. Visual and hearing needs, preferences or limitations.
12. Physical environment for risk.

Summary of Changes

Clarifications

- Revised the look-back period for Renewal Surveys from 12 months to 24 months and deleted the text regarding “current medications, including schedules and dosages” for factors 1 and 2.

Scoring	Met	Partially Met	Not Met
	The organization meets 6-12 factors	The organization meets 4-5 factors	The organization meets 0-3 factors

Data source Documented process

Scope of review **Product lines**
This element applies to First Surveys and Renewal Surveys for all product lines.

Documentation

NCQA reviews the organization's policies and procedures in place throughout the look-back period.

Look-back period *For First Surveys: 6 months.*
For Renewal Surveys: 24 months.

Explanation This element is a **structural requirement**. The organization presents its own documentation.

At a minimum, case management policies and procedures address how the organization documents and confirms that assessments have been completed. Assessments are not required to take place in a single encounter.

If another organization conducts assessments, the case management process explains how the organization obtains and documents the information.

If a state-mandated tool is used for assessments, the organization documents how factors not addressed by the tool are evaluated.

HEDIS LTSS measures

Organizations may submit performance results on the Comprehensive Assessment and Update (LTSS-CAU) measure instead of providing a documented process. If an organization chooses this voluntary option, it must meet the 5 percent threshold rate on the measure to receive credit.

Assessment

Assessment requires the case manager or other qualified individual to reach and document a conclusion about data or information collected. If the organization's CM system automatically generates suggestions, the case manager or other individual must still document their own conclusions. It is not sufficient to just have raw data or answers to questions. Policies describe the process to both collect information and document a summary of the meaning or implications of that data or information to the member's situation, so that it can be used in the case management plan.

The organization must reach a conclusion for each factor (unless otherwise stated in the explanation). This may be in separate summaries for each factor or in a combined summary, or in a combination of these.

Case management policies and procedures state why an assessment might not be appropriate for a factor (e.g., life-planning activities, in pediatric cases). The organization records the specific factor and the reason in the case management system and file.

Factor 1: Assessment of members' health status

Case management policies and procedures specify a process for assessing a member's health status, including active diagnoses. Assessment includes:

- Screening for presence or absence of physical conditions and their current status.
- The member's self-reported health status.
- Current medications, including schedules and dosages.

Factor 2: Documentation of clinical history

Case management policies and procedures specify a process for documenting clinical history, including:

- Past hospitalization and major procedures, including surgery.
- Significant past illnesses and treatment history.
- Relevant past medications.

The information may be reported by the member or collected from another source.

Factor 2 does not require assessment or evaluation.

Factor 3: Assessment of activities of daily living

Case management policies and procedures specify a process for assessing functional status related to activities of daily living, such as eating, bathing and mobility. Supports include both assistive technology and human assistance needed to complete a certain activity.

Factor 4: Assessment of instrumental activities of daily living

Case management policies and procedures specify a process for assessing functional status related to instrumental activities of daily living, such as housekeeping, money management and ability to navigate transportation. Supports include both assistive technology and human assistance needed to complete a certain activity.

Factor 5: Assessment of behavioral health status

Case management policies and procedures specify a process for assessing behavioral health status, including:

- Mental health conditions.
- Substance use disorders.

Factor 6: Cognitive functioning

Case management policies and procedures specify the process for assessing cognitive function, including:

- The member's ability to communicate and understand instructions.
- The member's ability to process information about an illness.

Factor 7: Assessment of social determinants of health

Social determinants of health are economic and social conditions that affect a wide range of health, functioning and quality-of-life outcomes and risks. Case management policies and procedures specify a process for identifying social determinants of health that may affect a member's ability to meet goals.

Because social determinants of health are a combination of influences, the organization considers more than one social determinant of health, for a comprehensive overview of the member's health.

Factor 8: Assessment of social functioning

Social functioning refers to an ability to interact easily and successfully with other people. Case management policies and procedures specify a process for assessing social functioning that may affect a member's mental and physical health.

Factor 9: Assessment of health beliefs and behaviors

Health beliefs and behaviors may reflect cultural and social beliefs about health problems, perceived benefits of action and barriers to action. Case management policies and procedures specify a process for assessing health beliefs and behaviors (e.g., optimism, self-efficacy, nutrition habits, physical activity and alcohol and tobacco use) that could improve or impede a member's ability to adhere to the case management plan.

Factor 10: Assessment of cultural and linguistic needs

Case management policies and procedures specify a process for assessing culture and language to identify potential needs or barriers to effective communication or care and acceptability of specific treatments. Policies and procedures also include consideration of cultural health beliefs and practices, preferred languages, health literacy and other communication needs.

Factor 11: Assessment of visual and hearing needs

Case management policies and procedures specify a process for assessing vision and hearing needs to identify potential needs or barriers to effective communication, care or well-being.

Factor 12: Assessment of risk in physical environment

Case management policies and procedures specify a process for assessing a member's physical environment to identify risks.

Exceptions

None.

Examples**Factor 2: Clinical history**

- Past inpatient stays.
- Medication list.
- Symptom history.
- Exposure to toxins.
- Health care providers regularly seen by the member.

Factor 3: Activities of daily living

- Grooming.
- Dressing.
- Bathing.
- Toileting.
- Eating.
- Transferring (e.g., getting in and out of chairs).
- Continence.
- Walking.

Factor 4: Instrumental activities of daily living

- Managing finances.
- Shopping.
- Preparing meals.
- Managing medications.
- Housework and basic home maintenance.
- Handling transportation (driving or navigating public transit).
- Using the telephone and other communication devices.

Factor 5: Behavioral health status

- Substance use disorders.
- Suicidal ideation.
- Depression.
- Psychosis.

Factor 6: Cognitive functioning

- Alert/oriented, able to focus and shift attention, comprehends and recalls direction independently.
- Requires prompting (cuing, repetition, reminders) only under stressful situations or unfamiliar conditions.
- Requires assistance and some direction in specific situation (e.g. on all tasks involving shifting attention) or consistently requires low stimulus environment due to distractibility.
- Requires considerable assistance in routine situations. Is not alert and oriented or is unable to shift attention and recall directions more than half the time.
- Totally dependent due to disturbances such as constant disorientation, coma, persistent vegetative state or delirium.

Factor 7: Social determinants of health

- Current housing and housing security.
- Access to local food markets.
- Exposure to crime, violence and social disorder.
- Residential segregation and other forms of discrimination.
- Access to mass media and emerging technologies.
- Social support, norms and attitudes.
- Access to transportation and financial barriers to obtaining treatment.

Factor 8: Social functioning

- Engagement with friends and family.
- Social isolation.
- Employment status.

Factor 9: Health beliefs and behaviors

- Optimism.
- Self-efficacy.
- Physical activity.
- Smoking.
- Alcohol use.
- Medication adherence.
- Beliefs and concerns about the condition or services the member is receiving.

Factor 10: Cultural and linguistic preferences

- Health care treatments or procedures discouraged or forbidden by religious or spiritual beliefs.
- Family traditions related to decision-making, illness, death and dying.
- Health literacy assessment.

Factor 11: Visual and hearing needs

- Visual impairment and need for, or use of, visual aids (e.g., talking clocks, large-font prescription labels).
- Hearing impairment and need for, or use of, hearing aids or other supports or devices (e.g., sign language interpreters, bed shakers).

Factor 12: Physical environment for risk

- Home-based initial assessment for fall risks, medication risks, accessibility of exits, access to emergency assistance (e.g., telephone, medical alert service).

Element C: Resource Assessment

The organization's case management process specifies assessment of the following resources:

1. Paid and unpaid caregiver resources, involvement and needs.
2. Available benefits within the organization.
3. Community resources.

Summary of Changes**Clarifications**

- Deleted "of requested activities of daily living/instrumental activities of daily living" and consolidated the second and third sentences under "Factor 1: Assessment of caregiver resources."
- Consolidated the first and second sentences under "Factor 2: Assessment of available benefits."

Scoring	Met	Partially Met	Not Met
	The organization meets 3 factors	The organization meets 2 factors	The organization meets 0-1 factors
Data source	Documented process		
Scope of review	Product lines		
	<i>This element applies to First Surveys and Renewal Surveys for all product lines.</i> Documentation NCQA reviews the organization's policies and procedures for assessing resources in place throughout the look-back period.		
Look-back period	<i>For First Surveys: 6 months.</i>		
	<i>For Renewal Surveys: 24 months.</i>		
Explanation	This element is a structural requirement. The organization presents its own documentation. At a minimum, case management policies and procedures address how the organization documents and confirms that assessments have been completed for each member. Assessments are not required to take place in a single encounter. If a state-mandated tool is used for the assessment, the organization documents how factors not addressed by the tool are evaluated. HEDIS LTSS measures Organizations may submit performance results on the Comprehensive Assessment and Update (LTSS-CAU) measure instead of providing a documented process. If an organization chooses this voluntary option, it must meet the 5 percent threshold rate on the measure to receive credit. Assessment Assessment requires the case manager or other qualified individual to reach and document a conclusion about data or information collected. If the organization's CM system automatically generates suggestions, the case manager or other individual must still document their own conclusions. It is not sufficient to just have raw data or answers to questions. Policies describe the process to both collect information and document a summary of the meaning or implications of that data or information to the member's situation, so that it can be used in the case management plan. The organization must reach a conclusion for each factor (unless otherwise stated in the explanation). This may be in separate summaries for each factor or in a combined summary, or in a combination of these. Case management policies and procedures state why an assessment might not be appropriate for a factor (e.g., life-planning activities, in pediatric cases). The organization records the specific factor and the reason in the case management system and file. Factor 1: Assessment of caregiver resources Case management policies and procedures specify a process for assessing the adequacy of paid and unpaid caregiver resources (e.g., family involvement in the		

case management plan and in carrying out the plan). Evaluation assesses availability, skills and caregiver capacity to provide support and considers and anticipates undue burden on the caregiver (e.g., unreasonable stress or strain) and caregiver support needs (e.g., training, respite).

Factor 2: Assessment of available benefits

Case management policies and procedures specify a process for assessing the adequacy of health benefits and available resources to fulfill the case management plan.

Factor 3: Assessment of community resources

Case management policies and procedures specify a process for assessing eligibility for supplemental community resources (e.g. organizations, facilities, services) that can address social determinants of health or other needs or barriers identified in the assessment. The assessment includes a determination of eligibility for supplemental community resources.

Exceptions

None.

Examples

Factor 1: Caregiver resources

- The member is independent and does not need caregiver assistance.
- Number of caregivers, and current assistance provided.
- Caregiver needs for training supportive services.

Factor 2: Available benefits

- Benefits covered by the organization and by providers.
- Services carved out by the purchaser.
- Services that supplement those the organization is contracted to provide, such as:
 - Community mental health.
 - Subsidized housing.
 - Palliative care programs.

Factor 3: Community resources

- Community mental health.
- Vocational programs.
- Volunteer companion services.
- Government aid (e.g., food stamps, maternal-child health programs, housing assistance).
- Senior centers.
- Adult day care.
- Support groups.
- Poverty outreach groups.
- Housing resources.
- Legal aid.
- Palliative care programs.

Element D: Comprehensive Assessment Implementation

An NCQA review of a sample of the organization's case management files demonstrates that the organization follows its documented processes for assessing:

1. Health status, including condition-specific issues.
2. Clinical history, including medications.
3. Activities of daily living, including use of supports.
4. Instrumental activities of daily living, including use of supports.
5. Behavioral health status.
6. Cognitive functioning.
7. Social determinants of health.
8. Social functioning.
9. Health beliefs and behaviors.
10. Cultural and linguistic needs, preferences or limitations.
11. Visual and hearing needs, preferences or limitations.
12. Physical environment for risk.
13. Paid and unpaid caregiver resources, involvement and needs.
14. Available benefits within the organization.
15. Community resources.

Summary of Changes

Clarifications

- Consolidated the first and second sentences in the first paragraph under "Factor 15: Assessment of community resources."

Scoring	Met	Partially Met	Not Met
	High (90-100%) on file review for at least 8 factors and medium (60-89%) on file review for any remaining factors	Low (0-59%) on file review for no more than 5 factors and high (90%-100%) or medium (60-89%) on file review for any remaining factors	Low (0-59%) on 6 or more factors

Data source Records or files

Scope of review **Product lines**

This element applies to First Surveys and Renewal Surveys for all product lines.

Documentation

NCQA reviews assessments in a random sample of up to 40 case management files. Files are selected from active or closed cases that were identified during the look-back period and remained open for at least 60 calendar days during the look-back period, from the date when the member was identified for case management.

	The organization must provide the identification date for each case in the file universe.
Look-back period	<i>For First Surveys:</i> 6 months. <i>For Renewal Surveys:</i> 12 months.
Explanation	This element evaluates the organization's assessment of health status, functioning, communication needs and resources, according to the policies evaluated in Element B and Element C.

HEDIS LTSS measures

Organizations may submit performance results on the Comprehensive Assessment and Update (LTSS-CAU) measure instead of completing the file review. If an organization chooses this voluntary option, it must meet the 90 percent threshold rate on the measure to receive credit.

Documentation to meet the factors includes evidence that assessments were completed and results were documented. A checklist of assessments without documentation of results does not meet the requirement.

Assessment components may be completed by other members of the care team and with the assistance of the member's family or caregiver. Assessment results for each factor must be clearly documented in case management notes, even if a factor does not apply.

If the member is unable to communicate because of infirmity, assessment may be completed by professionals on the care team, with assistance from the patient's family or caregiver.

If case management stops when a member is admitted to a facility and the stay is longer than 30 calendar days, a new assessment must be performed after discharge if the member is identified for case management.

Dispute file review results

Onsite file review is conducted in the presence of the organization's staff. The survey team works to resolve disputes that arise during the onsite survey. In the event that a dispute cannot be resolved, the organization must contact NCQA before the end of the onsite survey. File review results may not be disputed or appealed once the onsite survey is complete.

Assessment

Assessment requires the case manager or other qualified individual to reach and document a conclusion about data or information collected. If the organization's CM system automatically generates suggestions, the case manager or other individual must still document their own conclusions. For factors that require assessment, raw data or answers to questions do not meet the requirement.

There is a documented summary of the information's meaning or its implications for the member's situation, for use in the case management plan.

The organization must reach a conclusion for each factor (unless otherwise stated in the explanation). This may be in separate summaries for each factor or in a combined summary, or in a combination of these.

Files excluded from review

The organization excludes files from review that meet these criteria:

- Eligible members whom it cannot locate or contact after three or more attempts across a 2-week period, within the first 30 calendar days after identification, through at least two of the following mechanisms:
 - Telephone.
 - Regular mail.
 - Email.
 - Fax.
- Members enrolled in case management for less than 60 calendar days during the look-back period.
 - The organization provides evidence of the member's identification date and that the member was in case management for less than 60 calendar days during the look-back period.
- Employees of the organization and their dependents.

Files that meet these criteria and are inadvertently included in the organization's file review are scored NA for all factors. NCQA confirms that the files met the criteria for an NA score.

Factor 1: Assessment of member's health status

The file or case record documents a case manager's assessment of the member's current health status, including:

- Information on presence or absence of comorbidities and their current status.
- Self-reported health status.
- Current medications, including dosages and schedule.

Factor 2: Documentation of clinical history

The file or case record contains information on the member's clinical history, including:

- Past hospitalization and major procedures, including surgery.
- Significant past illnesses and treatment history.
- Past medications.

Dates are a necessary component of accurate documentation of the member's clinical history. To the extent possible, the organization collects dates as part of documenting clinical history; however, NCQA does not penalize an organization if a member or other individual providing the information cannot provide dates.

Factor 2 does not require assessment or evaluation.

Factor 3: Assessment of activities of daily living

The file or case record documents the results of the assessment of activities of daily living.

For activities the member needs assistance with, the case manager documents the reason and type of assistance. The case manager is not required to describe activities for which the member does not need assistance.

If the member needs no assistance with activities of daily living, the file or case record documents this (e.g., "Member is fully independent").

Factor 4: Assessment of instrumental activities of daily living

The file or case record documents the results of the assessment of instrumental activities of daily living.

For activities the member needs assistance with, the case manager documents the reason and type of assistance. The case manager is not required to describe activities for which the member does not need assistance.

If the member needs no assistance with instrumental activities of daily living, the file or case record documents this (e.g., “Member is fully independent”).

Factor 5: Assessment of behavioral health status

The file or case record documents the case manager’s assessment of:

- Mental health conditions.
- Substance use disorders.

Factor 6: Assessment of cognitive functioning

The file or case record documents the case manager’s assessment of:

- Cognitive functions.
 - The member’s ability to communicate and understand instructions.
 - The member’s ability to process information about an illness.

Factor 7: Assessment of social determinants of health

The case manager assesses social determinants of health, which are economic and social conditions that affect a wide range of health, functioning and quality-of-life outcomes, and assesses risks that may affect a member’s ability to meet goals.

Because social determinants of health are a combination of influences, the organization considers more than one social determinant of health, for a comprehensive overview of the member’s health.

Factor 8: Assessment of social functioning

The file or case record documents the case manager’s assessment of social functioning that may affect a member’s mental and physical health.

Factor 9: Assessment of health beliefs and behaviors

The file or case record documents the case manager’s assessment of health beliefs and behaviors (e.g., optimism, self-efficacy, nutrition habits, physical activity and alcohol and tobacco use).

Factor 10: Assessment of cultural and linguistic needs

The file or case record documents the case manager’s assessment of the member’s culture and language needs and their impact on communication, care or acceptability of specific treatments. At a minimum, the case manager evaluates:

- Cultural health beliefs and practices.
- Preferred languages.

Factor 11: Assessment of visual and hearing needs

The file or case record documents the case manager’s evaluation of the member’s vision and hearing. The document describes specific needs to consider in the case management plan and barriers to effective communication or care.

Factor 12: Assessment of physical environmental risk

The file or case record documents the case manager's assessment of the member's physical environment and identifies risks.

Factor 13: Assessment of caregiver resources

The file or case record documents the case manager's assessment of the adequacy of caregiver resources (e.g., family involvement in and decision making about the care plan) whether paid or unpaid, during initial member assessment. Documentation describes the resources in place and whether they are sufficient for the member's needs, and notes specific gaps to address.

Factor 14: Assessment of available benefits

The file or case record documents the case manager's evaluation of the adequacy of the member's health insurance benefits in relation to the needs of the case management plan. The evaluation goes beyond checking insurance coverage; it includes a determination of whether the resources available to the member are adequate to fulfill the case management plan.

Factor 15: Assessment of community resources

The file or case record documents the case manager's evaluation of the member's eligibility for community resources and the resources the member may need, and their availability.

If the member needs no community resources, the file or case record reflects this (e.g., "Member does not need community resources").

Exceptions

None.

Examples

None.

Element E: Person-Centered Assessments

The organization has a process to:

1. Assess members' service needs.*
2. Assess members' prioritized goals.*
3. Assess members' preferences.*
4. Assess members' life planning activities.
5. Identify members' preferred method of communication.

**Critical factors: These factors must be scored "yes" to score at least "Partially Met."*

Summary of Changes

Clarifications

- Revised the look-back period for Renewal Surveys from 12 months to 24 months for factor 1.
- Added "Medical or Physician Orders of Life-Sustaining Treatment forms" to the first paragraph under "Factor 4: Assessment of life-planning activities."

Scoring	Met	Partially Met	Not Met
	The organization meets 4-5 factors	The organization meets 3 factors	The organization meets 0-2 factors
Data source	Documented process		
Scope of review	Product lines <i>This element applies to First Surveys and Renewal Surveys for all product lines.</i> Documentation NCQA reviews the organization's process for completing person-centered assessments.		
Look-back period	<i>For First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	This element is a structural requirement. The organization presents its own documentation. Factors 1, 2 and 3 are critical factors; all factors must be scored "yes" for the organization to score at least "Partially Met" on this element. HEDIS LTSS measures Organizations may submit performance results on the Comprehensive Care Plan and Update (LTSS-CPU) measure instead of providing a documented process. If an organization chooses this voluntary option, it must meet the 5 percent threshold rate on the measure to receive credit. Person-centered planning involves viewing, listening to and supporting members, based on their strengths, abilities, aspirations and preferences, to make decisions for maintaining a life that is meaningful to them. The resulting care plan reflects the goals and interests of the member. Members should be involved in the care planning process to the extent they prefer. A caregiver who is involved in the plan development process may contribute to discussions about goals and other aspects of the process, but may not define goals for a member. Factor 1: Assessment of service needs Case management policies and procedures specify a process for assessing which services the member needs. The assessment includes a documentation of the services the member receives. Factor 2: Assessment of prioritized goals A member's goals are the foundation of person-centered care planning and address a desired outcome. Case management policies and procedures specify a process for assessing members' goals and priorities, which the organization uses to develop a person-centered case management plan (assessed in Element F). Designating goals as long-term or short-term is not sufficient to meet the requirement. The organization must rank or prioritize goals.		

Factor 3: Assessment of preferences

Preferences refer to members' inclinations toward lifestyle, living situation and how care is to be provided. Case management policies and procedures specify a process for assessing personal preferences for how case management and LTSS are delivered.

Factor 4: Assessment of life-planning activities

A member's expressed preferences can influence a case management plan. Case management policies and procedures specify a process for assessing whether members have completed life-planning activities such as wills, living wills, advance directives, health care powers of attorney and Medical or Physician Orders of Life-Sustaining Treatment forms.

If life planning activities are determined to be appropriate, the case manager documents what activities the member has taken and what documents are in place. If life planning activities are determined not to be appropriate, the case manager documents the reason in the case management record or file. A designated representative can make decisions on behalf of members who are incapacitated and cannot communicate life-planning preferences.

Documentation that the organization provided life planning information (e.g., brochure, pamphlet) to all members in case management meets the intent of this requirement.

Factor 5: Preferred communication methods

Case management policies and procedures specify a process for assessing a member's preferred method to receive communications from the organization (e.g., email, text, mail, phone).

Exceptions

None.

Examples**Factor 1: Service needs**

- Modifications to the home.
- Transportation services.
- Meal delivery.

Factor 2: Goals

- Go back to school.
- Volunteer at the local shelter.
- Go out with friends.
- Continue to live in my own home.
- Go to church.
- Attend my card game.

Factor 3: Preferences

- Where to live.
- With whom to live.
- When to go to bed.
- When and what to eat.

- Whom to involve in care planning.
- Which services and service providers to use.

Factor 4: Life-planning activities

- Legal documents, including wills, health care power of attorney.
- Designation of people permitted to make decisions on behalf of the member, obtain information about the member's health status and services received, be notified about transitions in care.
- Providing life-planning information (e.g., brochure, pamphlet) to all members in case management.
- Plan for caregiver transition in the event of death or illness that would prevent the caregiver from assisting the member.

Element F: Person-Centered Care Planning Process

The organization's care planning procedures address:

1. Development of an individualized case management plan that includes services needed members' preferences and prioritized goals.*
2. Identification of barriers to meeting members' goals and preferences or implementing the plan.
3. Development of a schedule for follow-up and communication with members.
4. Development of a plan for follow-up and communication with LTSS providers.
5. Development of an emergency back-up plan.
6. Development of a self-management plan.
7. Facilitation of referrals to resources and a follow-up process to determine whether members acted on referrals.
8. A process to capture whether members received services identified in the case management plan.
9. A process to assess members' progress against case management plans, at least every 12 months.

***Critical factors:** These factors must be scored "yes" to score at least "Partially Met."

Summary of Changes

Clarifications

- Revised the look-back period for Renewal Surveys from 12 months to 24 months and deleted the text regarding "services needed" for factors 1 and 8.

Scoring	Met	Partially Met	Not Met
	The organization meets 8-9 factors	The organization meets 4-7 factors	The organization meets 0-3 factors

Data source Documented process

Scope of review **Product lines**

This element applies to First Surveys and Renewal Surveys for all product lines.

Documentation

NCQA reviews the organization's process for creating individualized, person-centered case management plans.

Look-back period

For First Surveys: 6 months.

For Renewal Surveys: 24 months.

Explanation

This element is a **structural requirement**. The organization presents its own documentation.

Factor 1 is a critical factor; this factor must be scored "yes" for the organization to score at least "Partially Met" on this element.

HEDIS LTSS measures

Organizations may choose to meet this element by submitting performance results on the Comprehensive Care Plan and Update (LTSS-CPU) measure instead of providing a documented process. If an organization chooses this voluntary option, it must meet the 5 percent threshold rate on the measure to receive credit.

Factor 1: Individualized case management plan

Case management policies and procedures specify a process for working with members to develop an individualized case management plan based on the services needed and the prioritized goals and preferences assessed in Element E. Designating goals as long-term or short-term is not sufficient to meet the requirement. The personalized case management plan meets member needs and includes:

- Services needed.
- Prioritized goals.
 - Prioritized goals consider member and caregiver needs and preferences; they may be documented in any order, as long as the level of priority is clear.
- Resources to be utilized, including appropriate level of care.
- Collaborative approaches to be used, including level of family participation.

Factor 2: Identification of barriers

Case management policies and procedures specify a process for assessing barriers to receipt of services or to implementing the agreed-on case management plan. The organization may use assessments completed in Element B and Element C to identify barriers. A barrier analysis can assess:

- Language or literacy level.
- Access to reliable transportation.
- Understanding of a condition.
- Motivation.
- Financial or insurance issues.
- Cultural or spiritual beliefs.
- Visual or hearing impairment.
- Psychological impairment.

The organization documents that it assessed for the presence of barriers, even if none are identified.

Factor 3: Follow-up schedule

Case management policies and procedures have a process for determining if follow-up is appropriate or necessary (e.g., after a member is referred to a disease management program or health resource). The case management plan contains a schedule for follow-up that includes, but is not limited to:

- Counseling.
- Member education.
- Frequency of in-person and telephone check-ins.
- Self-management support.
- Determining when follow-up is not appropriate.

Factor 4: Communication with LTSS providers

LTSS providers are paid and unpaid members and organizations that provide long-term services and supports. Case management policies and procedures specify the roles and responsibilities of LTSS providers, case management plan details that are communicated to providers and a process for developing a follow-up schedule.

Factor 5: Emergency back-up plans

Emergency back-up plans account for short-term and long-term needs, and may address circumstances such as temporary replacements for personal care attendants and how to respond to power outages that affect equipment. The case management plan includes an emergency back-up plan customized to the member.

Factor 6: Self-management plans

Self-management is a member's role in managing the effects, physical and social consequences and lifestyle changes inherent in living with a chronic condition or a functional limitation. **Self-management plans** are activities that help members manage a condition and are based on instructions or materials provided to them or to their caregivers.

Case management policies and procedures specify a process for communicating the self-management plan to the member or caregiver (i.e., verbally, in writing).

Factor 7: Referrals to resources

Case management policies and procedures specify a process for facilitating referrals to other available resources that may benefit members. The organization may assist in the referral process; but members choose whether to act on referrals.

Factor 8: Receipt of services

Case management policies and procedures specify a process for verifying whether the member has received the services included in the case management plan.

Factor 9: Assessing progress

Case management policies and procedures specify a process for assessing member progress toward overcoming barriers to care and to meeting treatment goals, and for assessing and adjusting the care plan and its goals, as needed, at least every 12 months. Time frames for reevaluation of goals are specified in the case management plan.

Exceptions

None.

Examples**Factor 2: Assessment of barriers**

- Does the member understand the condition and treatment?
- Does the member want to participate in the case management plan?
- Does the member believe that participation will improve health?
- Are there financial or transportation limitations that may hinder the member from participating in care?
- Does the member have the mental and physical capacity to participate in care?

Source: Lorig, K., Patient Education, A Practical Approach (Thousand Oaks, CA: Sage Publications, 2001) 186–92.

Factor 3: Follow-up schedule

- Monthly telephone check-ins.
- Biannual home visits to assess progress and update the plan.
- Planned telephone check-in after a scheduled intervention (e.g., home modification) to assess its impact.

Factor 5: Emergency back-up plans

- Whom to call in the event of equipment failure.
- What to do if power goes out.
- What to do in the event of a natural disaster.

Factor 6: Self-management plan

- Perform activities of daily living (e.g., transfer/ambulation, bathing, dressing, toileting, eating/feeding).
- Perform instrumental activities of daily living (e.g., meals, housekeeping, laundry, telephone, shopping, finances).
- Using adaptive equipment (wheel chair, walker, cane).
- Ability to monitor their condition.
- Reporting an exacerbation of a condition or change in caregiver availability that requires a change in services.
- Money management.
- Paperwork (e.g., annual assessments, financial redetermination, utility assistance, food benefits, transportation).
- Engaging in community resources.

Element G: Implementing the Care Planning Process

NCQA's review of a sample of the organization's case management files demonstrates that the organization follows its documented processes for:

1. Assessment of members' need for service needs.
2. Assessment of members' prioritized goals.
3. Assessment of members' preferences.
4. Development and communication of self-management plans.
5. Assessment of members' life-planning activities.
6. Identification of members' preferred method of communication.
7. Development of an individualized case management plan that includes members' preferences and prioritized goals.
8. Identification of barriers to meeting goals or implementing the plan.
9. Development of a schedule for follow-up and communication with members.
10. Development of a plan for follow-up and communication with LTSS providers.
11. Development of an emergency back-up plan.
12. Documentation of whether members received the services identified in the case management plan.
13. Assessment of progress against case management plans and goals, and modification as needed.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	High (90-100%) on file review for at least 7 factors and medium (60-89%) on file review for any remaining factors	Low (0-59%) on file review for no more than 5 factors and high (90%-100%) or medium (60-89%) on file review for any remaining factors	Low (0-59%) on file review for 6 or more factors

Data source Records or files

Scope of review **Product lines**
This element applies to First Surveys and Renewal Surveys for all product lines.

Documentation

NCQA reviews assessments in a random sample of up to 40 case management files. Files are selected from active or closed cases that were identified during the look-back period and remained open for at least 60 calendar days during the look-back period, from the date when the member was identified for case management.

The organization must provide the identification date for each case in the file universe.

Look-back period *For First Surveys:* 6 months.
For Renewal Surveys: 12 months.

Explanation **HEDIS LTSS measures**

Organizations may submit performance results on the Comprehensive Care Plan and Update (LTSS-CPU) measure instead of completing the file review. If an organization chooses this voluntary option, it must meet the 90 percent threshold rate on the measure to receive credit.

Dispute of file review results

Onsite file review is conducted in the presence of the organization's staff. The survey team works to resolve disputes that arise during the onsite survey. In the event that a dispute cannot be resolved, the organization must contact NCQA before the end of the onsite survey. File review results may not be disputed or appealed once the onsite survey is complete.

Assessment

Assessment requires the case manager or other qualified individual to reach and document a conclusion about data or information collected. For factors that require assessment, raw data or answers to questions do not meet the requirement. There is a documented summary of the information's meaning or its implications for the member's situation, for use in the case management plan.

The organization must reach a conclusion for each factor (unless otherwise stated in the explanation). This may be in separate summaries for each factor or in a combined summary, or in a combination of these.

Files excluded from review

The organization excludes files from review that meet these criteria:

- Eligible members whom it cannot locate or contact after three or more attempts across a 2-week period, within the first 30 calendar days after identification, through at least two of the following mechanisms:
 - Telephone.
 - Regular mail.
 - Email.
 - Fax.
- Members enrolled in case management for less than 60 calendar days during the look-back period.
 - The organization provides evidence of the member's identification date and that the member was in case management for less than 60 calendar days during the look-back period.
- Employees of the organization and their dependents.

Files that meet these criteria and are inadvertently included in the organization's file review are scored NA for all factors. NCQA confirms that the files met the criteria for an NA score.

Factor 1: Assessment of service needs

The case manager documents the services the member needs in the care plan.

Factor 2: Assessment of case management plans and goals

The organization documents a plan for case management that:

- Is specific to the member's situation and needs.
- Includes goals that reflect issues identified in the member assessment and the supporting rationale for each selected goal.
 - Goals are specific, measurable and timebound (i.e., have a target completion date).

Case management goals are prioritized by high/low, numeric rank or similar designation. Priorities reflect the member or caregiver's input and preferences and priorities. Designating goals as long-term or short-term is not sufficient to meet the requirement; the organization must rank or prioritize goals.

Factor 3: Assessment of member preferences

The file or case record documents the member's personal preferences for how case management and LTSS are delivered.

Factor 4: Self-management plans

The self-management plan includes actions the member agrees to take to manage a condition or circumstances. The file or case record documents that the plan was communicated to the member. Communication may be verbal or written. The self-management plan documents the member's acknowledgment of and agreement with prescribed actions.

Factor 5: Assessment of life-planning activities

The file or case record documents the case manager's assessment of whether the member has in place or has considered the need for wills, living wills or advance directives, Medical or Physician Orders of Life-Sustaining Treatment (MOLST or POLST) forms and health care powers of attorney.

If life planning activities are determined appropriate, the case manager documents the member's activities and the documents that are in place.

If life planning activities are determined not appropriate, the case manager documents the reason in the case management record or file.

As an alternative to an assessment of life planning needs, the organization may provide life planning information (e.g., brochure, pamphlet) to members in complex case management during the time frame allowed for completing the initial assessment. The file must document that the information was provided and the date.

Factor 6: Identification of preferred method of communication

The file or case record documents the members' preferred method of communication. Member communication may be documented in the form of in-person appointments, telephone, email, or use of member web portals.

Factor 7: Development of case management plan

The file or case record documents a plan for case management that is specific to the member's situation and needs, and includes goals that reflect issues identified in the member assessment and the supporting rationale for goal selection.

Factor 8: Identification of barriers

The file or case record identifies barriers related to the member or to the member's circumstances, not to the case management process. The organization documents barriers to the member meeting the goals specified in the case management plan.

Factor 9: Follow-up and communication with members

The file or case record documents the next scheduled contact with the member, including the scheduled time or time frame which may be an exact or relative (e.g., "October 15," "in two weeks") date, and the contact method.

Factor 10: Follow-up and communication with LTSS providers

The file or case record documents the roles and responsibilities of LTSS providers, case management plan details and the follow-up schedule that are communicated to providers.

Factor 11: Assessment of emergency back-up plans

The file or case record documents an emergency back-up plan that accounts for short-term and long-term needs, and may address circumstances such as temporary replacements for personal care attendants and how to respond to power outages that affect equipment. The file includes an emergency back-up plan customized to the member.

Factor 12: Documentation of services received

The file or case record documents whether the individual received the services specified in the case management plan.

Factor 13: Assessment of progress

The organization documents the member's progress toward goals. If the member does not demonstrate progress over time, the organization reassesses the applicability of the goals to the member's circumstances and modifies the goals, as appropriate.

Exceptions

None.

Examples

None.

Element H: Critical Incident Management System

The organization uses a critical incident management system that:

1. Defines what constitutes a critical incident.
2. Identifies responsibility for managing, following-up and acting on critical incidents.
3. Tracks critical incidents.
4. Prompts an investigation.
5. Implements appropriate interventions.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 4-5 factors	The organization meets 2-3 factors	The organization meets 0-1 factors
Data source	Documented process, Reports		
Scope of review	Product lines <i>This element applies to First Surveys and Renewal Surveys for all product lines.</i> Documentation NCQA reviews the organization's process for managing critical incidents. NCQA also reviews the organization's reports documenting incidents tracked and implementation of interventions.		
Look-back period	<i>For First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	This element may not be delegated. Critical incidents are events or occurrences that cause harm to a member or LTSS provider, or that indicate risk (e.g., abuse, neglect, exploitation) to a member or LTSS provider's health or welfare. Critical incident management systems are organizationwide systems that can eliminate an immediate threat or investigate allegations that a member may be at risk. Critical incident management system policies and procedures specify task time frames and how the program meets factor requirements. The organization provides access to the critical incident management system or to reports detailing system operations. Factor 1: Defining critical incidents The organization defines what constitutes a critical incident (subject to its policy), and may use the state's definition.		

Factor 2: Identifying responsibility

Managing, following up on and acting on critical incidents may require participation from staff in multiple roles and functions. Quality, risk management or compliance staff or supervisors may be responsible for investigating an incident, oversight of the process and communicating with oversight agencies; case managers may be responsible for following up with the member and others. Each person involved in the process must understand the requirements of their role.

Factor 3: Tracking critical incidents

The organization documents critical incidents and tracks them from the initial report through follow-up. Reports may be used to track patterns and prevalence of serious adverse events among members and to inform strategies for managing risk, preventing errors and focusing quality improvement efforts.

Factor 4: Investigating critical incidents

The organization investigates critical incidents promptly, and may involve outside agencies (e.g., adult protective services, law enforcement), as appropriate.

Factor 5: Implementing interventions

The nature and scope of a critical incident may require immediate response. The organization has a process for determining appropriate actions and for intervening in emergent situations.

Exceptions

None.

Examples**Factor 1: Defining critical incidents**

The definition of “critical incident” may include an actual or alleged event, such as:

- Member is physically restrained.
- Member does not receive needed services.
- Member’s money or belongings are stolen.
- Member attempts suicide.
- Member is sexually exploited or abused.

Factor 5: Implementing interventions

- Immediately remove threats from the environment.
- Provide back-up if providers are removed after an incident.

Element I: Qualifications and Assistance for LTSS Providers

The organization:

1. Specifies qualifications for paid LTSS providers.
2. Requires background checks for paid LTSS providers.
3. Requires at least one additional screening tool to verify qualifications for paid LTSS providers.
4. Requires that paid LTSS providers have access to assistance to deliver services.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 4 factors	The organization meets 2-3 factors	The organization meets 0-1 factors
Data source	Documented process, Reports, Materials		
Scope of review	Product lines <i>This element applies to First Surveys and Renewal Surveys for all product lines.</i> Documentation NCQA reviews the organization's documented processes or materials for LTSS provider qualifications and the assistance available to them. NCQA also reviews the organization's reports for the results from the background checks and results from the additional screening tool.		
Look-back period	<i>For First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	LTSS providers are paid and unpaid providers and organizations that provide long-term services and supports. Factor 1: Defining qualifications Qualifications refers to the type and amount of experience and/or the type and amount of education the paid LTSS provider must have before providing services. To promote member safety in the home, the organization sets qualifications for paid LTSS providers, including those who provide self-directed services. The organization may set different qualifications for providers of self-directed services. Factors 2, 3: Background checks and additional screening tool For factor 2, the organization requires statewide background checks for criminal history, abuse and neglect for paid LTSS providers, prior to or within 90 calendar days of hire. The organization or state requirements may determine disqualifying crimes. The organization may conduct background checks or rely on background checks conducted by other agencies. NCQA does not consider it delegation if the organization uses another entity to conduct background checks.		

For factor 3, the organization selects and uses an additional appropriate screening tool to verify that paid LTSS providers meet the defined qualifications. An attestation alone is only acceptable for job history.

For factors 2 and 3, the organization's policies and procedures specify the types of paid LTSS providers included in the screening process. The organization may differentiate requirements between regular providers (i.e., who have a regular schedule) and ad hoc or back-up providers.

Source: Galantowicz, S., Crisp, S., Karp, N., Accius, J. Safe at Home? Developing Effective Criminal Background Checks and Other Screening Policies for Home Care Workers. (AARP, 2010).

Factor 4: Providing assistance

The organization's policies and procedures require that paid LTSS providers:

- Have access to necessary supports and resources to enable them to appropriately and safely complete assigned tasks.
- Have access to emergency services for their safety and protection.
- Have a means to communicate grievances.

Exceptions

None.

Examples

Factor 1: Defining qualifications

- Disqualifying offenses from a background check.
- Certifications.
- Education history.
- Job history.
- Relevant experience.
- Relevant training.

Factor 2: Background checks

- National FBI checks.
- Office of the Inspector General List of Excluded Members/Entities.
- State and county criminal records check.
- State Adult Protective and Child Protective Services registries.
- National and state sex offender registry.
- DMV records.
- Commercial databases.

Factor 3: Screening tools

- Reference checks.
- Interviews.
- Signed statements about job history.
- Alcohol/drug screening.
- Credit checks.

Factor 4: Providing assistance

- 24-hour advice.
- Emergency hotline to address risks to the member.
- Immediate telephone and emergency assistance to address threats to the LTSS provider.

LTSS 2: Measure and Improve Performance

The organization measures member experience, program effectiveness and participation rates, and works to improve.

Intent

The organization identifies opportunities to improve the way it coordinates services through a structured process of evaluating member experience, program effectiveness and participation rates

Element A: Experience with Case Management

At least annually, the organization evaluates experience with its case management program by:

1. Obtaining feedback from members.
2. Analyzing complaints from members.

Summary of Changes

Clarifications

- Moved the first paragraph under the factor 2 explanation to the second paragraph under the factor 1 explanation.
- Revised the explanation under “Factor 2: Analyzing complaints from members” to clarify how the organization analyzes member experience with its case management program.

Scoring	Met	Partially Met	Not Met
	The organization meets 2 factors	The organization meets 1 factor	The organization meets 0 factors

Data source Reports

Scope of review **Product lines**

This element applies to First Surveys and Renewal Surveys for all product lines.

Documentation

For First Surveys: NCQA reviews the organization’s most recent annual data collection and evaluation report that it collects feedback from members and analyzes member complaints.

For Renewal Surveys: NCQA reviews the organization’s most recent and previous year’s annual data collection and evaluation reports that it collects feedback from members and analyzes member complaints.

Look-back period *For First Surveys:* At least once during the prior year.

For Renewal Surveys: 24 months.

Explanation **Factor 1: Obtaining feedback from members**

At least annually, the organization obtains member feedback through focus groups or experience surveys. Feedback is specific to the LTSS program submitted for distinction.

To identify complaint patterns, the organization collects data on member complaints and appeals from the entire population of LTSS members or draws statistically valid samples from the population. If the organization uses a sample, it describes the sample universe and the sampling methodology.

Factor 2: Analyzing complaints from members

The organization analyzes complaints to identify opportunities to improve member experience with its case management program.

For initial measurement, the organization conducts quantitative and qualitative analysis of data.

For remeasurement, the organization conducts quantitative analysis, and conducts qualitative analysis if quantitative analysis demonstrates that stated goals were not met.

Refer to *Appendix 5: Glossary* for the full definition of and requirements for quantitative analysis and qualitative analysis.

Exceptions

None.

Examples

Factor 1: Obtaining feedback

Measures of experience

- Overall experience with the program.
- Respectful treatment by LTSS providers.
- Care manager helpfulness.
- Perception of quality of care/services.

Access to supports and services

- Limited access to care manager.
- Dissatisfaction with care manager.
- Dissatisfaction with care management plan.
- Timeliness of case management services.

Factor 2: Analyzing complaints

- Themes and significance in complaints about a specific service area or service provider.
- Themes and significance of complaints about a care manager.
- Themes and significance of complaints about a case management strategy.
- Volume of complaints provided by a member or population group.
- Changes in trends of complaints.

Element B: Track and Analyze a Measure of Effectiveness

At least annually, the organization monitors one measure to evaluate the effectiveness of its case management program. For the measure, the organization:

1. Identifies a relevant process or outcome.
2. Uses valid methods that provide quantitative results.
3. Sets a performance goal.
4. Clearly identifies measure specifications.
5. Collects and analyzes results.
6. Identifies opportunities for improvement, if applicable.

Summary of Changes**Clarifications**

- Revised “Factor 5: Analysis” to read “Factor 5: Quantitative and qualitative analysis” and added text to clarify how the organization analyzes its performance.

Scoring

Met	Partially Met	Not Met
The organization meets 4-6 factors	The organization meets 3 factors	The organization meets 0-2 factors

Data source

Reports

Scope of review**Product lines**

This element applies to First Surveys and Renewal Surveys for all product lines.

Documentation

For First Surveys: NCQA reviews the organization’s most recent report of assessment of the first measure of program effectiveness.

For Renewal Surveys: NCQA reviews the organization’s most recent and the previous year’s annual reports of assessment of the first measure of program effectiveness.

NCQA scores this element for each program the organization brings forward for distinction.

The score for the element is the average of the scores for all programs.

Look-back period

For First Surveys: At least once during the prior year.

For Renewal Surveys: 24 months.

Explanation**Measures**

Organizations are required to track three measures of effectiveness. The organization presents its results and analysis of each measure separately in Elements B–D. The intent of these elements is to establish a basis for sound quality measurement and improvement. Because different programs have different population bases, enrollment methods, service obligations and data access, the organization may select any measure that is most relevant and useful for its quality improvement program. Organizations may select process or outcome measures.

Note: NCQA does not publish or compare organizations based on measure results submitted for these elements.

Factor 1: Relevant process or outcome

Relevant process measures are measures of evidence-based case management actions, or policies or procedures that have a direct or indirect effect on the well-being or social outcomes of the case management of members served.

Relevant outcome measures assess how case management programs or interventions improve member well-being.

Evidence-based information is based on the best available scientific evidence, on professional standards or on expert opinion.

Factor 2: Valid methods and quantitative results

Measurement of case management effectiveness includes the use of quantitative information derived from valid methodology. NCQA considers the following criteria when evaluating a measure's validity:

- Numerator and denominator.
- Sampling methodology.
- Sample size calculation.
- Measurement periods and seasonality effects.

Factor 3: Performance goal

A **performance goal** is the organization's desired level of achievement. The organization establishes an explicit, quantifiable performance goal for the measure. The goal may be based on external benchmarks (i.e., known levels of best performance) or on the organization's actual performance from prior years.

Factor 4: Measure specifications

The organization describes the data source, the eligible population, the coding or other means of identifying the process or outcome. The intent is to provide detailed measure specifications that can guide valid measurement.

Factor 5: Quantitative and qualitative analysis

For initial measurement, the organization conducts quantitative and qualitative analysis of data.

For remeasurement, the organization conducts quantitative analysis, and conducts qualitative analysis if quantitative analysis demonstrates that stated goals were not met.

Refer to *Appendix 5: Glossary* for the full definition of and requirements for quantitative analysis and qualitative analysis.

Factor 6: Opportunities for improvement

The organization uses the results of its analysis to prioritize opportunities for improvement, which may be different each time the organization measures and analyzes the data. The organization is not required to identify a specific number of improvement opportunities.

Exception

Factor 6 is NA if the organization's analysis of results shows that there are no opportunities for improvement. NCQA evaluates whether this conclusion is reasonable, given assessment results.

Examples**Factor 1: Process measures**

- Timeliness of completion of initial assessment.
- Timeliness of completion of case management plan.
- Timeliness of contact (e.g., phone, visits, assessments).
- Timeliness of reporting critical incidents and complaints.
- Timeliness of initiation of services.
- Comprehensiveness of case management plan (addresses all prioritized goals).
- Percentage of transitions from the community in which the case management plan is shared with the receiving facility.
- Participation rates.

Outcome measures

- Improved quality of life.
- Percentage of members who have made progress toward achieving priority goals.
- Health status (e.g., SF-36® or SF-12® results).
- Experience with case management services.
- Readmission rates.
- Unmet needs addressed.
- Percentage of members admitted to skilled nursing facility.

Factor 4: Measure specifications**Table 1: Measure Specifications**

Name of Measure:	Long-Term Community Residence
Activity Objective:	Increase the percentage of enrolled members residing in the community (noninstitutional)
Quantifiable Measure:	Percentage of members residing in the community
Population Included:	100% of enrolled program participants
Data Source:	Care Management Record, field: current residence
Numerator:	Enrollees residing in a community setting (noninstitutional)
Denominator:	All members enrolled in the program
Exclusion:	Short-term institutional stays (e.g., rehabilitation)
Baseline Goal:	>80%
Time Frame:	January–December 31, 2022

Element C: Track and Analyze a Second Measure of Effectiveness

At least annually, the organization monitors a second measure to evaluate the effectiveness of its case management program. For the measure, the organization:

1. Identifies a relevant process or outcome.
2. Uses valid methods that provide quantitative results.
3. Sets a performance goal.
4. Clearly identifies measure specifications.
5. Collects and analyzes results.
6. Identifies opportunities for improvement, if applicable.

Summary of Changes**Clarifications**

- Revised “Factor 5: Analysis” to read “Factor 5: Quantitative and qualitative analysis” and added text to clarify how the organization analyzes its performance.

Scoring	Met	Partially Met	Not Met
	The organization meets 4-6 factors	The organization meets 3 factors	The organization meets 0-2 factors

Data source Reports

Scope of review **Product lines**

This element applies to First Surveys and Renewal Surveys for all product lines.

Documentation

For First Surveys: NCQA reviews the organization’s most recent report of assessment of the second measure of program effectiveness.

For Renewal Surveys: NCQA reviews the organization’s most recent and the previous year’s annual reports of assessment of the second measure of program effectiveness.

NCQA scores this element for each program the organization brings forward for distinction. The score for the element is the average of the scores for all programs.

Look-back period *For First Surveys:* At least once during the prior year.

For Renewal Surveys: 24 months.

Explanation **Measures**

Organizations are required to track three measures of effectiveness. The organization presents its results and analysis of each measure separately in Elements B–D. The intent of these elements is to establish a basis for sound quality measurement and improvement. Because different programs have different population bases, enrollment methods, service obligations and data access, the organization may select any measure that is most relevant and useful for its quality improvement program. Organizations may select process or outcome measures.

Note: NCQA does not publish or compare organizations based on measure results submitted for these elements.

Factor 1: Relevant process or outcome

Relevant process measures are measures of evidence-based case management actions, or policies or procedures that have a direct or indirect effect on the well-being or social outcomes of the case management of members served.

Relevant outcome measures assess how case management programs or interventions improve member well-being.

Evidence-based information is based on the best available scientific evidence, on professional standards or on expert opinion.

Factor 2: Valid methods and quantitative results

Measurement of case management effectiveness includes the use of quantitative information derived from valid methodology. NCQA considers the following criteria when evaluating a measure's validity:

- Numerator and denominator.
- Sampling methodology.
- Sample size calculation.
- Measurement periods and seasonality effects.

Factor 3: Performance goal

A **performance goal** is the organization's desired level of achievement. The organization establishes an explicit, quantifiable performance goal for the measure. The goal may be based on external benchmarks (i.e., known levels of best performance) or on the organization's actual performance from prior years.

Factor 4: Measure specifications

The organization describes the data source, the eligible population, the coding or other means of identifying the process or outcome. The intent is to provide detailed measure specifications that can guide valid measurement.

Factor 5: Quantitative and qualitative analysis

For initial measurement, the organization conducts quantitative and qualitative analysis of data.

For remeasurement, the organization conducts quantitative analysis, and conducts qualitative analysis if quantitative analysis demonstrates that stated goals were not met.

Refer to *Appendix 5: Glossary* for the full definition of and requirements for quantitative analysis and qualitative analysis.

Factor 6: Opportunities for improvement

The organization uses the results of its analysis to prioritize opportunities for improvement, which may be different each time the organization measures and analyzes the data. The organization is not required to identify a specific number of improvement opportunities.

Exception

Factor 6 is NA if the organization's analysis of results shows that there are no opportunities for improvement. NCQA evaluates whether this conclusion is reasonable, given assessment results.

Examples**Factor 1: Process measures**

- Timeliness of completion of initial assessment.
- Timeliness of completion of case management plan.
- Timeliness of contact (e.g., phone, visits, assessments).
- Timeliness of reporting critical incidents and complaints.
- Timeliness of initiation of services.
- Comprehensiveness of case management plan (addresses all prioritized goals).
- Percentage of transitions from the community in which the case management plan is shared with the receiving facility.
- Participation rates.

Outcome measures

- Improved quality of life.
- Percentage of members who have made progress toward achieving priority goals.
- Health status (e.g., SF-36® or SF-12® results).
- Experience with case management services.
- Readmission rates.
- Unmet needs addressed.
- Percentage of members admitted to skilled nursing facility.

Factor 4: Measure specifications**Table 1: Measure Specifications**

Name of Measure:	Long-Term Community Residence
Activity Objective:	Increase the percentage of enrolled members residing in the community (noninstitutional)
Quantifiable Measure:	Percentage of members residing in the community
Population Included:	100% of enrolled program participants
Data Source:	Care Management Record, field: current residence
Numerator:	Enrollees residing in a community setting (noninstitutional)
Denominator:	All members enrolled in the program
Exclusion:	Short-term institutional stays (e.g., rehabilitation)
Baseline Goal:	>80%
Time Frame:	January–December 31, 2022

Element D: Track and Analyze a Third Measure of Effectiveness

At least annually, the organization monitors a third measure to evaluate the effectiveness of its case management program. For the measure, the organization:

1. Identifies a relevant process or outcome.
2. Uses valid methods that provide quantitative results.
3. Sets a performance goal.
4. Clearly identifies measure specifications.
5. Collects and analyzes results.
6. Identifies opportunities for improvement, if applicable.

Summary of Changes**Clarifications**

- Revised “Factor 5: Analysis” to read “Factor 5: Quantitative and qualitative analysis” and added text to clarify how the organization analyzes its performance.

Scoring	Met	Partially Met	Not Met
	The organization meets 4-6 factors	The organization meets 3 factors	The organization meets 0-2 factors

Data source Reports

Scope of review **Product lines**

This element applies to First Surveys and Renewal Surveys for all product lines.

Documentation

For First Surveys: NCQA reviews the organization’s most recent report of assessment of the third measure of program effectiveness.

For Renewal Surveys: NCQA reviews the organization’s most recent and the previous year’s annual reports of assessment of the third measure of program effectiveness.

NCQA scores this element for each program the organization brings forward for distinction. The score for the element is the average of the scores for all programs.

Look-back period *For First Surveys:* At least once during the prior year.

For Renewal Surveys: 24 months.

Explanation **Measures**

Organizations are required to track three measures of effectiveness. The organization presents its results and analysis of each measure separately in Elements B–D. The intent of these elements is to establish a basis for sound quality measurement and improvement. Because different programs have different population bases, enrollment methods, service obligations and data access, the organization may select any measure that is most relevant and useful for its quality improvement program. Organizations may select process or outcome measures.

Note: NCQA does not publish or compare organizations based on measure results submitted for these elements.

Factor 1: Relevant process or outcome

Relevant process measures are measures of evidence-based case management actions, or policies or procedures that have a direct or indirect effect on the well-being or social outcomes of the case management of members served.

Relevant outcome measures assess how case management programs or interventions improve member well-being.

Evidence-based information is based on the best available scientific evidence, on professional standards or on expert opinion.

Factor 2: Valid methods and quantitative results

Measurement of case management effectiveness includes the use of quantitative information derived from valid methodology. NCQA considers the following criteria when evaluating a measure's validity:

- Numerator and denominator.
- Sampling methodology.
- Sample size calculation.
- Measurement periods and seasonality effects.

Factor 3: Performance goal

A **performance goal** is the organization's desired level of achievement. The organization establishes an explicit, quantifiable performance goal for the measure. The goal may be based on external benchmarks (i.e., known levels of best performance) or on the organization's actual performance from prior years.

Factor 4: Measure specifications

The organization describes the data source, the eligible population, the coding or other means of identifying the process or outcome. The intent is to provide detailed measure specifications that can guide valid measurement.

Factor 5: Quantitative and qualitative analysis

For initial measurement, the organization conducts quantitative and qualitative analysis of data.

For remeasurement, the organization conducts quantitative analysis, and conducts qualitative analysis if quantitative analysis demonstrates that stated goals were not met.

Refer to *Appendix 5: Glossary* for the full definition of and requirements for quantitative analysis and qualitative analysis.

Factor 6: Opportunities for improvement

The organization uses the results of its analysis to prioritize opportunities for improvement, which may be different each time the organization measures and analyzes the data. The organization is not required to identify a specific number of improvement opportunities.

Exception

Factor 6 is NA if the organization's analysis of results shows that there are no opportunities for improvement. NCQA evaluates whether this conclusion is reasonable, given assessment results.

Examples**Factor 1: Process measures**

- Timeliness of completion of initial assessment.
- Timeliness of completion of case management plan.
- Timeliness of contact (e.g., phone, visits, assessments).
- Timeliness of reporting critical incidents and complaints.
- Timeliness of initiation of services.
- Comprehensiveness of case management plan (addresses all prioritized goals).
- Percentage of transitions from the community in which the case management plan is shared with the receiving facility.
- Participation rates.

Outcome measures

- Improved quality of life.
- Percentage of members who have made progress toward achieving priority goals.
- Health status (e.g., SF-36® or SF-12® results).
- Experience with case management services.
- Readmission rates.
- Unmet needs addressed.
- Percentage of members admitted to skilled nursing facility.

Factor 4: Measure specifications**Table 1: Measure Specifications**

Name of Measure:	Long-Term Community Residence
Activity Objective:	Increase the percentage of enrolled members residing in the community (noninstitutional)
Quantifiable Measure:	Percentage of members residing in the community
Population Included:	100% of enrolled program participants
Data Source:	Care Management Record, field: current residence
Numerator:	Enrollees residing in a community setting (noninstitutional)
Denominator:	All members enrolled in the program
Exclusion:	Short-term institutional stays (e.g., rehabilitation)
Baseline Goal:	>80%
Time Frame:	January–December 31, 2022

Element E: Action and Remeasurement

Based on the results of its measurement and analysis of case management effectiveness, the organization:

1. Acts to improve on one measure of effectiveness, if applicable.
2. Acts to improve on one measure of experience, if applicable.
3. Remeasures to determine the action's impact on effectiveness, if applicable.
4. Remeasures to determine the action's impact on experience, if applicable.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 3-4 factors	The organization meets 2 factors	The organization meets 0-1 factors
Data source	Reports		
Scope of review	Product lines <i>This element applies to Renewal Surveys for all product lines.</i> Documentation NCQA reviews the organization's most recent and previous year's reports on annual improvement and remeasurement activities. NCQA scores this element for each program the organization brings forward for distinction. The score for the element is the average of the scores for all programs.		
Look-back period	<i>For Renewal Surveys: 24 months.</i>		
Explanation	The organization implements at least one intervention that addresses one or more opportunities identified in Elements B–D. Factors 1, 2: Acts to improve The organization selects interventions that have a reasonable chance of affecting results and describes the interventions, including the date of implementation and the period of interventions, when possible. Factor 3: Impact on effectiveness Using the methodology and performance measure specifications described in Elements B–D, the organization remeasures to determine the impact of interventions. Additional process measures regarding interventions may supplement remeasurement. The organization continues to remeasure to determine the effectiveness of the interventions, using methods consistent with initial measurements.		

Factor 4: Impact on experience

Using the methodology and experience measure specifications described in Element A, the organization remeasures to determine the impact of interventions. Additional experience measures regarding interventions may supplement remeasurement.

Exceptions

This element is NA for First Surveys.

Factors 1 and 3 are NA if the organization does not identify opportunities for improvement of effectiveness. NCQA evaluates whether this conclusion is reasonable, given the organization's analysis.

Factors 2 and 4 are NA if the organization does not identify opportunities for improvement of experience. NCQA evaluates whether this conclusion is reasonable, given the organization's analysis.

Examples None.

Element F: Active Participation

At least annually, the organization measures participation rates.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement

Data source Reports

Scope of review **Product lines**

This element applies to First Surveys and Renewal Surveys for all product lines.

Documentation

For First Surveys: NCQA reviews the most recent participation report completed during the look-back period for each program the organization brings forward.

For Renewal Surveys: NCQA reviews the most recent and previous year's participation report completed during the look-back period for each program the organization brings forward.

If the organization measures participation separately for each purchaser, NCQA reviews up to three reports that were distributed to purchasers during the look-back period.

NCQA scores this element for each program the organization brings forward for distinction.

The score for the element is the average of the scores for all programs.

Look-back period *For First Surveys:* At least once during the prior year.
For Renewal Surveys: 24 months.

Explanation Case management participation rate

The **participation rate** is the number of members who received an initial assessment and at least one additional interactive contact, divided by the number of members identified as eligible for the program.

Eligible members are members for whom the organization has been contracted to provide case management services.

Calculating active participation

The organization may not use the enrollment rate to calculate the active participation rate. The active participation rate is:

- *Numerator:* The number of members with completed assessments and at least one additional interactive contact.
- *Denominator:* Total number of eligible members.

Interactive contact is two-way interaction in which the member receives self-management support, health education or care coordination through one of the following methods:

- Phone.
- In-person contact (i.e., member or group).
- Online contact:
 - Interactive web-based module.
 - Live chat.
 - Secure email.

Interactive contact does not include:

- Completion of a health appraisal, except as noted in the client restrictions section below.
- Contacts made only to make an appointment, leave a message or acknowledge receipt of materials.

Client restrictions

Some programs do not identify eligible members because of restrictions by the client organization. These programs are based on attracting participation (e.g., using contacts through newsletters) from an employee population or a health plan's member population at large. NCQA expects that for these programs, the organization will calculate its rate of participation using an expected percentage of the at-large population with a specific condition. For example, if the rate of diabetes in the general U.S. population is 4 percent and the organization recruits 200 diabetics from an employer population of 10,000, it can estimate the participation rate as 50 percent of 400 diabetics ($10,000 \times 4\% = 400$).

Exceptions

None.

- Examples** The organization is contracted to provide case management to 100 members (the denominator) identified as needing LTSS.
- Of the 100 members identified, the organization is able to contact 80 members (the organization is unable to find or reach 20 members).
 - Of the 80 members reached, the organization can schedule an initial assessment with 78 members (2 members refused).
 - The organization conducts an initial scheduled assessment of 75 members (1 member dies, 1 is admitted to a skilled nursing facility, 1 refuses to meet the case manager on the day of the scheduled assessment).
 - Of the 75 assessments completed, case managers have interactive contact (in-person visits or telephone check-ins) with 60 members.
- In this scenario, the participation rate is 60/100.

Element G: Improving Participation Rates

At least annually, the organization:

1. Analyzes participation rates.
2. Identifies at least one opportunity for improvement.
3. Implements at least one action to improve participation rates.

Summary of Changes

Clarifications

- Added “Quantitative and qualitative analysis” subhead and text in the Explanation to clarify how the organization analyzes its performance.
- Removed the first sentence under the Factor 1 explanation.

Scoring	Met	Partially Met	Not Met
	The organization meets 3 factors	The organization meets 2 factors	The organization meets 0-1 factors

Data source Reports

Scope of review **Product lines**

This element applies to First Surveys and Renewal Surveys for all product lines.

Documentation

For First Surveys: NCQA reviews the organization’s most recent report for evidence that the organization analyzed participation rates.

For Renewal Surveys: NCQA reviews the organization’s most recent and previous year’s reports for evidence that the organization analyzed participation rates.

If the organization identifies opportunities for improvement by purchaser, NCQA reviews reports on at least three purchasers or a single report with information about all purchasers.

Look-back period *For First Surveys:* At least once during the prior year.
For Renewal Surveys: 24 months.

Explanation	Improving participation rates increases the number of eligible members who receive case management interventions. Quantitative and qualitative analysis <i>For initial measurement</i>, the organization conducts quantitative and qualitative analysis of data. <i>For remeasurement</i>, the organization conducts quantitative analysis, and conducts qualitative analysis if quantitative analysis demonstrates that stated goals were not met. Refer to <i>Appendix 5: Glossary</i> for the full definition of and requirements for quantitative analysis and qualitative analysis. Factor 1: Analysis Quantitative and qualitative data collection and analysis may be performed across all purchasers or by each purchaser, depending on the organization's preference. Factor 2: Opportunity for improvement Based on the results from the analysis in factor 1, the organization identifies at least one opportunity for improvement. Factor 3 No additional explanation required. Exceptions Factors 2 and 3 are NA: • If the organization's analysis of results shows that there are no opportunities for improvement. – NCQA evaluates whether this conclusion is reasonable, given assessment results.
Examples	None.

Element H: Transparency in Reporting Participation

The organization reports program participation rates to purchasers and is transparent about measure specifications by reporting:

1. The defined population included in the denominator.
2. How members are included in the numerator.
3. The specified measurement period and how it affects inclusion and exclusion in the numerator and denominator.

Summary of Changes

Clarifications

- Consolidated and reformatted the Exceptions language.

Scoring	Met	Partially Met	Not Met
	The organization meets 3 factors	The organization meets 2 factors	The organization meets 0-1 factors
Data source	Documented process, Reports, Materials		
Scope of review	Product lines <i>This element applies to First Surveys and Renewal Surveys for all product lines.</i> Documentation NCQA reviews up to three reports or materials that were distributed to purchasers during the look-back period, for evidence that the organization was transparent about its calculation method. If participation is measured by the purchaser, NCQA reviews the organization's documented process for providing measure specifications to each purchaser. NCQA also reviews evidence that the organization reports its active participation rate, as defined in Element F.		
Look-back period	<i>For First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	Measure components (e.g., denominator, numerator) may be defined in materials provided to purchasers with measure reports. Purchaser organization NCQA expects the primary audience for this disclosure to be the organization's purchasers. Factors 1–3 No additional explanation required. Exception This element is NA if the organization does not report, or is not required to report, measurement quality information to the state or to purchasers. The organization provides documentation (e.g., contracts, other service agreements) demonstrating that no clients receive measurement quality data reporting.		
Examples	None.		

LTSS 3: Care Transitions

The organization has a process to manage care transitions, identify problems that could cause care transitions and prevent unplanned transitions, when possible.

Intent

The organization works to reduce unplanned transitions by managing transitions and identifying problems that can result in transitions of care.

Element A: Process for Transitions of Care

To facilitate safe transitions of care, the organization has a documented process to:

1. Identify members who transition between settings.
2. Notify members' usual providers of a transition within a specified time frame.
3. Assign a consistent person or unit within the organization responsible for supporting a member throughout a transition within a specified time frame.
4. Communicate necessary information with the receiving setting within a specified time frame.
5. Communicate the care transition process to members and their designated representatives within a specified time frame.
6. Track transition status.
7. Collaborate with the discharge team on the discharge plan.
8. Reassess the appropriateness of an existing case management plan within a specified time frame, and modify as needed.
9. Discuss changes to the case management plan with members and their designated representatives within a specified time frame.
10. Document medications with members and their designated representatives and communicate identified discrepancies to an appropriate provider.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 7-10 factors	The organization meets 5-6 factors	The organization meets 0-4 factors
Data source	Documented process		
Scope of review	Product lines <i>This element applies to First Surveys and Renewal Surveys for all product lines.</i>		
	Documentation NCQA reviews the organization's documented process for managing planned and unplanned care transitions.		

Look-back period *For First Surveys:* 6 months.
 For Renewal Surveys: 24 months.

Explanation This element is a **structural requirement**. The organization presents its own documentation.

HEDIS LTSS measures

Organizations may choose to meet this element by submitting performance results on the Reassessment/Care Plan Update After Discharge (LTSS-RAC) measure instead of providing a documented process. If an organization chooses this voluntary option, it must meet the 5 percent threshold rate on the measure to receive credit.

A **care transition** is movement of members between care settings (e.g., from home to hospital) as their condition and care needs change during the course of a chronic or acute illness. Members moving between settings are particularly vulnerable to receiving fragmented and unsafe care if transitions are poorly coordinated. If the organization does not manage care transitions for all members who receive services, the documented process must describe the population for which it manages transitions.

Planned transitions include elective surgery or a decision to enter a long-term care facility. **Unplanned transitions** include sudden hospitalizations resulting from emergencies.

A **care setting** is a provider or place that delivers health care and health-related services, and includes:

- Acute care facilities.
- Emergency departments.
- Skilled nursing facilities.
- Custodial nursing facilities.
- Rehabilitation facilities.
- The home and community.

The **receiving setting** is the setting responsible for care after a transition.

Factor 1: Identify members who transition

The organization has a process to identify members who transition between settings.

Because members may transition between various types of settings for different reasons, the process should address transitions that impact the delivery of services provided by the organization. For example, an organization arranging for provision of LTSS in a home-based setting may choose to focus its process on transitions out of the home and into inpatient or residential facilities.

Factor 2: Notify usual providers

Usual care providers include LTSS providers, primary care practitioners or specialists responsible for members' care. The organization attempts to connect members with primary care practitioners if they do not have one.

Case management policies and procedures specify the process for:

- Notifying members' usual care providers about transitions between settings.
- The defined time frames for notification.
 - The organization establishes a time frame for carrying out activities after being notified of a transition.

Factor 3: Support during transition

The organization's policies and procedures specify a process for assigning a consistent person or unit to support members during a transition. When the organization is notified of a transition, the organization contacts the member or their designated representative within a specified time frame to communicate the person or unit responsible for supporting them during their transition.

Factor 4: Communicate key information

Sharing necessary information can facilitate continuity of care across settings. Case management policies and procedures specify a process for conveying information to the receiving setting, to help a member continue to receive vital care. Key information can include how the member's health status and goals impact the transition and how the transition impacts the plan of care. Sharing a case management plan meets the intent of this factor, but is not the only way organizations can share pertinent information during a transition between settings.

Factor 5: Communicate about the care transition process

The organization establishes a time frame for carrying out activities after being notified of a transition.

Case management policies and procedures specify a process for communicating about the transition process with members and their designated representatives. Members decide their designated representatives' level of involvement.

Factor 6: Track the status of transitions

Case management policies and procedures specify a process for tracking members who have transitioned to another care setting. Tracking the status of transitions will alert the organization when to resume services and when to initiate new services.

Factor 7: Collaborate with the discharge team

Case management policies and procedures specify a process (e.g., discharge plan) for collaborating with the discharge team to resume or initiate services for members who are transitioning back to their usual setting.

Factor 8: Reassess the existing case management plan

Case management policies and procedures specify a process for determining if the case management plan continues to meet a member's needs. The organization works with the member to modify the plan, if necessary. This may include screening for eligibility for additional services or reassessing goals or other components included in LTSS 1, Element B, Element C and Element G.

Factor 9: Communicate changes to the case management plan

The organization's case management policies and procedures specify a time frame and process for discussing changes to the case management plan with members and their designated representatives during transition from a care setting.

Factor 10: Documentation of medications

Case management policies and procedures specify a process for documenting new and existing medications during a transition between settings, and for informing an appropriate provider of discrepancies between the medication list and documented medications.

Exceptions

None.

Examples**Factor 1: Identify members who transition**

- Organization A's process indicates that the organization receives automated notifications when members are admitted to the local hospital.
- Organization B's process indicates that a coordinator has been assigned to check the admissions reports from the local hospitals on a daily basis to determine if members enrolled in the program have experienced a transition.

Factor 6: Track the status of transitions

The organization establishes a process to document:

- The current location of the member in the care record.
- The planned date of transfer to another location.
- The anticipated receiving setting.
- Contact information for those coordinating care at the current setting.

Factor 7: Collaborate with the discharge team

- Email correspondence.
- Telephone correspondence.
- Meeting notes.
- Documentation of information submitted to the receiving setting such as, but not limited to:
 - Original care plan information.
 - Description of the home environment.
 - Detail about informal supports available to the members.
 - Resources and support the LTSS case management organization is able to provide to the member upon return to the community.
- Updated discharge plan reflecting the roles and responsibilities of both the discharge setting and the LTSS case management organization.
- Updated care plan based on discharge orders.

Factor 10: Documentation of medications

The organization's process indicates that when a member experiences a care transition, the organization obtains a medication list from the last setting the member received care in. The organization's process also indicates that an LTSS care manager compares the medication list provided at discharge and the medications currently in the home. The LTSS care manager then identifies, documents and reports discrepancies to the member's appropriate provider.

Element B: Reducing Unplanned Transitions for Members

The organization uses available case management information to:

1. Identify members at high risk of an unplanned transition, at least monthly.
2. Take action to mitigate risk of unplanned transitions.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 2 factors	The organization meets 1 factor	The organization meets 0 factors
Data source	Reports, Materials		
Scope of review	Product lines		
	<i>This element applies to First Surveys and Renewal Surveys for all product lines.</i>		
	Documentation		
	NCQA reviews reports for evidence that the organization identifies members at high risk of unplanned transition and acts to reduce unplanned transitions.		
	NCQA also reviews evidence that the organization takes action to mitigate risk by providing information to members.		
Look-back period	<i>For First Surveys: 6 months.</i>		
	<i>For Renewal Surveys: 24 months.</i>		
Explanation	Factor 1: Identifying members at risk		
	To minimize avoidable and unplanned transitions, the organization monitors members' information (e.g., self-reported, case manager reports) and identifies members who are at risk of experiencing an unplanned transition.		
	The more frequently the organization receives reports or analyzes information, the better it can respond to a health issue before it results in an admission or a change in level of care.		
	NCQA does not prescribe a method for determining at-risk members.		
	Factor 2: Mitigating risk		
	As part of identifying and coordinating care to prevent potential problems, the organization takes action to mitigate risks of unplanned transitions identified in factor 1.		
	Exceptions		
	None.		

Examples**Factor 1: Identifying members at risk**

Use predictive modeling to identify members at risk by considering information from the following sources:

- In-home assessment.
- Case manager reports.
- LTSS provider reports.

Reports from family/friends/caregivers.

- Information reported by the member.
- Redacted reports that show predictive modeling to assign members a risk score.

Factor 2: Mitigating risk

- Arrange for meal delivery to improve nutritional status.
- Work with the member and arrange for home modifications to reduce fall risk (e.g., increase physical therapy, reduce clutter, tape down loose rugs, eliminate long electrical cords, install grab bars in the bathroom).
- Provide a brochure on prescription adherence.
- Provide educational materials on how to manage chronic conditions such as diabetes.
- Arrange for companion service or adult day program to reduce social isolation.
- Develop a triggering criterion to identify members whose condition might trigger an intervention.
- Care coordination for at-risk members.

Element C: Reducing Unplanned Transitions for the Population

At least annually, the organization uses available information to:

1. Analyze rates of unplanned admissions to facilities and emergency department visits, to identify areas for improvement.
2. Take action to address areas identified for improvement.

Summary of Changes**Clarifications**

- Moved text regarding quantitative and qualitative analysis from under “Factor 1: Analyzing population-based data and taking action” to be the first three paragraphs of the Explanation.

Scoring

Met	Partially Met	Not Met
The organization meets 2 factors	The organization meets 1 factor	The organization meets 0 factors

Data source

Reports

Scope of review**Product lines**

This element applies to First Surveys and Renewal Surveys for all product lines.

	Documentation NCQA reviews the organization's reports for evidence that it annually analyzes rates of unplanned admissions and takes action on areas identified for improvement.
Look-back period	<i>For First Surveys:</i> At least once during the prior year. <i>For Renewal Surveys:</i> 24 months.
Explanation	Quantitative and qualitative analysis <i>For initial measurement,</i> the organization conducts quantitative and qualitative analysis of data. <i>For remeasurement,</i> the organization conducts quantitative analysis, and conducts qualitative analysis if quantitative analysis demonstrates that stated goals were not met. Refer to <i>Appendix 5: Glossary</i> for the full definition of and requirements for quantitative analysis and qualitative analysis. Factor 1: Analyzing population-based data and taking action The organization completes a quantitative and qualitative analysis of admission rates for the population that receives services and identifies opportunities for improvement. Analysis includes patterns of unplanned admissions; readmissions; ED visits and repeat visits; and admission to participating and nonparticipating facilities. NCQA does not specify the type of information the organization uses to analyze admission rates. The organization is not required to include all these areas in its analysis, but at a minimum, it must evaluate rates of unplanned admissions to facilities and ED visits to identify areas for improvement. If the organization has no access to patient record systems, it may gather information about unplanned admissions during visits with participants. Factor 2: Taking action The organization implements interventions based on the causes of unplanned transitions for the population identified in factor 1. Exceptions None.
Examples	Factor 1: Reports • Redacted reports that identify members who experienced an unplanned transition, using claims data, assessment data, care plan data or other data. • Reports on analysis of root causes and opportunities for improvement. Factor 2: Taking action • Increase contact with subpopulations identified as having higher rates of unplanned admissions. • Address unmet needs that lead to unplanned transitions.

LTSS 4: Delegation of LTSS

If the organization delegates LTSS activities, there is evidence of oversight of delegated activities

Intent

The organization carefully monitors functions performed by other organizations.

Element A: Delegation Agreement

The written delegation document:

1. Is mutually agreed upon.
2. Describes the delegated activities and the responsibilities of the organization and the delegate.
3. Requires at least semiannual reporting by the delegated entity to the organization.
4. Describes the process by which the organization evaluates the delegate's performance.
5. Describes the remedies available to the organization, including revocation of the delegation, if the delegate does not fulfill its obligations.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 4-5 factors	The organization meets 3 factors	The organization meets 0-2 factors
Data source	Materials		
Scope of review	Product lines <i>This element applies to First Surveys and Renewal Surveys for all product lines.</i>		
	Documentation NCQA reviews delegation agreements in effect during the look-back of up to four randomly selected delegates, or reviews all delegates if the organization has fewer than four. The score for the element is the average of the scores for all delegates.		
Look-back period	<i>For First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	This element may not be delegated. This element applies to agreements that are in effect within the look-back period. The delegation agreement describes all delegated LTSS activities. A generic policy statement about the content of delegated arrangements does not meet this element.		

Factor 1: Mutual agreement

Delegation activities are mutually agreed on before delegation begins, in a dated, binding document or communication between the organization and the delegated entity.

NCQA considers the effective date specified in the delegation agreement as the mutually agreed-upon effective date. The effective date may be before or after the signature date on the agreement. If the agreement has no effective date, NCQA considers the signature date (the date of the last signature) as the mutually agreed upon effective date.

NCQA may accept other evidence of the mutually agreed-upon effective date: a letter, meeting minutes or other form of communication between the organization and the delegate that references the parties' agreement on the effective date of delegated activities.

NCQA requires submitted evidence for all other delegation factors to consider the same mutually agreed-upon date as the effective date for the delegate's performance of delegated activities.

Factor 2: Assigning responsibilities

The delegation agreement or an addendum thereto or other binding communication between the organization and the delegate specifies the LTSS activities:

- Performed by the delegate, in detailed language.
- Not delegated, but retained by the organization.
 - The organization may include a general statement in the agreement addressing retained functions (e.g., the organization retains all other LTSS functions not specified in this agreement as the delegate's responsibility).

If the delegate subdelegates an activity, the delegation agreement must specify that the delegate or the organization is responsible for subdelegate oversight.

Factor 3: Reporting

The organization determines the method of reporting and the content of the reports, but the agreement must specify:

- That reporting is at least semiannual.
- What information is reported by the delegate about delegated activities.
- How, and to whom, information is reported (i.e., joint meetings or to appropriate committees or members in the organization).

Factor 4: Performance monitoring

The delegation agreement specifies the organization's process for evaluating delegate performance.

Factor 5: Consequences for failure to perform

The delegation agreement specifies consequences if a delegate fails to meet the terms of the agreement and, at a minimum, circumstances that result in revocation of the agreement.

Exception

This element is NA if the organization does not delegate LTSS activities.

Examples None.

Element B: Predelegation Evaluation

For new delegation agreements initiated in the look-back period, the organization evaluated delegate capacity before the delegation document was signed.

Summary of Changes
Clarifications

- Revised the text under “Predelegation evaluation” to clarify that evaluation may include a review of the organization’s structure, processes and staffing.

Scoring

Met	Partially Met	Not Met
The organization evaluated delegate capacity before delegation began	The organization evaluated delegate capacity after the delegation began	The organization did not evaluate delegate capacity

Data source Reports

Scope of review**Product lines**

This element applies to First Surveys and Renewal Surveys for all product lines.

This element applies if delegation was implemented in the look-back period.

Documentation

NCQA reviews the organization’s predelegation evaluation of up to four randomly selected delegates, or reviews all delegates if the organization has fewer than four.

The score for the element is the average of the scores for all delegates.

Look-back period

For First Surveys: 6 months.

For Renewal Surveys: 12 months.

Explanation

This element may not be delegated.

NCQA-Accredited delegates

Automatic credit is available for this element if all delegates are NCQA-Accredited in Case Management for Long-Term Services and Supports (CM-LTSS), unless the element is NA.

Predelegation evaluation

The organization evaluated the delegate’s capacity to meet NCQA requirements within 12 months prior to implementing delegation. The evaluation may include a review of the organization’s structure, processes and staffing in order to determine its capability to perform the delegated function.

NCQA considers the date of the agreement to be the implementation date if the delegation agreement does not include an implementation date.

If the time between the predelegation evaluation and implementation of delegation exceeds the 12 months, the organization conducts another predelegation evaluation.

If the organization amends the delegation agreement to include additional LTSS activities within the look-back period, it performs a predelegation evaluation for the additional activities.

Exceptions

This element is NA if:

- The organization does not delegate LTSS activities.
- Delegation arrangements have been in effect for longer than the look-back period.

Examples

Predelegation evaluation

- Site visit.
- Telephone consultation.
- Documentation review.
- Committee meetings.
- Virtual review.

Element C: Review of LTSS Program

For delegation arrangements in effect for 12 months or longer, the organization:

1. Annually reviews its delegate's LTSS program.
2. Annually audits case management files against NCQA standards for each year that delegation has been in effect, if applicable.
3. Annually evaluates delegate performance against NCQA standards for delegated activities.
4. Semiannually evaluates regular reports, as specified in Element A.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 4 factors	The organization meets 2 factors	The organization meets 0 factors

Data source Reports

Scope of review **Product lines**

This element applies to First Surveys and Renewal Surveys for all product lines.

Documentation

NCQA reviews evidence of the organization's review from up to four randomly selected delegates, or all delegates if the organization has fewer than four.

For First Surveys: NCQA reviews the organization's most recent annual review, audit, performance evaluation and semiannual evaluation.

For Renewal Surveys: NCQA reviews the organization's most recent and the previous year's annual reviews, audits, performance evaluations and four semiannual evaluations.

The score for the element is the average of the scores for all delegates.

Look-back period

For First Surveys: Once during the prior year.

For Renewal Surveys: 24 months.

Explanation

This element may not be delegated.

Automatic credit is available for factors 2 and 3 if all delegates are NCQA-Accredited in Case Management for Long-Term Services and Supports (CM-LTSS), unless the element is NA.

Factor 1: Review of the LTSS program

Appropriate organization staff or committee reviews the delegate's LTSS program. At a minimum, the organization reviews parts of the LTSS program that apply to the delegated functions.

Factor 2: Annual file audit

If the organization delegates case management, it audits the delegate's case management files against NCQA standards. The organization uses one of the following to audit the files:

- 5 percent or 50 of its files, whichever is less.
- The NCQA "8/30 methodology" available at <http://www.ncqa.org/Programs/Accreditation/PolicyUpdatesSupportingDocuments.aspx>

The organization bases its annual audit on the responsibilities described in the delegation agreement and the appropriate NCQA standards.

Factor 3: Annual evaluation

No additional explanation required.

Factor 4: Evaluation of reports

No additional explanation required.

Exceptions

This element is NA if:

- The organization does not delegate LTSS activities.
- Delegation arrangements have been in effect for less than 12 months.

Factor 2 is scored NA if the organization does not delegate case management activities.

Examples

None.

Element D: Opportunities for Improvement

For delegation arrangements that have been in effect for more than 12 months, at least once in each of the past 2 years, the organization identified and followed up on opportunities for improvement, if applicable.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization has acted on identified problems, if any, at least once in each of the past 2 years that the delegation arrangement has been in effect	The organization took inappropriate or weak action, or acted only in the past year	The organization has not acted on identified problems

Data source Documented process, Reports, Materials

Scope of review **Product lines**
This element applies to First Surveys and Renewal Surveys all product lines.

Documentation

NCQA reviews reports of opportunities for improvement, of up to four randomly selected delegates, or from all delegates, if the organization has fewer than four, and for evidence that the organization took appropriate action to resolve issues.

For First Surveys, NCQA reviews the organization's most recent annual review and follow-up on improvement activities.

For Renewal Surveys, NCQA reviews the organization's most recent and previous year's annual reviews and follow-up on improvement opportunities.

The score for the element is the average of the scores for all delegates.

Look-back period *For First Surveys*: At least once during the prior year.
For Renewal Surveys: 24 months.

Explanation This element may not be delegated.

NCQA-Accredited delegates

Automatic credit is available for this element if all delegates are NCQA-Accredited in Case Management for Long-Term Services and Supports (CM-LTSS), unless the element is NA.

Identify and follow up on opportunities

The organization uses information from its predelegation evaluation, ongoing reports or annual evaluation to identify areas of improvement.

Exceptions

This element is NA if:

- The organization does not delegate LTSS activities.
- Delegation arrangements have been in effect for less than 12 months.
- The organization has no opportunities to improve performance.
 - NCQA evaluates whether this conclusion is reasonable, given assessment results.

Examples None.

Medicaid

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MED 1: Medicaid Benefits and Services*Refer to Appendix 1 for points*

The organization provides benefits and services specified by federal requirements for health plans administering to Medicaid members.

Intent

Medicaid health plans make benefits and services available to Medicaid members as specified by federal requirements.

Element A: Direct Access to Women’s Health Services

The organization allows women direct access to in-network women’s health specialists for covered routine and preventive health care services.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement

Data source Materials

Scope of review *This element applies to Interim Surveys, First Surveys and Renewal Surveys.*
NCQA reviews the organization’s marketing materials, member newsletters, member handbooks or other evidence that demonstrates that women have direct access to in-network specialists throughout the look-back period.

Look-back period *For Interim Surveys and First Surveys: 6 months.*
For Renewal Surveys: 24 months.

Explanation This element may not be delegated.
The organization allows women direct access to women’s health specialists for routine and preventive health care services. “Direct access” means that the organization cannot require women to obtain a referral or prior authorization as a condition to receiving such services from specialists in the network.

Women’s health specialists include, but are not limited to:

- Obstetricians.
- Gynecologists.
- Certified nurse midwives.

Routine and preventive health care services include, but are not limited to:

- Prenatal care.
- Breast exams.
- Mammograms.
- Pap tests.

Direct access does not prevent the organization from requesting or requiring notification from the practitioner for data collection purposes.

Exceptions

None.

Examples

None.

Element B: Direct Access to Specialists

The organization allows direct access to specialists, appropriate for the condition and identified needs of:

1. Members with special health care needs.
2. Members who need long-term services and supports (LTSS).

Summary of Changes

- No changes to this element.

Scoring

Met	Partially Met	Not Met
The organization meets the requirement	No scoring option	The organization does not meet the requirement

Data source

Materials

Scope of review

This element applies to Interim Surveys, First Surveys and Renewal Surveys.

NCQA reviews the organization's marketing materials, member newsletters, member handbooks or other evidence that demonstrates that members have direct access to specialists appropriate for their condition and identified needs throughout the look-back period.

Look-back period

For Interim Surveys and First Surveys: 6 months.

For Renewal Surveys: 24 months.

Explanation

This element may not be delegated.

The organization allows members with special health care needs and members who need LTSS services to have direct access to specialists, as appropriate for their condition and identified needs.

The organization assesses members to identify those with special health care needs.

Direct access means that the organization cannot require members to obtain a referral or prior authorization as a condition to receiving services from specialists in the network.

Direct access does not prevent the organization from requesting or requiring notification from specialists for data collection purposes.

Exception

Factor 2 is NA if the organization does not provide LTSS services.

Examples

None.

Element C: Second Opinion

The organization provides for a second opinion from an in-network provider or arranges for the member to obtain a second opinion outside the network.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Documented process, Materials		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organization's documented process and marketing materials, member newsletters, member handbooks, provider contract templates, provider communication or other materials for evidence that the requirements were met throughout the look-back period.		
Look-back period	<i>For Interim Surveys and First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	This element may not be delegated. Medicaid law requires that members have the right to a second opinion from a qualified health care professional. If an appropriate professional is not available in-network, the organization arranges for a member to obtain the second opinion out of network at no more cost to the member than if the service was obtained in-network. Exceptions None.		
Examples	None.		

Element D: Out-of-Network Services

If the organization is unable to provide a necessary and covered service to a member in-network, the organization must adequately and timely cover these services out of network for as long as the organization is unable to provide them.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Documented process, Materials		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organization's policies and procedures and communication materials for evidence that the requirements were met throughout the look-back period.		
Look-back period	<i>For Interim Surveys and First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	This element may not be delegated. If the network is unable to provide necessary services covered under its member contract, the organization provides adequate and timely coverage of these services out of network for as long as it is unable to provide them. The organization does not need to arrange or schedule out-of-network services, but it should provide necessary information for members to be able to arrange them. Exceptions None.		
Examples	None.		

Element E: Out-of-Network Cost to Member

If the organization approves a member to go out of network because it is unable to provide a necessary and covered service in-network, the organization:

1. Coordinates payment with the out-of-network practitioner.
2. Ensures that the cost to the member is no greater than it would be if the service was provided in-network.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Documented process, Reports, Materials		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organization's policies and procedures, communication materials and other evidence that the requirements were met throughout the look-back period.		
Look-back period	<i>For Interim Surveys and First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	This element may not be delegated. The organization coordinates payment for care or services provided by an out-of-network practitioner and ensures that the cost to members is no greater than it would be if the service was provided in-network. Exceptions None.		
Examples	Communication with out-of-network practitioners • Letter of agreement.		

Element F: Hours of Operation Parity

The organization requires the hours of operation that practitioners offer to Medicaid members to be no less than those offered to commercial members.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Materials		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organization's materials (e.g., practitioner contract templates, practitioner manual, practitioner newsletters) for evidence that the requirements were met throughout the look-back period.		
Look-back period	<i>For Interim Surveys and First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	This element may not be delegated. The organization ensures that network practitioners offer hours of operation that are no less (in number or scope) than the hours of operation offered to non-Medicaid members. If the practitioner serves only Medicaid recipients, hours offered to Medicaid managed care members are comparable to those offered to Medicaid fee-for-service members. Exceptions None.		
Examples	None.		

Element G: 24-Hour Availability of Contracted Services

The organization makes services included in its Medicaid contract available 24 hours a day, 7 days a week, when medically necessary.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Documented process, Materials		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organization's policies and procedures and materials (e.g., practitioner contract templates, practitioner manual, practitioner newsletters) for evidence that the requirements were met throughout the look-back period.		
Look-back period	<i>For Interim Surveys and First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	This element may not be delegated. The organization makes services included in its Medicaid contract available 24 hours a day, 7 days a week, when medically necessary. The organization's policies and procedures outline which medically necessary services warrant this type of availability. Exceptions None.		
Examples	None.		

Element H: Notification of Termination of a Practitioner or Practice Group

The organization provides written notification to affected members of termination of a practitioner or practice group by the later of 30 calendar days prior to the effective date of termination, or within 15 calendar days after receipt or issuance of the termination notice.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Documented process, Reports, Materials		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organization's policies and procedures or decision process, and evidence that members were notified of practitioner or practice group termination.		
Look-back period	<i>For Interim Surveys and First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	The organization notifies all members affected by the termination, including those who received primary care from, or were seen on a regular basis by, the terminated practitioner. Distribution of termination notice to members The organization distributes the notification to members by mail, fax or email, or on its website, if it informs members that the information is available online. The organization mails the notification to members who do not have fax, email or internet access. If the organization distributes information electronically, it must indicate that the information is available in paper format, upon request. Exceptions None.		
Examples	None.		

Element I: Advance Directives

The organization provides information to members about advance directives, in accordance with applicable state laws.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Documented process, Reports, Materials		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organization's policies and procedures for advance directives, evidence of applicable state law on advance directives, and reviews evidence that the organization provides information to members about advance directives.		
Look-back period	<i>For Interim Surveys and First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	This element may not be delegated. An advance directive is a document in which members specify the type and extent of health care services they want if they cannot make medical decisions. The organization provides information regarding advance directives to adult members. The information includes a description of applicable state law. If the state law changes, the organization updates its policies within 90 calendar days of the effective date of the change. Distribution of information about advance directives The organization distributes information to members by mail, fax or email, or on its website, if it informs members that the information is available online. The organization mails the information to members who do not have fax, email or internet access. Exceptions None.		
Examples	None.		

Element J: Physician Incentive Plans

The organization provides information about physician incentive plans to members, upon request.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Documented process, Reports, Materials		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organization's policies and procedures and reviews evidence that the organization distributes information about its physician incentive plans to members upon request.		
Look-back period	<i>For All Surveys:</i> 6 months.		
Explanation	A physician incentive plan is any compensation arrangement to pay a physician or physician group that may directly or indirectly have the effect of reducing or limiting the services provided to any plan member. When appropriate, the organization provides information about any physician incentive plans in place to members, upon request. The organization's report includes the date of the member's request and the date when the organization provided the information to the member. Distribution of physician incentive plans Upon request, the organization distributes information about physician incentive plans to members by mail, fax or email, or on its website, if it informs members that the information is available online. The organization mails the information to members who do not have fax, email or internet access. Exception This element is NA for organizations that do not utilize physician incentive plans.		
Examples	None.		

Element K: Machine-Readable Data

The organization makes the following available on its website in a machine-readable file and format:

1. Practitioner and provider directories.
2. Formulary drug lists.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Materials		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organization's website content or screenshots of its website as evidence that information is provided in a machine-readable file and format. NCQA also reviews a statement on the website or in other applicable documentation specifying that the information's format was in accordance with CMS regulations and guidance. Both must be in place throughout the look-back period.		
Look-back period	<i>For All Surveys:</i> 6 months.		
Explanation	The organization provides links to practitioner and provider directories and formulary drug lists in a machine-readable file and format on its website. The machine-readable file and format (as specified by the Department of Health and Human Services) provide the opportunity for software developers to create resources that aggregate information and create tools to improve transparency. Factor 1: Practitioner and provider directories Factor 1 applies to all network providers, practitioners, practitioner groups and facilities that have a network agreement. Factor 2: Formulary drug lists The formulary drug list includes the following information: • Covered medications (generic and name brand). • Medication tiers. Exceptions None.		
Examples	None.		

Element L: Excluded Practitioners and Providers

The organization does not employ or contract with practitioners and providers excluded from participation in federal health care programs.

Summary of Changes**Clarifications**

- Added in the Explanation that this element may not be delegated.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Documented process		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organization's policies and procedures regarding exclusion of practitioners and providers from employment or contract.		
Look-back period	<i>For All Surveys:</i> 6 months.		
Explanation	This element may not be delegated. The organization's policies and procedures describe its process for ensuring that it does not employ or contract with practitioners and providers excluded from participation in federal health care programs under either section 1128 or 1128A of the Social Security Act. The organization uses the verification process described in CR 1, Element A, factors 3 and 4 and CR 7, Element A, factor 1, and uses verification data from CR 3, Element B, CR 3, Element C and CR 7, Elements D and E to ensure that the organization does not employ or contract with practitioners and providers that are excluded from federal health care programs. Exceptions None.		
Examples	None.		

MED 2: Practice Guidelines

The organization is accountable for adopting and disseminating clinical practice guidelines relevant to its members for the provision of preventive, acute or chronic medical services and behavioral healthcare services.

Intent

The organization uses clinical practice guidelines to help practitioners make decisions about appropriate health care for specific clinical circumstances and behavioral healthcare services.

Element A: Adoption of Practice Guidelines

The organization adopts at least four evidence-based clinical practice guidelines, approved by its QI committee, that:

1. Are based on valid and reliable clinical evidence or a consensus of practitioners in the particular field.
2. Consider the needs of the organization's members.
3. Are adopted in consultation with contracted health care professionals.
4. Are reviewed and updated at least every two years, as applicable.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Documented process, Reports, Materials		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews at least four of the organization's clinical practice guidelines in place throughout the look-back period and evidence of updating the guidelines, as applicable, within the look-back period. <i>For factor 4, NCQA reviews the organization's policies and procedures for updating the guidelines if all guidelines are less than two years old.</i>		
Look-back period	<i>For Interim Surveys and First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	Factor 1: Guidelines based on clinical evidence The organization's QI Committee or other designated clinical committee approves clinical practice guidelines. The organization adopts guidelines from recognized sources. It uses one of the following in adopting its clinical practice guidelines: • Scientific evidence, or • Professional standards, or • Expert opinion.		

Factor 2: Guidelines that consider member needs

The organization adopts clinical practice guidelines that consider the needs of its members, which may include guidelines related to any applicable acute or chronic conditions, behavioral health-related issues and preventive or nonpreventive guidelines.

Factor 3: Adoption in consultation with contracted health care professionals

The organization adopts guidelines with feedback from board-certified, contracted practitioners from appropriate specialties who would use the guidelines. Evidence of appropriate specialty involvement may come through:

- Participation on a committee, **or**
- The organization's consideration of comments from contracted practitioners to whom guidelines were circulated.

Factor 4: Review and update

The organization reviews its guidelines against clinical evidence at least every two years, or more frequently, if national guidelines change within the two-year period.

Exceptions

None.

Related information

Behavioral healthcare guidelines. Because behavioral healthcare clinical practice guidelines often address primary, secondary or tertiary prevention as part of the management of acute or chronic conditions, the organization may adopt preventive or nonpreventive behavioral health clinical practice guidelines.

Collaborative activities. If the organization collaborates with other organizations to develop guidelines, either the collaborative group or the organization may review and update the guidelines.

Examples**Adopting guidelines**

The organization reviewed its HEDIS results for the past two years to determine the quality of care for the conditions reported in the measures. Analysis of the data and discussions with practitioners resulted in development of the following guidelines:

- Treatment of Diabetes.
- Treatment of Heart Failure.
- Screening for Depression in People With Diabetes.
- Screening and Treatment of Attention Deficit Hyperactivity Disorder (ADHD).

Behavioral health aspect of a medical guideline

The organization's diabetes guidelines include screening for depression in people with diabetes. The organization presents Treatment of Diabetes as a medical guideline and Screening for Depression in People With Diabetes as a behavioral health guideline. It may not present another screening guideline for depression.

Evidence-based guidelines

- National Guideline Clearinghouse (www.guideline.gov) guidelines.

Recognized sources

- Professional medical associations:
 - American Medical Association.
 - American College of Cardiology.
 - American Orthopedic Association.
 - American Academy of Pediatrics.
- Voluntary health organizations:
 - American Diabetes Association.
 - American Heart Association.
 - American Stroke Association.
 - American Lung Association.
- NIH Centers and Institutes:
 - National Heart Lung and Blood Institute.
 - National Cancer Institute.

Element B: Distribution of Practice Guidelines

The organization distributes the evidence-based guidelines it adopted in MED 2, Element A, to the appropriate practitioners and to members and potential members, upon request.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Documented process, Materials		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organization's policies and procedures and materials containing practice guidelines for distribution to practitioners, members and potential members, if requested.		
Look-back period	<i>For Interim Surveys and First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	Distributing guidelines The organization distributes clinical practice guidelines and revisions to all practitioners who are likely to use them. The organization also distributes practice guidelines to members and potential members, upon request. The organization distributes guidelines by mail, fax or email, or on its website, if it informs practitioners that the information is available online. The organization mails the information to practitioners, members and potential members who do not have fax, email or internet access. If the organization does not directly contract with practitioners, it may rely on the contracted entity to distribute practitioner information to practitioners.		

Exceptions

None.

Related information

Collaborative activities. If the organization collaborates with other organizations to develop guidelines, either the collaborative group or the organization may review, update and distribute the guidelines. The organization distributes guidelines to practitioners if the collaborative group does not.

Examples None.

Element C: Application of Guidelines

The organization makes decisions for member education that are consistent with its practice guidelines.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Documented process, Materials		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organization's policies and procedures and evidence that the organization's decision making regarding member education is consistent with its practice guidelines.		
Look-back period	<i>For Interim Surveys and First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	This element may not be delegated. The organization makes decisions regarding member education that are consistent with clinically relevant and evidence-based practice guidelines.		
	Exceptions None.		
Examples	None.		

MED 3: Practitioner Office Site Quality

The organization has a process to ensure that the offices of all practitioners meet its office-site standards.

Intent

The organization assesses the quality, safety and accessibility of office sites where care is delivered.

Element A: Performance Standards and Thresholds

The organization sets site performance standards and thresholds for:

1. Accessibility equipment.
2. Physical accessibility.
3. Physical appearance.
4. Adequacy of waiting and examining room space.
5. Adequacy of medical/treatment record keeping.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement

Data source Documented process

Scope of review *This element applies to Interim Surveys, First Surveys and Renewal Surveys.*
NCQA reviews the organization's site performance standards and thresholds.

Look-back period *For Interim Surveys and First Surveys: 6 months.*
For Renewal Surveys: 24 months.

Explanation This element may not be delegated.
This element applies to practitioners in the scope of credentialing as defined in *CR 1: Credentialing Policies, Element A.*

Factor 1: Accessible equipment

The organization has standards and thresholds for providing accessible equipment for members with physical or mental disabilities. This includes scales and exam tables and equipment for treatment and diagnosis.

Factor 2: Physical accessibility

The organization has standards and thresholds for:

- Ease of entry into the building or practice site.
- Accessible space in the building or practice site.
- Ease of access for physically disabled members.

Factor 3: Physical appearance

The organization has standards and thresholds for cleanliness, lighting and safety.

Factor 4: Adequacy of waiting and exam room space

The organization has standards and thresholds for the appropriate size and seating for waiting rooms.

Factor 5: Adequacy of medical/treatment record keeping

The organization has standards and thresholds for medical/treatment record orderliness, security, confidentiality and documentation practices.

Exceptions

Factors 1–4 are NA for practitioners who only provide care in members' homes and practitioners who provide telemedicine services.

Examples**Office-site criteria**

The organization includes the following components in its office-site criteria and sets thresholds for each component.

- Physical accessibility and appearance:
 - Handicapped accessible.
 - Well-lit waiting room.
 - Adequate seating.
 - Posted office hours.
- Paper-based medical records:
 - Secure/confidential filing system.
 - Legible file markers.
 - Records can be easily located.

Element B: Site Visits and Ongoing Monitoring

The organization implements appropriate interventions by:

1. Continually monitoring member complaints for all practitioner sites.
2. Conducting site visits of offices within 60 calendar days of determining that the complaint threshold was met.
3. Instituting actions to improve offices that do not meet site standards and thresholds in Element A.
4. Evaluating the effectiveness of the actions at least every six months, until deficient offices meet the site standards and thresholds in Element A.
5. Documenting follow-up visits for offices that had subsequent deficiencies.

Summary of Changes

- No changes to this element.

Scoring

Met	Partially Met	Not Met
The organization meets the requirement	No scoring option	The organization does not meet the requirement

Data source	Documented process, Reports, Records or files
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organization's policies and procedures, evidence of routine monitoring and evaluation of up to five practice sites during the look-back period.
Look-back period	<i>For Interim Surveys and First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>
Explanation	This element applies to practitioners in the scope of credentialing as defined in <i>CR 1: Credentialing Policies</i>, Element A. Factor 1: Monitor member complaints The organization: • Establishes a reasonable complaint threshold for an office-site visit that takes the severity of an issue into account. • Has a process to monitor and investigate member complaints related to the quality of practitioner office sites. Factor 2: Site visits The organization: • Uses a standardized survey form that incorporates office-site criteria for assessment of offices that have received member complaints. • Conducts site visits for complaints about physical accessibility, physical appearance and adequacy of waiting and examining room space (Element A, factors 1–3) if the complaint threshold is met. • Performs the site visit within 60 calendar days of the complaint threshold being met. Factors 3, 4 No additional explanation required. Factor 5: Follow-up visit If subsequent to correcting a deficiency, the office site meets the complaint threshold for a different standard, the organization performs a follow-up visit within 60 calendar days on the performance standard pertaining to that complaint. The organization develops and implements an action plan for improvement. Exceptions Factors 1 and 2 are NA for practitioners who only provide care in members' homes and who provide telemedicine services. Factors 2–5 are NA if all sites meet the organization's performance thresholds.
Examples	Methods for detecting deficiencies • Complaint monitoring. • Practice-specific member surveys. • Reports from Provider Relations staff visits. • Staff audit.

MED 4: Privacy and Confidentiality

The organization protects the confidentiality of member information and records.

Intent

The organization uses and discloses member protected health information (PHI) appropriately to protect member privacy.

Element A: Adopting Written Policies for Privacy and Confidentiality

The organization adopts written policies and procedures that address:

1. Information included in notification of privacy practices.
2. Access to PHI.
3. The process for members to request restrictions on use and disclosure of PHI.
4. The process for members to request amendments to PHI.
5. The process for members to request an accounting of disclosures of PHI.
6. Internal protection of oral, written and electronic information across the organization.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement

Data source Documented process

Scope of review *This element applies to Interim Surveys, First Surveys and Renewal Surveys.*
NCQA reviews the organization's policies and procedures.

Look-back period *For Interim Surveys and First Surveys: 6 months.*
For Renewal Surveys: 24 months.

Explanation This element may not be delegated.
Compliance with HIPAA regulations meets this element.
This element is not intended to conflict with the organization's responsibility to comply with HIPAA and other federal and state laws. The organization obtains all member consent and authorization required by HIPAA and other federal and state laws, and accesses, uses and shares the minimum amount of PHI necessary to perform the services it is obligated to perform.

Factor 1: Notification of privacy practices

PHI policies and procedures state that the organization notifies members of its privacy practices at enrollment and annually thereafter.

Factors 2–5: Processes for handling member requests

The organization has a process to receive, analyze and resolve member requests.

The organization is not required to approve all requests, but has a process for evaluating requests and deciding on a course of action.

The organization is not required to account for:

- Disclosures of treatment, payment and health care operations.
- Member-authorized disclosures.

Factor 6: Protecting PHI

The organization's policies and procedures state:

- How it protects the privacy of oral, written and electronic PHI from unauthorized or inappropriate use.
- That when it transmits PHI to another organization, as permitted by law, the PHI is protected according to the organization's specifications.

Exceptions

None.

Related information

Subscriber notification. Certain organizations, particularly those with open-access products, may not be able to provide the notice of privacy practices to all members at enrollment because they may not know who all members are. For these products, NCQA accepts notification of privacy practices at enrollment, although this may not be compliant with federal HIPAA privacy regulations.

Examples**Factor 6: Protections for oral PHI**

- Policies prohibiting staff discussions outside of the organization's offices (e.g., elevators, hallways).
- Procedures for ensuring that staff have contacted the correct member before revealing or discussing PHI.

Element B: Authorization

The organization has policies and procedures that address members' right to authorize or deny the release of PHI beyond uses for treatment, payment or health care operations.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Documented process		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organization's policies and procedures.		
Look-back period	<i>For Interim Surveys and First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	This element may not be delegated. Policies and procedures may: • Specify situations when the organization is required to obtain authorization for release of PHI. • Specify situations when the organization is not required to obtain authorization for release of PHI. • Give an example of authorization initiated by the organization.		
	Exceptions None.		
Examples	None.		

Element C: Communication of PHI Use and Disclosure

Upon member enrollment and annually thereafter, the organization distributes a notice of privacy practices to members that includes:

1. The organization's routine use and disclosure of PHI.
2. Use of authorizations.
3. Access to PHI.
4. Internal protection of oral, written and electronic PHI across the organization.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Reports, Materials		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organization's notice of privacy practices distributed to members. <i>For Interim Surveys and First Surveys:</i> NCQA also reviews the organization's most recent annual distribution of materials to members during the look-back period. <i>For Renewal Surveys:</i> NCQA reviews also the organization's most recent and previous annual distribution of materials to members during the look-back period.		
Look-back period	<i>For Interim Surveys and First Surveys:</i> 6 months for enrollment, and at least once during the prior year for annual distribution. <i>For Renewal Surveys:</i> 24 months.		
Explanation	This element may not be delegated. Distribution of PHI communication The organization distributes information to members by mail, fax or email, or on its website, if it informs members that the information is available online. The organization mails the information to members who do not have fax, email or internet access. Factors 1–4 No additional explanation required. Exceptions None.		
Examples	None.		

MED 5: Care Coordination

The organization has a process to coordinate services for Medicaid members and to maintain and share health records in accordance with professional standards.

Intent

The organization assigns a person or entity responsible for coordinating member health care services and maintains and shares medical record information in accordance with professional standards.

Element A: Coordinating Health Care Services for Members

The organization's care coordination process includes provisions for all members, including:

1. Having a person or entity formally assigned to coordinate health care services accessed by members.
2. Providing the contact information of the individuals coordinating healthcare services to members.
3. Ensuring coordination between settings of care.
4. Coordinating services members receive from any other organizations.
5. Coordinating services members receive in fee-for-service Medicaid.
6. Coordinating services members receive from community and social support providers.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement

Data source Documented process, Reports, Materials

Scope of review *This element applies to Interim Surveys, First Surveys and Renewal Surveys.*
NCQA reviews the organization's policies and procedures for coordinating care for all members for factors 1–6. NCQA also reviews reports or materials for factor 2, for evidence that all members were provided information on how to contact their designated person or entity.

Look-back period *For Interim Surveys and First Surveys: 6 months.*
For Renewal Surveys: 24 months for factors 1 and 2; 6 months for factors 3–6.

Explanation **Factor 1: Formally assigned person or entity**

The organization provides members with a person or entity responsible for coordinating the health care services accessed by members.

Factor 2: Contact information

The organization ensures that the member is provided information on how to contact their designated person or entity.

The organization distributes information to members by mail, fax or email, or on its website, if it informs members that the information is available online. The organization mails the information to members who do not have fax, email or internet access.

Factor 3: Coordination of services between settings of care

The organization's policies and procedures outline a process for coordinating between settings of care, including appropriate discharge planning for short-term and long-term hospital and institutional stays.

Factors 4–6

No additional explanation.

Exceptions

None.

Examples **Factor 4: Types of organizations**

- Managed care organizations.

Element B: Maintaining and Sharing Member Health Records

The organization requires:

1. Practitioners to maintain member health records, as appropriate and in accordance with professional standards.
2. Practitioners to share member health records, as appropriate and in accordance with professional standards.
3. Providers to maintain member health records, as appropriate and in accordance with professional standards.
4. Providers to share member health records, as appropriate and in accordance with professional standards.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement

Data source Documented process, Reports, Materials

Scope of review *This element applies to Interim Surveys, First Surveys and Renewal Surveys.*
NCQA reviews the organization's policies and procedures for maintaining and sharing health records. NCQA also reviews evidence that the organization shared health records with practitioners and providers during the look-back period.

Look-back period *For Interim Surveys and First Surveys: 6 months.*
For Renewal Surveys: 24 months.

Explanation The organization requires health records to be maintained in a manner that is current, detailed and organized to facilitate communication and coordination of care.

The organization establishes medical record standards by ensuring that practitioners and providers in its network comply with the standards.

The organization's medical record policies, procedures and documentation standards discuss the following:

- Medical record content.
- Medical record organization.
- Ease of retrieving medical records.
- Confidential patient information.
- Standards and performance goals for participating practitioners.

The organization distributes the medical record standards to all practitioners and appropriate staff members.

The organization's policies and procedures for primary care medical records reflect the following information:

- All services provided directly by a practitioner who provides primary care services.
- All ancillary services and diagnostic tests ordered by a practitioner.
- Reports of all diagnostic and therapeutic services for which a member was referred by a practitioner, such as:
 - Home health nursing.
 - Specialty physician.
 - Hospital discharge.
 - Physical therapy.

Medical record documentation standards

Each medical record includes the following information:

- History and physicals.
- Allergies and adverse reactions.
- Problem list.
- Medications.
- Documentation of clinical findings and evaluation for each visit.
- Preventive services/risk screening.

Confidentiality of medical records

- Records are stored securely.
- Only authorized personnel have access to records.
- Staff receive periodic training in member information confidentiality.

Sharing medical record information

The organization shares medical record information, as appropriate, with network providers and care coordinators.

Factors 1–4

No additional explanation required.

Exceptions

None.

Examples

None.

Element C: Care Plan

The organization's care planning process includes how the care plan is:

1. Developed by an individual trained in patient-centered planning.
2. Approved by the organization in a timely manner.
3. In accordance with any applicable state quality assurance and utilization review standards.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement

Data source Documented process

Scope of review *This element applies to Interim Surveys, First Surveys and Renewal Surveys.*
NCQA reviews the organization's policies and procedures for developing a care plan.

Look-back period *For All Surveys: 6 months.*

Explanation Factors 1–3 apply to all LTSS members. Factors 2 and 3 also apply to members with special health care needs, if the state requires, who have been determined through assessment to need a course of treatment or regular care monitoring.

Factor 1: Trained individual

The organization's policies and procedures specify a process for ensuring that the care plan is developed by a person who is trained in:

- Person-centered planning.
- Using a person-centered process and plan.
- Providing care that is driven by the preferences, needs and values of the individual.

Person-centered planning process and plan are defined in §441.301(c)(1) and (2).

Person-centered planning involves viewing, listening to and supporting individuals to make decisions for maintaining a life that is meaningful to them, based on their strengths, abilities, aspirations and preferences. The resulting care plan reflects the goals and interests of the individual. Individuals should be involved in the care planning process to the extent they prefer.

Factor 2: Approval of the care plan

The organization's policies and procedures specify whether approval of the care plan is required. If approval is required, the organization's process describes how it establishes an approval time frame to ensure timely case management services to members.

Factor 3: Quality assurance and utilization review

No additional explanation.

Exception

Factor 2 is NA if the organization does not require approval of treatment or service plans.

Examples

None.

MED 6: Initial Screening and Assessment of Members

The organization has a process to conduct an initial screening and an assessment of new members.

Intent

The organization conducts an initial screening of new members' health care needs and an assessment and shares this information with the state to prevent duplication of efforts.

Element A: Initial Screening of Member Needs

The organization conducts an initial screening of the health care needs of all new members within 90 calendar days of enrollment.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Documented process, Reports		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organization's policies and procedures for conducting an initial screening for new members within 90 days of enrollment and reviews reports showing that the organization performed the screening within the required time frame.		
Look-back period	<i>For Interim Surveys and First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	The organization conducts an initial screening of new members' health care needs, including whether they need case management services, within 90 calendar days of enrollment. At least 90 percent of the new member screenings must be completed in this time frame in order to meet the requirement. The initial screening is conducted through a survey type instrument used to gather information related to the health needs of all members. This instrument may be a health assessment that includes screenings for functional and social risk factors (e.g., physical disability, cognitive limitations, mental health issues, housing insecurity). If the organization is unable to complete the initial screening in the specified time frame, it makes three or more attempts to contact or locate the member (by telephone, regular mail, email or fax) and documents its efforts. Exceptions None.		
Examples	None.		

Element B: Sharing Identification and Assessment Results

The organization shares the results of its identification and assessment of members with:

1. The state.
2. Other organizations serving the member.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Documented process, Reports, Materials		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organization's policies and procedures for sharing its results and also reviews evidence that the organization shared result information with the state and, if applicable, with at least one other organization at least once during the look-back period.		
Look-back period	<i>For Interim Surveys and First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	To prevent duplicated services, the organization shares results of identification and assessment of member needs with the state or with other organizations serving the member. Factors 1, 2: Sharing information No additional explanation required. Exception Factor 2 is NA if there is a contract with the state that prohibits the organization from sharing with other organizations.		
Examples	None.		

MED 7: Quality Assessment and Performance Improvement

The organization establishes and implements an ongoing comprehensive quality assessment and performance improvement program for the services it provides to its members.

Intent

The organization establishes and implements an ongoing comprehensive quality assessment and performance improvement program.

Element A: Quality Assessment and Performance Improvement Program

The organization's comprehensive quality assessment and performance improvement program includes:

1. Mechanisms to detect underutilization and overutilization.
2. Mechanisms to assess the quality and appropriateness of care provided to members with special health care needs.
3. Mechanisms to assess the quality and appropriateness of care provided to members using LTSS.
4. Participation in efforts to prevent, detect and remediate critical incidents for members with LTSS needs.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement

Data source Documented process

Scope of review *This element applies to Interim Surveys, First Surveys and Renewal Surveys.*
NCQA reviews the organization's policies and procedures.

Look-back period *For Interim Surveys and First Surveys: 6 months.*
For Renewal Surveys: 24 months.

Explanation This element may not be delegated.

Factors 1, 2: Mechanisms for detecting and assessing

The organization's policies and procedures specify its mechanisms for detecting under- and overutilization of services and for assessing the quality and appropriateness of care provided to members with special needs.

Factor 3: Care provided for LTSS

The organization's policies and procedures address the ways in which it assesses quality and appropriateness of care given to members using LTSS.

The assessment must include an evaluation of care between settings and a comparison of services and supports received against what is documented in the member's case management plan.

Factor 4: Critical incidents

The organization's policies and procedures specifies the manner in which the organization participates in the state's efforts to prevent, detect and remediate critical incidents for members who need LTSS. Critical incidents are occurrences that adversely impact a member's health and welfare and the achievement of quality outcomes identified in person-centered planning.

The organization's efforts must be in a manner that are consistent with state requirements for home and community-based waiver programs.

Exceptions

Factors 3 and 4 are NA if the organization does not provide LTSS.

Examples

Factor 1: Mechanisms to detect underutilization and overutilization

The organization establishes thresholds for detecting underutilization and overutilization.

MED 8: Informing Members of Services

The organization informs members of services related to benefits and appeals.

Intent

Information regarding benefits and the appeal process is accessible to members.

Element A: Obtaining Benefit Information

The organization provides the following information to members in the member handbook:

1. Benefits provided by the state and how and where to access the benefits, including transportation.
2. Services not covered or provided for by the organization because of moral or religious objections.
3. How to obtain information from the state about how and where to access services not covered or provided because of moral or religious objections.
4. The extent to which, and how, members may obtain covered benefits from out-of-network providers, including family planning services and supplies.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement

Data source Materials

Scope of review *This element applies to Interim Surveys, First Surveys and Renewal Surveys.*
NCQA reviews the organization's member handbook.

Look-back period *For Interim Surveys and First Surveys: 6 months.*
For Renewal Surveys: 24 months for factors 1–3; 6 months for factor 4.

Explanation This element may not be delegated.

Factor 1: How to access benefits provided by the state

The organization informs members how and where to access benefits provided by the state, including how transportation (e.g., to obtain medically necessary services, such as trips to the pharmacy) is provided.

Factor 2: Services not covered by the organization due to moral or religious objections

The organization informs members when services are not covered or provided for any counseling or referral services due to moral or religious objections of the organization.

Factor 3: How to obtain information from the state

The organization informs members how to obtain information from the state about how and where to access services not covered or provided under factor 2.

Factor 4: How to obtain out-of-network services

The organization provides information to members about the extent to which, and how, they may obtain out-of-network services, including family planning services and supplies. Information includes a statement that the organization may not require a member to obtain a referral before choosing a family planning provider.

Exception

Factors 2 and 3 are NA if the organization does not have any services it does not provide because of moral or religious objections.

Examples None.

Element B: Obtaining Appeal Information

The organization provides information to members regarding how to request assistance from the organization in filing an appeal in the member handbook.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Materials		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organization's member handbook for evidence that it meets the requirements.		
Look-back period	<i>For Interim Surveys and First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	This element may not be delegated. The organization informs members about its process for requesting assistance in filing an appeal. Assistance may include providing language or interpreter services or aid for members with vision or hearing preferences. Exceptions None.		
Examples	None.		

Element C: Filing a Grievance and Requesting a State Fair Hearing

The organization provides the following information to members in the member handbook:

1. The right to file grievances.
2. The requirements and time frames for filing a grievance.
3. The availability of assistance with the filing process for grievances and appeals.
4. The right to request a State Fair Hearing after a member has received an adverse appeal decision.
5. That members may be required to pay the cost of services provided while the appeal or State Fair Hearing is pending.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Materials		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organization's member handbook for evidence that it meets the requirements.		
Look-back period	<i>For Interim Surveys and First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	This element may not be delegated. The organization's member handbook informs members about the grievance and appeal process. Factors 1–5 No additional explanation required. Exceptions None.		
Examples	None.		

Element D: Informing Members About the QI Program

The organization annually makes information about its QI program available to members.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Reports, Materials		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> <i>For Interim Surveys and First Surveys:</i> NCQA reviews the most recent distribution of QI program information for content, methods of distribution, timeliness and applicable notification of information availability. <i>For Renewal Surveys:</i> NCQA reviews the most recent and the previous year's two most recent annual distributions of QI program information for content, methods of distribution, timeliness and applicable notification of information availability.		
Look-back period	<i>For Interim Surveys and First Surveys:</i> At least once during the prior year. <i>For Renewal Surveys:</i> 24 months.		
Explanation	This is a structural requirement. The organization must present its own documentation. Communication of QI program information The annual QI program information includes information about QI program processes, goals and outcomes as they relate to member care and services, in language that is easy to understand. The organization may provide detailed information (such as a full program description), or information may be limited to an executive summary or outline of the QI program and its achievements. The organization distributes information by regular mail, email or, fax, through messages to members' mobile devices or on its website. The organization mails the information to members who do not have internet access. If the organization posts the information on its website, it notifies the target audience of the availability of the information on the web through another method listed. Note: Information may be made available by request if the organization notifies members that the information is available by request. Exceptions None.		

Related information

Collaborative activities. If the organization is involved in collaborative QI activities, either the collaborative group or the organization may distribute information about the QI program to members. The organization is responsible for providing information to members not included in the collaborative group's distribution process.

Examples None.

Element E: Informing Members About Member Handbook Changes

The organization provides written notification to members regarding any significant changes to the member handbook at least 30 days before the intended effective date of the change.

Summary of Changes

- No changes to this element.

Scoring

Met	Partially Met	Not Met
The organization meets the requirement	No scoring option	The organization does not meet the requirement

Data source Documented process, Reports, Materials

Scope of review *This element applies to Interim Surveys, First Surveys and Renewal Surveys.*
NCQA reviews the organization's policies and procedures and evidence that members were notified of any significant changes to the member handbook.

Look-back period *For All Surveys:* 6 months.

Explanation The organization gives all members notice of significant changes (as defined by the state) to the member handbook.

Distribution of termination notice to members

The organization distributes the notification to members by mail, fax or email, or on its website, if it informs members that the information is available online. The organization mails the notification to members who do not have fax, email or internet access.

If the organization distributes the notification electronically, it must indicate that the information is available in paper format, upon request.

Exception

This element is NA if there are no significant changes (as defined by the state) to the member handbook.

Examples None.

MED 9: UM Decisions About Payment and Services

The organization makes decisions about utilization management requests in a timely manner for Medicaid members.

Intent

The organization meets the established time frames for making decisions and authorizing payment for services.

Element A: Advance Notice

The organization's policies and procedures specify that for terminations, suspensions or reductions of previously authorized Medicaid-covered services, the organization gives electronic or written advance notice to practitioners and members at least 10 days before the date of action.

Summary of Changes

Clarifications

- Added in the Explanation that this element may not be delegated.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement

Data source Documented process, Reports, Materials

Scope of review *This element applies to Interim Surveys, First Surveys and Renewal Surveys.*
NCQA reviews the organization's policies and procedures in place throughout the look-back period and reviews evidence that members and practitioners were notified of terminations, suspensions or reductions of previously authorized Medicaid-covered services.

Look-back period *For All Surveys: 6 months.*

Explanation This element may not be delegated.
Even if there were no terminations, suspensions or reductions of previously authorized Medicaid-covered services, the organization must have policies and procedures for providing advance notice to members and practitioners.

Advance notice in cases of probable fraud

The organization's policies and procedures state that the time frame for advance notice may be shortened to 5 days before the date of action if:

- The agency has facts indicating that action should be taken because of probable fraud by the member, **and**
- The facts have been verified through secondary sources, if possible.

Exceptions from advance notice

The organization's policies and procedures specify that advance notice may be sent at any point up to the date of action, but no later than the date of action, if any scenario specified in § 428.213 is met.

Distribution of advance notice to members and practitioners

The organization distributes the notification by mail, fax or email, or on its website, if it informs members and practitioners that the information is available online. The organization mails the notification to members and practitioners who do not have fax, email or internet access.

Exceptions

None.

Examples None.

Element B: UM Denial Notifications

The organization's policies and procedures specify that the following information is included in the UM denial notification:

1. Information about exhausting the organization's appeal process and ability to request a State Fair Hearing.
2. Information about the member's access to and copies of all documents used to make the decision.
3. The circumstances under which an appeal of a denial can be expedited.
4. The member's right to have benefits continue pending the resolution of an external appeal and the circumstances under which the organization may seek reimbursement.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Documented process		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organization's policies and procedures.		
Look-back period	<i>For Interim Surveys and First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	This element may not be delegated. NCQA reviews the organization's policies and procedures that specify the content of UM denial notifications for its Medicaid members.		

Factor 1: Notification of the right to request a State Fair Hearing

The organization's policies and procedures specify that if the organization fails to make a decision in a timely manner regarding the member's appeal request, the member may initiate a State Fair Hearing.

Factor 2: Information used to make decisions

The organization specifies in its policies and procedures that UM denial notifications include member access to and copies of all relevant information used to make the denial decision. This includes any processes, strategies or evidentiary standards used in setting coverage limits, and any documentation, records and other information relevant to the member's request.

Factor 3: Expedited appeals

No additional information required.

Factor 4: Continuing coverage of benefits

The organization includes in its denial notifications to members (and their treating practitioners) a statement about the right to have benefits continue, pending the outcome of an external appeal.

The notification also includes information on how to request that benefits be continued, and the circumstances, consistent with state policy, under which members may be required to pay the costs of these services.

Exceptions

None.

Examples None.

Element C: LTSS Requests for Initial and Continuing Authorization of Services

The organization requires that decisions to deny LTSS services be made by an individual with expertise in LTSS.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement

Data source Documented process, Materials

Scope of review *This element applies to Interim Surveys, First Surveys and Renewal Surveys.*
NCQA reviews the organization's policies and procedures and materials for evidence that LTSS decision makers have expertise in LTSS.

Look-back period *For Interim Surveys and First Surveys: 6 months.*
For Renewal Surveys: 24 months.

Explanation	This element may not be delegated. The organization's policies and procedures specify that a decision to deny an LTSS service authorization request or to authorize a LTSS service in an amount, duration or scope that is less than requested is made by an individual with expertise in addressing members' LTSS needs. The organization specifies the expertise (e.g., relevant experience, training, education and certifications) needed to address members with LTSS needs. Exception This element is NA if the organization does not provide LTSS services.
Examples	None.

Element D: Coverage of Emergency and Post-Stabilization Services

The organization's policies and procedures require coverage of emergency services in the following situations:

1. To screen and stabilize the member without prior approval, where a prudent layperson, acting reasonably, would have believed that an emergency medical condition existed.
2. If an authorized representative, acting for the organization, authorized the provision of emergency services.
3. To provide post-stabilization care services for the member.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement

Data source	Documented process
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organization’s policies and procedures.
Look-back period	<i>For Interim Surveys and First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>
Explanation	This element may not be delegated. The organization provides, arranges for or otherwise facilitates all needed emergency services, including appropriate coverage of costs. The organization meets this element if its policies and procedures state that it covers all ER and post-stabilization care claims or does not deny any ER and post-stabilization care claims.

Factor 1: Prudent layperson

A prudent layperson is a person who is without medical training and who draws on practical experience to decide if there is a need to seek emergency medical treatment.

The organization's policies and procedures specify that it considers the member's presenting symptoms when determining if the definition of "prudent layperson" is met.

Factor 2: Organization representative

An organization representative is an employee or contractor who directs members to organization services.

Factor 3: Post-stabilization care services

Post-stabilization care services are services related to an emergency medical condition and provided after the member is stabilized, to maintain the stabilized condition or improve the condition.

The organization's policies and procedures specify that the attending emergency physician or the treating practitioner determines when the member is stabilized sufficiently for transfer or discharge.

Exceptions

None.

Examples

None.

Element E: Affirmative Statement About Incentives

The organization distributes a statement to all members and to all practitioners, providers and employees who make UM decisions, affirming the following:

1. UM decision making is based only on appropriateness of care and service and existence of coverage.
2. The organization does not specifically reward practitioners or other individuals for issuing denials of coverage.
3. Financial incentives for UM decision makers do not encourage decisions that result in underutilization.

Summary of Changes

- No changes to this element.

Scoring

Met	Partially Met	Not Met
The organization meets the requirement	No scoring option	The organization does not meet the requirement

Data source

Reports, Materials

Scope of review

This element applies to Interim Surveys, First Surveys and Renewal Surveys.
NCQA reviews the organization's affirmative statement against the required factors.

For First Surveys and Renewal Surveys: NCQA also reviews the organization's communication of the affirmative statement to each member and each practitioner, provider and employee who makes UM decisions at least once during the look-back period.

Look-back period

For Interim Surveys: Prior to the survey date.

For First Surveys and Renewal Surveys: 24 months.

Explanation

The organization's affirmative statement declares that the organization does not use incentives to encourage barriers to care and service. This element does not preclude the use of appropriate incentives for fostering efficient, appropriate care.

Distributing affirmative statement about incentives

The organization distributes its affirmative statement about incentives by mail, fax or email, or on its website, if it informs members, practitioners, providers and employees that the information is available on line. The organization mails its affirmative statement about incentives to recipients who do not have fax, email or internet access.

Factors 1–3

No additional explanation required.

Exceptions

None.

Examples

None.

MED 10: Grievances and Appeals

The organization has a process for handling grievances and appeals.

Intent

The organization establishes appropriate processes and time frames for grievances and appeals.

Element A: Policies for Grievances and Appeals

The organization's written policies and procedures specify that it:

1. Acknowledges receipt of grievances and appeals.
2. Oral inquiries made by members seeking to appeal a denial must be treated as appeals.
3. Considers information sent by members or their authorized representatives when making grievance and appeal decisions.
4. Provides grievance and appeal notifications in a format that is easy to understand, readily accessible and consistent with state and federal requirements.
5. Resolves a grievance and notifies the member of its decision within 90 calendar days of receipt of the grievance.
6. Provides oral notification to members of its decision on an expedited appeal.
7. Ensures that no punitive action is taken against a practitioner or provider that requests an expedited resolution or supports a member's appeal.
8. Handles denied requests for expedited appeals as a preservice or postservice request within the applicable time frames.
9. Allows members to initiate a State Fair Hearing when it fails to meet required decision and notification time frames for grievances and appeals.
10. Gives members reasonable assistance in completing forms and taking other procedural steps.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Documented process		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organization's policies and procedures.		
Look-back period	<i>For Interim Surveys and First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months for factors 1, 3–6, 8 and 9; 6 months for factors 2, 7 and 10.</i>		

Explanation	This element may not be delegated. This element applies to appeal decisions related to coverage or rescission of coverage. Members may appeal any denial decision. With member permission, the organization may refer an appeal directly to an IRO or State Fair Hearing without conducting an internal review. A grievance is defined as any expression of dissatisfaction or dispute (other than a denial) expressing dissatisfaction with any aspect of the organization's operations, activities, or behavior. Grievances include a member's right to dispute the organization's proposed extension of the time frame for making an authorization decision.
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Classification of appeal requests

Preservice appeal: An appeal of an adverse decision for coverage of care or services in advance of the member obtaining care or services.

Postservice appeal: A request to change an adverse decision for care or services that have been received by the member.

Expedited appeal: An appeal of an adverse decision for coverage of urgent care.

Factor 1: Acknowledging receipt of grievances and appeals

No additional explanation required.

Factor 2: Oral inquiries seeking to appeal

No additional explanation required.

Factor 3: Information sent by members for consideration in grievances and appeals

The organization considers all comments, documents, records and other information submitted by the member, even if the information was previously submitted and considered in the initial denial.

Factor 4: Notification of grievances and appeals

The organization uses language that is easy to understand and readily accessible to members in accordance with state and federal requirements, including, but not limited to, language services and auxiliary aids or assistance, upon member request.

Oral grievance. Grievances filed orally, may be responded to orally unless the member requests a written response or the grievance concerns quality of care. Grievances filed in writing are responded to in writing. The response should address all issues raised in the initial grievance and any resolution to those issues. For oral grievances that are resolved during the initial call, the organization should document the grievance and resolution.

Factor 5: Timely notification

A member may file a grievance with the organization at any time. The member may file a grievance either orally or in writing, in accordance with state law. Once a grievance has been filed, the organization resolves and notifies the member of its decision within 90 calendar days of receipt of the grievance.

A member may request an appeal either orally or in writing.

Extending the time frame to obtain additional information. Although there are allowable extensions for initial UM decisions, the organization may extend the time frame to obtain additional information by up to 14 calendar days after receipt of the appeal request, if the member agrees to extend the appeal time frame.

If the organization extends the decision time frames, it:

- Makes a reasonable effort to give the member prompt oral notification of the delay.
- Gives the member written notice of the reason for the decision to extend the time frame, within two calendar days.
- Informs the member of their right to file a grievance if they disagree with the decision.
- Resolves the appeal as expeditiously as the member's condition requires and no later than the date when the extension expires.

Factor 6: Oral notification of expedited appeals

The organization makes a reasonable effort to provide oral notification of expedited appeal decisions. The organization's policies and procedures specify what constitutes reasonable effort.

Factor 7: Punitive actions regarding expedited appeals

The organization's policies and procedures outline its process for ensuring that practitioners and providers are not penalized for requesting an expedited appeal or supporting a member's appeal.

Factor 8: Denials of expedited appeals

If the organization denies a request for an expedited appeal, it must reclassify the request as a preservice or postservice appeal request and meet the applicable time frames. Decisions regarding preservice and postservice appeals and notification to the member must occur within 30 calendar days of the request.

Factor 9: Initiating a State Fair Hearing

If the organization fails to meet the notice and timing requirements for grievances and appeals, the member is considered to have exhausted the organization's appeal process and may initiate a State Fair Hearing.

Factor 10: Providing assistance

The organization's policies and procedures:

- Specify how it helps members with the grievance and appeal process.
- Specify how it helps members complete necessary forms.
- Describe other assistance provided to members when requested, including access to auxiliary aids and services, interpreter services and toll-free numbers with TTY/TTD and interpreter capability.

Exceptions

None.

Examples

None.

Element B: Distribution of Information About the Grievance and Appeal Process

When it enters into a contract, the organization distributes a statement to all practitioners, providers and delegates that includes the following information:

1. The right to file grievances and appeals.
2. The requirements and time frames for filing a grievance or appeal.
3. The availability of assistance in the filing process.
4. The right to request a State Fair Hearing after a member has received an adverse appeal decision.
5. That members may be required to pay the cost of services provided while the appeal or State Fair Hearing is pending.

Summary of Changes

Clarifications

- Removed examples about practitioner contracts and delegation agreements from the scope of review.
- Revised the explanation to include a subhead and paragraph about methods of distribution.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Materials		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organization's materials for evidence that the requirements were met throughout the look-back period.		
Look-back period	<i>For Interim Surveys and First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	This element may not be delegated. Distributing grievance and appeal process information The organization distributes information about the grievance and appeal process to all new practitioners and organizations with which it contracts. The organization distributes its statement about grievances and appeals by mail, fax or email, or on its website, if it informs, practitioners, providers and delegates that the information is available on line. The organization mails its affirmative statement about incentives to recipients who do not have fax, email or internet access. Factors 1–5 No additional explanation required. Exceptions None.		
Examples	None.		

Element C: Grievance and Appeal Records

The organization's appeal policies and procedures specify that it maintains a written or electronic record of each grievance and appeal that includes:

1. The member's name.
2. A general description of the reason for the grievance or appeal.
3. The date each grievance or appeal was received.
4. The date of each review or, if applicable, the review meeting.
5. The decision or resolution of the grievance.
6. The decision or resolution of the appeal.
7. The date of the decision or resolution of the grievance.
8. The date of the decision or resolution of the appeal.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement

Data source Documented process

Scope of review *This element applies to Interim Surveys, First Surveys and Renewal Surveys.*
NCQA reviews the organization's policies and procedures for evidence that the requirements were met throughout the look-back period.

Look-back period *For Interim Surveys and First Surveys: 6 months.*
For Renewal Surveys: 24 months.

Explanation This element may not be delegated.

The manner in which the organization handles members' grievances demonstrates its commitment to ensuring quality care and service for its members. The organization maintains documentation of its handling and monitoring of grievances, in order to monitor actions taken.

Following standards for high-quality interaction with members means that any problems expressed by a member receive prompt and appropriate attention, regardless of the nature of those problems and whether they are oral or written, major or minor.

Factors 1–6: Documenting the reason and actions taken

The organization's policies and procedures specify that it maintains accurate records of all grievances and appeals that are received.

Factor 4: Date of each review

The organization's policies and procedures specify that the date of the review or the review meeting is documented each time the grievance or appeal is reviewed.

Exceptions

None.

- Examples** **Factor 4: The date of each review or, if applicable, the review meeting**
- The date when the request was reviewed to make the initial UM decision.
 - The date of the decision made by the State Fair Hearing.

Element D: Actions After an Appeal Is Overturned

The organization takes the following steps after an overturned internal or external appeal:

1. The organization provides or authorizes services within 72 hours of an overturned appeal, if services were not provided pending the outcome of the appeal.
2. The organization provides compensation for services provided while an appeal decision was pending.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement

Data source Documented process

Scope of review *This element applies to Interim Surveys, First Surveys and Renewal Surveys.*

NCQA reviews the organization's policies and procedures for evidence that the requirements were met throughout the look-back period.

Look-back period *For Interim Surveys and First Surveys: 6 months.*
For Renewal Surveys: 24 months.

Explanation This element may not be delegated.

The organization's policies and procedures describe its process for ensuring that services in dispute are provided or authorized if an appeal is overturned.

Factor 1: Services not provided pending the outcome of an appeal

The organization's policies and procedures specify that services are provided or authorized promptly and as expeditiously as the member's health condition requires, but no later than 72 hours from the date when the organization receives notification that the decision was overturned. Provider organizations deliver services within 72 hours; nonprovider organizations authorize services within 72 hours.

Factor 2: Compensation for services already provided

If the organization or a decision of the State Fair Hearing process overturns an appeal of services already received by the member while the appeal was pending, the organization's policies and procedures specify that the member is compensated in accordance with state policies and regulations.

Exceptions

None.

Examples

None.

MED 11: Continued Coverage

The organization provides continued coverage pending the outcome of an external appeal or State Fair Hearing.

Intent

The organization establishes a process for providing continued coverage of services for members while an external appeal or State Fair Hearing is pending.

Element A: Notification Regarding Continued Coverage

The organization provides the following information to members regarding continued coverage in the member handbook:

1. The member has continued coverage pending the outcome of an appeal or State Fair Hearing.
2. The member may be responsible for the costs associated with continued coverage if the appeal decision is adverse to the member.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement

Data source Materials

Scope of review *This element applies to Interim Surveys, First Surveys and Renewal Surveys.*

NCQA reviews the organization's member handbook.

Look-back period *For Interim Surveys and First Surveys: 6 months.*
For Renewal Surveys: 24 months.

Explanation This element may not be delegated.

Factor 1: Continued coverage pending the outcome

If a member requests continued coverage, the organization informs the member that benefits scheduled for reduction or termination will continue if the member files an appeal or requests a State Fair Hearing.

Factor 2: Member responsibility for costs associated with continued coverage

The organization informs members that if the result of an appeal is adverse to a member, they may be required to pay the cost of the services provided while the appeal or State Fair Hearing is pending.

If the outcome of the appeal is in the organization's favor, NCQA standards do not prohibit the organization from seeking reimbursement from the member for payments made.

Exceptions

None.

Examples

None.

Element B: Conditions of Continued Coverage Pending an Appeal Decision

The organization provides continued coverage pending the outcome of an external appeal or State Fair Hearing if all of the following occur:

1. The member appeals a utilization management denial within 60 calendar days of receiving notification of the denial.
2. The appeal involves the termination, suspension or reduction of previously authorized services.
3. The services were ordered by a practitioner or provider.
4. The period covered by the original decision has not expired.
5. The member files for continued coverage in a timely manner.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement

Data source Documented process

Scope of review *This element applies to Interim Surveys, First Surveys and Renewal Surveys.*
NCQA reviews the organization's policies and procedures for evidence that the requirements were met throughout the look-back period.

Look-back period *For Interim Surveys and First Surveys: 6 months.*
For Renewal Surveys: 24 months.

Explanation This element may not be delegated.

An external appeal is a request for an independent, external review of a final adverse determination made by the organization through its internal appeal process.

Continued coverage

This requirement applies to concurrent care decisions in which the organization approved an ongoing course of treatment and then denied, reduced or terminated the treatment before the end of the treatment period or number of treatments.

The organization continues to provide coverage and make payment for a currently approved ongoing course of treatment while the appeal is under review. The organization is obligated only to provide coverage to the end of the approved treatment or the final determination, whichever comes first, and subject to its regulatory and contractual obligations.

If the outcome of the appeal is in favor of the organization, NCQA standards do not prohibit the organization from seeking reimbursement from the member for payments for what was determined an uncovered service, subject to the organization's regulatory and contractual obligations.

Factors 1–4

No additional explanation required.

Factor 5: Timely manner

Members must request continued coverage within 10 calendar days of receiving a denial or within the intended effective date of the denial, whichever is later.

Exceptions

None.

Examples

A member requests 10 visits for physical therapy. The organization approves 6 physical therapy visits. Only a reduction/denial/termination of the 6 approved visits is subject to NCQA's requirements for continued coverage. Because these 4 visits are denied, continued coverage does not apply to the additional 4 visits and the organization is not required to pay for them, pending an appeal.

If the organization reduced the original 6 visits to 3 visits, it pays for the 6 visits pending an appeal. If the appeal outcome is in favor of the organization, the organization collects payment for the 3 denied visits, subject to the member's benefits plan and any state restrictions.

Element C: Duration of Continued Coverage

The organization's policies and procedures specify that it provides continued coverage until one of the following occurs:

1. The member withdraws the appeal or request for a State Fair Hearing.
2. The member fails to request a State Fair Hearing and continued coverage within 10 calendar days.
3. A State Fair Hearing issues a denial or upheld appeal.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement

Data source Documented process

Scope of review *This element applies to Interim Surveys, First Surveys and Renewal Surveys.*
NCQA reviews the organization's policies and procedures for evidence that the requirements were met throughout the look-back period.

Look-back period *For Interim Surveys and First Surveys: 6 months.*
For Renewal Surveys: 24 months.

Explanation This element may not be delegated.

Members must request both a State Fair Hearing and continued coverage within 10 calendar days of the organization's notification of the appeal decision. If the member requests a State Fair Hearing within 10 calendar days, the organization provides continued coverage until a decision has been made.

If the outcome of the appeal is in the organization's favor, NCQA standards do not prohibit the organization from seeking reimbursement from the member for payments made.

Factors 1–3: Duration of continued coverage

No additional explanation required.

Exceptions

None.

Examples None.

MED 12: Information Services for Members

The organization provides each member with the information necessary to understand their benefit coverage and obtain care.

Intent

The organization informs members about benefits and access to medical services.

Element A: Access and Cultural Considerations

The organization ensures that services are provided in a culturally competent manner to all members:

1. With limited English proficiency.
2. Of diverse cultural and ethnic backgrounds.
3. With a disability.
4. Regardless of sex.

Summary of Changes

Policy Changes

- Revised factor 4 to read “Regardless of sex.”

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement

Data source Documented process

Scope of review *This element applies to Interim Surveys, First Surveys and Renewal Surveys.*
NCQA reviews the organization’s policies and procedures.

Look-back period *For Interim Surveys and First Surveys: 6 months.*
For Renewal Surveys: 24 months.

Explanation This element may not be delegated.
The organization’s policies and procedures specify that it provides services to members in a culturally competent manner.

Factor 1: Limited English proficiency

The organization describes how it participates in state efforts to promote the delivery of services in a culturally competent manner to all members, including those with limited English proficiency.

Members with limited English proficiency do not speak English as their primary language, have a limited ability to read, write, speak or understand English and may be eligible to receive language assistance for a particular type of service, benefit or encounter.

Factors 2–4

No additional explanation required.

Exceptions

None.

Examples None.

Element B: Electronic Information

The organization's policies and procedures specify that it provides electronic information to members and potential members only if the information is:

1. Available upon request.
2. Readily accessible.
3. Also available in paper form, free of charge and within 5 business days.
4. In easy-to-understand language.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Documented process		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organization's policies and procedures.		
Look-back period	<i>For Interim Surveys and First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	This element may not be delegated. Factor 1: Available upon request The organization's policies and procedures specify how it informs members that electronic information is available upon request. Factor 2: Readily accessible information The organization's policies and procedures indicate that it provides information to members and potential members in a manner and format that is readily accessible. Readily accessible refers to electronic information and services that comply with modern accessibility standards.		

Factor 3: Availability of information

The organization's policies and procedures specify that it makes all electronic information available in paper format, upon request, free of charge and within five business days.

Factor 4: Easy-to-understand language

The organization's policies and procedures specify that the organization presents information in easy-to-understand language that is clear and uses words with common meaning, to the extent practical.

Exception

None.

Examples**Factor 2: Readily accessible**

- Section 508 guidelines.
- Section 504 of the Rehabilitation Act.
- W3C's Web Content Accessibility Guidelines 2.0.

Element C: Member Handbook

The organization's member handbook:

1. Informs existing and potential members how to request and access auxiliary aids and services.
2. Is available upon request.
3. Is available free of charge.
4. Includes a statement that the organization complies with all applicable federal and state laws.

Summary of Changes**Clarifications**

- Replaced the text under "Factor 1: Auxiliary aids and services" to clarify that the organization provides information about aids and services in addition to the information in the member handbook.

Scoring

Met	Partially Met	Not Met
The organization meets the requirement	No scoring option	The organization does not meet the requirement

Data source

Documented process, Reports, Materials

Scope of review

This element applies to Interim Surveys, First Surveys and Renewal Surveys.

NCQA reviews the organization's policies and procedures and reviews evidence that the organization's member handbook informs members about free access to auxiliary aids and services upon request and that the organization complies with federal and state laws.

Look-back period *For Interim Surveys and First Surveys:* 6 months.
For Renewal Surveys: 24 months for factors 1–3; 6 months for factor 4.

Explanation This element may not be delegated.

Factors 1–3: Auxiliary aids and services

The organization provides information about auxiliary aids and services (e.g., qualified interpreters, transcription services, assistive listening devices) for members and potential members with disabilities and how aids and services can be made available upon request and free of charge. The member handbook must inform members and potential members how to request and access auxiliary aids and services.

Factor 4: Compliance with other federal and state laws

The organization's member handbook states that the organization complies with all applicable federal and state laws, including:

- Title VI of the Civil Rights Act of 1964 as implemented by regulations at 45 CFR part 80.
- The Age Discrimination Act of 1975 as implemented by regulations at 45 CFR part 91.
- The Rehabilitation Act of 1973.
- Title IX of the Education Amendments of 1972 (regarding education programs and activities).
- Titles II and III of the Americans with Disabilities Act; and section 1557 of the Patient Protection and Affordable Care Act.

Exceptions

None.

Examples ***Factor 1: Auxiliary aids and services***

- Magnification software.
- Braille materials and displays.

Element D: Providing Information to Medicaid Members in the Practitioner Directory

The organization's written practitioner directory is available to existing and potential members:

1. With instructions for requesting and accessing auxiliary aids and services.
2. In alternative formats, upon request, free of charge.
3. In the prevalent non-English languages in its service area.
4. With taglines in the prevalent non-English languages in the state.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement

Data source Reports, Materials

Scope of review *This element applies to Interim Surveys, First Surveys and Renewal Surveys.*
NCQA reviews the organization's written practitioner directory for evidence that it meets the requirements throughout the look-back period and an assessment of its members' linguistic needs.

Look-back period *For Interim Surveys and First Surveys: 6 months.*
For Renewal Surveys: 24 months for factors 2–4; 6 months for factor 1.

Explanation Use of contracted translation services is not considered delegation.

Factor 1: Auxiliary aids and services

The practitioner directory contains information about auxiliary aids and services (e.g., qualified interpreters, transcription services, assistive listening devices) that are available upon request and free of charge for existing and potential members with disabilities, and instructions for requesting and accessing aids and services.

Factor 2: Availability of the directory in alternative formats

Alternative formats, including auxiliary aids and services, must also be made available upon request of the potential member or member, free of charge.

Factors 3: Availability of the directory in other languages

The organization may use the linguistic data used to satisfy NET 1, Element A to determine the non-English languages applicable to its membership.

Factor 4: Availability of the directory with taglines in other languages

The organization provides practitioner directories that include taglines in prevalent non-English languages in the state.

Taglines are short statements written in languages other than English that indicate the availability of language assistance services.

Taglines must be printed in a conspicuously visible font size.

The organization's practitioner directory informs members of the availability of written translation or oral interpretation, so they can understand the information provided, and the toll-free and TTY/TDY telephone number of the organization's Member Services department.

Exceptions

Factors 3 and 4 are NA if the organization can show that English is the principal spoken and written language of all members and prospective members.

Examples

Factor 2: Auxiliary aids and services

- Magnification software.
- Braille materials and displays.

Factor 3: Sources of data to determine linguistic needs

- Census Bureau data.
- Member surveys.
- Information from employer groups.
- Analysis of member complaints.
- Results of the Race/Ethnicity of Membership (RDM) and Language Diversity of Membership (LDM) HEDIS measures.

Element E: Providing Information to Medicaid Members in the Member Handbook

The organization's member handbook is available to existing and potential members:

1. In alternative formats, upon request, free of charge.
2. In the prevalent non-English languages in its service area.
3. With taglines in the prevalent non-English languages in the state.

Summary of Changes

Deletions

- Deleted factor 1 ("In regular and large print") and renumbered the remaining factors.
- Deleted "Factor 1: Availability of the member handbook in regular and large print" subhead and text in the Explanation and renumbered the remaining subheads.
- Deleted the Exception for factor 1.

Scoring

Met	Partially Met	Not Met
The organization meets the requirement	No scoring option	The organization does not meet the requirement

Data source

Reports, Materials

Scope of review

This element applies to Interim Surveys, First Surveys and Renewal Surveys.

NCQA reviews the organization's member handbook, an assessment of its members' linguistic needs and documentation that it meets the requirements throughout the look-back period.

Look-back period *For Interim Surveys and First Surveys:* 6 months.
 For Renewal Surveys: 24 months.

Explanation This element may not be delegated.
 Use of contracted translation services is not considered delegation.

Factor 1: Availability of the member handbook in alternative formats

Alternative formats, including auxiliary aids and services, must also be made available upon request of the potential member or member, free of charge.

Factor 2: Availability of the member handbook in other languages

The organization may use the linguistic data used to satisfy NET 1, Element A to determine the non-English languages applicable to its membership.

Factor 3: Availability of the member handbook with taglines in other languages

The organization provides member handbooks that include taglines in prevalent non-English languages in the state.

Taglines must be printed in a conspicuously visible font size.

The organization's member handbook informs members of the availability of written translation or oral interpretation, so they can understand the information provided, and the toll-free and TTY/TDY telephone number of the organization's Member Services department.

Exceptions

Factors 2 and 3 are NA if the organization can show that English is the principal spoken and written language of all members and prospective members.

Examples **Factor 1: Auxiliary aids and services**

- Magnification software.
- Braille materials and displays.

Factor 2: Sources of data to determine linguistic needs

- Census Bureau data.
- Member surveys.
- Information from employer groups.
- Analysis of member complaints.
- Results of the Race/Ethnicity of Membership (RDM) and *Language Diversity of Membership (LDM)* HEDIS measures.

Element F: Providing Information to Medicaid Members in Denial Notifications

Denial notifications sent by the organization to existing members:

1. Contain instructions for requesting and accessing auxiliary aids and services.
2. Are available in alternative formats, upon request, free of charge.
3. Are available in the prevalent non-English languages in its service area.
4. Contain taglines in the prevalent non-English languages in the state.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement

Data source Documented process, Reports, Materials

Scope of review *This element applies to Interim Surveys, First Surveys and Renewal Surveys.*
NCQA reviews the organization's policies and procedures, an assessment of its members' linguistic needs and other evidence that it meets the requirements throughout the look-back period.

Look-back period *For Interim Surveys and First Surveys: 6 months.*
For Renewal Surveys: 24 months for factors 2–4; 6 months for factor 1.

Explanation Use of contracted translation services is not considered delegation.

Factor 1: Auxiliary aids and services

Denial notifications contain information about auxiliary aids and services (e.g., qualified interpreters, transcription services, assistive listening devices) that are available upon request and free of charge for existing and potential members with disabilities, and instructions for requesting and accessing aids and services.

Factor 2: Availability of denial notifications in alternative formats

Alternative formats, including auxiliary aids and services, must also be made available upon request of the member, free of charge.

Factors 3: Availability of denial notifications in other languages

The organization may use the linguistic data used to satisfy NET 1, Element A to determine the non-English languages applicable to its membership.

Factor 4: Availability of denial notifications with taglines in other languages

The organization provides written denial notifications that include taglines in prevalent non-English languages in the state.

Taglines must be printed in a conspicuously visible font size.

The organization's denial notifications inform members of the availability of written translation or oral interpretation, so they can understand the information provided and the toll-free and TTY/TDY telephone number of the organization's Member Services department.

Exceptions

Factors 3 and 4 are NA if the organization can show that English is the principal spoken and written language of all members.

Examples

Factor 2: Auxiliary aids and services

- Magnification software.
- Braille materials and displays.

Factor 3: Sources of data to determine linguistic needs

- Census Bureau data.
- Member surveys.
- Information from employer groups.
- Analysis of member complaints.
- Results of the Race/Ethnicity of Membership (RDM) and Language Diversity of Membership (LDM) HEDIS measures.

Element G: Providing Information to Members in Appeal and Grievance Notifications

Appeal and grievance notifications sent to existing and potential members by the organization:

1. Contain information instructions for requesting and accessing auxiliary aids and services.
2. Are available in alternative formats, upon request, free of charge.
3. Are available in the prevalent non-English languages in its service area.
4. Contain taglines in the prevalent non-English languages in the state.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement

Data source Documented process, Reports, Materials

Scope of review *This element applies to Interim Surveys, First Surveys and Renewal Surveys.*
NCQA reviews the organization's policies and procedures, an assessment of its members' linguistic needs and other evidence that it meets the requirements throughout the look-back period.

Look-back period *For Interim Surveys and First Surveys: 6 months.*
For Renewal Surveys: 24 months for factors 2–4; 6 months for factor 1.

Explanation Use of contracted translation services is not considered delegation.

Factor 1: Auxiliary aids and services

Appeal and grievance notifications contain information about auxiliary aids and services (e.g., qualified interpreters, transcription services, assistive listening devices) that are available upon request and free of charge for existing and potential members with disabilities, and instructions for requesting and accessing aids and services.

Factor 2: Availability of the appeal and grievance notifications in alternative formats

Alternative formats, including auxiliary aids and services, must also be made available upon request of the potential member or member, free of charge.

Factors 3: Availability of the appeal and grievance notifications in other languages

The organization may use the linguistic data used to satisfy NET 1, Element A to determine the non-English languages applicable to its membership.

Factor 4: Availability of the appeal and grievance notifications with taglines in other languages

The organization provides notifications that include taglines in prevalent non-English languages in the state.

Taglines must be printed in a conspicuously visible font size.

The organization's appeal and grievance notifications inform members of the availability of written translation or oral interpretation, so they can understand the information provided, and the toll-free and TTY/TDY telephone number of the organization's Member Services department.

Exceptions

Factors 3 and 4 are NA if the organization can show that English is the principal spoken and written language of all members and prospective members.

Examples

Factor 2: Auxiliary aids and services

- Magnification software.
- Braille materials and displays.

Factor 3: Sources of data to determine linguistic needs

- Census Bureau data.
- Member surveys.
- Information from employer groups.
- Analysis of member complaints.
- Results of the Race/Ethnicity of Membership (RDM) and Language Diversity of Membership (LDM) HEDIS measures.

Element H: Interpreter Services for Medicaid Members

The organization provides oral interpreter or bilingual services in all languages to:

1. Existing members.
2. Prospective members.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Reports, Materials		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews evidence that interpreter or bilingual services were made available to existing and prospective members throughout the look-back period.		
Look-back period	<i>For Interim Surveys and First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	Use of contracted translation services is not considered delegation. Any materials sent to members must include information about the availability of oral interpretation or bilingual services. Factors 1–2 No additional explanation required. Exception This element is NA if the organization can show that all members' principal spoken and written language is English.		
Examples	None.		

Element I: Usability Testing of Member Materials

The organization evaluates its distributed materials for understandability and usefulness to members and prospective members at least every three years, and considers the following:

1. Reading level.
2. Intuitive content organization.

Summary of Changes**Deletions**

- Deleted factor 1 ("Font size") and renumbered the remaining factors.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Documented process, Reports		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organization's policies and procedures and evidence that it conducted usability testing.		
Look-back period	<i>For All Surveys:</i> At least once in the prior 36 months.		
Explanation	Usability testing is required for all materials that are distributed to members (e.g., directories, handbooks, denial notices). The organization is not required to conduct usability testing with an external audience. Testing with internal staff who were not involved in developing member materials meets the intent. Factors 1–2 No additional explanation required. Exceptions None. Related information <i>Information on usability testing.</i> For additional information on usability testing, refer to www.usability.gov. <i>Use of vendors for usability testing services.</i> If the organization contracts with a vendor to provide usability testing services, it provides access to the vendor's documentation. NCQA does not consider the relationship to be delegation and evaluates the vendor's documentation against the requirements.		
Examples	None.		

MED 13: Member Communications

The organization provides access to staff for members seeking information and assistance.

Intent

Members have access to the organization's telecommunication and special communication services.

Element A: Communication Services

The organization provides information to members in the member handbook about the availability of a toll-free number for:

1. Member Services.
2. Medical management.
3. Other departments that provide services directly to members.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Materials		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organization's member handbook for evidence that it provides information about the availability of its telecommunication services to its members.		
Look-back period	<i>For Interim Surveys and First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	This element may not be delegated. The organization provides members with information regarding the operating hours for the toll-free number for member services, medical management and any other department that provides services directly to members. Factors 1–3 No additional explanation required. Exception Factor 3 is NA if the organization does not have any additional departments that provide services directly to members.		
Examples	None.		

Element B: Offering Special Communication Assistance

The organization provides communication assistance, free of charge, to meet the special needs of members.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Documented process, Reports, Materials		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organization's policies and procedures that describe its process for providing communication assistance based on the needs of its population and evidence that the organization made communication assistance available to members when needed.		
Look-back period	<i>For Interim Surveys and First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	Based on the needs of its population, the organization has alternative communication formats for: • Vision impairment. • Hearing impairment. • Reading proficiency. The organization has a process for making member materials available in alternative formats, if applicable. Exceptions None.		
Examples	None.		

Element C: Notifying Members of Communication and Language Services

The organization informs members that communication and language services are available free of charge.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Reports, Materials		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organization's notification informing members of its communication services and other evidence that the requirements were met throughout the look-back period.		
Look-back period	<i>For Interim Surveys and First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	This element may not be delegated. The organization notifies members how to access language services and communication services, free of charge. Distribution of information The organization distributes information by regular mail, email or fax, through messages to members' mobile devices or on its website. The organization mails the information to members who do not have internet access. Exceptions None.		
Examples	None.		

MED 14: Practitioner and Provider Directories

The organization provides information to help members and prospective members choose practitioners and providers.

Intent

The organization's directories contain information that members and prospective members may use to select a physician and hospital.

Element A: Directory Data

The organization's directory includes the following information:

1. The website address.
2. Office/facility accommodations for members with disabilities or special needs.
3. Cultural and linguistic capabilities of the physician or clinical staff.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Materials		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organization's directory that is in place throughout the look-back period.		
Look-back period	<i>For Interim Surveys and First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	This requirement applies to all network providers, individual practitioners, practitioner groups and facilities that have a network agreement. Each field in the directory must be populated. If the information in a factor does not apply, the data field indicates "None" or "NA." Factor 1: Website address No additional explanation required. Factor 2: Accommodations for people with disabilities or special needs The directory specifies whether the practitioner office or facility has accommodations for members with physical disabilities; for example: • Adjustable-height exam tables. • Exam rooms that can accommodate wheelchairs. • Handrails in bathrooms.		

Factor 3: Cultural and linguistic capabilities of the physician or clinical staff

The directory includes information on the languages spoken, including American Sign Language, and whether the physician and staff have taken any cultural competence training.

The directory may have a separate field that lists languages spoken by nonclinical staff. The organization is not required to include English in the list of spoken languages.

Exceptions

None.

Related information

Effective July 1, 2021, CMS amended CFR 438.10(h)(1)(vii), which eliminated a phrase that required organizations to provide information on whether a practitioner completed cultural competency training. Unless this requirement is reinstated, NCQA will not review against this portion of the requirement in factor 3.

Examples None.

Element B: Pharmacy Directory Data

The organization's directory includes the following information about available pharmacies:

1. Name.
2. Pharmacy locations and phone numbers.
3. Pharmacy website address.
4. Pharmacy or facility accommodations for people with disabilities or special needs.
5. Cultural and linguistic capabilities of the pharmacist and pharmacy staff.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Materials		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organization's directory that is in place throughout the look-back period.		
Look-back period	<i>For Interim Surveys and First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		

Explanation Each field in the directory must be populated. If the information in a factor does not apply to a pharmacy, the data field indicates “None” or “NA.”

Factor 1: Name

The directory includes the name of the pharmacy.

Factor 2: Office location and phone numbers

The directory lists the physical addresses and phone numbers of pharmacy locations.

Factor 3: Website address

No additional explanation required.

Factor 4: Accommodations for people with disabilities or special needs

The directory specifies if the pharmacy has accommodations for members with physical disabilities or other special needs.

Factor 5: Cultural and linguistic capabilities of the pharmacist and pharmacy staff

The directory includes information on the languages spoken by the pharmacist and staff, including American Sign Language, and whether pharmacy staff have taken cultural competence training.

The directory may list languages spoken by the pharmacy staff in a separate field. The organization is not required to include English in the list of spoken languages.

Exceptions

None.

Related information

Effective July 1, 2021, CMS amended CFR 438.10(h)(1)(vii), which eliminated a phrase that required organizations to provide information on whether a practitioner completed cultural competency training. Unless this requirement is reinstated, NCQA will not review against this portion of the requirement in factor 5.

Examples None.

Element C: Behavioral Healthcare Directory Data

The organization's behavioral healthcare directory includes the following information for behavioral healthcare practitioners and providers:

1. Name.
2. Specialty.
3. Hospital affiliations.
4. Medical group affiliations.
5. Accepting new patients.
6. Cultural and linguistic capabilities of the practitioner or clinical staff.
7. Office locations and phone numbers.
8. Website address, if applicable.
9. Office or facility accommodations for members with disabilities or special needs.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Materials		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organization's behavioral healthcare directory that is in place throughout the look-back period.		
Look-back period	<i>For Interim Surveys and First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	This requirement applies to all behavioral healthcare network providers, individual practitioners, practitioner groups and facilities that have a network agreement. Each field in the directory must be populated. If the information in a factor does not apply, the data field indicates "None" or "NA." Factor 1: Name No additional explanation required. Factor 2: Specialty The directory lists all applicable specialties and lists applicable disciplines, for contracted behavioral healthcare practitioners and providers. The directory is not required to list subspecialty. Factor 3: Hospital affiliations The directory lists network hospitals where practitioners have admitting or attending privileges.		

Factor 4: Medical group affiliations

The directory lists network medical groups with which practitioners are affiliated.

Factor 5: Accepting new patients

The directory indicates whether practitioners or providers are accepting new patients.

Factor 6: Cultural and linguistic capabilities of the behavioral healthcare practitioner or clinical staff

The directory includes information on the languages spoken, including American Sign Language, and whether practitioners and other staff have taken cultural competence training.

The directory may have a separate field that lists languages spoken by nonclinical staff. The organization is not required to include English in the list of spoken languages.

Factor 7: Office location and phone numbers

The directory lists the physical addresses and phone numbers of office locations where practitioners practice.

Factor 8: Website address

No additional explanation required.

Factor 9: Accommodations for members with disabilities or special needs

The directory specifies if the practitioner's office or facility has accommodations for members with physical disabilities or other special needs.

Exceptions

None.

Related information

Effective July 1, 2021, CMS amended CFR 438.10(h)(1)(vii), which eliminated a phrase that required organizations to provide information on whether a practitioner completed cultural competency training. Unless this requirement is reinstated, NCQA will not review against this portion of the requirement in factor 6.

Examples

None.

Element D: Long-Term Services and Supports Provider Directory Data

The organization has a directory that includes the following information for LTSS providers:

1. Name.
2. Specialty.
3. Hospital affiliations.
4. Medical group affiliations.
5. Accepting new patients.
6. Cultural and linguistic capabilities of the LTSS provider or staff.
7. Office locations and phone numbers.
8. Website address, if applicable.
9. Office or facility accommodations for members with disabilities or special needs.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Materials		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organization's LTSS directory that is in place throughout the look-back period.		
Look-back period	<i>For Interim Surveys and First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	This requirement applies only to network providers. The directory includes all LTSS providers who provide services to Medicaid members, regardless of whether it is provided in the member's home, a worksite, a provider-owned or controlled residential setting, a nursing facility or other institutional setting. Every field must be populated. If a factor does not apply to a provider, the data field indicates "None" or "NA." Factor 1: Name No additional explanation required. Factor 2: Specialty The directory lists all applicable specialties, and is not required to list subspecialty.		

Factor 3: Hospital affiliations

The directory lists hospitals in the organization's network where providers have admitting or attending privileges.

Factor 4: Medical group affiliations

The directory lists medical groups in the organization's network with which the providers are affiliated.

Factor 5: Accepting new patients

The directory indicates whether providers are accepting new patients.

Factor 6: Cultural and linguistic capabilities of the LTSS provider or staff

The directory includes information on the languages spoken by the provider and staff, including American Sign Language, and whether the provider and staff have taken cultural competence training.

The directory may have a separate field that lists languages spoken by the practice's staff. The organization is not required to include English in the list of spoken languages.

Factor 7: Office location and phone numbers

The directory lists the physical addresses and phone numbers of practice office locations.

Factor 8: Website address

No additional explanation required.

Factor 9: Accommodations for members with disabilities or special needs

The directory specifies whether the provider's office or facility has accommodations for members with physical disabilities; for example:

- Exam rooms that can accommodate wheelchairs.
- Handrails in bathrooms.

Exception

This element is NA if the organization does not provide LTSS services.

Related information

Effective July 1, 2021, CMS amended CFR 438.10(h)(1)(vii), which eliminated a phrase that required organizations to provide information on whether a practitioner completed cultural competency training. Unless this requirement is reinstated, NCQA will not review against this portion of the requirement in factor 6.

Examples

None.

MED 15: Delegation of MED

If the organization delegates any NCQA-required MED activities, there is evidence of oversight of the delegated activities.

Intent

The organization remains responsible for and has appropriate structures and mechanisms to oversee delegated MED activities.

Element A: Delegation Agreement

The written delegation agreement:

1. Is mutually agreed upon.
2. Describes the delegated activities and the responsibilities of the organization and the delegated entity.
3. Requires at least semiannual reporting of the delegated entity to the organization.
4. Describes the process by which the organization evaluates the delegated entity's performance.
5. Describes the remedies available to the organization if the delegated entity does not fulfill its obligations, including revocation of the delegation agreement.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Materials		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews delegation agreements in effect during the look-back period from up to four randomly selected delegates, or reviews all delegates if the organization has fewer than four. The score for the element is the average of the scores for all delegates.		
Look-back period	<i>For Interim Surveys and First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	This element may not be delegated. This element applies to agreements that are in effect within the look-back period. The delegation agreement describes all delegated MED activities. A generic policy statement about the content of delegated arrangements does not meet this element.		

Factor 1: Mutual agreement

Delegation activities are mutually agreed on before delegation begins, in a dated, binding document or communication between the organization and the delegated entity.

NCQA considers the effective date specified in the delegation agreement as the mutually agreed-upon effective date. The effective date may be before or after the signature date on the agreement. If the agreement has no effective date, NCQA considers the signature date (meaning the date of last signature) as the mutually agreed-upon effective date.

NCQA may accept other evidence of the mutually agreed-upon effective date: a letter, meeting minutes or other form of communication between the organization and the delegate that references the parties' agreement on the effective date of delegated activities.

NCQA requires submitted evidence for all other delegation factors to consider the same mutually agreed-upon date as the effective date for the delegate's performance of delegated activities.

Factor 2: Assigning responsibilities

The delegation agreement or an addendum thereto or other binding communication between the organization and the delegate specifies the MED activities:

- Performed by the delegate, in detailed language.
- Not delegated, but retained by the organization.
 - The organization may include a general statement in the agreement addressing retained functions (e.g., the organization retains all other MED functions not specified in this agreement as the delegate's responsibility).

If the delegate subdelegates an activity, the delegation agreement must specify that the delegate or the organization is responsible for subdelegate oversight.

Factor 3: Reporting

The organization determines the method of reporting and the content of the reports, but the agreement must specify:

- That reporting is at least semiannual.
- What information is reported by the delegate about delegated activities.
- How, and to whom, information is reported (i.e., joint meetings or to appropriate committees or individuals in the organization).

The organization must receive regular reports from all delegates, even NCQA-Accredited or NCQA-Certified delegates.

Factor 4: Performance monitoring

The delegation agreement specifies how the organization evaluates the delegate's performance.

Factor 5: Consequences for failure to perform

The delegation agreement specifies consequences if a delegate fails to meet the terms of the agreement and, at a minimum, circumstances that result in revocation of the agreement.

Exception

This element is NA if the organization does not delegate MED activities.

Examples

None.

Element B: Provisions for PHI

If the delegation arrangement includes the use of protected health information (PHI) by the delegate, the delegation document also includes the following provisions.

1. The allowed uses of PHI.
2. A description of delegate safeguards to protect the information from inappropriate use or further disclosure.
3. A stipulation that the delegate ensures that subdelegates have similar safeguards.
4. A stipulation that the delegate provides individuals with access to their PHI.
5. A stipulation that the delegate informs the organization if inappropriate uses of the information occurs.
6. A stipulation that the delegate ensures that PHI is returned, destroyed or protected if the delegation agreement ends.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Materials		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews delegation agreements in effect during the look-back period from up to four randomly selected delegates, or reviews all delegates if the organization has fewer than four. The score for the element is the average of the scores for all delegates.		
Look-back period	<i>For Interim Surveys and First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	This element may not be delegated. Factor 1: Allowed uses of PHI The delegation agreement specifies PHI the delegate may use and disclose, and to whom PHI may be disclosed. Factors 2, 3: Delegate and subdelegate safeguards The organization provides reasonable administrative, technical and physical safeguards to ensure PHI confidentiality, integrity and availability and to prevent unauthorized or inappropriate access, use or disclosure of PHI.		

Factor 4: Access to PHI

The delegation agreement includes procedures to receive, analyze and resolve members' requests for access to their PHI.

Factor 5: Inappropriate use of PHI

The agreement specifies procedures for delegates to identify and report unauthorized access, use, disclosure, modification or destruction of PHI and the systems used to access or store PHI.

Factor 6: Disposal of PHI if delegation ends

No additional explanation required.

Exceptions

This element is NA if:

- The organization does not delegate MED activities.
- Delegation arrangements do not involve the use, creation or disclosure of PHI in any form.
 - The agreement states that the delegation arrangement does not involve PHI.
- Delegation arrangements are with covered entities.

Examples None.

Element C: Predelegation Evaluation

For new delegation agreements initiated in the look-back period, the organization evaluated delegate capacity to meet NCQA requirements before delegation began.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Reports		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> <i>This element applies if delegation was implemented in the look-back period.</i> NCQA reviews the organization's predelegation evaluation from up to four randomly selected delegates, or reviews all delegates if the organization has fewer than four. The score for the element is the average of the scores for all delegates.		
Look-back period	<i>For Interim Surveys and First Surveys: 6 months.</i> <i>For Renewal Surveys: 12 months.</i>		

Explanation

This element may not be delegated.

NCQA-Accredited/Certified delegates

Automatic credit is available for this element if all delegates are NCQA Accredited or NCQA Certified, unless the element is NA.

Predelegation evaluation

The organization evaluated the delegate's capacity to meet NCQA requirements within 12 months prior to implementing delegation.

NCQA considers the date of the agreement to be the implementation date if the delegation agreement does not include an implementation date.

If the time between the predelegation evaluation and implementation of delegation exceeds 12 months, the organization conducts another predelegation evaluation.

If the organization amends the delegation agreement to include additional activities within the look-back period, it performs a predelegation evaluation for the additional activities.

Exceptions

This element is NA if:

- The organization does not delegate MED activities.
- Delegation arrangements have been in effect for longer than the look-back period.

Examples**Predelegation evaluation**

- Site visit.
- Telephone consultation.
- Documentation review.
- Committee meetings.
- Virtual review.

Element D: Review of Delegate's MED Activities

For delegation arrangements in effect for 12 months or longer, the organization:

1. Annually reviews its delegate's policies and procedures.
2. Annually evaluates delegate performance against NCQA standards for delegated activities.
3. Semiannually evaluates regular reports, as specified in Element A.

Summary of Changes

- No changes to this element.

Scoring

Met	Partially Met	Not Met
The organization meets the requirement	No scoring option	The organization does not meet the requirement

Data source

Reports

Scope of review	<i>Factor 1 applies to Interim Surveys.</i>
	<i>All factors in this element apply to First Surveys and Renewal Surveys.</i>
	NCQA reviews reports from a sample of up to four randomly selected delegates, or reviews all delegates if the organization has fewer than four.
	<i>For All Surveys:</i> NCQA reviews the organization's evaluation of the delegate's policies and procedures (factor 1).
	<i>For First Surveys:</i> NCQA also reviews the organization's most recent annual audit, performance evaluation and semiannual report evaluation.
Look-back period	<i>For Renewal Surveys:</i> NCQA also reviews the most recent and previous year's annual audits, performance evaluations and four semiannual report evaluations.
	The score for the element is the average of the scores for all delegates.
Explanation	<i>For Interim Surveys and First Surveys:</i> At least once during the prior year.
	<i>For Renewal Surveys:</i> 24 months.
Examples	Explanation
	This element may not be delegated.
	NCQA-Accredited/Certified delegates
	Automatic credit is available for factor 2 if all delegates are NCQA Accredited or NCQA Certified, unless the element is NA.
	Factor 1: Review of the delegate's policies and procedures
	The appropriate organization staff or committee reviews the delegate's policies and procedures. At a minimum, the organization reviews sections of the policies and procedures that apply to the delegated functions.
	Factor 2: Annual evaluation
	No additional explanation required.
	Factor 3: Evaluation of reports
	No additional explanation required.
	Exceptions
	The element is NA if:
	• The organization does not delegate MED activities. • Delegation arrangements have been in effect for less than 12 months.
	Factor 2: Annual evaluation
	• Site visit. • Telephone consultation. • Documentation review. • Committee meetings. • Virtual review.

Element E: Opportunities for Improvement

For delegation arrangements that have been in effect for more than 12 months, at least once in each of the past 2 years, the organization identified and followed up on opportunities for improvement, if applicable.

Summary of Changes

- No changes to this element.

Scoring	<table border="1"> <thead> <tr> <th data-bbox="548 527 602 554">Met</th><th data-bbox="829 527 997 554">Partially Met</th><th data-bbox="1203 527 1305 554">Not Met</th></tr> </thead> <tbody> <tr> <td data-bbox="428 569 732 630">The organization meets the requirement</td><td data-bbox="805 569 1021 596">No scoring option</td><td data-bbox="1101 569 1414 630">The organization does not meet the requirement</td></tr> </tbody> </table>	Met	Partially Met	Not Met	The organization meets the requirement	No scoring option	The organization does not meet the requirement	
Met	Partially Met	Not Met						
The organization meets the requirement	No scoring option	The organization does not meet the requirement						
Data source	Documented process, Reports, Materials							
Scope of review	<i>This element applies to First Surveys and Renewal Surveys.</i> NCQA reviews reports of opportunities for improvement, from up to four randomly selected delegates, or from all delegates if the organization has fewer than four, and for evidence that the organization took appropriate action to resolve issues. <i>For First Surveys:</i> NCQA reviews the organization's most recent annual review and follow-up on improvement opportunities. <i>For Renewal Surveys:</i> NCQA reviews the organization's most recent and previous year's annual reviews and follow-up on improvement opportunities. The score for the element is the average of the scores for all delegates.							
Look-back period	<i>For First Surveys:</i> At least once during the prior year. <i>For Renewal Surveys:</i> 24 months.							
Explanation	This element may not be delegated. NCQA-Accredited/Certified delegates Automatic credit is available for this element if all delegates are NCQA Accredited or NCQA Certified, unless the element is NA. Identify and follow up on opportunities The organization uses information from its predelegation evaluation, ongoing reports or annual evaluation to identify areas of improvement. Exceptions This element is NA: • If the organization does not delegate MED activities. • If delegation arrangements have been in effect for less than 12 months. • If the organization has no opportunities to improve performance. – NCQA evaluates whether this conclusion is reasonable, given assessment results.							
Examples	None.							

Medicare Advantage

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MA 1: Disenrollment Practices

Refer to Appendix 1 for points

The MA organization may not, on the basis of any factor related to health status, deny, limit or condition the coverage or furnishing of benefits to individuals eligible to enroll in an MA plan offered by the organization.

Intent

The organization may not discriminate against prospective and enrolled members based on their health status.

Element A: Monitoring Disenrollment Practices

The MA organization has mechanisms in place to monitor:

1. Complaints from prospective and existing members related to discrimination based on their health status.
2. Disenrollment practices against members based on their health status.

Summary of Changes

- This is a new element.

Scoring	Met	Partially Met	Not Met
	The organization meets 2 factors	No scoring option	The organization meets 0-1 factors
Data source	Reports		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organization's reports that document enrollment and disenrollment and the reasons for disenrollment.		
Look-back period	<i>For Interim Surveys and First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	THIS IS A MUST-PASS ELEMENT. Medicare law requires that members of an MA organization receive covered services when medically necessary, without regard to their health status. The MA organization is prohibited by law from disenrolling an otherwise eligible member from the organization based on the member's health status. Exceptions None.		
Examples	None.		

Element B: Access to Antidiscrimination Laws

Not Applicable for MA Deeming

MA 2: ESRD Enrollment/Disenrollment

The MA organization may not enroll an individual who has been medically determined to have end-stage renal disease (ESRD), but a member who develops ESRD while enrolled in an MA organization may not be disenrolled for that reason.

Intent

The organization monitors ESRD enrollment/disenrollment.

Element A: Monitoring ESRD Enrollment/Disenrollment

The MA organization has a process in place for monitoring ESRD enrollment/disenrollment requirements.

Summary of Changes

- This is a new element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Documented process, Materials		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA assesses the organization's policies, procedures and process for complying with MA enrollment/disenrollment requirements concerning Medicare beneficiaries with ESRD. The organization's process may involve the Monthly Membership Report and Transaction Reply Monthly Activity Report issued by CMS.		
Look-back period	<i>For Interim Surveys and First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	THIS IS A MUST-PASS ELEMENT. Under Medicare law, an MA organization may not enroll an individual for coverage prior to January 1, 2021, if the individual has been medically determined to have ESRD. If a member develops ESRD while enrolled, the organization may not disenroll the member for that reason. The organization should describe this process in its policies and procedures. The organization's policies and procedures must also allow enrollment of members in the following cases: • Members currently enrolled in non-MA plans within the same organization that offers the MA plan. • Members who have elected a Medicare plan and who are medically determined to have ESRD before the effective date of coverage. • Members whose enrollment was terminated because of contract renewal, non-renewal or service area reduction who may make one election into a new Medicare plan.		

An MA organization may enroll individuals who have been medically determined to have ESRD on or after January 1, 2021.

Exceptions

None.

Examples

None.

MA 3: Initial Assessment “Best Effort”*Not Applicable for MA Deeming***MA 4: Advance Directives**

The MA organization adopts and implements policies and procedures on advance directives for its members.

Intent

The organization has a process for members’ advance directives.

Element A: Policies and Procedures

The MA organization’s written policies and procedures on advance directives:

1. Allow a member’s representative to manage care or treatment decisions when the member is incapacitated and unable to do so.
2. Comply with state law.
3. State that the MA organization is not required to provide care that conflicts with an advance directive.
4. Allow a member or the member’s representative to be involved in decisions about withholding resuscitative services or declining/withdrawing life-sustaining treatment.
5. State that the MA organization may not condition the provision of care or otherwise discriminate against a member based on whether the member has executed an advance directive.
6. Specify the time frames for completion of the advance directive.

Summary of Changes

- This is a new element.

Scoring	Met	Partially Met	Not Met
	The organization meets 6 factors	No scoring option	The organization meets 0-5 factors
Data source	Documented process		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i>		
	NCQA scores this element once for the organization.		
	NCQA reviews the MA organization’s advance directives policies and procedures to ensure that they contain the six factors.		
Look-back period	<i>For Interim Surveys and First Surveys: 6 months.</i>		
	<i>For Renewal Surveys: 24 months.</i>		

Explanation THIS IS A MUST-PASS ELEMENT.

To score Met, the organization's policies and procedures on advanced directives must satisfy all six factors listed in Element A.

An **advance directive** is a document in which members explain the type and extent of health care services they prefer if they become unable to make medical decisions. Medicare law requires an MA organization to follow a member's request made in an advance directive.

The organization's advance directives policies and procedures must allow the member's representative to manage care or treatment decisions when the member is unable to do so because of physical or mental limitations.

Factor 1: Member's representative

In addition to allowing a member's representative to manage care or treatment decisions when the member is incapacitated and unable to do so, the MA organization may give advance directive information to the member's family or surrogate in the same manner that it issues other materials about policies and procedures to the family of the incapacitated member or to a surrogate or other concerned persons, in accordance with state law. The MA organization is not relieved of its obligation to provide this information to the member once he or she is no longer incapacitated or unable to receive such information. Follow-up procedures must be in place to ensure that the information is given to the individual directly at the appropriate time.

Factor 2: State law

If a state law on advance directives affects an organization's compliance with this element, the organization is responsible for providing NCQA with a copy of the applicable state law on advance directives.

Factors 3–5

No additional explanation required.

Factor 6: Time frames for completing advance directive

The organization has policies in place regarding when a member should receive information on advance directives. Unless noted otherwise below, the organization provides the information to the adult member at the time of enrollment.

- *Hospital*: At the time of the member's admission as a patient.
- *Skilled nursing facility*: At the time of the member's admission as a resident.
- *Home health agency (HHA)*: In advance of the member coming under the care of the agency. The HHA may furnish advance directives information to a member at the time of the first home visit, as long as the information is furnished before care is provided.
- *Personal care services*: In advance of the individual coming under the care of the personal care services provider. The personal care provider may furnish advance directives information to a member at the time of the first home visit, as long as the information is furnished before care is provided.
- *Hospice program*: At the time of the initial receipt of hospice care.

Exceptions

None.

Examples None.

Element B: Conscience Protection Waiver Notice

If the MA organization has requested a conscience protection waiver from CMS, member materials must contain:

1. Clarification of any differences between organization-wide conscience objections and conscience objections that may be raised by individual practitioners.
2. The source of the state's legal authority permitting a conscience objection.
3. A description of the range of medical conditions, procedures and limitations affected by the conscience objection.

Summary of Changes

- This is a new element.

Scoring	Met	Partially Met	Not Met
	The organization meets 3 factors	No scoring option	The organization meets 0-2 factors
Data source	Materials		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> If the MA organization requested a conscience protection waiver from CMS, NCQA reviews member materials, such as member handbooks, newsletters or other relevant member documents, to determine whether the materials contain the required provisions. NCQA evaluates member materials that are actually in use and that have been sent to members. NCQA does not consider materials presented in draft form or those not yet distributed to members.		
Look-back period	<i>For Interim Surveys and First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	THIS IS A MUST-PASS ELEMENT. To score Met, the organization must address all three factors listed in Element B. Medicare law requires an MA organization and its network practitioners to follow requests made in a member's advance directive. Because of moral or religious beliefs, some MA organizations may decline to follow a member's advance directive request to withhold or withdraw life-sustaining care. This position is referred to as a conscience objection. Conscience protection waiver An MA organization that raises a conscience objection must request a conscience protection waiver from CMS that exempts the MA organization from the Medicare requirement. NCQA inquires whether the MA organization has requested a conscience protection waiver from CMS.		

Member materials

Medicare law requires an MA organization that requests a conscience protection waiver to notify members in member materials of the request and to clarify whether the waiver applies to the MA organization or to certain practitioners in the MA organization's network.

Member materials should identify the state law that permits the organization to raise the conscience objection and should describe the covered services to which the conscience objection applies.

Exception

This element is NA for MA organizations that have not requested a conscience protection waiver.

Examples None.

Element C: Advance Directives Rights Notification

The MA organization's written member materials:

1. Inform members at enrollment, and annually thereafter, of their right to accept or refuse treatment and to complete an advance directive, and inform them how to implement that right.
2. Inform members of their right to file a complaint with the state survey and certification agency.

Summary of Changes

- This is a new element.

Scoring	Met	Partially Met	Not Met
	The organization meets 2 factors	No scoring option	The organization meets 0-1 factors

Data source Materials

Scope of review *This element applies to Interim Surveys, First Surveys and Renewal Surveys.*

NCQA reviews materials provided to new members at enrollment, such as member handbooks, newsletters or other relevant member documents, to determine if the materials inform members of their right to accept or refuse treatment and to complete an advance directive and inform them how to complete an advance directive.

Additionally, NCQA reviews materials available to all members to determine if they inform members of their right to file a complaint with the state survey and certification agency concerning the organization's handling of advance directives.

NCQA evaluates member materials that are actually in use and that have been sent to members. NCQA does not consider materials presented in draft form or those not yet distributed to members.

Look-back period	<i>For Interim Surveys and First Surveys:</i> For factor 1, the organization is required to complete the activity at least once during the prior year. For factor 2, 6 months. <i>For Renewal Surveys:</i> 24 months.
Explanation	THIS IS A MUST-PASS ELEMENT. To score Met, the organization's member materials must address both factors listed in Element C. Medicare law requires MA organizations to ensure that members are fully informed of their rights concerning advance directives under state law. Materials provided at enrollment must tell members about their right to accept or refuse treatment offered by a practitioner and to complete an advance directive, and must tell members how to complete an advance directive. The information must reflect changes in state law as soon as possible, but no later than 90 days after the effective date of the state law. The organization must use its member materials to inform members that Medicare law gives them the right to file a complaint with the state survey and certification agency if they are dissatisfied with the organization's process for handling advance directives. Exceptions None.
Examples	None.

Element D: Advance Directives Community Outreach

The MA organization provides community education regarding advance directives either directly or indirectly through providers and other organizations.

Summary of Changes

- This is a new element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Reports, Materials		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the materials used for community education as well as reports demonstrating distribution and additional outreach efforts.		
Look-back period	<i>For All Surveys:</i> 6 months.		
Explanation	THIS IS A MUST-PASS ELEMENT. The organization provides community education regarding advance directives either directly or indirectly, through providers and other contracted organizations.		

The organization may develop materials specific to certain populations. However, all materials must include the following:

- Definition of what constitutes an advance directive.
- Details on the reasons why an advance directive is used. Specifically, to enhance an incapacitated member's control over his or her medical treatment.
- Description of all applicable state laws concerning advance directives.

Exceptions

None.

Examples None.

Element E: Medical Record Documentation

The MA organization has policies and procedures for medical record documentation of advance directives that require:

1. Documentation of advance directives to be in a prominent part of a member's medical record.
2. Documentation on whether or not a member has executed an advance directive to be included in the medical record.

Summary of Changes

- This is a new element.

Scoring	Met	Partially Met	Not Met
	The organization meets 2 factors	No scoring option	The organization meets 0-1 factors
Data source	Documented process		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organization's policies and procedures on documenting advance directives in members' medical records.		
Look-back period	<i>For Interim Surveys and First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	THIS IS A MUST-PASS ELEMENT. The medical record is a significant vehicle for a member to communicate the information contained in an advance directive. The MA organization demonstrates accountability by establishing policies and procedures documenting advance directives in medical records. Policies and procedures must require documentation of an advance directive to be in a prominent part of the medical record and must require the medical record to contain documentation about whether the member has executed an advance directive.		

The policies and procedures may be located in the organization's medical record documentation policies and procedures, in advance directives documentation or in policies and procedures from other relevant operational areas in the organization.

Exceptions

None.

Examples None.

Element F: Staff Training on Advance Directives

The MA organization demonstrates that its staff is provided education on advance directives.

Summary of Changes

- This is a new element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Documented process, Materials		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews documentation submitted by the MA organization that validates staff training on members' rights concerning advance directives.		
Look-back period	<i>For Interim Surveys and First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	THIS IS A MUST-PASS ELEMENT. MA organization staff who work in member services and benefits coverage must be aware of members' rights concerning advance directives. Medicare law requires the MA organization to demonstrate that staff have been educated on these rights. The MA organization may demonstrate that training has taken place through training agendas and accompanying attendance sheets, through a training curriculum on advance directives or through other means that can demonstrate staff received the required training. Staff training may be performed internally or externally. Exceptions None.		
Examples	None.		

MA 5: Notification to Practitioners Regarding Network Decisions

The MA organization notifies practitioners about the reason for not including the practitioner or group of practitioners in its network.

Intent

The organization notifies practitioners regarding network decisions.

Element A: Notification to Practitioners Regarding Network Decisions

The MA organization provides written notification to affected practitioners regarding the reason for its decision to not include the practitioner or group of practitioners in its network.

Summary of Changes

- This is a new element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Documented process, Materials		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organization's policies and procedures. NCQA also reviews the notification sent to affected practitioners throughout the look-back period.		
Look-back period	<i>For All Surveys:</i> 6 months.		
Explanation	THIS IS A MUST-PASS ELEMENT. The MA organization provides written notification to affected practitioners regarding the reason for its decision to not include the practitioner or group of practitioners in its network. In selecting its practitioners or groups of practitioners, the organization does not discriminate, in terms of participation, reimbursement or indemnification, against any health care professional, solely based on the practitioner's license or certification. This does not preclude the organization from: - Refusing to include a specific practitioner type in excess of the number necessary to meet the needs of the organization's members. - Using different reimbursement amounts for different specialists or for different practitioners in the same specialty. - Implementing measures designed to maintain quality and control costs. Exceptions None.		
Examples	None.		

MA 6: Credentialing Criteria and Performance Monitoring for Recredentialing

Not Applicable for MA Deeming

MA 7: Practitioner and Provider Participation

The MA organization adopts and implements policies and procedures for practitioner participation and also adopts and distributes clinical practice guidelines to practitioners and providers.

Intent

The organization has established policies and procedures and provided required notices relating to practitioner and provider participation.

Element A: Policies and Procedures

The MA organization's written policies and procedures:

1. State that the organization may not restrict a practitioner's ability to consult with members about their health status.
2. State that the organization may not operate a practitioner incentive plan.
3. State that the organization pays for private contracted providers for emergency services or urgently needed services.
4. Are distributed to practitioners upon entering into participation agreements with the organization.
5. Outline its process for encouraging practitioners and providers to participate in CMS and Health and Human Services (HHS) quality improvement initiatives as part of its quality improvement program.
6. State that contract providers confirm a member's enrollment with the MA organization.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 6 factors	No scoring option	The organization meets 0-5 factors
Data source	Documented process, Reports		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organization's policies and procedures, as well as reports demonstrating distribution of the policies to practitioners upon entering participation agreements with the organization.		
Look-back period	<i>For All Surveys: 6 months.</i>		

Explanation THIS IS A MUST-PASS ELEMENT.

The MA organization provides its medical policy, quality and medical management procedures to all practitioners and providers who have entered into participation agreements with the organization. The organization shall consult with practitioners and providers who have entered into participation agreements with the organization regarding its medical policy, quality and medical management procedures.

Factor 1: Practitioner consultations

The MA organization may not restrict a practitioner, acting within the lawful scope of practice from advising a member about the following, regardless of whether benefits for such care or treatment are provided under the plan:

- Member's health status, medical care or treatment for the member's condition or disease.
- Information sufficient to provide an opportunity to decide among all relevant treatment options.
- The risks, benefit and consequences of treatment or non-treatment.
- The opportunity for the member to refuse treatment and to express preferences about future treatment.

The MA organization is not required to provide reimbursement or provide coverage for the counseling or referral service if the organization makes available information on its policies regarding such service to prospective members before or during enrollment and to current members within 30 days before the effective date of the policy change and within 90 days after the date that the organization adopts a change in policy regarding such a counseling or referral service.

Factor 2: Practitioner incentive plans

The MA organization may not include practitioner incentives in its plan unless it has a process for complying with the following requirements:

- No payment is made, directly or indirectly, to reduce or limit medically necessary care.
- If the plan places a physician or physician group at substantial financial risk for services not provided by the physician or physician group, the organization provides stop-loss protection for the physician or physician group that is adequate and appropriate.
- For all physician incentive plans, the organization provides CMS with the information specified in 42 § CFR 422.210.

The MA organization must have a process for ensuring that all physicians and physician groups at substantial financial risk have either aggregate or per-patient stop-loss protection in accordance with the following requirements:

- Aggregate stop-loss protection must cover 90 percent of the costs of referral services that exceed 25 percent of potential payments.
- For per-patient stop-loss protection if the stop-loss protection provided is on a per-patient basis, the stop-loss limit (deductible) per patient must be determined based on the size of the patient panel and may be a combined policy or consist of separate policies for professional services and institutional services.

- Stop-loss protection must cover at least 90 percent of the costs of referral services above the deductible or an actuarial equivalent amount of the costs of referral services that exceed the per patient deductible limit and otherwise be determined consistent with 42 § CFR 422.202(f)(2)(iii).
- MA FFS plans are prohibited from operating physician incentive plans.

Factor 3: Private contracted practitioners

The MA organization does not pay for services to private contracted practitioners unless they are emergency services or urgently needed services. The MA organization pays for any services provided to a Medicare member considered to be emergency cases or urgently needed services.

Factor 4: Distribution to practitioners

The organization provides written information to practitioners regarding terms of payment, credentialing and other rules directly related to participation decisions.

The organization also provides written notice to practitioners regarding changes in participation rules before the changes are put into effect.

The MA organization uses the postal service, electronic mail, FAX or telephone to communicate the information to one of the following:

- The provider, the employer or billing agent of the provider.
- A partnership of which the provider is a member.
- Any party to which the provider assigns or reassigns benefits.

The MA organization has in effect a procedure under which:

- Any provider furnishing services to a member in an MA private fee-for-service plan, and who has not previously entered into a contract or agreement to furnish services under the plan, can receive instructions on how to request the payment information;
- The organization responds to the request before the entity furnishes the service; and
- The information the organization provides includes the following: billing procedures, the amount the organization will pay toward the service, the amount if the provider is permitted to collect from the member, the information described in §422.202(a)(1).

Announcements in newspapers, journals, or magazines or on radio or television are not considered communication of the terms and conditions of payment.

Contracts with providers must provide that, in order to be paid to provide services to plan members, providers must meet the requirements specified in §§422.204(b)(1)(i) and (b)(3).

Factor 5: Practitioner and provider participation in quality improvement initiatives

The organization must encourage its practitioners and providers to participate in CMS and HHS quality improvement initiatives as part of its quality improvement program.

Factor 6: Enrollment confirmation

NCQA ensures the MA organization has a policy, procedure or clause in the contract that ensures contracted providers confirm a beneficiary's enrollment in the MA organization. The proof can come from the enrollment card, CMS, intermediary/carrier or the MA organization.

Exceptions

None.

Examples None.

Element B: Rules for MA Fee-For-Service Plans

Practitioners are treated as having an agreement with the MA organization if:

1. The practitioner or other entity furnishes services that are covered under the plan to such a member.
2. If before providing such services, the practitioner or other entity has been informed of the member's enrollment under the plan.
3. The practitioner has been informed of the terms and conditions of payment for such services under the plan, or is given a reasonable opportunity to obtain information concerning such terms and conditions, in a manner reasonably designed to effect informed agreement by a provider.

Summary of Changes

- This is a new element.

Scoring	Met	Partially Met	Not Met
	The organization meets 3 factors	No scoring option	The organization meets 0-2 factors
Data source	Documented process		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organization's policies and procedures.		
Look-back period	For All Surveys: 6 months.		
Explanation	THIS IS A MUST-PASS ELEMENT. Practitioners and providers providing care to members of Medicare Choice plans are treated as having an agreement or contract with the Medicare organization, if the practitioner or provider provides services that are covered under the plan to the member, is informed before providing such services of the member's enrollment under the plan, and has been informed of the terms and conditions of payment for such services under the plan, or is given a reasonable opportunity to obtain information on such terms and conditions, in a manner reasonably designed to effect informed agreement by a provider.		

Exception

If there is an explicit agreement between the practitioner or other entity and the Medicare Choice organization, this element is NA.

Examples None.

Element C: Limitation on Provider Indemnification

The organization has a process in place for limitations on practitioner and provider indemnification.

Summary of Changes

- This is a new element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Documented process		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organization's policies and procedures.		
Look-back period	<i>For All Surveys:</i> 6 months.		
Explanation	THIS IS A MUST-PASS ELEMENT. The organization does not allow a health care practitioner or provider to indemnify the organization against any liability resulting from a civil action brought for any damage caused to a member resulting from a denial by the organization of medically necessary care.		
	Exceptions None.		
Examples	None.		

Element D: Conditions for Pooling of Patients

The organization has a process in place for the pooling of members that meets the following conditions:

1. The process is consistent with the relevant contracts governing the compensation arrangements for contracting physician or physician groups.
2. Physician or physician groups are at risk for referral services with respect to each of the categories of members being pooled.
3. The terms of the compensation arrangements permit physicians or physician groups to spread the risk across the categories of members being pooled.
4. The distribution of payments to physicians from the risk pool is not calculated separately by member category.
5. The terms of the risk borne by physicians or physician groups are comparable for all categories of members being pooled.

Summary of Changes

- This is a new element.

Scoring	Met	Partially Met	Not Met
	The organization meets 5 factors	No scoring option	The organization meets 0-4 factors
Data source	Documented process		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organization's policies and procedures.		
Look-back period	<i>For All Surveys:</i> 6 months.		
Explanation	THIS IS A MUST-PASS ELEMENT. No additional explanation required.		
	Exceptions None.		
Examples	None.		

Element E: Adoption and Distribution of Practice Guidelines

The MA organization has a formal mechanism to consult with the physicians who have agreed to provide services under the MA plan offered by the organization, regarding the organization's medical policy, quality improvement programs and medical management procedures and ensure that the following standards are met.

1. Practice guidelines and utilization management guidelines are based on reasonable medical evidence or a consensus of health care practitioners in the particular field.
2. Consider the needs of the organization's members.
3. Are adopted in consultation with contracted health care professionals.
4. Are reviewed and updated periodically.

Summary of Changes

- This is a new element.

Scoring	Met	Partially Met	Not Met
	The organization meets 4 factors	No scoring option	The organization meets 0-3 factors
Data source	Documented process, Reports, Materials		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organization's policies and procedures.		
Look-back period	For All Surveys: 6 months.		
Explanation	THIS IS A MUST-PASS ELEMENT. The organization communicates guidelines to providers and, as appropriate, to members. In addition, decisions with respect to utilization management, member education, coverage of services, and other areas in which the guidelines apply are consistent with the guidelines. Factor 1: Guidelines based on clinical evidence The organization's QI Committee or other designated clinical committee approves clinical practice guidelines. The organization adopts guidelines from recognized sources. It uses one of the following in adopting its clinical practice guidelines: • Reasonable medical evidence, or • A consensus of healthcare professionals in the particular field. Factor 2: Guidelines that consider member needs The organization adopts clinical practice guidelines that consider the needs of its members, which may include guidelines related to any applicable acute or chronic conditions, behavioral health-related issues and guidelines that address preventive or nonpreventive health concerns.		

Factor 3: Adoption in consultation with contracted healthcare professionals

The organization adopts guidelines with feedback from board-certified, contracted practitioners.

Factor 4: Review and update

The organization reviews its guidelines against clinical evidence at least every two years, or more frequently if national guidelines change within the two-year period.

Exceptions

None.

Examples

None.

MA 8: Practitioner and Provider Network*Not Applicable for MA Deeming***MA 9: Contracts—Medicare Services and Termination Notice**

The MA organization supports a network of physicians and providers with written arrangements that address the provision of services covered under the MA program. The contracts between the MA organization and a contracting physician provide at least 60 days written notice to each other before terminating the contract without cause.

Intent

Physician and provider contracts address confidentiality, compliance with Medicare law, contract termination and notification, and accuracy of member information.

Element A: Requirements for Physician Contracts*Not Applicable for MA Deeming***Element B: Written Notice of Contract Termination**

The MA organization's contracts with network physicians require each party to provide at least 60 days written notice to each other before terminating the contract without cause.

Summary of Changes

- This is a new element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement

Data source Materials

Scope of review *This element applies to Interim Surveys, First Surveys and Renewal Surveys.*
NCQA reviews the MA organization's current physician contract.

Look-back period *For Interim Surveys and First Surveys: 6 months.*
For Renewal Surveys: 24 months.

Explanation **THIS IS A MUST-PASS ELEMENT.**

Medicare law recognizes that the decision to terminate a contract between a network physician and an MA organization may have substantial impact on members. Therefore, the MA organization and its contracting physicians are required to give each other 60 days written notice before terminating a contract without cause.

NCQA reviews the MA organization's current physician contract to ensure that it contains a provision requiring both parties to give at least 60 days written notice before terminating the contract without cause.

Exceptions

None.

Examples

None.

Element C: Requirements for Provider Contracts*Not Applicable for MA Deeming***Element D: Actions Against Practitioners**

The MA organization has policies and procedures for reporting suspensions or terminations due to deficiencies in quality of care to the appropriate authorities.

Summary of Changes

- This is a new element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Documented process		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the MA organization's policies and procedures.		
Look-back period	<i>For Interim Surveys and First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	THIS IS A MUST-PASS ELEMENT. This element is a structural requirement. The organization must present its own documentation. Appropriate authority includes NPDB, state agency or other regulatory body, as appropriate. Policies and procedures describe: • What specific incidents are reportable. • How, and when, reporting to authorities occurs. • To whom incidents are reported. • Reporting responsibilities of organization staff. Exceptions None.		
Examples	None.		

MA 10: Contracts*Not Applicable for MA Deeming***MA 11: Appeals Process for Termination/Suspension**

On suspension or termination of a contract with a participating physician, the MA organization gives the affected physician written notice of the appeal process and the reason for the suspension or termination. The MA organization ensures that the majority of the appeal hearing panel members are peers of the affected physician. The organization notifies the physician in writing of an initial adverse decision.

Intent

The organization notifies participating physicians of the appeal process and ensures peer review of appeals.

Element A: Policies and Procedures for Suspension or Termination

The MA organization's policies and procedures regarding suspension or termination of a participating physician require the organization to:

1. Give written notification to the affected physician of the reasons for the action.
2. Give written notification to the affected physician of the appeal process.
3. Ensure that the majority of the hearing panel members are peers of the affected physician.

Summary of Changes

- This is a new element.

Scoring	Met	Partially Met	Not Met
	The organization meets 3 factors	No scoring option	The organization meets 0-2 factors
Data source	Documented process		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organization's policies and procedures.		
Look-back period	<i>For Interim Surveys and First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	THIS IS A MUST-PASS ELEMENT. To score Met, the organization's policies and procedures must cover all three factors in Element A. MA regulations require that a formal process, including an opportunity for appeal, must be followed when it suspends or terminates a physician's contract. The organization must have policies and procedures in place that require the affected physician to be notified of the appeals process in writing and the reasons for the suspension or termination. The organization provides the affected physician an opportunity to present information during the appeal process.		

The organization must ensure that the majority of the hearing panel members are peers of the affected physician. With regard to hearing panel members under this requirement, a **peer** is an appropriately trained and licensed physician in a practice similar to that of the affected physician. Panel members do not have to have identical specialty training.

Exceptions

None.

Examples None.

Element B: Policies and Procedures for the Network Application

The MA organization's policies and procedures regarding a physician's initial application to become a participating provider require the organization to give written notification to the affected physician if the application is denied.

Summary of Changes

- This is a new element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Documented process		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organization's policies and procedures.		
Look-back period	<i>For Interim Surveys and First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	THIS IS A MUST-PASS ELEMENT. MA regulations require an organization to notify a physician applicant in writing when the organization denies the applicant's request to join the organization's provider panel. NCQA reviews the organization's policies and procedures to determine whether the organization notifies a physician in writing when it denies the physician's application to become a participating provider. Exceptions None. Examples None.		

MA 12: Conscience Objection Notice

An MA organization that objects to covering, furnishing or paying for a particular counseling, referral or emergency service on moral or religious grounds must provide sufficient notice of its conscience protection observance to CMS and prospective and current members by submitting its conscience protection policies to CMS with its application for a Medicare contract, or within 10 days of submitting its bid proposal; providing explanatory information to prospective members before or during enrollment; and providing explanatory information to current members at least 30 days before the effective date of the policy change and within 90 days of adopting a new policy on conscience objection.

Intent

The organization notifies members and CMS of its conscience protection policies.

Element A: Conscience Protection Policies

The MA organization asserts conscience protection and has:

1. A process for submitting its conscience protection policies to CMS with its application for a Medicare contract, or within 10 days of submitting its bid proposal to CMS.
2. Provided explanatory information regarding conscience protection to prospective members before or during enrollment.
3. Provided explanatory information regarding conscience protection to current members annually.

Summary of Changes

- This is a new element.

Scoring	Met	Partially Met	Not Met
	The organization meets 3 factors	No scoring option	The organization meets 0-2 factors
Data source	Documented process, Materials		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organization's process for Factor 1. NCQA also reviews the organization's materials for evidence that the requirements were met for factors 2 and 3.		
Look-back period	<i>For Interim Surveys and First Surveys:</i> For factors 1 and 2, 6 months. For factor 3, the organization is required to complete the activity at least once during the prior year. <i>For Renewal Surveys:</i> 24 months.		
Explanation	THIS IS A MUST-PASS ELEMENT. To score Met, the organization must satisfy all three factors in Element A. Medicare law permits an MA organization to assert conscience protection when the organization objects to covering, furnishing or paying for a particular		

counseling, referral or emergency service based on moral or religious grounds. Under such circumstances, the law requires the organization to provide sufficient notice of its conscience protection policies to CMS and to prospective and current members.

Bid proposal

MA organizations annually submit to the federal government, proposed premium rates based on member use of benefits. This submission is called the **bid proposal**.

Factor 1: Notice of conscience protection to CMS

The organization has a process for notifying CMS of its conscience protection policies by submitting the policies with its application for an MA contract within 10 days of submitting its bid proposal, or for policy changes in accordance with all applicable requirements under subpart V of this part (see 42 CFR Subpart V).

Factors 2, 3: Written notice of conscience protection to members

An MA organization that asserts conscience protection must provide information explaining its conscience protection policies regarding counseling or referral services to prospective members before or during enrollment and to current members annually, within 30 days before the effective date of the policy change and within 90 days after the date the organization adopts a change in policy.

For prospective members, NCQA examines marketing materials such as member handbooks, newsletters or other relevant member documents distributed before and during enrollment, to ensure that explanatory information concerning the organization's conscience protection policies is included in the materials.

For current members, NCQA reviews explanatory information about the organization's conscience protection policies that it provided to members annually.

NCQA evaluates materials that are in use and are provided to prospective members and members currently enrolled in the organization. NCQA does not consider materials presented in draft form or those not yet in use.

Exception

This element is NA if the organization does not assert conscience protection for covering, furnishing or paying for particular counseling or referral services based on moral or religious grounds.

Examples

None.

MA 13: Direct Access to Practitioners*Not Applicable for MA Deeming***MA 14: Access to Services***Not Applicable for MA Deeming***MA 15: Emergency Services***Not Applicable for MA Deeming***MA 16: Standards for Medical Record Documentation**

The organization requires medical records to be maintained in a manner that is current, detailed and organized for effective and confidential patient care and quality review.

Intent

The organization has established medical record standards to facilitate communication, coordination and continuation of care.

Element A: Medical Record Criteria

The organization has policies and procedures that address the following information, and distributes the policies and procedures to practice sites:

1. Confidentiality of medical records.
2. Medical record documentation standards.
3. An organized medical record keeping system and standards for availability of medical records.
4. Performance goals to assess the quality of medical record keeping.

Summary of Changes

- This is a new element.

Scoring	Met	Partially Met	Not Met
	The organization meets 4 factors	No scoring option	The organization meets 0-3 factors
Data source	Documented process, Materials		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organization's policies and procedures and materials for evidence that the requirements are met.		
Look-back period	<i>For Interim Surveys and First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		

Explanation THIS IS A MUST-PASS ELEMENT.

The organization demonstrates accountability by establishing and distributing medical record standards and by ensuring that practitioners in its network comply with the standards.

The organization's medical record policies, procedures and documentation standards must discuss the following:

- Medical record content.
- Medical record organization.
- Ease of retrieving medical records.
- Confidential patient information.
- Standards and performance goals for participating practitioners.

The organization must distribute the medical record standards to all practitioners and appropriate staff members.

The organization's policies and procedures for primary care medical records reflect the following information.

- All services provided directly by a practitioner who provides primary care services.
- All ancillary services and diagnostic tests ordered by a practitioner.
- All diagnostic and therapeutic services for which a member was referred by a practitioner, such as:
 - Home health nursing reports.
 - Specialty physician reports.
 - Hospital discharge reports.
 - Physical therapy reports.

Factor 1: Confidentiality of medical records

- Records are stored securely.
- Only authorized personnel have access to records.
- Staff receive periodic training in member information confidentiality.

Factors 2–4

No additional explanation required.

Factor 2: Medical record documentation standards

Each medical record must include the following information:

- History and physicals.
- Allergies and adverse reactions.
- Problem list.
- Medications.
- Documentation of clinical findings and evaluation for each visit.
- Preventive services/risk screening.

Factor 3: Organized medical record keeping systems/standards for the availability of medical records

- Medical records are organized and stored in a manner that allows easy retrieval.
- Medical records are stored in a secure manner that allows access by authorized personnel only.

Factor 4: Performance goals to assess the quality of medical record keeping

The practitioner must meet the organization's established quantifiable performance goals for medical record keeping. NCQA does not prescribe a specific performance goal, but expects that the organization's performance goals are clearly defined in its medical record policies.

Exceptions

None.

Examples None.

Element B: Improving Medical Record Keeping

Not Applicable for MA Deeming

Element C: Distribution of Medical Record Information

The organization's policies and procedures specify that it maintains medical records and other health information regarding members in a manner that is:

1. Accurate.
2. Timely.
3. Safeguards the privacy of any identifiable information.

Summary of Changes

- This is a new element.

Scoring	Met	Partially Met	Not Met
	The organization meets 3 factors	No scoring option	The organization meets 0-2 factors
Data source	Documented process		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organization's policies and procedures.		
Look-back period	For All Surveys: 6 months.		
Explanation	THIS IS A MUST-PASS ELEMENT. Factor 1: Accurate information No explanation required.		

Factor 2: Timely access to information

The organization ensures that members have timely access to medical record and other health information. Medical information is released in accordance with applicable federal or state law, or pursuant to court orders or subpoenas.

Factor 3: Safeguarding identifiable information

The organization specifies to whom and for what purposes it will use member health information within the organization and to whom and for what purpose it will disclose member health information outside the organization.

Exceptions

None.

Examples

None.

MA 17: Quality Improvement Program

The MA organization measures and reports performance to CMS.

Intent

The organization measures and reports performance to CMS.

Element A: Data Collection and Reporting Performance to CMS

The organization has a process for collecting, analyzing and reporting all data, including performance data, to CMS.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement

Data source Documented process, Reports, Materials

Scope of review *This element applies to Interim Surveys, First Surveys and Renewal Surveys.*
 NCQA reviews the organization's policies and procedures.
 NCQA also reviews the organization's most recent report or other evidence that the organization meets the requirement and reviews for evidence of distribution to CMS.

Look-back period *For All Surveys: 6 months.*

Explanation **THIS IS A MUST-PASS ELEMENT.**

NCQA verifies that the MA organization has a process for satisfying the following requirements.

Data requirements for all plan types

The MA organization makes all collected information available to CMS.

Data requirements for MA coordinated care plans (CCPs) (except for regional MA plans) and including local PPO plans are offered by organizations that are licensed or organized under state law as HMOs.

The MA organization's quality improvement program does the following:

- Measures and reports performance and, in particular:
 - Measures performance under the plan, using the measurement tools required by CMS, and reports its performance to CMS. The standard measures may be specified in uniform data collection and reporting instruments required by CMS.
 - Ensures that the information it receives from providers of services is reliable and complete.

- Collects, analyzes and reports quality performance data identified by CMS.
- Makes available to CMS information on quality and outcomes measures that will enable beneficiaries to compare health coverage options and select among them.

Data requirements for MA regional plans and MA local plans that are PPO plans

The MA organization does the following:

- Measures performance under the plan using standard measures required by CMS and reports its performance to CMS. The standard measures may be specified in uniform data collection and reporting instruments required by CMS.
- Collects, analyzes and reports quality performance data identified by CMS.

Data requirements for physician incentive plans

The MA organization does the following:

- Makes available the information specified below to members who request it, as well as to CMS:
 - Whether or not the MA organization uses a physician incentive plan that affects the use of referral services.
 - The type of incentive arrangement.
 - Whether or not stop-loss protection is provided.

CAHPS data

The MA organization has a contract with a CAHPS survey vendor to conduct the Medicare CAHPS satisfaction survey of Medicare plan members in accordance with CMS specifications. The organization submits the survey data to CMS.

Reviewing data received

The MA organization reviews the data it receives from providers for reliability and completeness.

Corrective action

The MA organization corrects any issues identified through surveillance, complaints or other mechanisms.

Exceptions

None.

Examples

None.

Element B: Health Information System

The organization maintains a health information system that collects, analyzes and integrates the data necessary to implement its quality improvement program.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Documented process, Reports, Materials		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organization's policies and procedures. NCQA also reviews the organization's health information system used to implement its quality improvement program. The organization may provide annotated screen shots or other evidence of system functionality.		
Look-back period	<i>For All Surveys:</i> 6 months.		
Explanation	THIS IS A MUST-PASS ELEMENT. The organization maintains a health information system that collects, analyzes and integrates data necessary to improve its quality improvement program. Exceptions None.		
Examples	None.		

MA 18: Ensuring Appropriate Utilization

The MA organization monitors the impact of its UM program to detect and correct potential under- and overutilization of services.

Intent

The organization monitors and analyzes relevant data and acts to correct patterns of potential or actual inappropriate underutilization or overutilization.

Element A: Under-/Overutilization Thresholds

The organization sets thresholds for four data types, including behavioral health, and annually analyzes data against the established thresholds to detect under- and overutilization.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement

Data source Documented process, Reports

Scope of review *This element applies to Interim Surveys, First Surveys and Renewal Surveys.*
NCQA reviews the organization's policies and procedures and reports for evidence that the requirements are met.

Look-back period *For Interim Surveys and First Surveys:* The organization is required to complete the activity at least once during the prior year.
For Renewal Surveys: 24 months.

Explanation **THIS IS A MUST-PASS ELEMENT.**

The organization uses external or internal sources that provide distribution of performance data (e.g., by percentiles or mean rates) to set performance thresholds. The organization does not need to have an over- and underutilization threshold for each data type, but at least one data type must have a threshold to detect underutilization and at least one must have a threshold to detect overutilization.

Quantitative data analysis

Analysis of monitored data is critical to developing effective interventions; therefore, it must include practitioners and other personnel who understand processes of care and potential barriers to improvement.

To determine the meaning of the data, the organization must conduct a quantitative analysis against the established thresholds. The organization must:

- Compare the data in its four chosen data types to available sources of comparable data, which may be from internal or external sources.
- Assess the possibility of under- and overutilization.

The analysis must be performed at least annually and must include:

- Thresholds that the organization uses to flag potential under- and overutilization.
- Periodic (at least annual) reports of findings.
- Evidence that the analysis results in appropriately identified areas or procedures in need of improvement.

If an organization is part of a corporate family that collects and aggregates data from multiple organizations within its corporate family, it may use regional results (thresholds) as an external source for comparative analyses.

Exception

If behavioral healthcare is carved out of product lines brought forward for Accreditation, the organization must set thresholds for four types of utilization data, although one does not need to be specific to behavioral healthcare.

Examples

Data types

- Length of stay.
- Inpatient acute care days or discharge.
- Unplanned readmission.

Under or overutilization thresholds

The organization decides that its rates for selected procedures should not fall below the 10th percentile for the region.

Quantitative data analysis

The following list provides examples of comparable external data. Some sources provide distribution of data by percentiles or rates and standard deviations from the mean or data that allow comparison within regions and by age:

- Quality Compass provides comparative data by region.
- Other regional health reporting initiatives compile comparative data on HEDIS and other measures.
- Pharmacy benefit managers (PBM) may have comparative data on pharmaceutical use.
- Companies that provide utilization review guidelines have comparative data related to guidelines.

Associations and corporate organizations to which multiple organizations belong often provide comparative data, thresholds and benchmarks.

Element B: Informing Members and Providers

The organization communicates the results of the data analysis on utilization to members and providers of services.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Reports, Materials		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews reports or other documents for evidence of distribution to members and providers.		
Look-back period	<i>For All Surveys:</i> At least once in the prior year.		
Explanation	THIS IS A MUST-PASS ELEMENT. The organization annually distributes the information collected on the utilization of services and results of the data analysis in Element A to members and providers.		
Examples	None.		

MA 19: Delegation of Health Plan Standards

The organization includes a statement in its delegation agreements that delegates must adhere to MA regulations.

Intent

The organization's delegation agreements require delegates to adhere to MA regulations.

Element A: Delegate Adherence to MA Requirements

All delegation agreements include a statement that delegates must adhere to MA requirements.

Summary of Changes

- This is a new element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement
Data source	Documented process, Materials		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organization's policies and procedures and materials for evidence that the requirements are met.		
Look-back period	<i>For Interim Surveys and First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	THIS IS A MUST-PASS ELEMENT. The organization's delegation agreements for Quality Improvement (QI), Utilization Management (UM), Credentialing (CR), or Member Experience (ME) standards includes a statement requiring delegates to adhere to MA regulations. Exception This element is NA for organizations that do not delegate QI, UM, CR or ME activities.		
Examples	None.		

MA 20: QI Program Structure

The organization clearly defines its quality improvement (QI) structures and processes and assigns responsibility to appropriate individuals.

Intent

The organization has the QI infrastructure necessary to improve the quality and safety of clinical care and services it provides to its members.

Element A: QI Program Structure

The organization's QI program description specifies:

1. The QI program structure.
2. The behavioral healthcare aspects of the program.
3. How patient safety is addressed.
4. Involvement of a designated physician in the QI program.
5. Involvement of a behavioral healthcare practitioner in the behavioral aspects of the program.
6. Oversight of QI functions of the organization by the QI Committee.
7. An annual work plan.*
8. Objectives for serving a culturally and linguistically diverse membership.
9. Objectives for serving members with complex health needs.

**Critical factors: These factors must be scored "yes" to score at least "Partially Met."*

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 8-9 factors	No scoring option	The organization meets 0-7 factors

Data source Documented process, Reports

Scope of review *This element applies to Interim Surveys, First Surveys and Renewal Surveys.*

For Interim Surveys and First Surveys: NCQA reviews the organization's QI program description that is in place throughout the look-back period and the annual work plan.

For Renewal Surveys: NCQA reviews the organization's QI program description that is in place throughout the look-back period and the most recent and the previous year's annual work plans.

Look-back period *For Interim Surveys:* Prior to the survey date.

For First Surveys: 6 months.

For Renewal Surveys: 24 months.

Explanation THIS IS A MUST-PASS ELEMENT.

This element is a **structural requirement**. The organization must present its own documentation.

Factor 7 is a **critical factor** that must be scored “yes” for the organization to score at least “Met” on this element.

The QI program description

The QI program description is a comprehensive document or a set of documents that describes, in plain language, the QI program’s governance, scope, goals, measurable objectives, structure, responsibilities and annual work plan.

Factor 1: Program structure

The program description includes the following information about the QI structure:

- The QI program’s functional areas and their responsibilities.
- Reporting relationships of QI Department staff, QI Committee and any subcommittee.
- Resources and analytical support.
- Delegated QI activities, if the organization delegates QI activities.
- Collaborative QI activities, if any.

Factor 2: Behavioral healthcare

The program description outlines the organization’s efforts to monitor and improve behavioral healthcare.

Factor 3: Patient safety

The program description states how the organization works to improve the safety of clinical care and services provided to its members.

The organization may use performance data from QI activities conducted for other elements to determine safety initiatives to address for this element. Initiatives may focus on members, practitioners or providers.

Factor 4: Involvement of designated physician

The program description describes the role of the designated physician in the QI program, which includes participating in or advising the QI Committee or a subcommittee that reports to the QI Committee.

The physician may be a medical director, associate medical director or other physician assigned this task.

Factor 5: Involvement of designated behavioral healthcare practitioner

The program description specifies the role of the designated behavioral healthcare practitioner in the QI program, which includes participating in or advising the QI Committee or a behavioral healthcare subcommittee that reports to the QI Committee.

The behavioral healthcare practitioner must be a medical doctor or have a clinical PhD or PsyD and may be a medical director, clinical director, participating practitioner from the organization or behavioral healthcare delegate (if applicable).

Factor 6: QI Committee oversight

The program description defines the role, function and reporting relationships of the QI Committee and subcommittees, including committees associated with oversight of delegated activities.

- If participating practitioners are not members of the QI committee, they are involved in a clinical subcommittee or relevant ad hoc task forces.

Note: *Participating practitioners are external to the organization and part of the organization's network.*

- If organization staff are not members of the QI committee, they are involved in multidisciplinary work groups or subcommittees.

Factor 7: Annual work plan

The program description includes a work plan that reflects ongoing progress on QI activities throughout the year and addresses:

- Yearly planned QI activities and objectives for improving.
 - Quality of clinical care.
 - Safety of clinical care.
 - Quality of service.
 - Members' experience.
- Time frame for each activity's completion.
- Staff members responsible for each activity.
- Monitoring of previously identified issues.
- Evaluation of the QI program.

The work plan is a dynamic document. All bulleted requirements must be described for factor 7 to be scored "Yes."

Factor 8: Serving a diverse membership

The program description outlines the organization's approach to address the cultural and linguistic needs of its membership. The QI program description includes objectives or other objectives the organization deems appropriate.

Factor 9: Serving members with complex health needs

The program description outlines the organization's approach to addressing the needs of members with complex health issues, which may include:

- Physical disabilities.
- Developmental disabilities.
- Chronic conditions.
- Severe mental illness.

Exceptions

Factors 2 and 5 are NA if all purchasers of the organization's services carve out behavioral healthcare services (i.e., contract for a service or function to be performed by an entity other than the organization).

Related information

Collaborative activities. If the organization collaborates with other organizations on QI activities:

- It includes information about the collaborative and QI activities performed in the QI program description.
- It has communication and feedback mechanisms between the collaborative group and its internal QI Committee.

If the collaborative group has its own QI committee for carrying out functions, the organization may consider it to be a subcommittee of the QI Committee.

Examples**Factor 3: Patient safety improvement initiatives***Member-focused initiatives*

- Distribute information to members that improves their knowledge about clinical safety regarding their care.
 - Questions to ask surgeons prior to surgery.
 - Questions to ask about drug-drug interaction.
- Distribute information to members that facilitates informed decisions based on safety.
 - Facilities with computerized pharmacy order-entry systems.
 - Hospitals that use physicians trained in intensive care.
 - Providers that have best practices or outcomes often based on volume.

Practitioner and provider initiatives

- Collaborate with network practitioners and providers to:
 - Conduct in-service training focused on improving knowledge of safe practices (e.g., improving medical record legibility; establishing systems for timely follow-up of lab results).
 - Reduce identified adverse outcomes or polypharmacy issues.
 - Develop incentives for achieving safer clinical practices.

Factor 8: Objectives for serving diverse membership*Objectives*

- Reduce health care disparities in clinical areas.
- Improve cultural competency in materials and communications.
- Improve network adequacy to meet the needs of underserved groups.
- Improve other areas of needs the organization deems appropriate.

Approach

- Analyze significant health care disparities in clinical areas.
- Use practitioner and provider chart reviews and interviews to understand the differences in care provided and outcomes achieved.
- Conduct patient-focused interventions with culturally competent outreach materials that focus on risks specific to race/ethnicity/language.
- Conduct focus groups or key informant interviews with cultural or linguistic minority members to determine how to meet their needs.
- Identify and reduce a specific health care disparity.
- Provide information, training and tools to staff and practitioners to support culturally competent communication.

Factor 9: Serving members with complex health needs

- Provide case management teams that focus on:
 - Members who have had an organ transplant.
 - Members with HIV/AIDS, progressive degenerative disorders and metastatic cancers.
- Improve access to primary and specialty care to facilitate the receipt of appropriate services for members with complex health conditions.
- Coordinate care for members who receive multiple services.
- Identify and reduce barriers to services for members with complex conditions.

Element B: Annual Evaluation

The organization conducts an annual written evaluation of the QI program that includes the following information:

1. A description of completed and ongoing QI activities that address quality and safety of clinical care and quality of service.
2. Trending of measures of performance to assess performance in the quality and safety of clinical care and quality of service.
3. Analysis and evaluation of the overall effectiveness of the QI program and of its progress toward influencing networkwide safe clinical practices.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 3 factors	No scoring option	The organization meets 0-2 factors

Data source Reports

Scope of review *This element applies to Interim Surveys, First Surveys and Renewal Surveys.*
For Interim Surveys and First Surveys: NCQA reviews the organization's most recent annual evaluation report.
For Renewal Surveys: NCQA reviews the organization's most recent and the previous year's annual evaluation report.

Look-back period *For Interim Surveys and First Surveys:* At least once during the prior year.
For Renewal Surveys: 24 months.

Explanation **THIS IS A MUST-PASS ELEMENT.**

This is a **structural requirement**. The organization must present its own documentation.

The organization annually evaluates how well it met its performance goals and objectives for improving the quality and safety of clinical care and services specified within its QI program description. The organization summarizes its findings in an annual report.

Factor 1: Completed and ongoing QI activities

The annual evaluation summarizes completed and ongoing QI activities outlined within the QI program description.

Factor 2: Trending of QI measure results

The annual evaluation includes trending of QI measures results over time and comparison against performance objectives defined within the QI program description. The organization uses charts, graphs, tables or a combination thereof to display trend data.

Results do not need to be trended for First Surveys.

Factor 3: Analysis and evaluation of effectiveness

The annual evaluation includes the organization's determination of the overall effectiveness of the QI program and its progress in meeting safe clinical practice goals, based on an assessment of the performance in all aspects of the QI program. The summary of effectiveness addresses:

- Adequacy of QI program resources.
- QI Committee and subcommittee structure.
- Practitioner participation and leadership involvement in the QI program.
- Need to restructure or change the QI program for the subsequent year.

Exceptions

None.

Examples**Annual written evaluation contents**

- The title of each QI program initiative described in the work plan.
- A description of the program.
- The program's major accomplishments.
- Appropriate measures trended over time, including:
 - HEDIS data.
 - Organization-specific data.
- Barriers to achieving objectives.
- Recommended interventions for overcoming issues and barriers.
- Whether planned yearly activities were completed and objectives were met.
- Whether the QI program will be restructured or changed in the subsequent year.

MA 21: Program Operations*Not Applicable for MA Deeming***MA 22: Continuity and Coordination of Medical Care**

The organization monitors and takes action, as necessary, to improve continuity and coordination of care across the health care network.

Intent

The organization uses information at its disposal to facilitate continuity and coordination of medical care across its delivery system.

Element A: Identifying Opportunities

The organization annually identifies opportunities to improve coordination of medical care by:

1. Collecting data on member movement between practitioners.
2. Collecting data on member movement across settings.
3. Conducting quantitative and causal analysis of data to identify improvement opportunities.
4. Identifying and selecting one opportunity for improvement.
5. Identifying and selecting a second opportunity for improvement.
6. Identifying and selecting a third opportunity for improvement.
7. Identifying and selecting a fourth opportunity for improvement.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 6-7 factors	No scoring option	The organization meets 0-5 factors

Data source Reports

Scope of review *This element applies to Interim Surveys, First Surveys and Renewal Surveys.*

NCQA reviews the organization's medical care coordination data, quantitative and qualitative analysis and four opportunities for improvement selected from the data analysis.

For Interim Surveys and First Surveys: NCQA also reviews the organization's most recent report regarding annual data collection, evaluation and identification of opportunities.

For Renewal Surveys: NCQA also reviews the organization's most recent and the previous year's reports regarding annual data collection, evaluation and identification of opportunities.

Look-back period *For Interim Surveys and First Surveys:* At least once during the prior year.
For Renewal Surveys: 24 months.

Explanation **THIS IS A MUST-PASS ELEMENT.**

Continuity and coordination of medical care services is the facilitation, across transitions and settings of care, of:

- Patients getting the care or services they need, **and**
- Practitioners or providers getting the information they need to provide the care patients need.

The organization uses valid data collection and analysis methodology to identify opportunities for improvement.

Factors 1, 2: Collecting data

Care transitions refers to members moving between health care practitioners and across settings as their conditions and care needs change during the course of a chronic or acute illness. The organization collects data to assess coordination of care during care transitions.

Movement of members between practitioners includes the inception or cessation of patient care by a practitioner and coordination of care across practitioners who are concurrently or intermittently providing ongoing care for members.

Member movement across settings usually occurs as a member's health status changes (e.g., moving from home to hospital, moving from the hospital to a rehabilitation facility).

Data collected for factors 1 and 2 are analyzed in factor 3 and result in opportunities for factors 4–7. Because each opportunity in factors 4–7 must address a unique area of measurement, at least four measures must be presented based on the data collected for factors 1 and 2.

Factor 3: Analyzing data

For the initial measurement, the organization conducts an annual quantitative and qualitative analysis of data from factors 1 and 2 that compares results against goals.

For remeasurement, the organization conducts a quantitative analysis, and conducts a qualitative analysis, if its stated goals are not met.

Factors 4–7: Identifying and selecting opportunities

The organization identifies multiple areas, or measures, for improvement, based on its analysis and selects four opportunities for improvement.

NCQA considers each area, or measure, identified for improvement as one opportunity, and counts only one opportunity per measure for factors 4–7. Opportunities may address areas, or measures, related to transitions or settings and are not required to address both.

Because opportunities for improvement can change with each analysis, the organization may present different opportunities for each annual analysis.

Exceptions

None.

Related information

Collaboration through patient-centered medical home (PCMH) initiative. The use of a medical home initiative meets one opportunity for Element A if:

- The initiative is a direct result of the data collected and the analysis performed to meet factors 1–3.
- The organization provides evidence of active support for the PCMH model during the previous 12 months.

NCQA defines “active support” as any of the following:

- Helping with application fees for NCQA PCMH Recognition (beyond the NCQA program’s sponsor discount).
- Helping practices transform into a medical home.
- Providing other incentives for NCQA PCMH Recognition, such as pay-for-performance.
- Using NCQA PCMH Recognition as a criterion for inclusion in a restricted or tiered network.
- Reporting recognition status in the physician directory.

The organization may receive credit for a second or third opportunity if it can provide evidence of an analysis to determine whether the medical home initiatives can meet additional opportunities.

Automatic credit does not apply if the organization uses a medical home initiative to meet the requirements.

Use of HEDIS measures. Organizations may use HEDIS results that address movement between practitioners or movement across settings to identify relevant clinical issues. Although a HEDIS measure may be relevant to both movement between practitioners or across settings, each HEDIS measure may result in only one opportunity for factors 4–7.

Note: *The use of HEDIS measure results alone may not meet the intent of the element because the results may not evaluate continuity and coordination of care between practitioners or across settings. However, measure results may be used to identify a continuity and coordination of care area, or measure that goes beyond HEDIS results. If a measure assesses one area of measurement, it counts as one measure; if it assesses more than one area of measurement, it may be counted as two or more measures.*

Examples**Inpatient settings**

- Hospital:
 - Medical.
 - Surgical.
 - Rehabilitation.
 - Labor and delivery.
 - Pediatrics.
 - Orthopedics.
- Skilled nursing.
- Extended care facility.
- Inpatient hospice.

Outpatient settings

- Rehabilitation center.
- Physician's office.
- Surgery center.
- Urgent care center.
- Emergency department.
- Hospice.
- Home health.

Factor 1: Movement between practitioners

- Specialist to primary care practitioner.
- Practitioner surveys about communication and coordination issues.
- Consult report completeness.
- Consult reports received.
- HEDIS performance measure data:
 - *Comprehensive Diabetes Care (CDC)*—Eye Exam rate.

Factor 2: Movement across settings

- Readmission data.
- Inpatient facility to primary care practitioners.
- ED to primary care practitioners.
- Urgent care center to primary care practitioners.
- Inpatient to nursing home or post-acute care setting nursing home.
- ED visit or post-discharge follow-up calls.
- Home health visit within seven days of inpatient discharge.
- HEDIS performance measure data:
 - *Persistence of Beta-Blocker Treatment After a Heart Attack (PBH)*.
 - *Plan All-Cause Readmissions (PCR)*.
 - *Prenatal and Postpartum Care (PPC)*—Postpartum rate.
 - *Use of Opioids From Multiple Providers (UOP)*.

Element B: Acting on Opportunities

The organization annually acts to improve coordination of medical care by:

1. Acting on the first opportunity for improvement identified in Element A.
2. Acting on the second opportunity for improvement identified in Element A.
3. Acting on the third opportunity for improvement identified in Element A.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 3 factors	No scoring option	The organization meets 0-2 factors

Data source Documented process, Reports, Materials

Scope of review *This element applies to Renewal Surveys.*
 NCQA reviews the organization's most recent and the previous year's reports.
 Depending on the action taken, NCQA also reviews a documented process or materials.

Look-back period *For Renewal Surveys:* 24 months.

Explanation **THIS IS A MUST-PASS ELEMENT.**
 Activities related to coordination between medical care and behavioral healthcare do not meet the intent of this element.

Factors 1–3

Actions are based on three distinct prioritized opportunities from Element A.

Exceptions

None.

Related information

Selecting actions. Actions are based on three distinct opportunities; however, it is possible that the same action may address more than one identified opportunity.

Collaboration through PCMH initiative. The use a medical home initiative meets one opportunity for Element B if:

- The initiative is a direct result of the data collected and the analysis performed to meet Element A, factors 1–3.
- The organization provides evidence of active support for the PCMH model during the previous 12 months.

NCQA defines “active support” as any of the following:

- Helping with application fees for NCQA PCMH Recognition (beyond the NCQA program's sponsor discount).
- Helping practices transform into a medical home.

- Providing other incentives for NCQA PCMH Recognition, such as pay-for-performance.
- Using NCQA PCMH Recognition as a criterion for inclusion in a restricted or tiered network.
- Reporting Recognition status in the physician directory.

The organization may receive credit for a second or third opportunity if it provides evidence of an analysis to determine whether the medical home initiatives can meet additional opportunities.

Automatic credit does not apply if the organization uses a medical home initiative to meet the requirements.

Examples

Taking action

- Prompt specialists to send summaries of recommendations to practitioners who provide primary care services.
- Educate inpatient discharge planners or home health agencies on the use of discharge instructions.
- Use an ED case manager to follow up with members after an ED visit.
- Provide online tool kits and communicate their availability to practitioners.

Element C: Measuring Effectiveness

The organization annually measures the effectiveness of improvement actions taken for:

1. The first opportunity.
2. The second opportunity.
3. The third opportunity.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 3 factors	No scoring option	The organization meets 0-2 factors

Data source Reports

Scope of review *This element applies to Renewal Surveys.*
NCQA reviews the organization's most recent and the previous year's measurement of effectiveness.

Look-back period *For Renewal Surveys: 24 months.*

Explanation **THIS IS A MUST-PASS ELEMENT.**

Measurement cycle

To meet each factor, the organization must measure the effectiveness of its actions (e.g., remeasure) twice, and analyze the results of each of those remeasurements. A baseline measurement, analysis and intervention must

precede the first remeasurement. Therefore, the factor requires two full cycles of measurement, analysis and intervention, followed by one additional remeasurement and analysis.

Measuring effectiveness

The organization uses defined variables to measure performance of identified issues and collects data on one of the following:

- Activities.
- Events.
- Occurrences.
- Outcomes.

Measures are based on standards of care or practice guidelines that include objective clinical criteria from authoritative sources, such as:

- Clinical literature.
- Consensus panels.
- HEDIS measures.
- Measures that are part of the organization's ongoing monitoring.

Measures may be designed for a focused study or for an activity targeted to improve a process of care.

Quantitative results

The organization performs a quantitative analysis using valid methodology, and defines the following for each measure:

- The numerator and denominator.
- The sampling methodology.
- The sample size calculation.
- The measurement periods and seasonality effects.

Qualitative analysis

The organization performs a causal analysis if stated goals are not met.

Exceptions

None.

Related information

Collaborative activities. The organization receives credit in Element C for use of a PCMH initiative, for the conditions for which it received credit in Elements A and B. Data collection, analysis and measure of effectiveness may occur in the aggregate at the collaborative group level under the following conditions:

- The organization is involved in the collaborative efforts.
- The collaborative measure is relevant to the organization's population.
- If a sample is used, it is representative of the collaborative organizations but is not required to be stratified for each organization. All eligible members from each participating organization are eligible for the sampling frame.
- Opportunities for improvement can be identified at the collaborative level for all participating organizations.

- Collaborative interventions are performed in a pilot program that is made available to the entire eligible population.

Intermediate measures. The organization may evaluate effectiveness of an intervention by using the same measure specification used for the initial measurement to remeasure, or by conducting an intermediate measurement. An intermediate measurement can evaluate processes or outcomes related to the intervention. For example, if the goal of the organization's intervention is to increase a screening rate by encouraging members to schedule a screening appointment, its intermediate measurement of effectiveness might be to measure the rate of member contacts that resulted in scheduled appointments. The organization might wait to assess the intervention's success until the next annual measurement cycle.

Examples None.

Element D: Transition to Other Care

The organization helps with members' transition to other care, if necessary:

1. When their benefits end.
2. During transition from pediatric care to adult care.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 2 factors	No scoring option	The organization meets 0-1 factors
Data source	Documented process, Reports, Materials, Records or files		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews policies and procedures, and reviews evidence that the organization helped members transition to other care, as needed, throughout the look-back period.		
Look-back period	<i>For Interim Surveys and First Surveys: 6 months.</i> <i>For Renewal Surveys: 24 months.</i>		
Explanation	THIS IS A MUST-PASS ELEMENT. Factor 1: Exhausted benefits If covered benefits are exhausted while a member needs care, the organization notifies the member about alternatives and resources for continuing care and how to obtain it, as appropriate. The organization is not required to develop alternative resources.		

Factor 2: Transition from pediatric care to adult care

The organization is not required to manage the transition from pediatric care to adult care for every member. The organization may develop criteria (e.g., age) for notification to individuals or their caretakers.

Exceptions

None.

Examples**Identifying members whose benefit ended**

The organization identifies qualified individuals using daily case manager reports or requests for extension of needed services that were denied due to benefit limitations.

Transition of adolescents

- The organization assists pregnant adolescents in their transition from pediatrics to an adult primary care practitioner, OB/GYN, family practitioner or internist.
- The organization periodically assesses its membership for members reaching adulthood and those who have not chosen an adult primary care practitioner, and helps them select an adult primary care practitioner.
- The organization identifies members reaching adulthood who have not chosen an adult practitioner and helps them select an adult primary care practitioner.
- The organization's member newsletter contains information on how members reaching adulthood can get help choosing an adult primary care practitioner.

MA 23: Continuity and Coordination Between Medical Care and Behavioral Healthcare

The organization collaborates with behavioral healthcare practitioners to monitor and improve coordination between medical care and behavioral healthcare.

Intent

The organization collaborates with behavioral healthcare practitioners and uses information at its disposal to coordinate medical care and behavioral healthcare.

Element A: Data Collection

The organization annually collects data about opportunities for collaboration between medical care and behavioral healthcare in the following areas:

1. Exchange of information.
2. Appropriate diagnosis, treatment and referral of behavioral disorders commonly seen in primary care.
3. Appropriate use of psychotropic medications.
4. Management of treatment access and follow-up for members with coexisting medical and behavioral disorders.
5. Primary or secondary preventive behavioral healthcare program implementation.
6. Special needs of members with severe and persistent mental illness.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 5-6 factors	No scoring option	The organization meets 0-4 factors
Data source	Reports, Materials		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> <i>For Interim Surveys and First Surveys:</i> NCQA reviews the organization's evidence of collaboration (e.g., joint meeting minutes, communications) on data collection and most recent annual data collection report. <i>For Renewal Surveys:</i> NCQA reviews the organization's evidence of collaboration (e.g., joint meeting minutes, communications) on data collection and most recent and the previous year's annual data collection reports.		
Look-back period	<i>For Interim Surveys and First Surveys:</i> At least once during the prior year. <i>For Renewal Surveys:</i> 24 months.		
Explanation	THIS IS A MUST-PASS ELEMENT. The organization demonstrates collaboration between its behavioral healthcare delivery system and medical care delivery system.		

Collaboration with a managed behavioral healthcare organization (MBHO) for this element is not considered delegation.

Factor 1: Exchange of information

The organization collects data on exchange of information between behavioral healthcare and relevant medical delivery systems (e.g., medical/surgical specialists, organizational providers) measuring any or all of the following:

- Accuracy of information.
- Sufficiency of information.
- Timeliness of information.
- Clarity of information.
- Frequency of receiving information.

The organization meets the requirements of factor 1 if medical and behavioral healthcare practitioners can access each other's notes through a fully integrated EMR.

Factor 2: Diagnosis, treatment and referral of behavioral disorders

The organization collects data on:

- Behavioral disorders that may have been misdiagnosed or treated improperly, **or**
- Referrals that were unnecessary, too early, too late or to the incorrect type of behavioral healthcare practitioner.

Factor 3: Appropriate use of psychotropic medications

The organization collects data on behavioral and medical practitioner adherence to prescribing guidelines.

Collaborative efforts between the medical care and behavioral health care on improving the HEDIS measures *Antidepressant Medication Management (AMM)* or *Follow-Up Care for Children Prescribed ADHD Medication (ADD)* meets the requirements of this factor.

Factor 4: Management of coexisting medical and behavioral conditions

The organization collects data on issues around management of multiple conditions where there are both medical and behavioral health conditions and management across the continuum of care is an issue.

Factor 5: Prevention programs for behavioral healthcare

The organization collects data on issues that could be prevented if appropriate primary or secondary programs are developed and implemented. The organization identifies the programs that the collaboration deems most appropriate, but is not required to implement the program to meet the element.

Factor 6: Severe and persistent mental illness

The organization collects data on specific issues around the continuity and coordination of services for members with severe and persistent mental illness (SPMI).

Exceptions

This element is NA if all purchasers of the organization's services carve out or exclude behavioral healthcare.

Factor 6 is NA if the organization's membership does not include adult members.

Related information

Use of HEDIS measures. Organizations may use HEDIS results that address movement between practitioners or movement across settings to identify relevant clinical issues. Although a HEDIS measure may be relevant for more than one factor, the results of any HEDIS measure may be used for only one factor.

Note: *The use of HEDIS measure results alone may not meet the intent of the element because the results may not evaluate continuity and coordination of care between practitioners or across settings. However, measure results may be used to identify a continuity and coordination of care area, or measure that goes beyond HEDIS results. If a measure assesses one area of measurement, it counts as one measure; if it assesses more than one area of measurement, it may be counted as two or more measures.*

Examples**Factor 1: Exchange of information**

- Surveys of behavioral healthcare practitioners and other practitioners regarding information exchanged.
- Evaluation of solicited or unsolicited practitioner reports on communication between behavioral healthcare practitioners and medical practitioners, including protection of privacy.
- Evaluation of primary care practitioner medical records to determine if the practitioners receive behavioral healthcare practitioner feedback.

Factor 2: Diagnosis, treatment and referral

- Data on the use of primary care guidelines for treating or making referrals for treatment of problems such as eating disorders, depression, postpartum depression, substance abuse or attention deficit disorder.
- Results of the HEDIS measure *Antidepressant Medication Management (AMM)*.
- Results of the HEDIS measure *Follow-Up Care for Children Prescribed ADHD Medication (ADD)*.

Factor 3: Psychotropic medication use

- Analysis of pharmaceutical utilization data for appropriateness of a psychopharmacological medication.
- Evaluation of psychotropic medication utilization data on issues related to multiple-prescribing practitioners.
- Results of the HEDIS measure *Antidepressant Medication Management (AMM)*.
- Results of the HEDIS measure *Follow-Up Care for Children Prescribed ADHD Medication (ADD)*.
- Results of the organization's documented process for approving use of psychopharmacological medications in specific situations.
- Results of technology assessment to evaluate emerging psychopharmacological medications.

Factor 4: Managing coexisting conditions

- Data on the frequency of behavioral healthcare consultations for medical or surgical inpatients with secondary or tertiary mental health or substance abuse diagnoses.
- Pharmaceutical data on medication interactions to assess coordination of coexisting medical and behavioral problems.
- Results of the HEDIS measure *Cardiovascular Monitoring for People With Cardiovascular Disease and Schizophrenia (SMC)*.
- Results of the HEDIS measure *Diabetes Monitoring for People With Cardiovascular Diseases and Schizophrenia (SMD)*.
- Results of the HEDIS measure *Diabetes Screening for People With Schizophrenia or Bipolar Disorder Who Are Using Antipsychotic Medications (SSD)*.

Factor 5: Primary preventive programs

- Data on the need for a program on prevention of substance abuse.
- Data on the need for a program on parenting skills training.
- Data on the need for a program on bereavement counseling.
- Data on the need for a program on nutritional and body image for adolescents.
- Data on the need for a program on stress management for adults.

Factor 5: Secondary preventive programs

- Data on the need for developmental screening of children in primary care settings.
- Data on the need for ADHD screening of children in primary care settings.
- Data on the need for screening for eating disorders in adolescent females in primary care settings.
- Data on the need for behavioral health consultations for members hospitalized for targeted medical or surgical conditions that are known to be associated with behavioral complications or comorbidities.
- Data on the need for postpartum depression screening.

Factor 6: Severe and persistent mental illness

- Results of the HEDIS measure *Diabetes Screening for People With Schizophrenia or Bipolar Disorder Who Are Using Antipsychotic Medications (SSD)*.
- Results of HEDIS measure *Diabetes Monitoring for People With Diabetes and Schizophrenia (SMD)*.
- Results of the HEDIS measure *Metabolic Monitoring for Children and Adolescents on Antipsychotics (APM)*.
- Results of HEDIS measure *Cardiovascular Monitoring for People With Cardiovascular Disease and Schizophrenia (SMC)*.
- Data on frequency of assessment of substance use disorders.

Element B: Collaborative Activities

The organization annually conducts activities to improve the coordination of behavioral healthcare and general medical care, including:

1. Collaborating with behavioral healthcare practitioners.
2. Quantitative and causal analysis of data to identify improvement opportunities.
3. Identifying and selecting one opportunity for improvement from Element A.
4. Identifying and selecting a second opportunity for improvement from Element A.
5. Taking collaborative action to address one identified opportunity for improvement from Element A.
6. Taking collaborative action to address a second identified opportunity for improvement from Element A.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 5-6 factors	No scoring option	The organization meets 0-4 factors
Data source	Documented process, Reports, Materials		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> <i>For Interim Surveys and First Surveys:</i> NCQA reviews the organization's evidence of collaboration (e.g., joint meeting minutes, communications) on data collected in Element A, analyses, opportunities and actions. <i>For Renewal Surveys:</i> NCQA reviews the organization's most recent and the previous year's annual collaborations (e.g., joint meeting minutes, communications) on data collected in Element A, analyses, opportunities and actions. <i>For factors 5 and 6:</i> NCQA reviews reports. Depending on the action taken to address identified opportunities, NCQA may also review a documented process or materials.		
Look-back period	<i>For Interim Surveys and First Surveys:</i> At least once during the prior year. <i>For Renewal Surveys:</i> 24 months.		
Explanation	THIS IS A MUST-PASS ELEMENT. Factor 1: Collaboration The organization collaborates with behavioral healthcare practitioners to identify opportunities to improve continuity of care. Factor 2: Analysis The organization conducts a quantitative analysis of data that incorporates aggregate results and trends over time and compares results against a standard or goal. Tests of statistical significance are not required, but may be useful when analyzing trends. Results do not need to be trended for First Surveys.		

The organization collaborates on the quantitative results and conducts collaborative causal analysis if stated goals were not met.

Factors 3–6: Identifying, selecting and acting on opportunities

Working collaboratively, the organization identifies, selects and takes action on opportunities to improve coordination of care between medical care and behavioral healthcare.

The organization may identify multiple opportunities per issue and may identify different opportunities during each annual analysis of data.

Exceptions

Factors 2–6 are NA if all purchasers of the organization's services carve out or exclude behavioral healthcare. Even if behavioral healthcare is carved out or excluded, the organization must meet factor 1.

Related information

Collaboration through patient-centered medical home (PCMH) initiative. The use of a medical home initiative meets one opportunity for collaboration between medical care and behavioral healthcare in Element A if:

- The initiative is a direct result of the data collected and the analysis performed to meet factors 1–2.
- The organization provides evidence of active support for the PCMH model during the previous 12 months.

NCQA defines “active support” as any of the following:

- Helping with application fees for NCQA PCMH Recognition (beyond the NCQA program's sponsor discount).
- Helping practices transform into a medical home.
- Providing other incentives for NCQA PCMH Recognition, such as pay-for-performance.
- Using NCQA PCMH Recognition as a criterion for inclusion in a restricted or tiered network.
- Reporting Recognition status in the physician directory.

The organization can receive credit for a second opportunity if it can provide evidence of an analysis to determine whether the medical home initiatives can meet additional opportunities.

Automatic credit does not apply if the organization uses a medical home initiative to meet the requirements.

Examples**Opportunities for collaboration**

- Improve the process for members to authorize sharing of behavioral health information with primary care practitioners.
- Develop or implement primary care guidelines for assessing, treating and referring common behavioral problems.
- Increase nonbehavioral healthcare practitioner satisfaction with feedback from behavioral healthcare practitioners.
- Improve procedures for treating hospitalized members with coexisting medical and behavioral conditions.

- Improve identification and management of elderly members with multiple behavioral healthcare medications and potentially inappropriate dosages.
- Educate primary care practitioners about appropriate indications for referring patients with hyperactivity disorder or depression to behavioral healthcare specialists.
- For Medicaid products in states that carve out behavioral healthcare services, identify methods to improve dialogue between inpatient and aftercare providers.
- Provide tools to facilitate communication between a medical practitioner and the behavioral healthcare practitioner who is treating the medical practitioner's patient.
- Hold quarterly meetings with the behavioral healthcare organization to develop coordination activities.

Documenting collaboration

- QI Committee evaluation of collaborative activities.
- Participation of behavioral healthcare specialists on the QI Committee or on QI teams that report to the QI Committee.
- Joint meetings of organization and MBHO QI Committees.
- Exchange of ideas between the organization and the MBHO QI Committees (e.g., minutes, reports, questionnaires).

Activities for continuity and coordination of care

- Establish a system with behavioral healthcare practitioners for communicating members' prescribed medications to their primary care practitioner.
- Develop and implement a prevention program for a behavioral disorder commonly managed in a primary care setting.
- Place behavioral healthcare practitioners in high-volume or key primary-care settings.

Element C: Measuring Effectiveness

The organization annually measures the effectiveness of improvement actions taken for:

1. The first opportunity.
2. The second opportunity.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 2 factors	No scoring option	The organization meets 0-1 factors

Data source Reports

Scope of review *This element applies to Renewal Surveys.*

NCQA reviews the organization's most recent and the previous year's annual effectiveness report for two opportunities.

Look-back period *For Renewal Surveys: 24 months.*

Explanation **THIS IS A MUST-PASS ELEMENT.**

Measurement cycle

To meet each factor, the organization must measure the effectiveness of its actions (e.g., remeasure) twice, and analyze the results of each of those remeasurements. A baseline measurement, analysis and intervention must precede the first remeasurement; therefore, the factor requires two full cycles of measurement, analysis and intervention, followed by one additional remeasurement and analysis.

Measuring effectiveness

The organization uses specific indicators to measure performance of identified issues and collects data on one of the following:

- Activities.
- Events.
- Occurrences.
- Outcomes.

Measures are based on standards of care or practice guidelines that include objective clinical criteria from authoritative sources, such as:

- Clinical literature.
- Consensus panels.
- HEDIS measures.
- Measures that are part of the organization's ongoing monitoring.

Measures may be designed for a focused study or for an activity targeted to improve a process of care.

Quantitative analysis

The organization performs a quantitative analysis using valid methodology, and defines the following for each measure:

- The numerator and denominator.
- The sampling methodology.
- The sample size calculation.
- The measurement periods and seasonality effects.

Qualitative analysis

The organization performs a causal analysis if stated goals are not met.

Exception

This element is NA if all purchasers of the organization's services carve out or exclude behavioral healthcare.

Related information

Collaborative activities. Data collection, analysis and measurement of effectiveness can occur in the aggregate at the collaborative group level under the following conditions:

- The organization is involved in the collaborative efforts.
- The collaborative measure is relevant to the organization's population.
- Samples are representative of the collaborative organizations, but are not required to be stratified for each organization. All eligible members from each participating organization are eligible for the sampling frame.
- Opportunities for improvement can be identified at the collaborative level for all participating organizations.

The organization may:

- Use a collaborative-level goal or benchmark.
- Perform practitioner-focused interventions at the collaborative level.
- Perform member-focused interventions that include all eligible members.
- Perform collaborative interventions in a pilot program that is made available to the entire eligible population.

Intermediate measures. The organization may evaluate effectiveness of an intervention by using the same measure specification used for the initial measurement to remeasure, or by conducting an intermediate measurement. An intermediate measurement can evaluate processes or outcomes related to the intervention. For example, if the goal of the organization's intervention is to increase a screening rate by encouraging members to schedule a screening appointment, its intermediate measurement of effectiveness might be to measure the rate of member contacts that results in schedule appointments. The organization might wait to assess the intervention's success until the next annual measurement cycle.

Examples None.

MA 24: Utilization Program Structure

The organization's UM program has clearly defined structures and processes, and assigns responsibility to appropriate individuals.

Intent

The organization has a well-structured UM program and makes utilization decisions affecting the health care of members in a fair, impartial and consistent manner.

Element A: Written Program Description

The organization's UM program description includes the following:

1. A written description of the program structure.
2. The behavioral healthcare aspects of the program.
3. Involvement of a designated senior-level physician in UM program implementation.
4. Involvement of a designated behavioral healthcare practitioner in the implementation of the behavioral healthcare aspects of the UM program.
5. The program scope and process used to determine benefit coverage and medical necessity.
6. Information sources used to determine benefit coverage and medical necessity.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 5-6 factors	No scoring option	The organization meets 0-4 factors

Data source Documented process

Scope of review *This element applies to Interim Surveys, First Surveys and Renewal Surveys.*
NCQA reviews the organization's UM program description.

Look-back period *For Interim Surveys:* Prior to the survey date.
For First Surveys: 6 months.
For Renewal Surveys: 24 months.

Explanation **THIS IS A MUST-PASS ELEMENT.**

This element is a **structural requirement**. The organization must present its own documentation.

The UM program description is organized and written so that staff members and others can understand the program's structure, scope, processes and information sources used to make UM determinations.

Medical necessity review is a process to consider whether services that are covered only when medically necessary meet criteria for medical necessity and clinical appropriateness. A medical necessity review requires consideration of the

member's circumstances, relative to appropriate clinical criteria and the organization's policies.

NCQA's UM standards specify the steps in the medical necessity review. Medical necessity review requires that denial decisions be made only by an appropriate clinical professional as specified in NCQA standards.

Decisions about the following require medical necessity review:

- Covered medical benefits defined by the organization's Certificate of Coverage or Summary of Benefits.
- Preexisting conditions, when the member has creditable coverage and the organization has a policy to deny preexisting care or services.
- Care or services whose coverage depends on specific circumstances.
- Dental surgical procedures that occur within or adjacent to the oral cavity or sinuses and are covered under the member's medical benefits.
- Out-of-network services that are only covered in clinically appropriate situations.
- Prior authorizations for pharmaceuticals and pharmaceutical requests requiring prerequisite drug for a step therapy program.
- "Experimental" or "investigational" requests covered by the organization.

Decisions about the following do not require medical necessity review:

- Services in the member's benefits plan that are limited by number, duration or frequency.
- Extension of treatments beyond the specific limitations and restrictions imposed by the member's benefits plan.
- Care or services whose coverage does not depend on any circumstances.
- Requests for personal care services, such as cooking, grooming, transportation, cleaning and assistance with other activities of daily living (ADL).
- "Experimental" or "investigational" requests that are always excluded and never deemed medically necessary under any circumstance. In these instances, the organization either:
 - Identifies the specific service or procedure excluded from the benefits plan,
or
 - If benefits plan materials include broad statements about exclusions but do not specify excluded services or procedures, the materials state that members have the opportunity to request information on excluded services or procedures and the organization maintains internal policies or criteria for these services or procedures.

Medical necessity review of requests for out-of-network coverage

Requests for coverage of out-of-network services that are only covered when medically necessary or in clinically appropriate situations require medical necessity review. Such requests indicate the member has a specific clinical need that the requestor believes cannot be met in-network (e.g., a service or procedure not provided in-network; delivery of services closer or sooner than provided or allowed by the organization's access or availability standards).

If the certificate of coverage or summary of benefits specifies that the organization never covers an out-of-network service for any reason or if the request does not indicate the member has a specific clinical need for which out-of-network coverage may be warranted, the request does not require medical necessity review.

Factor 1: Program structure

The written description includes the following information about the UM program structure:

- UM staff's assigned activities.
- UM staff who have the authority to deny coverage.
- Involvement of a designated physician and a designated behavioral healthcare practitioner.
- The process for evaluating, approving and revising the UM program, and the staff responsible for each step.
- The UM program's role in the QI program, including how the organization collects UM information and uses it for QI activities.
- The organization's process for handling appeals and making appeal determinations.

Staff size. NCQA does not prescribe staff size or a method or criteria for determining staff size.

Factor 2: Behavioral healthcare aspects of the program

The program description specifies how the organization addresses sites of behavioral healthcare services (e.g., psychology groups) and the levels of behavioral healthcare services (e.g., inpatient psychiatric care, outpatient psychiatrist visits.) If the organization has a process for triage and referral to behavioral health services, the program description specifies the process.

Factor 3: Senior-level physician involvement

The program description specifies how a senior-level physician (a medical director, associate medical director or equivalent) is involved in UM activities, including implementation, supervision, oversight and evaluation of the UM program.

Factor 4: Designated behavioral healthcare practitioner involvement

The program description specifies how a designated behavioral healthcare physician or a doctoral-level behavioral healthcare practitioner is involved in implementing and evaluating the behavioral health aspects of the UM program.

The behavioral healthcare practitioner must be a physician or have a clinical PhD or PsyD, and may be a medical director, clinical director, participating practitioner from the organization or behavioral healthcare delegate (if applicable).

Factors 5, 6: Processes and information sources used to make determinations

The program description specifies:

- The UM functions, the services covered by each function or protocol and the criteria used to determine medical necessity, including:
 - How the organization develops and selects criteria.
 - How the organization reviews, updates and modifies criteria.
- How medical necessity and benefits coverage for inpatient and outpatient services are determined.

- The description of the data and information the organization uses to make determinations (e.g., patient records, conversations with appropriate physicians) and guide the UM decision-making process.
 - The description should not be burdensome for the member, the practitioner or the health delivery organization's staff.
- The triage and referral process for behavioral healthcare services (if applicable).
- How sites of service and levels of care are evaluated for behavioral healthcare services (if applicable).

The program description lists the information (e.g., patient records, conversations with appropriate physicians) the organization uses to make UM determinations.

Exceptions

This element is NA if the organization and its delegates do not make UM decisions and all services are automatically approved.

Factors 2, 4 and behavioral healthcare aspects of factor 5 are NA if all purchasers of the organization's services carve out or exclude behavioral healthcare.

Related information

Benefit plan exceptions. If the organization authorizes a service, grants an extension of benefits or makes an exception to a limitation in the benefits plan (e.g., the organization is required to approve 20 visits but allows 21 visits), the granting of an exception does not set precedent such that a subsequent denial of the same service or a request for an extension or exception is considered a medical necessity determination.

Examples

Factor 3: Senior-level physician involvement

The senior-level physician's responsibilities may include, but are not limited to:

- Setting UM policies.
- Supervising program operations.
- Reviewing UM cases.
- Participating on the UM Committee.
- Evaluating the overall effectiveness of the UM program.

Factor 4: Behavioral healthcare practitioner involvement

The designated behavioral healthcare practitioner's responsibilities may include, but are not limited to:

- Setting UM behavioral healthcare policies.
- Reviewing UM behavioral healthcare cases.
- Participating on the UM Committee.

MA 25: Clinical Criteria for UM Decisions

The organization uses written criteria based on sound clinical evidence to make utilization decisions, and specifies procedures for appropriately apply the criteria.

Intent

The organization applies objective and evidence-based criteria and takes individual circumstances and the local delivery system into account when determining the medical appropriateness of health care services.

Element A: UM Criteria

The organization:

1. Has written UM decision-making criteria that are objective and based on medical evidence.
2. Has written policies for applying the criteria based on individual needs.
3. Has written policies for applying the criteria based on an assessment of the local delivery system.
4. Involves appropriate practitioners in developing, adopting and reviewing criteria.
5. Annually reviews the UM criteria and the procedures for applying them, and updates the criteria when appropriate.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 4-5 factors	No scoring option	The organization meets 0-3 factors
Data source	Documented process, Reports, Materials		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> <i>For Interim Surveys:</i> NCQA reviews the organization's policies and procedures for factors 1–5.		
	<i>For First Surveys and Renewal Surveys:</i> NCQA reviews the following: • <i>For factors 1–3:</i> Policies and procedures in place throughout the look-back period. • <i>For factor 4:</i> Meeting minutes or other reports documenting the involvement of appropriate practitioners throughout the look-back period. • <i>For factor 5:</i> The most recent annual review and update (for First Surveys) or most recent and previous year's annual reviews and updates (for Renewal Surveys).		
Look-back period	<i>For Interim Surveys:</i> Prior to the survey date.		
	<i>For First Surveys:</i> 6 months for factors 1–4; at least once during the prior year for factor 5.		
	<i>For Renewal Surveys:</i> 24 months.		

Explanation THIS IS A MUST-PASS ELEMENT.

This element is a **structural requirement**. The organization must present its own documentation.

Factor 1: Written UM decision-making criteria

The organization has specific criteria to evaluate the necessity of medical and behavioral healthcare services requiring approval. The organization may address factors 2 and 3 as part of the UM criteria, or in separate, overriding documented processes for staff (e.g., standing instructions for staff to use when determining whether UM guidelines are appropriate for a specific situation).

Factor 2: Consideration of individual needs

The organization considers at least the following individual characteristics when applying criteria:

- Age.
- Comorbidities.
- Complications.
- Progress of treatment.
- Psychosocial situation.
- Home environment, when applicable.

Factor 3: Assessment of the local delivery system

The organization's UM policies and procedures require consideration of available services in the local delivery system and their ability to meet the member's specific health care needs when UM criteria are applied.

Factor 4: Practitioner involvement

Practitioners with clinical expertise in the area being reviewed have the opportunity to advise or comment on development or adoption of UM criteria, and on instructions for applying criteria. Although the organization may use practitioners that are its staff, non-staff network practitioners must also be involved in developing, adopting and reviewing criteria, because they are subject to application of the criteria. If an organization has been unable to involve network practitioners, it must document its attempts and provide the documentation to NCQA during the survey.

The organization may have practitioners review criteria if it does not develop its own UM criteria and obtains criteria from external entities.

Factor 5: Reviewing and updating criteria

The organization reviews its UM criteria and procedures against current clinical and medical evidence and updates them, when appropriate. If new scientific evidence is not available, a designated group may determine if further review of a criterion is necessary.

Exceptions

This element is NA if the organization and its delegates do not make UM decisions and all services are automatically approved.

Factor 5 is NA for UM criteria in use for less than 12 months.

Related information

Factors 2, 3: Applying criteria. Nationally developed procedures for applying criteria, particularly those for length of hospital stay, are often designed for “uncomplicated” patients and for a comprehensive delivery system; they may not be appropriate for patients with complications or for a delivery system with insufficient alternatives to inpatient care. Written UM procedures direct decision makers to alternative procedures or approaches (e.g., a secondary set of UM criteria and individual case discussions) when assessment indicates that UM guidelines are not appropriate.

Examples**Factor 3: Assessment of the local delivery system**

Assessment of available services in the local delivery system and their ability to meet a member’s specific health care needs could include:

- Availability of inpatient outpatient and transitional facilities.
- Availability of outpatient services in lieu of inpatient services such as surgicenters vs. inpatient surgery.
- Availability of highly specialized services, such as transplant facilities or cancer centers.
- Availability of skilled nursing facilities, subacute care facilities or home care in the organization’s service area to support the patient after hospital discharge.
- Local hospitals’ ability to provide all recommended services within the estimated length of stay.

Factor 4: Practitioner involvement

The organization solicits opinions about the UM criteria through either of the following:

- Practitioner participation on a committee.
- Distributing the UM criteria to applicable practitioners.

In large regional or national organizations, a central office may develop or adopt criteria if practitioners with clinical expertise are involved in their development or adoption.

Special Needs Plans

Standards for Special Needs Plans

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SNP 1: Enrollment Verification*Refer to Appendix 1 for points*

The organization verifies enrollment verification for special needs plan members.

Intent

The organization verifies enrollment verification for Chronic Special Needs Plan, Institutional Special Needs Plan and Dual-Eligible Special Needs Plan members.

Element A: Enrollment Verification for Chronic Special Needs Plan (C-SNP) Members

The organization appropriately verifies eligibility by:

1. Documenting the length of the period of deemed continued eligibility.
2. Providing a screenshot of the enrollment effective date shown in the organization's internal systems and a key to interpreting the screenshot.
3. Documenting receipt of the enrollment request.
4. Documenting verification of SNP eligibility prior to submission of the enrollment to CMS.
5. Documenting the completed enrollment request.
6. Providing practitioner verification that the member has a qualifying condition.
7. Documenting the date verification was received.
8. Providing documentation that the Pre-enrollment Qualification Assessment Tool was used to establish eligibility.

Summary of Changes

- No changes to this element.

Scoring

Met	Partially Met	Not Met
The file meets 8 factors	No scoring option	The file meets 0-7 factors

Data source

Records or files, Model of Care

Scope of review

This element applies to Interim Surveys, First Surveys and Renewal Surveys.

Where the MOC is silent (e.g., no factor in the MOC), surveyors will default to the requirements described in the regulation.

NCQA reviews the organization's records or files.

NCQA selects files of members who have been continuously enrolled for at least 13 months.

NCQA will review 30 files that reflect the general composition of the organization's membership. A minimum of five members will be selected from each SNP type (C-SNP, I-SNP, D-SNP). If there are less than five files in the file review universe for a specific SNP type, then all are included in the file review universe. The remaining number of files in the file review universe will be added from the SNP type with the greatest representation.

NCQA may take an organization's MOC into consideration in the selection of the file review universe (i.e., MOC with the highest enrollment for each SNP type). NCQA will consider the time frame for which the MOC is being evaluated. If there

are multiple approved MOC's, each will be reviewed according to the criteria approved with relevance to the specific document in effect.

Each file is scored on each factor. The total number of factors met provides the element score of Met or Not Met.

In order to achieve a score of Met, each file must meet all 8 factors.

Look-back period *For All Surveys:* 13 months.

Explanation **THIS IS A MUST-PASS ELEMENT.**

Factors 1–8: Enrollment Verification for C-SNP

NCQA will conduct a file review that includes files with missing or late HRAs or ICP dates, files that have experience transitions of care and files that have different types of clinical coordination activity.

NCQA will identify up-to-5 of the most common MOCs (MOCS with the highest enrollment) for each SNP type (to the extent possible) and then sample proportionally and with regard to clinical considerations.

Exception

This element is NA for I-SNP and D-SNP members.

Examples None.

Element B: Enrollment Verification for Institutional Special Needs Plan (I-SNP) Members

The organization appropriately verifies eligibility by:

1. Documenting the length of the period of deemed continued eligibility.
2. Providing a screenshot of the enrollment effective date shown in the organization's internal systems and a key to interpreting the screenshot.
3. Documenting receipt of the enrollment request.
4. Documenting verification of SNP eligibility prior to submission of the enrollment to CMS.
5. Documenting the completed enrollment request.
6. Confirming whether a member requires care from a skilled nursing facility, a nursing facility, an intermediate care facility for members with intellectual disabilities or an inpatient psychiatric facility.
7. Confirming receipt of the state-approved Level of Care Assessment, if community-based members are enrolled in the I-SNP.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The file meets 7 factors	No scoring option	The file meets 0-6 factors

Data source Records or files, Model of Care

Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> Where the MOC is silent (e.g., no factor in the MOC), surveyors will default to the requirements described in the regulation. NCQA reviews the organization's records or files. NCQA selects files of members who have been continuously enrolled for at least 13 months. NCQA will review 30 files that reflect the general composition of the organization's membership. A minimum of five files will be selected from each SNP type (C-SNP, I-SNP, D-SNP). If there are less than five members in the file review universe for a specific SNP type, then all are included in the file review universe. The remaining number of files in the file review universe will be added from the SNP type with the greatest representation. NCQA may take an organization's MOC into consideration in the selection of the file review universe (i.e., MOC with the highest enrollment for each SNP type). NCQA will consider the time frame for which the MOC is being evaluated. If there are multiple approved MOC's, each will be reviewed according to the criteria approved with relevance to the specific document in effect. Each file is scored on each factor. The total number of factors met provides the element score of Met or Not Met. In order to achieve a score of Met, each file must meet all 7 factors.
Look-back period	<i>For All Surveys: 13 months.</i>
Explanation	THIS IS A MUST-PASS ELEMENT. <i>Factors 1–7: Enrollment Verification for I-SNP</i> The organization confirms that there has been a need for an institutional level of care for 90 calendar days or longer. If the organization opts to enroll special-needs individuals before a 90-calendar-day length of stay, a needs assessment demonstrates that the member's condition makes it likely that the length of stay (or need for an institutional level of care) will be at least 90 calendar days. Exception This element is NA for C-SNP and D-SNP members.
Examples	None.

Element C: Enrollment Verification for Dual-Eligible Special Needs Plan (D-SNP) Members

The organization appropriately verifies and re-verifies eligibility by:

1. Documenting the length of the period of deemed continued eligibility.
2. Providing a screenshot of the enrollment effective date shown in the organization's internal systems and a key to interpreting the screenshot.
3. Documenting receipt of the enrollment request.
4. Documenting verification of SNP eligibility prior to submission of the enrollment to CMS.
5. Documenting the completed enrollment request.
6. Documenting Medicare and Medicaid eligibility prior to enrollment.
7. Documenting member attestation of eligibility for the election period submitted.
8. Documenting ongoing verification of SNP eligibility.

Summary of Changes

- No changes to this element.

	Met	Partially Met	Not Met
Scoring	The file meets 8 factors	No scoring option	The file meets 0-7 factors
Data source	Records or files, Model of Care		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> Where the MOC is silent (e.g., no factor in the MOC), surveyors will default to the requirements described in the regulation. NCQA selects files of members who have been continuously enrolled for at least 13 months. NCQA will review 30 files that reflect the general composition of the organization's membership. A minimum of five members will be selected from each SNP type (C-SNP, I-SNP, D-SNP). If there are less than five files in the file review universe for a specific SNP type, then all are included in the file review universe. The remaining number of files in the file review universe will be added from the SNP type with the greatest representation. NCQA may take an organization's MOC into consideration in the selection of the file review universe (i.e., MOC with the highest enrollment for each SNP type). NCQA will consider the time frame for which the MOC is being evaluated. If there are multiple approved MOC's, each will be reviewed according to the criteria approved with relevance to the specific document in effect. Each file is scored on each factor. The total number of factors met provides the element score of Met or Not Met. In order to achieve a score of Met, each file must meet all 8 factors.		
Look-back period	For All Surveys: 13 months.		

Explanation	THIS IS A MUST-PASS ELEMENT. <i>Factors 1–8: Enrollment Verification for D-SNP</i> No additional explanation needed. Exception This element is NA for I-SNP and C-SNP members.
Examples	None.

SNP 2: Assessing and Coordinating Care

The organization assesses and coordinates care for members.

Intent

The organization assesses and coordinates care by administering a comprehensive assessment, developing individual care plans and developing appropriate interdisciplinary care teams for members.

Element A: Health Risk Assessment (HRA)

For each member, the organization:

1. Completes an HRA that includes a comprehensive initial assessment and reassessment of the medical, psychosocial, cognitive, functional and mental health needs of the member.
2. Conducts the initial HRA 90 days before or after the enrollment effective date.
3. Conducts the annual HRA within 1 year of the initial assessment/previous HRA using a comprehensive risk assessment tool.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The file meets 3 factors	No scoring option	The file meets 0-2 factors
Data source	Records or files, Model of Care		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> Where the MOC is silent (e.g., no factor in the MOC), surveyors will default to the requirements described in the regulation. NCQA reviews the HRA and any other evidence that demonstrates that the organization completed a comprehensive initial assessment and reassessment for members, as applicable. NCQA also reviews the organization's MOC. If there is more than one approved MOC that covers a member during the review period, NCQA will review both MOCs. For factor 1, NCQA will review member files for completeness of the element for the applicable time within the 13-month look-back period. NCQA will consider the time frame for which the MOC is being evaluated. If there are multiple approved MOC's, each will be reviewed according to the criteria approved with relevance to the specific document in effect. NCQA reviews all applicable SNP files (total number of files from SNP 1, Elements A–C, Enrollment Verification). Each file is scored on each factor. The total number of factors met provides the element score of Met or Not Met. In order to achieve a score of Met, each file must meet all 3 factors.		

Look-back period	<i>For factor 1:</i> 13 months. <i>For factors 2, 3:</i> Up to 16 months, depending on enrollment date. Members must be continuously enrolled for at least 90 days and the organization must conduct the HRA within 90 days, either before or after the effective date of enrollment. <i>Please see explanation of factors 2 and 3 regarding timeliness tests.</i>
Explanation	THIS IS A MUST-PASS ELEMENT. The organization conducts an annual health risk assessment using a comprehensive health risk assessment tool. Factor 1: Health risk assessment NCQA reviews the same 30 SNP files for the Enrollment Verification standard for proper administration of the HRA. Factors 2, 3: Conducting the HRA Timeliness must be evaluated across all applicable members, and not the 30 files reviewed for factor 1. If there is no initial health risk assessment (IHRA) occurring within 90 days (before or after) of the effective enrollment date, the organization is required to complete a HRA as soon as possible. If the HRA is conducted beyond the 90 days following enrollment, the HRA will be considered a reassessment (AHRA). <i>For initial HRAs:</i> NCQA conducts a timeliness test at the universe level of members who have been continuously enrolled for at least 90 days, to determine whether the organization conducted a timely IHRA within 90 days (before or after) the effective date of enrollment. <i>For annual HRAs:</i> NCQA conducts timeliness test at the universe level of members who have either been continuously enrolled for 365 days or more, or new members who missed the deadline to complete an initial HRA, to determine whether the organization conducted timely annual health re-assessment HRAs (AHRAs). NCQA will take into account outreach attempts, as specified in approved MOCs for each member. If timeliness across all applicable members, including the consideration of outreach attempts, is greater than 90% for both factor 2 and factor 3, these factors are considered met for a given contract. If the timeliness percentage, including the consideration of outreach attempts, is below 90%, NCQA will request a CAP and subsequently validate that the CAP has been effective in getting the organization to the 90% threshold or higher, including consideration of outreach attempts.
	Exceptions None.
Examples	None.

Element B: Individual Care Plan (ICP)

For each member's ICP, the organization:

1. Completes the ICP in accordance with its Model of Care (MOC).
2. Develops a comprehensive ICP that addresses the needs of the member identified in the HRA.
3. Identifies goals and objectives that include measurable outcomes designed to meet the needs of the member, including access to services and benefits, identified in the HRA.
4. Reviews and revises the ICP, based on the member's health condition.
5. Provides documentation to demonstrate implementation of the ICP, such as proof of claims or documentation of social services received.
6. Facilitates member/caregiver participation in developing the ICP.
7. Coordinates communication among members/caregivers and practitioners.

Summary of Changes

- No changes to this element.

Scoring	Met The file meets 7 factors	Partially Met No scoring option	Not Met The file meets 0-6 factors
Data source	Reports, Records or files, Model of Care		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> Where the MOC is silent (e.g., no factor in the MOC), surveyors will default to the requirements described in the regulation. NCQA reviews the organization's MOC. NCQA also reviews all applicable SNP files (total number of files from SNP 1, Elements A–C, Enrollment Verification). Documentation noted in the files may include: case management notes, ICT documentation, systems information such as utilization management, claims data, prescription drug events (PDE), document of social services received for each of the files to determine whether the organization implemented the ICP. Each file is scored on each factor. The total number of factors met provides the element score of Met or Not Met. In order to achieve a score of Met, each file must meet all 7 factors.		
Look-back period	<i>For All Surveys:</i> 13 months.		
Explanation	THIS IS A MUST-PASS ELEMENT. Factors 1–7: Individual care plan NCQA reviews for appropriateness and implementation of the ICP for the same 30 SNP files selected for the Enrollment Verification standard. The organization develops and implements model of care components that: • Coordinate the delivery of care across healthcare settings, providers, and services to assure continuity of care.		

- Coordinate the delivery of specialized benefits and services that meet the needs of the most vulnerable members (frail members, disabled members and members near the end of life).

The organization collects, analyzes and reports data that measure health outcomes and access to care based on care coordination indicators (e.g., service and benefit utilization rates, timeliness of referrals or treatment).

The organization develops and implements a comprehensive ICP through an interdisciplinary care team that works in collaboration with the member to identify goals and objectives. The organization includes measurable outcomes designed to meet the needs of members and works to improve health status by including measures from functional, psychosocial or clinical domains (e.g., quality of life indicators, depression scales or chronic disease outcomes).

The organization provides access to services and benefits that address the member's needs.

Individual care plans are reviewed and revised based on changes in the member's health condition.

Exceptions

None.

Examples None.

Element C: Interdisciplinary Care Team (ICT)

The organization developed an appropriate ICT by including practitioners and staff with appropriate clinical expertise, as defined in the MOC, to manage the ICP and address the member's health needs.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets the requirement	No scoring option	The organization does not meet the requirement

Data source Records or files, Model of Care

Scope of review *This element applies to Interim Surveys, First Surveys and Renewal Surveys.*

Where the MOC is silent (e.g., no factor in the MOC), surveyors will default to the requirements described in the regulation.

NCQA reviews the organization's MOC.

NCQA reviews all applicable SNP files (total number of files from SNP 1, Elements A–C, Enrollment Verification).

In order to achieve a score of Met, each file must meet the element.

Look-back period *For All Surveys: 13 months.*

Explanation THIS IS A MUST-PASS ELEMENT.

The organization uses evidence-based practices and nationally recognized clinical protocols in assessing and coordinating care for members.

NCQA reviews the MOC's definition of "appropriate clinical expertise" to ensure that the organization developed an ICT capable of addressing the member's health needs.

The organization uses integrated systems of communication within the interdisciplinary care teams to address member health needs, as evidenced by measures from the care coordination domain (e.g., call center utilization rates, rates of beneficiary involvement in care plan development).

Exceptions

None.

Examples None.**Element D: Care Transitions**

The organization provides continuity of care to members as defined in the MOC by:

1. Developing care transition protocols.
2. Implementing the care transition protocols, when necessary.

Summary of Changes

- No changes to this element.

Scoring

Met	Partially Met	Not Met
The organization meets 2 factors	No scoring option	The organization meets 0-1 factors

Data source Records or files, Model of Care

Scope of review

This element applies to Interim Surveys, First Surveys and Renewal Surveys.

Where the MOC is silent (e.g., no factor in the MOC), surveyors will default to the requirements described in the regulation.

NCQA reviews the organization's MOC.

NCQA also reviews all applicable SNP files (total number of files from SNP 1, Elements A–C, Enrollment Verification). Documentation noted in the files may include:

- Case management and/or ICT notes.
- Correspondence with the enrollee's PCP, specialists, hospital, skilled nursing staff, assisted living facility, etc.
- Discharge planning and/or care setting transition discussions held with the enrollee, the enrollee's caregiver or authorized representative.

Each file is scored on each factor. The total number of factors met provides the element score of Met or Not Met.

Look-back period	<i>For All Surveys:</i> 13 months.
Explanation	THIS IS A MUST-PASS ELEMENT. Factors 1–2: Care transitions No explanation required. Exception Factor 2 is NA if there are no care transitions to implement.
Examples	None.

Element E: Administrative Processes, Training and Credentialing

The organization has a well-defined process to select and evaluate personnel involved in member care coordination across healthcare settings, providers and services to assure continuity of care. The process requires the organization to:

1. Ensure that the staff who reviewed, analyzed and stratified the HRA possessed the appropriate professional knowledge and credentials, as defined in the MOC.
2. Review and confirm that the vendor's policies and procedures match the MOC goals and CMS requirements, when applicable.
3. Ensure that the staff who reviewed the ICP possessed the professional knowledge and credentials, as defined in the MOC.
4. Document all training received by ICT staff on the MOC.
5. Distribute information on MOC training to network practitioners.

Summary of Changes

- No changes to this element.

	Met	Partially Met	Not Met
Scoring	The organization meets 5 factors	No scoring option	The organization meets 0-4 factors

Data source Materials, Records or files, Model of Care

Scope of review *This element applies to Interim Surveys, First Surveys and Renewal Surveys.*

Where the MOC is silent (e.g., no factor in the MOC), surveyors will default to the requirements described in the regulation.

NCQA reviews the organization's MOC.

NCQA reviews all applicable SNP files (total number of files from SNP 1, Elements A–C, Enrollment Verification).

Each file is scored on each factor. The total number of factors met provides the element score of Met or Not Met.

In order to achieve a score of Met, each file must meet the element.

Look-back period *For All Surveys:* 13 months.

Explanation **THIS IS A MUST-PASS ELEMENT.**

The organization uses an interdisciplinary team in the care management of its members. The organization outlines its process for selecting and evaluating personnel involved in member care and coordination across all healthcare settings, practitioners, providers and services assures continuity of care.

Factor 1: Appropriate staff

NCQA reviews evidence that the credentials of the staff involved in the member's HRA are consistent with those outlined in the organization's MOC.

Factor 2: Vendor's policies and procedures

NCQA reviews the organization's MOC and the vendor's policies and procedures to confirm that the vendor follows the organization's goals and CMS's requirements.

Factor 3: Staff review of the ICP

NCQA reviews evidence that the credentials of the staff involved in the member's ICP are consistent with those outlined in the organization's MOC.

Factors 4, 5: ICT staff training and distribution of information

NCQA will review the information distributed during training and other outreach activities, as well as documentation of any training activities developed for network staff.

Exceptions

None.

Examples None.

SNP 3: Plan Performance Monitoring and Evaluation of the Model of Care (MOC)

Under its quality improvement program, the organization appropriately monitors and evaluates its MOC by collecting, analyzing and reporting data.

Intent

Under its quality improvement program, the organization monitors and evaluates their MOC by collecting data, evaluating performance and taking action on identified opportunities.

Element A: Monitoring and Evaluating the MOC

The organization appropriately monitors and evaluates its MOC by:

1. Collecting data and evaluating its MOC performance.
2. Analyzing the results of its performance evaluation to identify opportunities for improvement of the MOC.
3. Taking action on identified opportunities, if applicable.
4. Reporting and communicating performance monitoring results and improvements to stakeholders, in accordance with the MOC.
5. Including the appropriate personnel to oversee the MOC's evaluation and monitoring process.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 5 factors	No scoring option	The organization meets 0-4 factors
Data source	Reports, Materials, Model of Care		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> Where the MOC is silent (e.g., no factor in the MOC), surveyors will default to the requirements described in the regulation. NCQA reviews the organization's MOC to ensure its process for collecting, analyzing, utilizing the results to improve care and reporting are in alignment with its documented process. NCQA only evaluates MOCs in this element that were reviewed in SNP 1. NCQA also reviews evidence through the organization's reports and materials that the organization monitors and evaluates its MOC, analyzes results of its performance evaluation, acts on identified opportunities, communicates results to stakeholders and includes the appropriate personnel in the process of monitoring and evaluating.		
Look-back period	<i>For All Surveys:</i> 13 months.		

Explanation **THIS IS A MUST-PASS ELEMENT.**

The organization monitors and evaluates its MOC by collecting, monitoring and reporting data about the MOCs with the largest enrollment that were in effect for SNP 1 and SNP 2, one for each SNP type or the organization monitors and evaluates its MOCs for the three SNP types with the largest enrollment.

The MOCs that are evaluated should correspond to the Care Coordination review period.

Factors 1–4: Evaluating MOC performance

The organization reports and materials demonstrate how it measures the effectiveness of its model of care through the collection, aggregation, analysis, and reporting of data that demonstrates all of the following:

- Access to care as evidenced by measures from the care coordination domain (e.g., service and benefit utilization rates, or timeliness of referrals or treatment).
- Improvement in member health status as evidenced by measures from functional, psychosocial, or clinical domains (e.g., quality of life indicators, depression scales, or chronic disease outcomes).
- Staff implementation of the SNP model of care as evidenced by measures of care structure and process from the continuity of care domain (e.g., HEDIS measures or medication reconciliation associated with care setting transitions indicators).
- Comprehensive health risk assessment as evidenced by measures from the care coordination domain (e.g., accuracy of acuity stratification, safety indicators, or timeliness of initial assessments or annual reassessments).
- Implementation of an individualized plan of care as evidenced by measures from functional, psychosocial, or clinical domains (for example, rate of participation by Interdisciplinary Team (IDT) members and members in care planning).
- A practitioner network having targeted clinical expertise as evidenced by measures from medication management, disease management, or behavioral health domains.
- Delivery of services across the continuum of care.
- Delivery of extra services and benefits that meet the specialized needs of the most vulnerable members as evidenced by measures from the psychosocial, functional, and end-of-life domains.
- Use of evidence-based practices and nationally recognized clinical protocols.
- Use of integrated systems of communication as evidenced by measures from the care coordination domain (e.g., call center utilization rates, rates of member involvement in care plan development).

Factor 5: Including appropriate personnel

The organizational chart documents the organization's reporting structure and staff responsible for administering the MOC program.

Exceptions

None.

Examples None.

Element B: Reporting Health Outcome Data to CMS

Under its quality improvement program, the organization has a process for collecting, analyzing and reporting data to CMS that measures health outcomes and indices of quality for its targeted special needs population. Under its quality improvement program, the organization also has a process for making information on quality and outcome measures available to CMS that will:

1. Enable members to compare health coverage options.
2. Enable CMS to monitor the organization's model of care performance.

Summary of Changes

- No changes to this element.

Scoring	Met	Partially Met	Not Met
	The organization meets 2 factors	No scoring option	The organization meets 0-1 factors
Data source	Documented process		
Scope of review	<i>This element applies to Interim Surveys, First Surveys and Renewal Surveys.</i> NCQA reviews the organizations policies and procedures.		
Look-back period	For All Surveys: 6 months.		
Explanation	THIS IS A MUST-PASS ELEMENT. No explanation required.		
	Exceptions None.		
Examples	None.		

Appendix 1

Element Points for 2022

Appendix 2

Delegation and Automatic Credit Guidelines

APPENDIX 2

DELEGATION AND AUTOMATIC CREDIT GUIDELINES

Summary of Changes

- Clarified under “Activities That May Not Be Delegated” that factor 5 in ME 2, Element A may be delegated to an organization with NCQA MHC Distinction.
- Updated the “Vendor” section in “Special Situations” to clarify when a vendor relationship exists for applicable requirements.
- Added the UM/CR System Control requirements to the applicable automatic credit tables.
- Updated the footnote for UM 5, Element D in Table 3 to reflect the factor changes in this element.
- Updated Table 10 based on changes made to the NCQA MHC Distinction program.
- Removed factor 6 from PHM 3, Element A in Table 15 because it was retired from this element.
- Added MED 1, Element L and MED 9, Element D to the list of “Activities That May Not Be Delegated” under “Medicaid Module Survey: Delegation and Automatic Credit Guidelines.”
- Added the following elements to Table 19:
 - MED 1, Elements J and K.
 - MED 5, Element C.
 - MED 8, Elements D and E.
 - MED 9, Element E.

Definitions

authority	Power granted to a delegate to perform an activity under its own direction, within agreed-on parameters.
automatic credit	Scoring a requirement as “met” for delegating an eligible NCQA activity to an NCQA-Accredited, NCQA-Certified or NCQA-Recognized delegate. Specific criteria must be met to receive automatic credit.
carve out	A payer’s (e.g., employer, state Medicaid agency, federal Medicare agency) benefits plan excludes a health care program that focuses on a specific disease or service (e.g., behavioral healthcare or case management) and makes another entity responsible for running the program or offering the service.
de facto delegation	An entity performs an NCQA-required activity (e.g., UM functions) for an organization without a formal agreement between the entity and the organization.
responsible/ responsibility	The organization must meet NCQA standards for all NCQA-related activities, whether the organization performs the activities or whether they are performed by a delegate or subdelegate.

structural requirement	Essential program, process and procedural component of NCQA standards that the organization is required to meet.
subdelegation	A delegate gives a third entity the authority to carry out a delegated activity. For example, an organization delegates credentialing (CR) activities to a managed behavioral healthcare organization (MBHO), which then delegates CR verifications to a credentials verification organization (CVO). In this case, the CVO is the subdelegate.

About Delegation

Delegation occurs when an organization gives another entity the authority to perform an activity that the organization would otherwise perform to meet a requirement in the NCQA standards and guidelines. For example, organizations often delegate:

- Behavioral healthcare.
- Utilization management.
- Complex case management.
- Health promotion.
- Credentialing.

Medical groups, independent physician associations and provider organizations may seek to be delegates, to reduce redundancy of effort and oversight.

When delegation exists, NCQA holds the delegating organization responsible for delegated NCQA activities and requires the organization to oversee that the delegate performs the activities in accordance with NCQA requirements. For information on how NCQA evaluates an organization's oversight of delegation and the delegate's performance, refer to *How NCQA Evaluates Delegation*.

Activities That May Not Be Delegated

Delegation is not permitted for the following standards and elements:

- QI 5: Delegation of QI.¹
- PHM 3: Delivery System Supports, Element B.
- PHM 7: Delegation of PHM.¹
- NET 6: Delegation of NET.¹
- UM 13: Delegation of UM.¹
- CR 8: Delegation of CR.¹
- ME 1: Statement of Members' Rights and Responsibilities, Element A.
 - Authority for this activity may not be delegated and must be performed by the organization; however, an organization may adopt the rights and responsibilities statement of its customers or payers (e.g., Medicaid organization adopts statement required by the state).
- ME 2: Subscriber Information, Element A, factors 1–4 and 6–16.
 - Factor 5 may be delegated to an organization with NCQA MHC Distinction.
- ME 3: Marketing Information.
- ME 7: Member Experience, Element D.
- ME 8: Delegation of ME.¹

¹ Delegation oversight must be performed by the organization, but delegates may be given the authority to oversee subdelegates. This authority is specified in the delegation agreement. Refer to *Subdelegation*.

Structural Requirements

The organization is responsible for the following procedural or structural components of the standards and guidelines, even if it delegates the entire allowable function or activity:

- QI 1: Program Structure and Operations.²
- QI 2: Health Services Contracting.
- PHM 1: PHM Strategy, Element A.
- PHM 5: Complex Case Management, Element C.²
- PHM 6: Population Health Management Impact, Elements A and B.²
- NET 1: Availability of Practitioners:²
 - Element B, factors 1, 2.
 - Element C, factors 1–4.
 - Element D, factors 1–3.
- NET 2: Accessibility of Services, Element A and Element B.²
- UM 1: Program Structure.²
- UM 2: Clinical Criteria for UM Decisions, Element A.
- UM 4: Appropriate Professionals, Elements A and B.
- UM 8: Policies for Appeals.
- UM 12: UM System Controls.²
- CR 1: Credentialing Policies.²
- CR 6: Notification to Authorities and Practitioner Appeal Rights.
- ME 7: Member Experience, Elements A and B.

An organization that delegates a structural requirement must provide its own documentation (e.g., policies and procedures) as evidence that it meets the requirement, unless the requirement is eligible for automatic credit, as specified in Tables 2 and 3.

If the organization delegates a function or activity associated with a structural requirement, it must require its delegates to adhere to the same standard.

If the organization delegates all functions associated with a structural requirement, as an alternative to developing its own documented processes or materials, it may formally adopt the delegate's documented process or materials as its own through its governing body, another group or individuals with appropriate authority. The organization must provide evidence of formal adoption.

The organization must produce its own reports.

If a wholly owned corporate family member performs all functions associated with a structural requirement for the organization, the organization may provide the documentation of its corporate family member in lieu of providing its own documentation or formally adopting the documentation of the corporate family member.

²Elements or factors may be eligible for automatic credit. Refer to Tables 2 and 3.

How NCQA Evaluates Delegation

NCQA evaluates delegation in two ways:

1. Directly evaluates delegate performance in conducting the delegated functions.
2. Evaluates whether the organization conducts the required delegate oversight.

Direct evaluation of delegate performance

For delegated functions that are not structural requirements, NCQA evaluates the performance of a delegate directly, for applicable file review and non-file review elements.

NCQA may waive direct evaluation if the function is performed by an NCQA-Accredited/Certified or NCQA-Recognized organization.

File-review elements

If the organization delegates PHM, CR or UM functions that NCQA evaluates using file review, NCQA includes delegate files in the random sample of files selected for the file-review portion of the survey. The organization must have access to the delegate's files and must make them available to surveyors during the onsite survey.

Note: NCQA reserves the right to request site visits to delegates and conduct additional file review, and makes arrangements in advance of the organization's survey if such action is necessary.

Non-file review elements in QI, PHM, NET and ME

Because of the potential impact that delegation of key functions can have on members, NCQA directly evaluates the performance of the delegate against specified standards when a significant portion of the membership is affected.

NCQA evaluates applicable non-file-review elements for a sample of up to four delegates if the organization delegates the following functions to an organization that is not an MBHO or a PBM and if the delegated functions affect 50% or more of its membership:

- QI.
- PHM (with the exception of *PHM 5: Complex Case Management*).
- NET.
- ME.

When evaluating a delegate directly, NCQA evaluates the delegate's documentation against the standard. The organization must have access to the delegate's materials, processes and other data sources that are relevant to the delegated activity, and must submit the required documentation at the time of the survey in the appropriate non-file-review elements.

The score for the element is based on the average score of both the organization's and delegate's documentation.

First, IRT converts element scores to percentages as follows:

Table 1A: Converting an element score to a percentage

Element Score	Percentage
Met	100%
Partially Met	50%
Not Met	0%

Second, the average score is calculated and converted back to element scores, as indicated in the table below:

Table 1B: Converting an average score to an element score

Average Score	Element Score
100%	Met
At least 50% but <100%	Partially Met
Less than 50%	Not Met

Delegation oversight

If the organization delegates functions to another organization, NCQA evaluates the organization's oversight of the delegate under the delegation oversight standard of the applicable category. This is always the final standard in the category. The delegation oversight standard is worth an additional point in the category if it is applicable.

Refer to *Appendix 1: Element Points for 2022*.

Subdelegation

When a delegate subdelegates to a third entity, either the delegate or the organization oversees the subdelegate's work. The delegation agreement between the organization and the delegate specifies the entity responsible for overseeing subdelegates. If the delegate oversees the subdelegate, the delegate must report to the organization regarding the subdelegate's performance. NCQA confirms that oversight of the subdelegate is performed according to its standards. The organization is responsible for oversight of all activities performed by the delegate and subdelegate on its behalf.

Selecting delegates

For each standard category, NCQA selects and reviews the oversight of a sample of four delegates for all product lines and products. For organizations with fewer than four delegates, NCQA reviews all delegates.

NCQA includes delegates who have terminated the relationship with or had the relationship terminated by the organization in the scope of review for oversight if the relationship terminated 12 months or less before the date of an Accreditation Survey or Certification Survey.

NCQA evaluates delegation oversight during the offsite survey. The organization sends NCQA information about its delegates in a specified format five weeks before submission of the completed survey tool. NCQA selects delegates to review at random and, approximately four weeks prior to submission of the completed survey tool, notifies the organization which delegates have been selected.

For information on the required information and the timeline for submission, refer to the "Organization Background" section of the survey tool, under the Survey Resources and Delegation Information tabs.

De facto delegation

If NCQA identifies de facto delegation at any point after selecting the delegates (including during the offsite survey), NCQA reserves the right to review oversight of the de facto delegates by selecting them at random to include up to two of the four delegates reviewed.

Oversight review waived NCQA waives review of oversight for some corporate relationships (e.g., when the organization is owned by or under common control with another organization that performs the function) and for some delegated functions. Refer to *Special Situations*.

Special Situations

Vendors

NCQA has determined that for some elements, when another entity performs the function, it is not considered delegation. Instead, NCQA considers this to be a vendor relationship. In a vendor relationship, the organization does not give another entity the authority to carry out a function that it would otherwise perform. A vendor relationship is more similar to a purchase relationship, where the organization obtains a product, solution or system from the vendor and maintains control over the implementation and manner and use of the vendor's product, solution or system.

NCQA does not evaluate oversight conducted by the organization of a vendor.

Using another organization to provide the functions evaluated by the following elements is considered a vendor relationship.

- NET 5, Element I: Usability Testing.
- PHM 2: Population Identification, Elements A, B and D.
 - A vendor relationship exists if the organization delegates these functions to a NCQA-Prevalidated Health IT Solution that receives a designation of "Organization Support."
- PHM 4: Wellness and Prevention, Elements A, B.
- PHM 5, Element B: Case Management Systems.
 - A vendor relationship exists if the organization delegates these functions to an NCQA-Prevalidated Health IT Solution that receives a designation of "Organization Support."
- ME 1, Element B: Distribution of Rights Statement.
 - The organization must create the written information but may distribute it through a mail service organization.
- ME 2, Element A: Subscriber Information.
 - The organization must create the written information but may distribute it through a mail service organization.
- ME 4, Element A: Functionality—Website.
- ME 5, Element A: Pharmacy Benefit Information, factors 3–5.
 - A vendor relationship exists if the organization contracts with pharmacies and provides a link to the pharmacies' website.

Using another organization to provide the functions evaluated by any other elements is considered delegation, including the use of a mail service organization for other distribution requirements (e.g., UM denial letters; NET 4, Element A, Notification of Termination).

The organization submits—and NCQA evaluates—the vendor's documentation to determine if the elements are met. If the vendor that performs the activities is NCQA Accredited or NCQA Certified, the organization under review may be eligible to receive automatic credit for specified activities. Refer to *Delegating to NCQA-Accredited, NCQA-Certified or NCQA-Recognized Organizations*.

When the organization submits its completed survey tool, it must submit a written acknowledgment or other official document that is mutually agreed-upon between the organization and the vendor and specifies the responsibilities of each entity with respect to the requirements listed above. The written acknowledgment or official document is not required to be as detailed as the delegation agreement, but it must specify the responsibilities of the organization and the vendor.

State Medicaid Health Home Regulations

Several state Medicaid agencies have developed mandatory health home programs that organizations must support. Some state Medicaid agencies retain oversight of the health home; others require organizations to oversee the health home. NCQA standards are applied differently in these two cases.

If the state Medicaid agency oversees a health home, NCQA considers the relationship between the organization and the health home to be carved out and not delegation.

- The health home is not reviewed under *PHM 7: Delegation of PHM*, **and**
- Complex case management files managed by the health home are not included in file review.

If the state Medicaid agency requires the organization to oversee a health home, NCQA considers the relationship between the organization and the health home to be delegation.

- The organization receives delegation oversight relief in PHM 7 as specified in the “Practice Not NCQA Recognized” column in Table 13.
- The organization must include all health homes it oversees in the delegation worksheet.
- Complex case management files managed by the health home are included in the universe of files for random sample selection and review during a survey.

NCQA reviews the contract between the state Medicaid agency and the organization to determine whether the state oversees a health home (the health home is carved out to the state) or the state requires the organization to oversee the health home.

Organizations mandated by their state Medicaid agency to support a health home are responsible for all the following:

- Determining if the state oversees the health home or if the organization is responsible for overseeing the health home.
- Notifying NCQA about the state’s health home arrangement before the survey date.
- Providing NCQA with a copy of the contract between the organization and the state.

Corporate Families

Corporate families represent a unique relationship between organizations that perform activities on each other’s behalf within the purview of NCQA standards.

To determine if a corporate family relationship exists, NCQA ascertains if the organization being reviewed shares common ownership or control with the organization performing the activity. The concepts of common ownership and control are based on two models:

1. *For-profit model*: The parent organization controls its wholly owned subsidiaries.
2. *Not-for-profit model*: The not-for-profit organization controls related organizations through a membership agreement or other governance structure. The concept of “control” in the not-for-profit model is akin to the “wholly owned” concept in the for-profit model.

If an organization that shares common ownership or control with the organization under review also performs activities required by NCQA:

- NCQA evaluates a written acknowledgment or an official document from the organization specifying where another corporate family member performs functions evaluated by NCQA requirements.
- NCQA evaluates documentation (including procedural or structural components of the standards and guidelines) from the corporate family member that performs the activity.
- NCQA does not evaluate or require documentation of oversight of the affiliated organization.
- If the corporate family member that performs the activities is NCQA Accredited, NCQA Certified or NCQA Recognized, the organization under review may be eligible to receive automatic credit for specified activities. Refer to *Delegating to NCQA-Accredited, NCQA-Certified or NCQA-Recognized Organizations*.

If the entity performing the activities is affiliated with a corporate family, but is not wholly owned or controlled by the same entity that owns or controls the organization under review, NCQA considers this to be delegation and all requirements contained in this appendix, including oversight, apply.

Delegating to NCQA-Accredited, NCQA-Certified or NCQA-Recognized Organizations

When an organization delegates defined activities to an entity or uses a wholly owned family member, subdelegate or vendor that is NCQA Accredited, NCQA Certified or NCQA Recognized, the organization under review benefits from NCQA's previous review of the delegate/vendor, as follows:

- *Oversight relief.* The organization is not required to perform certain oversight activities and receives full credit for the activities in its survey.
- *Automatic credit.* The organization receives full credit for meeting an element or a portion thereof, based on the delegate/vendor's NCQA status. Refer to Tables 2–19 for elements and factors eligible for automatic credit and additional eligibility criteria.

Criteria for oversight relief and automatic credit

The following minimum oversight relief and automatic credit criteria must be met.

- A signed delegation agreement must be in effect before submission of the survey tool.
 - When an affiliated organization shares common ownership or control with the organization being reviewed, in lieu of a signed delegation agreement, NCQA accepts a written acknowledgment between the entities or an official document specifying the responsibilities of each entity, with respect to the function reviewed.
- The delegate is NCQA Accredited, NCQA Certified or NCQA Recognized for the function on or before the date of submission of the organization's survey tool. (Refer to the exception below for terminated NCQA-Accredited delegates).

Additional criteria for automatic credit

In addition to the minimum criteria above, the delegating organization must meet the following requirements to receive automatic credit:

- The delegate's Accreditation Survey or Certification Survey included the specific elements or factors (or their equivalent, depending on the product) for which the organization seeks automatic credit.

- Elements or factors scored NA during the delegate’s survey are not eligible for automatic credit, because the delegate does not perform the associated activity.
- The organization is responsible for determining if delegated activities are covered in the scope of the delegate’s NCQA review.
- The delegation agreement must describe the responsibilities of the organization and the delegated entity, as required by the delegation oversight standards. (Refer to Element A in the appropriate delegation standard.) NCQA bases the scope of automatic credit on the description of delegated activities.
- The organization includes the delegation agreement, the delegate’s Accreditation/Certification certificate and a completed delegation worksheet showing the number and percentage of members or practitioners and providers (as applicable) delegation covered by the delegation arrangement when the survey tool is submitted.
- For *non-file review elements*, the NCQA-Accredited, NCQA-Certified or NCQA-Recognized delegate must perform the delegated function for at least:
 - 70% of the organization’s membership, **or**
 - 70% of practitioners and/or providers for CR or PN delegation, as applicable.

If there are two or more delegates, “70%” is cumulative for the same delegated function.

If the organization has two or more product lines and manages them the same, 70% is cumulative across all product lines. If the organization manages the product lines differently (e.g., delegating the activity for the commercial product line and not for Medicaid or Medicare product lines), 70% is calculated by product line.

Note: The 70% threshold also applies to vendor relationships as defined above.

- For *file-review elements*, the NCQA-Accredited or NCQA-Certified delegate conducted the activities required by the element for the selected file.

File review If the organization delegates 100% of CCM, UM or CR (including primary source verification *and* decision making) activities to an NCQA-Accredited or NCQA-Certified delegate for the entire look-back period of the applicable requirement, NCQA does not require the organization to submit a file universe from which to sample files. Instead, the organization completes the “100% AC” tab of the File Submissions Instructions workbook.

If the organization delegates less than 100% of file review activities for any category to an NCQA-Accredited or NCQA-Certified delegate, the organization must include its files and its delegate’s files in the file review universe. NCQA gives automatic credit to delegated file-review elements for selected delegate files.

CR files	If the organization only delegates primary source verification (and not decision making) to an NCQA-Certified CVO or NCQA-Accredited delegate, NCQA scores all delegated file review elements as present for correct primary source verification and reviews the organization's files to determine whether time-sensitive elements meet the time limits.
Automatic credit and subdelegation	If the delegate subdelegates to a third party that is NCQA Accredited, NCQA Certified or NCQA Recognized, the organization receives oversight relief and is eligible for automatic credit if it meets all other automatic credit criteria.
Other delegation arrangements	When activities are performed by an NCQA-Accredited, NCQA-Certified or NCQA-Recognized delegate, the organization is eligible for automatic credit for applicable elements and factors under the following circumstances if it meets all other automatic credit criteria.
Terminated delegation arrangements	• For non-file review requirements, if delegation terminated more than 90 calendar days before submission of the completed survey tool, the organization is eligible for automatic credit for the portion of the look-back period when the NCQA-Accredited/Certified/Recognized delegate conducted activities. • For non-file review requirements, if delegation terminated 90 calendar days or less before submission of the completed survey tool, the organization is eligible for automatic credit for the entire look-back period. • For file review requirements, if delegated activities required by the element were conducted while the delegation arrangement was in place, the organization is eligible for automatic credit for these files.
Delegates whose status lapsed	NCQA only offers automatic credit for the period when the status was valid. The score is based on the average of the organization's score and the automatic credit provided for the period of the delegate's Accreditation, Certification or Recognition status.

Automatic Credit for Delegating to an NCQA-Accredited Health Plan

The organization is eligible for automatic credit if the delegate is NCQA Accredited:

- For all product lines, and is therefore eligible for automatic credit regardless of product line match, **or**
- In the same product line as the client organization.

The organization is eligible for automatic credit for CR files if all the following apply:

- The delegate has a single network of practitioners with centralized credentialing.
- One credentialing committee and the same staff handle credentialing for all practitioners.
- All practitioners' credentialing files were subject to CR file review during the delegate's NCQA survey.

The organization is eligible for automatic credit for assessing organizational providers if all the following apply:

- The organizational providers are in the delegate's network.
- The same staff handle assessment for all organizational providers.
- All organizational providers were subject to review during the delegate's NCQA survey.

Product line match. NCQA does not grant the organization automatic credit for delegation of functions for a product line that its delegate had at the time of its survey but for which it chose not to seek NCQA Accreditation, even if the delegate and the organization belong to the same wholly owned corporate family. Such product lines were not in the scope of the delegate's NCQA review and are therefore ineligible for automatic credit. Product line match does not apply to organizations only seeking Accreditation for Exchange product lines.

Note: Product line match exception. The organization is eligible for automatic credit if its Accredited delegate has a single practitioner network with centralized credentialing for all product/product lines, but is not Accredited for the delegated product line.

Key: Y = Automatic credit available

NA = Requirement does not apply to the Evaluation Option

Table 2: Automatic credit by Evaluation Option for delegating to an NCQA-Accredited health plan

Health Plan Standards and Elements		EVALUATION OPTION		
		Interim	First	Renewal
QUALITY MANAGEMENT AND IMPROVEMENT				
QI 3: Continuity and Coordination of Medical Care				
A	Identifying Opportunities	NA	Y	Y
B	Acting on Opportunities	NA	NA	Y
C	Measuring Effectiveness	NA	NA	Y
D	Transition to Other Care	NA	Y	Y
QI 4: Continuity and Coordination Between Medical Care and Behavioral Healthcare				
A	Data Collection	NA	Y	Y
B	Collaborative Activities	NA	Y	Y
C	Measuring Effectiveness	NA	NA	Y
POPULATION HEALTH MANAGEMENT				
PHM 1: PHM Strategy				
B	Informing Members	Y	Y	Y
PHM 2: Population Identification				
A	Data Integration ³	Y	Y	Y
B	Population Assessment	Y	Y	Y
C	Activities and Resources	Y	Y	Y
D	Segmentation ³	Y	Y	Y
PHM 3: Delivery System Supports				
A	Practitioner or Provider Support ³	Y	Y	Y

³ Automatic credit is available if the delegate is Accredited under the 2018 standards and beyond.

Health Plan Standards and Elements		EVALUATION OPTION		
		Interim	First	Renewal
PHM 4: Wellness and Prevention				
A	Frequency of Health Appraisal Completion	NA	Y	NA
B	Topics of Self-Management Tools	NA	Y	Y
PHM 5: Complex Case Management				
A	Access to Case Management	Y	Y	Y
B	Case Management Systems	Y	Y	Y
C	Case Management Process ⁴	Y	Y	NA
D	Initial Assessment	NA	Y	Y
E	Case Management—Ongoing Management	NA	Y	Y
PHM 6: Population Health Management Impact				
A	Measuring Effectiveness ^{3, 4}	NA	Y	Y
B	Improvement and Action ^{3, 4}	NA	NA	Y
NETWORK MANAGEMENT				
NET 1: Availability of Practitioners				
A	Cultural Needs and Preferences	NA	Y	Y
B	Practitioners Providing Primary Care ⁵	NA	Y	Y
C	Practitioners Providing Specialty Care ⁶	NA	Y	Y
D	Practitioners Providing Behavioral Healthcare ⁷	NA	Y	Y
NET 2: Accessibility of Services				
A	Access to Primary Care ⁸ <i>Data collection and analysis only</i>	NA	Y	Y
B	Access to Behavioral Healthcare ⁹	NA	Y	Y
C	Access to Specialty Care	NA	Y	Y
NET 3: Assessment of Network Adequacy				
A	Assessment of Member Experience Accessing the Network	NA	Y	Y
B	Opportunities to Improve Access to Nonbehavioral Healthcare Services	NA	Y	Y
C	Opportunities to Improve Access to Behavioral Healthcare Services	NA	Y	Y

⁴ For PHM 5, Element C and PHM 6 (structural requirements), if activities are delegated to an NCQA-Accredited health plan, the organization is not required to provide its own documentation.

⁵ For NET 1, Element B, factors 1 and 2 (structural requirements), if activities are delegated to an NCQA-Accredited health plan, the organization is not required to provide its own documentation.

⁶ For NET 1, Element C, factors 1–4 (structural requirements), if activities are delegated to an NCQA-Accredited health plan, the organization is not required to provide its own documentation.

⁷ For NET 1, Element D, factors 1–3 (structural requirements), if activities are delegated to an NCQA-Accredited health plan, the organization is not required to provide its own documentation.

⁸ For NET 2, Elements A and B (structural requirements), if activities are delegated to an NCQA-Accredited health plan, the organization is not required to provide its own documentation.

⁹ Automatic credit for this element applies only to Medicaid for Renewal Surveys. It is scored NA for commercial, Medicare and Exchange Renewal Surveys.

Health Plan Standards and Elements		EVALUATION OPTION		
		Interim	First	Renewal
NET 4: Continued Access to Care				
A	Notification of Termination	NA	Y	Y
B	Continued Access to Practitioners	NA	Y	Y
NET 5: Physician and Hospital Directories				
A	Physician Directory Data	NA	Y	Y ⁹
B	Physician Directory Updates	NA	Y	Y ⁹
C	Assessment of Physician Directory Accuracy	NA	Y	Y
D	Identifying and Acting on Opportunities	NA	Y	Y
E	Searchable Physician Web-Based Directory	NA	Y	Y
F	Hospital Directory Data	NA	Y	Y ⁹
G	Hospital Directory Updates	NA	Y	Y ⁹
H	Searchable Hospital Web-Based Directory	NA	Y	Y
I	Usability Testing	NA	Y	Y
J	Availability of Directories	NA	Y	Y
UTILIZATION MANAGEMENT				
UM 2: Clinical Criteria for UM Decisions				
B	Availability of Criteria	Y	Y	NA
C	Consistency in Applying Criteria	NA	Y	Y
UM 3: Communication Services				
A	Access to Staff	Y	Y	Y
UM 4: Appropriate Professionals				
C	Practitioner Review of Nonbehavioral Healthcare Denials	NA	Y	Y
D	Practitioner Review of Behavioral Healthcare Denials	NA	Y	Y
E	Practitioner Review of Pharmacy Denials	NA	Y	Y
F	Use of Board-Certified Consultants	Y	Y	Y
UM 5: Timeliness of UM Decisions				
A	Notification of Nonbehavioral Healthcare Decisions	NA	Y	Y
B	Notification of Behavioral Healthcare Decisions	NA	Y	Y
C	Notification of Pharmacy Decisions	NA	Y	Y
D	UM Timeliness Report	NA	Y	Y
E	Interim: Policies and Procedures	Y	NA	NA
UM 6: Clinical Information				
A	Relevant Information for Nonbehavioral Healthcare Decisions	NA	Y	Y
B	Relevant Information for Behavioral Healthcare Decisions	NA	Y	Y
C	Relevant Information for Pharmacy Decisions	NA	Y	Y

Health Plan Standards and Elements		EVALUATION OPTION		
		Interim	First	Renewal
UM 7: Denial Notices				
A	Discussing a Denial With a Nonbehavioral Healthcare Reviewer	NA	Y	Y
B	Written Notification of Nonbehavioral Healthcare Denials	NA	Y	Y
C	Written Notification of Nonbehavioral Healthcare Appeal Rights/Process	NA	Y	Y
D	Discussing a Behavioral Healthcare Denial With a Reviewer	NA	Y	Y
E	Written Notification of Behavioral Healthcare Denials	NA	Y	Y
F	Written Notification of Behavioral Healthcare Appeal Rights/Process	NA	Y	Y
G	Discussing a Pharmacy Denial With a Reviewer	NA	Y	Y
H	Written Notification of Pharmacy Denials	NA	Y	Y
I	Written Notification of Pharmacy Appeals Rights/Process	NA	Y	Y
UM 9: Appropriate Handling of Appeals				
A	Preservice and Postservice Appeals	NA	Y	Y
B	Timeliness of the Appeal Process	NA	Y	Y
C	Appeal Reviewers	NA	Y	Y
D	Notification of Appeal Decision/Rights	NA	Y	Y
E	Final Internal and External Appeal Files	NA	Y	Y
F	Appeals Overturned by the IRO	NA	Y	Y
UM 10: Evaluation of New Technology				
A	Written Process	Y	Y	Y
B	Description of the Evaluation Process	Y	Y	Y
UM 11: Procedures for Pharmaceutical Management				
A	Pharmaceutical Management Procedures	Y	Y	Y
B	Pharmaceutical Restrictions/Preferences	Y	Y	Y
C	Pharmaceutical Patient Safety Issues	Y	Y	Y
D	Reviewing and Updating Procedures	Y	Y	Y
E	Considering Exceptions	Y	Y	Y
UM 12: UM System Controls				
A	UM Denial System Controls ¹⁰	Y	Y	Y
B	UM Denial System Controls Oversight ¹¹	NA	Y	Y
C	UM Appeal System Controls ¹⁰	Y	Y	Y
D	UM Appeal System Controls Oversight ¹¹	NA	Y	Y

¹⁰ Automatic credit is available if the delegate is Accredited under the 2020 standards and beyond.¹¹ Automatic credit is available if the delegate is Accredited under the 2022 standards.

Health Plan Standards and Elements		EVALUATION OPTION		
		Interim	First	Renewal
CREDENTIALING AND RECREDENTIALING				
CR 1: Credentialing Policies				
C	Credentialing System Controls ¹⁰	Y	Y	Y
D	Credentialing System Controls Oversight ¹¹	NA	Y	Y
CR 2: Credentialing Committee				
A	Credentialing Committee	Y	Y	Y
CR 3: Credentialing Verification				
A	Verification of Credentials	NA	Y	Y
B	Sanction Information	NA	Y	Y
C	Credentialing Application	NA	Y	Y
CR 4: Recredentialing Cycle Length				
A	Recredentialing Cycle Length	NA	NA	Y
CR 5: Ongoing Monitoring and Interventions				
A	Ongoing Monitoring and Interventions	NA	Y	Y
CR 7: Assessment of Organizational Providers				
A	Review and Approval of Provider	Y	Y	NA
B	Medical Providers	Y	Y	NA
C	Behavioral Healthcare Providers	Y	Y	NA
D	Assessing Medical Providers	NA	Y	Y
E	Assessing Behavioral Healthcare Providers	NA	Y	Y
MEMBER EXPERIENCE				
ME 1: Statement of Members' Rights and Responsibilities				
B	Distribution of Rights Statement	NA	Y	Y
ME 4: Functionality of Claims Processing				
A	Functionality—Website	NA	Y	NA
B	Functionality—Telephone Requests	NA	Y	NA
ME 5: Pharmacy Benefit Information				
A	Pharmacy Benefit Information—Website	NA	Y	NA
B	Pharmacy Benefits Information—Telephone	NA	Y	NA
C	QI Process on Accuracy of Information	NA	Y	Y
D	Pharmacy Benefit Updates	NA	Y	Y
ME 6: Personalized Information on Health Plan Services				
A	Functionality—Website	NA	Y	Y
B	Functionality—Telephone	NA	Y	Y
C	Quality and Accuracy of Information	NA	Y	Y
D	Email Response Evaluation	NA	Y	Y

Health Plan Standards and Elements		EVALUATION OPTION		
		Interim	First	Renewal
ME 7: Member Experience				
C	Annual Assessment of Nonbehavioral Healthcare Complaints and Appeals	NA	Y	Y
E	Annual Assessment of Behavioral Healthcare and Services	NA	Y	Y
F	Behavioral Healthcare Opportunities for Improvement	NA	NA	Y

Automatic Credit for Delegating to an NCQA-Accredited MBHO, NCQA-Accredited UM, CR or PN or NCQA-Certified CVO

For NCQA-Accredited MBHOs

The organization is eligible for automatic credit for behavioral requirements or the behavioral components of a requirement if the delegate is NCQA Accredited:

- For all product lines, and is therefore eligible for automatic credit regardless of product line match, **or**
- In the same product line as the client organization.

The organization is eligible for automatic credit for CR files if all the following apply:

- The delegate has a single network of practitioners with centralized credentialing.
- One credentialing committee and the same staff handle credentialing for all practitioners.
- All practitioners' credentialing files were subject to CR file review during the delegate's NCQA Survey.

The organization is eligible for automatic credit for assessing behavioral healthcare providers if all the following apply:

- The behavioral healthcare providers are in the delegate's network.
- The same staff handle assessment for all organizational providers.
- All behavioral healthcare providers were subject to review during the delegate's NCQA Survey.

Product line match. NCQA does not grant the organization automatic credit for delegation of functions for a product line that its delegate had at the time of its survey but for which it chose not to seek NCQA Accreditation, even if the delegate and the organization belong to the same wholly owned corporate family. Such product lines were not in the scope of the delegate's NCQA review and are therefore ineligible for automatic credit. Product line match does not apply to organizations only seeking Accreditation for Exchange product lines.

Note: Product line match exception. *The organization is eligible for automatic credit if its Accredited delegate has a single practitioner network with centralized credentialing for all product/product lines, but is not Accredited for the delegated product line.*

Appendix 3

Merger, Acquisition and Consolidation Policy

APPENDIX 3

MERGER, ACQUISITION AND CONSOLIDATION POLICY

Summary of Changes

- Revised text under “The MAC Policy” to clarify the purpose of the policy and when/how NCQA determines a MAC Survey is required.
- Clarified the definition of “New Entity.”
- Added a first and second bullet to the bulleted list under “Written Notice.”
- Revised the text under “MAC Evaluation and Outcomes” to clarify that NCQA may require a MAC Survey to evaluate changes in operational structure or to evaluate a New Entity after a merger.
- Added QI 1, Element E and the new UM/CR System Controls Oversight requirements to Table 1.
- Added MED 9, Element B and MED 10, Elements A–D to Table 3.

The MAC Policy

This Merger, Acquisition and Consolidation Policy (“MAC Policy”) applies to all health plans Accredited by NCQA. NCQA evaluates transactions involving Accredited organizations to determine if they impact the scope of review under NCQA’s Health Plan Accreditation standards. Mergers, acquisitions, consolidations and corporate reorganizations are treated the same under NCQA’s MAC Policy. The terms *merge*, *merged* and *merger* also refer to acquisitions, consolidations and reorganizations.

The MAC Policy distinguishes mergers among Accredited organizations only and those involving unaccredited organizations. For mergers among Accredited organizations, NCQA does not require a MAC Survey and the MAC Policy details how NCQA determines the impact, if any, on the Accredited organizations. NCQA may evaluate the Accredited organizations for continued compliance with the standards as a Discretionary Survey

A MAC Survey may be required when there is a merger among Accredited and unaccredited organizations. NCQA evaluates whether the merger results in changes to the operational structure or status of the Accredited organization. NCQA may require a MAC Survey to determine if there are changes in the Accreditable entity or a New Entity as a result of the merger. If required, a MAC Survey reviews a subset of the standards impacted by the merger. The MAC Policy also describes when a New Entity may be considered unaccredited and requires a Full Survey.

At its sole discretion, NCQA may amend this or any other NCQA Accreditation program policy with 90 calendar-days notice before implementation.

Submit questions about this NCQA MAC Policy at My NCQA (<https://my.ncqa.org>).

Definitions

Accreditable entity	The entity that is legally and financially accountable for care provided to members. NCQA considers the following factors, among others, when determining what constitutes an Accreditable entity: • Number of legal entities involved. • Number of distinct practitioner and provider networks. • Centralization of key functions. • Number of licenses.
acquisition	The purchase or other transfer of at least 50% ownership or control of a health plan, or the purchase or transfer of a health plan's assets, including its members, to another entity.
acquisition date	The closing date of the purchase or other transfer of at least 50% ownership or control of a health plan, or the purchase or transfer of a health plan's assets, including its members, to another entity.
complete consolidation	The combination of all functions addressed under NCQA standards, of two or more separate Accreditable entities owned by one legal entity. Complete consolidation may or may not result in one Accreditable entity. If it does not, a separate Accreditation decision is issued for each Accreditable entity.
consolidation	The complete or partial combination of functions addressed under NCQA standards, of two or more separate Accreditable entities owned by one legal entity.
consolidation date	The date when a consolidating entity or plan stops performing a function and the remaining consolidating entity or plan performs the function fully.
Discretionary Survey	A survey of an organization with a current Accreditation status, assessing continued compliance with standards in effect at the time of the organization's most recent Accreditation decision. A Discretionary Survey is not a MAC Survey.
expiration date	The date when the organization's Accreditation status expires.
First Evaluation Option	A survey for any organization new to NCQA Accreditation. First Evaluation Options include most elements; the organization must submit HEDIS and CAHPS measures in the next calendar year following the effective date of Accreditation status.
functions	Categories of operations addressed by NCQA's health plan standards (e.g., QI, UM).
Interim Evaluation Option	A survey for any organization new to NCQA Accreditation. The Interim Evaluation Option includes non-file review elements only. The organization must submit HEDIS and CAHPS measures in the next calendar year following the effective date of Accreditation status. The Interim Evaluation Option is valid for up to 18 months.

MAC Survey	A survey that NCQA may conduct within 6 months following a merger or complete consolidation, during which a subset of the health plan standards are reviewed.
merger	The combination of two or more formerly separate legal entities (e.g., two corporations) into one legal entity.
merger date	The date when two or more formerly separate legal entities become one legal entity.
New Entity	The new or surviving entity following a merger or complete consolidation. A New Entity may also result following a merger if there is a new Accreditable Entity or a change to the operations of an existing Accreditable Entity.
partial consolidation	The combination of some (but not all) functions addressed under NCQA standards, of two or more separate Accreditable entities owned by one legal entity. A partial consolidation does not change the number of Accreditable Entities or lead to a MAC Survey or a Full Survey, but it may result in further NCQA review.
reorganization	The process of reorganizing or altering the corporate structure of an organization, including the creation of a new organization or the dissolution of the organization as an entity. Filing for petition for bankruptcy or the initiation of receivership, liquidation or state insurance supervision should be reported to NCQA as Reportable Events under the NCQA Accreditation program policy, not under the MAC Policy.
reorganization date	The effective date of the new entity, dissolution or corporate restructuring plan.
Renewal Evaluation Option	Surveys for organizations that have a current Accreditation status other than Interim. In addition, organizations must submit HEDIS and CAHPS measures and be reevaluated on them during each year of Accreditation.
weighted average score	The weighted average of the scores of all Accredited organizations involved in a merger or complete consolidation, based on each entity's enrollment at the time.

Written Notice

Timing of written notice	An NCQA-Accredited organization involved in a merger, acquisition, consolidation or reorganization must submit written notice of such action to NCQA within 30 calendar days following the merger, acquisition, consolidation or reorganization date, or earlier, if possible. Submit notices electronically to SIG@ncqa.org or by fax to 202-955-3599 directed to the attention of the Structural Issues Group (SIG). Although written notice is required after a transaction, NCQA and the organization will discuss in advance the potential impact a pending transaction may have on Accreditation status and survey scheduling.
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Content of written notice

The organization must outline the nature of the transaction in the written notice (to the extent possible), including, but not limited to:

- Names of the legal entities involved in the transaction and their NCQA Accreditation status.
- Brief description of the transaction and available documentation, if any (e.g., press releases, Articles of Merger, regulatory filing).
- Number of members affected.
- Projected time frames for merger and consolidation activities.
- Service areas.
- Board structure.
- Licensure.
- Committee structures.
- Areas of consolidation.
- Significant structural or staffing changes.
- Combination of, or changes to, practitioner and provider networks.
- How products or product lines will be marketed (e.g., the HMO commercial product line will be marketed by the merged entity as one statewide product/product line under a common name, “ABC Behavioral Healthcare of New York”).
- For transaction details that are not final at the time of written notice to NCQA, the organization should provide preliminary expectations and a timeline that indicates when each item will be final.

MAC Evaluation and Outcomes

Some organizations may merge or completely consolidate with other organizations in stages over a period of months. Because of their complexity, NCQA examines these situations on a case-by-case basis. A look-back period of 6 months applies to each transaction, to determine if a serial merger or complete consolidation situation exists and to decide the type of survey that may be necessary.

Initial assessment

When NCQA receives notice of a merger, acquisition or consolidation (hereafter referred to as a *merger*), it evaluates the transaction’s impact on Accreditation status and score. Evaluation considers member data, existing Accreditation status level, expiration dates of status level and standard scores of Accredited organizations involved in the transaction.

Depending on the results of the evaluation, NCQA may not require a survey, may require a MAC Survey, or may require an Accreditation Survey similar to the organization’s original survey, if it determines the New Entity is unaccredited due to merger or consolidation.

When a MAC Survey is required

NCQA requires a MAC Survey when an Accredited organization (or organizations) merges with an unaccredited organization and the unaccredited organization (or organizations) contributes 50% or less to the total membership. Refer to *Unaccredited New Entity* for information on mergers that involve an unaccredited organization contributing more than 50%.

NCQA may require a MAC Survey to evaluate the changes in operational structure to the Accreditable entity, or to evaluate the New Entity as a result of the merger.

Survey timing:

- If required by NCQA, the Accreditable entity or New Entity must undergo a MAC Survey within 6 months of the merger date and must undergo a Full Survey under the Renewal Evaluation Option on or before the expiration date of the Accredited organization involved in the merger.
- NCQA waives the MAC Survey requirement if the Accredited organization's expiration date is 12 months or less from the merger date.

Accreditation status:

- When NCQA determines a MAC survey is required, the New Entity's Accreditation status is listed and noted as *Merger Review in Process until the survey is complete*.
- If only one Accredited organization is involved in the merger and the New Entity passes the MAC Survey (or if NCQA waives it), the new score and Accreditation status are based on the Accredited organization's score and status.
- A New Entity that fails the MAC Survey receives Denied Accreditation status.
- An organization that refuses to undergo a MAC Survey or a Full Survey required by NCQA loses its Accreditation status.

When a MAC Survey is not required

NCQA does not require a MAC Survey if the merger involves only NCQA-Accredited organizations; however, NCQA reserves the right to conduct a MAC Survey or require a Full Survey if the merger involves an Accredited health plan with Interim Accreditation status. The date of the next Full Survey is based on the larger Accredited organization's expiration date. The new score and Accreditation status are based on the largest organization's score and status. If a health plan with Interim Accreditation status is involved in the merger, NCQA does not consider its score and status.

Unaccredited New Entity

When an Accredited organization merges with an unaccredited organization or organizations and the unaccredited organization contributes more than 50% of the total merged membership, NCQA considers the result to be a New Entity that is unaccredited. In this situation the New Entity does not have an Accreditation status. If it applies for an Accreditation under the First Evaluation Option, its status is noted as *Scheduled*.

Scheduling and cost

NCQA may schedule a MAC Survey within 6 months after the merger or consolidation date, to assess if the Accreditable entity or New Entity is in compliance with a subset of health plan Accreditation standards (described below).

If NCQA determines that a MAC Survey is necessary, the survey includes all affected product lines and products.

The organization bears the cost of a MAC Survey in accordance with NCQA pricing policies in effect at the time of the survey.

MAC Survey Process**What the MAC Survey includes**

The MAC Survey includes a review of the organization's description of changes to the Accreditable entity and/or the New Entity, including, if applicable, a timetable for reporting HEDIS results based on the merged/consolidated product lines and products of the New Entity.

For organizations Accredited under the First or Renewal Evaluation Option, the MAC Survey includes an onsite review for continued compliance with Accreditation standards and file review of the organization's processes for complex case management, UM and CR, including:

- Case management files.
- UM denials.
- UM appeals.
- Credentialing.

The MAC Survey uses standards in effect at the time of the merger or consolidation and the 8/30 file review process for applicable requirements. NCQA randomly selects files that reflect activities from the time of the merger or consolidation.

Refer to Tables 1–4 for standards reviewed during a MAC Survey.

Relevant documentation

The organization provides relevant documentation for the MAC Survey, including, but not limited to:

- QI program description.
- QI work plan.
- QI Committee structure.
- QI Committee meeting minutes.
- UM program description/policies.
- UM appeals policies and procedures.
- CR policies and procedures.
- CR Committee meeting minutes.
- Complex case management policies and procedures.
- HEDIS reports (Renewal Evaluation Options only).
- A description of HEDIS reporting systems (Renewal Evaluation Options only).

To demonstrate compliance with the standards, the New Entity may submit the following information:

- New documents showing policies for the merged health plan, **or**
- Premerger documents still in effect, **and**
- A plan for creating policies and procedures for the New Entity.

**HEDIS
submission**

For organizations Accredited under the Renewal Evaluation Option, before the New Entity's next survey, for the annual review and update of Accreditation status based on updated HEDIS results, the organization may either submit separate HEDIS results for all previously Accredited product lines and products of the New Entity, or combined HEDIS results.

At the New Entity's next Full Survey, it must submit HEDIS data reflecting care and service provided by the merged/completely consolidated product lines and products. Submission of combined HEDIS data is required only if the complete consolidation resulted in one Accreditable entity.

Scoring for MAC Survey

If a MAC Survey is required, NCQA uses a pass/fail scoring methodology. To pass, the New Entity must earn 80% of the applicable points for each standard category.

The use of quantitative ratings is an internal tool for making an Accreditation decision and is based on NCQA's professional judgment and on the organization's numerical score. The Review Oversight Committee (ROC) makes the final Accreditation determination.

Table 1: 2022 Health Plan Standards for MAC Survey

2022 Standards/Elements	
QUALITY MANAGEMENT AND IMPROVEMENT	
QI 1: Program Structure and Operations	
A	QI Program Structure ⁰¹
B	Annual Work Plan
C	Annual Evaluation
D	QI Committee Responsibilities
E	Promoting Organizational Diversity, Equity and Inclusion
POPULATION HEALTH MANAGEMENT	
PHM 5: Complex Case Management	
B	Case Management Systems
C	Case Management Process ¹
D	Initial Assessment
E	Case Management—Ongoing Management
UTILIZATION MANAGEMENT	
UM 1: Program Structure	
A	Written Program Description ¹
B	Annual Evaluation
UM 2: Clinical Criteria for UM Decisions	
UM 4: Appropriate Professionals	
A	Licensed Health Professionals ¹
B	Use of Practitioners for UM Decisions ¹
C	Practitioner Review of Nonbehavioral Healthcare Denials
D	Practitioner Review of Behavioral Healthcare Denials
E	Practitioner Review of Pharmacy Denials
F	Use of Board-Certified Consultants
UM 5: Timeliness of UM Decisions	
UM 6: Clinical Information	
UM 7: Denial Notices	
UM 8: Policies for Appeals¹	
UM 9: Appropriate Handling of Appeals	
UM 12: UM System Controls	
CREDENTIALING AND RECREDENTIALING	
CR 1: Credentialing Policies	
A	Practitioner Credentialing Guidelines ¹

¹ Optional standards/elements. The New Entity may forego review on these requirements. In this case, the remaining standards reviewed carry proportionately more weight in determining the overall outcome of the MAC Survey.

2022 Standards/Elements	
B	Practitioner Rights ¹
C	Credentialing System Controls
D	Credentialing System Controls Oversight
CR 2: Credentialing Committee	
CR 3: Credentialing Verification	
CR 4: Recredentialing Cycle Length	
CR 5: Ongoing Monitoring and Interventions	

Table 2: 2022 LTSS Distinction Standards for MAC Survey

2022 Standards/Elements	
LONG-TERM SERVICES AND SUPPORTS	
LTSS 1: Core Features	
A	Program Description
B	Assessment of Health, Functioning and Communication Needs
C	Resource Assessment
D	Comprehensive Assessment Implementation
E	Person-Centered Assessments
F	Person-Centered Care Planning Process
G	Implementing the Care Planning Process
LTSS 3: Care Transitions	
A	Process for Transitions of Care

Table 3: 2022 Medicaid Standards for MAC Survey

2022 Standards/Elements	
MEDICAID	
MED 3: Practitioner Office Site Quality	
MED 8: Informing Members of Services	
D	Informing Members About the QI Program
MED 9: UM Decisions About Payment and Services	
B	UM Denial Notifications
D	Coverage of Emergency and Post-Stabilization Services
E	Affirmative Statement About Incentives
MED 10: Grievances and Appeals	
A	Policies for Grievances and Appeals
B	Distribution of Information About the Grievance and Appeal Process
C	Grievance and Appeal Records
D	Actions After an Appeal is Overturned

Table 4: 2022 MA Special Needs Plans Standards for MAC Survey

2022 Standards/Elements
SPECIAL NEEDS PLANS
SNP 1: Enrollment Verification
SNP 2: Assessing and Coordinating Care
SNP 3: Plan Performance Monitoring and Evaluation of the Model of Care (MOC)

Appendix 4

Answers to Commonly Asked Questions

APPENDIX 4

ANSWERS TO COMMONLY ASKED QUESTIONS

Policies and Procedures

For non-file review elements that require completion of annual activities within a look-back period of two years, how many annual cycles must an organization undergoing a Renewal Survey complete?

Organizations must complete required annual activities at least twice (two cycles) within the 2 years before submission. A 2-month grace period applies to annual activities. For an Initial Survey, NCQA looks for completion of one annual cycle within the 12-month look-back period.

Are electronic signatures acceptable for review and approval of minutes and policies and procedures? Must organizations also maintain a handwritten signature page, or will an electronic signature suffice?

An electronic signature is acceptable if the organization demonstrates use of appropriate controls to ensure that only the indicated individual could enter the signature. Organizations are not required to maintain a handwritten signature page in addition to the electronic signature.

How does NCQA handle a situation where a plan sponsor or account takes responsibility under federal law and is therefore accountable for an NCQA-required activity?

NCQA's definition of delegation is when an organization gives the authority to another organization to perform an activity that it would otherwise perform. This is not the case when a purchaser takes responsibility for a required activity, and therefore it is not delegation.

If an NCQA requirement conflicts with a state or federal requirement, the state or federal requirement supersedes the NCQA requirement. If an organization would violate federal requirements if it did not allow the purchaser to take responsibility for an NCQA-required function, NCQA holds the organization harmless for those functions and scores the relevant elements, or parts of elements, NA.

If the organization's purchasers do not take responsibility for required activities, the organization is expected to meet NCQA requirements for the activities. The organization must provide appropriate documentation in the Survey Tool about activities for which a purchaser is responsible.

Interim Surveys

How many sets of minutes are required for Interim Surveys? Many organizations seeking Interim Accreditation will barely be able to have one or two meetings before survey, especially new health plans, like co-ops.

If there were fewer than three meetings, the organization presents the minutes from the meetings it held.

For organizations coming through Interim Surveys, what documentation is expected to meet the delegation requirements?

For Interim Evaluation Option, if the delegation agreement is not final, NCQA accepts draft agreements and communications between organizations. NCQA reviews agreements and communications stating when the agreement will be complete for Element A (Delegation Agreement) in the applicable categories (QI, PHM, NET, UM, CR, ME), and reviews documented processes and reports for Element B (Predelegation Evaluation).

Delegates may provide documentation to support NCQA's review.

NCQA reserves the right to review and score completed delegation agreements during the length of Accreditation.

If “materials” are a data source, must they be distributed for Interim Surveys?

No. Materials must be created and approved, but are not required to have been distributed.

Is a medical record review required for an Exchange product coming through an Interim Survey in Florida or Pennsylvania?

No. Because Exchanges come under federal statutes, which do not require medical record review, medical record review is not included in Interim Surveys in Florida and Pennsylvania.

What documentation and evidence are required for interpreter or bilingual services since there may be no membership? What is required as evidence?

If an organization's membership has a need for interpreter or bilingual services, the organization must offer them to its membership.

If an organization's membership has no need for these services, the element is NA.

If marketing materials will not be in final form at the time of an Interim Survey, what is required as evidence?

Materials that have been approved by the organization but are not in final layout form are acceptable to meet ME 3, Elements A and B.

Many organizations seeking Interim status will not have enrolled any members and therefore will not have distributed materials. ME 2 (Subscriber Information) requires materials that have been sent to members. What documentation is expected for Interim Surveys?

Organizations going through Interim Survey must present subscriber materials to meet the requirements, but materials are not required to have been distributed. Draft materials are appropriate if they are approved by the organization, but not by the regulator.

Given that the QI Committee may have done little other than review and approve policies and procedures, what is expected in the way of practitioner involvement?

Organizations must provide evidence that practitioners are involved in review and approval of procedures.

Delegation

The survey tool allows documentation for only four delegates. How should an organization with more than four delegates populate the delegation standards?

A delegation worksheet in the “Background” section of the survey tool allows organizations to provide information on all of its delegates. Because the worksheet is an Excel document, rows can be added to accommodate all delegates for every delegated activity. The delegation selection process begins before the IRT Survey Tool is submitted. The organization completes the delegation worksheet and returns it to NCQA five weeks before submission. If the organization has more than four delegates, NCQA selects a random sample of four delegates and informs the organization four weeks before submission of the IRT Tool.

When populating the delegation standards in the survey tool for submission, organizations perform a readiness evaluation of delegation elements, and include supporting documentation for the delegation oversight elements when it submits the Survey Tool. Those elements will be reviewed by the surveyors during the offsite survey.

If an organization delegates a portion of the activities assessed in the UM file review to an Accredited delegate (e.g., gathering information for nonurgent preservice and postservice decisions, or appeal processing but not decision making or notification of the resolution of denials or appeals), does NCQA score any delegated file review element as present?

It depends. If NCQA determines that functions performed by the delegate meet the criteria for automatic credit (i.e., they are specifically referenced in the delegation agreement and the functions were included in the scope of the delegate’s Accreditation survey), the organization may be eligible to receive automatic credit for selected elements or factors in the denial and appeal file review.

Does NCQA review delegation oversight if an organization terminates a delegated relationship six months before its scheduled survey?

Yes. NCQA’s review of delegation oversight includes active delegated relationships and relationships terminated 12 months or less before the date of an Accreditation/Certification survey. NCQA does not review delegation oversight of an organization whose delegation relationships have been terminated for more than 12 months before submission.

Must UM guidelines be consistent across delegates?

Yes. Organizations must ensure consistent application of medical necessity decision making against criteria. One way to do this is to require all delegates to use the same UM criteria; however, this may not be feasible, especially in an environment in which delegates work with multiple organizations that require different criteria. In this situation, organizations should ensure that UM criteria selected by the medical group or IPA are, at a minimum, based on sound clinical evidence, are updated periodically and are applied consistently.

What types of PBM organization arrangements does NCQA review during a survey?

NCQA focuses on the organization’s oversight of the PBM and reviews pharmacy denials included in a random sample of UM denials.

May a national corporate office perform credentialing oversight of a nationally contracted delegate on behalf of its local organizations?

Yes. Oversight results of such oversight must be available at each local organization. The sample selection for the credentialing file audit must be random and is not required to include a proportionate number of files for each local organization, unless there are differences in how local providers are credentialed.

The underlying rationale is that the process is tested randomly so that results are equally valid for all parties using the process. Conversely, if the delegate's credentialing system is not centralized, separate oversight audits should be conducted for each local organization.

What does NCQA require when an organization rescinds CR delegation?

NCQA requires an unbroken string of recredentialing at least every three years. If an organization can obtain files from its delegate, it is not required to start over with initial credentialing; it may simply continue the process that the delegate began and recredential practitioners when they are due.

If the organization cannot obtain files from the delegate, it must perform initial credentialing within six months of the delegate's termination date. The organization is responsible for ensuring that credentialing occurs according to NCQA standards.

If an organization establishes a new contract with a physician organization (PO) that has up-to-date credentialing using NCQA standards, must it also credential the physician organization's practitioners?

The answer depends on the delegation scenario. If an organization delegates both data collection and decision making to the PO, the credentialing cycle belongs to the PO, not to the organization. The PO does not have to recredential its practitioners before the normal three-year cycle to accommodate a newly signed contract and delegation agreement.

If the organization only delegates data collection to the PO and the organization makes the credentialing decision, then the credentialing cycle belongs to the organization. The organization must perform initial credentialing of the PO's practitioners.

If a delegate does not have the same credentialing cycle as an organization, may the organization adopt the delegate's cycle?

Yes. If the delegate made credentialing decisions and practitioners were recredentialed every three years, and if the organization can obtain the credentialing files from the delegate, it may adopt the delegate's cycle to meet NCQA standards.

Does NCQA consider it delegation if several Accredited organizations collaborate with another Accredited organization or vendor to conduct credentialing oversight audits, using an agreed-upon data collection tool, and if each organization has access to the information?

No. NCQA encourages organizations to collaborate and share data. Because the Accredited organization or vendor is using a tool approved by a group of organizations and does not make decisions for them, NCQA does not consider this to be delegation and it does not require a delegation agreement. NCQA supports sharing of data among Accredited organizations. However, each organization must review the results independently to determine if a delegate's performants meets the requirements of the organization's agreement with the delegate.

Are organizations penalized for deficiencies or noncompliance resulting from an uncooperative delegate?

Yes. Before delegation, organizations must ensure that all necessary precautions (e.g., access to files during the survey; meeting NCQA standards) are outlined in a mutually agreed-upon and signed document. Because organizations enter into delegation arrangements voluntarily, they are held accountable for meeting all NCQA standards, regardless of circumstances.

May some elements be performed by another entity without meeting the delegation requirements?

Yes. NET 5, Element I; PHM 4, Elements A–B; and ME 4, Element A may be performed by another entity without meeting the delegation requirements.

General**For elements that require information to be distributed to members or practitioners (e.g., ME 2, Element A), how detailed must the notice of availability of information on the organization’s website be?**

The notice is not required to include the posted information verbatim, but must include a description specific enough to give readers a clear idea of the topic and the general content, and must include a link to the information. The organization may group or summarize the information by theme. For example, for ME 2, Element A, the notification may state: “We have posted information on our website [URL] that will help you understand the benefits and services covered in your benefits plan; plan restrictions; how to obtain health care services and medications; how to file a claim for payment (if needed); how submit a complaint; how to appeal an adverse decision; and how [organization] evaluates new technology.”

Population Health Management**May follow-up prompts be scheduled in a case manager’s electronic calendar?**

Yes, if the case manager’s schedule can be accessed by other case managers. “Prompts” for follow-up must meet the organization’s process and the specific case management plan. The organization must demonstrate prompts to NCQA during file review.

Utilization Management**How does NCQA handle the DNOD/Grijalva Fast-Track appeal process during an Accreditation review?**

The DNOD/Grijalva Fast-Track Appeal process is not similar to NCQA’s current denial and appeal file review processes; therefore, the denial and appeal file review does not include DNOD denials or Grijalva Fast-Track Appeals for the Medicare product line. These files should not be included in the random sample selected for file review.

Many denials involve providers, as well as practitioners and members. Are organizations required to notify providers of denials?

No. NCQA does not require organizations to send providers a copy of the denial letter.

May organizations develop their own UM decision-making criteria? How should criteria be referenced?

Yes, an organization may develop evidence-based, UM decision-making criteria. The denial notice must reference the specific criterion for the condition or procedure requested. A generic reference such as “our internal UM criteria” does not meet the intent of UM 7, Element B, factor 2, but a specific reference, such as “our Internal Criteria for Gallbladder Removal” satisfies the requirement.

Are Medicare Part D pharmacy denials and appeals included in the universe of UM files?

Yes. For Medicare Advantage (MA) plans, Part D denials and appeals are included in the universe of files eligible for review and are handled the same as other pharmaceutical decisions.

Are vision and chiropractic files included in the UM denial/appeal file review?

Yes. NCQA includes vision and chiropractic files in the UM denial/appeal file review for care that can be covered under an organization’s medical benefits.

Credentialing

Does NCQA require verification of education and training for a previously board-certified practitioner upon expiration of board certification?

No. NCQA does not require further verification of education and training for a previously board-certified practitioner, even upon expiration of board certification (i.e., the organization need not verify residency training or graduation from medical school). If a previously verified board certification used to verify education and training expires, the organization is not required to re-verify education and training.

May the organization exclude employee files from file review samples?

Yes. An organization that exercises this option must do so uniformly and consistently.

Should practitioners who are initially credentialed and recredentialed within the 36-month period before submission of the initial credentialing list, be placed on the list?

Yes. Practitioners who were initially credentialed within the 36-month period before submission of the list for sampling should be placed on the initial credentialing list, whether or not they have been recredentialed.

May an organization use a 24-month recredentialing cycle?

Yes. If an organization uses a 2-year (24-month) recredentialing cycle, or if the state mandates that it continues to use the 2-year cycle, NCQA applies the 36-month timeliness requirement for Accreditation purposes.

If an organization contracts with a health delivery organization outside the United States, how does it comply with the NCQA requirement for state or federal licensure?

If the health delivery organization is not under state or federal licensure systems, the organization makes its own determination about licensing, Accreditation and acceptability.

Does NCQA consider it an independent relationship if an organization contracts with a PPO network to provide health services to members who need care outside its service area and if it encourages members to obtain care from that network when they are outside the network?

Yes, if the organization includes information about the PPO network in member materials, or on an ID card that is used to direct members to the network (e.g., network name, phone number, logo) or if there are incentives for members to see the PPO's practitioners.

If the organization executes this type of contractual arrangement, it must credential those practitioners or delegate credentialing to the PPO network in order to meet the intent of NCQA credentialing standards.

What information must be verified for specialty training in order to ensure that member materials are consistent with information gathered during the credentialing process?

Organizations verify specialty training according to their credentialing policies and procedures, which must outline how the organization ensures that listings in member materials (and practitioner directories) are consistent with the credentialing information obtained, including for specialty training. NCQA reviews the organization's policies and procedures to determine whether they meet CR 1, Element A, factor 11.

Must paper credentialing files *and* electronic credentialing be maintained?

No. Maintaining one or the other is acceptable.

Population Health & Member Experience

Must an organization demonstrate compliance for certain PHM and ME standards/elements, even if services are not purchased by an employer group?

Yes. Organizations must demonstrate the functional capability to perform the following PHM and ME standards, even if the functions have not been purchased by an employer:

- PHM 4, Element A.
- ME 4, Elements A, B.
- ME 3, Elements A, B.
- ME 5, Elements A, B.

Must an organization demonstrate functional capability for performing a service, even if no clients have purchased the service?

Yes. If no employer or plan sponsor has purchased the services, organizations must demonstrate functional capability and may provide a “test” or “demo” ID and data for assessment of applicable ME requirements.

Appendix 5

Glossary

APPENDIX 5

GLOSSARY

Summary of Changes

- Replaced “centralized triage and referral” with “triage and referral.”

“6 months”	A 6-month period, with no grace period.
“12 months”	A 12-month period, with no grace period.
“24 months”	A 24-month period, with no grace period.
accessibility	The extent to which a patient can obtain available services when they are needed. <i>Services</i> refers to both telephone access and ease of scheduling an appointment, if applicable.
ACO	Accountable care organization. A group of health care providers who provide coordinated care and chronic disease management, with the goal of improving patient care, and whose payment is tied to achieving health care-quality goals and outcomes that result in lower costs.
active course of treatment	Treatment whose discontinuation could cause a recurrence or worsening of a condition under treatment and interfere with anticipated outcomes. Treatment typically involves regular practitioner visits to monitor the status of an illness or disorder, provide direct treatment, prescribe medication or other treatment or modify a treatment protocol.
adverse event	An injury that occurs in the course of a member receiving health care services from a practitioner.
affinity program	A program offered by the organization in collaboration with vendors, community programs or state/federal grant-funded programs, offering member discounts or other services not covered by the organization’s benefits plan.
after hours	The period after an organization’s stated hours of operation (i.e., after normal business hours, after office hours, after-hours care).
agent	An organization that contracts with a primary source or an NCQA-recognized source to provide credentialing information.
ambulatory	A setting where nonacute care is provided on an outpatient basis to patients who leave the setting on the same day they receive treatment (e.g., urgent care centers, walk-in clinics or outpatient surgery centers).
analysis	The process of conducting quantitative analysis <i>and</i> qualitative analysis, unless otherwise specified.
annual	A 12-month period, plus a 2-month grace period (12–14 months).

appeal	A request to change an adverse decision made by the organization. A member or the member's authorized representative may appeal an adverse decision.
ASO	Administrative services only. A contract between a managed care entity and a self-funded purchaser, where the managed care entity performs only administrative services (e.g., claims processing, actuarial analysis, utilization review).
attestation	A signed statement by a practitioner confirming the validity, correctness and completeness of a credentialing application.
authorized representative	An individual who acts on behalf of another individual through consent or under applicable law. An organization may establish procedures for determining whether an individual is authorized to act on behalf of a member. For urgent care decisions, an organization allows a health care practitioner with knowledge of the member's medical condition (e.g., a treating practitioner) to act as the authorized representative.
availability	The extent to which an organization geographically distributes practitioners of the appropriate type and number to meet the needs of its members.
behavioral change model	A method or process that supports members in accomplishing tangible beneficial health behavior changes.
behavioral healthcare	Evaluation and treatment of psychological and substance abuse disorders.
benchmark	For a particular indicator or performance goal, the industry measure of best performance for a process or outcome.
board certified	A practitioner has satisfied the requirements/standards of a nationally recognized specialty board and received the board's specialist certification.
board-certified consultant	A practitioner external to an organization who holds certification from an ABMS, AOA or other specialty board and acts in an advisory capacity to the organization.
CAHPS	Consumer Assessment of Healthcare Providers and Systems. A set of standardized surveys that measure patient satisfaction with the experience of care. CAHPS is sponsored by the Agency for Health Care Research and Quality (AHRQ).
care plan	Information about the member that facilitates communication, collaboration and continuity of care across settings when members experience transitions.
care setting	The provider or place where a member receives health care and health-related services. In any setting, a designated practitioner has ongoing responsibility for the member's medical care.

carve out	A payer's (e.g., employer, Medicaid, Medicare) exclusion of a health care program or service from an organization's benefits plan, making another entity responsible for the program or service.
case management	A collaborative process of assessment, planning, facilitation, care coordination, evaluation and advocacy for options and services to meet the comprehensive medical, behavioral health and psychosocial needs of an individual and the individual's family, while promoting quality, cost-effective outcomes.
case management plan	An individualized, person-centered plan to meet the member's needs. Case management plans may include prioritized goals; a reevaluation time frame; resources to be utilized, including the appropriate level of care; plans for continuity of care (e.g., transition of care, transfer between settings); or collaborative approaches (e.g., family participation).
category	A hierarchical, logical group of standards. The highest level of the hierarchy; within each category are standards, elements and factors.
children and adolescents	Persons 2–19 years of age.
chronic care	Management of diseases or conditions that are usually of slow progression and long continuance (e.g., hypertension, asthma, diabetes).
clean files	Credentialing files that meet the organization's criteria for participation and are not required to be sent to the credentialing committee for review.
clinical appropriateness	Based on judgment of a health care practitioner, applicability of a requested service to a member's case in terms of type, frequency, extent, site and duration. For example, a request to receive out-of-network services, based on a member's assertion that appropriate services are not available in network, requires clinical judgment to assess the clinical circumstances and determine if network practitioners have the required expertise. Whether services are medically necessary might not be in question.
clinical expertise	An individual with practical and theoretical knowledge gained through experience treating a specific condition or health issue.
clinical privileges	A practitioner is authorized by a health care facility to provide defined patient care services at the facility, based on the practitioner's license, education, training, experience, competence and ability.
closed formulary	A formulary structure in which some drugs are not covered. A <i>tiered formulary</i> is not a closed formulary.
community-based resource	Health care services or programs located where members live or work.

complaint	An expression of dissatisfaction with an aspect of the organization's operations or activities, including the actions of network providers and practitioners. <i>Note: NCQA's definition of complaint is inclusive of the CMS definition of "grievance."</i>
complex case management	A program of coordinated care and services for organization members who have experienced a critical event or diagnosis that requires extensive use of resources.
concurrent request	A request for coverage of medical care or services made while a member is in the process of receiving the requested medical care or services, even if the organization did not previously approve the earlier care.
confidentiality	Treatment of information that an individual has disclosed in a relationship of trust with the expectation that it will not be divulged to others without permission or in a manner inconsistent with the understanding of the original disclosure.
continuity of care	A process for ensuring that care is delivered seamlessly across a multitude of delivery sites and transitions in care throughout the course of a disease.
continuum	Health care services that expand across the entire delivery system and are available to members to meet their health care needs.
covered entity	According to HIPAA regulations, a health plan, health care clearinghouse or health care provider that transmits health information by electronic means, in connection with an electronic health care transaction.
CR	Credentialing. A process of reviewing and evaluating qualifications of licensed independent practitioners.
credentials	Documented evidence of professional licensure, education, certification or other qualifications required to perform certain health care roles.
critical incidents	Events or occurrences that cause harm to a member or LTSS provider or serve as indicators of risk to the health or welfare of the member or provider, such as abuse, neglect and exploitation.
critical incident management systems	Organizationwide systems that can be used to eliminate an immediate threat or to investigate allegations that a member or LTSS provider may be at risk of harm.
cultural competence	The ability of practitioners and systems to respect and respond to diverse member values, beliefs, behaviors and needs (e.g., social, cultural, linguistic) when providing health care services.
culture	The shared values, ideals and beliefs of a group of people.
customer services	Administrative systems that enroll members, provide information on using the organization's services, respond to member concerns and help members access clinical services.

CVO	Credentials verification organization. An organization that conducts primary-source verification of practitioner credentials for other organizations. The NCQA CVO Certification program evaluates CVO management of many aspects of its credentials verification operations, as well as the process it uses for continuous improvement of services.
data collection	The process of gathering and measuring information systematically, with the goal of answering stated research questions, testing hypotheses and evaluating outcomes.
de-identify	Removal of individual identifiers. Under the HIPAA Privacy Rule, protected health information is de-identified if all individual identifiers are removed. There are 18 categories of identifiers that include name; street address and ZIP code; telephone and fax number; dates (except year) directly related to a person, including date of birth and dates of service; email address and URL; Social Security Number; medical record number and account number; vehicle identifiers, including license plate number; device identifiers and serial number; and any other unique identifying number, characteristic or code.
decision variable	A principle, standard or specification that aids in decision making.
delegation	An organization gives another entity the authority to perform certain functions on its behalf. Although the organization may delegate the authority to perform a function, it may not delegate the responsibility for ensuring that the function is performed appropriately.
denial	Nonauthorization of coverage for care or services based on medical necessity, clinical appropriateness or benefit coverage. Partial approval and care termination when the practitioner does not agree are also considered denials.
digital services	Internet-based access and downloadable applications for smartphones and other devices.
disability	Physical or mental impairment that substantially limits one or more major life activities (e.g., seeing, hearing, speaking, walking, breathing, performing manual tasks, learning, self-care, working).
discharge planning	Comprehensive evaluation of a patient's health needs to arrange for appropriate care following discharge from an institutional clinical care setting.
DM	Disease management. A multidisciplinary, continuum-based approach to health care delivery that proactively identifies populations with, or at risk for, established medical conditions. Disease management supports the practitioner-patient relationship and plan of care, and emphasizes prevention of complications using cost-effective, evidence-based practice guidelines and patient empowerment strategies such as self-management.
DM program	An organization's disease-specific or condition-specific package of ongoing services and assistance that includes education and interventions.

documented process	Policies and procedures, process flow charts, protocols and other mechanisms that describe the methodology used to complete a task.
e-appointment	A clinician appointment made electronically through a secure site.
e-prescribing	A physician accesses the formulary electronically (to check for drug interactions, to print a prescription and, in some cases, to send a prescription directly to the pharmacy).
e-visit	A secure method of electronic/email communication between physicians and members after the first face-to-face visit has taken place.
element	The scored component of a standard that provides details about performance expectations. NCQA evaluates each element to determine the degree to which the organization meets the standard's requirements.
eligible members	Members who meet the criteria for participation in a benefit program or a contracted service.
EPO	Exclusive provider organization. Similar to HMOs in that members receive care from affiliated providers but do not need a referral to see a specialist in the EPO network. EPOs do not offer out-of-network benefits.
ethnicity	A shared culture and way of life, especially reflected in language, religion and material culture products and distinguishing from OMB's use of the term to mean only Hispanic ethnicity ("Unequal Treatment: Confronting Racial and Ethnic Disparities in Healthcare," IOM, 2003).
evidence-based guideline	Clinical guidelines, statements of recommendation, algorithms or materials created through an unbiased and transparent process of systematic review, appraisal and the best clinical findings to aid in delivery of optimum clinical care. The guidelines are based on the best available scientific evidence, professional standards or expert opinion.
Exchange	A government-regulated marketplace of insurance plans with different tiers or levels of coverage. Formerly <i>Marketplace</i> .
expedited appeal	An appeal of an adverse decision for coverage of urgent services.
external appeal	A request for a third-party, independent review of an organization's adverse determination.
factor	A scored item in an element. For example, an element may require the organization to demonstrate that its policies and procedures include four specific items; each item is a factor.
falsified documents	Documents provided by an applicant that have been redrafted, reformatted or fabricated, in whole or in part, with false or misleading information to substantiate compliance with NCQA standards and guidelines.

fraudulent information	Includes false or otherwise misleading oral statements made by an applicant, or by another Accredited, Certified or Recognized person on behalf of the applicant, to substantiate compliance with NCQA standards and guidelines or to otherwise influence the outcome of an NCQA Survey.
free-standing facility	An outpatient center that is separate from a hospital or other inpatient facility and whose primary focus is providing immediate or short-term medical services on an outpatient basis.
FSA	Flexible spending account. An employer-established, tax-advantage account funded by an employee to pay for qualified medical expenses with pre-tax dollars.
generic substitution	A bioequivalent copy of a pioneer (innovator) name-brand pharmaceutical whose patent has expired; typically, less expensive and sold under the common or generic name for the name-brand pharmaceutical.
goal setting	Establishing specific, measurable and time-targeted objectives.
grievance	A request for an organization to change a decision. See <i>appeal</i> .
group model	An HMO model of one or more physician group practices that are not owned by the HMO, but operate as independent partnerships or professional corporations. The HMO contracts with the medical group to provide a wide array of professional services and to pay for hospital care or care from specialists outside its network.
HA	Health appraisal. An instrument used to collect and analyze an individual's demographics, health-related behaviors, lifestyle and health conditions and compare the collected information with standardized data. Results are used to predict the individual's risks of developing specific medical conditions and provide information about how to reduce risks.
health coaching	Ongoing person-to-person support and motivation, with the goal of changing behavior and improving health and well-being.
health information line	Access to health-related inquiry resources (secure telephone service or electronic communication) staffed by licensed health care practitioners.
HEDIS	Healthcare Effectiveness Data and Information Set. A set of standardized performance measures designed to allow reliable comparison of the performance of managed health care plans.
high-impact specialist	A type of specialist who treats specific conditions that have serious consequences for the member and require significant resources.
high-risk/high-utilization program	A program aimed at members who frequently use emergency department services, have frequent hospitalizations or are high risk individuals (e.g., dually eligible for Medicare and Medicaid, have a high-risk pregnancy).
high-volume specialist	A specialist who treats a significant portion of an organization's membership.

HMO	Health maintenance organization. An organized health care system that is accountable for financing and delivering comprehensive health services to an enrolled population, and for assessing access and ensuring quality and appropriate care. Services are rendered by practitioners affiliated with the health care system. To receive reimbursement in an HMO, members must obtain all services from an affiliated provider and must comply with a predefined authorization system.
hospital case management program	A program designed to coordinate care for a member during inpatient admission and discharge planning.
HSA	Health savings account. A tax-advantage account used to pay for qualified medical expenses of the account holder, spouse or dependents.
IDS	Integrated delivery system. A group of hospitals, physicians, other providers, insurers or community agencies working together to coordinate and deliver a broad spectrum of services to their community.
inbound communications	Conversations initiated by a member or practitioner via telephone, electronic communication, fax or in person.
incentive	A reward earned by engaging in a suggested activity.
independent external review	A review performed by an independent review organization, typically after an organization's appeal process has been completed. See <i>IRO</i> .
independent relationship	An organization directs its members to see a specific practitioner or group of practitioners, including all practitioners that members may select as primary care practitioners. Not synonymous with <i>independent contract</i> .
inpatient	A setting where patients are admitted for diagnostic, therapeutic (surgical and nonsurgical), rehabilitative or psychiatric services provided by or under the supervision of physicians on a 24-hour basis.
interactive contact	A reciprocal action between an organization and a member that focuses on self-management support or health education.
intervention	A planned and defined action based on an assessment, taken to increase the probability of a desired outcome (e.g., include phone calls, emails, mailings, coaching, home visits, advice, reminders, tools and biometric devices provided to help manage health or disease).
IPA model	Independent practice association model. A health plan that contracts with physicians in independent practices, or one or more association of physicians in independent practices, or one or more multispecialty group practices, but which is organized predominantly around solo/single specialty practices.
IRO	Independent review organization. A third-party entity that provides unbiased, objective opinions based on clinical evidence.

least restrictive setting	The setting that is clinically appropriate for managing a member's condition and medical needs, and that aligns best with the member's preferences.
licensed independent practitioner	An individual permitted by law to provide services without direction or supervision, within the scope of the individual's licensure or certification and in accordance with individually granted clinical privileges.
locum tenens	A medical practitioner who temporarily takes the place of another practitioner. Locum practitioners are governed by regulatory bodies and include physicians, dentists, chiropractors, optometrists, pharmacists, social workers, psychologists.
long-term care facility	A facility that primarily provides skilled nursing care and rehabilitation services.
LTSS providers	Paid and unpaid members and organizations that provide long-term services and supports.
marketing materials	Advertising information (e.g., brochures, scripts, presentations) to members or prospective members.
materials	Prepared information that an organization provides to its members and practitioners, including written and electronic communication, websites, scripts, brochures, reviews and clinical guidelines or contracts or agreements with practitioners, delegates and vendors.
MBHO	Managed behavioral healthcare organization. A system of behavioral healthcare delivery that manages quality, utilization and cost of services and measures performance in the area of mental and substance abuse disorders.
medical evidence	The best-available clinical research in the diagnosis, treatment and management of disease to promote effectiveness and safety of care.
medical management system	A system designed to ensure that members receive appropriate health care services. Medical management systems include, but are not limited to, utilization management, quality improvement, case management and complaint and resolution.
medical necessity	Refers to services or supplies for diagnosing, evaluating, treating or preventing an injury, illness, condition or disease, based on evidence-based clinical standards of care. Medically necessary services are accepted health care services and supplies provided by health care entities, appropriate to evaluation and treatment of a disease, condition, illness or injury and consistent with the applicable standard of care. Determination of medical necessity is based on specific criteria. Note: This definition is based on CMS and American College of Medical Quality (ACMQ) definitions.
member	A person insured or otherwise provided coverage by a health insurance organization.

member participation rate	The number of members who received an initial assessment and at least one additional interactive service, divided by the number of patients identified as eligible for the program.
mixed model	An organization that uses a combination of Staff, Group, Network, IPA or Direct Contract models. If the majority of practitioners are included in one model, NCQA classifies the organization as that model.
monitor	A periodic or ongoing activity to determine opportunities for improvement or effectiveness of interventions.
narrow network	A tailored, tiered or high-performance network that has 80% or less overlap with the broadest network in the product/product line coming forward for Accreditation.
non-life threatening emergency	An emergency situation where clinical evidence shows that a person requires immediate care, but that lack of care would not lead to death.
nonsubordinate	An individual who is not under the authority of another person in the organization.
nonurgent request	A request for coverage of medical care or treatment for which application of the time periods for making a decision does not jeopardize the life or health of the member or the member's ability to regain maximum function, and would not subject the member to severe pain.
normal business hours	The period of the organization's stated hours of operation.
NPBD	National Practitioner Data Bank. A federally mandated agency that is the repository of information about settled malpractice suits and adverse acts, sanctions or restrictions against the practice privileges of a physician.
objective	A measurable target or result that a program aims to achieve within a time frame, based on verifiable evidence.
ombudsman	An impartial advocate appointed by the state to investigate and make recommendations for resolving a dispute.
open formulary	An organization covers the cost of all pharmaceuticals, regardless of whether they are on its formulary list.
opt in	A process for eligible members to choose to receive health care services or participate in a health care program (voluntary participation).
opt out	A process for eligible members to choose not to receive health care services or participate in a health care program (voluntary participation). Members are assumed to be in the program unless they opt out (opt-out enrollment).
organization representative	An organization employee or contractor who directs members to services.

overutilization	Providing clinical services that are not clearly indicated or providing services in excessive amounts or in a higher-level setting than is required.
participation rate	The number of eligible members participating in a program, divided by all members identified as eligible to be involved in the program, regardless of stratification or intervention level or enrollment process.
passive participation rate	The percentage of eligible patients who do not actively opt out of a program, regardless of their level of involvement.
patient identification	A process for identifying individuals as eligible for a program or set of services, using criteria that are often condition-specific.
patient participation rate	The percentage of eligible patients involved in a DM program, regardless of their level of involvement in the program. Varies by participation process (active vs. passive).
patient safety	Interventions that reduce the risk of adverse events related to exposure to medical care across a range of diagnoses or conditions.
PBM	Pharmacy benefit management [organization]. An organization contracted to recommend, develop or pay pharmaceutical claims.
PCMH	Patient-centered medical home. A health-care setting that facilitates partnerships between patients and their personal physicians and, when appropriate, with the patient's family. Care is facilitated by registries, information technology, the exchange of health information and by other means, to ensure that patients get the care necessary, when and where they need and want it, in a culturally and linguistically appropriate manner.
peer review	Evaluation or review of colleague performance by professionals with similar types and degrees of expertise (e.g., evaluation of a physician's credentials and practice by another physician).
performance measure	A quantifiable measure of performance.
performance/program goal	A desired level of achievement measured by specific objectives and targets. Goals may be expressed as desired minimum performance levels (thresholds), industry-best performance (benchmarks) or permitted variance from the standard. Performance goals usually are not static, but change as performance improves or as the standard of care is refined.
person-centered planning	A philosophy that involves viewing, listening to, and supporting a member, based on their strengths, abilities, aspirations and preferences, to make decisions to maintain a life that is meaningful to them.
plan sponsor	• The employer, in the case of an employee benefits plan established or maintained by a single employer. • The employee organization, in the case of an employee benefits plan established or maintained by an employee organization.

- The association, committee, joint board of trustees or similar group of representatives of parties who establish or maintain the benefits plan, in the case of an employee benefits plan established or maintained by two or more employers, or jointly by one or more employers and one or more employee organization.
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plan sponsor certification A written statement that the organization agrees to the requirement for protection of PHI.

planned transition An arranged change in a member's treatment or care setting (e.g., elective surgery, a decision to enter a long-term care facility).

policies and procedures A formal documented process adopted by the organization that describes the course of action the organization will follow and the methods that will be carried out to achieve the policy objectives. If the scope of review indicates that policies and procedures are reviewed, a formal policy and procedure document must be provided as evidence to demonstrate performance. Policies and procedures must include an effective date.

population based A study in which the subjects are drawn from a defined population that is representative of the entire eligible population.

POS Point of service. Coverage options that combine HMO features and out-of-network coverage with economic incentives for using network practitioners. POS options may be offered by health plans or indemnity insurers. In this type of health plan, members may choose to receive services either within the health plan's health care system (from an in-network practitioner) or outside the health plan's health care delivery system (from an out-of-network practitioner).

post-service Assessing appropriateness of medical services on a case-by-case or aggregate basis after services were provided.

PPO network Preferred provider organization network. An organization that contracts with independent providers at a discount for services.

practice One or a group of practitioners at a single geographic location who practice together. *Practicing together* means that, for all the practitioners in a practice:

- The single site is the location of practice for at least the majority of their clinical time.
- The nonphysician staff follow the same procedures and protocols.
- Paper or electronic medical records for all patients treated at the practice site are available to and shared by all physicians, as appropriate.
- The same systems, whether electronic (computer) or paper-based, and procedures support both clinical and administrative functions (e.g., scheduling time, treating patients, ordering services, prescribing, keeping medical records, follow-up).

practice independently	Practitioners who treat members without the supervision of another practitioner.
practice site	An office or facility where one or more practitioners provide care or services.
practice-based assessment	An evaluation of screening or treatment provided by practitioners or practices to members who represent a specific population.
practitioner	A licensed or certified professional who provides medical care or behavioral healthcare services.
preservice appeal	An appeal of an adverse decision for coverage of a service in advance of the member obtaining care or services.
preventive health services	Health care services designed for prevention and early detection of illness in asymptomatic people, generally including routine physical examinations, tests and immunizations.
primary care	Routine care of individuals with common health problems and chronic illnesses that can be managed on an outpatient basis.
primary prevention	Activities undertaken to prevent the occurrence of a disorder, disease or condition by eliminating causative agents, removing risk factors and enhancing patient competence (e.g., exercise, nutrition, alcohol and drug education, stress management programs).
primary source	The entity that originally conferred or issued a credential.
primary source verification	Verification of credentialing information directly from the entity (e.g., state licensing board) that conferred or issued the original credential.
prior review/ authorization/ certification/ determination	Prior assessment that proposed services (such as hospitalization) are appropriate for a particular patient and will be covered by an organization. Payment for services depends on whether the patient and the category of service are covered by the member's benefit plan.
product line	A distinct population (commercial, Exchange, Medicare, Medicaid) brought forward by an organization for evaluation.
professional standards of care	Stated ethical or legal requirements to exercise the level of care, diligence and skill prescribed in a professional's code of practice.
program	A collection of services and activities to manage member health. See <i>services</i> .
program scope	The extent of involvement of an organization's departments and units in the QI program and in organizationwide activities. Also refers to clinical, service and patient-experience activities.
provider	An institution or organization that provides services, such as a hospital, residential treatment center, home health agency or rehabilitation facility.

provisional credentialing	A type of credentialing that enables someone to practice as a health care or education practitioner, with certain restrictions.
prudent layperson	An individual who is without medical training and draws on practical experience to decide if there is a need to seek emergency medical treatment.
purchaser	An entity (e.g., health plan, provider) that contracts with an organization to provide case management services.
push messaging	Messages using telephone, short message service (SMS) messages, email, multimedia messaging, cell broadcast, picture messages and automated surveys with the intent of providing health care information.
QA	Quality assurance. A formal set of activities to review and safeguard the quality of medical services provided. Includes quality assessment and implementation of corrective actions to address deficiencies identified in the quality of care and services provided to individuals or populations.
QI	Quality improvement. Implementing corrective actions, based on assessment results, aimed at addressing identified deficiencies and improving outcomes.
qualitative analysis	An examination of the underlying reason for or cause of results, including deficiencies or processes that may present barriers to improvement or cause failure to reach a stated goal. Qualitative analysis must draw conclusions about why the results are what they are and involves staff responsible for executing a program or process. Also called a <i>causal, root cause</i> or <i>barrier</i> analysis.
quantitative analysis	A comparison of numeric results against a standard or benchmark, trended over time. Quantitative analysis must draw conclusions about what results mean. Unless specified, tests of statistical significance are not required, but may be useful when analyzing trends. NCQA does not require that results be trended for First Surveys.
quality assessment	Measurement and evaluation of the success of care and services offered to individuals, groups or populations.
quality of care	The degree to which health services increase the likelihood of desired health outcomes and are consistent with current professional knowledge.
race	A biological categorization of genetically transferred physical characteristics (e.g., skin color, eye color, hair color, bone and jaw structure).
received setting	The setting responsible for the member's care after a transition.
recognized source	An organization that develops or disseminates evidence-based clinical practice guidelines, such as a professional medical association, voluntary health organization, NIH Centers and Institutes.

records or files	Case management, UM denial and appeal or CR files that show direct evidence of action or performance in an element.
relevant outcome measure	The desired result of evidence-based interventions. Measures may be short term (e.g., effects of a medication) or long term (e.g., effects of a case management program on a specific member population).
relevant process measure	A clinical process or procedure that has a direct or indirect effect on the health outcomes of a specific member population.
rental/leased network	A network of practitioners that leases its panel to another network or insurer to expand practitioner access and negotiate discounted fees. Also referred to as a “brokerage leased network.”
reports	Aggregated sources of evidence of action or performance on an element, including management reports; key indicator reports; summary reports from member reviews; system output giving information such as number of member appeals; minutes; and other documentation of actions that the organization has taken.
residential	A facility, or a discrete part of a facility, that provides a 24-hour, therapeutically planned and professionally staffed group living and learning environment to live-in residents who require psychiatric care or substance abuse treatment but do not require acute medical care.
scope of review	For an element, the extent of the organization’s programs that NCQA evaluates. See the <i>Policies and Procedures</i> for additional information about the scope of review.
self-management tool	An interactive tool that helps members determine health risk factors, provides guidance on health issues and recommends ways to improve health, or supports reducing risk or maintaining low risk.
secondary prevention	Activities undertaken to ensure early identification and prompt treatment of an illness or disorder, with the goal of reducing the prevalence of the condition and shortening its duration (e.g., pre-term pregnancy program, early case finding, depression screening in the elderly).
sending setting	The setting responsible for care before a transition.
sensitive information	Includes PHI, as defined under HIPAA regulations, and other personal, private health information.
services	Activities or interventions in which individuals can participate to reach a specified health goal.
setting	The location where care is rendered (e.g., hospital, residential, ambulatory facility).

SPMI	Serious [and] persistent mental illness. A diagnosable mental, behavioral or emotional disorder (e.g., schizophrenia, bipolar disorder, major depression) resulting in functional impairment that substantially interferes with or limits one or more major activities (e.g., maintaining interpersonal relationships, activities of daily living, self-care, employment, recreation). An organization may use its state’s definition of SPMI.
staff model	Practitioners are direct employees of an organization, are salaried and have offices in the organization’s buildings.
step-therapy protocol	Specifies the sequence in which different pharmaceuticals are prescribed for a condition. If a patient does not obtain the desired clinical effect or experiences undesirable side effects at one step, pharmaceutical choices may be tried at another step. For some conditions, step-therapy protocols may also be “additive,” combining multiple steps for more aggressive treatment of a condition or for treatment at different or advanced stages of a disease.
stratification	Classification according to disease severity or other clinical criteria (e.g., readiness to change or other risk factors).
systemic identification	Use of a rules-based, consistent, population-based process of identification.
TDD/TTY	Telecommunications device for the deaf/teletypewriter. An electronic device for text communication via a telephone line, used when one or more parties have hearing or speech difficulties.
therapeutic interchange	Dispensing a pharmaceutical that is different from the one prescribed for a given condition. Interchange may occur within or across drug classes used to treat the condition. Interchanged pharmaceuticals are deemed to have similar therapeutic objectives but may have different mechanisms of action, side-effect profiles and other properties. Requires the prescribing physician’s explicit authorization. Also called <i>therapeutic substitution</i> or <i>therapeutic exchange</i> .
tiered formulary	A list of covered pharmaceuticals with varying copayments, categorized by preference level. Preferred pharmaceuticals have a lower copay than nonpreferred pharmaceuticals.
tiered network	A network with different cost-sharing rates for different provider levels.
transition	Movement between care settings (e.g., from home to hospital) as condition and care needs change during the course of a chronic or acute illness.
transition process	The period from identifying a member at risk for a care transition through completion of a transition.
transitional case management program	A program for evaluating and coordinating the post-hospitalization needs of patients who may be at risk of rehospitalization.

treating practitioner	In the context of denial notices, where NCQA requires that the organization notify the treating practitioner of a denial, the practitioner who requested the care, service or procedure subject to medical necessity review. If a practitioner refers a member for a treatment, service or procedure, for preservice requests, either the referring practitioner or the practitioner who will ultimately provide the treatment, service or procedure may be considered the treating practitioner.
triage and referral	A system of assessing a member's needs, determining the appropriate level of service and connecting the member with a service. Assessment that occurs at the treatment source is considered direct access to care, not triage and referral.
UM	Utilization management. Evaluating and determining coverage for and appropriateness of medical care services, as well as providing needed assistance to clinicians or patients, in cooperation with other parties, to ensure appropriate use of resources.
UM criteria	Objective, quantifiable guidelines used to assess the appropriateness of specific health care decisions and services.
understandability	The language and content of a document is written so that its meaning is easily comprehended by the target audience. Testing for understandability could include a reading level assessment.
underutilization	Failure to provide appropriate or indicated services, or provision of an inadequate quantity or a lower level of services than required.
unplanned transition	Unexpected movement from one care setting to another (e.g., hospitalization resulting from an emergency).
upon enrollment	The date when a member enrolls in the organization.
urgent request	A request for medical care or services where application of the time frame for making routine or non-life threatening care determinations: • Could seriously jeopardize the life or health of the member or the member's ability to regain maximum function, based on a prudent layperson's judgment, or • Could seriously jeopardize the life, health or safety of the member or others, due to the member's psychological state, or • In the opinion of a practitioner with knowledge of the member's medical or behavioral condition, would subject the member to adverse health consequences without the care or treatment that is the subject of the request.
usability	A measure of an interface or product's effectiveness, efficiency and adequacy.
usual care providers	LTSS providers, primary care practitioners or specialists responsible for a member's care.

usual setting	The setting where a member receives regular care; this may be the member's home or a residential care facility.
utilization review	A formal evaluation of the coverage, medical necessity, efficiency or appropriateness of health care services and treatment plans.
valid methodology	A data collection technique that produces consistent, reproducible results and shows a clear relationship between collected data and conclusions drawn from analysis of the data.
vendor relationship	A relationship between a client organization and another entity, where the organization obtains a product, solution or system from the entity and maintains control over the implementation and manner and use of the entity's product or services to perform a function.
well-being	At a minimum, includes the presence of positive emotions and moods (e.g., contentment, happiness), absence of negative emotions (e.g., depression, anxiety), satisfaction with life, fulfillment and positive functioning.

Appendix 6

Standards by Product Line and Survey Type

Appendix 7

MA Deeming Module Requirements by Deeming Category

